

NCLEX-PN

Question Bank

Volume 1 — Questions 1-2000

2,000 Practice Questions with Detailed Answers & Explanations

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Coordinated Care

Prioritization

1. A practical nurse begins a shift on a medical-surgical unit and receives report on four clients. Which client should the nurse assess first?

- A. A client 1 day after cataract surgery who reports mild eye itching
- B. A client with heart failure who has new crackles and oxygen saturation of 88%
- C. A client with osteoarthritis requesting a PRN analgesic for knee pain rated 6/10
- D. A client with type 2 diabetes whose prebreakfast blood glucose is 168 mg/dL

Correct Answer: B

New crackles with an oxygen saturation of 88% suggests impaired oxygenation and possible fluid overload, which is the highest priority because airway and breathing problems come first. Mild itching after cataract surgery is less urgent, pain medication can be given after unstable findings are addressed, and a glucose of 168 mg/dL is elevated but not immediately life-threatening.

2. The practical nurse is caring for four clients in a long-term care facility. Which situation requires immediate follow-up?

- A. A resident with dementia is wandering into another resident's room
- B. A resident with chronic constipation has not had a bowel movement in 2 days
- C. A resident receiving warfarin has sudden slurred speech and right arm weakness
- D. A resident with chronic back pain asks for a heating pad before lunch

Correct Answer: C

Sudden slurred speech and unilateral weakness may indicate a stroke or bleeding complication and require immediate action. Wandering needs prompt redirection for safety, but it is not as emergent as acute neurologic change. Constipation over 2 days and a request for heat therapy are lower priority.

3. A practical nurse is reviewing tasks to complete after morning medication pass. Which action should the nurse complete first?

- A. Return a phone call from a client's family member asking for an update
- B. Reassess a client 30 minutes after giving morphine for severe pain
- C. Restock the treatment cart with clean dressings
- D. Fax a routine referral form for physical therapy

Correct Answer: B

After administering morphine, the nurse must first evaluate the client's response and monitor for adverse effects such as oversedation or respiratory depression. The family phone call, supply restocking, and routine referral can occur after the client is reassessed.

4. The practical nurse is caring for four postpartum clients. Which client should be seen first?

- A. A client 12 hours postpartum with moderate lochia rubra and a firm fundus
- B. A client 1 day postpartum requesting instruction on bottle-feeding technique
- C. A client 2 hours postpartum with a boggy fundus and heavy vaginal bleeding
- D. A client 18 hours postpartum reporting perineal discomfort rated 4/10

Correct Answer: C

A boggy fundus with heavy bleeding suggests postpartum hemorrhage, which is an immediate threat to circulation and must be addressed first. The other findings are expected or nonurgent teaching and comfort needs.

5. A practical nurse in the emergency department receives these updates. Which client is the priority?

- A. A client with a forearm fracture who reports pain 8/10 despite a splint

- B.** A client with vomiting and diarrhea whose potassium level is 2.9 mEq/L
- C.** A client with renal colic waiting for discharge instructions
- D.** A client with a migraine asking for a darkened room

Correct Answer: B

A potassium level of 2.9 mEq/L is significantly low and increases the risk for dangerous cardiac dysrhythmias, so that client is the priority. Severe pain matters, but life-threatening electrolyte imbalance takes precedence. Discharge teaching and environmental comfort are lower priority.

- 6.** The practical nurse has just received report on four clients. Which client should the nurse assess first?
- A.** A client with chronic obstructive pulmonary disease whose oxygen saturation is 91%, the same as earlier today
 - B.** A client with a urinary tract infection who has a temperature of 100.4°F (38°C)
 - C.** A client 6 hours after thyroid surgery who reports a feeling of neck tightness and difficulty swallowing
 - D.** A client with a pressure injury who needs a scheduled dressing change

Correct Answer: C

Neck tightness and difficulty swallowing after thyroid surgery can signal swelling or bleeding that may rapidly obstruct the airway. The COPD oxygen saturation is unchanged from baseline, a mild fever with UTI is expected and less urgent, and a routine dressing change can wait until the airway concern is addressed.

- 7.** A practical nurse is making rounds in a rehabilitation unit. Which finding requires the most immediate action?
- A.** A client with a spinal cord injury says, "I have a pounding headache"
 - B.** A client with a below-knee amputation requests help transferring to a chair
 - C.** A client with a recent stroke becomes frustrated during speech exercises
 - D.** A client with multiple sclerosis asks when the next dose of medication is due

Correct Answer: A

In a client with spinal cord injury, a sudden pounding headache may indicate autonomic dysreflexia, which can become life-threatening if not managed promptly. The other issues are important but do not suggest an immediate physiologic emergency.

- 8.** The practical nurse is caring for four pediatric clients. Which client should the nurse assess first?
- A.** A toddler with gastroenteritis who has had one episode of emesis after oral fluids
 - B.** A school-age child with asthma who is using accessory muscles to breathe
 - C.** An adolescent with a fractured ankle who wants to know when lunch will arrive
 - D.** An infant with diaper rash who cries during cleaning

Correct Answer: B

Accessory muscle use is a sign of increased work of breathing and possible respiratory distress, making this child the highest priority. The other clients have discomfort or minor issues, but no sign of immediate airway or breathing compromise.

- 9.** A practical nurse on a mental health unit receives report on four clients. Which client should the nurse see first?
- A.** A client with depression who refuses breakfast but is sitting quietly in the day room
 - B.** A client with panic disorder who says, "I feel shaky when groups are crowded"
 - C.** A client with alcohol use disorder who is diaphoretic, tremulous, and seeing insects on the wall
 - D.** A client with insomnia asking whether the bedtime snack will be served soon

Correct Answer: C

Diaphoresis, tremors, and visual hallucinations suggest severe alcohol withdrawal, which can rapidly progress to seizures and cardiovascular instability. Refusing a meal, anxiety in groups, and a snack request are lower priority than a potentially life-threatening withdrawal state.

10. The practical nurse is reviewing these newly reported findings. Which one should the nurse address first?

- A.** A client with pneumonia reports pleuritic chest pain when coughing
- B.** A client with cirrhosis has ammonia level slightly above the previous day's value but is alert and oriented
- C.** A client receiving a blood transfusion develops chills and low back pain 15 minutes after the infusion starts
- D.** A client with cellulitis says the antibiotic caused mild nausea after lunch

Correct Answer: C

Chills and low back pain shortly after a blood transfusion begins suggest a transfusion reaction and require immediate action. Pleuritic pain with pneumonia may be expected, a mild lab trend without mental-status change is less urgent, and nausea after antibiotics is a lower-priority adverse effect.

11. A practical nurse is assigned four clients at the start of the shift. Which client should be assessed first?

- A.** A client with sickle cell disease who reports severe leg pain and has a temperature of 101.9°F (38.8°C)
- B.** A client with chronic kidney disease scheduled for routine daily weights
- C.** A client with stable angina asking when morning medications will be given
- D.** A client with eczema requesting lotion for dry skin

Correct Answer: A

In a client with sickle cell disease, fever raises concern for infection, and severe pain may indicate vaso-occlusive crisis. This combination requires prompt assessment because complications can escalate quickly. The other needs are routine or nonurgent.

12. The practical nurse is caring for four clients on a postoperative unit. Which client is the priority?

- A.** A client 8 hours after hip surgery with urine output of 30 mL over the last hour and stable vital signs

B. A client 24 hours after bowel surgery who has absent bowel sounds and mild abdominal distention

C. A client 2 hours after abdominal surgery who is restless, has a respiratory rate of 28/min, and oxygen saturation of 89%

D. A client 1 day after laparoscopic surgery who reports incisional pain rated 7/10

Correct Answer: C

Restlessness, tachypnea, and oxygen saturation of 89% indicate acute respiratory compromise and possible hypoxemia, which takes priority. Urine output of 30 mL/hr is borderline but not the most urgent with stable vital signs. Mild distention and absent bowel sounds can be expected early after bowel surgery, and pain management follows stabilization of breathing.

13. A practical nurse receives change-of-shift report on four clients with diabetes. Which client should the nurse assess first?

- A.** A client whose fingerstick glucose is 62 mg/dL and who is sweaty and confused
- B.** A client whose fingerstick glucose is 240 mg/dL and who is thirsty
- C.** A client who asks for a different breakfast tray because of food preferences
- D.** A client who needs teaching about rotating insulin injection sites

Correct Answer: A

A glucose of 62 mg/dL with diaphoresis and confusion indicates symptomatic hypoglycemia, which can quickly worsen and requires immediate treatment. Hyperglycemia at 240 mg/dL is important but usually less immediately dangerous than neuroglycopenic symptoms. Food preferences and teaching are lower priority.

14. The practical nurse is caring for clients in a clinic. Which telephone message should the nurse return first?

- A.** A client taking furosemide reports muscle cramps and palpitations
- B.** A client prescribed iron tablets reports dark stools
- C.** A client using a nicotine patch says the patch left mild skin redness
- D.** A client with seasonal allergies asks whether pollen counts are usually higher in the morning

Correct Answer: A

Muscle cramps and palpitations in a client taking furosemide may reflect hypokalemia with cardiac effects and should be addressed first. Dark stools are an expected effect of iron therapy, mild local patch redness is usually not urgent, and the allergy question is routine education.

15. A practical nurse is assigned to four clients in a skilled nursing facility. Which client should the nurse assess first?

- A.** A client with Parkinson disease who is 20 minutes late for a scheduled dose of carbidopa-levodopa
- B.** A client with heart failure who gained 2 lb since yesterday and reports increased shortness of breath when lying flat
- C.** A client with chronic venous insufficiency who has dry, flaky skin on both lower legs
- D.** A client with a healed stroke who wants assistance calling a family member

Correct Answer: B

Weight gain with worsening orthopnea suggests fluid overload and possible heart failure exacerbation, creating a higher risk for respiratory compromise. A late Parkinson medication dose should be addressed, but it is less urgent than signs of decompensating heart failure. Dry skin and a phone call request are nonurgent.

16. The practical nurse is caring for four clients on a medical unit. Which client should the nurse assess first?

- A.** A client with pancreatitis who reports abdominal pain of 7/10 and asks for the next PRN analgesic
- B.** A client with pneumonia whose oxygen saturation dropped from 95% to 90% and who is now more restless
- C.** A client with anemia who reports fatigue after walking to the bathroom
- D.** A client with a urinary catheter who asks when it will be removed

Correct Answer: B

The client with pneumonia has a change in respiratory status: decreasing oxygen saturation and new restlessness, which can indicate worsening hypoxia. Airway and breathing problems take priority over pain, expected fatigue, or routine questions. Choice A matters, but pain without signs of instability is not as urgent as respiratory decline.

17. A practical nurse in a long-term care facility is told about these situations during rounds. Which resident should the nurse see first?

- A.** A resident with chronic heartburn who asks for an antacid before dinner
- B.** A resident with dementia who is calling out and trying to remove a sweater
- C.** A resident with COPD who is more drowsy than usual and has respirations of 10/min after receiving an opioid 30 minutes ago
- D.** A resident with osteoarthritis who says both knees ache after physical therapy

Correct Answer: C

The priority is possible opioid-induced respiratory depression. Drowsiness plus a respiratory rate of 10/min, especially in a resident with COPD, suggests impaired ventilation and risk for rapid deterioration. The other findings need attention but are not as immediately life-threatening. Choice B may reflect discomfort or confusion, but it does not outweigh depressed respirations.

18. The practical nurse is reviewing the plan of care for four clients. Which action should the nurse complete first?

- A.** Reinforce teaching for a client newly prescribed a low-sodium diet
- B.** Obtain a sterile dressing setup for a scheduled wound change
- C.** Recheck a client who received sublingual nitroglycerin 5 minutes ago for chest pain
- D.** Document intake and output totals from the previous shift

Correct Answer: C

Chest pain requires prompt follow-up to determine whether symptoms are improving and whether additional action is needed. Reassessment after nitroglycerin is time-sensitive because ongoing ischemic pain may signal an emergency. Teaching, gathering supplies, and documentation are important but can wait. The likely trap is choosing documentation because it is overdue, but client condition comes first.

19. A practical nurse receives report on four clients in an outpatient infusion center. Which client is the priority?

- A.** A client receiving IV antibiotics who reports itching and has new wheezes
- B.** A client receiving iron infusion who says the IV site feels cool
- C.** A client receiving fluids for dehydration who asks to use the restroom
- D.** A client receiving chemotherapy who asks whether nausea medicine can be given early

Correct Answer: A

Itching with new wheezes during an infusion suggests an acute hypersensitivity reaction that threatens the airway. This requires immediate intervention. A cool IV site may indicate infiltration, which is important but usually not as urgent as airway compromise. The restroom request and nausea question are lower priority.

20. The practical nurse is caring for four clients in a rehabilitation center. Which client should the nurse assess first?

- A.** A client with a new colostomy who says the pouch leaked once during the night
- B.** A client recovering from a stroke whose blood pressure is 198/104 mm Hg and who now has a severe headache
- C.** A client with chronic low back pain who requests a warm blanket before therapy
- D.** A client with a healed femur fracture who wants help finding a misplaced hearing aid

Correct Answer: B

Severely elevated blood pressure with a new severe headache can indicate a hypertensive emergency or another acute neurologic problem, so this client is the priority. The colostomy leak, comfort request, and missing hearing aid are nonurgent in comparison. The key is recognizing a potentially unstable change rather than focusing on routine rehab needs.

21. A practical nurse on a surgical unit has these tasks to complete. Which should be done first?

- A.** Administer a scheduled stool softener to a client 2 days after surgery
- B.** Call central supply for additional IV tubing
- C.** Evaluate a client 1 hour after antiemetic administration because the client is still vomiting
- D.** Print discharge instructions for a client leaving later this afternoon

Correct Answer: C

Persistent vomiting despite treatment can lead to aspiration, fluid imbalance, and delayed recovery, so reassessment is the priority. Scheduled medications, supply issues, and discharge paperwork can usually wait briefly. The best-answer reasoning is to address a current client response to treatment before routine tasks.

22. The practical nurse is caring for four clients at the beginning of the shift. Which client should be seen first?

- A.** A client with liver disease who is slightly more forgetful than yesterday and has hand-flapping tremors
- B.** A client with gout who requests an ice pack for toe pain
- C.** A client with psoriasis who asks when the medicated shampoo will be available
- D.** A client with cataracts who wants assistance signing a menu form

Correct Answer: A

Increasing forgetfulness and asterixis in a client with liver disease suggest worsening hepatic encephalopathy, which can progress and requires prompt assessment. The other clients have comfort or routine needs. A likely trap is underestimating mild mental-status changes, but subtle neurologic deterioration can signal significant instability.

23. A practical nurse working in a pediatric clinic receives these calls from parents. Which call should the nurse return first?

- A.** A parent says a child taking amoxicillin developed a diffuse rash and swelling of the lips
- B.** A parent says a child with eczema is scratching more at bedtime
- C.** A parent asks whether a prescribed multivitamin can be mixed with juice
- D.** A parent reports a child with a cast says the skin feels itchy under it

Correct Answer: A

Rash plus lip swelling after amoxicillin suggests an allergic reaction with possible airway involvement, so this call takes priority. Eczema itching, a medication administration question, and itching under a cast are less urgent. The key is recognizing signs that could progress to anaphylaxis.

24. The practical nurse is assigned four clients on a telemetry unit. Which client should the nurse assess first?

- A.** A client with heart failure whose ankles are more swollen than yesterday but who denies shortness of breath
- B.** A client with atrial fibrillation whose monitor shows a heart rate of 132/min and who reports feeling lightheaded
- C.** A client with hypertension whose scheduled antihypertensive medication is due in 20 minutes
- D.** A client with a permanent pacemaker who asks when the dressing can be removed

Correct Answer: B

A rapid heart rate with lightheadedness suggests decreased cardiac output and possible hemodynamic instability, making this the priority. Increased ankle edema without dyspnea needs follow-up but is less immediate. A medication that is due soon and a routine question are lower priority. The likely trap is choosing the heart failure client because edema is concerning, but symptomatic tachyarrhythmia is more urgent.

25. A practical nurse on a maternal-newborn unit receives report on four clients. Which client should the nurse assess first?

- A.** A newborn 6 hours old with acrocyanosis and a temperature of 36.8°C (98.2°F)
- B.** A postpartum client who says one breast feels firm and tender before feeding
- C.** A newborn with jitteriness whose mother has diabetes and whose heel-stick glucose is 38 mg/dL
- D.** A postpartum client who asks for help using a sitz bath

Correct Answer: C

A jittery newborn with a glucose of 38 mg/dL may be experiencing symptomatic hypoglycemia and needs prompt intervention. Acrocyanosis in an otherwise stable newborn can be expected, breast fullness before feeding is common, and sitz bath assistance is routine. The best answer focuses on the immediate risk of neurologic harm from low blood glucose.

26. A practical nurse on a medical-surgical unit is deciding whom to assess first after lunch. Which client is the priority?

- A.** A client with chronic kidney disease who reports nausea after eating half of the meal
- B.** A client with ulcerative colitis who has had six loose stools since morning and now reports dizziness when standing
- C.** A client with a hernia repair who says the prescribed oral analgesic did not fully relieve pain
- D.** A client with hyperlipidemia who asks for written information about diet choices

Correct Answer: B

Frequent diarrhea plus dizziness on standing suggests volume depletion and possible orthostatic compromise, which can worsen quickly. The other clients have discomfort, mild nausea, or teaching needs. A common trap is focusing on pain control, but signs of circulatory instability take priority.

27. The practical nurse is caring for four clients in a skilled nursing facility. Which situation requires the most immediate action?

- A.** A resident with Parkinson disease suddenly becomes unable to swallow water and has a wet-sounding voice
- B.** A resident with chronic constipation asks for prune juice with breakfast
- C.** A resident with hearing impairment says the television volume is too low
- D.** A resident with insomnia reports waking up twice during the night

Correct Answer: A

Sudden difficulty swallowing with a wet voice suggests acute aspiration risk and possible airway compromise. This finding is far more urgent than constipation management, hearing needs, or sleep concerns. The priority is preventing respiratory complications and assessing airway protection.

28. A practical nurse is assisting with care in an oncology unit. Which client should be assessed first?

- A.** A client receiving radiation therapy who has dry skin in the treatment area
- B.** A client with neutropenia whose temperature is 100.8°F (38.2°C)
- C.** A client with alopecia who asks about head coverings
- D.** A client with nausea who wants crackers before medication time

Correct Answer: B

Fever in a client with neutropenia may signal infection with limited immune response and requires prompt assessment. Dry skin from radiation, questions about appearance changes, and mild nausea are important but not as urgent. The critical point is that even a modest fever can be significant in an immunocompromised client.

29. The practical nurse is planning the order of care for four home health clients. Which client should the nurse visit first?

- A.** A client with chronic venous insufficiency who needs reinforcement about leg elevation
- B.** A client 3 days after starting a diuretic who reports a 4-pound weight loss and improved ankle swelling
- C.** A client with a stage 2 pressure injury who needs a dressing change later this morning
- D.** A client with type 1 diabetes who has influenza, is vomiting, and reports moderate ketones in the urine

Correct Answer: D

A client with type 1 diabetes who is vomiting and has moderate urine ketones is at risk for diabetic ketoacidosis and needs prompt assessment. The other clients are stable or improving, and the dressing change is important but not as urgent. The best-answer reasoning is to prioritize the client with signs of a potentially life-threatening metabolic complication.

30. A practical nurse in an assisted-living facility is notified of these client situations. Which client should the nurse assess first?

- A.** A client with type 2 diabetes who is sweating, shaky, and has a capillary blood glucose of 54 mg/dL
- B.** A client with chronic obstructive pulmonary disease who asks for the next scheduled inhaler treatment in 30 minutes
- C.** A client with rheumatoid arthritis who reports morning hand stiffness rated 6/10
- D.** A client with constipation who has not had a bowel movement in 2 days and requests a laxative

Correct Answer: A

The client with diaphoresis, shakiness, and a blood glucose of 54 mg/dL is the priority because these are signs of symptomatic hypoglycemia, which can rapidly progress to seizures, loss of consciousness, and neurologic injury if not addressed immediately. Choice B involves a need that is time-sensitive but not currently emergent. Choices C and D reflect discomfort and a nonurgent elimination concern. The key prioritization principle is to address the most immediate threat to physiologic stability first.

31. A practical nurse on a medical unit receives these updates from the assistive personnel. Which client should the nurse assess first?

- A.** A client with heart failure who gained 2 lb since yesterday and has 1+ ankle edema
- B.** A client 6 hours after thyroid surgery who is restless and says the neck dressing feels tight
- C.** A client with gastritis who asks for a PRN antacid after breakfast
- D.** A client with a knee immobilizer who needs help getting to the bathroom

Correct Answer: B

A tight neck dressing and restlessness after thyroid surgery raise concern for postoperative bleeding or airway compression, which can rapidly become life-threatening. That makes this client the priority. Mild weight gain and ankle edema need follow-up but are not as immediately dangerous. The antacid request and toileting assistance are important, but they do not outweigh a possible airway emergency.

32. At the start of the shift, the practical nurse reviews these client findings. Which client should be seen first?

- A.** A client with a recent below-knee amputation who reports incisional pain rated 8/10
- B.** A client with cirrhosis who has not finished breakfast because of poor appetite
- C.** A client with sickle cell disease who reports new left-sided chest pain and shortness of breath
- D.** A client with cellulitis who asks when the next antibiotic dose is due

Correct Answer: C

New chest pain and shortness of breath in a client with sickle cell disease suggest acute chest syndrome, a potentially life-threatening complication requiring immediate assessment. Severe postoperative pain is important but is not the first priority over impaired oxygenation or cardiopulmonary compromise. Poor appetite and a medication-time question are lower priority.

33. The practical nurse is deciding which task to complete first during morning rounds. Which action has the highest priority?

- A.** Reposition a client who is due for turning and skin care in 20 minutes

- B.** Reinforce incentive spirometer teaching for a client scheduled for discharge tomorrow

- C.** Check a client with diabetes who received regular insulin and is now pale and diaphoretic before breakfast arrives

- D.** Refill the water pitcher for a client on a regular diet

Correct Answer: C

Pallor and diaphoresis after insulin administration suggest hypoglycemia, which can progress quickly and requires immediate assessment and intervention. Repositioning and teaching are important but can wait briefly. Refilling a water pitcher is routine care. The key trap is choosing the scheduled task rather than the client showing signs of an acute complication.

34. A practical nurse in a long-term care facility is notified of four resident situations. Which resident should the nurse evaluate first?

- A.** A resident with heart failure who has crackles at the lung bases and is more short of breath while sitting in a chair
- B.** A resident with chronic constipation who reports no bowel movement for 3 days
- C.** A resident with glaucoma who says the eyedrops sting briefly after administration
- D.** A resident with osteoarthritis who requests acetaminophen before afternoon activities

Correct Answer: A

Increased shortness of breath with crackles suggests worsening fluid overload and impaired gas exchange, making this the priority. Constipation and pain control are important but not immediately life-threatening. Brief stinging with eyedrops can occur and is not the most urgent issue here.

35. A practical nurse is caring for four clients in a postoperative unit. Which client should the nurse assess first?

- A.** A client 1 day after abdominal surgery with absent bowel sounds and mild nausea
- B.** A client 4 hours after a transurethral resection who has bright red urine with several small clots in the drainage bag

C. A client 8 hours after hip surgery who reports calf pain when the foot is dorsiflexed and has one leg more swollen than the other

D. A client after laparoscopic surgery who has shoulder pain rated 5/10

Correct Answer: C

Calf pain with unilateral swelling after surgery suggests possible deep vein thrombosis, which is urgent because of the risk for pulmonary embolism. Mild nausea and absent bowel sounds can be expected after abdominal surgery. Shoulder pain after laparoscopy is commonly related to residual gas. Bright red urine with a few small clots may occur early after a transurethral resection, so it is concerning but less immediately dangerous than suspected thromboembolism.

36. During medication pass, the practical nurse learns about these client concerns. Which requires immediate follow-up?

A. A client receiving warfarin says the gums bled a little during toothbrushing this morning

B. A client receiving digoxin reports seeing yellow halos and has a pulse of 52/min

C. A client taking iron says the stools are dark green to black

D. A client taking an oral antibiotic reports a mild metallic taste

Correct Answer: B

Yellow halos and bradycardia are classic signs of possible digoxin toxicity and require immediate follow-up. Mild gum bleeding with warfarin warrants assessment but is less urgent than likely toxicity affecting cardiac conduction. Dark stools with iron and a metallic taste with some antibiotics can be expected effects. The key is recognizing the combination of visual changes and a low pulse.

37. A practical nurse in a rehabilitation unit must decide whom to assess first after receiving report. Which client is the priority?

A. A client with multiple sclerosis who reports increasing fatigue by late afternoon

B. A client with a spinal cord injury who has a pounding headache, flushed skin above the injury, and blood pressure of 188/96 mm Hg

C. A client recovering from knee replacement who says the CPM machine is uncomfortable

D. A client with peripheral neuropathy who asks for help trimming toenails

Correct Answer: B

A pounding headache, severe hypertension, and flushing in a client with spinal cord injury suggest autonomic dysreflexia, which is a medical emergency. The other situations are important but not immediately life-threatening. This item tests recognition of a high-risk syndrome that can be missed if the nurse focuses only on pain or comfort complaints.

38. The practical nurse is helping manage care for four clients on an evening shift. Which client should be assessed first?

A. A client with a chest tube whose drainage increased from 20 mL to 40 mL over the last hour

B. A client with a history of seizures who is difficult to arouse and has noisy respirations after receiving a sedative 45 minutes ago

C. A client with dermatitis who reports itching under an intact dressing

D. A client with gastroesophageal reflux who asks whether supper can be reheated

Correct Answer: B

Difficulty arousing the client with noisy respirations after a sedative suggests oversedation and compromised airway. Airway and breathing take priority. A moderate increase in chest tube drainage should be monitored, but it is not the most urgent finding presented. Itching and meal questions are lower priority.

39. A practical nurse receives change-of-shift report on four clients. Which client should the nurse see first?

A. A client with chronic kidney disease whose arteriovenous fistula has a palpable thrill and asks when dialysis starts

B. A client with peptic ulcer disease who had one dark stool this morning and now reports sudden severe abdominal pain with a rigid abdomen

C. A client with shingles who wants another blanket because the room feels cool

D. A client with benign prostatic hyperplasia who voided 200 mL and says the stream is still weak

Correct Answer: B

Sudden severe abdominal pain with a rigid abdomen suggests possible perforation and peritonitis, which is an emergency. The fistula finding is expected because a palpable thrill indicates patency. Feeling cool and weak urinary stream are nonurgent compared with an acute abdomen.

40. The practical nurse has time to complete only one assessment immediately. Which client should be assessed first?

- A.** A client with hyperthyroidism who is anxious and asks why the room feels warm
- B.** A client receiving a blood transfusion who developed chills, low back pain, and a sense of uneasiness 10 minutes after it started
- C.** A client with chronic venous insufficiency who wants help rewrapping compression bandages
- D.** A client with migraine who asks for the lights to be dimmed

Correct Answer: B

Chills, low back pain, and uneasiness shortly after a transfusion begins suggest an acute transfusion reaction, which requires immediate action. The other clients are uncomfortable but stable. The timing after the transfusion starts is the key cue that makes this the priority.

41. A practical nurse in a mental health setting is told about these client behaviors. Which client needs immediate assessment first?

- A.** A client with depression who stays in bed and refuses breakfast
- B.** A client with mania who is pacing and speaking rapidly
- C.** A client admitted for alcohol withdrawal who is diaphoretic, tremulous, and newly disoriented
- D.** A client with anxiety who asks for a quiet place to sit

Correct Answer: C

Diaphoresis, tremors, and new disorientation in alcohol withdrawal may indicate worsening withdrawal with risk for seizures or delirium tremens, requiring immediate assessment. Depression, mania, and anxiety all need care, but this client has the most acute physiologic risk.

42. The practical nurse is caring for four clients in a rural emergency department waiting area. Which client should be taken to a treatment room first?

- A.** A client with a wrist fracture who rates pain as 9/10 and has intact finger movement
- B.** A client with gastroenteritis who has dry lips and says, "I feel weak when I stand up"
- C.** A client with a kidney stone who is vomiting but is alert and answering questions appropriately
- D.** A client with facial droop that began 30 minutes ago and new slurred speech

Correct Answer: D

Sudden facial droop and slurred speech suggest acute stroke, and rapid assessment is critical because time-sensitive treatment may be needed. The other clients are symptomatic but do not show the same threat to neurologic function. The trap is choosing the highest pain rating rather than the most time-critical condition.

43. A practical nurse on a pediatric unit is told about these clients. Which child should the nurse assess first?

- A.** A child with croup who has inspiratory stridor at rest and suprasternal retractions
- B.** A child with otitis media who cries when swallowing
- C.** A child with gastroenteritis who had one loose stool after lunch
- D.** A child with a healing arm fracture who says the cast edges feel rough

Correct Answer: A

Stridor at rest with suprasternal retractions indicates significant upper airway obstruction and requires immediate assessment. The other children are uncomfortable but stable. Stridor only when upset would be less urgent; stridor at rest is the key finding.

44. The practical nurse is organizing care for four clients in a dialysis center. Which client should be assessed first?

- A.** A client whose blood pressure is 146/88 mm Hg and who says the treatment chair is uncomfortable
- B.** A client who completed dialysis and now reports muscle cramps in both legs

- C. A client whose arteriovenous access is bleeding through the dressing after the needles were removed
- D. A client waiting for dialysis who asks whether a family member can sit nearby

Correct Answer: C

Active bleeding from an arteriovenous access can lead to significant blood loss and requires immediate assessment and intervention. Muscle cramps after dialysis can occur and should be addressed, but ongoing bleeding is the higher priority. The blood pressure given is not critical, and the family question is nonurgent.

45. A practical nurse in a skilled nursing facility receives these reports during the lunch hour. Which client should the nurse assess first?
- A. A client with Parkinson disease who is coughing during meals and has a wet, gurgling voice after swallowing
 - B. A client with chronic heart failure who has 2+ ankle edema before the scheduled afternoon diuretic
 - C. A client with osteoarthritis who reports knee pain rated 7/10 after walking to the dining room
 - D. A client with dementia who is upset because another resident took the seat they usually use

Correct Answer: A

A wet, gurgling voice and coughing with swallowing suggest impaired airway protection and possible aspiration, which makes this the highest priority. Airway problems come before discomfort, chronic edema, or behavioral upset. Choice B needs follow-up, but ankle edema before a scheduled diuretic is not as immediately dangerous as signs of aspiration. Choice C involves pain, and choice D involves emotional distress, but neither is more urgent than a potential airway compromise.

46. The practical nurse is receiving morning report on four clients. Which client should the nurse assess first?
- A. A client with type 1 diabetes who is awake, reports shakiness, and has a capillary glucose of 54 mg/dL
 - B. A client with chronic obstructive pulmonary disease who asks for assistance setting up breakfast

- C. A client 2 days after cataract surgery who says the prescribed eye drops sting briefly
- D. A client with rheumatoid arthritis who requests help buttoning a shirt

Correct Answer: A

A blood glucose of 54 mg/dL with shakiness indicates symptomatic hypoglycemia, which can quickly progress to seizures, loss of consciousness, or other serious complications if not treated promptly. The other clients have needs, but none show an immediate physiologic threat. The common trap is choosing the client with COPD, but needing breakfast setup is not as urgent as active hypoglycemia.

47. A practical nurse is planning the order of care for four clients. Which client should be seen first?
- A. A client with a new colostomy who is embarrassed about emptying the pouch
 - B. A client receiving IV potassium who reports burning at the insertion site and has redness along the vein
 - C. A client with chronic back pain who requests a warm compress
 - D. A client scheduled for discharge who needs printed instructions reviewed

Correct Answer: B

IV potassium is irritating to veins, and burning with redness suggests infiltration or phlebitis that requires prompt assessment and intervention. Potassium administered incorrectly can cause tissue injury, and the infusion may need to be stopped. The other concerns are important but not as time-sensitive. A likely trap is choosing the discharge teaching task because it has a deadline, but physiologic risk comes first.

48. After receiving report, the practical nurse must decide which postoperative client to check first. Which client is the priority?
- A. A client 12 hours after bowel surgery with pain rated 7/10 despite ordered PRN medication 2 hours ago
 - B. A client 24 hours after hernia repair who has not yet passed flatus
 - C. A client 6 hours after a carotid endarterectomy who has frequent swallowing and a firm area developing at the neck incision

D. A client 1 day after breast surgery who asks when the drain will be removed

Correct Answer: C

Frequent swallowing and a firm area at the neck incision after carotid surgery suggest a hematoma, which can rapidly compromise the airway. That makes this the highest priority. Unrelieved pain needs follow-up, but it is not as immediately life-threatening. The trap is focusing on pain severity in choice A instead of recognizing early airway danger in choice C.

49. The practical nurse in a clinic receives four phone messages. Which call should the nurse return first?

A. A client taking furosemide says, "I'm having leg cramps at night."

B. A client with asthma says, "I used my rescue inhaler three times in the last hour and I'm still short of breath."

C. A client with eczema says, "The new lotion feels greasy."

D. A client with osteoarthritis says, "My refill is not ready yet."

Correct Answer: B

Persistent shortness of breath despite repeated use of a rescue inhaler suggests worsening bronchospasm and possible respiratory compromise. This requires immediate follow-up. Leg cramps with furosemide may indicate electrolyte loss, but the client with uncontrolled breathing symptoms is more urgent. The other concerns are routine and can wait.

50. A practical nurse is rounding on four antepartum clients. Which client should the nurse assess first?

A. A client at 10 weeks' gestation with nausea who asks whether small frequent meals may help

B. A client at 24 weeks' gestation who reports a new gush of fluid and feels something "bulging" in the vagina

C. A client at 30 weeks' gestation with mild ankle swelling at the end of the day

D. A client at 36 weeks' gestation who wants to know when to pack a hospital bag

Correct Answer: B

A gush of fluid with something bulging in the vagina suggests possible umbilical cord prolapse or another obstetric emergency requiring immediate assessment. This threatens fetal oxygenation and cannot wait. The other clients have expected discomforts or teaching needs. The key is recognizing that sudden fluid loss plus bulging tissue is not a routine complaint.

51. The practical nurse is caring for four clients on a medical unit. Which client should the nurse assess first?

A. A client with *Clostridioides difficile* infection who had three loose stools during the night and asks for juice

B. A client with Addison disease who is vomiting, weak, and has a blood pressure of 84/50 mm Hg

C. A client with iron-deficiency anemia who reports fatigue when walking to the bathroom

D. A client with psoriasis who asks whether oatmeal baths can help itching

Correct Answer: B

Vomiting, weakness, and hypotension in a client with Addison disease suggest possible adrenal crisis, which can rapidly become life-threatening. The client needs immediate assessment and prompt intervention. The client with diarrhea also needs care, but the severe hypotension in choice B indicates greater immediate instability.

52. At change of shift, the practical nurse reviews these laboratory and assessment findings. Which client should the nurse see first?

A. A client receiving heparin whose activated partial thromboplastin time is mildly above the expected therapeutic range and who has no bleeding

B. A client with neutropenia who has an oral temperature of 100.8°F (38.2°C)

C. A client with chronic kidney disease whose potassium level is 5.3 mEq/L and who is waiting for lunch

D. A client with hypothyroidism whose sodium level is 133 mEq/L and who reports dry skin

Correct Answer: B

Fever in a client with neutropenia can indicate infection with limited immune response and may progress quickly to sepsis. That requires immediate assessment. Mildly elevated aPTT without bleeding and a potassium of 5.3 mEq/L are concerns, but they are not as urgent as possible neutropenic infection. The trap is overreacting to abnormal lab values without considering which finding signals the highest immediate risk.

53. The practical nurse is assigned four residents in a long-term care facility. Which resident should be evaluated first?

- A.** A resident with dementia who is wandering into another room
- B.** A resident with diabetes who is sweating, shaky, and refusing to finish lunch
- C.** A resident with chronic venous insufficiency who asks for an extra pillow to elevate the legs
- D.** A resident with hearing loss who says the television remote is missing

Correct Answer: B

Sweating and shakiness in a resident with diabetes are classic signs of hypoglycemia, which can quickly worsen if not addressed. Wandering is a safety concern, but active hypoglycemia is the more immediate physiologic threat. The other requests are nonurgent.

54. A practical nurse in an outpatient infusion center is told about four clients. Which client should the nurse assess first?

- A.** A client receiving chemotherapy who reports tingling at the IV site and whose surrounding skin looks pale and swollen
- B.** A client receiving a vitamin infusion who asks for a blanket because the room feels cool
- C.** A client receiving maintenance fluids who wants to know how much longer the infusion will take
- D.** A client waiting for a scheduled antibiotic infusion who says the chair is uncomfortable

Correct Answer: A

Tingling, pallor, and swelling at the IV site during chemotherapy suggest extravasation, which can cause serious tissue injury and requires immediate intervention. The other clients have comfort or information needs. The key is recognizing that not all IV complaints are equal; vesicant infiltration is urgent.

55. The practical nurse is caring for four clients on a neurological unit. Which client should the nurse assess first?

- A.** A client with Bell palsy who needs help applying an eye patch
- B.** A client with Parkinson disease who says the morning dose of medication seems less effective than usual
- C.** A client with meningitis who suddenly becomes difficult to arouse and has vomited once
- D.** A client with trigeminal neuralgia who asks for a soft toothbrush

Correct Answer: C

A sudden decrease in level of consciousness with vomiting in a client with meningitis suggests possible increased intracranial pressure or neurologic deterioration. This is an immediate priority. The other clients have important but nonemergent needs. A common trap is focusing on medication timing in Parkinson disease rather than an acute change in neurologic status.

56. A practical nurse in a postpartum unit must decide which client to assess first. Which client is the priority?

- A.** A client 8 hours after birth who has a boggy uterus, heavy lochia, and feels dizzy when sitting up
- B.** A client 1 day after birth who asks for help with breastfeeding latch
- C.** A client 2 days after cesarean birth who rates incisional pain at 6/10 before ambulation
- D.** A client 12 hours after birth who has not yet had a bowel movement

Correct Answer: A

A boggy uterus with heavy lochia and dizziness suggests postpartum hemorrhage, which can rapidly lead to hypovolemia and shock. This client needs immediate assessment. The other clients need routine postpartum care or teaching. The key cue is the combination of uterine atony and signs of blood loss.

57. The practical nurse receives four reports from assistive personnel. Which client should the nurse follow up on first?

- A.** A client with a Foley catheter whose urine output for the last 4 hours totals 100 mL
- B.** A client with pneumonia whose pulse oximeter reading dropped from 95% to 89% while walking back from the bathroom
- C.** A client with a fractured ankle who says the cast feels itchy inside
- D.** A client with hepatitis who ate only half of breakfast

Correct Answer: B

A drop in oxygen saturation to 89% in a client with pneumonia indicates impaired oxygenation and requires prompt assessment. Choice A is also concerning because urine output is low, but oxygenation problems take priority over renal perfusion concerns when ranking immediate physiologic threats. The other findings are less urgent.

58. The practical nurse is assigned four clients in a community health setting. Which client should be contacted first?

- A.** A client with hypertension who says, "I ran out of pills yesterday."
- B.** A client with depression who says, "Nothing matters anymore, and I have the pills I need."
- C.** A client with allergic rhinitis who asks whether pollen counts are higher this week
- D.** A client with chronic constipation who says, "The stool softener has not helped yet."

Correct Answer: B

This statement suggests suicidal ideation with a possible plan and means, which requires immediate follow-up to protect client safety. Running out of antihypertensive medication is important but not as immediately life-threatening as a suicide risk. The trap is underestimating indirect wording that strongly signals self-harm danger.

59. A practical nurse is reviewing four clients before lunch. Which client should be assessed first?

- A.** A client with syndrome of inappropriate antidiuretic hormone who has a new headache, confusion, and nausea
- B.** A client with emphysema who wants the fan turned toward the bed
- C.** A client with constipation who says the bran cereal tastes bland
- D.** A client with a healed pressure injury who asks whether walking after lunch is allowed

Correct Answer: A

New headache, confusion, and nausea in a client with SIADH suggest worsening hyponatremia, which can progress to seizures and neurologic compromise. That client should be assessed first. The other clients have comfort or routine concerns. The key is recognizing subtle neurologic changes as high-risk cues.

60. The practical nurse is caring for four clients on a surgical floor. Which task should take priority right now?

- A.** Bring fresh ice water to a client who is allowed clear liquids
- B.** Recheck a client who received morphine 20 minutes ago and now has a respiratory rate of 8/min
- C.** Help a client complete menu choices for tomorrow
- D.** Change the linens for a client returning from physical therapy in 30 minutes

Correct Answer: B

A respiratory rate of 8/min after morphine indicates possible opioid-induced respiratory depression, which is an immediate airway and breathing concern. This takes priority over comfort, planning, and routine room care. The stem tests whether the nurse recognizes a medication effect that can become life-threatening very quickly.

61. The practical nurse is preparing to begin a shift in a rehabilitation unit. Which client should the nurse assess first?

- A.** A client with multiple sclerosis who reports needing help to the bathroom
- B.** A client 1 day after a stroke who suddenly has slurred speech that is worse than earlier
- C.** A client with osteoarthritis who asks for the next scheduled analgesic
- D.** A client with a healed hip fracture who wants assistance putting on shoes

Correct Answer: B

Worsening slurred speech after a recent stroke may indicate new neurologic deterioration and requires immediate assessment. The other clients have important but nonurgent needs related to comfort or assistance with activities. A common trap is choosing the client who needs toileting help first, but an acute change in neurologic status takes priority over routine assistance.

62. A practical nurse in a long-term care facility is told about these residents at the start of rounds. Which resident should the nurse see first?

- A.** A resident receiving digoxin who reports nausea and sees yellow halos around lights
- B.** A resident with chronic constipation who has not had a bowel movement in 2 days
- C.** A resident with cataracts who says the room seems dim this morning
- D.** A resident with knee pain who asks for a warm blanket before therapy

Correct Answer: A

Nausea and yellow visual halos in a client taking digoxin suggest possible digoxin toxicity, which can progress to serious dysrhythmias and requires prompt assessment. Constipation, chronic vision limitations from cataracts, and a request for comfort measures are lower priority. The likely trap is minimizing the visual complaint as an eye problem rather than linking it to medication toxicity.

63. The practical nurse is caring for four clients in an urgent care clinic. Which client should be evaluated first?

- A.** A school-age child with a sore throat who has a temperature of 100.4°F (38°C)
- B.** An older adult with a urinary tract infection who is newly confused and trying to climb off the stretcher
- C.** A young adult with a sprained wrist who rates pain as 8/10
- D.** A middle-aged adult with seasonal allergies who requests a note for work

Correct Answer: B

New confusion is an acute change in mental status and also creates an immediate safety risk because the client is trying to climb off the stretcher. This client needs prompt evaluation to prevent injury and identify the cause of deterioration. The child with low-grade fever and the client with pain need care, but they are more stable. The work note request is lowest priority.

64. The practical nurse is covering four clients on a medical-surgical unit. Which client should the nurse assess first?

- A.** A client with pancreatitis who reports abdominal pain of 7/10 and is due for a PRN analgesic
- B.** A client with heart failure who has crackles at the lung bases and 1+ ankle edema at the end of the day
- C.** A client 18 hours after thyroidectomy who is restless, has a high-pitched voice, and develops noisy breathing
- D.** A client with gastroesophageal reflux disease who says the evening meal caused heartburn

Correct Answer: C

Restlessness, voice changes, and noisy breathing after thyroidectomy suggest airway compromise, possibly from edema or bleeding, and require immediate assessment. Airway problems take priority over pain, chronic fluid overload findings, or heartburn. A common trap is choosing the client with heart failure because crackles sound serious, but the postoperative airway threat is the most immediate danger.

Assignment and delegation

65. A practical nurse is caring for four clients with the help of one experienced UAP. Which task is most appropriate to delegate to the UAP?

- A.** Reinforce teaching about how to use an incentive spirometer after surgery
- B.** Obtain and record a routine set of vital signs for a stable client
- C.** Assess a client's pain 30 minutes after an opioid was given
- D.** Evaluate whether a new antihypertensive medication is effective

Correct Answer: B

Routine data collection for a stable client, such as obtaining and recording vital signs, is appropriate to delegate to experienced assistive personnel. Teaching, assessment, and evaluation remain the nurse's responsibility. A common trap is choosing reinforcement of teaching, but client education still requires nursing judgment.

66. A practical nurse is assigning care for the next shift. Which client should the practical nurse care for personally rather than assign to an assistive personnel worker?

- A.** A stable client who needs help with a bed bath
- B.** A client who needs intake and output measured
- C.** A client who reports new shortness of breath after ambulation
- D.** A client who needs linens changed after breakfast

Correct Answer: C

New shortness of breath requires nursing assessment and possible immediate intervention, so the nurse should care for this client personally. Bathing, measuring intake and output, and changing linens are routine tasks that may be delegated when the client is stable. The key is recognizing the change in condition.

67. A practical nurse is working with a UAP who says, "The client's blood pressure is 84/50 mm Hg." What is the nurse's best action first?

- A.** Ask the UAP to recheck the blood pressure and report back at the end of rounds
- B.** Immediately assess the client and determine whether the reading is accurate and clinically significant
- C.** Document the value in the chart and continue passing medications
- D.** Tell the UAP to notify the provider

Correct Answer: B

A markedly low blood pressure may indicate instability, so the nurse must assess the client promptly. The nurse cannot delegate the interpretation of the finding or delay evaluation. Telling the UAP to notify the provider skips the nurse's responsibility to assess first and gather relevant information.

68. The practical nurse asks a UAP to ambulate a postoperative client for the first time after surgery. Which response by the UAP requires the nurse to intervene?

- A.** "I'll let you know if the client feels dizzy while walking."
- B.** "I'll report how far the client walks."
- C.** "I'll assess whether the client is ready for the first ambulation."
- D.** "I'll make sure the client has nonskid footwear on."

Correct Answer: C

Determining whether a postoperative client is ready for first ambulation requires assessment and nursing judgment, so the nurse must intervene. Reporting distance walked, observing for dizziness, and using safety measures are appropriate parts of delegated ambulation assistance once the nurse has assessed readiness.

69. A practical nurse is planning assignments for a team that includes one LPN/LVN and one UAP. Which task is most appropriate to assign to the LPN/LVN rather than the UAP?

- A. Collect a urine specimen from a stable client
- B. Feed a client who has no swallowing problems
- C. Change a sterile dressing for a client with a healing wound, if permitted by agency policy
- D. Transport a client to radiology

Correct Answer: C

Sterile dressing changes involve nursing skill and are appropriate for a licensed nurse when allowed by role and facility policy. The other tasks are routine, nonsterile activities often appropriate for assistive personnel with stable clients. The phrase about agency policy matters because assignment depends on role and setting.

70. A practical nurse delegates morning weights to a UAP on a pediatric unit. At noon, the weights still are not documented because the unit became busy. What is the nurse's best action?

- A. Document estimated weights based on yesterday's values
- B. Ignore the omission because weights can wait until the next shift
- C. Reassess priorities and ensure the weights are obtained because the nurse remains accountable for delegated care
- D. Report the UAP to the manager before addressing the client care need

Correct Answer: C

Delegation transfers the task, not accountability. The nurse must follow up, reprioritize, and make sure the required care is completed. Charting estimated data is unsafe, and focusing first on discipline rather than client care misses the immediate responsibility.

71. A client with a new colostomy asks for help emptying the pouch. Which action by the practical nurse is best?

- A. Delegate the procedure to the UAP because pouch emptying is a routine task
- B. Perform the procedure while teaching the client, then consider delegation of routine care later if the client is stable

- C. Ask the family to handle the procedure so the client learns faster
- D. Delay the procedure until the wound care nurse is available

Correct Answer: B

When a client needs education and adaptation to new ostomy care, the nurse should perform the task and provide teaching. Once the client is stable and the need is routine, some aspects may later be delegated according to policy. The trap is focusing only on the motor task and missing the teaching component.

72. The practical nurse is caring for four clients and has one UAP available. Which task is unsafe to delegate?

- A. Turn a client every 2 hours to reduce pressure injury risk
- B. Assist a client with toileting after the nurse has assessed mobility
- C. Measure blood glucose on a client who is diaphoretic, shaky, and newly confused
- D. Refill a stable client's water pitcher if no fluid restriction is ordered

Correct Answer: C

A client who is diaphoretic, shaky, and newly confused may be clinically unstable and needs immediate nursing assessment. Even though obtaining a blood glucose can be routine, this situation involves an acute change in condition. Stable turning, toileting assistance after assessment, and refilling water are more appropriate delegated tasks.

73. A UAP reports that a client refused a scheduled bath and became tearful when encouraged. What should the practical nurse do next?

- A. Tell the UAP to insist because hygiene is required
- B. Ask the UAP to chart that the client was noncompliant
- C. Assess the client to explore the reason for refusal and determine whether further intervention is needed
- D. Cancel all hygiene care for the rest of the day

Correct Answer: C

A tearful refusal may reflect pain, depression, fear, fatigue, or another concern that requires nursing assessment. The nurse should not simply label the client noncompliant or direct the UAP to pressure the client. Delegated care still requires the nurse to assess problems and adjust the plan.

74. Which instruction by the practical nurse best promotes safe delegation to a UAP who will feed a client with right-sided weakness after a stroke?

- A.** "Feed the client quickly so the meal does not get cold."
- B.** "If the client coughs while eating, stop feeding and notify me right away."
- C.** "Encourage the client to drink through a straw with every bite."
- D.** "If the client seems sleepy, finish the meal while the client rests."

Correct Answer: B

Clear directions about what to observe and when to report are essential for safe delegation. Coughing while eating may signal aspiration risk and requires the nurse's prompt assessment. The other options promote unsafe feeding practices or ignore changes in the client's condition.

75. A practical nurse is supervising a newly hired UAP. Which task should the nurse avoid delegating until the UAP's competency for that task is confirmed?

- A.** Making an occupied bed for a stable client
- B.** Recording oral intake after meals
- C.** Applying elastic antiembolism stockings to a postoperative client
- D.** Bringing a client clean gowns and toiletries

Correct Answer: C

Some delegated tasks may be appropriate only after the nurse verifies training and competency. Applying antiembolism stockings requires correct technique and awareness of circulation and skin concerns. The other tasks are more basic supportive activities. This question focuses on the nurse's duty to consider both task and worker competence.

76. The practical nurse is receiving shift report on four clients. Which client is most appropriate to assign to an experienced UAP for routine care?

- A.** A client 1 hour after returning from a bronchoscopy
- B.** A client admitted with chest pain who is awaiting diagnostic results
- C.** A client with dementia who has stable vital signs and needs help getting dressed
- D.** A client with a new onset of irregular pulse

Correct Answer: C

An experienced UAP can help a stable client with dressing and other routine activities of daily living. The other clients are unstable, newly admitted with potentially serious symptoms, or recovering from a recent procedure and need nursing assessment and monitoring.

77. A practical nurse tells a UAP, "Please collect a clean-catch urine specimen from Ms. Lopez after you explain the steps to her." Which part of this delegation is problematic?

- A.** The nurse should not use the client's name when delegating
- B.** The nurse delegated specimen collection for a stable client
- C.** The nurse expected the UAP to provide client teaching about the procedure
- D.** The nurse delegated a task that always requires sterile technique by a licensed nurse

Correct Answer: C

Collecting a specimen may be delegated for a stable client, but initial teaching about how to perform the clean-catch procedure should be done by the nurse. The issue is not naming the client; it is delegating teaching. A likely trap is assuming specimen collection itself is never delegable.

78. A practical nurse delegates postoperative ambulation of a stable client to a UAP. Thirty minutes later, the UAP reports, "The client said the incision felt wet, so I helped the client back to bed." What is the nurse's priority action?

- A.** Praise the UAP and check the client at the next scheduled round

- B. Ask the UAP whether the client still wants to walk later
- C. Immediately assess the incision and the client's overall status
- D. Tell the UAP to reinforce the dressing

Correct Answer: C

A wet incision after ambulation may indicate drainage, bleeding, or wound disruption, so the nurse must assess immediately. The UAP acted appropriately by stopping the activity and reporting. The nurse should not delay assessment or delegate dressing decisions without first evaluating the client.

79. Which assignment is most appropriate for the practical nurse to make to a UAP on a long-term care unit?
- A. Observe a resident with chronic confusion for sudden changes in behavior and determine whether the resident has delirium
 - B. Offer fluids and record intake for a resident at risk for dehydration
 - C. Decide whether a pressure injury is healing normally during perineal care
 - D. Evaluate the effectiveness of a laxative given earlier in the shift

Correct Answer: B

Offering fluids and recording intake are routine supportive tasks that may be delegated. Determining delirium, judging wound healing, and evaluating medication effectiveness require nursing assessment or evaluation. The wording "determine" and "evaluate" helps identify choices that stay with the nurse.

80. A practical nurse is caring for a stable client who had a Foley catheter removed 6 hours ago. Which task is most appropriate to delegate to an experienced UAP?
- A. Assess the client for bladder distention
 - B. Ask the client about burning with urination
 - C. Measure and record the client's first voided urine amount
 - D. Determine whether urinary retention is present

Correct Answer: C

Measuring and recording the amount of urine voided is a routine data-collection task that can be delegated to an experienced UAP for a stable client. Assessing for bladder distention, asking focused symptom questions to interpret possible complications, and determining whether retention is present require nursing assessment and judgment.

81. A practical nurse is assigning tasks for a home health visit. Which activity should the nurse perform rather than delegate to a home health aide/UAP?
- A. Assist a stable client with putting on shoes before walking
 - B. Check whether a newly reddened heel area is blanchable
 - C. Help a client bathe after the nurse has completed the assessment
 - D. Reposition a client in bed using pillows for support

Correct Answer: B

Checking whether new redness is blanchable is an assessment that helps identify possible pressure injury development, so the nurse should perform it. The other options are routine supportive care tasks that may be delegated for a stable client. A common trap is choosing bathing or repositioning because skin is involved, but the key issue is whether the task requires assessment and interpretation.

82. A practical nurse delegates finger-stick blood glucose checks before breakfast to a UAP for several stable clients with diabetes. Which finding reported by the UAP requires the nurse to see the client first?
- A. A glucose of 118 mg/dL in a client who is eating breakfast
 - B. A glucose of 136 mg/dL in a client who is due for routine insulin
 - C. A glucose of 62 mg/dL in a client who says, "I feel shaky"
 - D. A glucose of 154 mg/dL in a client who has no symptoms

Correct Answer: C

A glucose of 62 mg/dL with shakiness suggests symptomatic hypoglycemia and requires prompt nursing assessment and intervention. The other glucose values are less urgent in stable clients. The delegation itself may be appropriate, but the nurse remains responsible for interpreting abnormal findings and responding to urgent changes.

83. The practical nurse is working with a float UAP who has not worked on the unit before. Which delegated task requires the nurse to provide the most specific instructions because of client safety risks?

- A.** Refill water pitchers for clients without fluid restrictions
- B.** Transport a stable client by wheelchair to physical therapy
- C.** Apply a bed alarm for a client already identified as a high fall risk and report if it activates
- D.** Bring extra blankets to a client who reports feeling cold

Correct Answer: C

Applying and monitoring a bed alarm for a client at high fall risk requires clear directions about setup, when to notify the nurse, and what the UAP should and should not do if the alarm sounds. The other tasks are routine and lower risk. The trap is assuming bed alarms are simple equipment tasks; they are routine only when accompanied by precise supervision and reporting expectations.

84. A practical nurse delegates collection of a stool specimen for occult blood testing to a UAP. Which statement by the UAP shows correct understanding?

- A.** I will tell the client the test means colon cancer is present if it is positive.
- B.** I will collect the specimen and let you know if the stool looks black or tarry.
- C.** I will decide whether the client should avoid certain foods before the test.
- D.** I will interpret the result and notify the provider if it is abnormal.

Correct Answer: B

Collecting the specimen and reporting an abnormal observation such as black, tarry stool is appropriate for a UAP. Explaining diagnostic meaning, deciding preparation instructions, and interpreting results require nursing knowledge and judgment. The likely trap is thinking the UAP can manage the whole testing process rather than the specific task delegated.

85. A practical nurse is planning care for four clients with help from one experienced UAP. Which task is best to delegate?

- A.** Observe whether a client starting a blood transfusion develops chills
- B.** Perform passive range-of-motion exercises for a client with longstanding contractures
- C.** Evaluate the pain relief after a PRN analgesic was given 45 minutes ago
- D.** Teach a client with a new walker how to transfer safely

Correct Answer: B

Passive range-of-motion exercises for a client with longstanding contractures are a routine, predictable care activity that may be delegated to an experienced UAP. Monitoring for transfusion reactions, evaluating medication effectiveness, and teaching safe transfer technique require licensed nursing assessment, evaluation, or instruction.

86. A practical nurse asks a UAP to obtain orthostatic vital signs for a stable client. Which instruction is most important for the nurse to include?

- A.** Tell the client why the provider ordered the test.
- B.** Document the results after the end of the shift.
- C.** Stop and call me right away if the client becomes dizzy when changing positions.
- D.** Skip the standing measurement if the client says it takes too long.

Correct Answer: C

The most important instruction is to report dizziness immediately because position changes can create a fall risk and may indicate hemodynamic instability. Teaching the reason for the test is not the UAP's role, delaying documentation is inappropriate, and skipping a prescribed component without notifying the nurse is unsafe.

87. On a rehabilitation unit, a practical nurse is delegating care for a client who had a left-sided stroke 2 weeks ago and is medically stable. Which task is inappropriate to delegate to a UAP?

- A.** Assist the client with grooming at the sink while using a gait belt
- B.** Help the client transfer to the toilet using the technique taught by therapy
- C.** Decide whether the client can advance from thickened liquids to thin liquids at lunch
- D.** Place the client's affected arm on a pillow when returning the client to bed

Correct Answer: C

Deciding whether to advance liquid consistency requires clinical judgment and is outside the UAP role. Assisting with grooming, transfers using an established plan, and positioning the affected arm are routine supportive tasks for a stable client when the nurse has assessed the client and the UAP is competent. The trap is confusing follow-through on an established care plan with making a new clinical decision.

88. A practical nurse delegated routine postoperative vital signs on a stable client to a UAP. The UAP reports, "The heart rate went from 88 to 124, and the client looks pale." Which action by the nurse is best?

- A.** Ask the UAP to repeat the vital signs in 1 hour
- B.** Assess the client immediately
- C.** Tell the UAP to document the findings and continue rounds
- D.** Instruct the UAP to call the surgeon directly

Correct Answer: B

A sudden increase in heart rate with pallor may indicate acute deterioration, so the nurse should assess the client immediately. Repeating the vital signs later delays care, documentation alone is insufficient, and the UAP should not bypass the nurse to call the surgeon. Delegation does not transfer the nurse's responsibility to interpret abnormal findings and act promptly.

89. The practical nurse is supervising a UAP who is helping care for a client receiving hospice care. Which task is appropriate to delegate?

- A.** Determine whether the client's increased restlessness is caused by uncontrolled pain
- B.** Discuss code status with the family when they have questions
- C.** Provide mouth care and reposition the client for comfort
- D.** Evaluate whether the current opioid dose should be changed

Correct Answer: C

Providing mouth care and repositioning for comfort are routine comfort measures appropriate to delegate. Determining the cause of restlessness, discussing code status, and evaluating opioid dosing require nursing assessment, communication, and judgment. The likely trap is assuming hospice care changes scope boundaries; it does not.

90. A practical nurse is caring for a client with a chest tube connected to suction and is assisted by an experienced UAP. Which task, if delegated, would be unsafe?

- A.** Help the client sit in a chair after the nurse assesses stability
- B.** Measure and record oral intake for the shift
- C.** Report if the drainage system tips over during linen change
- D.** Determine whether tidaling in the water-seal chamber is expected

Correct Answer: D

Determining whether tidaling is expected requires knowledge of the chest drainage system and clinical interpretation, so it should not be delegated to a UAP. Helping the client to a chair after assessment, measuring intake, and reporting a tipped drainage system are appropriate delegated tasks. The trap is that all choices involve a chest tube client, but only one requires interpretation.

91. A practical nurse on a busy medical-surgical unit delegates to a UAP: "Please help Mr. Daniels to the bathroom and get his morning weight." Thirty minutes later, the UAP says, "He looked weak, so I let him stay in bed and skipped the weight." Which aspect of delegation was weakest?

- A.** The right task, because weights can never be delegated
- B.** The right circumstance, because the nurse should not delegate any care on a busy unit
- C.** The right direction and communication, because the nurse did not give clear instructions about what to report or how to proceed if the client seemed weak
- D.** The right person, because UAPs should never assist clients to the bathroom

Correct Answer: C

The weakest element was direction and communication. The nurse should include specific instructions such as reporting weakness before changing the plan and notifying the nurse if the client cannot safely stand for a weight. The task itself may be delegable, a busy unit does not prohibit delegation, and UAPs can assist with toileting when the client is appropriate.

92. A practical nurse is caring for four clients with one experienced UAP. Which situation is best to keep for the nurse rather than delegate?

- A.** A client with chronic COPD who needs assistance setting up the lunch tray
- B.** A client with a healed hip incision who needs help putting on anti-slip socks
- C.** A client with heart failure who reports that breathing is harder than it was an hour ago
- D.** A client with arthritis who needs help opening hygiene supplies

Correct Answer: C

A report of worsening breathing in a client with heart failure suggests a possible change in condition and requires nursing assessment. The other tasks are routine assistance that can generally be delegated to an experienced UAP. The key is recognizing unstable or changing status, not the diagnosis alone.

93. A practical nurse assigns a UAP to assist a client with dementia during meals. Which observation by the UAP should be reported to the nurse immediately?

- A.** The client eats dessert before the sandwich
- B.** The client pockets food in the cheek and begins coughing
- C.** The client needs extra time to finish the meal
- D.** The client asks for the same beverage twice

Correct Answer: B

Pocketing food and coughing suggest possible dysphagia and aspiration risk, which requires immediate reporting and nursing follow-up. Eating items out of order, taking longer to eat, or repeating a beverage request are less urgent observations. This question tests the nurse's responsibility to set reporting expectations for delegated care.

94. A practical nurse is supervising an experienced UAP who has been asked to apply sequential compression devices (SCDs) to a postoperative client. Which finding means the nurse should intervene and perform further assessment before the UAP applies the devices?

- A.** The client says the sleeves feel warm after they inflate
- B.** The client has a new complaint of unilateral calf pain
- C.** The client asks whether the sleeves can be removed for skin care
- D.** The client wants the pump placed farther from the bed because of noise

Correct Answer: B

New unilateral calf pain may indicate a possible deep vein thrombosis and requires nursing assessment before SCDs are applied. The other issues involve comfort or routine use and do not by themselves suggest a contraindication. The trap is focusing on equipment application as a routine task while missing the need to reassess when new symptoms appear.

95. The practical nurse is working with an experienced UAP on a long-term care unit. Which task is most appropriate to delegate to the UAP?

- A.** Assess a resident's new complaint of burning with urination
- B.** Reinforce teaching about insulin self-administration
- C.** Transfer a stable resident from bed to chair using a gait belt
- D.** Evaluate whether a pressure injury dressing is effective

Correct Answer: C

Transferring a stable resident with a gait belt is an appropriate delegated task because it is routine, has a predictable outcome, and does not require nursing assessment, teaching, or evaluation. Assessing a new urinary complaint, reinforcing medication teaching, and evaluating wound care all require nursing judgment.

96. A practical nurse is receiving morning assignment on a medical unit with one UAP. Which client should the nurse care for personally instead of assigning routine care to the UAP?

- A.** A stable client who needs help eating breakfast after a wrist fracture
- B.** A client returning from the post-anesthesia care unit 20 minutes ago
- C.** A resident who needs linens changed after spilling water in bed
- D.** A client scheduled for discharge later today who needs help getting dressed

Correct Answer: B

A client who has just returned from the post-anesthesia care unit requires ongoing assessment and nursing judgment, so the nurse should provide that care personally. The other tasks are routine assistance activities that may be delegated to a UAP if the clients are stable.

97. The practical nurse asks a UAP to measure and record intake and output for a client with heart failure. Which observation by the nurse shows that the delegation process needs improvement?

- A.** The nurse explains which fluids count toward intake
- B.** The nurse confirms when totals should be reported
- C.** The nurse tells the UAP to report urine output less than 30 mL/hr
- D.** The nurse assumes the UAP knows how to measure emesis because the UAP has worked on the unit before

Correct Answer: D

Delegation includes giving clear instructions and verifying expectations rather than assuming understanding. Even experienced personnel may need task-specific clarification. Choices A, B, and C reflect appropriate direction and supervision by the nurse.

98. An experienced UAP tells the practical nurse, "Mrs. Lee says her new pain pill is making her dizzy and nauseated. What should I tell her to do?" What is the nurse's best response?

- A.** Tell her to skip the next dose if she still feels dizzy
- B.** Ask her whether the dizziness started before or after the medication
- C.** I will assess her now; please stay with her until I get there
- D.** Document the complaint in the intake-and-output section

Correct Answer: C

A new report of dizziness and nausea after medication requires nurse assessment. The best action is for the nurse to assess the client promptly while having the UAP remain with the client for safety. Asking follow-up assessment questions, advising medication changes, and choosing unrelated documentation are outside the UAP's role or do not address the immediate need.

99. The practical nurse is delegating care to a newly hired UAP. Which task is safest to assign first?

- A.** Assist a stable client with a bed bath
- B.** Apply a condom catheter to a client with skin breakdown
- C.** Ambulate a client 6 hours after first-time opioid administration
- D.** Feed a client who coughed repeatedly during the previous meal

Correct Answer: A

Assisting a stable client with a bed bath is routine and carries the lowest risk, making it the safest first assignment for a newly hired UAP. The other options involve greater risk or require more assessment and judgment: skin breakdown, possible medication-related instability, and aspiration risk.

100. A practical nurse delegates routine morning weights to a UAP on a pediatric unit. Which client should the nurse instruct the UAP to weigh first because the result is most time-sensitive?

- A.** A child with nephrotic syndrome receiving diuretic therapy
- B.** A child with a healed arm fracture waiting for breakfast
- C.** A child admitted for observation after a mild concussion who is sleeping
- D.** A child with eczema scheduled for discharge teaching later in the day

Correct Answer: A

Daily weight is especially time-sensitive for a child with nephrotic syndrome receiving diuretics because fluid status must be monitored closely. The other clients do not have the same immediate need for weight-based evaluation. This question focuses on directing a delegated task according to clinical priority.

101. The practical nurse is assigning tasks after change of shift. Which assignment is inappropriate for a UAP?

- A.** Collect a clean-catch urine specimen from a stable client after the nurse explains the procedure to the client
- B.** Measure the amount of drainage in a Hemovac and report the total to the nurse
- C.** Remove a peripheral IV catheter from a client being discharged
- D.** Provide oral hygiene to a stable client with partial dentures

Correct Answer: C

Removing a peripheral IV catheter is not an appropriate task for a UAP because it requires licensed nursing responsibility according to common delegation principles used in nursing practice. Collecting a specimen, measuring drainage, and providing oral hygiene are routine tasks that may be delegated when the client is stable and the nurse provides needed instructions.

102. A practical nurse delegates ambulation of a stable postoperative client to a UAP. Which statement by the UAP requires the nurse to intervene immediately?

- A.** I'll put nonskid footwear on before we walk.
- B.** If the client says the incision pulls a little, I'll keep going unless I see drainage.
- C.** I'll help the client sit at the bedside before standing.
- D.** I'll call you if the client feels faint during the walk.

Correct Answer: B

The nurse should intervene because the UAP should not independently decide to continue ambulation when a postoperative client reports new discomfort at the incision. The nurse must assess the complaint. The other statements reflect appropriate safety measures during ambulation.

103. The practical nurse is delegating to one UAP and one licensed practical/vocational nurse peer is not available. Which task should the nurse assign to the UAP?

- A. Observe a client for effectiveness of a newly applied oxygen therapy order
- B. Perform tracheostomy suctioning for a client with thick secretions
- C. Obtain a routine pulse oximetry reading for a stable client with chronic lung disease
- D. Teach a client how to use an incentive spirometer after surgery

Correct Answer: C

Obtaining a routine pulse oximetry reading on a stable client is a standard data-collection task that may be delegated to a UAP. Evaluating oxygen therapy, suctioning a tracheostomy, and teaching incentive spirometer use require nursing assessment, intervention, or teaching.

104. The practical nurse delegated collection of vital signs on four stable clients to a UAP. Which report from the UAP should the nurse follow up on first?

- A. A client with hypertension has a blood pressure of 148/88 mm Hg
- B. A client with pneumonia has an oral temperature of 38.3 C (100.9 F)
- C. A client with chronic atrial fibrillation has a heart rate of 96/min
- D. A client with chronic back pain rates pain as 6 out of 10

Correct Answer: B

A client with pneumonia and an elevated temperature may be showing worsening infection or inadequate treatment response and should be followed up promptly. The blood pressure is mildly elevated but not the highest priority, the atrial fibrillation heart rate is not markedly abnormal for the condition described, and pain needs attention but is less urgent than possible infection progression.

105. A practical nurse on a surgical unit delegates evening care to an experienced UAP. Which task is inappropriate to delegate?

- A. Empty and record urine from a client's indwelling catheter
- B. Assist a stable client to use the incentive spirometer every hour while awake
- C. Change the linens of a client on bed rest
- D. Reposition a stable client every 2 hours

Correct Answer: B

Helping a client use an incentive spirometer involves teaching, coaching, and evaluation of technique, which are nursing responsibilities. Emptying a catheter bag, changing linens, and repositioning a stable client are routine tasks appropriate for delegation.

106. The practical nurse tells a UAP, "Please obtain a urine specimen from Mr. Ortiz sometime this morning." At noon, the specimen has not been collected because the client voided in the toilet twice. Which change would best improve future delegation?

- A. Assign the task to a different UAP next time
- B. Tell the UAP to document that the client was noncompliant
- C. Give a clearer time frame and specific instructions about notifying the nurse before the client voids
- D. Wait until the provider asks again before reordering the specimen

Correct Answer: C

The problem is a lack of clear direction. Effective delegation includes a defined time frame, task details, and instructions about when to report back. Reassigning blame or delaying action does not correct the communication gap that caused the missed specimen.

107. A practical nurse is supervising a UAP who is feeding clients on a subacute unit. Which client should the nurse assess personally before delegating feeding assistance?

- A. A client with osteoarthritis who needs the tray opened and meat cut
- B. A client with Parkinson disease who eats slowly but has no swallowing precautions
- C. A client who had a stroke yesterday and is awaiting a swallowing evaluation
- D. A client with a leg cast who cannot reach the water pitcher easily

Correct Answer: C

A client who had a recent stroke and is awaiting a swallowing evaluation is at risk for dysphagia and aspiration, so the nurse must assess and should not delegate feeding before safety is established. The other clients have routine assistance needs that may be delegated if they are otherwise stable.

108. The practical nurse delegates a routine blood pressure check to a UAP for a stable client taking antihypertensive medication. The UAP reports a blood pressure of 82/50 mm Hg and says, "The client looks sleepy, so I let him rest." What is the nurse's priority action?

- A. Ask the UAP to recheck the blood pressure in 1 hour
- B. Assess the client immediately
- C. Document the value and continue medication rounds
- D. Tell the UAP to offer the client more oral fluids

Correct Answer: B

A blood pressure of 82/50 mm Hg with a change in appearance requires immediate nurse assessment. The UAP has reported significant abnormal data, and the nurse must evaluate the client rather than delay, document without assessment, or direct an intervention before determining the cause.

109. The practical nurse is caring for clients in an outpatient clinic with one trained UAP. Which task is most appropriate to delegate to the UAP?

- A. Call a client to explain that a screening test result was abnormal

- B. Obtain height and weight for a client being seen for an annual visit
- C. Determine whether a client's chest pain is likely related to anxiety
- D. Review discharge instructions after a skin biopsy

Correct Answer: B

Obtaining height and weight is a routine, standard task appropriate for a trained UAP. Explaining abnormal results, assessing chest pain, and reviewing discharge instructions require nursing judgment, assessment, or teaching and should not be delegated.

110. A practical nurse is planning care for four clients and must decide which task to assign to an unlicensed assistive personnel (UAP). Which task is most appropriate to delegate?

- A. Reinforcing teaching about a newly prescribed low-sodium diet
- B. Collecting a clean-catch urine sample from a stable adult who has already received instructions
- C. Assessing a client who reports shortness of breath after walking to the bathroom
- D. Evaluating whether a client understood discharge instructions

Correct Answer: B

Collecting a clean-catch urine sample from a stable client is a routine task with a predictable outcome, so it is appropriate to delegate to a UAP. Teaching, assessment, and evaluation remain the nurse's responsibility. A common trap is choosing A because it sounds simple, but even reinforcement of new teaching requires nursing judgment about understanding and response.

111. The practical nurse delegates routine bedtime care to a UAP for several stable clients. Which additional task should the nurse perform personally?

- A. Repositioning a client with chronic left-sided weakness
- B. Emptying a urinary drainage bag and measuring output
- C. Applying a prescribed topical medication to a new blistering rash
- D. Providing fresh water to a client without swallowing problems

Correct Answer: C

Applying a prescribed medication, especially to a new skin problem, requires nursing judgment and cannot be delegated to a UAP.

Repositioning, measuring urinary output, and offering fresh water are routine care tasks that may be delegated when the client is stable. The likely trap is B, but measuring and recording output is generally routine if no assessment or interpretation is required.

112. A practical nurse on a rehabilitation unit is working with a float UAP who is unfamiliar with the unit. Which factor is most important for the nurse to consider before delegating a task?

- A.** Whether the task is one the nurse dislikes doing
- B.** Whether the task is routine and the UAP has the demonstrated ability to perform it safely
- C.** Whether the client prefers help from assistive personnel instead of the nurse
- D.** Whether the task can be finished quickly

Correct Answer: B

Delegation decisions depend on the nature of the task, client stability, predictability of outcome, and the delegatee's competence. A task should not be delegated simply because it is fast or convenient. Client preference may matter, but it does not override safety and competency. The central issue is whether this specific UAP can safely perform that specific routine task.

113. The practical nurse asks a UAP to obtain a stool specimen from a client being evaluated for possible gastrointestinal bleeding. Two hours later, the UAP reports, "The sample looked unusual, so I threw it away and will get another one later." What is the nurse's best response?

- A.** Thank the UAP for avoiding contamination and ask for a new specimen later
- B.** Tell the UAP not to worry because the laboratory can test any later specimen
- C.** Explain that unusual findings should be reported immediately and review specimen-handling expectations
- D.** Document the missed specimen and continue with other care

Correct Answer: C

The UAP should report abnormal or unusual findings rather than make an independent decision to discard a potentially important specimen. The nurse must address the immediate delegation problem by clarifying expectations and reinforcing that changes or abnormal observations require prompt reporting. The trap is A, which sounds supportive but fails to correct unsafe decision-making.

114. A practical nurse is coordinating a busy morning on a medical-surgical unit. Which task is most appropriate to assign to a licensed practical/vocational nurse (LPN/LVN) team member rather than to a UAP?

- A.** Measuring a stable client's finger-stick blood glucose if trained by unit policy
- B.** Ambulating a client who walked independently yesterday
- C.** Administering a scheduled oral medication to a client with chronic hypertension
- D.** Transporting a client to radiology

Correct Answer: C

Medication administration requires licensed nursing responsibility and is appropriate for an LPN/LVN team member, not a UAP. The other tasks are commonly delegated to trained assistive personnel when the client is stable and agency policy allows. The main distinction is that UAPs perform routine supportive tasks, while licensed nurses can carry out tasks requiring nursing licensure.

115. During morning rounds, a practical nurse delegates vital signs on stable clients to a UAP. Which client assignment is inappropriate to delegate?

- A.** A client 2 days after cataract surgery awaiting discharge instructions
- B.** A client with dehydration who started IV fluids 30 minutes ago after a blood pressure of 88/54 mm Hg
- C.** A client with osteoarthritis receiving routine pain medication for knee discomfort
- D.** A client with chronic hypertension whose blood pressure has been stable all week

Correct Answer: B

A client with recent hypotension and newly initiated treatment is unstable and requires nursing judgment, follow-up, and likely reassessment by the nurse. Routine vital signs on stable clients can be delegated, but trending response to a current problem should not be delegated without close nurse involvement. The trap is A, but discharge teaching is a nursing task; the question asks specifically about vital-sign collection.

116. A UAP tells the practical nurse, "I don't feel comfortable transferring Mr. Daniels by myself because he was unsteady earlier." What is the nurse's best action?

- A.** Tell the UAP to proceed because transfer assistance is part of the job
- B.** Ask another UAP to do the transfer without further assessment
- C.** Reassess the client and adjust the plan before deciding who should assist with the transfer
- D.** Document that the UAP refused the assignment

Correct Answer: C

When a UAP reports a safety concern, the nurse should reassess the client and determine the safest plan. Delegation is dynamic; a task that seemed routine may no longer be appropriate if the client's condition or mobility has changed. A and B ignore the reported concern, and D mislabels a safety issue as simple refusal.

117. A practical nurse delegates oral intake recording for a client with nausea. At the end of the shift, the intake record is blank. The UAP says, "The client only took small sips, so I didn't think it mattered." Which action by the nurse addresses the delegation issue best?

- A.** Complete the record from memory and say nothing to avoid conflict
- B.** Tell the UAP that charting is not necessary unless the client drinks a full serving
- C.** Clarify that all intake must be measured or estimated as instructed and verify understanding for future care
- D.** Reassign all intake and output tasks to another staff member permanently

Correct Answer: C

The nurse should address the gap through supervision, clear expectations, and confirmation of understanding. Intake and output are important even in small amounts, especially for a symptomatic client. A avoids correction, B gives incorrect guidance, and D is excessive without first addressing training and communication. Effective delegation includes follow-up and feedback.

118. The practical nurse is caring for a client 1 day after a stroke who has right-sided weakness and occasional coughing with liquids. Which task may the nurse delegate to a UAP?

- A.** Deciding whether the client can safely eat breakfast at the bedside
- B.** Feeding the client breakfast without further assessment because the tray has arrived
- C.** Assisting the client with face washing and grooming after the nurse evaluates swallowing risk and sets precautions
- D.** Teaching the client to use a chin-tuck technique during swallowing

Correct Answer: C

Because the client may have swallowing problems, the nurse must assess safety before any feeding-related task. Grooming assistance is routine and can be delegated after the nurse evaluates the client and establishes precautions. A, B, and D all involve assessment, judgment, or teaching. The likely trap is B because feeding may seem routine, but this client is not yet clearly safe to feed.

119. A practical nurse is deciding whether to delegate dressing changes to a UAP. Which client situation makes delegation inappropriate?

- A.** A stable client needs help changing into a clean gown before the dressing change
- B.** A stable client needs a dry, intact elastic wrap reapplied exactly as previously placed after bathing
- C.** A client has a new postoperative wound with increased drainage and surrounding redness
- D.** A stable client needs linens changed because drainage leaked onto the sheet

Correct Answer: C

A new postoperative wound with increased drainage and redness requires nursing assessment and judgment, so delegation to a UAP is inappropriate. The other tasks involve routine hygiene or basic assistance. The question turns on client condition, not just the word 'dressing.' When findings are changing or abnormal, the nurse must assess personally.

120. Near the end of the shift, a practical nurse learns that a UAP completed a delegated capillary blood glucose check on a stable client but did not report a reading of 62 mg/dL because the client said, "I feel fine." Which aspect of delegation failed most directly?

- A. The right circumstance, because glucose checks can never be delegated
- B. The right person, because only licensed staff may use a glucometer under any circumstance
- C. The right direction and communication, because expectations for reporting abnormal findings were not effectively established or understood
- D. The right task, because blood glucose results are never important unless symptoms occur

Correct Answer: C

If unit policy allows trained UAPs to obtain routine glucose checks, the major failure here is that abnormal results were not reported promptly. That points most directly to inadequate direction, communication, or verification of understanding about what must be reported immediately. A and B are overgeneralized statements, and D is unsafe because asymptomatic hypoglycemia still requires nurse follow-up.

121. A practical nurse has four possible tasks to assign during a home-health follow-up visit with a trained assistive worker. Which task is appropriate to delegate?

- A. Determining whether a healing pressure injury is improving compared with last week
- B. Teaching the family how to perform passive range-of-motion exercises correctly
- C. Observing and reporting the amount of fluid left in a graduated container used to track intake for the day

D. Deciding whether a client's new ankle swelling requires the provider to be notified

Correct Answer: C

Observing and reporting intake data is a routine task that may be delegated. The other options require assessment, interpretation, or teaching, which remain nursing responsibilities. The trap is A because it may sound observational, but deciding whether a wound is improving requires skilled assessment and comparison over time.

122. The practical nurse delegates morning hygiene to a UAP for a client with chronic dementia who is usually calm. Midway through care, the UAP reports that the client became newly agitated, pushed the washcloth away, and shouted incoherently. What should the nurse do first?

- A. Instruct the UAP to finish quickly so the client stays on schedule
- B. Ask the UAP whether this behavior happens every morning and continue with other tasks if it does
- C. Assess the client for possible causes of the sudden change before deciding whether care should continue
- D. Document that the client refused care and reschedule hygiene for the evening

Correct Answer: C

A sudden behavior change may signal pain, delirium, hypoxia, infection, fear, or another problem requiring nursing assessment. Once the client's condition appears less predictable, the nurse must reassess rather than simply push forward with delegated care. A and D ignore the change, and B delays needed assessment. The key principle is that delegation must be reconsidered when the client's status changes.

123. A practical nurse is supervising a UAP who has been delegated to apply antiembolism stockings for stable postoperative clients. Which observation requires the nurse to intervene?

- A. The UAP removes the stockings briefly for skin care and reapplies them smoothly
- B. The UAP reports that one client's toes are cool and pale after the stockings were applied
- C. The UAP applies the stockings before the client gets out of bed in the morning

D. The UAP asks the nurse to verify the correct stocking size for a client with borderline measurements

Correct Answer: B

Cool, pale toes suggest impaired circulation and require immediate nursing assessment and intervention. A and C describe expected actions, and D reflects appropriate use of supervision when the UAP recognizes uncertainty. The important delegation concept is that UAPs may perform routine tasks, but the nurse must intervene when abnormal findings are reported.

124. The practical nurse is caring for four clients with help from an experienced UAP. Which task is most appropriate for the nurse to delegate to the UAP?

- A.** Check a newly admitted client's pain level after the first dose of prescribed analgesic
- B.** Assist a stable client with chronic arthritis to the bathroom using the client's usual walker
- C.** Teach a client how to use an incentive spirometer after surgery
- D.** Inspect a reddened heel to determine whether skin breakdown is worsening

Correct Answer: B

Assisting a stable client with a routine toileting activity using the client's usual walker is an appropriate delegated task because it is predictable, does not require nursing judgment, and involves established care. The other options require assessment, evaluation, or teaching, which remain the nurse's responsibility. A is evaluation of response to medication, C is teaching, and D is assessment of skin status.

125. A practical nurse is assigning tasks to a UAP on a long-term care unit. Which task is most appropriate to delegate?

- A.** Assessing a resident's new complaint of burning with urination
- B.** Changing the dressing on a skin tear that was noted for the first time this morning
- C.** Assisting a stable resident with a scheduled shower and routine oral care
- D.** Teaching a resident how to use an incentive spirometer after pneumonia

Correct Answer: C

Routine, noninvasive care for a stable client is appropriate to delegate to a UAP. Assisting with a scheduled shower and oral care does not require nursing assessment, teaching, or clinical judgment. The other options involve assessment of a new symptom, care for a newly identified wound, or client teaching, which should be performed by licensed nursing staff.

126. The practical nurse delegates ambulation of a postoperative client to a UAP. Which instruction is most important for the nurse to include before the task begins?

- A.** Walk the client at least three times even if the client becomes tired
- B.** Encourage the client to increase speed with each lap in the hallway
- C.** Stop and report immediately if the client becomes dizzy, weak, or short of breath
- D.** Document the distance walked only if the client completes the full plan

Correct Answer: C

Effective delegation requires clear directions about what to do and what findings must be reported. The UAP should know to stop the activity and notify the nurse about symptoms such as dizziness, weakness, or shortness of breath because these may signal instability or intolerance of activity. The other options are unsafe or incomplete because they prioritize finishing the task over monitoring the client's response.

127. A practical nurse is supervising a newly hired UAP during a busy shift. Which situation requires the nurse to intervene first?

- A.** The UAP is helping a stable client choose clothing before physical therapy
- B.** The UAP is collecting a routine urine output measurement from a client with a chronic catheter
- C.** The UAP is feeding lunch to a client who has a newly placed nasogastric tube and has not yet been assessed by the nurse
- D.** The UAP is making an occupied bed for a client who was bathed earlier

Correct Answer: C

The nurse should intervene first when the delegated task involves an unassessed or potentially unstable situation. A client with a newly placed nasogastric tube requires nursing assessment before feeding or related assistance decisions are made. The other tasks involve routine care for stable clients and are generally appropriate for a UAP.

128. A practical nurse delegated collection of morning weights to a UAP for several clients with heart failure. At noon, the nurse notices one weight is 2.3 kg higher than yesterday. The UAP says, "I thought the scale was off, so I wrote it down and moved on." Which response by the nurse best addresses the delegation problem?

- A.** Tell the UAP to ignore the result because daily weights often vary
- B.** Explain that unexpected findings should be reported promptly and review when rechecking and notifying the nurse are required
- C.** Ask the UAP to delete the weight from the record until tomorrow's value is available
- D.** Decide that only nurses should ever obtain daily weights on clients with heart failure

Correct Answer: B

The best response is to correct the communication and supervision gap. A large unexpected weight increase in a client with heart failure may be clinically important, so the UAP should know to recheck per instruction and report the finding promptly rather than simply record it and continue. Choice D is too broad; the issue is not that weights can never be delegated, but that abnormal results must be recognized and reported appropriately.

129. A practical nurse is caring for four clients with support from an unlicensed assistive personnel (UAP). Which task is most appropriate to delegate to the UAP?

- A.** Reinforcing teaching about how to use an incentive spirometer after abdominal surgery
- B.** Collecting a clean-catch urine specimen from a stable client who received instructions earlier
- C.** Assessing a client's report of new tingling in the fingers after a cast was applied

D. Evaluating whether a client's pain medication was effective 30 minutes after administration

Correct Answer: B

The UAP can appropriately collect a clean-catch urine specimen from a stable client after the nurse has already provided instructions. This is a routine task that does not require nursing judgment. Reinforcing teaching, assessing new tingling after cast application, and evaluating response to medication all require licensed nursing assessment or teaching responsibility and should not be delegated to UAP.

Client rights and advocacy

130. A nurse enters a semi-private room and finds a visitor asking the client, "What surgery are you having tomorrow?" The client looks away and stays silent. What is the nurse's best response?

- A.** Answer the visitor's question because family and friends usually need updates
- B.** Tell the visitor that information can be shared only if the client agrees
- C.** Ask the visitor to wait outside and then explain the surgery in the hallway
- D.** Assume the client wants privacy and tell the visitor to leave permanently

Correct Answer: B

The nurse should protect the client's right to privacy and let the client decide whether information is shared. Choice B supports client autonomy without making assumptions. Choice A violates confidentiality. Choice C still risks inappropriate disclosure. Choice D is too extreme because the nurse has not first clarified the client's wishes.

131. A competent adult client with a new diagnosis says, "I don't want this treatment right now. I need time to think." Which action by the nurse is most appropriate?

- A.** Tell the client that delaying treatment is not allowed once the provider recommends it
- B.** Witness the consent form anyway because the provider already explained the treatment
- C.** Support the client's decision to delay and notify the provider of the client's wishes
- D.** Ask the client's spouse to make the decision so treatment can begin on time

Correct Answer: C

A competent adult has the right to refuse or delay treatment after receiving information. The nurse's role is to respect that decision, support the client, and communicate it to the provider. Choice A ignores autonomy. Choice B is inappropriate because consent must be voluntary. Choice D wrongly transfers decision-making to someone else when the client is capable.

132. During discharge teaching, a client with limited reading ability nods during every explanation but cannot describe the medication schedule when asked. What is the nurse's best advocacy action?

- A.** Document that the client attended teaching and move on to the next discharge
- B.** Repeat the same instructions more quickly to avoid embarrassing the client
- C.** Use plain language, picture-based instructions, and ask the client to teach back the schedule
- D.** Ask a family member to sign the papers so discharge is not delayed

Correct Answer: C

Advocacy includes promoting equitable access to understandable care information. Using plain language and teach-back helps confirm understanding without shaming the client. Choice A focuses on completion rather than comprehension. Choice B does not address the barrier. Choice D shifts responsibility away from the client and does not ensure understanding.

133. A hospitalized client says, "I want my daughter to hear everything about my care, but I do not want my son told anything." Which nursing action best supports the client's rights?

- A.** Share updates with both children because they are immediate family
- B.** Ask the provider to decide which family members should receive information
- C.** Document the client's preferences and follow them when sharing information
- D.** Refuse to speak with any family member to avoid making a mistake

Correct Answer: C

The client has the right to determine who receives personal health information. Documenting and honoring those preferences supports confidentiality and advocacy. Choice A ignores the client's wishes. Choice B shifts a nursing responsibility unnecessarily. Choice D is overly restrictive when the client has clearly given permission for one person.

134. A client who speaks little English is scheduled for a procedure. The adult grandchild says, "I can translate. Let's just get this done." What should the nurse do first?

- A.** Accept the grandchild as interpreter because consent is a family matter
- B.** Proceed with the provider's explanation and ask the client to sign if the grandchild agrees
- C.** Request a qualified interpreter so the client can receive accurate information before consenting
- D.** Cancel the procedure because informed consent is impossible when English is limited

Correct Answer: C

For informed decision-making, the client needs clear, accurate communication. Using a qualified interpreter best protects the client's rights and reduces the risk of omissions or pressure from family members. Choice A may compromise accuracy and autonomy. Choice B is invalid because the grandchild cannot substitute for the client's informed decision. Choice D is unnecessary because communication support is available.

135. A postoperative client tells the nurse, "The provider explained the procedure so fast that I signed without understanding the risks." What is the nurse's best action?

- A.** Reinforce the client's signature because the form is already complete
- B.** Notify the provider that the client has questions and pause preparation until concerns are addressed
- C.** Explain all procedure risks in detail and obtain a new signature
- D.** Tell the client that questions should have been asked before signing

Correct Answer: B

The nurse should advocate for valid informed consent by stopping the process and notifying the provider that the client needs further explanation. The nurse may witness consent, but obtaining informed consent through detailed procedural risk discussion is not the nurse's role in this context. Choice A ignores a serious consent problem. Choice C oversteps. Choice D is dismissive and unsafe.

136. A client with early dementia is alert, able to state a consistent choice, and says, "I do not want my son making decisions for me while I can still speak for myself." Which response by the nurse is best?

- A.** Explain that the son automatically becomes the decision-maker after a dementia diagnosis
- B.** Assess and document the client's ability to express preferences, and support the client's stated wishes
- C.** Ask the son to sign all treatment forms now to avoid future confusion
- D.** Delay all care decisions until a court determines who is responsible

Correct Answer: B

The key issue is the client's current decision-making ability, not the diagnosis label alone. If the client can understand and express a choice, the nurse should support and document those preferences. Choice A wrongly assumes loss of rights. Choice C bypasses the client. Choice D is unnecessary when the client can currently participate.

137. A nurse overhears a staff member laughing about a client's mental health history at the nurses' station where visitors are nearby. What is the nurse's best immediate action?

- A.** Ignore the comment and report it at the end of the shift
- B.** Join the conversation to learn whether the information is accurate
- C.** Stop the discussion and move communication about the client to an appropriate private setting
- D.** Tell nearby visitors that the conversation was not about any specific client

Correct Answer: C

The immediate priority is to stop the privacy breach and protect the client's confidentiality. Then follow facility policy for reporting as needed. Choice A delays intervention while the breach continues. Choice B worsens the problem. Choice D does not correct the inappropriate disclosure.

138. A client with severe pain says, "I don't want to bother anyone, so I just wait until the pain is unbearable." Which nursing response best reflects advocacy?

- A.** Pain is expected after surgery, so waiting is usually best
- B.** You should ask only when the provider rounds to avoid extra medication use
- C.** You have the right to pain management; let's review how to report pain early and what options are available
- D.** If you are really in pain, it will be obvious without you saying anything

Correct Answer: C

Advocacy includes helping the client understand the right to appropriate symptom management and how to access it. Choice C supports communication and informed participation. Choice A minimizes the client's experience. Choice B creates an unnecessary barrier. Choice D is unsafe because pain is subjective and must be reported and assessed.

139. A client says, "I want to leave the hospital now," even though the provider recommends staying for monitoring. The client is alert, oriented, and able to explain the risks of leaving. What should the nurse do?

- A.** Block the exit until the provider personally speaks with the client
- B.** Acknowledge the client's right to leave, notify the provider, and follow facility process for discharge against advice
- C.** Remove the IV and tell the client to go immediately without further discussion
- D.** Tell the client that insurance will refuse payment, so leaving is not permitted

Correct Answer: B

A competent client generally has the right to leave after being informed of risks. The nurse should advocate by ensuring communication, documentation, and appropriate process rather than using threats or force. Choice A may violate the client's rights. Choice C skips needed communication and documentation. Choice D uses coercion and may be inaccurate.

140. A client with metastatic cancer has a documented advance directive declining resuscitation. During a transfer, a family member cries, "Do everything no matter what the papers say!" The client is now unresponsive. What should the nurse do?

- A.** Follow the family's request because relatives can change the plan in an emergency
- B.** Start resuscitation until the provider confirms the document again
- C.** Advocate for the client's documented wishes and immediately communicate the directive to the team
- D.** Wait to act until all family members agree on what should happen

Correct Answer: C

The nurse's duty is to advocate for the client's known wishes as documented in the advance directive. Family distress does not automatically override the client's stated preferences. Choice A improperly substitutes family wishes. Choice B disregards the documented directive. Choice D creates harmful delay and ignores the client's autonomy.

141. A teenager requests testing for a sensitive condition and asks the nurse not to tell the parent who is in the waiting room. The nurse is unsure what information can legally be kept private in this setting. What is the best action?

- A.** Promise complete secrecy to gain the teenager's trust
- B.** Immediately disclose everything to the parent because minors never have privacy rights
- C.** Delay all care until the parent gives written permission for every discussion
- D.** Protect the client's privacy as much as possible and consult facility policy or the appropriate supervisor about the limits of confidentiality

Correct Answer: D

When confidentiality rules may depend on setting or policy, the nurse should avoid making absolute promises, protect privacy, and seek guidance on the applicable limits. Choice A is risky because the nurse may not be able to keep that promise. Choice B is too broad and may violate the client's rights. Choice C unnecessarily delays care and assumes a rule not established here.

142. A client with a hearing impairment is nodding yes to every question during preoperative preparation. The assistive personnel says, "The client agreed, so we're ready." Which action by the nurse best protects the client's rights?

- A.** Proceed because repeated nodding shows understanding and consent
- B.** Have the assistive personnel explain the procedure again using simpler words
- C.** Use an effective communication method for the client, verify understanding directly, and address any questions before proceeding
- D.** Ask a roommate whether the client usually understands written information

Correct Answer: C

Nodding alone does not establish understanding. The nurse must ensure the client can communicate effectively and has the opportunity to ask questions before care proceeds. Choice A risks invalid consent. Choice B relies on assistive personnel for a responsibility requiring nursing judgment. Choice D is an inappropriate privacy breach and not a valid assessment method.

143. A nurse is preparing a client for discharge to a shelter. The only prescribed medication requires refrigeration, and the client says, "I don't have a refrigerator or reliable transportation." What is the nurse's best advocacy action?

- A.** Continue discharge because medication access is the client's responsibility after leaving
- B.** Document nonadherence risk and ask the client to do the best possible
- C.** Contact the provider and case management to address barriers and explore a realistic discharge plan

D. Tell the client to skip doses until a refrigerator becomes available

Correct Answer: C

Advocacy includes addressing barriers that affect safe access to care after discharge. The nurse should coordinate with the provider and case management to seek a feasible plan. Choice A ignores equitable access. Choice B identifies the issue but does not act on it. Choice D is unsafe and could harm the client.

144. A client tells the nurse, "I do not want my diagnosis discussed during bedside shift report when my roommate is awake." Which adjustment by the nurse best respects the client's rights while still supporting continuity of care?

- A.** Skip shift report for that client entirely to maintain privacy
- B.** Give a brief bedside safety check, then discuss sensitive details in a more private location
- C.** Continue the full bedside report because staff communication is more important than privacy concerns
- D.** Ask the roommate to wear headphones so the normal report can continue

Correct Answer: B

The nurse should balance privacy with safe handoff by limiting bedside discussion to essential immediate information and moving sensitive details to a private setting. Choice A could harm continuity of care. Choice C dismisses the client's privacy rights. Choice D does not reliably protect confidentiality and shifts the burden to another patient.

145. A client admitted for complications of diabetes tells the nurse, "My employer keeps calling. You can tell my supervisor I'm here, but do not give any details about my condition." Later, the supervisor calls the unit and asks why the client was admitted and when the client can return to work. What is the nurse's best response?

- A.** Provide the diagnosis but not the treatment plan, because the client allowed the nurse to confirm the admission
- B.** State that the client is hospitalized and refer all other questions to the client for permission

C. Give a brief update because the supervisor needs information for staffing purposes

D. Refuse to confirm that the client is hospitalized because no information can be shared

Correct Answer: B

The client specifically allowed the nurse to confirm the hospitalization but not disclose medical details. The nurse should follow the client's stated limits and direct the supervisor to the client for any additional permission. Choice A exceeds the client's consent, and choice D ignores the client's expressed wish to allow limited disclosure.

146. A client scheduled for a blood transfusion quietly tells the nurse, "My religious beliefs do not allow me to receive blood products." The consent form is unsigned, and the provider is waiting to begin treatment. Which action should the nurse take first?

- A.** Ask the family to encourage the client to accept the transfusion
- B.** Notify the provider that the client is refusing the transfusion and support the client's stated choice
- C.** Request that the client sign the refusal form immediately without further discussion
- D.** Delay reporting the refusal until laboratory values are reviewed again

Correct Answer: B

The nurse's first duty is to advocate for the client's right to refuse treatment and promptly notify the provider. Supporting the client's stated decision protects autonomy and allows discussion of alternatives. Choice C focuses on paperwork before advocacy, and choice A pressures the client rather than supporting an informed, voluntary decision.

147. An older adult client with a fractured wrist is accompanied by an adult child who answers every question. When the nurse asks about pain, the client starts to speak, and the adult child says, "I handle all of that." What is the nurse's best action?

- A.** Direct all questions to the adult child because family members know the client best
- B.** Continue the interview later only if the provider requests it

- C. Politely ask the adult child to allow the client to answer and assess the client's wishes directly
- D. Assume the client wants the adult child to speak because no objection was made

Correct Answer: C

The nurse should support the client's right to speak for themselves whenever possible. Politely redirecting the conversation to the client helps assess preferences, symptoms, and decision-making ability directly. Choices A and D wrongly assume family control, and choice B delays advocacy.

148. A client with limited mobility says, "Physical therapy is scheduled on the second floor, but I've missed two visits because no one arranged transport." Which nursing action best demonstrates advocacy?

- A. Tell the client to ask a family member to assist with transportation needs
- B. Document the missed visits and wait to see whether the problem happens again
- C. Coordinate with the care team to arrange reliable transport so the client can access ordered therapy
- D. Advise the client to refuse therapy until scheduling improves

Correct Answer: C

Advocacy includes addressing barriers that prevent equitable access to needed care. Coordinating transportation directly addresses the problem and supports the plan of care. Choice B is too passive, and choice A shifts responsibility away from the healthcare team.

149. A client is sedated after a painful procedure. The client's spouse says, "I'm going to sign the surgical consent for tomorrow since my partner is sleepy." The client has no documented surrogate and was alert earlier in the day. What should the nurse do?

- A. Allow the spouse to sign because spouses automatically provide consent in this situation
- B. Witness the spouse's signature so the procedure is not delayed
- C. Explain that consent should be obtained from the client when the client is able to participate, and notify the provider

- D. Ask another nurse to decide whether the spouse may sign

Correct Answer: C

A sedated client may not be able to provide informed consent, and the nurse should not assume a spouse can sign without proper authority or necessity. The best action is to protect the client's rights by ensuring consent is obtained appropriately and by notifying the provider. Choices A and B make an unsafe legal assumption.

150. A client receiving hospice services says, "I want to review my choices again before I decide whether to continue this medication." The practical nurse is busy and knows the medication was already discussed yesterday. What is the best action?

- A. Tell the client the decision has already been addressed and should not be delayed
- B. Encourage the client to ask questions and help arrange further discussion with the appropriate team member
- C. Advise the client to follow the family's recommendation to avoid conflict
- D. Withhold all medications until the client makes final decisions about the entire plan of care

Correct Answer: B

Clients have the right to revisit decisions and ask for clarification before accepting or refusing care. The nurse advocates by encouraging questions and facilitating communication with the appropriate provider. Choice A dismisses the client's autonomy, and choice C substitutes family wishes for the client's rights.

151. A client with newly diagnosed cancer tells the nurse, "Do not tell my brother about this diagnosis. He gets upset and tells everyone." A few hours later, the brother arrives and says, "I'm family, so I have a right to know what is going on." Which response by the nurse is best?

- A. "I can only share information the client has permitted us to discuss."
- B. "Because you are immediate family, I can give you a brief summary."
- C. "I will tell you the diagnosis, but not the treatment plan."

D. “If you promise not to repeat it, I can explain the situation.”

Correct Answer: A

Family relationship alone does not override the client’s privacy choices. The nurse should respect the client’s clear instructions and limit disclosure accordingly. Choices B, C, and D all violate the client’s stated wishes.

152. A client who recently immigrated says, “I don’t understand these forms, but I’m embarrassed to ask again.” The nurse notices the client has signed several documents without asking questions. Which action is the best advocacy measure?

- A.** Assure the client that the forms are routine and can be explained later if needed
- B.** Ask the client to identify which signature is most important so teaching can be limited
- C.** Pause the process, assess what the client understands, and obtain communication support as needed before more forms are signed
- D.** Have a family member summarize the forms to save time

Correct Answer: C

Advocacy means protecting the client’s right to understand care and consent-related information before signing. The nurse should stop the process, assess understanding, and arrange appropriate support. Choice D may be inaccurate or coercive, and choice A minimizes the problem.

153. A client with chronic kidney disease says, “I can’t afford all these follow-up visits, so I’ll probably just stop coming.” Which nursing action best supports client advocacy?

- A.** Tell the client that skipping visits is noncompliant and dangerous
- B.** Explore financial barriers and refer the client to available case management or community resources
- C.** Advise the client to wait until symptoms become severe before seeking care
- D.** Suggest reducing prescribed visits without contacting the healthcare team

Correct Answer: B

Advocacy includes identifying barriers to ongoing care and connecting the client with resources that may improve access. Choice B addresses the actual obstacle while supporting the plan of care. Choices A and C are judgmental or unsafe, and D changes care outside the proper process.

154. An alert client says, “I don’t want any more pain medicine because it makes me feel confused.” The provider later writes, “Administer pain medication every 4 hours while awake.” The client again declines the dose. What should the nurse do?

- A.** Administer the medication because the provider prescribed it
- B.** Ask assistive personnel to convince the client to take the medication
- C.** Respect the refusal, assess pain and concerns, and communicate the client’s decision to the provider
- D.** Document the refusal after giving half the dose

Correct Answer: C

A competent client has the right to refuse medication. The nurse should assess the reason for refusal, respect the decision, and communicate with the provider about alternatives or needed follow-up. Choices A and D violate autonomy, and B is inappropriate delegation and coercion.

155. A client undergoing evaluation for injuries keeps looking at a partner before answering questions. When the nurse asks whether the client feels safe at home, the partner answers, “Of course.” What is the nurse’s priority advocacy action?

- A.** Accept the partner’s response because the client did not disagree
- B.** Separate the client from the partner and assess the client privately
- C.** Delay further questions until discharge teaching begins
- D.** Ask the partner to complete the history form for accuracy

Correct Answer: B

Possible intimidation or abuse requires the nurse to create a safe opportunity for the client to speak privately. Separating the client from the partner is the best immediate advocacy step. Choices A and D allow the partner to control the interaction, and C delays needed assessment.

156. A client with mild developmental disability is scheduled for a nonemergency procedure. The client can explain what the procedure is for, ask relevant questions, and clearly state a choice. The client's sibling says, "Just talk to me instead. I always make the decisions." What is the nurse's best response?

- A.** Direct the discussion to the sibling because a disability prevents valid consent
- B.** Include the sibling only if the client wants support, but continue addressing the client as the decision-maker
- C.** Ask the provider to postpone the procedure until the sibling can sign the consent
- D.** Obtain consent from both the client and sibling to avoid conflict

Correct Answer: B

Decision-making ability is based on the client's actual capacity, not on the presence of a disability label. Because the client demonstrates understanding and can communicate a choice, the nurse should continue to address the client directly and include the sibling only as the client wishes. Choice A is a common but incorrect assumption.

157. A client says, "I want my pastor involved in my care decisions, but my family says that is unnecessary." The family asks the nurse not to call the pastor. Which action by the nurse is most appropriate?

- A.** Honor the family's request because they are the main support system
- B.** Tell the client that spiritual support is not part of medical decision-making
- C.** Ask the client whether the pastor should be contacted now and follow the client's wishes
- D.** Wait until the family leaves before discussing it again

Correct Answer: C

The client's preferences guide care. If the client wants spiritual support involved, the nurse should clarify the request and act on the client's wishes. Choice A improperly prioritizes family preference, and choice B dismisses the client's values.

158. A client says, "I signed the procedure form because everyone seemed in a rush, but now I have more questions and I'm not sure I want it." The transport team has arrived to take the client to the procedural area. What should the nurse do first?

- A.** Send the client because the form is already signed
- B.** Tell the client to discuss concerns after arriving in the procedure area
- C.** Stop the transport and notify the appropriate provider that the client has unresolved questions and may not be giving informed consent
- D.** Explain the procedure in full and obtain a new signature

Correct Answer: C

A signature alone does not ensure informed consent. If the client expresses uncertainty or lack of understanding, the nurse should stop the process and notify the appropriate provider so the client can receive needed information before proceeding. Choice D exceeds the nurse's role in obtaining the provider's informed consent discussion.

159. A client in a long-term care setting says, "The aide comes into my room without knocking, even when I'm dressing." Which nursing response best reflects advocacy for client rights?

- A.** "That happens sometimes in busy units, so try to be ready earlier."
- B.** "You still have a right to privacy. I will address this concern and help prevent it from happening again."
- C.** "You should speak directly to the aide next time instead of reporting it."
- D.** "As long as your care is completed, room entry is not a rights issue."

Correct Answer: B

Privacy and dignity remain client rights in every care setting. The nurse should acknowledge the concern, advocate for respectful care, and take steps to address the behavior. Choices A and D minimize the violation, and choice C shifts responsibility away from the nurse.

160. A hospitalized client who is alert and oriented says, "I do not want my test results discussed while my roommate is in the room." Which action by the nurse best protects the client's rights?

- A.** Explain that sharing brief updates at the bedside is part of normal care
- B.** Wait until privacy can be provided before discussing the results
- C.** Ask the roommate to promise not to repeat the information
- D.** Discuss the results quietly so only the client can hear

Correct Answer: B

The nurse should protect the client's privacy by delaying the discussion until a private setting or adequate privacy is available. Quietly speaking in front of a roommate still risks disclosure, and a roommate's promise does not protect confidentiality. Normal workflow does not override the client's right to privacy.

161. During morning care, a client says, "I want to take a shower after breakfast, not right now." The nursing assistant says the schedule requires all baths before 7 a.m. What should the nurse do?

- A.** Support the client's preference and revise the plan if possible
- B.** Tell the client that unit routines must be followed
- C.** Instruct the assistant to complete the bath as quickly as possible
- D.** Document that the client is being uncooperative

Correct Answer: A

Respecting client rights includes honoring reasonable preferences and promoting participation in care planning. If the request is safe and feasible, the nurse should advocate for adjusting the schedule. The trap is choosing unit convenience over the client's autonomy and dignity.

162. A client says, "I do not want any students involved in my care today." What is the nurse's best response?

- A.** "You have the right to refuse student participation, and I will respect that choice."
- B.** "Students are part of the team, so you cannot refuse."
- C.** "Let the student come in once, and then decide."
- D.** "Refusing students may delay your care."

Correct Answer: A

Clients have the right to participate in decisions about who is involved in their care. The nurse should acknowledge and honor the refusal without pressure or guilt. The other responses undermine autonomy by coercing or dismissing the client's choice.

163. A client with hearing loss nods during teaching but gives unrelated answers when the nurse asks follow-up questions. The provider is waiting for the client to sign consent for a scheduled procedure. Which nursing action is best?

- A.** Ask a family member to summarize the information for the client
- B.** Proceed with the paperwork because the client appears agreeable
- C.** Obtain communication support and clarify understanding before the process continues
- D.** Have the client sign now and ask questions after returning from the procedure

Correct Answer: C

Advocacy requires ensuring the client can receive and understand information before decision-making proceeds. The nurse should address the communication barrier first, such as using hearing devices, written communication, or another appropriate support. A nod does not prove understanding, and signing first is unsafe and inappropriate.

164. A client on a medical unit tells the nurse, "Please do not let my estranged parent visit me." Later, the parent arrives and says, "I'm immediate family, so I can go in whenever I want." What is the nurse's best action?

- A.** Allow the visit because immediate family members have automatic access
- B.** Ask the client in front of the parent to explain the situation
- C.** Follow the client's expressed wishes and limit the visit according to facility process
- D.** Allow a brief visit as long as the parent agrees not to upset the client

Correct Answer: C

The alert client's wishes about visitors should guide care, and the nurse should advocate for those wishes using established facility procedures. Family relationship alone does not override the client's preferences. Asking the client to explain in front of the unwanted visitor may increase distress and pressure.

165. A nurse overhears a staff member joking about a client's sexually transmitted infection at the nurses' station where visitors are nearby. What is the nurse's best immediate action?

- A.** Ignore it because no names were mentioned
- B.** Join the conversation to learn which client was involved
- C.** Address the inappropriate discussion and move the conversation to a private, professional setting
- D.** Wait until the end of the shift to mention that the comment was unprofessional

Correct Answer: C

The nurse should intervene promptly because the discussion violates privacy and professional standards, even if a full name was not used. Immediate correction helps protect client dignity and confidentiality. Waiting until later fails to stop the ongoing breach.

166. A client with chronic pain says, "I do not want my adult children making decisions for me while I'm in the hospital. Please speak directly to me." Later, the children repeatedly answer questions before the client can respond. Which nursing action is most appropriate?

- A.** Continue the discussion with the children to avoid conflict
- B.** Direct questions to the client and reinforce the client's role in decision-making
- C.** Ask the children to sign the care plan since they seem involved
- D.** Limit communication with the client because the family may know more

Correct Answer: B

When the client is capable of participating, the nurse should address the client directly and support the client's stated preferences. This action advocates for autonomy without excluding supportive family members entirely. The distractors inappropriately shift control away from the client.

167. An uninsured client is prescribed a wound-care supply that the client says cannot be purchased after discharge. Which action by the nurse best demonstrates advocacy?

- A.** Tell the client to reuse old supplies carefully
- B.** Discharge the client with the current prescription and document the concern
- C.** Notify the appropriate team member to explore lower-cost options and community resources
- D.** Advise the client to skip wound care on days when money is limited

Correct Answer: C

Advocacy includes helping the client access realistic, safe care after discharge. Collaborating with the appropriate team member to identify affordable alternatives or community resources addresses the barrier directly. The other choices either ignore the barrier or promote unsafe care.

168. A client says, “I changed my mind. I do not want my photograph taken for the teaching file anymore.” The staff member holding the camera says, “But you already agreed yesterday.” What should the nurse do?

- A.** Support the client’s current decision and stop the photography
- B.** Encourage the client to continue because the photo could help others
- C.** Ask the provider to persuade the client to cooperate
- D.** Allow the photo if the client’s face is not fully visible

Correct Answer: A

A client may withdraw permission for optional participation, such as educational photography, and the nurse should advocate for that choice. Prior agreement does not eliminate the client’s current right to refuse. The incorrect options prioritize staff goals over the client’s autonomy and privacy.

169. A client with limited English proficiency is brought to the preprocedure area. The client’s bilingual teenage grandchild says, “I can interpret everything.” The client appears anxious and keeps looking between the staff and grandchild. What is the nurse’s best action?

- A.** Use the grandchild because it will be faster and more comfortable for the client
- B.** Delay important communication until a qualified interpreter can be used
- C.** Speak louder in simple English while the grandchild explains difficult words
- D.** Ask the client to sign now and review details after the procedure with an interpreter

Correct Answer: B

For important care communication, the nurse should advocate for communication the client can reliably understand, using a qualified interpreter rather than relying on a minor family member. This protects accuracy, privacy, and the client’s ability to make decisions. Speed and convenience are not the priority here.

170. A client with early dementia is alert, calm, and able to state, “I do not want my nephew to manage my money or sign anything for me.” The nephew tells the nurse, “I’m taking over because the client forgets things sometimes.” What is the nurse’s best response?

- A.** Accept the nephew’s direction because family usually makes financial and health decisions
- B.** Ask the nephew to continue since it will reduce the client’s stress
- C.** Recognize the client’s expressed wishes and report the concern to the nurse in charge or appropriate team member
- D.** Tell the client that occasional forgetfulness means the nephew must take control

Correct Answer: C

The nurse should not assume incapacity based only on some memory loss. The client’s clearly expressed wishes should be respected, and the concerning family pressure should be communicated through the appropriate chain to protect the client. The trap is equating diagnosis or forgetfulness with automatic loss of decision-making rights.

171. A client receiving care for injuries says quietly, “Please don’t give the printed discharge papers to the person who drove me here.” A visitor at the bedside says, “I always keep all of the paperwork.” The discharge packet includes follow-up contacts and shelter resources. What is the nurse’s priority action?

- A.** Hand the packet to the visitor to avoid creating tension
- B.** Place the packet on the bedside table and leave the room
- C.** Arrange a private opportunity to review and give the materials directly to the client
- D.** Ask the visitor whether the client can safely take the paperwork home

Correct Answer: C

The client has directly expressed a safety-related preference, so the nurse should protect confidentiality and provide the materials privately to the client. Asking the visitor for permission or handing over the packet could place the client at greater risk. This is an advocacy issue involving both privacy and safety.

172. A client says, "I do not want to discuss my diagnosis where my employer can hear me on speakerphone." The employer is already on the line because the unit clerk connected the call to the bedside phone. What should the nurse do first?

- A. Continue the call briefly to avoid appearing rude
- B. Ask the employer to hold while the nurse explains the diagnosis to the client
- C. End or remove the employer from the conversation and speak with the client privately about preferences
- D. Tell the client that employment matters require open communication

Correct Answer: C

The immediate priority is to stop the unwanted disclosure and restore the client's control over the conversation. After privacy is protected, the nurse can clarify what information, if any, the client wants shared. Continuing the call even briefly risks violating confidentiality.

173. A client who is alert and oriented tells the nurse, "I know the provider recommended a blood transfusion, but I do not want it." The client's spouse becomes upset and says, "You have to convince my partner to agree." Which action by the nurse is most appropriate?

- A. Ask the spouse to sign the consent form because the spouse supports the transfusion
- B. Explain the treatment again and continue urging the client until the client agrees
- C. Support the client's stated decision and notify the provider of the refusal
- D. Delay documenting the refusal until the family reaches an agreement

Correct Answer: C

A competent, alert client has the right to refuse treatment, even if family members disagree. The nurse's role is to advocate for the client's decision, ensure it is communicated, and notify the provider. Choice A is incorrect because the spouse cannot override an alert client's decision. Choice B is incorrect because repeated pressure becomes coercive rather than advocacy. Choice D is incorrect because the refusal should be documented promptly, not postponed for family consensus.

174. An alert, oriented adult client is preparing for discharge and says, "I do not want my neighbor to know why I was hospitalized." The neighbor arrives to drive the client home and asks the nurse, "What medications were prescribed, and what follow-up visit is this for?" What should the nurse do?

- A. Review the discharge instructions with both the client and neighbor because the neighbor is providing transportation
- B. Ask the client privately whether any health information may be shared with the neighbor before discussing the discharge plan
- C. Give the medication list to the neighbor but avoid discussing the diagnosis
- D. Refuse to discharge the client until a family member arrives instead of the neighbor

Correct Answer: B

The nurse should first clarify the client's wishes privately and obtain permission before sharing any health information. An alert, oriented client has the right to control who receives personal health information. Choice A is incorrect because transportation does not give someone a right to private medical details. Choice C is a common trap: medications and follow-up plans are still protected health information. Choice D is incorrect because a neighbor may provide transportation; the issue is confidentiality, not who is allowed to pick up the client.

175. An alert adult client scheduled for surgery says, "I signed the consent form earlier, but now I don't want the operation." The provider is not on the unit yet, and transport has arrived to take the client to pre-op. What should the nurse do first?

- A. Tell the client the procedure can be discussed again after arrival in pre-op
- B. Notify the surgical team that the client is refusing and stop the transfer
- C. Ask the client's spouse to help persuade the client to proceed
- D. Send the client to pre-op because the consent form is already signed

Correct Answer: B

A client who is alert and has decision-making capacity may withdraw consent at any time before the procedure. The nurse should immediately protect that right by stopping the transfer and notifying the surgical team. A signed form does not cancel the client's later refusal. Asking family to persuade the client shifts pressure onto the client instead of supporting autonomy.

176. A nurse is caring for a competent adult client whose sibling asks, "Can you tell me what the biopsy showed? I handle all the family medical issues." The client is resting and has not given permission to share information. Which response by the nurse is best?

- A.** I can only discuss the results with the client unless the client authorizes sharing that information
- B.** I can give you a brief summary because you are immediate family
- C.** I will tell you the results now and document that you requested them
- D.** I can share the results if you promise not to tell anyone else

Correct Answer: A

The nurse must protect confidentiality and the client's right to control disclosure of personal health information. Family relationship alone does not permit sharing when the client has not authorized it. Documenting the request or receiving a promise of secrecy does not make disclosure appropriate.

177. A client with terminal illness has a documented do-not-resuscitate order. During a bedside handoff, a newly assigned nurse says, "If the client arrests, we should start compressions until the provider arrives." Which action by the charge nurse best advocates for the client?

- A.** Ask the family what they want done if the client arrests
- B.** Clarify that the documented order must be followed and ensure the team is aware of it
- C.** Wait to discuss the issue unless the client actually arrests

D. Remove the order from the chart until the provider can review it

Correct Answer: B

Advocacy includes ensuring the client's expressed end-of-life decisions are honored and clearly communicated. The charge nurse should immediately correct the misunderstanding and make sure the care team knows the documented order. Waiting creates risk, and family wishes do not override an existing valid order simply because staff are uncertain.

178. A client with limited literacy smiles and says, "Don't worry, I signed the paper," when asked about discharge instructions. Which nursing action best supports the client's rights?

- A.** Assume the client understands because the form is signed
- B.** Ask the client to describe, in their own words, how to manage care at home
- C.** Give the client the written instructions again and move on
- D.** Ask a family member to explain the instructions later

Correct Answer: B

Advocacy includes protecting the client's right to understandable information and meaningful participation in care. Using teach-back helps the nurse assess understanding without relying on literacy level or a signature. A signed paper does not prove comprehension, and deferring teaching to family reduces direct support for the client.

179. A client says, "I don't want that nursing assistant helping me with toileting anymore." When the nurse asks why, the client replies, "I just don't feel comfortable with that person." Which nursing action is most appropriate?

- A.** Tell the client a reason is required before assignments can be changed
- B.** Explain that all staff are trained and the client should try to cooperate
- C.** Explore the concern and arrange care in a way that protects the client's dignity and preferences when possible
- D.** Document the statement but continue the assignment unless the client files a written complaint

Correct Answer: C

The nurse should take the concern seriously, assess for safety or boundary issues, and advocate for care that respects the client's dignity and preferences when possible. Requiring the client to justify discomfort or dismissing the concern can undermine the client's rights. The trap is assuming reassignment only matters if there is a formal complaint.

180. A confused older adult arrives with an adult daughter who says, "My mother gets upset if anyone tells her too much. Just explain the diagnosis to me, not to her." The client is awake, makes eye contact, and answers simple questions appropriately. What should the nurse do?

- A. Provide all teaching to the daughter because she requested it
- B. Ask the daughter to leave so no teaching is given to her at all
- C. Direct explanations to the client first and assess the client's understanding and decision-making ability
- D. Avoid discussing the diagnosis until a social worker is available

Correct Answer: C

The client's rights are the priority. If the client appears able to participate, the nurse should speak directly to the client and assess understanding and capacity rather than automatically shifting control to family. The likely trap is assuming age or mild confusion means the daughter should automatically receive information instead.

181. A client receiving opioid medication for severe pain says, "The pills help, but the nurse keeps delaying them because she thinks I'll get addicted." The medication is prescribed as needed and the doses requested have been within the ordered interval. What is the nurse manager's best action?

- A. Support the staff nurse's judgment because caution with opioids is always safest
- B. Review the pain assessments and medication administration record, then address any failure to follow the prescription and pain plan

C. Tell the client that some delay is expected with controlled substances

D. Discontinue the opioid and ask the provider for a nonpharmacologic plan only

Correct Answer: B

Advocacy includes protecting the client's right to appropriate pain management based on assessment and prescribed therapy. The nurse manager should evaluate whether the medication was withheld inappropriately and correct practice if needed. The trap is letting personal bias about addiction override assessment, the prescription, and the client's expressed need.

182. A client with a new ostomy says quietly, "Please don't practice with the bag change while my cousin is watching." The cousin says, "It's fine, I'm family." What should the nurse do?

- A. Proceed because family support is helpful for discharge teaching
- B. Ask the client whether the cousin should step out during the teaching and care
- C. Continue only the teaching portion and postpone the actual care
- D. Allow the cousin to stay unless the provider writes a restriction

Correct Answer: B

The client's preference controls who is present during care and teaching. The nurse should immediately clarify and honor the client's wishes about privacy. Family support can be useful, but only with the client's permission. The trap is assuming a relative may stay by default.

183. A postoperative client becomes drowsy after pain medication. The client's partner says, "Sign the procedure follow-up paperwork now so we can get it done." The client opens eyes briefly but cannot explain what is being signed. What is the nurse's best action?

- A. Witness the signature because the partner is present
- B. Ask the partner to sign instead
- C. Delay signing until the client can participate adequately and notify the appropriate team member if needed

D. Guide the client's hand so the signature matches the chart

Correct Answer: C

A client should be able to understand and participate meaningfully before signing important care-related documents. Because this client is not adequately alert to explain what is being signed, the nurse should delay the process and protect the client's rights. The partner's presence does not replace the client's participation, and helping the client sign would be unethical.

184. At a long-term care facility, a resident says, "I know it's almost dinner, but I want to stay in bed and skip the dining room tonight." A staff member says, "Everyone must come to the dining room unless they're sick." Which nurse response best supports resident rights?

- A.** The resident may choose whether to attend, so let's ask about preferences and offer alternatives
- B.** The staff member is correct because routines help the unit run smoothly
- C.** The resident should attend because socialization is part of treatment
- D.** The resident may skip dinner only if the provider writes an order

Correct Answer: A

Residents retain the right to make reasonable choices about daily routines and personal preferences. The nurse should support autonomy while exploring needs and safe alternatives, such as tray service if appropriate. The common trap is placing unit convenience above the resident's rights.

185. A client with a history of domestic violence is preparing for discharge. The abusive partner calls the unit and says, "I need to know exactly when the client is leaving so I can pick them up." The client previously stated, "Do not give my schedule to that person." What is the nurse's best action?

- A.** Provide the discharge time because transportation is important
- B.** Refuse to share the information and follow facility safety procedures to protect the client
- C.** Ask the caller to verify the client's date of birth, then provide the time

D. Transfer the call to the client's room so the client can handle it

Correct Answer: B

The nurse must protect the client's safety, privacy, and expressed wishes. Discharge timing can be sensitive information, especially in the context of abuse. The nurse should not disclose it and should activate appropriate safety planning steps. Verification questions do not make disclosure appropriate when the client has already refused permission.

186. A competent client says, "I do not want to participate in this research study anymore." A staff member replies, "But the study medication has already started, so you have to finish." What should the nurse do?

- A.** Tell the client withdrawal is not allowed after treatment begins
- B.** Encourage the client to continue to avoid disappointing the research team
- C.** Support the client's decision to withdraw and notify the appropriate research personnel
- D.** Ask the client's family to decide whether withdrawal is reasonable

Correct Answer: C

A competent client has the right to refuse or stop participation in research. The nurse should advocate for that right and notify the appropriate personnel so the withdrawal is handled correctly. The likely trap is confusing enrollment paperwork with an obligation to continue.

187. A nurse sees an assistive personnel member fastening a mitten restraint onto an alert client who is trying to pull at an IV line. There is no restraint order, and the client says, "I don't want these on." What should the nurse do first?

- A.** Tell the assistive personnel to keep the restraints on until the provider rounds
- B.** Remove the restraint, assess the client's immediate safety needs, and intervene with least restrictive measures
- C.** Document that the client is uncooperative and continue the restraint
- D.** Ask the family whether the restraint is acceptable

Correct Answer: B

The nurse must immediately protect the client's rights and safety by stopping an unauthorized restraint and reassessing the situation. Least restrictive alternatives should be used when possible. Family approval does not replace proper clinical and legal processes, and labeling the client uncooperative does not justify ignoring the client's rights.

188. A client is admitted for treatment of an infectious condition. Housekeeping staff begin placing a large sign with the client's diagnosis on the outer door so visitors will know why precautions are needed. Which nursing action is best?

- A.** Allow the sign because infection control takes priority over privacy
- B.** Replace the sign with precaution information that does not reveal the diagnosis
- C.** Move the sign inside the room where only staff can see it after entry
- D.** Ask the client to approve the diagnosis being posted on the door

Correct Answer: B

The nurse should support necessary precautions while still protecting confidentiality. Posting the diagnosis publicly discloses private health information unnecessarily. The better approach is to use signage that communicates required precautions without naming the condition. The trap is assuming safety measures justify avoidable disclosure.

189. A hospitalized adult client is awake, oriented, and speaks limited English. The client's adult son says, "I'll translate for my mother. Don't call an interpreter because it will take too long." The nurse needs to teach the client about a newly prescribed medication and confirm understanding. What is the best action by the nurse?

- A.** Ask the son to interpret because he knows the client's language and can help the nurse finish teaching faster
- B.** Provide the medication handout in English and ask the client to sign that teaching was completed

C. Use a qualified medical interpreter to provide the teaching and verify the client's understanding directly

D. Delay all medication teaching until the provider is available to explain the prescription

Correct Answer: C

Using a qualified medical interpreter best protects the client's right to receive accurate information and participate in care decisions. The nurse should communicate with the client directly and verify understanding, rather than relying on a family member who may omit, change, or filter information. Choice A is a common trap because it seems convenient, but convenience does not outweigh the client's right to clear, unbiased communication. Choice B does not ensure informed understanding, especially when the client has limited English proficiency. Choice D unnecessarily delays needed teaching.

190. A client who is scheduled for a blood transfusion says to the nurse, "I changed my mind. I do not want any blood products for personal reasons." The provider's prescription is active, and another nurse says, "The hemoglobin is low, so we have to give it." What is the nurse's best action?

- A.** Begin the transfusion because the provider already prescribed it
- B.** Ask the family to persuade the client to accept the transfusion
- C.** Respect the refusal, hold the transfusion, and notify the provider
- D.** Delay the transfusion briefly, then administer it if the client does not change the decision

Correct Answer: C

A competent client has the right to refuse treatment, even if the treatment is recommended. The nurse should advocate for that right by not initiating the transfusion and by notifying the provider of the refusal. Choice A and D violate client autonomy. Choice B is inappropriate because the decision belongs to the client, not the family.

191. During morning care in a semiprivate room, a client says, "I don't want to discuss my diagnosis while my roommate and visitors are here." The provider is expected to round in a few minutes. Which nursing action best protects the client's rights?

- A.** Tell the client that brief bedside discussions are part of routine care
- B.** Ask the provider to discuss the client's condition in a more private setting
- C.** Encourage the client to speak quietly so others cannot hear
- D.** Wait until the provider arrives, then remind the visitors not to interrupt

Correct Answer: B

The nurse should advocate for the client's privacy by arranging for health information to be discussed in a more private setting. This directly addresses the client's stated concern. Choice A dismisses the client's right. Choice C still leaves a risk of being overheard. Choice D does not solve the privacy problem because the roommate and visitors would still be present.

192. A client with chronic kidney disease says, "I want to leave the hospital today even though the provider wants me to stay." The client is alert, oriented, and able to explain the risks of leaving. Which response by the nurse is best?

- A.** You cannot leave until the provider writes a discharge order
- B.** If you leave, your insurance will refuse to pay for your care
- C.** Let me review the risks with you and notify the provider of your decision
- D.** I will ask your family to come in before you make that choice

Correct Answer: C

An alert, decision-capable client has the right to make decisions about care, including leaving after being informed of the risks. The nurse's role is to support informed decision-making, communicate the decision, and follow facility process. Choice A incorrectly removes the client's autonomy. Choice B is coercive and may not be true. Choice D shifts decision-making to the family instead of the client.

193. A client says, "I told the night nurse I do not want students involved in my care." Later that morning, a student nurse enters with the assigned nurse and begins preparing supplies for a dressing change. What should the assigned nurse do?

- A.** Explain that students are part of the health care team, so the client must allow participation
- B.** Allow the student to observe only, because observation does not affect the client's rights
- C.** Ask the client to cooperate this one time because the procedure is important for learning
- D.** Honor the client's refusal and assign the dressing change to licensed staff only

Correct Answer: D

Clients have the right to refuse involvement in teaching activities such as student participation in care. The nurse should advocate for that preference and arrange care accordingly. Choice A is incorrect because student participation is not mandatory. Choice B is still inappropriate if the client refused student involvement. Choice C pressures the client and fails to respect the client's stated wishes.

194. A client with advanced illness has an advance directive stating no feeding tube placement. During a hospitalization, the client becomes unable to communicate. The spouse tells the nurse, "Do not mention that document. I want everything done, including a feeding tube." The advance directive is in the chart. What is the nurse's best action?

- A.** Arrange for the feeding tube because the spouse is now the decision-maker
- B.** Delay action until the family reaches agreement about what to do
- C.** Inform the provider of the documented directive and advocate that the client's stated wishes be followed
- D.** Remove the advance directive from the chart until its legal status can be reviewed later

Correct Answer: C

When a valid advance directive is available, the nurse should advocate for the client's previously expressed wishes, especially when the client can no longer speak for themselves. Choice A is a common trap: surrogate input does not override documented client wishes in this scenario. Choice B delays needed advocacy. Choice D is clearly inappropriate and undermines the client's rights.

Confidentiality and privacy

195. During change-of-shift report in a semiprivate room, the nurse begins discussing one client's new diagnosis while the roommate and a visitor are present. Which action should the practical nurse take first?

- A. Continue the report but lower the speaking volume
- B. Ask the roommate's visitor to leave the room immediately
- C. Move the discussion to a more private area
- D. Document that privacy could not be maintained during report

Correct Answer: C

The priority is to protect confidentiality by moving the conversation to a private area. Lowering the voice does not adequately protect private information, and asking the visitor to leave does not address the roommate's presence. Documentation does not correct the breach.

196. A practical nurse prints a client's laboratory results for morning rounds and realizes the papers were left unattended at the nurses' station for 10 minutes. Which action is most appropriate?

- A. Place the results in the chart and take no further action
- B. Shred the results and reprint them later
- C. Retrieve the papers at once and report the privacy incident per facility policy
- D. Ask staff whether anyone read the papers before deciding what to do

Correct Answer: C

Leaving protected health information unattended creates a potential privacy breach. The nurse should immediately secure the documents and follow facility policy for reporting the incident. Taking no action or waiting to see whether someone looked at the papers fails to address the breach. Shredding and reprinting alone does not meet reporting responsibilities.

197. The practical nurse is assisting with discharge teaching for a high-profile local athlete. A staff member from another unit says, "I just want to know why he was admitted." What is the best response?

- A. "I'll tell you briefly, but do not repeat it."
- B. "You may review the chart after your shift ends."
- C. "Only staff directly involved in his care should access that information."
- D. "It is acceptable to discuss because he is a public figure."

Correct Answer: C

Client information is shared only with staff who need it for care. Curiosity, even by another health care worker, does not justify access. Public status does not reduce privacy rights. Choice A is a trap because it tries to limit further disclosure, but the initial disclosure is still improper.

198. A nursing student posts on social media after clinical: "Tough day caring for a 42-year-old burn patient rescued from the downtown warehouse fire." The post does not include the client's name. Which statement best explains why this is inappropriate?

- A. It is acceptable because no name was used
- B. It is inappropriate because details in the post may still identify the client
- C. It is acceptable if the account is private
- D. It is inappropriate only if a photo of the client is included

Correct Answer: B

Even without a name, enough specific details can identify a client. Privacy can be breached through age, event, diagnosis, location, or timing. Private accounts do not make disclosure acceptable, and photos are not required for a breach to occur.

199. A client asks the practical nurse, "Can my spouse receive updates about my treatment while I'm in surgery?" What is the best initial nursing action?

- A.** Ask the client whom they want information shared with and follow facility procedures for authorization
- B.** Tell the client that spouses automatically receive all updates
- C.** Advise the client to wait until after surgery to decide
- D.** Document that no information will be released to anyone

Correct Answer: A

The nurse should determine the client's wishes and ensure information sharing follows the facility's authorization process. A spouse does not automatically have unlimited access to information. Waiting delays an important privacy decision, and documenting no release ignores the client's expressed request.

200. The practical nurse receives a text from an unrecognized number that says, "This is Dr. Lee. Send me Ms. Gomez's blood glucose trends from today." What should the nurse do first?

- A.** Reply with the values because the sender identified as the provider
- B.** Send a photo of the chart because it is faster
- C.** Verify the request through approved communication channels before sharing any information
- D.** Ignore the text and delete it without informing anyone

Correct Answer: C

The nurse should first verify the identity of the requester and use approved, secure communication methods. A text from an unknown number is not enough to confirm authorization. Sending a photo of the chart adds another privacy risk. Simply deleting the message does not address a potentially improper request for protected information.

201. A client on a medical-surgical unit tells the practical nurse, "I don't want my roommate overhearing my teaching about HIV medications." Which intervention is best?

- A.** Speak quietly at the bedside and continue
- B.** Provide the teaching in a private area if possible
- C.** Ask the roommate not to listen
- D.** Delay all teaching until discharge day

Correct Answer: B

Sensitive teaching should be moved to a private area when possible to protect confidentiality and support honest discussion. Speaking quietly at the bedside may still allow others to overhear. Asking the roommate not to listen does not ensure privacy. Delaying necessary education is not appropriate.

202. The practical nurse is floating to another unit and opens the electronic record of a former client "just to see how she is doing." Which statement is most accurate?

- A.** This is acceptable because the nurse previously cared for the client
- B.** This is acceptable if no information is printed
- C.** This is inappropriate because access must be related to current job responsibilities
- D.** This is appropriate if the nurse does not tell anyone what was seen

Correct Answer: C

Accessing a record out of curiosity is a confidentiality violation. Prior involvement in care does not grant ongoing access without a current need to know. Whether information is printed or discussed does not change the improper access itself.

203. A practical nurse is caring for an adolescent client who asks that a pregnancy test result not be discussed in front of accompanying family members. The family later asks, "What did the test show?" What is the nurse's best action?

- A.** State that results cannot be discussed without the client's permission and arrange a private conversation with the client
- B.** Share the result because family members brought the client to the visit
- C.** Tell the family that they will need to check the online portal later
- D.** Avoid answering and leave the room without explanation

Correct Answer: A

The nurse should protect the client's privacy and clearly state that results cannot be shared without permission, then continue communication privately with the client. The escorting family does not automatically gain access to confidential information. Deflecting to the portal or leaving the room avoids the issue instead of handling it professionally.

204. At the end of the shift, a practical nurse finds a worksheet in a uniform pocket containing room numbers, diagnoses, and medication times. Which action is best?

- A.** Take the worksheet home to finish charting accurately
- B.** Return the worksheet to the unit unsecured so day shift can use it
- C.** Dispose of the worksheet according to secure facility procedure and avoid removing such documents from the clinical area
- D.** Keep the worksheet until the client is discharged, then throw it away

Correct Answer: C

Client care notes that contain identifying information must be handled securely. The best action is to dispose of the worksheet according to facility procedure and correct the unsafe practice of carrying it away from the care area. Taking it home or keeping it longer extends the privacy risk.

205. A hospitalized client's adult child says, "My mother is confused, so you should give me her password to the patient portal." Which response by the practical nurse is most appropriate?

- A.** "I can give you the password since you are family."
- B.** "Passwords should not be shared. We can follow the proper process for authorized access."
- C.** "Use your mother's birth date and room number to log in."
- D.** "I will print the entire chart for you instead."

Correct Answer: B

Passwords are personal security credentials and should not be shared. If the family member is to receive access, it must occur through the appropriate authorization process. Offering substitutes such as printing the chart or suggesting guessed login information also violates privacy and security standards.

206. A practical nurse overhears an assistive personnel staff member discussing a client's cancer diagnosis in the cafeteria. What should the nurse do?

- A.** Wait to see whether the client complains before acting
- B.** Join the conversation so the details are accurate
- C.** Address the staff member promptly and follow facility reporting procedures for the breach
- D.** Document the diagnosis more clearly in the chart

Correct Answer: C

Discussing client information in a public area is a confidentiality breach. The nurse should intervene promptly and follow policy for reporting and addressing the incident. Waiting for a complaint delays action, and joining the conversation increases the breach.

207. A client who is alert and oriented tells the nurse, "Do not share my diagnosis with my brother." Later, during a bedside visit, the brother asks, "Why is she getting that medication?" The client is resting with eyes closed and does not respond. What is the best nursing response?

- A.** Explain the medication because the brother is present at the bedside

- B.** Discuss the diagnosis quietly so the client can continue resting
- C.** Tell the brother that you cannot discuss the client's care without the client's permission
- D.** Give a partial explanation without naming the diagnosis

Correct Answer: C

The client has already expressed a clear wish that information not be shared with the brother. The nurse must honor that directive, even if the brother is present and the client appears asleep. Partial disclosure is still a breach. Presence at the bedside does not equal authorization.

208. A practical nurse is preparing to leave a voicemail for a client who missed a follow-up appointment. Which message best protects client privacy?

- A.** Hello, this is the clinic calling about your abnormal test results. Please call us back today.
- B.** Hello, this is Jamie from Dr. Patel's office. Please return our call at 555-0184 regarding your appointment.
- C.** Hello, this is the medical office calling to discuss your new medication for depression. Please call before 5 PM.
- D.** Hello, this is your nurse. We need to reschedule your visit for high blood pressure management.

Correct Answer: B

Choice B shares the minimum necessary information: the caller's name, the office, a callback number, and a general reason to return the call. It avoids revealing diagnoses, test results, or treatment details that someone else could overhear. Choices A, C, and D inappropriately disclose private health information in the voicemail.

209. A practical nurse is updating a client's daughter at the nurses' station when the client states, "I don't want any information discussed where other people can hear it." Which action should the nurse take?

- A.** Continue the conversation because the daughter is family and the client is present
- B.** Ask the daughter to lower her voice and finish the update quickly

- C.** Move the discussion to a private area after confirming the client's permission to share information
- D.** Tell the daughter to read the information on the whiteboard in the client's room instead

Correct Answer: C

Choice C best protects confidentiality and respects the client's stated preference. Even when the client allows information sharing with family, the nurse should avoid discussing protected information where others may overhear. Choice A ignores the client's privacy concern. Choice B does not adequately prevent incidental disclosure. Choice D could still expose private information and does not address the need for a private conversation.

210. A practical nurse is reviewing assigned tasks with an unlicensed assistive personnel (UAP). Which instruction best supports client confidentiality during routine care?

- A.** "If a family member asks questions, answer if the person seems involved in the client's care."
- B.** "Discuss client information only with staff who need it to provide care."
- C.** "Share updates with other clients if it helps explain unit delays."
- D.** "Leave the assignment sheet at the front desk so anyone can find you if needed."

Correct Answer: B

B is correct because confidentiality means limiting access to client information to personnel who need that information to provide care. A is a common trap because family involvement does not automatically authorize disclosure. C and D both inappropriately expose private information.

211. A client is being transported for an x-ray. While in a crowded elevator, the transporter asks the practical nurse, "Is this the client with the positive tuberculosis test?" What is the best response by the nurse?

- A.** "I'll discuss the client's information with you in a private area."
- B.** "Yes, but the client has already started treatment."
- C.** "No, that is a different client from this hall."
- D.** "Ask the charge nurse after the x-ray is finished."

Correct Answer: A

A is correct because even confirming or denying sensitive information in a public space can violate privacy. The nurse should move the conversation to a private setting if the information is needed for care. B and C both disclose information inappropriately. D delays communication but does not directly protect confidentiality in the moment.

212. A practical nurse is caring for a client who asks, "Can you tell my neighbor that I'm doing better if she calls the unit?" Which action is most appropriate?

- A.** Tell the client that only immediate family can receive updates.
- B.** Ask the client to identify specifically what information may be shared and with whom.
- C.** Inform the client that telephone updates are never allowed.
- D.** Tell the client to have the neighbor call after discharge.

Correct Answer: B

B is correct because the client has the right to direct who may receive information, but the nurse should clarify the client's wishes before sharing anything. A and C are too absolute. D ignores the client's current preference and does not address confidentiality appropriately.

213. A practical nurse is documenting on a computer in a shared work area. The nurse needs to step away quickly to assist with a client who is calling for help. Which action is best?

- A.** Leave the record open because the nurse will return in less than 1 minute.
- B.** Ask a visitor near the desk to watch the screen.
- C.** Log off or lock the computer before leaving.
- D.** Turn the monitor slightly toward the wall and walk away.

Correct Answer: C

C is correct because electronic records must be secured when left unattended, even briefly. A and D still allow possible unauthorized viewing. B is inappropriate because a visitor should never be involved in protecting access to client records.

214. A practical nurse is giving report to the oncoming nurse in a private conference area. A dietary employee enters to restock supplies and remains in the room. The report includes details about a client's substance use history. What should the nurse do?

- A.** Continue because the dietary employee works in the facility.
- B.** Lower the voice and finish quickly.
- C.** Pause the report until only staff involved in the client's care are present.
- D.** Ask the dietary employee to promise not to repeat what is heard.

Correct Answer: C

C is correct because access to client information should be limited to those who need it for care. Working in the facility alone does not authorize hearing private report details. B and D do not solve the confidentiality problem.

215. A practical nurse receives a request from the physical therapy department for a client's latest mobility restrictions before a treatment session. The nurse also receives a request from a neighbor of the client, who says, "I drove her here, so I should know what's going on." Which request may the nurse appropriately address?

- A.** Neither request, because no information can ever be shared.
- B.** Only the neighbor's request, because the neighbor assisted with transportation.
- C.** Only the physical therapy request, because it relates to the client's care.
- D.** Both requests, because each person has a reason for asking.

Correct Answer: C

C is correct because interdisciplinary team members involved in the client's care may receive information needed to provide that care. The neighbor does not gain access simply by helping with transportation. A is too broad because necessary care-related sharing is allowed.

216. A practical nurse is teaching a newly hired assistive personnel (AP) about confidentiality. Which statement by the AP indicates a need for further teaching?

- A. "I should avoid discussing clients in the hallway."
- B. "I can review the chart of any client on the unit if I am curious about the diagnosis."
- C. "I should keep written notes with client information out of public view."
- D. "I need to report privacy concerns to the nurse."

Correct Answer: B

B is correct because accessing a chart out of curiosity is not permitted; staff should view only information needed to perform assigned duties. A, C, and D reflect appropriate confidentiality practices.

217. The practical nurse is caring for a client who speaks limited English. During medication teaching, the client's adult grandson says, "I can translate everything. Just tell me all the medical details." The client appears uncomfortable and looks away. What is the nurse's best action?

- A. Continue using the grandson as the interpreter because he is family.
- B. Ask the grandson to sign that he will keep the information private.
- C. Pause the discussion and arrange for appropriate interpretation while protecting the client's privacy preferences.
- D. Give the teaching materials to the grandson and document that education was completed.

Correct Answer: C

C is correct because the client's discomfort suggests a privacy concern, and the nurse should protect confidentiality while ensuring accurate communication. A is a common trap because family translation can interfere with privacy and accuracy. B does not resolve the client's discomfort. D is incomplete and may improperly shift private information to the grandson.

218. A practical nurse notices that a fax containing a client's wound care plan was sent to the wrong outpatient office. Which action should the nurse take first?

- A. Call the receiving office, explain the error, and request secure destruction of the fax according to facility procedure.
- B. Wait to see whether the outpatient office calls back.
- C. Send the correct fax immediately and ignore the original transmission.
- D. Tell the client at discharge if the issue comes up later.

Correct Answer: A

A is correct because the nurse should respond promptly to limit further disclosure and follow facility procedure for privacy incidents. B delays action. C addresses care coordination but ignores the breach. D postpones an immediate problem instead of addressing it.

219. A practical nurse is precepting a new graduate. Which situation is the clearest violation of confidentiality?

- A. Reviewing the medication administration record before giving a scheduled dose
- B. Reporting abnormal findings to the supervising registered nurse
- C. Discussing a client's prognosis with a friend after work without using the client's name
- D. Sharing a client's intake and output totals during handoff report

Correct Answer: C

C is correct because discussing client information with someone not involved in care is a confidentiality violation, even if the client's name is omitted. Details may still identify the client or reveal protected information. A, B, and D are care-related uses of information.

220. At a community clinic, a practical nurse calls a client from the waiting room using the client's full name and diagnosis-related service, saying, "Mr. Daniels, come for your HIV follow-up visit." What is the main confidentiality concern?

- A. The nurse used the client's last name instead of the first name.
- B. The nurse disclosed sensitive health information where others could hear it.

- C. The nurse should never speak to clients in the waiting room.
- D. The nurse should have asked another client to relay the message.

Correct Answer: B

B is correct because the problem is the public disclosure of sensitive visit information. A is not the main issue. C is too absolute; clients may be called from a waiting room, but private information should not be announced. D would worsen confidentiality.

221. A practical nurse is asked by a police officer at the nurses' station, "Tell me whether the client in room 214 admitted using illegal drugs." The client is stable and no consent for sharing information has been discussed. What is the nurse's best initial action?

- A. Provide the information because law enforcement requested it.
- B. Ask the officer to wait and follow facility procedure for handling the request.
- C. Tell the officer everything except the client's laboratory results.
- D. Ask the client's roommate whether the officer's statement is true.

Correct Answer: B

B is correct because the nurse should not release client information simply because it is requested by law enforcement. The proper next step is to follow facility procedure and appropriate channels before any disclosure. A and C improperly release information. D is entirely inappropriate and creates another privacy breach.

222. A practical nurse is completing interdisciplinary rounds in a semiprivate room. The client's roommate is awake, and the team wants to discuss the client's new diagnosis, discharge barriers, and family conflict. Which action best protects confidentiality while still supporting care coordination?

- A. Proceed at the bedside because rounds are part of treatment.
- B. Ask the roommate to listen quietly and not repeat what is heard.

C. Limit bedside discussion to immediate care needs and move sensitive details to a private area.

- D. Cancel rounds entirely until the client is transferred to a private room.

Correct Answer: C

C is correct because it balances necessary care coordination with protection of sensitive information. Bedside rounds can occur, but private topics should be moved to a setting where unauthorized persons cannot overhear. A and B inappropriately expose private details. D is unnecessary and may interfere with care.

223. A practical nurse working in a long-term care facility receives this written note from a client's daughter: "Mom forgets everything. Please leave all updates and test results with me, even if she says not to." The client is awake, usually able to make needs known, and has previously asked that some information not be shared with family. What should the nurse do next?

- A. Follow the daughter's request because family usually manages long-term care decisions.
- B. Review the client's current preferences and share information only as authorized.
- C. Stop documenting discussions with the client to avoid family conflict.
- D. Give the daughter complete access because memory problems automatically remove privacy rights.

Correct Answer: B

B is correct because the nurse should not assume that family requests override the client's preferences. The client's current wishes and authorization should guide disclosure. A and D are common traps because family involvement does not automatically eliminate the client's privacy rights. C is inappropriate because accurate documentation remains essential.

224. A practical nurse is chatting with a coworker in the cafeteria during lunch. The coworker asks, "How is the young burn patient from room 306 doing?" The coworker is assigned to another unit and is not involved in the client's care. Which response by the nurse is most appropriate?

- A. Give a brief update without using the client's name
- B. Share only information that the family already knows
- C. State that client information should be discussed only with staff involved in the client's care
- D. Answer the question because both people are employees of the facility

Correct Answer: C

Confidential information should be shared only with people who need it to provide care. The coworker is not part of this client's care team, so the nurse should not discuss the client's condition. A common trap is thinking that leaving out the client's name makes the discussion acceptable, but details such as age, diagnosis, or room number may still identify the client.

225. While making home visits, a practical nurse receives a text message from a friend who works at another clinic: "I heard your patient Mrs. Lane was admitted last week. What happened to her?" Which action should the nurse take?

- A. Reply with a short summary because the friend is also a healthcare worker
- B. Ignore the message or respond that client information cannot be shared
- C. Provide the information if the friend promises to keep it private
- D. Send only the diagnosis and no other details

Correct Answer: B

The nurse should not disclose client information to a person who is not involved in the client's care. The safest response is not to share information and, if replying, to state that confidentiality prevents discussion. Being a healthcare worker does not create a right to know. The trap is assuming professional status alone allows access.

226. A practical nurse prints a client's laboratory results at the unit printer and finds another client's discharge instructions already sitting in the output tray. What is the nurse's best action?

- A. Read the papers to identify whose instructions they are
- B. Leave the papers there so the correct nurse can collect them later
- C. Secure the papers and give them to the appropriate staff member according to facility procedure
- D. Place the papers in a public recycle bin because they were printed by mistake

Correct Answer: C

Printed health information should be protected from unnecessary viewing. The nurse should secure the document and route it appropriately according to facility procedure. Leaving it unattended risks a privacy breach, and placing it in regular recycling is unsafe. Reading more of the document than needed is also inappropriate.

227. During change of shift, a practical nurse realizes that the handoff report is taking place in a hallway where visitors are seated nearby. Which action is most appropriate?

- A. Continue the report because shift handoff is required
- B. Speak more quietly but include all details
- C. Move the report to a more private location
- D. Skip important details and let the next nurse read the chart later

Correct Answer: C

The nurse should protect confidentiality by moving the discussion to a private area. Handoff is necessary, but it must occur in a setting that limits unauthorized disclosure. Simply lowering the voice may not be enough if visitors can still overhear.

228. A client tells the practical nurse, "If my employer calls, you may tell them only that I am in the hospital and not discuss anything else." Later that day, the employer calls and asks, "What condition is the client being treated for, and when can the client return to work?" Which response is best?

- A. Provide the diagnosis but not the treatment plan

- B.** Confirm the hospitalization and state that no additional information may be shared
- C.** Refuse to confirm the client is hospitalized
- D.** Discuss the return-to-work date because it is work-related

Correct Answer: B

The nurse should follow the client's stated authorization exactly. The client allowed disclosure only of the hospitalization, not the diagnosis or expected return date. Choice C ignores the client's permission, while A and D go beyond it. This question tests careful attention to the limits of authorized sharing.

229. A practical nurse enters a semiprivate room to teach a client about a newly prescribed antidepressant. The client's roommate and the roommate's visitors are present. What should the nurse do first?

- A.** Give the teaching as planned because medication education cannot be delayed
- B.** Ask the visitors to stay but not comment during the teaching
- C.** Assess whether the client wants the discussion moved to a private setting or limited until privacy is available
- D.** Hand the client the medication information sheet and leave without discussion

Correct Answer: C

The nurse should first protect privacy by addressing the setting and the client's preference before discussing sensitive information. Education is important, but the environment matters. A common trap is assuming necessary care always justifies open discussion, but sensitive teaching should be moved or limited when others are present.

230. A practical nurse notices another staff member searching the electronic record of a well-known community member who was admitted earlier that day. The staff member is not assigned to the client and says, "I just want to see why everyone is talking about this case." What should the nurse do?

- A.** Say nothing because the staff member is licensed
- B.** Tell the staff member to stop and report the concern through the appropriate chain of command

- C.** Allow it as long as no information is printed
- D.** Ask the staff member to close the chart after checking only the admitting diagnosis

Correct Answer: B

Accessing a chart out of curiosity is a privacy violation, even if the person works in the facility. The nurse should intervene and report the issue according to policy. The main trap is confusing general employment or licensure with a need-to-know basis for access.

231. In a rehabilitation unit, a practical nurse receives a phone call from a person who says, "I am the client's brother, and I need the latest test results right now." The client is currently in therapy and has not identified approved phone contacts in the chart. Which action is most appropriate?

- A.** Give a limited update because the caller is family
- B.** Ask the caller to verify the client's date of birth, then provide the results
- C.** Tell the caller that information cannot be released at this time without confirmed authorization
- D.** Transfer the call to another nurse so the unit remains helpful

Correct Answer: C

Without confirmed authorization, the nurse should not release health information by phone. Relationship claims and identifying information from the caller do not, by themselves, establish permission to disclose. A frequent trap is assuming family membership automatically allows access.

232. A practical nurse is documenting care on a portable device at the bedside. The client's visiting friend stands nearby and begins reading the screen. What is the nurse's best response?

- A.** Continue charting because the friend is a guest of the client
- B.** Turn the device away and continue only after protecting the screen from view
- C.** Ask the client whether the friend may read the charting
- D.** Read the note aloud so the friend hears the same information

Correct Answer: B

The nurse should immediately protect the display from unauthorized viewing. Visitors do not have automatic access to the client's record. Even if the client later wants information shared, the nurse should first stop the unintended exposure. The trap is delaying action while seeking permission after the privacy risk has already occurred.

233. A practical nurse is preparing a client for discharge. The client's spouse says, "I handle all the bills, so give me the written instructions and medication list." The client is alert and says, "I want to look at everything myself first, and then I'll decide what to share." Which action should the nurse take?

- A.** Give the spouse the papers because spouses usually assist after discharge
- B.** Provide one copy to the spouse and one to the client
- C.** Give the materials to the client and involve the spouse only as the client allows
- D.** Refuse to include the spouse in any discharge discussion

Correct Answer: C

An alert client directs who receives personal health information. The nurse should provide the discharge information to the client first and include the spouse only as authorized by the client. Choice D goes too far because the spouse may be involved if the client wishes. The key principle is client control over disclosure when decision-making ability is intact.

234. At the end of the shift, a practical nurse realizes that a worksheet used for client care includes room numbers, diagnoses, and medication times. The nurse is leaving the unit. Which action best protects confidentiality?

- A.** Take the worksheet home in case questions come up later
- B.** Leave the worksheet folded in a uniform pocket for the next shift
- C.** Dispose of the worksheet using the approved secure method or return it per policy
- D.** Place the worksheet face down at the nurses' station

Correct Answer: C

Client worksheets containing identifying information must be secured or destroyed using approved methods. Taking them home, leaving them in clothing, or placing them unattended at the desk risks unauthorized disclosure. This tests practical privacy protection in routine workflow.

235. A practical nurse is caring for an adolescent client. The client's parent asks to stay in the room while the client answers questions about sexual history and pregnancy concerns. The client becomes quiet and avoids eye contact. Which nursing action best protects confidentiality while supporting care?

- A.** Continue the interview with the parent present because parents usually need complete information
- B.** Ask the client privately whether some questions should be discussed without the parent in the room
- C.** Tell the parent to leave because family members are never allowed during assessments
- D.** Skip the sensitive questions to avoid conflict

Correct Answer: B

The nurse should create an opportunity to assess the adolescent client's privacy needs and preferences, especially for sensitive topics. Choice B protects confidentiality and supports accurate assessment. Choice A may suppress important information, C is too absolute, and D compromises care.

236. A practical nurse receives a voicemail from a client's adult child stating, "Dad wants me to manage everything now. Call me back with his biopsy result." The chart contains no documented permission, and the client is awake, oriented, and scheduled to return from a procedure in 20 minutes. What is the best action?

- A.** Return the call and leave the result on voicemail because the family member initiated the contact
- B.** Call back and provide only whether the result is normal or abnormal
- C.** Wait to verify the client's wishes before sharing any information

D. Assume the family member is authorized because an adult child often helps with decisions

Correct Answer: C

Because the client is available soon and can express preferences, the nurse should first verify the client's wishes before disclosing any information. Voicemail is especially risky because identity and privacy cannot be confirmed. The trap is assuming that a close family relationship or helpful role equals authorization.

237. A practical nurse finds a photo on social media showing a de-identified wound from the unit, posted by another staff member with the caption, "Crazy case tonight." No name, room number, or face is visible. Which interpretation is most accurate?

- A.** It is acceptable because no name or face is shown
- B.** It is acceptable if the post is shared only with private followers
- C.** It may still violate confidentiality and should be reported through the appropriate channels
- D.** It is acceptable if the staff member took care of the client personally

Correct Answer: C

Posting client-related images or details on social media can violate confidentiality even when obvious identifiers are removed. Clinical images, timing, context, and unusual features can still identify a client or represent improper use of client information. Private account settings do not eliminate the risk. The nurse should treat this as a potential privacy breach and report it appropriately.

238. A practical nurse is giving bedside care when a client says quietly, "Do not discuss my diagnosis in front of my neighbor when she visits later." Two hours later, the neighbor arrives and asks the nurse, "What exactly is wrong with him? I am helping him at home." What is the nurse's best response?

- A.** Explain the diagnosis because the neighbor says she will be helping with care at home
- B.** Ask the neighbor to wait while the nurse checks whether the client wants information shared

C. Provide a brief summary without details because the neighbor is not family

D. Tell the neighbor the diagnosis but avoid discussing the treatment plan

Correct Answer: B

The client has already expressed a clear preference not to discuss the diagnosis in front of this person. The nurse should protect confidentiality and confirm the client's wishes before sharing any information. Choice A is a common trap because helping at home does not automatically authorize access to private health information. Choices C and D are still privacy violations because they disclose health information without the client's permission.

239. A practical nurse is riding the elevator with a respiratory therapist after leaving a client's room. The therapist says, "The client in 214 has such a sad story." Several visitors are also in the elevator. Which response by the nurse is most appropriate?

- A.** Discuss the client quietly so only the therapist can hear
- B.** Wait until the elevator is less crowded and then continue the conversation
- C.** Tell the therapist that client information should be discussed only in a private area
- D.** Ask the visitors to step out because staff need to talk about the client

Correct Answer: C

Client information should not be discussed in public areas where others may overhear. Redirecting the conversation to a private setting best protects confidentiality. Choice A and B still allow a privacy breach, and D is inappropriate because staff should not use a public elevator for confidential discussion.

240. A practical nurse is calling a provider's office to report a client's change in condition from the nurses' station. Family members of other clients are nearby. What is the best action by the nurse?

- A.** Provide the full report quickly before anyone can interrupt
- B.** Move to a more private location before sharing identifying information
- C.** Ask nearby visitors not to listen to the phone call

D. Use the client's room number only and discuss the rest of the details openly

Correct Answer: B

When discussing protected health information by phone, the nurse should reduce the risk of others overhearing by moving to a private area when possible. A and D still risk disclosure. C places responsibility on bystanders rather than on the nurse to protect confidentiality.

241. A practical nurse receives a paper census sheet that lists client names, room numbers, and diagnoses. The nurse no longer needs it after the shift. Which action is most appropriate?

- A.** Place it in the regular trash can after folding it
- B.** Take it home to review assignments for the next shift
- C.** Leave it at the desk in case another nurse needs it
- D.** Dispose of it in the designated secure shred container

Correct Answer: D

Documents containing client identifiers must be destroyed according to facility policy, such as secure shredding. Throwing it in regular trash, taking it home, or leaving it unattended risks unauthorized disclosure.

242. A client asks the practical nurse, "Can my daughter sit in while you explain my new diet plan?" What is the nurse's best response?

- A.** Proceed with teaching only after confirming the client's permission
- B.** Refuse because teaching must always be done one-on-one
- C.** Teach the daughter separately so the client does not need to hear it again
- D.** Ask the daughter to sign the chart before remaining in the room

Correct Answer: A

The client controls who may be present for discussion of health information when the client has decision-making capacity. Confirming permission supports both confidentiality and client autonomy. B is too rigid, C excludes the client from the teaching, and D is not the key issue here.

243. A practical nurse logs into the electronic health record on a workstation and is called away for an urgent assist in another room. What should the nurse do first?

- A.** Leave the chart open because the nurse will return shortly
- B.** Ask a nearby unlicensed assistive person to watch the screen
- C.** Log off or lock the workstation before leaving
- D.** Turn the monitor slightly away from the hallway and leave

Correct Answer: C

Locking or logging off the workstation prevents unauthorized viewing or access to the client's record. A and D still leave protected information exposed. B is inappropriate because watching the screen is not the same as securing access.

244. A practical nurse is asked to fax a client's wound care orders to a home health agency that will continue treatment after discharge. Which action best protects confidentiality while supporting care coordination?

- A.** Send the fax to the number written on an old referral form without checking it
- B.** Fax the record after verifying the recipient and using only the necessary information
- C.** Refuse to fax anything because all faxing violates privacy rules
- D.** Send the entire chart so the agency has complete background information

Correct Answer: B

Appropriate information sharing for ongoing treatment is allowed, but the nurse should verify the destination and send only the minimum necessary information relevant to care coordination. A risks misdirection, C is too absolute, and D shares more information than needed.

245. A practical nurse caring for a client who speaks limited English asks a bilingual housekeeping employee to translate a discussion about surgical risks because no interpreter is immediately available. Which concern is most important?

- A.** The employee may not know the client's room number
- B.** Housekeeping staff cannot translate casual conversation at work

- C. Using an unqualified interpreter can compromise both accuracy and privacy
- D. The provider, not the nurse, should decide who enters the room

Correct Answer: C

Using an unqualified person to interpret sensitive health information can lead to errors and unnecessary disclosure of private information. C identifies the main confidentiality and safety concern. A is irrelevant, B is too broad, and D avoids the core issue.

246. A practical nurse receives a request from the billing department for details about a client's mental health history. The nurse notes that only the current admission diagnosis is needed to correct an insurance coding error. What principle should guide the nurse's response?

- A. Share all past and present diagnoses because the request comes from the same facility
- B. Share only the information needed to complete the specific billing task
- C. Refuse to provide any information because financial staff are not clinicians
- D. Tell the billing department to ask the client's family instead

Correct Answer: B

The minimum necessary principle supports sharing only the information required for the stated work-related purpose. A discloses more than needed. C is too restrictive because some internal sharing may be appropriate for operations. D is inappropriate because family members are not the proper source for billing documentation.

247. A practical nurse accidentally hands one client the after-visit summary for another client in the discharge area. The nurse immediately retrieves it before the client reads beyond the first line. What is the priority action after securing the paper?

- A. Document and report the privacy incident according to facility policy
- B. Say nothing because the paper was recovered quickly
- C. Ask the client not to mention the event to anyone

- D. Place the paper back in the chart and continue discharge

Correct Answer: A

After stopping the exposure, the nurse should report and document the privacy incident according to policy so it can be evaluated and managed appropriately. B and C hide the error. D ignores the need for follow-up after a potential breach.

248. A practical nurse receives change-of-shift report and learns that a client's test results are highly sensitive. Later, the nurse sees those results displayed on a hallway computer monitor while the assigned nurse steps away. What is the best immediate action?

- A. Read the results fully so the nurse can be updated later
- B. Leave the screen alone because touching another nurse's workstation is inappropriate
- C. Protect the information by locking the screen if possible and notifying the assigned nurse
- D. Turn the monitor off and avoid mentioning it to prevent embarrassment

Correct Answer: C

The priority is to protect the client's private information immediately and then alert the responsible staff member. A is unauthorized access. B fails to stop an ongoing risk. D may interfere with work and still avoids proper follow-up.

249. In a clinic, a practical nurse calls out, "Mr. Lopez, your HIV counseling appointment is ready," in a crowded waiting room. Which revision best protects privacy while still identifying the client for care?

- A. Call only the client's name and ask the client to come with the nurse
- B. Announce the full reason for the visit more quietly
- C. Ask everyone in the waiting room to remain silent before calling the client
- D. Use the client's diagnosis instead of the appointment type

Correct Answer: A

Calling only the client's name is the least revealing way to identify the person while maintaining workflow. B still discloses sensitive information. C is unrealistic and unnecessary. D is even more inappropriate because it directly states protected health information.

250. A practical nurse receives an email from a staff educator requesting examples of charting errors for an in-service presentation. The educator asks the nurse to send screenshots from recent client records because "names can be cropped later." What is the best response?

- A.** Send the screenshots now because the educator works for the facility
- B.** Send screenshots only from clients who were discharged that day
- C.** Decline to send client-record screenshots and follow approved methods for de-identified teaching materials
- D.** Print the screenshots instead of emailing them

Correct Answer: C

Even if names are cropped later, sharing screenshots from actual records through unapproved methods creates a confidentiality risk. The nurse should use approved, properly de-identified educational processes. A and B assume internal use alone makes it acceptable. D changes the format but not the privacy problem.

251. A practical nurse is caring for a client who asks that no health information be shared with family members. Later that day, the client's spouse approaches the nurse at the desk and says, "I'm the spouse, so you have to tell me what the doctor said." What is the nurse's best response?

- A.** Share the update because spouses automatically have full access to all medical information
- B.** Ask the spouse which information seems most important, then decide what is reasonable to disclose

C. State that information cannot be shared without the client's permission and offer to speak with the client about what the client wants disclosed

D. Refer the spouse to another nurse who may be more familiar with confidentiality rules

Correct Answer: C

The client has the right to control who receives personal health information. The nurse should not disclose information to the spouse when the client has stated that family members are not to receive updates. Offering to clarify the client's wishes supports both privacy and communication. Choice A is a common trap because marriage does not automatically override the client's expressed privacy preferences. Choice B still involves unauthorized disclosure, and D avoids the nurse's responsibility.

252. During a busy shift, a practical nurse realizes that a medication administration record for one client was left open on a computer screen in a semi-private work area for several minutes. No unauthorized person is known to have viewed it. Which action is most appropriate for the nurse to take first?

- A.** Close or secure the record immediately, then follow facility policy for reporting the potential privacy incident
- B.** Delete the medication entry so no one can see the information later
- C.** Wait to see whether anyone complains before taking further action
- D.** Ask a coworker to watch the screen while the nurse finishes another task

Correct Answer: A

The priority is to stop any further exposure by securing the record immediately. After that, the nurse should follow facility policy for reporting and documenting a potential privacy incident. Choice C is unsafe because privacy concerns should not be ignored simply because no complaint has been made. Choice B is inappropriate because altering the record is not a correct response, and D leaves the information exposed instead of securing it.

253. A practical nurse is preparing discharge teaching for a client with a new colostomy. The client says, "My brother will be helping me at home, but I do not want him to know anything about my past treatment for alcohol use disorder." The brother is present and asks to receive copies of all discharge instructions. Which action by the nurse best protects confidentiality while supporting safe care at home?

- A.** Provide the brother with the complete discharge packet because he will be assisting with care
- B.** Refuse to involve the brother in any teaching because selective sharing is not possible
- C.** Review with the client what information may be shared, provide only the care instructions the client agrees to disclose, and keep unrelated history private
- D.** Ask the brother to step out, then send him the full packet later after the client is discharged

Correct Answer: C

The nurse should use the minimum necessary approach and follow the client's stated preferences. Information needed for colostomy care can be shared if the client agrees, while unrelated sensitive history should remain private. Choice A wrongly assumes that a helper should receive the entire record. Choice B is too absolute because the client can authorize limited sharing. Choice D still violates confidentiality by sending the full packet against the client's wishes.

254. A practical nurse is updating a client's plan of care in the hallway outside the room when the client says, "You can tell my neighbor everything because she helps me with appointments." The neighbor is standing beside the bed. Which action by the nurse is most appropriate?

- A.** Discuss the client's full health information because the client gave verbal permission in front of the neighbor
- B.** Ask the neighbor to step out, then confirm with the client what information may be shared before discussing details
- C.** Refuse to discuss any information with the neighbor because only immediate family can receive updates

D. Provide only the diagnosis and medication list because those are necessary for appointments

Correct Answer: B

The nurse should first verify the client's wishes privately and clarify the scope of permission before sharing information. This protects confidentiality and supports the client's right to control who receives health information. Choice A moves too quickly without confirming exactly what may be disclosed. Choice C is incorrect because clients may authorize sharing with people other than family. Choice D still discloses private information without clearly established consent.

255. A practical nurse is preparing a written shift worksheet to help remember care tasks. Which entry on the worksheet best supports safe care while protecting client privacy if the sheet is misplaced?

- A.** Client initials, room number, and brief task reminders only
- B.** Client full name, diagnosis, date of birth, and medication times
- C.** Client medical record number, full address, and provider name
- D.** Client full name, insurance number, and laboratory values

Correct Answer: A

Using the minimum necessary identifying information helps the nurse organize care while reducing privacy risk if the worksheet is lost. Initials, room number, and task reminders are less revealing than full names and detailed personal or clinical data. The other options include unnecessary protected information such as date of birth, address, insurance number, and detailed diagnoses or results.

256. A practical nurse is caring for an adolescent client in an outpatient setting. The client's parent asks to read the nurse's notes from today's visit. The client quietly says, "I don't want my parent to hear everything we talked about." Which response by the nurse is best?

- A.** Give the parent the notes because parents always have unlimited access to a minor's information

- B.** Refuse to speak with either the client or the parent until the provider arrives
- C.** Tell the client that parents must be informed of every discussion in outpatient care
- D.** Explain that privacy rights can vary by situation and facility policy, and consult the charge nurse or policy before sharing the information

Correct Answer: D

When privacy rights may depend on the client's age, type of care, consent status, or applicable policy, the nurse should avoid making absolute promises or disclosures and should follow facility policy and chain of command. Choice D is safest because it recognizes that access is not always simple or universal. Choice A is too absolute. Choice C also overstates the rule. Choice B delays communication without addressing the immediate confidentiality concern appropriately.

257. A practical nurse is documenting in the electronic health record when a friend who works in radiology says, "I heard your client was admitted after an overdose. I just want to check the chart because I know the family." Which reply by the nurse is most appropriate?

- A.** That is acceptable if the friend promises not to tell anyone else
- B.** Access is allowed because the friend works for the same facility
- C.** The chart should be accessed only by staff directly involved in the client's care or duties that require the information
- D.** The friend may read only the laboratory results but not the progress notes

Correct Answer: C

Health information should be accessed only for legitimate care-related or job-required purposes. Personal curiosity, even by an employee, does not justify viewing a record. Choice C correctly applies the need-to-know standard. Choices A and B ignore confidentiality rules, and choice D still allows unauthorized access.

258. At the end of the shift, a practical nurse needs to give report to the oncoming nurse about a client with a newly positive test result for a communicable disease. The unit is busy, and visitors are walking through the hallway. What is the best action?

- A.** Give the full report at the desk but lower the voice
- B.** Wait and provide report in a more private area where only involved staff can hear
- C.** Write the sensitive result on a sticky note for the next nurse instead of speaking aloud
- D.** Omit the test result from report to avoid any risk to privacy

Correct Answer: B

The nurse must communicate important clinical information for continuity of care, but it should be done in a setting that protects privacy. Choice B balances confidentiality with safe handoff. Choice A still risks disclosure in a public area. Choice C creates an insecure written record. Choice D is unsafe because withholding significant information can compromise care.

259. A practical nurse is transporting a client to physical therapy when the client's adult daughter walks alongside the wheelchair and asks, "How did my mother's biopsy turn out? She forgets things, so you can just tell me." The client is alert, oriented, and does not respond. What is the nurse's best action?

- A.** Briefly share the result because the daughter is involved in the client's care
- B.** Ask the client whether she wants the information discussed with her daughter present before sharing anything
- C.** Tell the daughter the result now, then verify with the client afterward
- D.** Refuse to discuss any information with family members under any circumstance

Correct Answer: B

The nurse should first confirm the client's permission before discussing protected health information with the daughter, because the client is alert and able to decide who may receive information. Choice A and C assume family involvement automatically gives access, which is a common confidentiality error. Choice D is too absolute; information may be shared with family if the client authorizes it.

Advance directives

260. A client with advanced heart failure tells the nurse, "If my heart stops, I do not want anyone to try to restart it." What is the nurse's best initial action?

- A.** Ask the provider to place a do-not-resuscitate order if the client confirms this wish
- B.** Tell the family so they can make the decision together
- C.** Document the statement and take no further action unless the client arrests
- D.** Explain that only clients on hospice can refuse resuscitation

Correct Answer: A

A client's wish about resuscitation should be clarified and communicated promptly so the provider can write the appropriate order. The nurse advocates for the client's stated preference. Family input does not replace a capable client's decision, and simply documenting the comment is not enough to guide emergency care.

261. A nurse is admitting a competent adult client for surgery. The client says, "I completed a living will last year." Which statement by the nurse is most appropriate?

- A.** Bring me the document if available so it can be placed in your record and communicated to the care team
- B.** That document is only used after surgery if complications occur
- C.** A living will automatically names who can make decisions for you
- D.** You will not need that document because you are signing the surgical consent

Correct Answer: A

Advance directive documents should be requested, reviewed, and included in the medical record when available so the team can honor the client's wishes. A living will is not limited to postoperative complications, does not automatically appoint a decision-maker, and is not replaced by procedure consent.

262. A client arrives from a long-term care facility with a copy of a durable power of attorney for health care naming the client's daughter as surrogate. The client is awake, oriented, and clearly states treatment preferences. Who should make the current health care decisions?

- A.** The daughter, because she is listed on the document
- B.** The provider, because treatment decisions are medical
- C.** The client, because the client currently has decision-making capacity
- D.** The nurse, because the nurse is at the bedside

Correct Answer: C

A surrogate decision-maker acts when the client cannot make or communicate decisions. If the client currently has capacity, the client's own choices guide care. A common trap is assuming the named surrogate always takes over once the document exists.

263. A postoperative client becomes pulseless. The nurse sees a valid do-not-resuscitate order in the chart. Several family members at the bedside begin shouting, "Do everything!" What should the nurse do?

- A.** Begin chest compressions until the provider arrives
- B.** Follow the do-not-resuscitate order while calling for appropriate support and notifying the provider
- C.** Ask the family to vote on what they want done
- D.** Delay action until the charge nurse can review the chart

Correct Answer: B

When a valid do-not-resuscitate order is present, the nurse should honor it and avoid initiating resuscitation. The nurse should also notify the provider and obtain support for the family. Family demands do not override an existing order based on the client's wishes. Delaying action creates risk and confusion.

264. During admission, a client says, "I don't have an advance directive, and I don't want one." Which response by the nurse is best?

- A.** You must complete one before discharge
- B.** I will note your decision and let you know you can ask for information later if you change your mind
- C.** Your next of kin will automatically decide everything, so nothing else matters
- D.** Let's wait until you are more ill to talk about it

Correct Answer: B

Clients have the right to accept or decline information and to complete or not complete an advance directive. The nurse should document the discussion and keep the option open for future questions. The nurse should not coerce the client or postpone education as if it only matters when the client becomes critically ill.

265. An older adult is admitted from home with pneumonia. The client reports having an advance directive, but no copy is available. Which nursing action is most appropriate?

- A.** Assume the client is full code until the document is found and stop asking questions
- B.** Ask the client about current wishes, document the discussion, and try to obtain the directive according to facility process
- C.** Ask the family to create a new directive from memory
- D.** Refuse admission until the original document is brought in

Correct Answer: B

If the document is unavailable, the nurse should still assess the client's current wishes, document them, and work through the facility process to obtain the directive. Refusing admission is inappropriate, and family members should not recreate legal documents from memory. Assuming status without further clarification is not the best advocacy step when the client can speak.

266. A client with metastatic cancer has a signed advance directive stating refusal of artificial feeding in a specified end-stage condition. The client is now confused and unable to make decisions. The client's son says, "Start tube feeding anyway. Mom didn't mean that." What is the nurse's best response?

- A.** We should follow the client's documented wishes and notify the provider of your concerns
- B.** The son's request takes priority because he is the closest relative
- C.** Tube feeding should be started until the ethics committee meets next week
- D.** Advance directives are suggestions, so the team can ignore them if the family disagrees

Correct Answer: A

When the client lacks decision-making capacity and a valid directive addresses the current situation, the team should honor the client's documented wishes. The son's disagreement does not automatically override the client's prior decisions. A common trap is giving family preference priority over the capable client's previously expressed choices.

267. A hospitalized client asks the nurse, "Can you witness my advance directive?" What is the nurse's best first action?

- A.** Witness it immediately because the nurse is already assigned to the client
- B.** Review facility policy and verify the client is voluntarily signing while competent before proceeding
- C.** Tell the client only an attorney can be present during signing
- D.** Ask the family to sign on the client's behalf to save time

Correct Answer: B

Requirements for witnessing vary by jurisdiction and facility, so the nurse should first follow facility policy and verify that the client is competent and acting voluntarily. The nurse should not assume any bedside nurse may automatically witness, and family signing for a competent client is inappropriate.

268. A client says, "My brother is my health care proxy, but I do not want him involved right now. I can make my own decisions." Which nursing action is best?

- A.** Document the client's statement and continue to take decisions directly from the client
- B.** Call the brother for all decisions because the proxy document is already in effect
- C.** Ask the provider to determine whether the brother's opinion is more reasonable
- D.** Limit the client to minor decisions until the proxy agrees

Correct Answer: A

A competent client remains the decision-maker. The nurse should document the client's statement and continue to accept decisions from the client. A proxy does not replace a capable adult simply because the document exists.

269. The nurse is caring for four clients. Which situation requires the highest priority follow-up related to advance directives?

- A.** A client scheduled for discharge asks for blank advance directive forms
- B.** A client says, "I should probably talk to my daughter about my wishes someday"
- C.** A client going for an invasive procedure says, "I do not want to be kept alive on machines," but no directive or related order is in the chart
- D.** A client wants to know whether a living will should be kept at home

Correct Answer: C

The client facing an invasive procedure has expressed a potentially urgent treatment preference, yet there is no documented directive or related order. This requires prompt clarification and communication before a situation arises in which the preference matters immediately. The other situations are important but less urgent.

270. A client with severe dementia is admitted from a group home. A document in the chart names a longtime friend as durable power of attorney for health care. The client's estranged adult child arrives and states, "I'm family, so I make all decisions now." What should the nurse do first?

- A.** Accept the adult child's decisions because blood relatives come first
- B.** Ask both parties to decide together to avoid conflict
- C.** Refer to the existing legal document and notify the provider and charge nurse about the disagreement
- D.** Ignore the document until the hospital lawyer gives a formal opinion

Correct Answer: C

When conflict arises, the nurse should rely on the client's existing legal documentation and escalate the disagreement appropriately. The named surrogate in a valid document is not displaced simply because a relative objects. Waiting passively or treating all opinions as equal risks violating the client's chosen arrangement.

271. A nurse is documenting admission questions about advance directives. Which chart entry is best?

- A.** Client asked about future care; family seemed supportive
- B.** Advance directive status discussed; client states no current document and declines further information at this time
- C.** Client probably does not understand code status
- D.** No issues related to end-of-life preferences

Correct Answer: B

Good documentation is specific, objective, and relevant. Choice B clearly records that the topic was addressed, the client reports no document, and the client declined more information. The other options are vague, judgmental, or incomplete.

272. A client says, "I want my spouse to make decisions for me if I'm ever unable, but I haven't completed any paperwork." Which nursing response is best?

- A.** Then your spouse already has full authority in every situation
- B.** I can provide information on how to document that choice so your wishes are clearer to the health care team
- C.** You should wait until you are critically ill before naming a decision-maker
- D.** That choice only matters if you are refusing all treatment

Correct Answer: B

The nurse should provide information that helps the client formally document wishes and identify a preferred surrogate. Saying the spouse automatically has full authority in every situation is too broad. Advance care planning is not limited to critical illness or refusal of treatment.

273. A client admitted for treatment of sepsis is alert and able to make decisions. The chart contains an older living will that refuses mechanical ventilation in a terminal condition. The client now states, "If my breathing gets worse from this infection, I do want a breathing machine if it could help me recover." What is the nurse's best action?

- A.** Follow the living will because written directives always take priority over current statements
- B.** Ask the family to decide whether the client really wants ventilation
- C.** Notify the provider of the client's current wishes and ensure the change is documented according to facility policy
- D.** Explain that the client cannot change the directive after hospital admission

Correct Answer: C

A competent client's current expressed wishes guide care. The nurse should communicate the client's statement promptly and ensure appropriate documentation or updating of the directive per policy. Choice A is a common trap: an older document does not override a capable client's current decisions. Choice B is incorrect because family members do not replace the decision-making authority of an alert, competent adult. Choice D is false; clients can change their decisions while they retain decision-making capacity.

274. During discharge teaching, a client asks, "What is the main purpose of a durable power of attorney for health care?" Which response by the nurse is best?

- A.** It allows a person you choose to make health care decisions for you if you become unable to decide for yourself
- B.** It lists every treatment you will accept or refuse in each possible future situation
- C.** It prevents providers from discussing your condition with your family under any circumstances
- D.** It becomes active whenever you are admitted to a hospital, even if you can still make decisions

Correct Answer: A

A durable power of attorney for health care designates a surrogate decision-maker to act if the client loses decision-making capacity. Choice B describes a more treatment-specific directive, such as a living will, not the main purpose of naming a proxy. Choice C confuses decision-making authority with privacy rules. Choice D is incorrect because the client's own decisions remain primary as long as the client has capacity.

275. A nurse is admitting an alert adult client who says, "I signed papers years ago, but I've changed my mind about some of my treatment wishes." Which nursing response is most appropriate?

- A.** "Once an advance directive is signed, it cannot be changed."
- B.** "Your family must approve any changes before they can be recognized."
- C.** "You can discuss your current wishes with the provider so the record can be updated."

D. "The original document automatically overrides anything you say now."

Correct Answer: C

A competent client may express current treatment wishes, and those wishes should be communicated and documented so the health care team can follow them appropriately. Choice A is incorrect because advance directives are not permanently fixed. Choice B is a common trap: family members do not have to approve a competent client's choices. Choice D is wrong because a capable client's current decisions guide care.

276. An alert client tells the nurse, "I want to review what my advance directive says before I sign the admission papers." Which action should the nurse take first?

- A. Tell the client that the directive can be reviewed after admission is complete
- B. Provide the client with the available copy or assist in obtaining it for review
- C. Ask the family to explain the document to the client
- D. Have the client sign consent forms first to avoid treatment delays

Correct Answer: B

The nurse should support the client's right to review the advance directive by providing or helping obtain the document. This respects autonomy and informed decision-making. Delaying review until later or pushing other signatures first undermines that right. Asking family to explain the document is less appropriate than directly supporting the client and using the health care team as needed.

277. A nurse is caring for a client who says, "I don't understand the difference between a living will and choosing a health care proxy." Which explanation by the nurse is best?

- A. A living will names a person to decide, while a proxy lists specific treatments
- B. A living will states treatment wishes, while a health care proxy designates someone to decide if needed
- C. Both documents are used only for clients in hospice care
- D. Both documents apply only after a court reviews them

Correct Answer: B

A living will typically records treatment preferences, and a health care proxy or durable power of attorney for health care names a decision-maker if the client loses capacity. Choice A reverses the two. Choices C and D add restrictions that are not the defining purpose of these documents.

278. A client who is scheduled for a procedure says quietly, "My son keeps answering questions for me, but I want to make my own decisions." The son then says, "I already told everyone what she wants." Which nursing action is most appropriate?

- A. Continue the discussion with the son because family knows the client best
- B. Ask the client privately about her wishes and support her decision-making role
- C. Postpone all decisions until the ethics committee meets
- D. Follow the son's wishes unless the client has a notarized directive

Correct Answer: B

When the client is capable, the nurse should protect the client's autonomy by speaking with the client directly and privately if needed. Family input does not override a competent client's authority. An ethics consult may be useful in some conflicts, but it is not the first step here. A notarized directive is not what determines whether a capable client may speak for herself.

279. A nurse notes that a client's chart contains an advance directive, but the care team's handoff report does not mention it. Which action is best?

- A. Assume the provider already discussed it with the client
- B. Wait until discharge planning to clarify whether it matters
- C. Communicate the presence of the advance directive to the team and ensure it is readily available in the record
- D. Remove the document from the chart to prevent confusion unless the family requests it

Correct Answer: C

Advance directives are only useful in practice if the team knows they exist and can access them. The nurse should promote safe, coordinated care by communicating the document's presence and ensuring it is available in the record. Assuming others know, delaying follow-up, or removing the document risks care that does not match the client's wishes.

280. A client with progressive neurologic disease asks, "Why should I talk about advance directives now when I'm still doing okay?" Which response by the nurse is best?

- A. "You should wait until you are seriously ill so your wishes are more realistic."
- B. "These discussions are most helpful while you can clearly state your values and preferences."
- C. "Advance directives are mainly needed so hospitals can limit expensive treatments."
- D. "You only need one if your family members disagree with each other."

Correct Answer: B

Advance care planning is most effective while the client can thoughtfully communicate goals, values, and preferences. Choice A delays planning unnecessarily. Choice C misrepresents the purpose of advance directives. Choice D is too narrow because directives are helpful even in supportive families.

281. A client is confused after a stroke and cannot communicate choices. The chart contains no advance directive. Two adult children disagree about treatment, and both ask the nurse to "pick the right side." What is the nurse's best response?

- A. Choose the option that seems least invasive and document the decision
- B. Explain that the nurse cannot choose among family members' preferences and should notify the provider according to facility process
- C. Support the family member who has visited the client most often
- D. Ask the unit secretary to decide which child should speak for the client

Correct Answer: B

The nurse should not personally choose among competing family preferences. The appropriate action is to recognize the conflict, avoid acting beyond role, and involve the provider and established process for identifying the proper decision-maker. Choices A, C, and D inappropriately substitute personal judgment or irrelevant factors for proper legal-ethical process.

282. Which charting entry best documents a client's statement about advance directives during admission?

- A. Client probably understands end-of-life planning and seems comfortable with current family decisions.
- B. Advance directive status addressed.
- C. Client states, "I have a health care proxy at home naming my partner." No copy available at admission; provider notified per facility process.
- D. Family says the client once completed some kind of legal paperwork.

Correct Answer: C

The best documentation is specific, objective, and includes the client's own statement plus the follow-up action taken. Choice A is vague and interpretive. Choice B is too general to be useful. Choice D relies on secondhand, uncertain information instead of clearly documenting what is known.

283. A client says, "I have a living will, so that means I never need to talk with my family or nurses about my wishes." Which response by the nurse is best?

- A. "That is correct; the written form replaces future discussion."
- B. "Discussion is still helpful so your wishes are understood and available when needed."
- C. "Only your provider needs to know what is in the document."
- D. "A living will is used only if you are permanently unconscious."

Correct Answer: B

Even when a written directive exists, communication remains important so the care team and potential surrogates understand the client's preferences. Choice A is a trap because a document alone may not answer every situation. Choice C limits communication too much. Choice D is inaccurate because living wills may address broader treatment situations.

284. A competent client with an advance directive asks the nurse, "Can I refuse one treatment and still accept another if my condition changes?" Which response is best?

- A.** "No. Once any refusal is written, you must refuse all related treatments."
- B.** "Yes. Your treatment decisions can be specific, and your current wishes should guide care."
- C.** "Only the court can decide whether your preferences may be mixed."
- D.** "No. Accepting one treatment cancels your entire advance directive permanently."

Correct Answer: B

Advance care planning is not an all-or-nothing choice. A competent client may express specific preferences and clarify current wishes based on the situation. Choices A and D wrongly present treatment planning as rigid and automatic. Choice C adds an unnecessary legal barrier.

285. A client with chronic lung disease becomes increasingly short of breath. The chart includes an old advance directive, but parts of the scanned document are unreadable. The client is now lethargic and unable to participate in decision-making. Which action by the nurse is the priority?

- A.** Rely on memory from a prior admission about what the document probably said
- B.** Ask the family to vote on what seems most reasonable
- C.** Immediately seek a legible copy or verified clarification through available records and notify the provider of the uncertainty
- D.** Ignore the document because any unreadable section makes the entire directive unusable

Correct Answer: C

When an advance directive is potentially relevant but unclear, the priority is to urgently clarify and communicate the uncertainty so decisions are based on the best available verified information. Memory and family voting are unreliable substitutes. Ignoring the document altogether is also inappropriate because some parts may still be usable or a clearer copy may be available quickly.

286. A long-term care resident with fluctuating cognition is most alert in the morning. The nurse plans to discuss whether the resident wants information about advance directives. When is the best time to begin this conversation?

- A.** During the evening when family can answer most questions
- B.** At the time of greatest alertness so the resident can participate as fully as possible
- C.** Only after the resident loses all decision-making ability
- D.** Immediately after sedating medication is given so the resident stays calm

Correct Answer: B

Because decision-making ability may fluctuate, the nurse should maximize the client's participation by choosing the time of greatest alertness. Waiting until capacity is lost defeats the purpose. Sedation can impair participation, and family presence does not replace the resident's own involvement when possible.

287. A client is transferred from another facility with a POLST-type medical order and a separate advance directive. The receiving nurse notices the two documents appear inconsistent regarding life-sustaining treatment. What should the nurse do first?

- A.** Follow whichever document was signed first because it is the original statement
- B.** Set both documents aside until discharge planning can sort them out
- C.** Recognize the discrepancy, promptly notify the provider, and seek clarification using the client's current decision-making ability if possible
- D.** Ask a family member which document seems more accurate and treat that answer as final

Correct Answer: C

Potential conflict between directive documents requires prompt clarification, especially when life-sustaining treatment decisions may be affected. If the client can currently state preferences, those wishes are highly important. Choice A incorrectly relies on age of the document alone. Choice B delays a potentially urgent issue. Choice D gives final authority to a family member without proper basis.

288. A nurse is orienting a newly hired assistive personnel staff member, who asks, "If a family member wants to know whether a client has a do-not-resuscitate order or other advance directive, can I answer?" Which instruction by the nurse is best?

- A. "Yes, if the family member seems upset and needs reassurance."
- B. "Yes, because advance directive status is routine information for visitors."
- C. "No. Refer questions about the client's protected health information and plan of care to the nurse."
- D. "No. Advance directives can never be discussed with any family member under any circumstance."

Correct Answer: C

Advance directive information is part of the client's protected health information and should be handled according to role and authorization. Assistive personnel should refer such questions to the nurse. Choice D is too absolute because authorized discussion may occur appropriately with the care team and permitted decision-makers. Choices A and B improperly encourage unauthorized disclosure.

289. An older adult admitted for heart failure says, "I never completed an advance directive because I thought only people who are dying need one." Which response by the nurse is best?

- A. Advance directives are usually completed only after a provider determines that a condition is terminal.
- B. You can still make your own decisions now, but an advance directive lets you state your wishes in case you cannot speak for yourself later.

C. If you do not have an advance directive, the hospital will automatically make treatment decisions for you.

D. Once you sign an advance directive, your family will no longer be involved in your care decisions.

Correct Answer: B

B is correct because advance directives are intended to communicate a person's preferences for future care if the person later becomes unable to make or express decisions. They are not only for clients who are terminally ill. A is incorrect because completion is not limited to terminal conditions. C is incorrect because treatment decisions are not simply turned over to the hospital by default. D is incorrect because families may still be involved, especially if discussion and support are needed; an advance directive does not automatically exclude them.

290. During admission, an adult client says, "I completed an advance directive in another state, so it probably does not matter here." Which response by the nurse is most appropriate?

- A. "Advance directives are only valid in the state where they were signed."
- B. "We should ignore it until the provider decides whether it can be used."
- C. "It may still be relevant, so I will notify the appropriate team member and make sure it is reviewed according to facility policy and state law."
- D. "You will need to complete a completely new document before we can place anything in your chart."

Correct Answer: C

An out-of-state advance directive should not be dismissed automatically. The safest nursing action is to ensure the document is identified, communicated, and reviewed according to applicable policy and law. Choice A is too absolute, B delays appropriate follow-up, and D assumes a new document is always required.

291. A client asks the nurse, "If I don't have an advance directive, can the hospital make me sign one before I get treatment?" Which response is best?

- A. "Yes, every admitted client must sign one before any care can be given."

- B. "No. You can receive care whether or not you complete an advance directive."
- C. "Only clients older than 65 are asked to sign one."
- D. "Only surgical clients are required to complete one."

Correct Answer: B

Clients should be informed about advance directives, but they are not required to complete one in order to receive care. Choice B protects client rights. The other options incorrectly present completion as mandatory for all clients or for selected groups.

292. A client tells the nurse, "I already filled out an advance directive, but I don't want a copy of it in my hospital record." Which action is most appropriate?

- A. Tell the client that all advance directives must be placed in the chart without exception.
- B. Ask the family to provide the document anyway because family wishes take priority.
- C. Explain the value of having the document available to guide care, then follow facility process based on the client's decision and document the discussion.
- D. Wait until the client becomes unable to decide, then request the document from the provider.

Correct Answer: C

The nurse should provide clear information about why an available advance directive can help guide care, respect the client's current decision-making role, and document the conversation. A is overly coercive, B improperly elevates family wishes over the capable client, and D delays needed clarification.

293. A nurse is caring for a client who has decision-making capacity. The client's spouse says, "Don't talk to him about advance directives. He gets upset." Which action should the nurse take?

- A. Avoid the topic because the spouse requested it
- B. Discuss advance directives privately with the client and determine the client's wishes
- C. Ask the provider to obtain the spouse's permission first

- D. Document that the spouse declined advance directive information

Correct Answer: B

A capable client has the right to receive information and make personal decisions, including whether to discuss advance directives. The nurse should speak with the client directly and privately if appropriate. A and D improperly substitute the spouse's preference for the client's rights, and C is unnecessary because the spouse does not control a capable adult's decisions.

294. A nurse enters a client's room and hears two adult children arguing about whether their parent would want life-sustaining treatment. The client is alert and says, "Please stop talking about me like I'm not here." Which nursing action is best?

- A. Ask the children to decide together, since they know the client best
- B. Tell the family that treatment decisions belong to the provider until discharge
- C. Refocus the discussion on the client's own wishes and offer to discuss advance directives directly with the client
- D. Request an ethics consult immediately before speaking further with anyone

Correct Answer: C

Because the client is alert and expressing preferences, the nurse should center the conversation on the client's own decisions and wishes. A bypasses the competent client, B is incorrect because providers do not take over personal decision-making from a capable client, and D may be useful later but is not the best immediate step.

295. A client says, "My daughter is the person I named to make decisions if I can't, so she should start making them now because she's better at it." The client is alert and oriented. Which response by the nurse is best?

- A. "That person usually acts only if you are unable to make or communicate your own decisions."
- B. "Once you name someone, your own choices no longer matter."
- C. "Your daughter can begin signing all consents as soon as she arrives."

D. "That means your provider no longer needs to explain treatments to you."

Correct Answer: A

A surrogate or proxy is generally intended to step in if the client cannot make or communicate decisions. A respects the current decision-making capacity of the client. B, C, and D incorrectly remove the capable client from the decision process.

296. A nurse is preparing discharge teaching for a client newly diagnosed with a chronic illness. The client says, "I'm overwhelmed, but I do want to think about advance directives after I go home." Which nursing action is most appropriate?

- A.** Document the client's interest and provide information on how to obtain further guidance after discharge
- B.** Insist that the client complete the forms before leaving the facility
- C.** Postpone all discussion because advance directives should not be mentioned at discharge
- D.** Tell the client that only an attorney can provide any information about advance directives

Correct Answer: A

The nurse should support the client's interest, document it, and provide appropriate resources or referral information. B is coercive, C is unnecessarily restrictive, and D overstates the need for an attorney when clients can often receive general information and assistance through routine health care channels.

297. A client's chart states that an advance directive is on file. During morning care, the client says, "I don't want my brother involved in my decisions anymore." What should the nurse do first?

- A.** Tell the client that an existing document cannot be changed during hospitalization
- B.** Assess the client's current understanding and notify the appropriate team member so the client's concerns can be addressed promptly
- C.** Ask the brother to explain why the client made that statement
- D.** Remove the advance directive from the chart immediately

Correct Answer: B

When a capable client expresses concern about an existing directive or decision maker, the nurse should clarify the client's current wishes and communicate the need for prompt review through the proper process. A is too absolute, C inappropriately shifts focus to the family member, and D skips necessary verification and procedure.

298. A client with advanced illness says, "I have a do-not-resuscitate order, so that means the staff should stop all treatments now." Which response by the nurse is best?

- A.** "Yes, a do-not-resuscitate order means no more medical treatment of any kind."
- B.** "A do-not-resuscitate order refers specifically to withholding resuscitation if your heart or breathing stops, not automatically stopping all other care."
- C.** "Yes, but only if your family agrees with the order."
- D.** "A do-not-resuscitate order and an advance directive mean exactly the same thing."

Correct Answer: B

A DNR order specifically addresses resuscitation if cardiac or respiratory arrest occurs; it does not automatically cancel all treatment. A is overly broad, C incorrectly makes family agreement the deciding factor, and D wrongly equates two related but different documents or orders.

299. A nurse is caring for a client who speaks limited English and asks for help understanding information about advance directives. The client's adult grandson says, "I can translate everything for her." Which action is best?

- A.** Use the grandson so the conversation can happen more quickly
- B.** Delay the discussion until discharge instructions are given in writing
- C.** Arrange for a qualified interpreter and then discuss the information with the client
- D.** Ask the provider to skip the discussion because it is too complicated through an interpreter

Correct Answer: C

Advance directive discussions involve important legal and personal choices, so accurate communication with the client is essential. A qualified interpreter best supports informed, autonomous decision-making. Using a family member as the default interpreter can create errors or pressure. B and D unnecessarily delay or avoid the client's right to information.

300. A nurse finds an unsigned advance directive form at the bedside of a client who is now unresponsive after rapid deterioration. The family says, "He was going to finish it later, so follow what it says." Which action should the nurse take?

- A. Treat the unsigned form as a completed directive because the family confirmed it
- B. Place the form in the chart as an active advance directive immediately
- C. Report the finding and follow established decision-making and documentation processes rather than assuming the unsigned form is legally completed
- D. Discard the form because it has no value at all

Correct Answer: C

An unsigned form should not automatically be treated as a completed directive. The nurse should communicate the finding and proceed through facility policy and established decision-making channels. A and B assume legal completion without support. D is also incorrect because the form may still provide contextual information even though it should not be treated as a fully executed directive without proper review.

301. During an admission interview, which question by the nurse is most appropriate for screening whether an advance directive already exists?

- A. "Do you have any written instructions or a person you have chosen to speak for you if you become unable to make decisions?"
- B. "You don't want resuscitation, correct?"
- C. "Which family member should we allow to make decisions for you now?"
- D. "Why have you not completed the required advance directive form yet?"

Correct Answer: A

Choice A is neutral, respectful, and broad enough to identify either written treatment wishes or a designated decision maker. B is leading, C assumes the client currently needs a surrogate, and D incorrectly suggests the form is required.

302. A client with metastatic cancer has decision-making capacity and tells the nurse, "My family wants every treatment possible, but I want comfort-focused care if things worsen." The family later asks the nurse to leave that out of the chart to avoid conflict. Which action is most appropriate?

- A. Honor the family's request to prevent emotional distress
- B. Document the client's stated wishes accurately and communicate them to the appropriate team members
- C. Wait until the client loses capacity so the family can make the final decision
- D. Record only the family's preferences because they will likely be making decisions later

Correct Answer: B

The capable client's wishes should be documented accurately and communicated so care planning reflects the client's preferences. A, C, and D wrongly prioritize the family's wishes over the client's current autonomy and risk unsafe, unethical care.

303. A nurse is teaching a group of clients about advance care planning. Which statement by the nurse is most accurate?

- A. "Advance directives are useful only in emergency departments."
- B. "Advance care planning is something people can consider before a crisis happens."
- C. "Once completed, advance directives should never be reviewed again."
- D. "Only clients with terminal illness benefit from discussing advance directives."

Correct Answer: B

Advance care planning is most useful when considered before a crisis, while the person can reflect and communicate clearly. A and D make the topic too narrow, and C is incorrect because wishes and documents may need review as circumstances change.

304. A nurse is admitting an alert adult client for elective surgery. The client says, "I have an advance directive, but I changed my mind about some of it last month." Which nursing action is most appropriate?

- A.** Tell the client that the document on file must be followed until after surgery
- B.** Ask the client to clarify the current wishes and notify the provider so the record can be updated
- C.** Instruct the family member present to explain which parts of the directive were changed
- D.** Remove the existing advance directive from the chart because it may no longer be valid

Correct Answer: B

A client with decision-making capacity can state current wishes about care. The nurse should clarify what the client wants now and communicate that information so the health record can be updated appropriately. Choice A is incorrect because current capable wishes take priority over simply relying on an older document on file. Choice C is a trap because the family member is not the primary source when the client can speak for himself or herself. Choice D is incorrect because the nurse should not independently remove legal documents from the chart; the priority is to assess, document, and communicate the client's current wishes.

305. An alert adult client says, "I'm only 32, so I don't need to think about an advance directive yet." Which response by the nurse is best?

- A.** "That is fine, because advance directives are mainly for older adults."
- B.** "You can decide later if you become seriously ill and are unable to speak."
- C.** "Advance directives are for any competent adult and can help communicate your wishes if you cannot speak for yourself."
- D.** "You should complete one now because all hospitalized clients are expected to have one."

Correct Answer: C

Advance directives are intended for competent adults of any age, not only older adults. They help guide care if the person later cannot communicate decisions. Choice A is incorrect because age alone does not determine whether advance care planning is appropriate. Choice B is a trap because if the client becomes unable to speak, it may be too late to express preferences personally. Choice D is incorrect because clients are not required to complete an advance directive.

306. A competent client asks, "Can I change my advance directive after I sign it?" Which response by the nurse is most appropriate?

- A.** "Yes, as long as you still have decision-making capacity and follow the required process."
- B.** "No, once it is signed it cannot be changed."
- C.** "Only your health care provider can change it for you."
- D.** "Only your family can approve changes after hospitalization."

Correct Answer: A

A client with decision-making capacity may revise an advance directive, though the exact process depends on applicable requirements and facility procedure. Choice A is best because it is accurate without overstating details. Choices B, C, and D incorrectly suggest the document is fixed or controlled by others rather than by the capable client.

307. A nurse is admitting a client who says, "I don't want an advance directive." Which action by the nurse is most appropriate?

- A.** Ask the family to persuade the client to complete one before treatment begins.
- B.** Document the client's decision and continue care without pressure.
- C.** Explain that a refusal means the provider will make all future decisions.
- D.** Delay the admission until the client signs the form.

Correct Answer: B

A competent client has the right to decline an advance directive. The nurse should document the decision and continue appropriate care. Choice A is inappropriate because it pressures the client. Choice C is misleading because refusal of an advance directive does not mean the client gives up all decision-making authority. Choice D is wrong because treatment and admission should not be delayed solely because the client declines to sign.

308. A client asks the nurse, "What is the main purpose of naming a health care proxy?" Which response is best?

- A. "To let someone make health care decisions for you if you cannot make them yourself."
- B. "To let someone manage your bank account during hospitalization."
- C. "To allow a family member to override your decisions whenever they disagree."
- D. "To give the hospital permission to release all records to anyone in your family."

Correct Answer: A

A health care proxy or surrogate is designated to make health care decisions if the client loses decision-making ability. Choice B refers to financial matters, not health care decision-making. Choice C is wrong because a capable client's own decisions take priority. Choice D confuses decision-making authority with broad privacy release.

309. A client with decision-making capacity tells the nurse, "I want to complete an advance directive, but my son says he should fill it out for me because he knows me best." Which response by the nurse is most appropriate?

- A. "Your son may complete it for you because he is your closest relative."
- B. "The provider should complete it and you can sign later without reading it."
- C. "Because this expresses your wishes, you should complete it yourself if you choose to do so."
- D. "It is better to wait until you are more seriously ill before completing it."

Correct Answer: C

An advance directive reflects the client's own preferences. If the client has capacity, the client should make those decisions. Choice A incorrectly gives decision-making control to a family member. Choice B is unsafe and violates informed, voluntary decision-making. Choice D delays planning without reason and may reduce the chance to clearly document the client's wishes.

310. A nurse is reviewing a newly admitted client's chart and sees no documentation about advance directives. Which action should the nurse take first?

- A. Document that the client declined to discuss advance directives.
- B. Ask the client whether an advance directive already exists.
- C. Provide a blank form and instruct the client to sign it.
- D. Call the client's next of kin to ask about end-of-life wishes.

Correct Answer: B

The first step is to assess whether the client already has an advance directive. Choice A assumes a refusal without asking. Choice C skips assessment and pressures the client toward completion. Choice D is premature because the client should be asked directly when able to participate.

311. A client tells the nurse, "I signed papers months ago, but I'm not sure whether they cover the kind of care I would want now." Which nursing action is most appropriate?

- A. Tell the client that any signed document remains final no matter what.
- B. Encourage the client to review the document and discuss possible updates while the client has capacity.
- C. Advise the client to ignore the document until a family member requests it.
- D. Tell the client that treatment decisions cannot be discussed until the provider is present.

Correct Answer: B

When a client is unsure whether an existing advance directive still reflects current wishes, the appropriate action is to support review and possible revision while the client has decision-making capacity. Choice A is incorrect because the client may be able to revise the document. Choice C dismisses the client's concern. Choice D unnecessarily blocks discussion that the nurse can appropriately facilitate.

312. An older adult client says, "I don't understand these advance directive forms very well." Which nursing action is best?

- A.** Hand the client the paperwork and ask for a signature before discharge.
- B.** Ask the client's roommate to explain the forms in simple terms.
- C.** Provide clear explanations, answer questions, and offer the appropriate resources for further assistance.
- D.** Tell the client to wait until an emergency occurs to decide.

Correct Answer: C

The nurse should support informed decision-making by giving understandable information, answering questions, and connecting the client with appropriate resources as needed. Choice A turns a voluntary process into a task to complete. Choice B violates privacy and uses an inappropriate person for teaching. Choice D delays planning until a time when the client may no longer be able to participate.

313. A client who is alert and oriented says, "I want my sister involved in decisions if I become unable to decide, but I do not want my ex-spouse making decisions." Which nursing action is most appropriate?

- A.** Document the client's statement and assist the client in obtaining information about formally designating a decision-maker.
- B.** Tell the client that family members will decide among themselves if needed.
- C.** Advise the client that only a current spouse can be named for health care decisions.
- D.** Wait until the client loses capacity before documenting the preference.

Correct Answer: A

The nurse should document the capable client's stated preferences and help the client access the proper process for formal designation. Choice B is less appropriate because it leaves the issue unresolved and may invite conflict. Choice C is incorrect because a client is not limited to naming a current spouse. Choice D delays an important action that should happen while the client can still decide.

314. A hospitalized client says, "I've never talked with my family about what I would want if I could not speak for myself." Which nursing suggestion is most appropriate?

- A.** "It may help to discuss your values and wishes with them now, so they better understand your preferences."
- B.** "You should avoid talking about it because those conversations increase family conflict."
- C.** "Only written instructions matter, so there is no need to discuss it."
- D.** "Wait until you are critically ill so the conversation feels more urgent."

Correct Answer: A

Discussing wishes with family can improve understanding and reduce confusion if the client later cannot communicate. Choice B overgeneralizes and discourages planning. Choice C is a trap because conversation does not replace documentation, but it is still valuable. Choice D delays an important discussion until a more stressful and potentially less effective time.

315. A nurse is caring for a client who has decision-making capacity. The client says, "I want information about advance directives, but I don't want to complete anything today." Which response by the nurse is best?

- A.** "That is your choice. I can provide information now so you can review it when you are ready."
- B.** "You need to complete the forms today or your wishes cannot be honored."
- C.** "Then there is no reason to discuss advance directives at all."
- D.** "I will ask your family to decide whether you should complete one."

Correct Answer: A

Advance care planning should be voluntary and informed. Providing information now while respecting the client's choice not to complete documents today is appropriate. Choice B is coercive and inaccurate. Choice C misses the opportunity for education. Choice D improperly shifts the decision away from a capable client.

316. A client becomes unconscious after surgery. The chart contains a valid advance directive naming the client's niece as the health care proxy. The client's adult son arrives and says, "I'm the oldest child, so I make the decisions." What is the nurse's best action?

- A.** Ask the son to decide because children have priority over other relatives.
- B.** Follow the client's documented designation and notify the appropriate team members.
- C.** Delay all decisions until the entire family reaches agreement.
- D.** Use the surgeon as the decision-maker because family members disagree.

Correct Answer: B

When a valid advance directive names a health care proxy and the client lacks capacity, the designated person should be recognized according to the client's documented choice. Choice A incorrectly substitutes family hierarchy for the client's expressed wishes. Choice C may delay needed care. Choice D ignores the client's prior designation.

317. A client with capacity has a documented advance directive requesting no life-prolonging treatment in a specified terminal condition. The client is now alert, understands the situation, and tells the nurse, "I want full treatment for now." The family says, "Use the written directive instead." Which action should the nurse take?

- A.** Follow the written directive because documents always override current statements.
- B.** Follow the family's request because they are more objective.
- C.** Recognize the capable client's current wishes and communicate them to the provider and team.

D. Ignore both the client and family until an ethics consult is completed.

Correct Answer: C

A client who currently has decision-making capacity directs care in the present. The advance directive guides decisions when the client cannot decide or communicate. Choice A is a common trap because it treats the document as automatically superior even when the client is capable. Choice B wrongly gives the family control over a capable client's choices. Choice D causes unnecessary delay.

318. A nurse is caring for an unresponsive client whose chart includes an advance directive. The provider writes a treatment plan that appears inconsistent with the documented wishes. What should the nurse do first?

- A.** Carry out the orders exactly as written because provider orders always take priority.
- B.** Contact the provider and clarify the apparent inconsistency with the client's documented wishes.
- C.** Ask the client's visitors what they think the client would want.
- D.** Remove the advance directive from the chart until the team agrees.

Correct Answer: B

When the nurse identifies a possible conflict between the care plan and the client's documented advance directive, the first action is to advocate for the client by clarifying the discrepancy with the provider. Choice A fails to address a possible violation of client wishes. Choice C relies on informal opinions rather than documented information. Choice D is inappropriate and unsafe.

319. A long-term care client has a valid advance directive in the facility record. After transfer to the hospital, the receiving nurse cannot locate any documentation of it in the transfer packet. The client is now confused and unable to give reliable history. Which action by the nurse is most appropriate?

- A.** Assume no advance directive exists because it was not in the packet.

- B.** Request the directive from the transferring facility and notify the care team that confirmation is being obtained.
- C.** Ask a family member to create a new directive immediately.
- D.** Wait until the client regains capacity before addressing the issue.

Correct Answer: B

If a known advance directive is missing during transfer, the nurse should try to obtain the existing document promptly and keep the team informed. Choice A is unsafe because absence from the packet does not prove absence of the directive. Choice C is inappropriate because others cannot simply create a new directive for the client. Choice D delays action in a situation where the information may be urgently relevant.

320. During admission, a client asks, "If I write down my wishes in an advance directive, does that mean the staff will stop treating me?" Which response by the nurse is best?

- A.** "Yes, because an advance directive means you are refusing future medical care."
- B.** "No. An advance directive helps guide care based on your wishes if you cannot speak for yourself."
- C.** "Yes, but only if your family agrees with the document."
- D.** "No, because advance directives are used only for financial decisions."

Correct Answer: B

An advance directive communicates the client's preferences for future health care decisions if the client becomes unable to decide or communicate. It does not automatically mean treatment stops. Choice A is incorrect because advance directives are not the same as refusing all care. Choice C is a common trap because family opinions do not replace the purpose of the client's stated wishes. Choice D is incorrect because advance directives address health care, not financial matters.

321. A client with decision-making capacity tells the nurse, "I already have an advance directive, but I do not want my family to read it right now." Which nursing action is most appropriate?

- A.** Give the family a copy because they may need it later

- B.** Ask the provider to decide whether the family should see it
- C.** Maintain the client's privacy and share the document only according to facility policy and the client's authorization
- D.** Remove the document from the chart until the client becomes unable to decide

Correct Answer: C

Advance directives are part of the client's health information and should be handled with respect for confidentiality and the client's rights. The nurse should protect privacy and disclose the document only as permitted by the client's wishes and facility policy. Choice A violates confidentiality. Choice B delays a nursing responsibility related to privacy. Choice D is incorrect because the document should remain available in the record for care planning.

322. A client admitted for worsening heart failure says, "I completed an advance directive years ago in another state." Which action by the nurse is best?

- A.** Tell the client the document is invalid because it was not completed locally
- B.** Instruct the client to complete a new document immediately
- C.** Ask the client to provide a copy for the record and notify the appropriate team member so it can be reviewed according to facility policy
- D.** Wait to address the issue until the client becomes critically ill

Correct Answer: C

The best action is to obtain and document the client's existing directive and ensure it is reviewed appropriately. This supports coordinated care and respect for client wishes. Choice A is too absolute; the nurse should not assume invalidity. Choice B is inappropriate because the client should not be pressured to create a new document immediately. Choice D delays important planning that should occur before a crisis.

323. A client who is alert and has decision-making capacity says, "My advance directive says no feeding tube, but after talking with my provider, I want one if it becomes necessary." Which nursing response is best?

- A.** "Your written directive cannot be changed once it has been signed."

- B.** "Your family will need to approve any change before it can be followed."
- C.** "Because you can make decisions now, tell the provider your current wishes so they can be addressed and documented."
- D.** "The existing document must be followed until you are discharged."

Correct Answer: C

A client with decision-making capacity can express current treatment preferences, and those wishes should be communicated and documented. The nurse should support the client in making the current wishes known to the provider and team. Choice A is wrong because it treats the directive as permanent and unchangeable. Choice B is a trap because family approval is not required for a capable client's decision. Choice D ignores the client's present autonomy.

324. A nurse is caring for a confused client who cannot make decisions. Two adult children are arguing at the bedside about treatment. The chart contains a signed document naming the client's neighbor as the health care proxy. One child says, "A neighbor should not be making these choices." What is the nurse's best action?

- A.** Ask the adult children to vote on the treatment decision
- B.** Follow the preferences of the oldest child because family takes priority
- C.** Recognize the named proxy as the decision-maker and notify the provider and charge nurse of the family conflict
- D.** Delay all treatment until the family reaches an agreement

Correct Answer: C

When a valid advance directive names a health care proxy and the client lacks decision-making capacity, the named proxy is the appropriate surrogate decision-maker. The nurse should advocate for that role and communicate the conflict to the team. Choice A is incorrect because treatment decisions are not made by family vote. Choice B reflects a common but incorrect assumption that the oldest relative automatically decides. Choice D is unsafe because necessary care should not be delayed while family members argue.

Informed consent

325. A nurse is preparing a client for a bronchoscopy. The client says, "I signed the form, but I still don't really understand why I need this test." What is the nurse's best action?

- A.** Explain the procedure in detail and ask the client to sign a new form
- B.** Notify the provider that the client has questions before the procedure continues
- C.** Tell the client the provider would not have ordered the test unless it was necessary
- D.** Document that the client is anxious and continue with preprocedure preparation

Correct Answer: B

The provider performing or ordering the procedure is responsible for giving the information needed for informed consent, including risks, benefits, and alternatives. If the client states they do not understand, the nurse should stop and notify the provider. Choice A is a common trap because the nurse may reinforce teaching, but should not take over the provider's responsibility for obtaining informed consent.

326. Which situation requires the nurse to question whether informed consent is valid?

- A.** The client signs the consent form after receiving pain medication that causes drowsiness
- B.** The client asks the nurse where the procedure will be performed
- C.** The client requests that a family member sit with them before surgery
- D.** The client signs the consent form using their dominant hand

Correct Answer: A

Informed consent requires that the client have capacity and be able to understand the information and make a voluntary decision. A client who is sedated or drowsy from medication may not be able to provide valid consent. The other options do not by themselves suggest that consent is invalid.

327. A client who speaks limited English is scheduled for a central venous catheter insertion. The client's adult daughter says, "I can translate for my mother." What should the nurse do?

- A. Allow the daughter to interpret because she knows the client's history
- B. Ask another bilingual staff member from the unit to interpret if available
- C. Use a qualified medical interpreter to support the consent discussion
- D. Have the client sign the form if she nods when the daughter explains it

Correct Answer: C

A qualified medical interpreter should be used so the client can receive accurate information and ask questions before consenting. Family members may omit, change, or misunderstand information, so relying on the daughter is not the best approach. A bilingual staff member is not automatically the right choice unless qualified to interpret in that role.

328. The nurse witnesses a client sign a surgical consent form. Which statement best describes the nurse's role?

- A. The nurse confirms that the client fully understands all procedure risks and alternatives
- B. The nurse certifies that the surgery is necessary and urgent
- C. The nurse verifies the client's identity and witnesses the signature as voluntary
- D. The nurse assumes responsibility for obtaining the informed consent

Correct Answer: C

When witnessing a consent signature, the nurse verifies the client's identity and that the signature appears voluntary. The nurse does not certify the medical necessity of the procedure or assume the provider's duty to explain risks, benefits, and alternatives. Choice A is tempting because nurses assess understanding, but the legal responsibility for obtaining informed consent rests with the provider performing the procedure.

329. A competent adult client is scheduled for surgery. After the provider has explained the procedure and the client has signed consent, the client tells the nurse, "I've changed my mind. I don't want it done." What should the nurse do first?

- A. Tell the client it is too late to refuse because the form is already signed
- B. Notify the provider and stop further preparation for the procedure
- C. Ask the family to help persuade the client to continue
- D. Send the client to surgery and let the surgeon discuss it there

Correct Answer: B

A competent client may withdraw consent at any time before the procedure. The nurse should stop preparation and notify the provider promptly. Choices A and D ignore the client's right to refuse, and C threatens voluntariness by pressuring the client.

330. A nurse is reviewing consent for a thoracentesis with a client. Which client statement most clearly indicates a need for the nurse to delay the procedure and notify the provider?

- A. "I know they are removing fluid from around my lung."
- B. "I understand this may help me breathe easier."
- C. "I signed because my son told me I had no choice."
- D. "I want to know when I can eat after the procedure."

Correct Answer: C

Informed consent must be voluntary. If the client says they signed because a family member pressured them, the consent may not be valid. The nurse should delay the procedure and notify the provider. The other statements reflect understanding or routine questions rather than coercion.

331. A client is brought to the emergency department unconscious after a motor vehicle crash and has life-threatening internal bleeding. No family members are available. Which action is most appropriate?

- A. Delay treatment until written consent can be obtained from a relative
- B. Proceed with emergency treatment under implied consent
- C. Wait until the client regains consciousness to discuss options
- D. Ask two nurses to sign the operative consent form for the client

Correct Answer: B

In an emergency, when immediate treatment is needed to preserve life or prevent serious harm and the client cannot consent, treatment generally proceeds under implied consent. Delaying care could endanger the client. Nurses or relatives do not simply sign on behalf of a competent adult in this circumstance.

332. A nurse is caring for a client who has been declared legally incompetent by the court and is scheduled for a nonemergent procedure. Who should provide the informed consent?

- A. The nurse caring for the client
- B. The client's roommate who knows the client's wishes
- C. The legally authorized representative
- D. The charge nurse on the unit

Correct Answer: C

For a client who has been legally declared incompetent, consent for nonemergent treatment should be provided by the legally authorized representative. The nurse, roommate, and charge nurse do not have authority to give consent simply because they are available or familiar with the client.

333. A client signs a consent form for a hysterectomy. In the preoperative area, the client asks the nurse, "What are the chances I could have bleeding or damage to nearby organs? Nobody explained that part." What is the nurse's best response?

- A. Those risks are rare, so you do not need to worry about them

B. I can explain the surgical risks so we do not delay the case

C. You should ask the anesthesia provider after you get to the operating room

D. I will notify the surgeon because more discussion is needed before surgery

Correct Answer: D

Questions about procedural risks should be addressed by the provider obtaining informed consent before the procedure continues. The nurse should not minimize the concern or independently provide the full risk-benefit discussion in place of the surgeon. Directing the client to ask later in the operating room is unsafe because consent should be informed beforehand.

334. A nurse is obtaining admission information from a client scheduled for amputation. The client says, "I don't want anyone to tell my brother about my surgery." Later, the brother arrives and asks the nurse to bring in the consent form so he can review it with the client. What is the nurse's best response?

- A. Bring the form because close relatives have a right to see it
- B. Refuse to speak with the brother and ask him to leave the unit
- C. Ask the client whether the client wants the brother involved before sharing consent information
- D. Explain the procedure to the brother but avoid discussing the risks

Correct Answer: C

Client autonomy and privacy apply to consent-related information. Because the client has already expressed a wish to limit disclosure, the nurse should ask the client directly before involving the brother. Family relationship alone does not give automatic access to the client's health information or consent discussion.

335. A client is scheduled for a lumbar puncture. The provider obtained informed consent yesterday. This morning, after receiving a sedative, the client becomes confused and says, "What am I even having done today?" What should the nurse do?

- A.** Proceed because the consent was already signed yesterday
- B.** Have the spouse sign a second consent form
- C.** Hold the procedure and notify the provider of the change in the client's condition
- D.** Reorient the client briefly and send the client for the procedure

Correct Answer: C

A significant change in the client's condition or understanding before a procedure can affect whether consent remains valid in practice. The nurse should hold the procedure and notify the provider rather than assuming yesterday's signature is still sufficient in the face of current confusion. The spouse cannot automatically re-consent for a competent adult simply because confusion developed after sedation.

336. A provider asks a nurse to obtain informed consent for a bone marrow biopsy because the provider is running late. The client has not yet spoken with the provider about the procedure. Which response by the nurse is best?

- A.** I can witness the signature after you explain the procedure to the client
- B.** I can obtain the consent because I know the procedure well
- C.** The provider needs to discuss the procedure with the client before consent is obtained
- D.** The nursing supervisor should obtain the consent instead

Correct Answer: C

The provider responsible for the procedure must explain the purpose, risks, benefits, and alternatives so the client can make an informed decision. The nurse should not substitute for that role. Choice A would only be appropriate after the provider has completed the consent discussion.

337. A nurse is preparing a 17-year-old client for a nonemergent surgery. The client understands the procedure and wants it done, but the parent who usually makes medical decisions has not arrived yet. Which action is most appropriate?

- A.** Accept the adolescent's signature because the client understands the surgery
- B.** Proceed without consent because the surgery is beneficial
- C.** Delay the nonemergent procedure until consent is obtained from the appropriate authorized person
- D.** Ask the surgeon to sign for the client because the parent is unavailable

Correct Answer: C

For a nonemergent procedure, consent must come from the appropriate authorized decision-maker when the client is a minor under usual circumstances. The client's understanding is important, but it does not by itself replace the need for authorized consent. Because this is nonemergent, delaying is the safest action. The distractor in A is tempting because assent and understanding matter, but they are not the same as legal consent here.

338. A client signs consent for removal of the gallbladder. In the operating room, the surgeon discovers a nonemergent ovarian cyst and tells the nurse, "We'll remove that too while we're here." What is the nurse's best action?

- A.** Prepare for the second procedure because the client is already anesthetized
- B.** Question the additional procedure because it is outside the consent given
- C.** Document the finding and let the postoperative nurse address it later
- D.** Call the family member in the waiting room to provide verbal consent

Correct Answer: B

Consent for one procedure does not automatically cover a separate nonemergent procedure. The nurse should question the plan because the added surgery is outside the consent provided. A family member in the waiting room does not simply replace the client's authority in this situation. Emergency findings that require immediate action are different, but this stem specifies nonemergent.

339. A client is scheduled for a paracentesis. Just before transport, the client says, "I know the doctor talked to me earlier, but I was in too much pain to pay attention. Can you explain exactly what risks are involved so I can decide whether to go through with it?" Which action by the nurse is most appropriate?

- A.** Explain the procedure, risks, and benefits so the client can make an informed decision
- B.** Ask the client to sign the consent form again after receiving pain medication
- C.** Notify the provider that the client needs further explanation before the procedure
- D.** Send the client to the procedure area because the provider already discussed it earlier

Correct Answer: C

The provider performing the procedure is responsible for giving the information needed for informed consent, including risks, benefits, alternatives, and the opportunity to ask questions. Because the client says they could not pay attention and needs the risks explained before deciding, the nurse should stop and notify the provider. Choice A is a common trap: the nurse may reinforce teaching, witness the signature, and identify lack of understanding, but should not take over the provider's duty to provide the full informed-consent discussion for the procedure.

340. A client is scheduled for a cardiac catheterization later today. While completing the preprocedure checklist, the nurse notes that the consent form is unsigned. The client says, "I want the test, but the provider never came back after mentioning it this morning." Which action by the nurse is most appropriate?

- A.** Ask another nurse to witness the client's signature so the procedure is not delayed
- B.** Explain the purpose, risks, and alternatives of the procedure to the client
- C.** Notify the provider that informed consent has not been obtained
- D.** Have the client sign the form because verbal agreement was already given

Correct Answer: C

The provider performing the procedure is responsible for giving the information needed for informed consent, including risks, benefits, and alternatives. The nurse should recognize that this discussion has not occurred and notify the provider. Choice A is incorrect because witnessing a signature does not fix missing informed-consent teaching. Choice B exceeds the nurse's role if the provider has not completed the explanation. Choice D confuses willingness to proceed with valid informed consent.

341. A client with severe abdominal pain is scheduled for a nonemergent endoscopy. Just before signing the consent form, the client receives IV opioid medication and becomes drowsy, slurs words, and says, "Whatever you put in front of me, I'll sign it." What should the nurse do?

- A.** Allow the client to sign because pain medicine was ordered by the provider
- B.** Delay the consent process and notify the provider that the client may not be able to provide informed consent now
- C.** Ask a family member to sign because the client already agreed to the procedure earlier
- D.** Have the client sign and document that sedation was given before consent

Correct Answer: B

A client must be capable of understanding and making a voluntary decision at the time consent is obtained. Drowsiness, slurred speech, and impaired judgment after opioid administration call consent validity into question. The nurse should delay the process and notify the provider. Choice A and D ignore impaired decision-making capacity. Choice C is incorrect because a family member cannot simply sign for a competent adult just because the adult is temporarily impaired after medication.

342. A client who is about to sign consent for a diagnostic procedure asks, "Do I have any other choices besides this test?" Which response by the nurse is best?

- A.** The provider should discuss alternatives with you before you decide
- B.** Most clients choose this test because it is the quickest option
- C.** If you are unsure, you can just sign now and change your mind later
- D.** There are probably no other options if the test was scheduled

Correct Answer: A

Alternatives are part of informed consent and must be explained by the provider obtaining consent. The nurse should support the client's right to receive that information before deciding. Choice B pressures the client. Choice C minimizes the need for informed decision-making before signing. Choice D wrongly assumes scheduling proves no alternatives exist.

343. A nurse is preparing a client for elective hernia repair. The client reads the consent form, asks several questions, and then says, "I understand the surgery, but I do not want blood products under any circumstances." What is the nurse's priority action?

- A.** Tell the client that blood products are routinely given during surgery when needed
- B.** Document the statement and notify the provider before the procedure
- C.** Ask the client to wait and discuss it with family members
- D.** Instruct the client to sign the regular consent form first and address the issue after surgery

Correct Answer: B

The client's statement reflects a treatment limitation that could affect the plan of care and must be communicated before the procedure. The nurse should document and notify the provider so the client's wishes can be addressed appropriately. Choice A dismisses the client's right to refuse specific treatments. Choice C shifts decision-making away from the competent client. Choice D is unsafe because the issue must be clarified before proceeding.

344. A client is scheduled for a skin biopsy under local anesthesia. The provider has already explained the procedure. Which nursing action is appropriate when the client is ready to sign the consent form?

- A.** Provide legal advice about the wording of the form
- B.** Sign the form for the client if hand weakness is present
- C.** Witness the client's signature and document that the signature was observed
- D.** Promise the client that complications are very rare

Correct Answer: C

The nurse's role includes witnessing the client's signature and documenting that the signature was observed, if agency policy permits. The nurse is not responsible for legal counseling or guaranteeing outcomes. Choice B is inappropriate because signing for a client is not the nurse's role in this context. Choice D is misleading and does not support informed decision-making.

345. A client signs consent for insertion of a feeding tube. Thirty minutes later, while waiting for transport, the client says, "I know I signed, but now I feel pressured by my family. I don't think I want this." Which action should the nurse take first?

- A.** Tell the client the procedure can be canceled after arrival in the procedure room
- B.** Notify the provider and stop preparation for the procedure
- C.** Explain that the signed form means the client has already agreed
- D.** Ask the family to step in and help the client decide

Correct Answer: B

Consent must be voluntary, and a competent client may withdraw consent at any time before the procedure. The nurse should stop preparation and notify the provider. Choice A delays action and risks proceeding despite refusal. Choice C incorrectly treats the signature as irreversible. Choice D may increase pressure rather than protect autonomy.

346. A client is scheduled for a colonoscopy. The client speaks fluent conversational English but has difficulty understanding medical terms. After the provider explains the procedure in English, the client smiles and nods but then quietly asks the nurse, "What does perforation mean?" What is the nurse's best action?

- A.** Define the term in simple language and ask the provider to return if the client still has questions about the decision
- B.** Tell the client not to worry because serious complications are uncommon
- C.** Ask the client to sign the form since the provider already explained everything
- D.** Advise the client to look up unfamiliar words after the procedure

Correct Answer: A

The nurse may reinforce or clarify teaching in simple language within the nurse's scope, such as defining an unfamiliar term. If the client's question suggests uncertainty about whether to proceed, the provider should return to continue the informed-consent discussion. Choice B minimizes the concern. Choice C assumes nodding equals understanding. Choice D is inappropriate because understanding is needed before consent.

347. A client with hearing impairment is scheduled for a nonemergent procedure. The client uses sign language as the primary method of communication and reads written English poorly. The provider is ready to obtain informed consent. Which action by the nurse best supports valid consent?

- A.** Use written notes because they are faster than arranging additional services
- B.** Ask a nursing assistant who knows a few signs to interpret the discussion

C. Arrange for a qualified interpreter so the client can receive the explanation in the preferred language format

D. Ask the client's spouse to interpret because the spouse knows the client's communication style

Correct Answer: C

Valid informed consent requires that the client understand the explanation. If the client's primary language format is sign language and written English is limited, a qualified interpreter is needed. Choice A does not address the client's communication needs. Choice B uses unqualified interpretation. Choice D risks inaccuracies and conflicts and does not replace a qualified interpreter.

348. A client arrives in the preoperative area for an elective procedure and says, "The surgeon explained the operation, but I never heard what could happen if I decide not to have it." Which response by the nurse is best?

- A.** That information is part of the consent discussion, so I will notify the provider before you proceed
- B.** If the surgery was recommended, there is probably no reasonable alternative to it
- C.** The risks of refusing treatment are not usually included in informed consent
- D.** I can list the consequences of refusing so you can sign now

Correct Answer: A

A complete informed-consent discussion includes information needed for a meaningful decision, which commonly includes the nature of the procedure, risks, benefits, alternatives, and likely consequences of refusing. If the client reports that this part was not explained, the nurse should notify the provider. Choice B and C dismiss the client's right to complete information. Choice D inappropriately substitutes the nurse for the provider in obtaining informed consent.

349. A provider obtained consent for a procedure from a client yesterday. Overnight, the client developed acute confusion from hypoxia and can no longer understand questions. The procedure is elective and can safely be postponed. What is the nurse's best action?

- A.** Send the client as scheduled because the consent was signed while the client was competent
- B.** Proceed because the provider, not the nurse, decides whether consent remains valid
- C.** Hold the procedure and notify the provider that the client's current condition affects the ability to confirm ongoing consent
- D.** Have the client's roommate verify that the client wanted the procedure

Correct Answer: C

Although consent may have been signed earlier, the nurse must advocate when the client's current condition raises concern about understanding and the ability to participate in care. Because the procedure is elective and the client is now acutely confused, the nurse should hold the procedure and notify the provider. Choice A is a trap that treats the signature as the only issue. Choice B ignores the nurse's duty to act on a change in status. Choice D is irrelevant and inappropriate.

350. During preparation for a liver biopsy, a client says, "My pastor told me not to sign anything until I pray about it more." The provider is waiting and the unit is busy. What is the nurse's best response?

- A.** Tell the client the decision should be based on medical facts, not spiritual beliefs
- B.** Explain that delaying could affect the schedule and ask the client to sign now
- C.** Support the client's request for more time and notify the provider of the delay
- D.** Ask the pastor to give permission by phone so the procedure can continue

Correct Answer: C

Informed consent must be voluntary and respectful of the client's values, including spiritual beliefs. The nurse should support the client's request for more time and notify the provider. Choice A dismisses client beliefs. Choice B introduces pressure, which threatens voluntariness. Choice D wrongly shifts the decision from the client to another person.

351. A client is scheduled for elective surgery and tells the nurse, "I can read the form, but I do not understand many of the medical words in it." Which action is best?

- A.** Tell the client to sign if the provider already reviewed the procedure verbally
- B.** Read the form aloud exactly as written and ask the client to sign afterward
- C.** Assess what the client understands and arrange for the provider to clarify unclear information before signing
- D.** Ask the client's adult child to explain the form in simpler terms

Correct Answer: C

The issue is understanding, not simply the ability to read aloud. The nurse should assess what the client understands and ensure the provider clarifies information needed for an informed decision. Choice A assumes prior explanation was sufficient despite the client's stated confusion. Choice B may repeat unfamiliar terms without improving understanding. Choice D improperly relies on family to provide the consent explanation.

352. A competent adult client is refusing a recommended procedure. The client's spouse says, "Do not listen to him. He is scared and not thinking straight. I am his wife, and I consent." What should the nurse do?

- A.** Accept the spouse's consent because family members can override refusal when a procedure is recommended
- B.** Delay the procedure and arrange for the spouse to sign as next of kin
- C.** Support the client's decision and inform the provider that the client is refusing the procedure
- D.** Ask the provider to obtain signatures from both the client and spouse

Correct Answer: C

A competent adult client has the right to refuse treatment, and a spouse cannot override that refusal. The nurse should support the client's autonomy and notify the provider. Choice A and B incorrectly substitute family authority for the client's decision-making rights. Choice D suggests shared authority where the competent client alone is the decision-maker.

353. A client is being transported for a procedure when the nurse notices the consent form is signed with an illegible mark. The client has limited hand control and usually signs this way. The form has no witness signature, and there is no note describing how the client signed. What should the nurse do next?

- A.** Send the client because the exact signature style does not matter
- B.** Complete the documentation and witness process according to policy before the procedure proceeds
- C.** Rewrite the client's name on the signature line to make the form clearer
- D.** Ask the transporter to witness that the form is probably the client's

Correct Answer: B

When a client signs with a mark or altered signature style, the consent still requires proper witnessing and documentation according to policy to show that the client personally signed it. Choice A ignores a documentation problem that could affect validity. Choice C is falsification. Choice D uses an inappropriate witness and guessing rather than proper verification.

354. A client scheduled for a biopsy says, "The provider explained the procedure and risks clearly, but I still feel unsure. What would you do if you were me?" Which nursing response is most appropriate?

- A.** I think you should go ahead because the provider would not recommend it otherwise
- B.** Only you can make this decision; I can help you review your questions and ask the provider to return
- C.** If you are this uncertain, you should refuse the procedure

D. Just sign now so you do not lose your appointment time

Correct Answer: B

The nurse should support the client's decision-making without coercion or substituting personal opinion. Offering to help the client organize questions and involve the provider again protects autonomy and informed choice. Choice A is directive and may pressure the client. Choice C is also directive. Choice D prioritizes convenience over valid consent.

355. A client is scheduled for an elective bronchoscopy. After the provider leaves the room, the client tells the nurse, "I still do not understand why this test is needed for me." What is the nurse's best action?

- A.** Explain the purpose of the bronchoscopy in simpler terms so the client can sign the form
- B.** Ask another nurse to witness the signature because the provider already discussed the test
- C.** Notify the provider that the client does not understand the indication for the procedure
- D.** Have the family explain the reason for the procedure to the client

Correct Answer: C

The provider is responsible for giving the information needed for informed consent, including the nature and purpose of the procedure. If the client says they do not understand why the procedure is needed, the nurse should stop the process and notify the provider. Choice A is a trap because the nurse may reinforce teaching, but should not replace the provider's informed-consent discussion for key elements such as indication, risks, benefits, and alternatives.

356. A client has signed consent for a central venous catheter insertion. While setting up the room, the nurse hears the client tell the provider, "Actually, I changed my mind. I don't want this done." Which action should the nurse take first?

- A.** Document the statement and continue preparing the sterile field in case the client reconsiders
- B.** Tell the client that the form has already been signed and the procedure should proceed

- C.** Stop the preparation and inform the provider that the client is withdrawing consent
- D.** Ask the client's family to help persuade the client to continue

Correct Answer: C

A competent client may withdraw consent at any time before a procedure. The nurse's first action is to stop preparation and make sure the provider knows the client has withdrawn consent. Choices B and D violate client autonomy. Choice A delays the priority action and may imply pressure to proceed.

357. A nurse is caring for a client who just signed consent for an elective procedure. Which finding would cause the nurse the greatest concern about whether the consent is valid?

- A.** The client asks for reading glasses before reviewing the form
- B.** The client states, "My daughter helped me decide."
- C.** The client says, "I know this can help, but I don't really understand the possible dangers."
- D.** The client requests a few more minutes before transport

Correct Answer: C

Valid informed consent requires understanding. A client who does not understand the possible risks has not received or processed essential information. Choice B is not necessarily a problem; clients may involve family in decision-making as long as the choice remains voluntary. Choices A and D support, rather than undermine, a valid consent process.

358. A client is scheduled for a lumbar puncture. The provider obtained informed consent, and the client is now ready to sign the form. Which nursing action is appropriate?

- A.** Witness the client's signature on the consent form
- B.** Review all major risks and benefits before the client signs
- C.** Sign the form for the client if the client agrees verbally
- D.** Explain that signing means the client may not change their mind later

Correct Answer: A

An appropriate nursing role is to witness the client's signature, confirming the person signing is the client. The nurse does not obtain the informed consent discussion itself. Choice B oversteps into the provider's responsibility if used as the primary consent explanation. Choice C is inappropriate, and choice D is false because consent may be withdrawn before the procedure.

359. A provider is ready to obtain consent for an elective procedure from a client whose primary language is not English. The client's adult son says, "I speak both languages. I can translate so we can get this done quickly." What should the nurse do?

- A.** Allow the son to translate because family members know the client's preferences
- B.** Ask the son to summarize only the main points while the nurse witnesses the signature
- C.** Request a qualified medical interpreter before the consent discussion continues
- D.** Have the client sign now and ask questions later through an interpreter

Correct Answer: C

A qualified medical interpreter should be used so the client can receive accurate information and ask questions freely. Family members may omit, add, or filter information, even when trying to help. Choice A and B risk inaccurate communication, and D is unsafe because understanding must come before signing.

360. A nurse is reviewing a chart before transporting a client for a scheduled procedure. The consent form is signed, but the client now has a new prescription for IV midazolam that was administered 10 minutes ago. The client is drowsy but arousable. Which action is best?

- A.** Proceed with transport because the form was signed before the medication was given
- B.** Assess whether the client is still willing to proceed and report any hesitation immediately
- C.** Ask the provider to obtain a second signature after the procedure
- D.** Have the client's next of kin confirm the consent

Correct Answer: B

Because the consent was signed before sedation, the key issue now is whether the client still wishes to proceed and whether any change has occurred. The nurse should assess and report any withdrawal of consent or new concerns immediately. Choice A is incomplete because a client may revoke consent after signing. Choice D is inappropriate when the client had capacity and has not been shown to need a surrogate decision-maker.

361. A client is scheduled for insertion of a permanent pacemaker. While the provider is obtaining consent, the client repeatedly looks at an adult child before answering and finally says, "I'll sign because my daughter wants me to." What is the nurse's best response?

- A. Witness the signature because the client verbally agreed
- B. Tell the daughter to leave so the client can sign without distraction
- C. Pause the process and report concern that the decision may not be voluntary
- D. Ask the daughter to sign as the responsible family member

Correct Answer: C

Informed consent must be voluntary. The client's statement suggests possible coercion or undue pressure, so the nurse should pause the process and report the concern. Choice B may be helpful in some situations, but the priority is to address the voluntariness concern rather than independently manage the family dynamic. Choices A and D ignore client rights.

362. A client who is alert and oriented is scheduled for a bone marrow biopsy. The provider has already left after obtaining consent. The client asks the nurse, "What are the possible complications if I go through with this?" Which response by the nurse is best?

- A. "The provider is the best person to answer that, so I will contact the provider before anything proceeds."
- B. "Most people do fine, so there is probably nothing important to worry about."
- C. "The main risk is pain, but signing means you accept whatever happens."

D. "You already signed, so your questions have already been addressed."

Correct Answer: A

Questions about risks and complications are part of the informed-consent discussion that the provider must address. The nurse should ensure the provider returns to answer before the procedure continues. Choice B minimizes the concern, C is inaccurate and coercive, and D falsely suggests that signing ends the client's right to ask questions.

363. A client is being prepared for an elective procedure and has a signed consent form in the chart. During the preprocedure assessment, the nurse determines that the client is disoriented and cannot explain the procedure in any meaningful way. Which action is most appropriate?

- A. Proceed because the form is already signed and filed
- B. Ask the unlicensed assistive personnel to verify the signature with the client
- C. Hold the procedure and notify the provider of the change in decision-making capacity
- D. Re-explain the procedure quickly and have the client sign a new form

Correct Answer: C

A change in decision-making capacity before an elective procedure can affect whether consent remains valid and whether the procedure should proceed. The nurse should hold the process and notify the provider. Choice A relies on paperwork instead of current client status. Choice D is inappropriate because a disoriented client may lack capacity to provide valid consent.

364. A nurse is preparing a client for a thoracentesis. Which statement by the client best indicates understanding that informed consent is an ongoing process rather than a one-time signature?

- A. "Once I signed, the procedure has to be done."
- B. "If I think of more questions, I can still ask them before the procedure starts."
- C. "The nurse is responsible for deciding whether this procedure is best for me."

D. “My family can overrule me if they think I am making a bad choice.”

Correct Answer: B

Informed consent continues through the period before the procedure; the client may ask additional questions and make decisions during that time. Choices A, C, and D all conflict with client autonomy and the actual roles of the health care team.

365. A provider asks the nurse to “just get the consent signed” for an elective procedure because the schedule is running late. The client has not yet spoken with the provider and says, “I don't know much about this procedure.” What should the nurse do?

- A.** Obtain the signature because delaying care could inconvenience the unit
- B.** Explain the procedure, risks, and alternatives so the schedule can stay on time
- C.** Refuse to witness the consent until the provider has discussed the procedure with the client
- D.** Have the client sign now and write a note that teaching is still needed

Correct Answer: C

The nurse should not witness consent when the client has not had the required informed-consent discussion with the provider. Client understanding and voluntariness take priority over convenience or schedule pressure. Choice B oversteps the nursing role in obtaining informed consent. Choices A and D would allow an invalid process to continue.

366. A client signed consent this morning for an elective procedure. This afternoon, the procedure plan changes from a diagnostic biopsy to a biopsy with possible excision of additional tissue if a lesion is found. Which action by the nurse is best?

- A.** Send the client because the original consent covers any reasonable extension of the procedure
- B.** Notify the provider that additional consent discussion is needed for the changed plan
- C.** Explain the added step to the client and obtain initials on the old form
- D.** Ask the circulating nurse to clarify the plan in the operating room

Correct Answer: B

A material change in the planned procedure requires a new or updated informed-consent discussion by the provider. The original consent for one procedure does not automatically authorize a substantially expanded plan in an elective setting. Choice C is a trap because the nurse cannot independently substitute for the provider's consent discussion. Choice D delays correction until too late in the process.

367. A nurse is caring for four clients awaiting elective procedures. Which client situation should the nurse address first because it poses the greatest immediate threat to valid informed consent?

- A.** A client who signed the consent form but now says, “I did not know there was a chance of paralysis.”
- B.** A client who asks whether recovery will take days or weeks
- C.** A client who wants to call a sibling before leaving the unit
- D.** A client who asks whether personal belongings will be locked up during the procedure

Correct Answer: A

A client who states they were unaware of a serious risk may not have given truly informed consent, making this the highest-priority issue to address before the procedure proceeds. The other concerns are important but do not as directly indicate that essential consent information is missing. The trap is focusing on general client anxiety rather than on the specific cue that understanding of major risk is absent.

368. Immediately before an elective procedure, a client says, “I know I signed, but I am afraid my husband will be angry if I back out.” The spouse is standing at the bedside and says, “We are doing this today.” What is the nurse's best next action?

- A.** Ask the spouse to step out so the nurse can assess the client's wishes privately
- B.** Remind the client that the provider already reserved time and equipment
- C.** Proceed because fear alone does not invalidate a signed form

D. Request security because the spouse is likely to become disruptive

Correct Answer: A

The nurse should first create privacy to assess whether the client's decision is voluntary and whether the client wants to continue or withdraw consent. This action directly addresses possible coercion while protecting the client's autonomy. Choice B is coercive, C ignores a major voluntariness concern, and D may be unnecessary escalation before assessment.

369. A competent adult client signs consent for a procedure after the provider's explanation. Later, the client asks the nurse, "If I say no now, can anyone force me because I already agreed earlier?" Which response is best?

- A.** "No. As long as you can make your own decisions, you may change your mind before the procedure."
- B.** "Yes, because the signed consent form is a legal contract that cannot be reversed."
- C.** "Your family would decide that, since they were present during the discussion."
- D.** "Only the nurse can decide whether it is too late to cancel."

Correct Answer: A

A competent client retains the right to refuse or withdraw consent before the procedure. A signed consent form documents the process but does not remove the client's autonomy. Choices B, C, and D incorrectly transfer decision-making away from the client.

370. A client is scheduled for an elective cardiac catheterization. The provider explained the procedure and left. The client tells the nurse, "I'm not sure I want this. Are there any other options besides having this done?" What is the nurse's best action?

- A.** Explain the alternative treatment options so the client can decide
- B.** Tell the client that signing the form means the procedure should proceed as planned
- C.** Notify the provider that the client has questions about alternatives before consenting
- D.** Ask the family member to help the client decide quickly

Correct Answer: C

The provider obtaining informed consent is responsible for explaining the procedure, risks, benefits, and alternatives. If the client asks about alternatives, the nurse should stop the process and notify the provider so those questions can be addressed. Choice A is a common trap because teaching is part of nursing care, but detailed discussion of procedural alternatives for informed consent belongs to the provider performing or ordering the procedure. Choices B and D ignore the client's right to make a voluntary, informed decision.

371. A nurse is asked to witness a client's signature on a consent form for a scheduled procedure. Which action is appropriate when serving as the witness?

- A.** Confirm that the client appears to sign voluntarily and is the person named on the form
- B.** Verify that the provider fully explained all risks, benefits, and alternatives
- C.** Assess whether the procedure is medically necessary before allowing the client to sign
- D.** Provide the detailed explanation if the client has not yet spoken with the provider

Correct Answer: A

When witnessing the signature, the nurse verifies the identity of the signer and that the signature appears voluntary. The nurse is not responsible for giving the full procedural explanation or independently determining medical necessity. Choice B is tempting, but the nurse cannot personally validate every detail of the provider's discussion; instead, the nurse should act if the client indicates lack of understanding.

372. A client is scheduled for a nonemergency procedure. Which situation most clearly means the consent should not be signed at this time?

- A.** The client asks whether recovery will take several days
- B.** The client received a prescribed opioid analgesic 5 minutes ago and is now very sleepy
- C.** The client wants a family member present during the discussion
- D.** The client asks the nurse where the procedure will be performed

Correct Answer: B

Valid informed consent requires that the client have decision-making capacity and not be impaired when signing. A client who has become very sleepy immediately after medication may not be able to provide valid consent. The other situations do not by themselves prevent valid consent. Choice A may mean the client needs more information, but it does not automatically invalidate the process.

373. A competent adult client is scheduled for an elective surgery. The client's sibling says, "I already signed the paperwork because my brother gets nervous." What is the nurse's best response?

- A.** Accept the signature because family members may sign when the client is anxious
- B.** Ask the sibling to sign again in the nurse's presence
- C.** Explain that the client must sign personally if able and notify the provider of the issue
- D.** Proceed as long as the client verbally agrees with the sibling's decision

Correct Answer: C

A competent adult client must provide their own informed consent. Anxiety alone does not transfer decision-making authority to a family member. The nurse should not accept the sibling's signature and should notify the provider so the consent process can be done correctly. Choice D is a trap because verbal agreement does not fix an invalid signature process for a competent adult who can sign.

374. A client is being prepared for an elective procedure. The consent form was signed yesterday. Today the nurse finds the client alert and oriented but saying, "I thought this was just to look at the problem, not to remove anything." What is the nurse's priority action?

- A.** Have the client sign the same form again
- B.** Hold preparation and notify the provider that the client may not understand the planned procedure
- C.** Tell the client the provider would not schedule the procedure unless removal might be needed
- D.** Document the statement and continue because the form is already signed

Correct Answer: B

The client's statement suggests a possible misunderstanding of what is planned, so the nurse should stop further preparation and notify the provider. Informed consent requires understanding, not just a signature. Choice A is incorrect because repeating a signature without clarifying the misunderstanding does not create valid consent. Choice D incorrectly treats the form as sufficient by itself.

375. A client with hearing impairment is scheduled for a procedure requiring informed consent. The client uses hearing aids and reads written information well. Which nursing action best supports valid consent?

- A.** Remove the hearing aids to limit distraction during the explanation
- B.** Rely only on the client's nodding because it shows agreement
- C.** Ensure communication aids are available and that the client can receive and ask about the explanation
- D.** Ask a family member to decide whether the client understood enough to sign

Correct Answer: C

Valid informed consent depends on effective communication. The nurse should help ensure the client has the needed aids and opportunity to receive information and ask questions. Choice B is a trap because nodding does not confirm understanding. Choices A and D interfere with the client's autonomy and communication access.

376. The provider obtained informed consent for a scheduled procedure. Just before transport, the client says to the nurse, "I signed because everyone kept pressuring me, and I didn't feel like I could say no." What should the nurse do first?

- A.** Document the statement and send the client because the form is complete
- B.** Stop the transport process and report concern about possible coercion
- C.** Tell the client that pressure from family is common before procedures
- D.** Ask the transporter to wait while the nurse convinces the client to proceed

Correct Answer: B

Consent must be voluntary. A statement suggesting pressure or coercion raises immediate concern about whether consent is valid, so the nurse should stop transport and report the concern. Choice C minimizes a serious legal and ethical issue. Choice D is wrong because the nurse should not persuade the client to proceed.

377. A confused older adult is scheduled for an elective procedure. The chart shows no legal guardian or health care proxy, and the client cannot explain the procedure or its purpose. Which action by the nurse is most appropriate?

- A.** Ask the nearest relative at the bedside to sign immediately
- B.** Proceed because the provider already wrote the order for the procedure
- C.** Hold the procedure and notify the provider that an appropriate authorized decision-maker is needed
- D.** Ask another nurse to witness the client's mark on the consent form

Correct Answer: C

If the client lacks decision-making capacity for an elective procedure, the nurse should not proceed without an appropriate authorized decision-maker according to facility and legal requirements. Choice A is a trap because the nearest relative is not automatically authorized in every situation. Choices B and D ignore the lack of valid consent.

378. A client is scheduled for a skin graft. The provider has explained the procedure, and the consent form is on the bedside table unsigned. The client says, "I understand what they told me, and I want to go ahead." Which action by the nurse is appropriate?

- A.** Witness the client's signature if the client is competent and has no further questions for the provider
- B.** Sign the client's name because verbal agreement has already been given
- C.** Explain that consent can be signed only in the procedure area
- D.** Refuse to let the client sign until a family member is present

Correct Answer: A

If the provider has already given the necessary explanation and the competent client indicates understanding and willingness, the nurse may witness the client's signature. Choice B is clearly inappropriate because no one signs for a competent client. Choices C and D add barriers that are not required.

379. A client who signed consent for an elective colonoscopy asks the nurse, "What happens if I choose not to have this today?" Which response is best?

- A.** You have the right to refuse, and the provider should discuss the possible consequences and other options with you
- B.** Because you already signed, refusing now would require court approval
- C.** If you refuse, the facility can discharge you for noncompliance
- D.** Once the IV is started, refusal is no longer allowed

Correct Answer: A

A competent client retains the right to refuse an elective procedure even after signing consent. The provider should discuss the consequences of refusal and available alternatives. The other choices incorrectly suggest the client loses autonomy after signing or after preparation has begun.

380. A nurse is caring for a client who is to sign consent for a blood transfusion. Which finding most strongly suggests the client has decision-making capacity at this time?

- A.** The client states, "My spouse told me this is what I should do."
- B.** The client can describe the treatment in general terms and explain a reason for accepting it
- C.** The client says, "I trust the doctor, so details do not matter."
- D.** The client has already signed consent for another treatment earlier in the week

Correct Answer: B

A client showing capacity can demonstrate understanding in a meaningful way and express a reasoned choice. Choice A suggests reliance on another person's preference, not the client's own informed decision. Choice C does not show understanding, and choice D is unrelated to current capacity for this specific decision.

381. A client is being transported to the operating room for an elective procedure. In the hallway, the client quietly tells the nurse, "I want to ask one more question before this starts." The surgical team is waiting. What is the nurse's best action?

- A.** Return the client to a private area or pause transport and notify the provider so the question can be addressed before proceeding
- B.** Tell the client the opportunity for questions ended when the consent form was signed
- C.** Advise the client to ask after the procedure because the schedule cannot be delayed
- D.** Answer the question regardless of its content so the procedure can stay on time

Correct Answer: A

Informed consent is an ongoing process, and the client must have the opportunity to ask questions before the procedure. The nurse should pause the process and involve the provider as needed. Choice D is a common trap because some questions may exceed the nurse's role, especially if they involve risks, benefits, or alternatives. Choices B and C wrongly prioritize schedule over client rights.

382. A provider asks the nurse to witness consent for an elective procedure. The client says, "I understand the benefits, but I was never told what serious problems could happen." The provider has stepped off the unit. What should the nurse do?

- A.** Witness the signature because the client understands the benefits
- B.** Explain the major complications from personal experience and then witness the signature
- C.** Delay witnessing the signature and contact the provider to complete the informed consent discussion

D. Ask the unlicensed assistive personnel to stay with the client until transport arrives

Correct Answer: C

The client's statement shows that the consent discussion is incomplete. The nurse should not witness the signature until the provider addresses the missing information. Choice B is incorrect because the nurse should not substitute a personal explanation of procedural risks for the provider's duty in informed consent. Choice D does not address the immediate problem.

383. A client with limited literacy is scheduled for an elective procedure. After the explanation, the client says, "I don't read well, so I just sign wherever people tell me." Which nursing action best promotes valid informed consent?

- A.** Ask the client to sign now to avoid embarrassment in front of staff
- B.** Arrange for the information to be reviewed in an understandable verbal format and verify the client's understanding before proceeding
- C.** Request that a roommate help explain the form in simpler words
- D.** Tell the client the signature is mostly routine paperwork

Correct Answer: B

The nurse should advocate for communication in a form the client can understand and help verify that the client has meaningful understanding before the process continues. Choice D is a dangerous minimization of informed consent. Choice C violates privacy and uses an inappropriate person to explain sensitive information.

384. A client is scheduled for an elective bronchoscopy in 2 hours. After receiving a prescribed sedative, the client becomes drowsy but can still answer simple questions. The provider arrives with the consent form and asks the nurse to have the client sign it now. What is the nurse's best response?

- A.** Ask the client to sign quickly before the sedative causes more drowsiness
- B.** Have a family member sign because the procedure is already scheduled

- C.** Delay signing and notify the provider that consent should be obtained when the client is able to make an informed decision
- D.** Witness the signature if the client states the provider already explained the procedure earlier

Correct Answer: C

Informed consent requires that the client have decision-making capacity and give consent voluntarily. A sedated, drowsy client may not be able to fully understand information or make an informed decision, so the nurse should delay signing and notify the provider. Choice A ignores impaired capacity. Choice B is inappropriate for an elective procedure when a family member is not automatically authorized to consent. Choice D is a trap because prior explanation alone does not make consent valid if the client is currently impaired.

385. A client is scheduled for an elective laparoscopic cholecystectomy. When the nurse reviews the chart, the consent form is signed, but the client's prescribed interpreter has not yet arrived. The client speaks very little English and nods yes to most questions. Which action should the nurse take?

- A.** Proceed because the signed form is already in the chart
- B.** Ask a family member to confirm that the client understands the procedure
- C.** Hold preparation and arrange qualified language assistance before proceeding
- D.** Explain the procedure slowly using simple English words

Correct Answer: C

Informed consent requires that the client understand what is being consented to and make the decision voluntarily. A language barrier can prevent true understanding, so the nurse should stop the process and obtain qualified language assistance. A signed form alone does not prove informed consent. Using family members as interpreters can create errors or pressure, and simplifying English does not ensure adequate comprehension.

386. A client is scheduled for insertion of a central venous catheter. After the provider explains the procedure, the nurse asks the client to describe what will happen. Which client statement best shows understanding needed for informed consent?

- A.** They are putting a tube into a large vein to give treatment, and there are risks like infection or bleeding
- B.** I signed the paper because my provider thinks this is best
- C.** I do not know exactly what they are doing, but I trust the team
- D.** This means I definitely will not need any more needle sticks

Correct Answer: A

The best evidence of understanding is the client's ability to state, in their own words, the basic purpose of the procedure and acknowledge possible risks. Choice A reflects teach-back and realistic understanding. Choices B and C show trust or compliance rather than informed understanding. Choice D shows an inaccurate absolute belief, which suggests misunderstanding.

387. A client is scheduled for elective knee arthroscopy this afternoon. The client is alert, oriented, and has no legal restrictions on decision-making. The spouse tells the nurse, "Don't tell my husband that the surgeon mentioned a chance of nerve injury. He gets anxious and might cancel." What is the nurse's best response?

- A.** Respect the spouse's request because reducing stress is therapeutic
- B.** Ask the surgeon to speak only with the spouse about the risks
- C.** Explain that the client has the right to receive complete information and make the decision personally
- D.** Document the spouse's concern and continue preparing the client for surgery

Correct Answer: C

A competent adult client has the right to receive relevant information and decide whether to accept or refuse a procedure. The nurse must advocate for that right. Withholding risk information to avoid cancellation undermines informed consent. Choices A, B, and D all support bypassing the client's autonomy.

388. In the preoperative area, a client says, "I know I signed consent for the procedure, but I changed my mind and do not want it anymore." The surgeon has already scrubbed in, and the operating room is ready. Which action by the nurse is most appropriate?

- A.** Tell the client it is too late to cancel because the consent form is legally binding
- B.** Notify the surgical team immediately and stop further preparation
- C.** Ask the family to encourage the client to proceed since everything is prepared
- D.** Give the prescribed preoperative medication to help the client relax before deciding

Correct Answer: B

A client may withdraw consent for an elective procedure at any time before it begins. The nurse should immediately stop the process and notify the surgical team. A signed consent form does not remove the client's right to refuse. Family pressure is inappropriate, and sedating the client before the issue is addressed could further impair decision-making.

389. A client arrives in the emergency department with severe internal bleeding after a motor vehicle crash. The client is unconscious, no family is present, and immediate surgery is needed to prevent death. There is no advance directive available. Which statement best describes how care should proceed?

- A.** Treatment must wait until a family member can be reached for consent
- B.** Only noninvasive care can be given until the client regains consciousness
- C.** The nurse should sign the operative consent because the situation is urgent
- D.** Emergency treatment may proceed under implied consent because delay would threaten the client's life

Correct Answer: D

In a true emergency, when a client lacks decision-making capacity and delaying treatment would risk serious harm or death, care may proceed under implied consent. The nurse should not sign as the decision-maker. Waiting for family or limiting care despite a life-threatening emergency would be inappropriate when urgent intervention is necessary.

Discharge and referrals

390. A client with newly diagnosed heart failure is being prepared for discharge. Which action by the practical nurse best supports a safe discharge plan?

- A.** Telling the client to call the clinic only if severe symptoms occur
- B.** Confirming the client understands medication, diet, daily weights, and follow-up appointments
- C.** Giving the client a stack of printed instructions just before transport arrives
- D.** Assuring the client that home adjustment will be easy after a few days

Correct Answer: B

A safe discharge plan includes verifying understanding of key self-management needs such as medications, diet, symptom monitoring, and follow-up care. Simply handing over papers or giving reassurance does not confirm readiness. Waiting until severe symptoms occur is unsafe because early reporting is often needed.

391. A client is being discharged after treatment for pneumonia and now requires home oxygen. Which referral is most appropriate for the nurse to anticipate?

- A.** Durable medical equipment service
- B.** Speech therapy
- C.** Cardiac rehabilitation
- D.** Pastoral care only

Correct Answer: A

A durable medical equipment service is the most appropriate referral because the client needs home oxygen equipment arranged for safe discharge. Speech therapy and cardiac rehabilitation do not address the immediate equipment need. Pastoral care may support coping but does not meet the oxygen setup need.

392. The practical nurse is reinforcing discharge teaching for a client with a new colostomy. Which client statement shows a need for further teaching?

- A.** I will watch the skin around the stoma for redness.
- B.** I should empty the pouch before it gets too full.
- C.** If I have questions, I can contact the ostomy nurse or clinic.
- D.** I will stop drinking fluids so the pouch does not fill as often.

Correct Answer: D

Stopping fluids to reduce pouch output is unsafe and indicates misunderstanding. The other statements reflect appropriate discharge teaching: monitoring peristomal skin, emptying the pouch regularly, and seeking follow-up help from appropriate resources.

393. A client who lives alone is being discharged after a hip fracture repair. The client says, "I can manage at home," but also reports difficulty getting to the bathroom without help. What is the nurse's priority action?

- A.** Document the client's preference and complete the discharge
- B.** Ask the provider to delay discharge without further assessment
- C.** Report the safety concern and collaborate on home support needs before discharge
- D.** Instruct the client to stay in bed most of the day at home

Correct Answer: C

The nurse should identify the safety concern and collaborate with the team about support needs before discharge. The issue is not simply the client's stated preference; it is whether the discharge plan is safe. Telling the client to stay in bed is inappropriate and increases risk for complications.

394. A client with diabetes is leaving the hospital with a new insulin regimen. Which outcome best shows effective discharge teaching?

- A.** The client nods while the nurse reviews written instructions.
- B.** The client states that a family member will read the instructions later.
- C.** The client correctly demonstrates how to draw up insulin and explains when to take it.
- D.** The client says, "I took medicine before, so I know what to do."

Correct Answer: C

Return demonstration plus accurate explanation is the best evidence that teaching was effective. Nodding, relying on someone else to read instructions, or assuming prior medication experience does not confirm understanding of a new insulin regimen.

395. A client with a recent stroke has mild swallowing difficulty and is preparing for discharge. Which referral should the nurse expect to be most helpful?

- A.** Respiratory therapy
- B.** Occupational health
- C.** Speech-language pathology
- D.** Radiology scheduling

Correct Answer: C

Speech-language pathology is the most appropriate referral for swallowing difficulty because that discipline evaluates and treats dysphagia and helps reduce aspiration risk. Respiratory therapy may help with breathing needs, but the key issue here is swallowing safety.

396. The nurse is preparing discharge instructions for a client with limited reading ability. Which approach is best?

- A. Use medical terms so the client becomes familiar with them
- B. Provide only written instructions to avoid embarrassment
- C. Speak clearly, use simple language, and ask the client to explain the plan back
- D. Direct all teaching to the family and skip the client

Correct Answer: C

Simple language combined with teach-back is the best approach because it promotes understanding and respects the client's participation in care. Using complex terminology, relying only on written materials, or excluding the client can lead to unsafe discharge teaching.

397. A client is being discharged with a new walker. Which action by the nurse is most important before the client leaves?

- A. Make sure the walker color matches the client's preference
- B. Confirm the client can use the walker correctly and safely
- C. Ask the family to store the walker in the garage until needed
- D. Suggest that the client practice with the walker after getting home

Correct Answer: B

The priority is to confirm safe and correct use of the assistive device before discharge. Delaying practice until home can increase fall risk. Appearance and storage issues are not the main safety concern at discharge.

398. A client is to be discharged after an uncomplicated postoperative stay. The spouse asks the nurse to review all discharge instructions while the client is asleep, saying, "I handle everything." What is the nurse's best response?

- A. Provide the teaching only to the spouse because the spouse requested it
- B. Wait until the client is awake and include the client in the teaching whenever possible
- C. Refuse to discuss any discharge plan with the spouse under any circumstances

D. Ask the spouse to sign that the client understands the instructions

Correct Answer: B

The client should be included in discharge teaching whenever possible because the client has the right to participate in care decisions and understand post-discharge needs. Teaching only the spouse without involving the capable client is inappropriate. Refusing all discussion with the spouse is too absolute; with the client's involvement or permission, family participation may be helpful.

399. A client with peripheral neuropathy is being discharged home. The home has several loose rugs and poor lighting in the hallway. Which nursing action is best?

- A. Advise the client to walk less often at home
- B. Recommend removing tripping hazards and improving lighting before discharge
- C. Tell the client that home hazards are a family issue, not a nursing concern
- D. Suggest wearing socks instead of shoes indoors

Correct Answer: B

The nurse should address home safety hazards that increase fall risk as part of discharge planning. Removing loose rugs and improving lighting directly reduces risk. Advising less walking can worsen mobility, and socks indoors may increase slipping risk compared with supportive footwear.

400. A client is being discharged after treatment for a deep vein thrombosis and will take an anticoagulant at home. Which statement by the client requires immediate follow-up?

- A. I will keep my follow-up appointments.
- B. I will tell my provider about any unusual bleeding.
- C. If I miss a dose, I will double the next one.
- D. I will use a soft toothbrush.

Correct Answer: C

Doubling the next anticoagulant dose after a missed dose can be dangerous and requires immediate correction. The other statements show appropriate understanding of basic anticoagulant precautions and follow-up.

401. The practical nurse is preparing a client for discharge after hospitalization for depression. Which finding is the highest priority to communicate to the charge nurse or provider before the client leaves?

- A. The client says the medication schedule seems complicated.
- B. The client says transportation to counseling may be difficult.
- C. The client says, "Nothing will matter once I get home anyway."
- D. The client asks whether follow-up should be in person or virtual.

Correct Answer: C

A hopeless statement may signal suicide risk and must be addressed before discharge. Although medication complexity and transportation barriers matter, they are not the highest priority compared with a possible immediate safety threat.

402. A client with a new below-knee amputation is planned for discharge in 24 hours. Which situation creates the strongest reason to question discharge readiness?

- A. The client asks for extra dressing supplies.
- B. The client reports phantom limb sensations.
- C. The client has not yet demonstrated safe transfer from bed to wheelchair.
- D. The client wants the follow-up appointment written on a calendar.

Correct Answer: C

Safe transfer ability is essential for immediate post-discharge function and safety. If the client cannot transfer safely, the discharge plan needs reevaluation. Phantom sensations can occur after amputation and do not alone show discharge is unsafe. Asking for supplies or a written appointment reflects engagement, not unpreparedness.

403. A client who speaks limited English is being discharged with several new medications. The client's bilingual teenage child offers to translate. What is the nurse's best action?

- A. Use the child as the main interpreter because the child knows the family best
- B. Provide the instructions in English and ask the child to explain them later at home

C. Arrange qualified language assistance and include the client directly in the teaching

D. Postpone all discharge teaching until the next clinic visit

Correct Answer: C

Qualified language assistance best supports accurate, safe discharge teaching and respects the client's direct participation. Using a child as the main interpreter can lead to errors, omissions, and privacy concerns. Delaying all teaching until after discharge is unsafe.

404. A client is being discharged after an exacerbation of chronic obstructive pulmonary disease. The client says, "I want to quit smoking this time, but I don't know where to start." Which nursing action is best before discharge?

- A. Advise the client to try stopping without assistance to avoid depending on outside support
- B. Provide a referral to a smoking-cessation program and review how to access it after discharge
- C. Tell the client to wait until the follow-up appointment to discuss quitting methods with the provider
- D. Focus discharge teaching only on prescribed medications because lifestyle changes are not part of discharge planning

Correct Answer: B

The best action is to connect the client with an appropriate referral before discharge. A smoking-cessation program supports continuity of care and addresses the client's stated need for help getting started. Choice A ignores evidence-based support and places the burden entirely on the client. Choice C delays needed help when the client is ready now. Choice D is incorrect because discharge planning includes referrals and health-promotion resources, not just medication instructions.

405. A client is being discharged after a hospitalization for cellulitis. The prescribed oral antibiotic is available only at a pharmacy across town, and the client says, "I don't have a car and my bus route doesn't go there." Which action by the practical nurse best supports a safe discharge?

- A. Advise the client to skip the medication until transportation becomes available
- B. Notify the nurse or case manager so the transportation and medication-access barrier can be addressed before discharge
- C. Tell the client to ask a neighbor for help after arriving home
- D. Proceed with discharge because the prescription has already been written

Correct Answer: B

The best action is to communicate the barrier before discharge so the team can help arrange a workable plan, such as a different pharmacy, delivery, or transportation resources. Safe discharge planning includes identifying barriers to obtaining treatment. Choice C might help, but it relies on an uncertain plan. Choices A and D allow an avoidable interruption in treatment.

406. An older adult is being discharged after treatment for dehydration. The client lives with an adult daughter who works long hours. During discharge teaching, the client says, "I forget whether I already took my pills." Which referral would be most appropriate for the nurse to suggest?

- A. Dietitian services only
- B. Home health nursing for medication management support
- C. Speech therapy for communication training
- D. Respiratory therapy for breathing exercises

Correct Answer: B

Home health nursing is the best referral because the immediate problem is safe medication management at home. The nurse can help reinforce teaching, assess adherence, and identify ongoing needs. The other options do not address the client's stated risk of medication errors.

407. A client is preparing for discharge 1 day after cataract surgery. Which statement by the client indicates that the nurse should reinforce the discharge instructions?

- A. I will avoid rubbing my eye.
- B. I will call the provider if I have increasing pain.
- C. I can drive myself to my follow-up visit tomorrow.
- D. I will use my eye drops as directed.

Correct Answer: C

The client should not plan to drive immediately after cataract surgery, so this statement indicates a need for further teaching and discharge planning, including transportation for follow-up care. The other statements reflect appropriate understanding of common postoperative instructions.

408. A client with chronic kidney disease is being discharged with instructions for a low-sodium diet. Which response by the practical nurse best checks the client's understanding before discharge?

- A. Do you understand the diet restrictions?
- B. You know not to use table salt, right?
- C. Tell me what foods you would choose for meals tomorrow.
- D. Your family can read the diet sheet when you get home.

Correct Answer: C

Having the client describe specific meal choices uses teach-back and gives the nurse a clear way to evaluate understanding. Choices A and B invite yes/no answers and may overestimate understanding. Choice D shifts responsibility away from assessing the client's readiness before discharge.

409. A client is being discharged after a mild stroke with residual right-arm weakness. The client says, "Buttoning my shirt and opening containers are hard now." Which referral is most appropriate?

- A. Occupational therapy
- B. Respiratory therapy
- C. Cardiac rehabilitation
- D. Audiology

Correct Answer: A

Occupational therapy focuses on helping clients perform activities of daily living and adapt to functional limits, such as dressing and managing household tasks. The other referrals do not address the client's difficulty with self-care tasks caused by arm weakness.

410. A client with newly diagnosed hypertension is being discharged with a prescription for medication and written instructions. The client nods during teaching but avoids eye contact and says, "I'll figure it out at home." Which action is best?

- A.** Document that teaching was completed because the client received written instructions
- B.** Ask the provider to cancel the discharge because the client seems uninterested
- C.** Use a calm, nonjudgmental approach to reassess understanding and barriers to following the plan
- D.** Direct all further teaching to the client's visiting friend instead

Correct Answer: C

The client's response suggests possible confusion, overwhelm, or another barrier. The best action is to reassess understanding and explore what may interfere with following the plan. Choice A is unsafe because receiving paperwork does not prove readiness. Choice B is premature without further assessment. Choice D may violate client-centered teaching if the client has not asked for that arrangement.

411. A client is being discharged after a cesarean birth. The client says there is no crib at home yet and the baby will sleep on a couch cushion "for a few nights." What is the nurse's priority action?

- A.** Complete discharge because the client can buy a crib later
- B.** Notify the registered nurse or appropriate team member to address the unsafe home plan before discharge
- C.** Instruct the client to place pillows around the baby during sleep
- D.** Suggest that the baby sleep in the client's bed to make feeding easier

Correct Answer: B

An unsafe sleep plan is a significant discharge safety concern and should be escalated so the team can help address it before discharge. Choices C and D increase risk rather than reduce it. Choice A ignores an immediate newborn safety issue.

412. A client is being discharged after treatment for a venous leg ulcer and needs ongoing dressing changes. The spouse says, "I want to help, but I feel nervous about doing it wrong." Which nursing action is best before discharge?

- A.** Tell the spouse that the skill will become easier with time at home
- B.** Arrange for return demonstration of the dressing change and communicate the need for follow-up support
- C.** Advise the spouse to watch online videos after discharge
- D.** Limit teaching to the client because family members should not be involved

Correct Answer: B

A return demonstration is the best way to assess whether the caregiver can perform the dressing change safely, and follow-up support may be needed. Choice A is reassuring but does not verify competence. Choice C is unreliable as the primary plan. Choice D ignores an available caregiver who is asking for help and may be part of the client's discharge plan.

413. A client is being discharged after a short hospitalization for asthma. The provider prescribed two inhalers: one for daily control and one for sudden symptoms. Which client statement requires follow-up by the nurse?

- A.** I will keep the quick-relief inhaler with me when I leave home.
- B.** I will use the controller inhaler even when I feel well.
- C.** I will stop both inhalers once my cough is gone for a few days.
- D.** I will ask if I should get refills before I run out.

Correct Answer: C

Stopping both inhalers based on temporary symptom improvement shows misunderstanding of the discharge plan, especially the purpose of a daily controller medication. Choices A, B, and D reflect safer self-management. The common trap is treating all inhalers as if they are only for immediate symptoms.

414. A client with chronic low back pain is being discharged from rehabilitation. The client reports needing help with bathing and meal preparation at home but says, "I don't need a nurse coming to my house." Which referral would best match the client's stated needs?

- A. Hospice care
- B. Assistive services for help with activities of daily living
- C. Telemetry monitoring
- D. Isolation precautions teaching

Correct Answer: B

The client's main needs involve assistance with daily activities, so supportive in-home assistive services are the best match. Hospice care is not indicated by the scenario. Telemetry and isolation teaching do not address bathing and meal preparation needs.

415. A client with a new tracheostomy is scheduled for discharge tomorrow. During a teaching session, the client's partner successfully performs suction setup but says, "If the tube comes out, I'll just wait for the home nurse to arrive." What is the nurse's best action?

- A. Continue with discharge because the partner learned the routine parts of care
- B. Praise the partner for honesty and document that emergency teaching is optional
- C. Recognize that discharge readiness is incomplete and report the need for further emergency teaching and evaluation
- D. Tell the partner not to worry because accidental decannulation is rare

Correct Answer: C

A caregiver for a client with a new tracheostomy must be prepared for urgent problems, not just routine care. The statement shows incomplete readiness, so the need for further teaching and evaluation should be reported before discharge. Choices A, B, and D minimize a potentially serious safety gap.

416. A client is being discharged after surgical repair of an ankle fracture. The prescription includes crutches. During preparation for discharge, the client becomes sweaty and unsteady while using the crutches and says, "I'll just learn once I get home." What is the best nursing action?

- A. Discharge the client with written crutch instructions only
- B. Report that the client is not demonstrating safe use of the assistive device and requires further evaluation before discharge
- C. Ask a family member to sign that they accept responsibility for the risk
- D. Tell the client to avoid walking until the follow-up appointment next week

Correct Answer: B

The client is not safely demonstrating the ability to use required equipment, so discharge readiness should be questioned and reported for further evaluation. Choice A ignores a clear fall risk. Choice C does not remove the nurse's responsibility to address safety concerns. Choice D is impractical and does not solve the mobility problem.

417. A client with newly diagnosed seizure disorder is preparing for discharge. The client says, "I can still bathe in the tub when I'm home, as long as I keep the bathroom door unlocked." Which response by the nurse is best?

- A. That is fine if someone checks on you every 30 minutes.
- B. You should avoid bathing until your follow-up visit.
- C. Showers are generally safer than tub baths because of the risk of injury if a seizure occurs.
- D. Use the tub only on days when you feel rested.

Correct Answer: C

For a client with seizures, tub bathing creates a higher risk of injury or drowning if a seizure occurs. Teaching should emphasize safer alternatives such as showering. Choices A and D do not adequately reduce risk. Choice B is too broad and not as helpful as giving a safer specific alternative.

418. A client is being discharged after treatment for a diabetic foot infection. The provider prescribes wound care, glucose monitoring, and a follow-up visit in 2 days. The client quietly tells the nurse, "I don't have electricity right now, and my refrigerator isn't working." Which concern is the priority to address before discharge?

- A. Whether the client will be comfortable doing dressing changes
- B. Whether written instructions should be printed in larger font
- C. Whether the client has a safe plan for storing medications and carrying out the treatment regimen at home
- D. Whether the client prefers morning or afternoon appointments

Correct Answer: C

Loss of electricity can directly affect safe medication storage and the client's ability to carry out the prescribed treatment plan, making this the priority issue to address before discharge. Choice A matters, but the environmental barrier may make the entire plan unsafe. Choices B and D are secondary compared with the immediate home-resource problem.

419. A client is being discharged after a myocardial infarction. The provider has prescribed a cardiac diet, activity pacing, several new medications, and follow-up with a cardiology clinic in 1 week. The client says, "I live alone, and English is not my first language. My nephew usually helps me understand medical forms, but he is out of town." Which action by the practical nurse is best before discharge?

- A. Ask the client to sign the discharge papers now and review the instructions again at the follow-up visit

- B. Request a qualified medical interpreter and review the discharge plan using teach-back
- C. Give the client the written instructions and highlight the most important parts to read later
- D. Call the nephew and leave a detailed voicemail with the client's discharge instructions

Correct Answer: B

A qualified medical interpreter helps ensure the client understands medications, diet, activity limits, and follow-up care before leaving. Using teach-back checks actual understanding instead of assuming the client understood. Choice C provides information but does not address the language barrier or confirm comprehension. Choice A delays needed clarification until after discharge, which is unsafe. Choice D is inappropriate because it does not ensure immediate client understanding and may violate privacy if detailed health information is left on voicemail.

420. A client is preparing for discharge after treatment for pneumonia. The provider prescribed home oxygen. Which finding is most important for the practical nurse to report before discharge?

- A. The client says, "My granddaughter will visit after school."
- B. The client states, "I keep a small wood stove going most evenings."
- C. The client asks whether a humidifier can be used in the bedroom.
- D. The client wants written instructions to take home.

Correct Answer: B

A wood stove presents a significant fire and oxygen-safety concern that must be addressed before discharge. The nurse should report and help coordinate teaching and planning. The other options may require education, but they are not the most urgent safety issue.

421. A client with a new colostomy is being discharged tomorrow. Which outcome best shows the client is ready for discharge teaching to be considered effective?

- A. The client watches the nurse change the pouch without asking questions
- B. The client states that a home health nurse will handle all ostomy care

C. The client correctly demonstrates emptying the pouch and explains when to seek help for skin irritation

D. The client says, "I read the booklet, so I should be fine."

Correct Answer: C

Discharge readiness is best shown by return demonstration plus understanding of when to get help. Watching, reading, or assuming someone else will do all care does not confirm the client can manage safely at home.

422. A client is being discharged after treatment for major burns to the arm and chest. The provider prescribes dressing changes and range-of-motion exercises at home. Which referral is most appropriate to support the client's recovery?

A. Speech therapy

B. Occupational therapy

C. Respiratory therapy

D. Audiology

Correct Answer: B

Occupational therapy is appropriate for helping the client maintain function, mobility for daily activities, and recovery related to movement and self-care after burn injury. Speech therapy and audiology do not address these needs. Respiratory therapy may be needed in some burn cases, but the scenario specifically highlights dressing care and range-of-motion needs.

423. The practical nurse is reviewing discharge plans for four clients. Which client should the nurse identify as the highest priority for referral to social services before discharge?

A. A client with seasonal allergies who wants help comparing over-the-counter medications

B. A client with a healed wrist sprain who asks for a work excuse note

C. A client with newly prescribed insulin who says there is not enough food at home this week

D. A client with mild eczema who asks whether to switch laundry detergent brands

Correct Answer: C

A client starting insulin who lacks adequate food is at risk for unsafe glucose management after discharge. Social services can help address food insecurity and access issues. The other concerns are less urgent and do not pose the same immediate risk to safe recovery.

424. A client is being discharged after a hip fracture repair. The client lives alone in a second-floor apartment with no elevator. Which action by the practical nurse is best?

A. Ask whether the client has ever climbed stairs before the injury

B. Arrange for discharge and instruct the client to move slowly at home

C. Collaborate with the team to evaluate whether the home setting and discharge plan are safe

D. Tell the client to stay with a friend only if pain becomes severe

Correct Answer: C

The home environment may not be safe after hip repair, especially when the client lives alone and must manage stairs. The nurse should collaborate with the care team to reassess discharge planning and needed support. Choice B minimizes a major safety issue. Choice A gathers limited information but does not address the problem. Choice D delays planning until after discharge.

425. A client with newly diagnosed dysphagia after a stroke is preparing for discharge. Which referral should the practical nurse expect to be most important before the client goes home?

A. Speech-language pathology

B. Podiatry

C. Dermatology

D. Audiology

Correct Answer: A

Speech-language pathology is the most appropriate referral for dysphagia because swallowing safety and diet recommendations are central to discharge planning. The other specialties do not address the immediate swallowing concern.

426. A client is scheduled for discharge after treatment for deep vein thrombosis and will take an anticoagulant at home. Which client statement requires the most immediate follow-up by the practical nurse?

- A. "I'll use a soft toothbrush."
- B. "I'll call if I notice black stools."
- C. "I usually take aspirin when I get headaches."
- D. "I'll keep my follow-up lab appointment."

Correct Answer: C

Taking aspirin with an anticoagulant can increase bleeding risk and requires follow-up teaching before discharge. The other statements reflect appropriate understanding of bleeding precautions and follow-up care.

427. A client with chronic obstructive pulmonary disease is being discharged with several new medications. During teaching, the client's spouse answers every question for the client. The client looks away and stays silent. What is the best action by the practical nurse?

- A. Accept the spouse's answers because caregivers often manage medications at home
- B. Ask the spouse to leave immediately and continue teaching only with the client
- C. Direct some questions to the client and assess the client's own understanding of the discharge plan
- D. Skip the teaching session and leave written instructions at the bedside

Correct Answer: C

The nurse should assess the client's own understanding rather than assuming the spouse's responses reflect the client's readiness. This supports safe discharge and respects the client's participation in care. Choice A may miss a knowledge gap. Choice B is unnecessarily abrupt. Choice D does not verify understanding.

428. A client is being discharged after treatment for a pressure injury. The home plan includes a special mattress overlay, repositioning schedule, and weekly nurse visits. Which statement by the client shows the need for further discharge teaching?

- A. "I'll change positions even if the mattress feels comfortable."
- B. "If the skin looks more red or starts draining, I'll report it."

- C. "Once the overlay is delivered, I won't need to turn as often."
- D. "I'll keep the follow-up visits so the wound can be checked."

Correct Answer: C

A support surface helps but does not replace repositioning. The client still needs frequent turning and skin protection. The other statements show appropriate understanding of monitoring and follow-up.

429. A practical nurse is planning discharge teaching for a client with limited reading ability who is starting tube feedings at home. Which approach is best?

- A. Provide detailed written instructions only, so the client can review them privately
- B. Use plain language, demonstrate the procedure, and ask the client to return-demonstrate the steps
- C. Direct all teaching to the family member to save the client from embarrassment
- D. Delay teaching until the morning of discharge so the instructions are fresh

Correct Answer: B

Using plain language, demonstration, and return demonstration is the best way to support understanding and safe care when reading ability is limited. Written instructions alone may not be effective. Teaching only the family member excludes the client. Waiting until the last minute increases the chance of poor learning and missed problems.

430. A client with a below-knee amputation is preparing for discharge from rehabilitation. The client says, "I can get around with my walker, but I have no idea how I'll bathe safely at home." Which referral is most appropriate?

- A. Occupational therapy
- B. Respiratory therapy
- C. Radiology
- D. Cardiac rehabilitation

Correct Answer: A

Occupational therapy focuses on safe performance of daily activities such as bathing, dressing, and home adaptation after loss of limb. The other options do not address the client's stated need.

431. A client is being discharged after treatment for tuberculosis and will continue medications at home. The client says, "I felt better after a few days last time, so I stopped taking them." What is the practical nurse's priority action?

- A.** Reinforce the importance of completing the full medication regimen and notify the team about the adherence concern
- B.** Tell the client to restart the medications only if symptoms return
- C.** Focus on diet teaching first because nutrition is essential to recovery
- D.** Continue discharge because the client has already had tuberculosis teaching before

Correct Answer: A

Stopping tuberculosis medication early is a major adherence and public health concern. The nurse should immediately reinforce completion of therapy and communicate the concern to the team so discharge planning can address follow-up needs. Choices B and D are unsafe. Choice C addresses a lesser priority than medication adherence.

432. A client is being discharged after treatment for opioid overdose. The client will return home with a parent. Which nursing action is most important before discharge?

- A.** Encourage the client to rest for several days and avoid upsetting conversations
- B.** Arrange follow-up support for substance-use treatment and reinforce the discharge plan with the client and parent
- C.** Advise the parent to control all of the client's finances immediately
- D.** Delay any referral discussions until the client requests help

Correct Answer: B

After an opioid overdose, coordinated discharge planning should include follow-up support for substance-use treatment and clear reinforcement of the plan. This addresses safety and continuity of care. Choice A is incomplete and does not address the central discharge need. Choice C is not the nurse's best immediate action. Choice D misses an important opportunity for referral and support.

433. A client is being discharged after a new diagnosis of heart failure. The provider prescribes a low-sodium diet, daily weights, and several medications. During discharge teaching, the client says, "I share a shelter room, and I don't have a scale." What is the nurse's best action before discharge?

- A.** Tell the client to estimate fluid retention by how tight shoes feel
- B.** Ask the charge nurse to delay teaching until the day of follow-up
- C.** Notify the appropriate team member to help arrange community resources for obtaining a scale
- D.** Instruct the client to weigh once a week at a pharmacy instead of daily

Correct Answer: C

Daily weights are an important part of home management for heart failure, so the lack of access to a scale creates a discharge barrier that should be addressed before the client leaves. Referral for community or discharge-planning support is the best action because it helps remove the obstacle safely. Choice D is a common trap: weekly weights are not an adequate substitute when the plan calls for daily monitoring. Choice A is unreliable, and delaying teaching does not solve the problem.

434. A client is preparing for discharge after cataract surgery. The client says, "I know I shouldn't drive today, but I'll be fine taking the bus alone. My daughter can come next week if I need help then." Which response by the nurse is best?

- A.** Ask the client to sign a form stating transportation risks were explained
- B.** Reinforce discharge instructions and help arrange transportation or support for the trip home today
- C.** Tell the client to stay in the waiting room until vision feels normal again
- D.** Suggest the client close one eye during the bus ride to reduce dizziness

Correct Answer: B

The best response addresses the immediate discharge need: safe transportation and support after a procedure that can affect vision.

Reinforcing instructions and helping arrange assistance promotes safety and appropriate discharge planning. Choice A focuses on documentation rather than solving the problem. Choice C is not a safe or practical plan, and Choice D gives inappropriate advice that could increase risk.

435. A client is being discharged after a mild stroke with residual weakness in the dominant hand. The client will need daily blood glucose checks and insulin injections at home. Which referral is most appropriate before discharge?

- A. Speech-language pathology
- B. Home health nursing
- C. Respiratory therapy
- D. Cardiac rehabilitation

Correct Answer: B

Home health nursing is the best referral because the client needs ongoing support with glucose monitoring, insulin administration, and safety assessment in the home. Speech-language pathology would be more appropriate for swallowing or communication problems, respiratory therapy for breathing-related needs, and cardiac rehabilitation for cardiac recovery.

436. A client is ready for discharge after abdominal surgery. The provider prescribed a walker for temporary use at home. Which action by the practical nurse is best before the client leaves?

- A. Tell the client to buy any walker at a pharmacy after discharge
- B. Ask the unlicensed assistive personnel to explain walker safety after discharge paperwork is signed
- C. Confirm that the walker is available and that the client can use it safely before discharge
- D. Delay all discharge teaching until the client reports pain is completely gone

Correct Answer: C

The best action is to confirm the equipment is available and the client can use it safely before discharge. Safe discharge planning includes verifying needed equipment and the client's ability to use it. Telling the client to purchase equipment later risks an unsafe transition. UAP should not replace nursing assessment and discharge teaching. Waiting for complete absence of pain is unnecessary and unrealistic.

437. A client with newly diagnosed type 2 diabetes is preparing for discharge. The client speaks very little English, and the adult grandchild offers to interpret the discharge instructions. Which action by the practical nurse is best?

- A. Use the grandchild because family members know the client's needs best
- B. Provide written instructions in English and ask the family to review them later
- C. Request a qualified medical interpreter for discharge teaching
- D. Postpone all teaching until the follow-up clinic visit

Correct Answer: C

A qualified medical interpreter is the best choice to support accurate, safe communication and informed participation in discharge planning. Family members may omit, add, or misunderstand information. English-only written instructions are not appropriate when the client has limited English proficiency. Delaying all teaching would leave the client unprepared for care at home.

438. A client is being discharged after treatment for cellulitis and will continue oral antibiotics at home. Which client statement shows correct understanding of the discharge plan?

- A. I'll stop the pills when the redness is gone.
- B. I'll take the antibiotic until it is finished unless my provider changes it.
- C. I'll save the extra doses in case the infection comes back.
- D. I'll double the next dose if I forget one.

Correct Answer: B

Taking the antibiotic until it is finished, unless the provider changes the plan, shows correct understanding. Stopping early can lead to incomplete treatment. Saving leftover doses is unsafe because antibiotics should be taken only as prescribed for the current illness. Doubling a missed dose can increase risk of adverse effects.

439. A client with chronic kidney disease is being discharged with a new low-potassium diet. The practical nurse has completed teaching. Which action best evaluates whether the teaching was effective?

- A.** Ask the client whether the instructions seemed clear
- B.** Give the client a printed list of foods to review at home
- C.** Have the client choose a meal from several options and explain why it fits the diet
- D.** Ask the family member if the client usually follows diet instructions

Correct Answer: C

Having the client apply the information by choosing an appropriate meal and explaining the choice is the best evaluation of learning. This teach-back style approach measures understanding, not just exposure to information. Asking whether instructions were clear or giving handouts does not show actual comprehension. A family member's opinion is less reliable than the client's own demonstration.

440. A client is being discharged after an exacerbation of multiple sclerosis. The client says, "My daughter works days, so I'll be alone most of the time." The home plan includes transfer assistance, meal preparation, and bathing help. Which referral is most appropriate?

- A.** Occupational health
- B.** Assistive personnel through home care services
- C.** Hospice care
- D.** Dialysis services

Correct Answer: B

Home care assistive personnel are appropriate to help with personal care and routine support such as bathing and meal preparation, based on the care plan and supervision requirements. Occupational health is unrelated to home support needs. Hospice is for end-of-life care, which is not indicated here. Dialysis services are unrelated to the scenario.

441. A practical nurse is discharging a client who has a new prescription for a controlled pain medication after orthopedic surgery. Which instruction is most important to include to reduce the risk of accidental harm at home?

- A.** Store the medication in a secure location out of reach of others
- B.** Stop taking the medication as soon as appetite returns
- C.** Take the medication only with a full meal every time
- D.** Crush the medication if it is hard to swallow

Correct Answer: A

Safe storage in a secure location is especially important for a controlled pain medication to prevent accidental ingestion, misuse, or diversion. The other choices are not universally correct. Appetite does not determine when pain medication should stop, not all such medications must be taken with a full meal, and some formulations should never be crushed.

442. A client is being discharged after treatment for dehydration caused by vomiting and diarrhea. The provider has written prescriptions and follow-up instructions. The client says, "I don't have a car, and the clinic is two bus rides away." Which response by the practical nurse is best?

- A.** Advise the client to call the clinic only if symptoms return
- B.** Document the concern and notify the appropriate team member to address transportation needs before discharge
- C.** Tell the client to ask neighbors for rides after going home
- D.** Cancel the follow-up appointment because attendance is uncertain

Correct Answer: B

Transportation is a discharge barrier that can interfere with follow-up care, so the nurse should document it and involve the appropriate team member, such as case management or social work, before discharge. Telling the client to manage it alone does not address the barrier. Canceling the appointment reduces continuity of care.

443. A client is being discharged after a transient ischemic attack. The provider recommends follow-up with outpatient physical therapy, but the client says, "I don't think I need that." Which action by the practical nurse is best?

- A.** Respect the refusal, explore the client's concerns, and reinforce the purpose of the referral
- B.** Tell the client that therapy is mandatory after this diagnosis
- C.** Ask the family to sign the referral forms for the client
- D.** Remove the referral from the plan without telling the provider

Correct Answer: A

The nurse should respect the client's right to participate in decisions while exploring concerns and explaining the reason for the referral. This supports advocacy and informed decision-making. Telling the client therapy is mandatory is coercive. Family cannot simply make the decision for a competent client, and the nurse should not independently remove the referral without communicating with the team.

444. A client is scheduled for discharge with a new indwelling urinary catheter after urinary retention. Which outcome best indicates the client is ready for discharge?

- A.** The client states that a home nurse will do all catheter care
- B.** The client verbalizes that cloudy urine is expected with long-term catheter use
- C.** The client demonstrates how to keep the drainage bag below bladder level and when to report problems
- D.** The client says the catheter can be disconnected briefly for easier walking

Correct Answer: C

Correct catheter management includes keeping the drainage bag below bladder level and knowing when to report problems such as fever, pain, leakage, or poor drainage. Depending entirely on a future home nurse does not show readiness for self-care. Cloudy urine is not simply expected and may signal infection. Disconnecting the system unnecessarily raises infection risk.

445. A client with heart disease is being discharged on several medications. During teaching, the client becomes tearful and says, "I won't be able to pay for all of these." Which action by the practical nurse is the priority?

- A.** Continue teaching so all medication information is covered before discharge
- B.** Suggest the client take only the most important medications until the next appointment
- C.** Notify the appropriate team member to help address medication-cost barriers before discharge
- D.** Tell the client to discuss costs with the pharmacist after leaving the hospital

Correct Answer: C

Medication cost is a major barrier to treatment adherence and safe discharge. The priority is to involve the appropriate team member before discharge so options can be explored, such as assistance programs or alternative plans from the prescriber. Suggesting the client choose medications independently is unsafe. Delaying the issue until after discharge increases risk of nonadherence and readmission.

446. A client who is legally blind is being discharged with a new insulin regimen. The spouse says, "I'll just draw it up for him every time." Which response by the practical nurse is best?

- A.** That's fine, as long as the spouse is home for every dose
- B.** The client should skip insulin doses when alone to avoid mistakes
- C.** The nurse should arrange teaching and referral for adaptive strategies and resources to support safe medication administration

D. Switching to oral medication is the simplest solution

Correct Answer: C

The best action is to support a safe, realistic discharge plan by arranging teaching and referral for adaptive methods and resources. This may include skill training, accessible devices, and follow-up support. Relying only on the spouse may fail when the spouse is unavailable. Skipping doses is unsafe, and changing medications is not a nursing decision.

447. A practical nurse is reviewing discharge planning for a client with a new tracheostomy who will go home with family. Which information is most critical to verify before discharge?

- A.** The family has selected a date for the follow-up appointment
- B.** A neighbor is willing to visit once a week
- C.** Required supplies and emergency equipment are available, and caregivers can use them correctly
- D.** The client prefers written instructions over verbal teaching

Correct Answer: C

For a client going home with a tracheostomy, the most critical issue is whether necessary supplies and emergency equipment are available and whether caregivers can use them safely. This directly affects airway safety at home. Follow-up timing and teaching preferences matter, but they do not outweigh immediate airway-related readiness.

448. A client with a recent spinal cord injury is being discharged to home. The discharge plan includes a wheelchair, pressure-relief schedule, and bowel program. Which situation requires the practical nurse to intervene before discharge?

- A.** The client can describe the bowel program but wants a written schedule too
- B.** The family says they will move furniture later to make space for the wheelchair
- C.** The client asks for the provider's office number in case questions come up
- D.** The caregiver correctly explains the pressure-relief schedule

Correct Answer: B

If the home is not prepared for wheelchair access before discharge, the client may be unable to move safely through the environment, making this a discharge barrier that requires intervention now. Wanting written instructions and contact numbers supports safe care. A caregiver who can explain the pressure-relief schedule demonstrates readiness.

449. A client with chronic obstructive pulmonary disease is preparing for discharge with home oxygen. During the discharge review, the client says, "My electricity was shut off once last winter, but I should be able to manage." Which action by the practical nurse is best?

- A.** Continue the discharge because the oxygen prescription has already been written
- B.** Ask the client to sign that the oxygen risks were explained and proceed with discharge
- C.** Notify the registered nurse or provider and request case management or social work assistance before discharge
- D.** Tell the client to use the oxygen only during the day until the electric bill is caught up

Correct Answer: C

Home oxygen depends on a safe and reliable plan. A history of electricity shutoff creates a discharge safety concern and a need for referral to case management or social work to help address resources, equipment planning, and safe follow-up. Choice A ignores a major barrier to safe care at home. Choice B focuses on paperwork rather than solving the problem. Choice D gives unsafe advice because the client should not independently reduce prescribed oxygen use based on finances.

450. A client is being discharged after hospitalization for heart failure. The provider's discharge prescriptions include a low-sodium diet, daily weights, and a follow-up appointment in 1 week. The client says, "I live alone, and I forget things sometimes." Which action by the practical nurse is most appropriate before discharge?

- A.** Ask the client to sign the discharge papers and review the instructions again quickly

- B.** Request a referral for home health services to reinforce teaching and monitor adherence at home
- C.** Tell the client to call the clinic only if shortness of breath becomes severe
- D.** Delay discharge until a family member can move in with the client

Correct Answer: B

Home health services are appropriate because the client has an identified risk for difficulty managing the plan at home and may need reinforcement of teaching and monitoring after discharge. Choice A does not address the client's forgetfulness. Choice C gives incomplete guidance and does not support safe follow-up. Choice D is unnecessary because the safer and more practical step is arranging support services rather than assuming discharge must wait for a live-in family member.

451. A client is preparing for discharge after a total hip arthroplasty. The client will return home with a partner and has prescriptions for pain medication and outpatient follow-up. Which finding is the highest priority for the practical nurse to report before discharge?

- A.** The client asks whether driving can resume in 2 weeks
- B.** The client states that the raised toilet seat has not yet been delivered to the home
- C.** The partner says the client can use a low, soft couch for most of the day
- D.** The client reports mild incisional soreness rated 3 out of 10 after walking

Correct Answer: B

A missing raised toilet seat is a discharge safety concern because the client may not be able to use the bathroom safely at home after hip surgery without the needed equipment. This should be reported so the discharge plan can be corrected before the client leaves. Choice C is concerning and should be corrected with teaching, but unavailable essential equipment is the more immediate barrier to a safe discharge. Choice A is a routine follow-up question. Choice D describes expected mild postoperative discomfort.

452. An older adult is being discharged after treatment for pneumonia. The practical nurse reviews the plan for oral antibiotics, increased fluids, and a follow-up visit. Which client statement best indicates a need for referral to additional community support before discharge?

- A.** "My neighbor can pick up my prescriptions this evening."
- B.** "I usually write instructions on a calendar so I do not forget."
- C.** "I have not had enough food in the house since my check ran out last week."
- D.** "I will call if my cough is not improving in a few days."

Correct Answer: C

Lack of food at home signals an unmet basic need and possible inability to follow the recovery plan, so the client may need referral to case management, social services, or community food resources before discharge. Choice A identifies available support. Choice B shows use of a helpful strategy. Choice D may reflect a need for some teaching about specific return precautions, but it does not show the same urgent need for community resource referral.

453. A client with a new colostomy is being discharged home. The spouse will assist with care. Which outcome best shows that discharge teaching and referral planning have been effective?

- A.** The client states that the home health nurse will explain ostomy care after discharge
- B.** The spouse says that written instructions can be read later if problems occur
- C.** The client empties and changes the pouch using correct technique and identifies when to contact the provider
- D.** The client says that having a colostomy should not affect any daily routines

Correct Answer: C

Effective discharge teaching is best shown by return demonstration plus understanding of when to seek help. That indicates the client is prepared to manage care safely at home. Choice A relies on future teaching instead of current readiness. Choice B delays learning until after discharge, which is unsafe. Choice D reflects unrealistic understanding because an adjustment period and care changes are expected.

454. A client is being discharged after an ischemic stroke with mild dysphagia and right-sided weakness. The client lives in a second-floor apartment with no elevator. The provider has prescribed a modified-texture diet, daily medications, and follow-up rehabilitation. Which referral should the practical nurse prioritize to support a safe discharge?

- A. Pastoral care for emotional support during recovery
- B. Occupational therapy to discuss return-to-work planning
- C. Social services or case management to address home access and needed support resources
- D. A routine smoking-cessation class in 3 months

Correct Answer: C

The immediate discharge problem is whether the client can safely access and function in the home environment. A second-floor apartment without elevator access, combined with weakness and swallowing-related care needs, creates a major barrier that case management or social services should address before discharge through coordination of resources, equipment, or placement options. Choice B may be useful later, but return-to-work planning is not the priority. Choice A may help coping but does not resolve the discharge barrier. Choice D is not the urgent need right now.

Quality improvement

455. A practical nurse on a medical-surgical unit notices that several clients scheduled for morning medications are receiving them 45 to 60 minutes late because the medication room becomes crowded during shift change. Which action best reflects a quality improvement approach?

- A. Report the coworkers responsible for the delays to the nurse manager for disciplinary action
- B. Track when delays occur, look for patterns, and work with the team to test a change in workflow
- C. Tell clients that medication times are approximate and should not be questioned
- D. Begin giving medications early to avoid any chance of delay

Correct Answer: B

Quality improvement focuses on identifying a system problem, collecting data, and testing changes to improve outcomes. Option B addresses the process causing delays rather than blaming individuals. Option A focuses on punishment, not system improvement. Option C dismisses a care issue, and option D may create new safety problems by giving medications outside the prescribed time.

456. A clinic reviews its no-show rate for follow-up visits and finds that missed appointments are highest among clients who depend on public transportation. Which quality improvement response is most appropriate?

- A. Discharge clients who miss more than one appointment
- B. Schedule every client at the same time of day for consistency
- C. Explore barriers and test reminders or transportation-support referrals for affected clients
- D. Document the pattern and wait 6 months to see whether it improves on its own

Correct Answer: C

Quality improvement seeks to identify causes of poor outcomes and design changes that improve access and follow-through. Transportation is a likely barrier, so exploring it and testing targeted interventions is the best approach. Option A punishes clients and may worsen inequity. Option B does not address the barrier. Option D delays needed action despite a known pattern.

457. The nurse manager asks a practical nurse to participate in a quality improvement project about client falls. Which data would be most useful for determining whether a new fall-prevention plan is working?

- A.** The number of nurses who say they are trying harder
- B.** The fall rate before and after the new plan is started
- C.** The average age of the nursing staff on the unit
- D.** The number of visitors entering the unit each day

Correct Answer: B

To evaluate whether a fall-prevention change is effective, the team should compare fall data before and after the intervention. That directly measures the outcome of interest. Staff opinion alone does not show improvement, and the other options are not closely tied to the effectiveness of the fall-prevention plan.

458. A nurse notices that two clients with heart failure were readmitted within a week of discharge because they did not understand how to weigh themselves daily or when to call the provider. Which quality improvement action should the nurse suggest first?

- A.** Standardize discharge teaching and verify understanding before clients leave
- B.** Avoid discharging clients with heart failure until they can repeat the entire textbook content
- C.** Stop giving written instructions because they may confuse clients
- D.** Tell the provider that readmissions are unavoidable in clients with chronic illness

Correct Answer: A

A clear first improvement step is to standardize the teaching process and confirm client understanding, such as with teach-back. This directly addresses the identified gap. Option B is unrealistic and not client-centered. Option C removes a helpful support rather than improving it. Option D treats a modifiable problem as unavoidable.

459. A unit is trying to reduce specimen-labeling errors. Which statement by the practical nurse best shows understanding of quality improvement?

- A.** "If one staff member made the mistake, replacing that person should solve the problem."
- B.** "We should look at when and how labels are applied so we can find weak points in the process."
- C.** "These errors happen in every facility, so tracking them is not very helpful."
- D.** "Once we remind everyone to be careful, no further follow-up is needed."

Correct Answer: B

Quality improvement examines the care process to identify where errors are most likely to occur, then tests system changes. Option B reflects that systems-based approach. Option A focuses too narrowly on blame. Option C ignores the value of measurement. Option D assumes education alone is enough and skips reassessment.

460. A long-term care facility starts using a checklist before client transfers from bed to wheelchair. After 1 month, staff want to know whether the checklist is being used consistently. Which measure is the best example of a process measure?

- A.** The percentage of transfers in which staff completed the checklist
- B.** The number of client fractures during the month
- C.** The average length of stay for residents on the unit
- D.** The number of family compliments about the facility

Correct Answer: A

A process measure evaluates whether the planned action is actually being carried out. The percentage of transfers with completed checklists directly measures use of the intervention. Fractures are an outcome measure, length of stay is not specific to the transfer process, and family compliments are too indirect.

461. On a rehabilitation unit, call-light response times have increased. A practical nurse suggests reviewing staffing, client acuity, and the times of day when delays are worst before changing the routine. What is the main purpose of this step?

- A.** To identify likely contributors to the problem before choosing an intervention
- B.** To prove that the delays are caused by clients using the call light too often
- C.** To avoid involving unlicensed assistive personnel in improvement efforts
- D.** To delay action until the problem resolves without changes

Correct Answer: A

Reviewing staffing, acuity, and timing helps the team analyze the problem and identify contributing factors so changes are targeted rather than random. Option B jumps to blame without evidence. Option C is unrelated, and option D misuses data gathering as an excuse not to act.

462. A practical nurse participates in a meeting about reducing pressure injuries. The team decides to test a turning reminder tool on one hallway for 2 weeks instead of changing the whole facility at once. What is the best rationale for this plan?

- A.** Small tests of change allow the team to evaluate feasibility and make adjustments before wider use
- B.** A change should never be used throughout a facility, even if it works well
- C.** Pressure injury prevention is the responsibility of only one hallway's staff
- D.** Testing on a small scale guarantees the change will succeed everywhere else

Correct Answer: A

Quality improvement commonly uses small tests of change so teams can assess whether an intervention is workable, safe, and effective before broader implementation. Option D is a trap because a successful small test does not guarantee identical results in every setting. Options B and C are incorrect because they misunderstand the purpose of pilot testing.

463. A home health agency tracks medication reconciliation errors at admission. After introducing a new reconciliation form, errors drop for 2 weeks and then return to the previous level. Which next action is most appropriate?

- A.** Conclude the project failed and return immediately to the old form
- B.** Blame the newest employees because the improvement did not last
- C.** Reevaluate how the form is being used and what barriers are affecting consistency
- D.** Stop measuring errors because the results are discouraging

Correct Answer: C

When initial gains are not sustained, the team should study implementation barriers, variation in use, and workflow issues. That helps determine whether the problem is the tool, the process, or both. Option A is premature. Option B focuses on blame instead of systems. Option D removes the very data needed for improvement.

464. During a quality improvement review, a practical nurse reports that insulin administration errors occur most often when one nurse is interrupted multiple times during the medication pass. Which intervention best targets the identified cause?

- A.** Post a visible medication pass interruption-reduction plan and reinforce its use
- B.** Ask nurses to complete all insulin doses at the end of the shift instead
- C.** Require the nurse involved to write an apology for each error
- D.** Increase the number of chart audits unrelated to medication administration

Correct Answer: A

If interruptions are a major contributor, an intervention designed to reduce interruptions during medication administration directly addresses the cause. Option B would create unsafe delays. Option C is punitive and does not improve the system. Option D may increase workload without targeting the specific problem.

465. A unit compares two months of data and finds that catheter-associated urinary tract infections decreased after a new insertion checklist was adopted. Before concluding that the checklist caused the improvement, which additional consideration is most important?

- A.** Whether client volume, catheter use, or documentation practices also changed during that period
- B.** Whether the unit manager strongly prefers checklists
- C.** Whether staff can memorize the exact infection totals without reviewing the report
- D.** Whether the team presented the results at a staff celebration

Correct Answer: A

Quality improvement interpretation requires looking for other factors that may have influenced the outcome. Changes in client census, catheter use, or documentation could affect the infection rate and should be considered before assigning cause. The other options do not meaningfully strengthen the conclusion.

466. A practical nurse joins a team working to lower falls on a mixed medical unit. Data show most falls happen when clients try to toilet without help between 0500 and 0700. Which proposed change is most targeted to the data?

- A.** Place fall-risk posters in the staff lounge
- B.** Increase purposeful rounding and toileting assistance during the high-risk early morning period
- C.** Teach all discharged clients about home throw-rug safety
- D.** Move every client's breakfast to noon

Correct Answer: B

The data identify a specific pattern: unassisted toileting during a certain time window. Increasing rounding and toileting assistance during that period directly addresses the highest-risk situation. Option A is too indirect, option C is unrelated to the inpatient problem, and option D does not logically address the cause.

467. A quality improvement team wants to reduce delays in pain reassessment after PRN analgesics are given. Which indicator best reflects whether the team is improving the care process rather than only documenting symptoms?

- A.** The percentage of PRN analgesic doses followed by reassessment within the expected time frame
- B.** The number of clients who requested stronger medications during the month
- C.** The average age of clients receiving PRN analgesics
- D.** The total number of pain diagnoses listed on admission

Correct Answer: A

If the goal is to improve timely pain reassessment after PRN medication, the clearest process indicator is the percentage of doses followed by reassessment within the expected time. The other options may describe the population or medication needs but do not show whether the care process improved.

468. During review of incident reports, a practical nurse notices that several documentation omissions occur on days when clients transfer rapidly between the emergency department and the inpatient unit. Which quality improvement recommendation is most appropriate?

- A.** Advise staff to chart from memory at the end of the shift to save time
- B.** Develop a standardized handoff and transfer documentation process to reduce missed information
- C.** Stop accepting clients from the emergency department during busy hours
- D.** Remove incident reporting because it makes staff anxious

Correct Answer: B

The omissions are associated with a transition point in care, so a standardized handoff and transfer documentation process is the best systems-level intervention. Option A increases risk of inaccuracy. Option C is impractical and does not improve the process. Option D eliminates learning from errors instead of fixing them.

469. A facility launches a project to improve vaccine screening on admission. After staff education, screening rates improve briefly but drop when agency nurses begin covering weekend shifts. Which conclusion is most accurate?

- A.** The problem is solved because the initial improvement proved the project worked
- B.** The education was enough, and weekend results should be ignored
- C.** The process may not be built into the workflow strongly enough for consistent use across different staff
- D.** Agency nurses should be excluded from caring for newly admitted clients

Correct Answer: C

A drop in performance when staffing changes suggests the improvement was not fully integrated into the workflow or system. Sustainable quality improvement should function reliably across staff groups, including temporary coverage. Option A ignores the loss of reliability. Option B dismisses meaningful data. Option D blames a group rather than improving the process.

470. A practical nurse on a postoperative unit notices that clients often wait a long time for prescribed antiemetic medication after reporting nausea. To begin a quality improvement effort, which action is most appropriate first?

- A.** Ask the provider to change all antiemetic prescriptions to a different medication
- B.** Collect data on when nausea is reported, when medication is administered, and where delays occur
- C.** Tell staff to respond faster whenever a client says nausea is severe

D. Document each delayed dose as individual staff negligence

Correct Answer: B

Quality improvement starts by clearly defining the problem and gathering data to understand the current process. Tracking when nausea is reported, when the medication is given, and where delays happen helps identify the cause of the delay before solutions are chosen. Choice C is too vague, and choices A and D jump to conclusions without evidence.

471. A clinic wants to improve blood pressure recheck rates for clients with elevated readings. Which measure is the best outcome measure for this project?

- A.** The percentage of staff who attended an in-service on hypertension follow-up
- B.** The number of reminder cards stocked at the checkout desk
- C.** The percentage of clients with elevated blood pressure who complete a recheck visit within the recommended time
- D.** The number of exam rooms with blood pressure education posters

Correct Answer: C

An outcome measure reflects the actual result of care. Here, the desired result is that clients return for timely blood pressure rechecks. Staff attendance, reminder card supply, and posters are structure or process-related items, not the client outcome itself.

472. A long-term care unit is reviewing a rise in evening medication omissions. Which finding best suggests a system issue rather than an isolated staff mistake?

- A.** One nurse forgot a dose during an unusually busy shift
- B.** A newly hired nurse made one charting error on the first day of orientation
- C.** Missed doses increase repeatedly when medications are delivered late from the pharmacy
- D.** A nurse admits forgetting to initial one administration record

Correct Answer: C

Quality improvement focuses on patterns and systems. Repeated omissions linked to delayed pharmacy delivery point to a recurring process problem affecting care. The other options describe single events that may require follow-up but do not show a broader system trend.

473. A practical nurse is part of a team trying to improve discharge teaching for clients prescribed a new inhaler. Which action best supports quality improvement?

- A.** Assume the teaching is effective if no client complains
- B.** Use teach-back and track how many clients can correctly demonstrate inhaler use before discharge
- C.** Provide the written instructions only to save staff time
- D.** Wait for readmissions to increase before changing the teaching process

Correct Answer: B

Quality improvement uses specific actions and measurable results. Teach-back plus tracking correct demonstration provides direct evidence about whether the teaching process is working. Choices A, C, and D rely on assumptions or delayed recognition of problems rather than active evaluation.

474. A unit introduces bedside shift report to improve communication. After 3 weeks, the team wants to know whether staff are actually performing bedside report as intended. Which data point would be most useful?

- A.** Client satisfaction scores from the previous year
- B.** The percentage of shift changes in which bedside report is observed or documented
- C.** The total number of licensed staff on payroll
- D.** The average length of stay for clients on the unit

Correct Answer: B

This question asks whether the new process is being carried out, so the best measure is adherence to bedside shift report. That is a process measure. Satisfaction scores and length of stay are broader outcomes, and payroll size does not show whether the intervention is being used.

475. A practical nurse reviews incident data and finds that most glucometer documentation errors occur on one unit during the first hour after shift change. Which interpretation is best?

- A.** The nurses on that unit are careless and need immediate discipline
- B.** The unit should stop checking blood glucose during that hour
- C.** The pattern suggests a workflow factor near shift change that should be investigated
- D.** The errors are probably random because every unit has some documentation problems

Correct Answer: C

A cluster of errors in one place and time suggests a process or workflow issue, such as interruptions, handoff confusion, or competing tasks. Quality improvement seeks to understand those contributing factors before assigning blame. Choices A and D ignore the pattern, and B removes needed care instead of improving the process.

476. A pediatric clinic wants to reduce delays in giving scheduled immunizations during well-child visits. The team plans to test a change by placing vaccine supplies in each exam room for one week. Which statement best explains why this is a useful quality improvement strategy?

- A.** A small test of change lets the team evaluate the idea before adopting it more widely
- B.** Any change made for one week is automatically considered successful
- C.** Short tests remove the need to compare data from before and after the change
- D.** Testing a change on a small scale guarantees the problem will be solved permanently

Correct Answer: A

A small test of change helps a team learn whether an intervention is feasible and effective before expanding it. It supports practical improvement with lower risk and easier adjustment. Choices B, C, and D overstate what a short test can prove.

477. A practical nurse joins a quality improvement team reviewing delayed wound dressing changes. The team wants to understand the current process from the provider order to completion of the dressing change. Which tool would be most helpful?

- A. A process map of the steps involved
- B. A list of staff vacation requests
- C. A copy of the unit's fire drill log
- D. A summary of client diagnoses by room number

Correct Answer: A

A process map helps the team visualize each step in the workflow and identify where delays, handoff gaps, or confusion may occur. The other options do not describe the care process and would not help analyze dressing-change delays.

478. A facility starts a new skin assessment form on admission to reduce missed pressure injuries. One month later, completion of the form is high, but the number of pressure injuries identified after 48 hours has not changed. Which conclusion is most appropriate?

- A. The project failed because the form completion rate is irrelevant
- B. The team should stop the project immediately because documentation improved first
- C. The new form improved compliance with the assessment process, but the impact on outcomes is not yet demonstrated
- D. The unchanged pressure injury rate proves staff are filling out the form dishonestly

Correct Answer: C

High completion shows improvement in the process measure, but unchanged outcome data mean the team cannot yet conclude that client outcomes improved. Quality improvement often requires examining both process and outcome measures over time. Choices A, B, and D draw unsupported conclusions.

479. A practical nurse suggests posting each nurse's monthly medication error count on a public staff bulletin board to reduce errors. Which response by the nurse manager best reflects sound quality improvement practice?

- A. Public comparison is best because fear of embarrassment usually improves performance

- B. The plan should focus instead on examining system contributors to errors while maintaining a just culture

- C. Medication errors should not be measured because reporting them increases unit tension

- D. Only providers, not nursing staff, influence medication safety outcomes

Correct Answer: B

Effective quality improvement promotes safety by identifying system contributors and encouraging reporting without a punitive atmosphere. A just culture supports learning and improvement. Public shaming can discourage reporting and does not address root causes. Choices C and D are also inconsistent with safe quality practice.

480. A clinic tracks the percentage of clients who receive written instructions after a new diagnosis of diabetes. The rate rises from 40% to 95%, but many clients still return unable to explain glucose monitoring. What is the best next step for the quality improvement team?

- A. Conclude the project is complete because the documentation target was met
- B. Measure client understanding and revise the teaching process, such as adding return demonstration or teach-back
- C. Stop giving written instructions because they clearly do not matter
- D. Focus only on whether staff sign the education form on every visit

Correct Answer: B

The process measure improved, but the desired learning outcome did not. The next step is to evaluate client understanding directly and improve the teaching method. Choice A confuses documentation with effective teaching, and choices C and D are either too extreme or too narrow.

481. A practical nurse is reviewing monthly data on client falls after a new rounding protocol began. Which pattern most strongly suggests special-cause variation rather than normal month-to-month fluctuation?

- A. The fall rate changes slightly up and down around the previous average
- B. The fall rate remains exactly the same for two consecutive months

- C.** The fall rate shows one sudden, unusually large spike after a period of relative stability
- D.** The number of occupied beds is lower during one holiday week

Correct Answer: C

A sudden large spike after stable performance suggests a nonrandom event or special cause that should be investigated. Small up-and-down changes around the average are more consistent with common-cause variation. Option D may be relevant context, but by itself it does not show variation in the fall data.

482. A unit wants to decrease delays in obtaining STAT laboratory specimens. The current average time from order entry to specimen collection is 50 minutes. After adding a dedicated phlebotomy pager, the average falls to 35 minutes, but the range between cases remains very wide. Which interpretation is best?

- A.** The intervention improved average performance, but inconsistency remains and the process still needs further analysis
- B.** The project is complete because any drop in the average proves the process is reliable
- C.** The wide range means the lower average is meaningless and should be ignored
- D.** The dedicated pager should be removed because variation is always worse than a high average

Correct Answer: A

A lower average suggests improvement, but wide variation means performance is still inconsistent. In quality improvement, both central tendency and reliability matter. Choice B overstates the result, while C and D dismiss useful information instead of interpreting it appropriately.

483. A practical nurse on a dialysis unit notices repeated delays in administering a time-sensitive antibiotic because the medication is not available when clients return from treatment. Which proposed measure would best balance improvement in timely administration with the risk of an unintended consequence?

- A.** Track only how often the antibiotic is given within the scheduled window

B. Track antibiotic timeliness and also monitor the number of doses prepared but wasted when clients do not return as expected

C. Track only the total number of dialysis clients per day

D. Track how many staff say the new medication process feels faster

Correct Answer: B

A balancing measure looks for unwanted effects of a change. If medications are prepared earlier to improve timeliness, waste may increase when schedules change. Monitoring both timely administration and wasted doses gives a fuller picture. Choices A, C, and D do not assess unintended consequences adequately.

484. A quality improvement team introduces hourly pain rounds on one surgical unit. Reported pain scores decrease during the trial, but at the same time a new postoperative order set, additional float staff, and lower census are also present. Which conclusion is most appropriate?

- A.** Hourly pain rounds definitely caused the lower pain scores
- B.** The lower pain scores mean the other changes had no effect
- C.** The team should recognize that multiple simultaneous changes limit the ability to attribute the improvement to hourly rounds alone
- D.** The data should be discarded because quality improvement cannot be done when more than one change occurs

Correct Answer: C

When several changes occur at the same time, it becomes difficult to isolate the effect of one intervention. The improvement may be real, but attribution should be cautious. Choice A is too certain, B ignores confounding factors, and D is unnecessarily absolute.

485. A practical nurse on a rehabilitation unit notices that several clients miss scheduled physical therapy because transport does not arrive on time. The team wants to improve this process. Which aim statement is best for a quality improvement project?

- A.** Clients should get to therapy faster whenever possible.

B. Staff will try harder to reduce delays in client transport.

C. Increase the percentage of clients arriving to physical therapy within 10 minutes of the scheduled time from 60% to 90% in 8 weeks.

D. Transport staff need more education about being on time.

Correct Answer: C

A strong quality improvement aim is specific, measurable, and time-limited. Choice C names the process, gives a baseline and target, and sets a time frame. Choices A and B are too vague to guide measurement, and D jumps to a presumed solution before clearly defining the goal.

486. A long-term care team is reviewing frequent late administration of bedtime medications. Which question best helps the team distinguish a process problem from individual blame?

A. Which nurse had the most late medication passes last month?

B. What steps occur between medication preparation and administration, and where do delays usually happen?

C. Who should be written up for not finishing on time?

D. Which staff member has the least experience on the unit?

Correct Answer: B

Quality improvement focuses on understanding the process and locating where breakdowns occur. Choice B examines the workflow and supports system improvement. The other options focus mainly on identifying or blaming individuals rather than studying the process that produces the problem.

487. A clinic tests reminder phone calls to reduce missed follow-up appointments for clients with heart failure. Which result is the best example of an outcome measure?

A. Number of staff trained to make reminder calls

B. Percentage of scheduled clients who received a reminder call

C. Percentage of clients who attended their follow-up visit within 7 days of discharge

D. Average time staff spent making each phone call

Correct Answer: C

An outcome measure reflects the result of care or the effect of the change on clients. Attendance at follow-up visits is the desired result, so C is the outcome measure. B is a process measure because it tracks whether the intervention was done. A and D do not measure the client-level result.

488. A practical nurse on a medical unit is helping with a quality improvement project to reduce specimen-labeling errors. The team plans to trial a new bedside label-check routine with one nurse and two clients during one shift, review the results, and then decide whether to revise the change. Which quality improvement approach does this describe?

A. A small test of change before wider implementation

B. A root cause analysis after a sentinel event

C. A disciplinary review of unsafe staff behavior

D. A retrospective chart audit for annual reporting

Correct Answer: A

This describes testing a change on a very small scale, learning from the results, and then adjusting before broader rollout. That is a core quality improvement approach. It is not root cause analysis, which is typically used after a serious event, and it is not primarily a disciplinary or annual reporting activity.

489. A skilled nursing facility wants to reduce resident falls during toileting. Before changing practice, the team collects two weeks of data on when falls occur, where they occur, and whether staff were assisting. What is the main purpose of collecting this baseline data?

A. To determine whether the staff involved should be disciplined

B. To compare future results with current performance

C. To prove that no changes are needed

D. To replace the need for any further measurement

Correct Answer: B

Baseline data establish current performance so the team can judge whether a later change leads to improvement. Without a baseline, it is hard to know if results are actually better. Choices A and C misuse data, and D is incorrect because ongoing measurement is still needed after the baseline is set.

490. A practical nurse says, "Call light times improved after we moved the supply cart, so the cart change must have caused the improvement." Which additional information would most weaken that conclusion?

- A.** The team measured response times every day for 1 week before and 1 week after the change.
- B.** During the same week, the unit also added one extra assistive staff member on every shift.
- C.** Several staff members reported that the cart was easier to access.
- D.** Clients on the unit had a lower average acuity that month than the previous month.

Correct Answer: B

If another major change happened at the same time, it becomes difficult to attribute improvement to the supply cart alone. Adding staff directly affects call light response and is the strongest competing explanation. Choice D could also influence results, but B is the more direct confounder. A strengthens measurement quality rather than weakening the conclusion, and C is only staff opinion.

491. A unit is trying to improve completion of admission pain assessments. The charge nurse wants the first step after noticing low completion rates. Which action is most appropriate?

- A.** Immediately require all nurses to attend remediation
- B.** Replace the electronic record template with a new one that same day
- C.** Clarify the current performance gap by reviewing when and where assessments are being missed
- D.** Stop tracking the measure until staff morale improves

Correct Answer: C

Before selecting a solution, the team should define the problem clearly and understand the pattern of misses. Choice C supports that step. A and B move to corrective action too quickly without understanding why the gap exists. D prevents improvement because the problem is no longer being measured.

492. A practical nurse is helping review a new bedside handoff checklist. Which finding best reflects a balancing measure?

- A.** Percentage of handoffs in which the checklist was used
- B.** Percentage of clients who could correctly name their nurse after shift change
- C.** Average length of shift-to-shift report after the checklist was introduced
- D.** Number of staff who signed the checklist education roster

Correct Answer: C

A balancing measure looks for an unintended consequence of the change. If a bedside handoff checklist improves communication but makes report much longer, that matters. Choice C checks for that tradeoff. A is a process measure, B is more of an outcome or effect measure, and D only tracks education attendance.

493. A clinic tests a new workflow for same-day prescription refills. After the first 2 days, staff report the process feels smoother, but no refill turnaround times have been recorded. What is the best interpretation?

- A.** The project is successful because staff believe the new process works
- B.** The change should be spread to all clinics immediately
- C.** The team needs objective data before deciding whether the change improved performance
- D.** The project should be stopped because anecdotal feedback has no value

Correct Answer: C

Staff impressions can be useful, but quality improvement decisions should be supported by objective measurement. Choice C is best because the team still needs data on refill turnaround times. A and B rely too heavily on opinion, while D is too extreme because staff feedback can still help refine a test.

494. A home health agency wants to reduce missed visits caused by scheduling problems. Which change would best support a small, rapid quality improvement test?

- A.** Rewrite the agency's entire scheduling policy before collecting any data
- B.** Try a new confirmation text message process with one nurse's clients for 3 days and compare missed visits with the prior week
- C.** Train every employee in the agency on a new software system at once
- D.** Wait 6 months, then review whether missed visits seem lower

Correct Answer: B

A small, rapid test uses a limited scale, occurs over a short time, and includes comparison data. Choice B matches that approach. A and C are large-scale changes without initial testing, and D delays learning instead of supporting quick improvement cycles.

495. A practical nurse reviews data from a project intended to improve urine specimen collection labeling. Week 1: 6 errors, Week 2: 5 errors, Week 3: 6 errors, Week 4: 5 errors. A new labeling script is introduced. Weeks 5 through 10: 1, 0, 1, 0, 1, 0 errors. Which interpretation is most appropriate?

- A.** The lower error counts after week 4 suggest a meaningful shift associated with the new process
- B.** The pattern proves the change will remain effective permanently
- C.** The data show no important difference because some errors still occurred
- D.** The project failed because the error count did not become exactly zero every week

Correct Answer: A

The pattern changes clearly after the intervention, with sustained lower counts rather than random up-and-down fluctuation around the previous level. That suggests a meaningful improvement associated with the new process. B is too certain because ongoing monitoring is still needed. C and D ignore the strong improvement just because occasional errors remain.

496. A unit's pressure-injury prevention team finds that turning documentation is often incomplete on night shift. One staff member says, "Let's just remind everyone to chart better." Which response best reflects stronger quality improvement reasoning?

- A.** That is enough because documentation problems are usually caused by carelessness
- B.** The team should first examine whether workflow, staffing, prompts, or the record design contribute to missed documentation
- C.** Night-shift staff should receive formal warnings before more data are reviewed
- D.** The best solution is to assign all turning documentation to one nurse each night

Correct Answer: B

Quality improvement looks beyond simple reminders and asks what in the system makes the desired action hard to do consistently. Workflow barriers, staffing patterns, missing prompts, or poorly designed documentation tools may all contribute. A and C overemphasize individual blame. D may create new safety and workload problems without addressing root system issues.

497. A clinic wants to reduce delays in starting morning appointments. Data show that the first client of the day is roomed on time only 45% of the time. The manager proposes changing check-in, room assignment, staffing arrival times, and supply restocking all at once next Monday. What is the main quality improvement concern with this plan?

- A.** The changes are too likely to improve client satisfaction
- B.** Making several changes at once makes it hard to know which change affected the result
- C.** Morning delays should be handled only by administration, not clinical staff
- D.** The team should avoid collecting any more baseline information

Correct Answer: B

When multiple changes are introduced at the same time, the team cannot easily tell which change helped, which did not, or whether one created problems. Testing changes in a more structured way supports clearer learning. A is not a concern, C is incorrect, and D goes against sound QI practice.

498. A practical nurse participates in a project to reduce delays in obtaining prescribed pain medication from the automated dispensing system. The team measures the time from client request to medication administration. After a change, the median time improves, but nurses now interrupt one another more often while accessing the machine. Which next action is best?

- A.** Ignore the interruptions because the main measure improved
- B.** Stop the project permanently because every change creates some inconvenience
- C.** Study the increased interruptions as a possible unintended consequence and adjust the process if needed
- D.** Assume the interruptions are unrelated because the original goal involved medication timing

Correct Answer: C

Quality improvement includes looking for unintended consequences. Even if the main measure improves, the team should evaluate whether the new interruptions create safety risks or workflow problems and then refine the process. A and D dismiss important balancing information, and B is an overreaction.

499. A practical nurse on a clinic improvement team says, "We should choose a measure that staff can control directly while still relating clearly to the problem." The project goal is to improve completion of depression screening at annual visits. Which measure best fits that statement?

- A.** Percentage of annual-visit clients with a completed depression screening form before the visit ends
- B.** Percentage of clients whose depression symptoms improve within 6 months

C. Number of community mental health resources available in the county

D. Statewide rate of hospitalizations related to mental health crises

Correct Answer: A

The team can directly influence whether the screening is completed during the visit, and that measure closely matches the stated problem. B is important clinically but is less directly controlled by the clinic's screening workflow and is influenced by many factors. C and D are too far removed from the team's process.

500. A practical nurse joins a quality improvement team that wants to reduce delays in discharge teaching. Which statement is the best example of a clear process measure for this project?

- A.** The percentage of clients who receive discharge teaching before the discharge order is written
- B.** The percentage of clients who can correctly repeat all home-care instructions 48 hours after discharge
- C.** The percentage of clients whose discharge teaching is completed before they leave the unit
- D.** The number of clients readmitted within 30 days for any cause

Correct Answer: C

A process measure tracks whether the care step itself happened as intended. Completing discharge teaching before the client leaves the unit directly measures the process being improved. Choice B is an outcome measure because it reflects what the client retained. Choice D is also an outcome measure and is affected by many factors beyond teaching. Choice A uses a timing point that may not fit the actual workflow and does not best capture completion before discharge.

501. A unit is starting a quality improvement project after noticing a rise in client falls. Which action by the practical nurse best supports a systems-focused approach?

- A.** Identify the staff member involved in the most recent fall and recommend discipline first
- B.** Review when and where falls occur and whether common conditions are present

- C. Wait to collect information until another fall happens so the details are more current
- D. Ask management to replace all fall-prevention equipment before examining the pattern

Correct Answer: B

Quality improvement focuses on understanding the process and the system, not starting with blame. Reviewing time, location, and contributing conditions helps identify patterns that can guide change. Choice A focuses on individual blame too early. Choice C delays needed assessment. Choice D jumps to a solution before the problem is understood.

502. A clinic wants to improve completion of vaccine screening at check-in. The team plans a small first test of change. Which plan is most appropriate?

- A. Train the entire clinic staff and launch the new process for all clients for one month
- B. Ask one staff member to test the new screening form with the first three clients of the morning
- C. Change the electronic record, staffing pattern, and client flow on the same day
- D. Wait until a perfect clinic-wide policy is written before trying any changes

Correct Answer: B

A small first test reduces risk and helps the team learn quickly. Testing with one staff member and a few clients is a practical rapid-cycle approach. Choice A is too large for an initial test. Choice C changes too many variables at once. Choice D delays learning and goes against iterative improvement.

503. During a quality improvement meeting, the manager says, "Our goal is zero medication-administration delays by next week." Which response by the practical nurse best reflects sound quality improvement reasoning?

- A. That goal is ideal because quality improvement works best with broad, aspirational language only
- B. That goal should be postponed until the team can identify which staff members are responsible

C. That goal may not be useful unless it is made more specific, measurable, and realistic for testing

D. That goal is strong because medication delays are always caused by client noncompliance

Correct Answer: C

Improvement goals work best when they are specific, measurable, and realistic enough to guide action and evaluation. "Zero by next week" may be too vague or unrealistic for an initial project. Choice A ignores the need for measurable targets. Choice B returns to blame rather than improvement. Choice D incorrectly assumes one cause.

504. A quality improvement team tests a new admission checklist to improve allergy documentation. After one week, the documentation rate improves from 68% to 95%, but admission interviews now take longer. How should the increased interview time be classified?

- A. A balancing measure
- B. A structure measure
- C. A sentinel event
- D. A root cause

Correct Answer: A

A balancing measure checks whether improvement in one area creates a new problem elsewhere. Longer admission interviews may be an unintended tradeoff of the new checklist. A structure measure refers to the care setting or resources. A sentinel event is a serious unexpected event, and a root cause is an underlying contributor, not the type of measure shown here.

505. A practical nurse is reviewing data from a project to improve completion of daily weights for clients with heart failure. The completion rate by week is 62%, 64%, 63%, 65%, 64%, and 63%. After a bedside reminder card is introduced, the next six weeks are 64%, 63%, 65%, 64%, 63%, and 64%. Which interpretation is most appropriate?

- A. The reminder card clearly improved performance because the average after the change is slightly higher
- B. The reminder card likely had little effect because the pattern stayed essentially the same

- C. The reminder card caused harm because the rate dropped at least once after the change
- D. The data prove staff stopped trying once the reminder card was introduced

Correct Answer: B

The values before and after the change remain in about the same narrow range, so there is no convincing sign of meaningful improvement. Choice A overstates a trivial difference. Choice C mistakes normal week-to-week variation for harm. Choice D makes an unsupported conclusion about staff motivation.

506. A nursing unit wants to reduce incomplete postoperative vital-sign documentation. The team asks why the documentation is missed and learns that the flowsheet is stored in a different tab than the medication record, and staff often do not see it during busy periods. This finding most strongly suggests which type of improvement issue?

- A. A systems and workflow problem
- B. A need to lower documentation standards
- C. A client teaching deficit
- D. A confidentiality breach

Correct Answer: A

If the documentation tool is hard to find within the workflow, the issue is most likely related to system design and process flow. Quality improvement looks for barriers in the work system. Choice B is inappropriate because standards should not be lowered to match a flawed process. Choices C and D do not address the problem described.

507. A practical nurse suggests using client satisfaction scores as the only measure for a project intended to reduce delays in obtaining prescribed wound-care supplies. Which response by the team leader is best?

- A. That is enough because satisfaction always shows whether a process improved
- B. That measure is useful, but the team also needs a direct measure of supply delay
- C. Satisfaction scores should replace all staff-reported data because they are more objective
- D. The team should avoid measuring delay because staff might feel criticized

Correct Answer: B

Client satisfaction may add useful information, but it does not directly measure whether supply delays improved. A direct process or time-based measure is needed to assess the target problem. Choice A overstates the value of satisfaction alone. Choice C incorrectly treats satisfaction as more objective than all other data. Choice D avoids measurement instead of improving care.

508. A long-term care facility is trying to reduce omitted skin checks on admission. One nurse says, "We just need to tell everyone to be more careful." Which reply by the practical nurse best supports effective quality improvement?

- A. That should solve it because most quality problems come from staff not paying attention
- B. Before deciding on a fix, we should learn when the checks are being missed and what in the process contributes
- C. We should stop documenting skin checks until the staff become more consistent
- D. We should assume the problem is unique to the newest employees

Correct Answer: B

Effective quality improvement begins by understanding the process and its contributing factors. Saying "be more careful" is a weak intervention if the true problem is workflow, timing, unclear responsibility, or documentation design. Choices A and D rely on assumptions about individuals. Choice C would reduce safety and documentation quality.

509. A clinic tests a new checkout script to improve scheduling of follow-up visits. On Monday, one nurse uses the script with five clients and reports that it was confusing. What is the best next step in the quality improvement cycle?

- A. Adopt the script across the clinic immediately because it was tested in practice
- B. Abandon the project because the first test did not go smoothly
- C. Revise the script based on what was learned and test it again on a small scale
- D. Discipline the nurse for not using the script successfully

Correct Answer: C

Small tests are designed to produce learning. If the first trial shows confusion, the team should refine the change and retest rather than scale it up, abandon it immediately, or blame staff. Choice A moves too quickly. Choice B ignores the value of iterative learning. Choice D conflicts with improvement principles.

510. A practical nurse on a pediatric unit is helping with a quality improvement effort to reduce missing signatures on consent forms before procedures. Which measure is the best outcome measure for this project?

- A. The percentage of charts audited each week
- B. The number of staff taught the consent policy
- C. The percentage of procedures delayed because consent was incomplete
- D. The time the charge nurse spends sending reminder messages

Correct Answer: C

An outcome measure reflects the result of the process on care delivery. Procedures delayed because consent was incomplete show the effect of the problem on actual care. Choice A is an audit activity, not the outcome. Choice B is a training measure. Choice D reflects staff effort, not the project result.

511. A unit introduces bedside barcode scanning to reduce specimen-identification errors. Error rates fall, but nurses now report frequent delays because scanners are difficult to locate at the start of each shift. Which action best reflects strong quality improvement thinking?

- A. Ignore the delays because the original error rate improved
- B. Treat the delays as important feedback and examine equipment placement and access
- C. Remove barcode scanning immediately because every change that adds time is a failure
- D. Conclude the delays are due to poor time management by individual nurses

Correct Answer: B

Quality improvement includes watching for unintended consequences and adjusting the system accordingly. If scanners are hard to find, equipment placement and access are likely workflow issues worth addressing. Choice A ignores a balancing concern. Choice C overreacts without evaluating possible fixes. Choice D jumps to blame without evidence.

512. A rehabilitation unit wants to improve completion of mobility-safety checks before transfers. The manager proposes a new checklist, extra staff education, a revised room layout, and a new documentation prompt, all beginning the same day. What is the greatest quality improvement concern?

- A. The team may not be able to tell which change, if any, actually affected results
- B. The plan is too focused on client safety rather than staff convenience
- C. The project uses more than one measure, which weakens improvement work
- D. The plan should avoid changing the environment because only staff behavior matters

Correct Answer: A

When many interventions are launched at once, it becomes difficult to know what caused any improvement or worsening. Strong quality improvement often tests changes in a more controlled way so learning is clearer. Choice B is not the central issue. Choice C is incorrect because multiple measures can be helpful. Choice D wrongly dismisses environmental factors.

513. A practical nurse reviews monthly data from a project to reduce omitted evening blood glucose checks. For eight months before the change, omissions ranged from 7 to 10 per month. After a new reminder step was added, omissions for the next seven months were 2, 1, 2, 1, 0, 2, and 1. Which conclusion is most justified?

- A. The change may be associated with meaningful improvement because the pattern shifted lower and stayed lower
- B. The result is probably random because some variation remains after the change

- C. The change failed because omissions did not immediately reach zero every month
- D. The data are uninterpretable unless staff satisfaction was measured at the same time

Correct Answer: A

The post-change data show a clear and sustained shift to a much lower range than the baseline pattern, which supports the conclusion that the change may be associated with improvement. Choice B ignores the importance of the sustained shift. Choice C uses an unrealistic all-or-nothing standard. Choice D adds a useful possible measure but not a requirement for interpreting the clear pattern shown.

514. A home health agency is trying to reduce missed wound-care visits. The team learns that visit times are often entered in one software system, while supply delivery information is tracked in another system that field staff cannot easily view. Which intervention is most likely to address the underlying quality problem?

- A. Remind nurses weekly that missed visits are unacceptable
- B. Require each nurse to call the office before every visit regardless of supply status
- C. Improve coordination by making scheduling and supply information easier to access in the workflow
- D. Ask clients to keep calling until a nurse arrives

Correct Answer: C

The agency identified a coordination and information-flow problem within the system. Improving access to scheduling and supply information targets the likely root workflow barrier. Choice A is a weak reminder-based response. Choice B adds burden without fixing the fragmented information process. Choice D shifts responsibility to clients rather than improving the system.

515. A practical nurse is helping with a quality improvement project on a medical unit after several clients waited a long time for prescribed pain medication. Which project aim is written most appropriately?

- A. Improve pain medication administration as much as possible soon
- B. Decrease the average time from client request to pain medication administration on the medical unit from 35 minutes to 20 minutes within 8 weeks
- C. Make sure every nurse gives pain medication faster starting today
- D. Reduce all medication errors and client complaints on every unit this month

Correct Answer: B

B is best because a strong quality improvement aim is specific, measurable, focused, and time limited. It names the process being improved, the current and desired performance, the setting, and the timeframe. A is too vague, C is not measurable and focuses on individuals rather than a clear aim, and D is overly broad because it combines multiple problems across all units.

516. A clinic is trying to improve interpreter use for clients with limited English proficiency. Baseline chart review shows that language preference is documented in 92% of visits, but qualified interpreter use for eligible clients occurs in only 41% of visits. Which change should the team test first to address the most likely gap in care?

- A. Post a general reminder about cultural sensitivity in the staff break room
- B. Add a step at check-in that flags clients needing interpreter services and alerts the nurse before the visit begins
- C. Ask nurses to speak more slowly when clients seem confused
- D. Start measuring client satisfaction only at the end of each month

Correct Answer: B

B is best because the data suggest the clinic usually identifies language preference but does not reliably act on that information. A check-in flag and alert target the workflow gap between documentation and interpreter use. A is too nonspecific, C is not an appropriate substitute for qualified interpreter services, and D adds measurement without testing a change likely to improve the process.

517. A quality improvement team wants to reduce missed turning and repositioning for bedbound clients. For the first 2 weeks, staff use a paper turning log on one hallway during day shift only. Several nurses report the log is hard to find, and entries are often completed late from memory. What is the best next step?

- A.** Stop the project because the first test did not immediately succeed
- B.** Expand the paper log to all hallways so more data can be collected
- C.** Adapt the change by placing the log at each bedside or embedding the reminder into the usual workflow, then test again on a small scale
- D.** Require written counseling for staff who forgot to document repositioning

Correct Answer: C

C is best because small tests of change are used to learn from barriers and refine the process. The problem appears to be poor fit with workflow, so the team should adapt the change and retest on a limited scale. A ignores useful learning, B spreads a flawed process before improving it, and D treats the issue as individual fault instead of addressing the system problem identified during the test.

518. A long-term care facility is reviewing a quality improvement project to reduce omitted morning doses of a Parkinson medication. For 10 weeks before any change, omissions per week were 4, 5, 4, 6, 5, 4, 5, 6, 4, and 5. After the team introduced a medication-cart cue and a handoff reminder, the next 8 weeks were 5, 0, 1, 0, 1, 0, 1, and 0. Which interpretation is most appropriate?

- A.** The improvement is probably due to normal week-to-week variation, so no conclusion can be made

- B.** The data suggest a meaningful improvement after the change, although the team should continue monitoring to confirm the new level is sustained

- C.** The project failed because one post-change week still had 5 omissions

- D.** The facility should assume the problem is solved permanently and stop collecting data now

Correct Answer: B

B is best because the post-change pattern shows a clear shift to a much lower level after the intervention, which is more consistent with real improvement than with ordinary fluctuation. However, quality improvement requires ongoing monitoring to make sure the gain continues. A ignores the strong pattern change, C overreacts to the first post-change point without considering the full trend, and D is unsafe because successful improvements still need follow-up.

Interdisciplinary information sharing

519. A practical nurse is preparing to give end-of-shift report on four clients. Which information is most important to communicate first to the oncoming nurse?

A client with osteoarthritis who requested a warm blanket 30 minutes ago

A client with pneumonia whose oxygen saturation dropped from 95% to 89% during ambulation

A client with diabetes who ate 75% of lunch and has a bedtime snack ordered

A client with a healing pressure injury who needs dressing change supplies restocked

- A.** A client with osteoarthritis who requested a warm blanket 30 minutes ago

- B.** A client with pneumonia whose oxygen saturation dropped from 95% to 89% during ambulation

- C.** A client with diabetes who ate 75% of lunch and has a bedtime snack ordered

- D.** A client with a healing pressure injury who needs dressing change supplies restocked

Correct Answer: B

A drop in oxygen saturation with activity suggests a change in respiratory status and requires prompt follow-up, so it is the highest-priority information to share. The other items are routine comfort, nutrition, or supply issues that can be addressed after urgent clinical changes. A common trap is choosing something left unfinished, but unresolved tasks are not always the highest priority.

520. A practical nurse is calling the health care provider about a client who has newly developed confusion and a temperature of 38.9°C (102°F). Which statement best demonstrates organized interdisciplinary communication?

- A. "This client doesn't look right and needs to be seen soon."
- B. "I think the antibiotics are not working. What do you want to do?"
- C. "I'm calling about Mr. Diaz in room 214, who became confused over the last hour and now has a temperature of 38.9°C. His blood pressure is 98/56 mm Hg, pulse is 112/min, and urine output has decreased since noon."
- D. "Can you come assess this client when you have time?"

Correct Answer: C

This response clearly identifies the client, the urgent concern, and relevant assessment data, which supports safe interdisciplinary communication. The other options are vague, incomplete, or based on unsupported conclusions. The most common error is reporting a feeling or opinion instead of concise, objective findings.

521. During interdisciplinary rounds, which statement by the practical nurse is most appropriate for advocating for a client's discharge needs?

- A. "The client is pleasant and should do fine at home."
- B. "The client lives alone, uses a walker, and says there is no one available to help with daily wound care."
- C. "The client wants to leave today, so discharge should happen as planned."
- D. "Physical therapy can decide whether the client needs anything."

Correct Answer: B

Effective interdisciplinary sharing includes specific functional and social information that affects safe discharge planning. This option gives concrete data the team can act on. The other responses are vague, assumptive, or shift responsibility instead of contributing useful information.

522. A practical nurse receives a postoperative client from the PACU. Which handoff information should the nurse clarify immediately?

- A. The client received an antiemetic in recovery
- B. The estimated blood loss was documented as "minimal"
- C. The report states "pain controlled" but gives no pain rating, medication, or time last medicated
- D. The dressing is dry and intact

Correct Answer: C

To continue safe postoperative care, the receiving nurse needs specific pain details, including the client's pain level, what medication was given, and when it was administered. "Pain controlled" is too vague to guide ongoing assessment and medication timing. The other data may be less detailed than ideal but are not as immediately unsafe.

523. The practical nurse is documenting a phone report to the provider after reporting a client's worsening shortness of breath. Which documentation is best?

- A. "Doctor notified. Awaiting orders."
- B. "Provider made aware that client seems worse."
- C. "Reported increased shortness of breath, respiratory rate 28/min, crackles at bases, and oxygen saturation 90% on 2 L/min by nasal cannula to provider at 1435; received order to increase oxygen to 3 L/min and obtain chest x-ray."
- D. "Called provider about breathing issue; see orders."

Correct Answer: C

Good documentation of interdisciplinary communication includes the reason for contact, objective findings, time, and any resulting orders or actions. The other options are too vague and do not create a clear clinical record. A common mistake is documenting that a provider was notified without recording what information was actually communicated.

524. A practical nurse is caring for a client who speaks limited English. The nurse needs to explain a newly ordered urine specimen collection. Which action best supports accurate interdisciplinary communication?

- A.** Ask the client's teenage daughter to interpret because she is already at the bedside
- B.** Use simple English and gestures, then proceed if the client nods
- C.** Request a qualified medical interpreter before teaching the client about the procedure
- D.** Wait until the provider returns to the unit to explain the procedure

Correct Answer: C

A qualified interpreter helps ensure accurate, safe communication and reduces the risk of misunderstanding. Family members, especially minors, should not be relied on for medical interpretation when an interpreter is needed. The trap is choosing a convenient option that may seem practical but does not support clear, reliable communication.

525. The practical nurse notices that a client's medication administration record lists a penicillin allergy, but a newly entered antibiotic order is for ampicillin. What is the nurse's priority action related to interdisciplinary information sharing?

- A.** Administer the first dose and observe closely for a reaction
- B.** Hold the medication and promptly notify the appropriate licensed team member or provider about the discrepancy
- C.** Ask the client whether the allergy was probably mild before deciding what to do
- D.** Remove the allergy from the record if the client cannot remember the reaction

Correct Answer: B

The nurse should not give a medication when there is a possible allergy conflict. Promptly communicating the discrepancy helps prevent harm and allows the order to be clarified. Asking for more history may be useful later, but it does not replace the need to hold the drug and escalate the concern.

526. At change of shift, the practical nurse reports that a client with heart failure gained 2.2 kg (4.8 lb) in 2 days, has ankle edema, and is more short of breath when walking to the bathroom. Which additional information would make this handoff most complete?

- A.** The client's room number and favorite beverage
- B.** The client's ejection fraction from 5 years ago
- C.** The client's current intake and output trend and most recent lung assessment findings
- D.** The name of the client's primary family contact

Correct Answer: C

Current intake and output and recent lung findings directly relate to worsening fluid overload and help the next nurse assess urgency and response to treatment. The other information may be useful in some contexts but does not best complete this focused handoff. The key is choosing data that are both current and directly tied to the change in condition.

527. A practical nurse overhears a nursing assistant discussing a client's diagnosis in the cafeteria. What is the nurse's best action?

- A.** Ignore the conversation because the assistant did not mention the client's room number
- B.** Join the conversation to correct inaccurate details
- C.** Tell the assistant privately that discussing client information in a public area is inappropriate and report the breach according to facility policy
- D.** Wait to see whether the client complains before taking action

Correct Answer: C

The nurse should address the confidentiality breach promptly and follow policy for reporting it. Public discussion of client information is inappropriate even if some identifiers are omitted. A common trap is underestimating a privacy breach because not every identifying detail was spoken aloud.

528. A client is being transferred from a medical unit to a rehabilitation facility. Which information is most essential for the practical nurse to include in the transfer report to promote continuity of care?

- A.** The names of staff members who cared for the client during this admission
- B.** The client's current functional status, mobility limitations, recent vital sign trends, medications, and ongoing care needs
- C.** The client's opinion about hospital food and sleep quality
- D.** A detailed summary of every normal laboratory value since admission

Correct Answer: B

Transfer communication should focus on the current status and information needed for safe ongoing care, especially function, treatment needs, and recent clinical trends. The other choices contain either nonessential details or excessive information that may obscure priorities. The best handoff is both complete and clinically focused.

529. The practical nurse is preparing to notify the provider about a client's blood glucose of 46 mg/dL after treatment with juice and glucose gel. The repeat glucose is 52 mg/dL, and the client remains diaphoretic. Which additional information is most important to have ready before making the call?

- A.** The client's preferred pharmacy
- B.** The date of the client's last influenza vaccine
- C.** The client's current symptoms, most recent insulin or diabetes medication administration, and ability to swallow safely
- D.** The total number of visitors currently at the bedside

Correct Answer: C

Before contacting the provider, the nurse should gather information that directly affects immediate management of persistent hypoglycemia: ongoing symptoms, what diabetes medication was given, and whether the client can safely take more oral treatment. The other details do not support urgent decision-making. The trap is collecting background information that is true but not actionable in the moment.

530. A practical nurse receives verbal instructions from a provider during a noisy emergency situation. The nurse is unsure whether the provider said morphine 2 mg IV or 4 mg IV. What is the best action?

- A.** Administer 2 mg because using the lower dose is safer than delaying care
- B.** Ask another nurse what dose is usually given and use that amount
- C.** Immediately clarify the order with the provider by repeating back the medication and dose before administration
- D.** Document the order as heard and wait to see whether the client reports pain relief

Correct Answer: C

If a verbal order is unclear, the nurse must clarify it directly with the provider and use read-back to verify accuracy before giving the medication. Guessing, even with a lower dose, is unsafe because either underdosing or overdosing can harm the client. This question tests safety in communication, not speed alone.

531. A client with a new colostomy is scheduled for discharge tomorrow. During rounds, the practical nurse learns that the client avoids looking at the stoma, the spouse says, "I'll do all of it at home," and the client has not yet emptied the pouch independently. Which communication by the nurse is best?

- A.** "Discharge teaching is complete because the spouse is willing to help."
- B.** "The client may need reinforcement, but the provider can address that later."
- C.** "The client has not demonstrated self-care for the colostomy and appears reluctant to engage; the team should reassess readiness, teaching needs, and home support before discharge."

D. "Since the surgery was uncomplicated, discharge should proceed as planned."

Correct Answer: C

This statement accurately shares behavioral cues, skill deficits, and the need for team reassessment before discharge. It supports coordinated planning rather than assuming a willing spouse solves the problem. The trap is focusing only on medical stability while missing functional readiness and teaching needs.

532. A practical nurse is contributing to an interdisciplinary care conference for a client with recurrent falls. Which report is most likely to improve the team's plan of care?

A. "The client fell again sometime overnight."

A. "The client fell again sometime overnight."

B. "The client is just noncompliant with instructions."

C. "The client had two falls when attempting to toilet without help between 0500 and 0700, reports urgency on awakening, and leaves the walker out of reach when getting up."

D. "Falls are common in older clients, so close observation is needed."

Correct Answer: C

This option provides specific, actionable pattern information: timing, activity, symptoms, and environmental factors. That helps the team target interventions such as scheduled toileting, walker placement, and morning assistance. The other choices are vague, judgmental, or too general to support a strong interdisciplinary plan.

533. A practical nurse is updating the care team about a client who has become increasingly withdrawn. Which observation is most important to communicate promptly?

A. The client prefers to keep the room dark and declines group activities

B. The client says, "Everyone would be better off without me," and has stopped eating meals

C. The client has asked for extra time before bathing in the morning

D. The client wants family to visit later in the day instead of early morning

Correct Answer: B

A statement suggesting hopelessness or self-harm risk requires prompt communication to the team because it may indicate an immediate safety concern. Social withdrawal alone is concerning, but direct verbal cues of despair elevate urgency. The key is recognizing when information is not just relevant, but emergent.

534. A practical nurse is giving a handoff report on a client admitted with dehydration and vomiting. Which statement best reflects effective interdisciplinary information sharing?

A. The client had a pretty rough night and still seems weak.

B. The client is dehydrated and probably needs more attention today.

C. The client has vomited 4 times since 0200, has dry mucous membranes, urine output of 120 mL over 8 hours, and blood pressure 92/54 mm Hg.

D. The client is not doing as well as expected, so please keep an eye on things.

Correct Answer: C

Choice C is best because it gives specific, relevant clinical data another team member can act on: frequency of vomiting, assessment findings, urine output, and blood pressure. The other options are vague and rely on subjective impressions rather than clear information.

535. A practical nurse is preparing information for interdisciplinary rounds about a client recovering from a stroke. Which detail is most important to share with the team when planning care for the day?

A. The client prefers oatmeal instead of eggs for breakfast.

B. The client requires two-person assistance to transfer and coughed while drinking water this morning.

C. The client's daughter brought in clean clothing from home.

D. The client watched television most of the evening.

Correct Answer: B

Choice B is correct because transfer assistance needs and coughing with liquids affect safety, mobility, and possible swallowing concerns, making them highly relevant to nursing, therapy, and dietary planning. The other details may be true but are not priority interdisciplinary information.

536. A practical nurse is leaving a voicemail message for a home health agency about a client who will be discharged tomorrow. Which information is most important to include to support continuity of care?

- A.** The client enjoys talking with staff and is motivated to go home.
- B.** The client was admitted 6 days ago and had several visitors this week.
- C.** The client has a new leg wound, needs daily dressing changes, uses a cane, and lives on the second floor without an elevator.
- D.** The client would like visits scheduled in the afternoon if possible.

Correct Answer: C

Choice C includes the client's ongoing care needs, mobility status, and home environment barriers, all of which are essential for safe discharge follow-up. The other options are secondary details and do not provide the clinical or functional information the receiving agency most needs.

537. A licensed practical nurse receives report from a nursing assistant that a client 'just doesn't seem right.' Which response by the nurse best promotes effective interdisciplinary information sharing?

- A.** Document the statement in the chart and continue scheduled tasks.
- B.** Ask the assistant to describe specific changes such as behavior, vital signs, breathing, intake, or activity level.
- C.** Tell the assistant to report only if the client becomes unresponsive.
- D.** Assume the comment refers to the client's usual anxiety.

Correct Answer: B

Choice B is best because effective communication requires clarifying vague concerns into specific observable findings. That information helps the nurse assess the client and decide what must be reported to the broader team. The other responses dismiss a potentially important change.

538. A practical nurse is updating the care team about a client with chronic obstructive pulmonary disease. Which report is most complete and useful?

- A.** The client is breathing worse today.
- B.** The client seems anxious and tired.
- C.** The client has increased cough with thick yellow sputum, respiratory rate 30/min, oxygen saturation 88% on prescribed oxygen at 2 L/min by nasal cannula, and diminished breath sounds bilaterally.
- D.** The client may need another breathing treatment soon.

Correct Answer: C

Choice C is correct because it combines symptoms, objective measurements, current oxygen use, and assessment findings. This allows other team members to understand the severity of the change and respond appropriately. The other options are incomplete or too subjective.

539. A practical nurse is participating in an interdisciplinary meeting for a client with newly diagnosed dysphagia. Which nursing statement is most helpful to the team?

- A.** The client is upset about being in the hospital.
- B.** The client ate poorly yesterday.
- C.** The client coughed after thin liquids, tolerated pureed foods better, and needs reminders to take small bites and remain upright during meals.
- D.** The client would rather not change the diet.

Correct Answer: C

Choice C provides specific observations that inform the work of nursing, speech therapy, and dietary services. It identifies what increases aspiration risk and what strategies appear safer. The other statements are too general to guide coordinated care.

540. A practical nurse receives a transfer report from the emergency department for a client with chest pain. Which missing information should the nurse obtain first to support safe interdisciplinary care?

- A. Whether the client arrived by ambulance or private car
- B. The client's preferred first name
- C. The time of the last chest pain episode, current pain level, and treatments already given
- D. Whether the client finished the meal tray in the emergency department

Correct Answer: C

Choice C is the priority because current symptoms and recent interventions are essential for immediate ongoing care and communication with the provider and receiving team. The other details may be useful later but are not the most urgent for continuity and safety.

541. A practical nurse is reporting a new pressure injury concern during rounds. Which statement best supports an interdisciplinary response?

- A. The client's skin looks bad on the lower back.
- B. There is a 2-cm open area over the sacrum with surrounding redness; the client needs help turning and often slides down in bed.
- C. The client probably developed the wound because of age.
- D. Housekeeping should be more careful with the bedding.

Correct Answer: B

Choice B is best because it gives location, size, surrounding condition, and contributing factors related to mobility and positioning. That information helps nursing, wound care, and rehabilitation staff coordinate prevention and treatment. The other options are vague, speculative, or irrelevant.

542. A practical nurse is caring for a client who is deaf and uses American Sign Language. The provider arrives unexpectedly and begins explaining a consent-related procedure while the client's adult son starts interpreting. What is the nurse's best action to support accurate interdisciplinary communication?

- A. Allow the son to interpret because he already knows the client's history.
- B. Summarize the explanation later using written notes only.
- C. Request a qualified interpreter and delay the discussion until accurate communication can occur, unless emergency circumstances require immediate action.
- D. Ask the son to interpret only the parts about risks and benefits.

Correct Answer: C

Choice C is correct because consent-related communication must be accurate and complete. A qualified interpreter supports reliable communication across disciplines and protects the client's rights. Family members may unintentionally omit, alter, or filter information, making them a poor choice for this role when a qualified interpreter is needed.

543. A practical nurse notes in the chart that a client has had no bowel movement for 5 days, abdominal distention, decreased appetite, and nausea. Which communication to the provider is most appropriate?

- A. The client is constipated again and wants something for it.
- B. The client has not had a bowel movement in 5 days and now has abdominal distention, nausea, decreased appetite, and hypoactive bowel sounds.
- C. The client has stomach issues and may need a laxative.
- D. Can you prescribe the same thing that worked last time?

Correct Answer: B

Choice B gives objective, organized findings that describe the problem clearly and support decision-making. The other choices are too vague, assume a treatment plan, or fail to include important assessment details.

544. A practical nurse is contributing to care planning for a client with Parkinson disease who will be discharged to an assisted-living facility. Which information is most important to communicate to the receiving team?

- A.** The client enjoys classical music before bedtime.
- B.** The client needs medications given on a consistent schedule, requires extra time to swallow pills, and is at increased fall risk during transfers.
- C.** The client previously worked as a school bus driver.
- D.** The family hopes the move will improve socialization.

Correct Answer: B

Choice B is most important because it identifies time-sensitive medication needs, swallowing considerations, and fall risk, all of which directly affect safe daily care after transfer. The other options may help with personalization but are not the highest-priority interdisciplinary information.

545. A practical nurse receives end-of-shift report on four clients. Which report should the nurse question for being too incomplete to support safe continuity of care?

- A.** Client with cellulitis: receiving IV antibiotics, temperature 37.8°C, redness remains within marked borders, pain 3/10 after acetaminophen at 1700.
- B.** Client with heart failure: walked 50 feet with assistance, crackles at bases, 1+ ankle edema, urine output 900 mL this shift.
- C.** Client after hip repair: stable and doing okay.
- D.** Client with gastroenteritis: two loose stools this afternoon, tolerated broth, IV fluids infusing, mucous membranes less dry.

Correct Answer: C

Choice C should be questioned because 'stable and doing okay' is too vague to support safe handoff. It omits pain status, mobility, neurovascular checks, medications, and other relevant postoperative details. The other reports contain concrete findings and current status information.

546. During interdisciplinary rounds, the practical nurse reports that a client with delirium was calm overnight after receiving a PRN sedative. Which additional information is most important for the nurse to include to avoid misleading the team?

- A.** The client used to work night shift before retirement.
- B.** The client slept 6 hours without asking for a snack.
- C.** The client became difficult to arouse this morning and required extensive assistance to take oral medications safely.
- D.** The family was pleased the client was quieter overnight.

Correct Answer: C

Choice C is critical because it changes the meaning of the overnight improvement. The client may not simply be calmer; the sedative may be contributing to oversedation and impaired function. Without that follow-up information, the team could draw the wrong conclusion about the intervention's success.

547. A practical nurse is asked during rounds whether a client with pneumonia is ready for discharge later today. Which response best supports sound interdisciplinary decision-making?

- A.** Yes, because the client says he wants to go home.
- B.** Probably, because the fever is lower than yesterday.
- C.** The client still becomes short of breath walking to the bathroom, needs 2 L/min oxygen to keep saturation above 92%, and has not demonstrated correct inhaler use.
- D.** That is mainly the provider's decision.

Correct Answer: C

Choice C is best because it presents functional status, oxygen needs, and a teaching gap, all of which directly affect discharge readiness and require team attention. The other responses rely on one limited factor or avoid the nurse's responsibility to contribute relevant information.

548. A practical nurse is updating the provider about a client with sepsis who has an indwelling urinary catheter. The nurse reports fever, tachycardia, hypotension, and decreased urine output. Which additional information would most strengthen the communication?

- A. The client's spouse is on the way to visit.
- B. The urine in the drainage bag appears cloudy, and the catheter was inserted 9 days ago.
- C. The client usually prefers oral medications.
- D. The client asked whether discharge could still happen tomorrow.

Correct Answer: B

Choice B adds a likely source of infection and relevant timing, which helps the provider and team analyze the situation and plan next steps. The other details do not materially improve urgent interdisciplinary decision-making in a client with suspected sepsis.

549. A practical nurse is calling the provider about a client with heart failure who has increased shortness of breath. Which report best supports effective interdisciplinary decision-making?

A client with heart failure is more short of breath than this morning. Breath sounds now have crackles at the bases, oxygen saturation decreased from 95% to 89% on room air, respirations are 28/min, and bilateral ankle edema is 2+.

Which additional information is most important for the nurse to include?

- A. The client watched television most of the afternoon
- B. The client gained 2.2 lb (1 kg) since yesterday
- C. The client's daughter visited during lunch
- D. The client prefers to sleep with the door closed

Correct Answer: B

A daily weight gain of 2.2 lb (1 kg) provides objective evidence of fluid retention and helps the provider connect the worsening respiratory status to possible heart failure exacerbation. Effective interdisciplinary communication includes timely, relevant clinical data that supports action. The other options do not meaningfully guide treatment or explain the change in condition.

550. A practical nurse is preparing for interdisciplinary rounds on a client admitted 2 days ago with uncontrolled diabetes. Which update is most important to communicate to the team?

The client is eating 25% of meals, says insulin is sometimes skipped at home because the client cannot afford supplies, and had blood glucose readings of 278 mg/dL and 301 mg/dL today.

- A. The client likes sugar-free gelatin
- B. The client usually naps after breakfast
- C. The client reports difficulty paying for diabetic supplies
- D. The client asked whether the hospital coffee is decaffeinated

Correct Answer: C

Financial barriers to obtaining supplies are highly relevant to discharge planning and require coordination with the provider, case manager, and possibly social services. Interdisciplinary information sharing should highlight factors that affect safety and the ability to follow the treatment plan after discharge. The meal intake and glucose values matter clinically, but the affordability barrier is the key piece that drives team-based planning across disciplines.

551. A practical nurse overhears a nursing assistant say during shift change, "Mr. Lopez was really difficult all morning." Which response by the nurse best improves interdisciplinary information sharing?

- A. Tell me exactly what behaviors you observed and when they occurred.
- B. He is often like that, so just document he was uncooperative.

C. Let's wait and see if the next shift has the same problem.

D. You should avoid mentioning personality issues in report.

Correct Answer: A

Effective information sharing replaces vague labels with objective, time-linked observations. Asking for specific behaviors and timing helps turn a subjective statement into usable clinical information, such as refusal of care, confusion, pain behaviors, or changes from baseline. Choice B reinforces a nonobjective label, choice C delays needed assessment, and choice D could suppress relevant observations instead of clarifying them.

552. A client is being transferred from a medical unit to an inpatient rehabilitation facility after a hip fracture repair. Which information is most important for the practical nurse to communicate to the receiving team?

A. The client enjoys crossword puzzles in the evening

B. The client requires two-person assist for transfers and must follow hip precautions

C. The client prefers soup instead of sandwiches at lunch

D. The client's family plans to visit this weekend

Correct Answer: B

Transfer ability and required movement precautions are essential for safe handoff because they affect mobility, fall prevention, and therapy planning immediately on arrival. Interdisciplinary communication should emphasize information that changes how the receiving team provides care. The other choices may be useful later but are not the priority for a safe transfer.

553. A practical nurse is updating the dietitian about a client with chronic kidney disease. Which statement is the best example of effective interdisciplinary communication?

A. The client is not eating well and seems unhappy with the food.

B. The client ate about half of breakfast and does not like vegetables.

C. The client consumed 20% of meals over the last 24 hours and reports nausea after eating.

D. The client says the tray service is slow and the menu is confusing.

Correct Answer: C

Choice C gives objective intake data over a defined time period and includes a symptom that may explain the poor intake, making it actionable for the dietitian and the rest of the team. Effective interdisciplinary sharing should be specific, measurable, and clinically useful. The other choices are either vague, overly limited, or focused on nonclinical preferences.

554. A practical nurse is caring for a client with a new colostomy. During interdisciplinary rounds, the surgeon says the client may be discharged tomorrow. Which nursing information is most important to share at that time?

A. The client asked whether showering is allowed after discharge

B. The client has not yet demonstrated independent pouch emptying or skin care

C. The client prefers morning teaching sessions

D. The client's spouse brought in loose-fitting clothing

Correct Answer: B

A client's ability to perform essential self-care is critical to discharge readiness and should be clearly communicated to the team. If the client cannot yet manage pouch emptying and skin care, discharge planning may need more teaching, wound-ostomy support, or home assistance. The other choices are less important for deciding whether discharge is safe tomorrow.

555. A practical nurse receives a provider's telephone prescription for a client with severe pain. To promote accurate interdisciplinary communication and reduce error, which action should the nurse take first after writing down the prescription?

A. Ask another nurse to decide whether the prescription sounds appropriate

B. Read back the prescription exactly as written to the provider for verification

C. Wait until the provider enters the prescription electronically later

D. Give the medication now and clarify the prescription at the next rounding time

Correct Answer: B

Reading back a telephone prescription exactly as written helps verify the medication, dose, route, and timing before it is carried out. This is a key safety step in interdisciplinary communication because verbal orders are vulnerable to misunderstanding. The other choices either delay needed clarification or introduce unsafe assumptions.

556. During morning rounds, a practical nurse reports that a client with pneumonia has had a temperature of 38.6°C (101.5°F), productive cough, and fatigue. Which additional information would be most important to include for the interdisciplinary team?

- A. The client prefers juice over water with meals
- B. The client has been watching old movies overnight
- C. The client's oxygen saturation is 90% on 2 L/min by nasal cannula
- D. The client says the room is colder than at home

Correct Answer: C

Oxygen saturation on current oxygen support gives the team immediate information about respiratory status and the client's response to treatment. In pneumonia, this is more important for shared decision-making than comfort preferences or unrelated observations. The trap is focusing only on symptoms like fever and cough without including objective measures of oxygenation.

557. A practical nurse is contributing to discharge planning for a client who will receive home wound care. The client speaks limited English, and the adult grandchild usually translates at home. Which action best supports safe interdisciplinary information sharing?

- A. Ask the grandchild to interpret all discharge instructions to save time
- B. Request a qualified medical interpreter for team teaching and home-care coordination
- C. Provide written instructions in English and ask the client to sign receipt
- D. Delay all teaching until the home-care nurse visits after discharge

Correct Answer: B

A qualified medical interpreter supports accurate, complete communication among the client, nurse, provider, and home-care team. This helps prevent misunderstanding about wound care, follow-up, and warning signs. Family translation may omit or alter information, and delaying or relying on English-only materials can jeopardize safe continuity of care.

558. A practical nurse notices that a provider's progress note says a client is "ambulating independently," but the nurse assessed the client 30 minutes ago and found the client needed one-person assist with a gait belt. What is the nurse's best action?

- A. Document the nursing assessment and promptly clarify the discrepancy with the provider and team
- B. Assume the provider observed the client at a stronger moment and leave the note unchanged
- C. Change the provider's note so the chart matches the nursing assessment
- D. Wait until discharge planning begins to mention the difference

Correct Answer: A

Conflicting information in the record can lead to falls, unsafe transfers, and flawed discharge plans. The nurse should document accurate findings and communicate the discrepancy promptly so the team is working from the same current picture. Changing another clinician's note is inappropriate, and delaying clarification risks harm.

559. A practical nurse is preparing a written update for the interdisciplinary team about a client with bipolar disorder who slept only 2 hours and has been pacing, talking rapidly, and interrupting others. Which documentation is best to share?

- A. Client is manic and impossible to manage.
- B. Client is acting strange and making the staff uncomfortable.
- C. Client slept 2 hours, paced hallway for 40 minutes, spoke rapidly, and frequently interrupted conversation during the shift.
- D. Client had a bad night and needs stronger medication.

Correct Answer: C

Choice C uses observable, measurable behaviors rather than labels or conclusions. Objective behavioral data is more useful to the interdisciplinary team because it supports assessment, treatment planning, and evaluation of changes over time. The other options are judgmental, vague, or make unsupported treatment recommendations.

560. A practical nurse is sending a secure electronic message to the provider about a client whose potassium level is 2.9 mEq/L. Which message best demonstrates effective interdisciplinary communication?

- A.** Potassium low. Please advise.
- B.** Lab issue on 214. I am worried.
- C.** Client in room 214 has potassium 2.9 mEq/L, reports leg cramps, and has irregular pulse at 110/min. Request evaluation and orders.
- D.** Abnormal results are in the chart if you want to review them.

Correct Answer: C

Choice C is specific, concise, and actionable. It includes the abnormal value, relevant symptoms, and a clear request for follow-up. Effective interdisciplinary messaging should reduce ambiguity and support timely response. The other choices are too vague or put the burden on the receiver to search for critical information.

561. A practical nurse is working with physical therapy and nursing assistive personnel to mobilize a client after abdominal surgery. Which information is most important for the nurse to share before the first hallway ambulation?

- A.** The client used to coach high school baseball
- B.** The client reports pain 8/10 with movement and became dizzy when sitting at the bedside
- C.** The client prefers to walk after lunch instead of before lunch
- D.** The client wants family present during physical therapy sessions

Correct Answer: B

Severe movement-related pain and dizziness with position change are safety-critical findings that can alter the ambulation plan and may require reassessment, medication timing, or additional assistance. Sharing this before ambulation supports coordinated, safe care. The other options are secondary preferences, not immediate safety concerns.

562. A practical nurse is asked to fax records to a dialysis center that will receive a client tomorrow. Which action best reflects appropriate interdisciplinary information sharing?

- A.** Send the entire chart so the center has every possible detail
- B.** Fax only the most recent medication list because that is the only urgent information
- C.** Send the authorized transfer information needed for continuity of care and verify the correct destination before sending
- D.** Ask the client's roommate to confirm the dialysis center's fax number

Correct Answer: C

Interdisciplinary information sharing must balance continuity of care with privacy and accuracy. Sending the needed authorized records to the verified destination supports safe transition while avoiding unnecessary disclosure. Sending everything is excessive, sending too little can impair care, and using a roommate to confirm destination information breaches privacy.

563. At the end of the shift, a practical nurse is giving report on a client receiving IV antibiotics for cellulitis. Which statement is the best handoff?

- A.** The client is stable and should be fine overnight.
- B.** The leg still looks bad, but there were no major problems today.
- C.** Cellulitis of left lower leg; received 1800 IV antibiotic as ordered, temperature decreased from 38.2°C to 37.6°C, pain is 3/10 after medication, and redness extends 1 cm beyond the marked border from this morning.
- D.** The provider knows about the infection, and labs are in the chart.

Correct Answer: C

Choice C gives diagnosis, treatment provided, response to treatment, and a specific reassessment finding about the redness. That level of detail supports continuity and helps the oncoming nurse recognize whether the client is improving or worsening. The other options are vague and do not provide actionable clinical information.

564. A practical nurse is participating in bedside rounds for a client hospitalized with COPD exacerbation. Which information is most important to share with the interdisciplinary team to support the client's plan of care for the next 24 hours?

The client used pursed-lip breathing during care, walked 20 feet with rest periods, oxygen saturation dropped to 88% with activity and returned to 93% after 2 minutes on prescribed oxygen, and states, "I get too tired to fix meals at home."

- A.** The client prefers the room lights dimmed in the evening.
- B.** The client became tired with activity, had exertional oxygen desaturation, and reports difficulty preparing meals at home.
- C.** The client has COPD and is receiving oxygen therapy.
- D.** The client uses pursed-lip breathing during care.

Correct Answer: B

B is correct because it combines current functional status, a measurable response to activity, and a likely discharge concern that may require input from nursing, respiratory therapy, and case management or dietary services. A is low priority. C is too general and does not help the team make decisions. D is relevant but incomplete because it lacks the broader clinical and home-management issues.

565. A practical nurse cares for a client who had a stroke 3 days ago. During a speech therapy session, the client coughs after sipping thin liquids. Which communication by the nurse best supports interdisciplinary care?

The client is alert, follows simple commands, and asks for water frequently.

- A.** Tell the unlicensed assistive personnel to avoid giving drinks until the next shift.
- B.** Document that the client dislikes water and may be uncooperative with therapy.
- C.** Notify the registered nurse and speech-language pathologist that the client coughed with thin liquids and requests water often.
- D.** Wait to see whether coughing happens again at dinner before reporting it.

Correct Answer: C

C is correct because coughing with thin liquids after stroke may signal dysphagia and aspiration risk, which requires prompt communication to the RN and speech-language pathologist. A delays broader team awareness and uses an incomplete approach. B is inaccurate and judgmental. D risks harm by delaying action.

566. A practical nurse is updating the interdisciplinary team about a client with a stage 2 pressure injury. Which report is the most useful?

The wound is on the sacrum and the client has limited mobility.

- A.** The skin looks worse today and probably needs a different treatment.
- B.** The client has a sore on the lower back and does not like being turned often.
- C.** The sacral wound measures 2 cm by 1 cm, has a pink wound bed with scant serous drainage, and the client needs reminders to turn every 2 hours.
- D.** The pressure injury is still present and should be monitored.

Correct Answer: C

C is correct because it gives objective, specific data the team can use: location, size, wound characteristics, drainage, and adherence concerns. A and D are too vague. B includes subjective wording and lacks measurable details.

567. A practical nurse is caring for a client with newly diagnosed hypertension. The provider has prescribed diet changes, and the dietitian left written teaching materials in English. The client says, "I read very little, even in my own language." What should the nurse communicate to the team first?

- A.** The client may not follow the diet after discharge.
- B.** The client has low health literacy and will need teaching adapted beyond written English materials.
- C.** The client should ask a family member to explain the handout.
- D.** The dietitian should return another day when the client feels better.

Correct Answer: B

B is correct because it identifies a specific barrier that affects interdisciplinary teaching and discharge planning. The team can adjust education methods and involve interpreter or teaching support as needed. A is an unsupported prediction. C may be inappropriate and does not solve the literacy problem. D delays care without addressing the underlying need.

568. A practical nurse receives morning laboratory results for four clients. Which result should the nurse communicate first to the interdisciplinary team?

Client 1: Hemoglobin 10.2 g/dL after surgery yesterday.

Client 2: International normalized ratio (INR) 2.4 while taking warfarin.

Client 3: Blood glucose 68 mg/dL before breakfast and the client is awake and sweating.

Client 4: Sodium 134 mEq/L with no neurologic symptoms.

- A.** Client 1 hemoglobin 10.2 g/dL
- B.** Client 2 INR 2.4
- C.** Client 3 blood glucose 68 mg/dL with sweating
- D.** Client 4 sodium 134 mEq/L

Correct Answer: C

C is correct because symptomatic hypoglycemia requires prompt action and communication. The other results may need follow-up, but they are less urgent based on the information given. This question tests not just recognizing an abnormal value, but identifying which information the team needs immediately to protect the client.

569. During an interdisciplinary conference, the social worker asks whether a client recovering from bowel surgery can manage ostomy care at home alone. Which nursing report is most helpful?

- A.** The client seems motivated and says everything will be fine.
- B.** The client watched the ostomy teaching and asked several questions.
- C.** The client emptied the pouch with verbal cues but was unable to cut the skin barrier to fit the stoma independently.
- D.** The client lives alone and has a neighbor nearby.

Correct Answer: C

C is correct because it gives direct, observable information about the client's actual performance with self-care tasks. That helps the team plan home support, additional teaching, or referral. A is subjective, B shows engagement but not skill, and D is social context without showing ability.

570. A practical nurse notes that a postoperative client has an active physical therapy order for weight-bearing as tolerated. However, the latest orthopedic note states, "Non-weight-bearing right leg until reevaluation." What is the nurse's best action to support safe interdisciplinary information sharing?

- A.** Follow the physical therapy order because it was entered first.
- B.** Ask the client which instruction seems more correct.
- C.** Hold ambulation and promptly clarify the conflicting instructions with the registered nurse and appropriate providers before therapy continues.
- D.** Wait until the next round of charting to see whether the note is updated.

Correct Answer: C

C is correct because conflicting activity instructions create immediate safety risk. The nurse should stop potentially unsafe action and seek clarification through the appropriate team members. A and D could harm the client. B is not a reliable way to resolve contradictory orders.

571. A practical nurse is leaving a voicemail on a secure provider line about a client with worsening nausea and vomiting. Which message best supports effective interdisciplinary communication?

- A.** This client is very sick and needs something stronger right away. Please call me back.
- B.** Ms. Reed in room 204 has vomited three times since 0600, cannot keep oral fluids down, has dry mucous membranes, urine output of 120 mL in 8 hours, and reports abdominal pain rated 7 out of 10. Please call the unit for further assessment and orders.
- C.** The client is still nauseated and probably dehydrated.
- D.** Please review the chart when you have time because the client is not doing well.

Correct Answer: B

B is correct because it includes specific, actionable, objective data and a clear request for follow-up. A is emotional but not informative. C is too vague and relies on assumption. D lacks urgency cues and meaningful clinical details.

572. A practical nurse is caring for a client with dementia who becomes agitated every evening. The team is discussing whether medication changes are needed. Which nursing information is most important to share?

- A.** The family says the client was always stubborn at home.
- B.** The client is harder to manage at night than during the day.
- C.** Agitation begins most evenings around shift change, decreases when noise is reduced, and the client settles after toileting and a snack.
- D.** The client has dementia and should probably receive a sedative.

Correct Answer: C

C is correct because it identifies the timing, pattern, and response to nonpharmacologic measures, which are highly useful to the interdisciplinary team. A is judgmental and not clinically helpful. B is too general. D jumps to a treatment conclusion instead of sharing assessment data.

573. A practical nurse is preparing a handoff for a client who will start hemodialysis today. Which piece of information is most important to communicate to the dialysis nurse before transport?

- A.** The client prefers warm blankets during treatments.
- B.** The client has a left arm arteriovenous fistula with a palpable thrill and no blood pressure or venipuncture should be done on that arm.
- C.** The client watched a video about dialysis yesterday.
- D.** The family plans to visit this afternoon.

Correct Answer: B

B is correct because access protection is essential for safe coordinated care and should be clearly communicated across disciplines. A and D are lower priority comfort or scheduling details. C is not the most important information before treatment begins.

574. A practical nurse is assisting with discharge planning for a client with a new prescription for home oxygen. Which finding is most important to communicate to the interdisciplinary team?

- A.** The client asks whether portable oxygen tanks are heavy.
- B.** The client says, "My daughter can probably stop by after work."
- C.** The client lives in a second-floor apartment and says the electricity was shut off twice in the last 3 months.
- D.** The client wants to go home before noon.

Correct Answer: C

C is correct because it identifies a major safety and access barrier that directly affects whether home oxygen can be used reliably. This information is crucial for case management, social work, and discharge planning. The other choices may matter, but they are not as urgent or consequential.

575. A practical nurse is communicating with the pharmacist about a client who has become increasingly drowsy. The client started a new opioid 12 hours ago. Which information is most important for the nurse to include?

- A.** The client usually takes naps in the afternoon.
- B.** The client rates pain as 3 out of 10 and does not want visitors.
- C.** The client is difficult to arouse, respiratory rate is 10/min, and oxygen saturation is 90% on room air.
- D.** The client received the medication from the night nurse.

Correct Answer: C

C is correct because level of consciousness, respiratory rate, and oxygenation are critical data when communicating possible opioid-related oversedation. A, B, and D are less useful for immediate medication-related decision-making.

576. A practical nurse is in an interdisciplinary meeting about a client with repeated falls. The provider says the client is "noncompliant" because the bed alarm was off twice overnight. Which nursing response best improves information sharing?

- A.** The client keeps turning the alarm off and is not cooperative.
- B.** The client was found attempting to toilet without assistance after receiving a diuretic at 2100 and again at 0300; the client said the alarm noise caused embarrassment with the roommate present.
- C.** The falls are the night shift's problem because that is when they happen.
- D.** A restraint may be the easiest solution.

Correct Answer: B

B is correct because it reframes the issue with objective observations, timing, possible contributing factors, and the client's perspective. That helps the team develop safer solutions. A uses labeling instead of useful data. C is inappropriate and unhelpful. D jumps to an extreme intervention without adequate assessment.

577. A practical nurse is reviewing a discharge summary before it is sent to a long-term care facility. The summary lists the client as a full code. During this admission, the client signed a valid do-not-resuscitate order that is in the chart. What should the nurse do first?

- A.** Send the summary because the receiving facility can review the chart later.
- B.** Cross out the incorrect code status and initial the change before faxing.
- C.** Stop the transfer communication and immediately notify the registered nurse and provider so the discrepancy is corrected before records are sent.
- D.** Ask the family what code status they prefer now.

Correct Answer: C

C is correct because code-status discrepancies are high-risk and must be corrected through the proper chain before transfer information is shared. A could propagate dangerous misinformation. B alters the record inappropriately. D is not the right first step when a valid order already exists in the chart.

578. A practical nurse is caring for a client with sepsis who was transferred from the emergency department 1 hour ago. The medication administration record shows an antibiotic due now, but the handoff report states, "First dose already given downstairs." No dose is documented anywhere in the chart. What is the nurse's best action?

- A.** Administer the antibiotic now to avoid delay in treatment.
- B.** Assume the emergency department nurse forgot to chart and reschedule the medication.

C. Immediately verify whether the dose was actually administered by checking available records and contacting the emergency department or pharmacy, then communicate findings to the registered nurse and provider as needed.

D. Wait until the next scheduled dose and document that the previous shift reported giving it.

Correct Answer: C

C is correct because the nurse must resolve the discrepancy using reliable sources before the client either misses a needed dose or receives a duplicate dose. A and D risk medication error. B relies on assumption rather than verification.

579. A practical nurse is calling the provider about a client whose condition has changed. Which report best demonstrates clear interdisciplinary information sharing?

A. Your client does not look right and needs to be seen soon.

B. The client is having problems again like before and is not improving.

C. Mr. Lopez, room 204, reports chest pressure rated 8 out of 10 for 15 minutes, blood pressure is 168/94 mm Hg, heart rate is 112/min, oxygen saturation is 91% on 2 L/min by nasal cannula, and the provider was notified after the client was placed in semi-Fowler position.

D. The client says the pain medicine is not working, so I think something is wrong.

Correct Answer: C

This report is specific, organized, and clinically useful. It identifies the client, describes the problem, includes objective assessment data, and states an action already taken. That helps the provider respond quickly and appropriately. The other choices are vague, incomplete, or based mostly on opinion rather than actionable information.

580. A practical nurse is caring for a client who speaks limited English and is scheduled for physical therapy after a hip fracture. The client smiles, nods during teaching, and later tries to stand without using the prescribed walker. Which information is most important for the nurse to share with the interdisciplinary team?

A. The client seems cooperative and pleasant during care.

B. The client prefers family members to stay in the room.

C. The client may not understand mobility instructions and attempted to stand without the walker despite teaching.

D. The client nodded during the teaching session.

Correct Answer: C

The priority is sharing evidence that the client may not understand safety instructions, because this creates an immediate fall risk and affects how the team should teach and plan care. Option D is a common trap: nodding can be mistaken for understanding, but the unsafe behavior shows that understanding may be incomplete. Options A and B are less important than the safety issue.

581. During shift report, a practical nurse learns that a client with diabetes is scheduled for a wound care consult. Which newly observed finding should the nurse communicate first to the interdisciplinary team?

A. The client says the dressing tape pulls on the skin during removal.

B. The wound drainage has changed from scant serous fluid to thick drainage with a foul odor, and the client's temperature is 38.4°C (101.1°F).

C. The client asks whether the wound will leave a scar.

D. The previous dressing change took 20 minutes to complete.

Correct Answer: B

A change to foul-smelling thick drainage plus fever suggests possible infection and requires prompt interdisciplinary communication for evaluation and treatment. This information is more urgent than discomfort with tape, concerns about scarring, or the time needed for care. The key is recognizing a significant change in condition that affects the plan of care.

582. A practical nurse is reviewing the chart before an interdisciplinary discharge meeting for a client who had a below-knee amputation. The physical therapy note says the client transfers with one-person assist using a gait belt. The nursing note from 30 minutes ago states the client required two staff members and nearly fell during transfer to the commode. What is the nurse's best action?

- A.** Use the physical therapy note because it was written by the discipline specializing in mobility.
- B.** Wait until the next shift to see whether the client's transfers improve before saying anything.
- C.** Share the most recent nursing findings with the team and clarify that the client's current transfer status may be less stable than earlier documentation showed.
- D.** Remove the nursing note because conflicting information may confuse the discharge team.

Correct Answer: C

The nurse should communicate the most recent, safety-relevant assessment and clarify the discrepancy so the team can make accurate discharge decisions. Interdisciplinary information sharing depends on timely updates when a client's status changes. Option A is a trap because one discipline's earlier note does not outweigh newer evidence of instability. Option B delays needed communication, and option D is inappropriate because documentation should not be removed to avoid conflict.

583. A practical nurse is preparing for interdisciplinary rounds on a client admitted with heart failure. Which nursing update is most important to share with the team to support coordinated care planning?

- A.** The client says the hospital food tastes bland.

- B.** The client gained 2.2 lb (1 kg) since yesterday and becomes short of breath when walking to the bathroom.

- C.** The client watched the assigned heart-failure education video this morning.

- D.** The client prefers to bathe before breakfast rather than after breakfast.

Correct Answer: B

A rapid weight gain with increased shortness of breath suggests worsening fluid retention and declining status in a client with heart failure. This is the most important information for the interdisciplinary team because it may affect medical treatment, nursing monitoring, activity planning, and discharge readiness. Choice C is useful but less urgent. Choices A and D are lower-priority preferences that do not have the same immediate impact on clinical decision-making.

Legal and ethical scope

584. A practical nurse is caring for a competent adult client who says, "I don't want the blood transfusion my provider recommended." The client's spouse tells the nurse, "Please go ahead anyway. He doesn't understand how serious this is." What is the nurse's best action?

- A.** Ask the spouse to sign the consent form for the client
- B.** Notify the provider that the client is refusing the transfusion
- C.** Prepare the client for transfusion because the spouse agrees
- D.** Delay documentation until the family reaches a decision

Correct Answer: B

A competent adult has the right to refuse treatment, even if family members disagree. The nurse should communicate the refusal to the provider and document the client's decision. The spouse cannot override the client's wishes when the client has decision-making capacity. A common trap is assuming the family's concern allows treatment to proceed, but client autonomy takes priority.

585. A client scheduled for a bedside procedure asks the practical nurse, "What are the chances this could go wrong?" Which response by the nurse is most appropriate?

- A.** The provider will explain the risks and benefits before you decide
- B.** There are no real risks because this is a routine procedure
- C.** You should sign the consent now so the procedure is not delayed
- D.** I can explain the legal form because nurses obtain all informed consent

Correct Answer: A

Informed consent requires explanation of the procedure, risks, benefits, and alternatives by the provider performing or ordering the procedure, according to facility policy and role expectations. The nurse may witness the signature and reinforce teaching, but should not replace the provider's discussion of risks. The trap is choosing an answer that sounds helpful but places the nurse outside the proper scope.

586. During a meal break in the hospital cafeteria, a nursing assistant says loudly, "The young man in room 412 is here because he overdosed again." Which action by the practical nurse is best?

- A.** Ignore the statement because the nursing assistant is off the unit
- B.** Tell the nursing assistant to stop and remind them to discuss client information only in appropriate settings
- C.** Join the conversation so the full story is not misunderstood
- D.** Wait until the end of the shift to see whether anyone complains

Correct Answer: B

Client information should be shared only with appropriate team members in private, work-related settings. The nurse should intervene promptly to protect confidentiality and reinforce proper practice. Waiting increases the privacy breach, and being off the unit does not remove the duty to protect client information.

587. A practical nurse is assigned to an older adult client whose daughter says, "Do not tell my mother she has cancer. It would upset her too much." The client is alert and asks, "What did the biopsy show?" What should the nurse do first?

- A.** Tell the daughter she must leave the room
- B.** Explain to the client that the family requested privacy
- C.** Assess whether the provider has discussed the results with the client
- D.** Change the subject to avoid causing emotional distress

Correct Answer: C

The alert client has a right to information about their condition, but the nurse should first determine whether the provider has already discussed the biopsy results and what information has been communicated. This supports truthful communication while staying within scope. A common trap is either hiding information solely because family requests it or independently disclosing results without clarifying the care team's communication.

588. A client with intermittent confusion signed a consent form for surgery earlier in the day. Just before transport, the client becomes disoriented and says, "I never agreed to this." What is the nurse's priority action?

- A.** Send the client to surgery because the form is already signed
- B.** Ask a family member to confirm that the client wanted surgery
- C.** Hold transport and notify the provider immediately
- D.** Reorient the client quickly and have the client sign again

Correct Answer: C

A client who is expressing uncertainty or refusal immediately before a procedure requires prompt reassessment of consent and decision-making capacity. The nurse should stop the process and notify the provider. A signed form alone does not settle the issue if the client's current statements raise concern. The trap is treating the paper form as more important than the client's present condition and expressed wishes.

589. The parent of a 6-year-old client asks the practical nurse not to document that the child fell at home before admission, saying, "I don't want people getting the wrong idea." Which response is best?

- A.** I can leave it out if the provider already knows
- B.** I need to document relevant facts accurately in the medical record
- C.** I will chart it later only if the child has injuries
- D.** It is better not to include home incidents in hospital documentation

Correct Answer: B

The nurse has a legal and ethical duty to document relevant assessment data truthfully and accurately. Omitting important facts can compromise care and create legal risk. The likely trap is trying to reduce family distress by altering the record, but documentation must remain factual and complete.

590. A practical nurse receives a verbal order over the phone from a provider for a medication change. Which action best supports safe and legally sound practice?

- A.** Enter the order from memory after finishing other tasks
- B.** Write down the order and read it back to the provider for verification
- C.** Ask another nurse to guess the usual dose if the call is unclear
- D.** Delay recording the order until the provider signs the chart

Correct Answer: B

Writing the order down and reading it back helps verify accuracy and reduces communication errors. This is both a safety and legal protection measure. Relying on memory or guessing a dose is unsafe, and delaying documentation increases the chance of mistakes.

591. A client tells the practical nurse, "If my heart stops, I want everything done," but the chart contains a current provider order indicating do-not-resuscitate status. What should the nurse do first?

- A.** Assume the chart order is final and ignore the client's statement
- B.** Immediately clarify the discrepancy with the provider and follow facility policy

C. Tell the family to decide which instruction should be used

D. Cross out the existing order because the client changed their mind

Correct Answer: B

A conflict between the client's current expressed wishes and the medical record requires immediate clarification. The nurse should not ignore the statement, ask the family to decide, or alter orders independently. The trap is assuming old documentation always controls without reassessing the client's current preferences and proper process for updating orders.

592. A practical nurse is floated to a clinic and is asked to perform a task that is not permitted for the nurse's role under that setting's policy. The charge nurse says, "You'll be fine. We do this all the time." What is the best response?

- A.** Perform the task because the charge nurse approved it
- B.** Refuse the task and request an assignment within the nurse's role and competence
- C.** Perform the task if another staff member stays nearby
- D.** Do the task once, then document that the charge nurse insisted

Correct Answer: B

The nurse is accountable for practicing within legal scope, competence, and policy. Another staff member's reassurance does not make an out-of-scope assignment acceptable. The safest action is to decline and request a lawful assignment. A common trap is believing that supervision or documentation removes personal accountability.

593. After a medication error, the practical nurse completes the required incident report. Which statement is most accurate about documenting the event in the client's medical record?

- A.** The nurse should chart that an incident report was completed
- B.** The nurse should chart the facts of the event, the client's response, and notifications made
- C.** The nurse should avoid any charting until risk management reviews the case

D. The nurse should include personal opinions about who caused the error

Correct Answer: B

The medical record should include objective facts, assessments, interventions, client response, and required notifications. Incident reports are typically not referenced in the client's chart. Personal blame and delayed charting are inappropriate. The key trap is confusing internal reporting documents with clinical documentation.

594. A client who had sedation 30 minutes ago is asked to sign consent for an invasive procedure that was added to the plan. Which action by the practical nurse is best?

- A.** Witness the signature because the client is awake
- B.** Ask the spouse to sign instead, even though the client is present
- C.** Question the client's capacity to provide informed consent at this time and notify the provider
- D.** Explain the procedure quickly so the client can sign before becoming sleepy again

Correct Answer: C

Recent sedation may impair the client's ability to understand and make an informed decision. The nurse should recognize the concern, hold the process, and notify the provider. Being awake is not enough to prove valid consent. The trap is equating alertness with full decisional capacity.

595. A practical nurse notices that a coworker documented a full wound assessment at 0900, but the wound dressing is still sealed and dated from the previous evening. What is the nurse's best initial action?

- A.** Add a chart entry accusing the coworker of falsifying documentation
- B.** Confront the coworker publicly at the nurses' station
- C.** Follow the chain of command to report the concern according to policy
- D.** Ignore it because the client has not been harmed

Correct Answer: C

Possible falsification of the medical record is a serious legal and ethical concern. The nurse should address it through the proper reporting process. Public confrontation and accusatory charting are inappropriate, and lack of immediate harm does not remove the duty to act. The main trap is waiting for proven injury before reporting a serious practice concern.

596. A practical nurse is caring for a client who says, "Don't tell my adult son anything about my condition." Later, the son arrives and says, "I pay her bills, so you have to update me." What is the nurse's best response?

- A.** Share a brief update because the son is involved financially
- B.** Ask the son which details he wants to know first
- C.** Explain that the client's information cannot be shared without the client's permission
- D.** Provide the update as long as no written records are shown

Correct Answer: C

Client confidentiality is not based on who pays bills or helps at home. If a competent client refuses information sharing, the nurse must protect privacy unless there is appropriate authorization or another lawful basis to disclose. The trap is assuming family involvement automatically grants access to protected information.

597. A practical nurse is asked to obtain a client's signature on a consent form for surgery. The client says, "I already signed, but no one told me there was a chance I might need a colostomy." What should the nurse do?

- A.** Witness the signature because the provider can explain later
- B.** Tell the client that complications are always possible and not important to consent
- C.** Stop the signing process and notify the provider that the client has unanswered questions
- D.** Ask the family to reassure the client so the procedure can stay on schedule

Correct Answer: C

A client who indicates a significant gap in understanding has not completed a fully informed decision-making process. The nurse should pause the consent process and notify the provider so questions can be addressed. The trap is focusing on keeping the schedule moving rather than ensuring valid informed consent.

598. At the end of a shift, a practical nurse realizes a 1400 pain reassessment was not charted, although the reassessment was actually completed at that time. Which action is most appropriate?

- A.** Enter the reassessment with the current time and indicate it is a late entry for the 1400 assessment
- B.** Chart the reassessment at 1400 to make the record look complete
- C.** Skip the entry because late charting is not allowed
- D.** Ask a coworker to document the reassessment since they remember the client

Correct Answer: A

If documentation was delayed but the care was actually provided, the nurse should make a late entry according to policy, using the current time and clearly identifying the actual time of the assessment. Backdating is falsification, and asking another person to chart care they did not provide is inappropriate. The trap is trying to make the record appear uninterrupted instead of accurate.

599. An alert adult client with severe burns says, "I want to leave now, even if it could kill me." The provider has explained the risks, and the client correctly repeats them back. The client's brother tells the nurse, "Stop him. He is making a terrible choice." What is the nurse's best action?

- A.** Ask security to prevent the client from leaving until the family agrees
- B.** Support the client's right to leave after following facility procedure and notifying the provider
- C.** Tell the brother to sign a form accepting responsibility for the client's decision
- D.** Delay discharge until the client promises to return for treatment tomorrow

Correct Answer: B

A competent adult who demonstrates understanding of the risks has the right to refuse treatment and leave, even when the decision is unwise. The nurse should protect client autonomy, notify the provider, and follow agency procedure. Preventing the client from leaving without legal justification could violate the client's rights. The common trap is choosing a family-centered response instead of recognizing that the client's decision controls.

600. A practical nurse is caring for a client with a new diagnosis of HIV infection. Another nurse who is not assigned to the client says, "I heard room 206 got some bad news. What exactly is going on?" Which response is most appropriate?

- A.** Share only the diagnosis because nurses should be aware of infection risks
- B.** Discuss the case quietly at the nurses' station so visitors cannot hear
- C.** State that client information is shared only with staff directly involved in the client's care
- D.** Tell the nurse to ask the provider because providers may disclose diagnoses

Correct Answer: C

Confidential information should be shared only with personnel who need it for the client's care. The other nurse is not assigned to the client, so the practical nurse should decline to disclose the diagnosis. Quietly discussing it still violates privacy if there is no need to know. The trap is thinking that a healthcare role alone gives unlimited access to private information.

601. A practical nurse is preparing to witness a client's signature on a surgical consent form. The client asks, "Can you explain exactly how the surgeon will do the operation and all the possible complications?" What should the nurse do?

- A.** Provide a full explanation so the surgery is not delayed
- B.** Tell the client that signing is required before asking further questions
- C.** Notify the provider because the client needs more information before consent can be considered informed

D. Have a family member explain the procedure in simple language

Correct Answer: C

Informed consent requires that the client receive adequate information about the procedure, risks, benefits, and alternatives from the provider performing or responsible for the procedure. The nurse may witness the signature and reinforce teaching, but should not replace the provider's explanation. The trap is picking A because the nurse wants to help, but doing so exceeds the nurse's role in obtaining informed consent.

602. A practical nurse accidentally administers an antihypertensive medication to the wrong client. The client is stable, and the charge nurse has been notified. Which action should the nurse take next?

- A.** Document the medication error in the client's chart and complete the agency reporting process
- B.** Wait until the end of the shift to see whether the client develops symptoms
- C.** Ask the charge nurse to chart the event so there is only one account
- D.** Record the incident only on the agency event form, not in the medical record

Correct Answer: A

After an error, the nurse should continue to prioritize client safety, document relevant facts and the client's response in the medical record, and complete the required agency reporting process. Waiting delays monitoring and treatment. The event form does not replace charting of the client's status and care. The trap is D, which confuses internal reporting with clinical documentation.

603. A client tells the practical nurse, "I signed papers last year saying I do not want machines keeping me alive." The chart contains no copy of any advance directive. What is the nurse's best action?

- A.** Document the statement and notify the charge nurse or provider so the client's wishes can be clarified
- B.** Assume the client has a do-not-resuscitate order and place a note above the bed

C. Tell the client advance directives are handled only at hospital admission

D. Ask the family to decide what treatment the client should receive

Correct Answer: A

The client's statement should be taken seriously, documented, and communicated so the team can verify and clarify the client's wishes. The nurse should not assume that an advance directive automatically equals a current do-not-resuscitate order. Family members do not override a capable client's expressed wishes. The trap is B, which skips the needed verification process.

604. A practical nurse is caring for a client who says quietly, "My neighbor works in this hospital. Please don't let anyone tell her I'm here." Which action best protects the client's privacy?

- A.** Explain that hospital employees may view any chart if they keep the information private
- B.** Tell the client that visitors, not staff, are the main privacy concern
- C.** Share the concern with appropriate staff and follow facility measures to limit unnecessary disclosure of the client's presence
- D.** Advise the client to avoid speaking with any employee other than the nurse

Correct Answer: C

The nurse should act on the client's privacy concern by using approved measures to protect confidentiality and by informing appropriate staff who can help implement those protections. Staff do not have a right to access a chart without a care-related need. The trap is A, which wrongly assumes employment alone allows access.

605. A practical nurse working in long-term care sees a coworker sign a resident's medication administration record for a narcotic before giving the medication. What is the nurse's best initial action?

- A.** Immediately report the coworker to the police
- B.** Confront the coworker privately and direct the coworker to correct the unsafe practice at once
- C.** Ignore it unless the resident complains of pain later

D. Cosign the record so the documentation appears complete

Correct Answer: B

Charting a medication before administration is unsafe and legally problematic. The best initial action is to address the situation promptly and directly so resident safety is protected and the practice can be corrected; then the nurse should follow agency reporting procedures as needed. Calling the police is not the first nursing action in this scenario. The trap is A, which skips immediate workplace intervention and the chain of command.

606. A provider asks the practical nurse to translate informed-consent information for a client who speaks limited English because the unit interpreter is busy. The client's teenage daughter says, "I can translate for my mother." What is the nurse's best response?

- A.** Use the daughter because family members are usually more comforting
- B.** Translate the discussion personally since the nurse understands basic medical Spanish
- C.** Request a qualified interpreter and delay the consent discussion until accurate communication can occur
- D.** Have the client sign now and review the details later with an interpreter

Correct Answer: C

Informed consent requires accurate communication. A qualified interpreter should be used rather than relying on a minor family member or a staff member using informal language skills for a complex legal and clinical discussion. Signing first and explaining later does not meet the standard for informed consent. The trap is A, which may feel convenient but risks errors, omissions, and coercion.

607. Near the end of the shift, a practical nurse realizes a blood glucose value was entered into the wrong client's electronic chart 20 minutes earlier. What is the most appropriate action?

- A.** Delete the entry from the record without telling anyone so the chart stays clean
- B.** Log out and leave the mistake for health information management to fix the next day

C. Follow agency procedure to correct the erroneous entry and notify the appropriate supervisor or nurse responsible for the clients

D. Copy the same value into the correct client's chart and leave the original entry in place

Correct Answer: C

Errors in the medical record must be corrected promptly using the facility's approved process so the chart remains accurate and legally sound. The nurse should also notify the appropriate person because incorrect data can affect care decisions. Deleting or ignoring the entry can create additional legal and safety problems. The trap is D, which partially fixes the problem but still leaves dangerous misinformation in the wrong chart.

608. A client with metastatic cancer tells the practical nurse, "I don't want any more chemotherapy. I just want to be comfortable." The client's adult children insist that staff continue "everything possible." What should the nurse do first?

- A.** Support the capable client's expressed wishes and inform the provider of the change in goals
- B.** Follow the children's request because they will be responsible after discharge
- C.** Tell the client that stopping treatment requires a court order
- D.** Avoid documenting the discussion until the family reaches agreement

Correct Answer: A

When a capable client expresses a treatment preference, that preference guides care. The nurse should advocate for the client's wishes, document the discussion, and notify the provider so the plan of care can be reviewed. Family disagreement does not override a competent client's decisions. The trap is B, which wrongly gives decision-making authority to family over the client.

609. In a crowded elevator, a respiratory therapist asks the practical nurse, "Is Mr. Lawson in 318 the one being evaluated for tuberculosis?" What is the best response by the nurse?

- A.** Answer briefly because the therapist is part of the healthcare team

- B.** Say that client information should be discussed in a private setting and continue the conversation later if clinically necessary
- C.** Whisper the answer so other people in the elevator cannot hear
- D.** Confirm the diagnosis but avoid using the client's full name

Correct Answer: B

Even when another team member may have a legitimate need for information, confidential details should be discussed privately and only as needed for care. A public elevator is not an appropriate place for the conversation. The trap is A, which overlooks that privacy rules apply to where and how information is shared, not only to whom.

610. A practical nurse is told by a provider, "Just put your initials on this telephone order sheet for me. I gave the order earlier, and we both know what I meant." The nurse did not hear the original order. What is the best response?

- A.** Initial the sheet because the provider is accepting responsibility
- B.** Refuse to authenticate the order and request that the provider enter or verify it appropriately
- C.** Ask another nurse to initial the order if that nurse is less busy
- D.** Write the order from memory and add a note that the provider was in a hurry

Correct Answer: B

The nurse must not authenticate an order that was not personally received or verified. Doing so creates false documentation and legal risk. The appropriate response is to refuse and require proper provider verification according to policy. The trap is A, which confuses the provider's authority with the nurse's duty to maintain accurate records.

611. A practical nurse in a rehabilitation unit posts a de-identified photo of an unusual wound dressing setup on a private social media account for "educational purposes." No name or room number is shown. Which statement is most accurate?

- A.** This is acceptable because no identifying information was written in the post

- B.** This is acceptable if the account privacy settings are limited to friends only
- C.** This may still violate professional and privacy standards, so client images should not be shared without proper authorization and policy compliance
- D.** This is acceptable if another nurse comments that the image is anonymous

Correct Answer: C

Posting client-related images on social media can violate privacy and professional boundaries even when obvious identifiers are omitted. Images, timing, context, or unique features may still identify the client, and agency policy and authorization requirements must be followed. The trap is A, which assumes removal of a name alone makes online sharing acceptable.

612. A practical nurse is asked by a charge nurse to start an IV push medication in a setting where unit policy permits only RNs to administer IV push medications. The client needs the medication soon. What is the nurse's best action?

- A.** Administer the medication because the client's need makes policy less important
- B.** Decline to perform the task and promptly seek an authorized clinician to give the medication
- C.** Administer the medication if the charge nurse agrees to stay in the room
- D.** Give half the dose now and wait for an RN to give the rest

Correct Answer: B

The nurse must practice within authorized role limits and facility policy, even when the need is urgent. The safest action is to decline the prohibited task and obtain the appropriate licensed professional without delay. Supervision does not make an unauthorized act permissible. The trap is C, which sounds safer but still exceeds legal and policy boundaries.

613. A practical nurse is assigned to reinforce teaching for a client who will perform sterile dressing changes at home. During the session, the nurse realizes the client was never told that the wound culture showed methicillin-resistant *Staphylococcus aureus* (MRSA), and the client asks, "Why do I need these extra precautions?" Which action is most appropriate for the practical nurse?

- A. Explain the culture result and prescribe the home infection-control plan in detail
- B. Tell the client not to worry about the reason and continue only with the dressing steps
- C. Notify the RN or provider that the client has questions requiring initial teaching and clarification, then reinforce the plan after that teaching occurs
- D. Ask the unlicensed assistive personnel to review the precautions because the client is being discharged soon

Correct Answer: C

The practical nurse should recognize the limit of role scope: reinforcing previously established teaching is appropriate, but providing initial teaching or explaining new diagnostic information generally requires the RN or provider according to role and agency policy. After the client receives that initial explanation, the practical nurse can reinforce the instructions for safe care at home. Choice A goes beyond reinforcement into initial teaching and plan development. Choice B ignores the client's right to understand care. Choice D is inappropriate because teaching about precautions should not be delegated to unlicensed assistive personnel.

614. A client who is alert and oriented asks to see a copy of the medical record before discharge. The unlicensed assistive personnel tells the nurse, "We do not let clients read their charts on the unit." What is the nurse's best response?

- A. Tell the client that only the provider can decide whether records may be viewed
- B. Explain that the client can request access through the facility's established process
- C. Refuse the request because chart access would violate staff confidentiality
- D. Hand the chart to the client immediately and leave the room

Correct Answer: B

Clients generally have a right to access their health information, but the nurse should follow the facility's approved process rather than casually handing over the record. Choice D skips the required process and safeguards. Choices A and C incorrectly deny or limit the client's rights.

615. A practical nurse is floated to a clinic area and is asked to perform a task that is outside the nurse's role under state law but is routinely done there by another type of licensed staff member. What is the nurse's best action?

- A. Perform the task because the clinic has used that workflow for years
- B. Perform the task if another nurse verbally agrees to be responsible
- C. Decline the task and notify the supervisor that it is outside the nurse's legal scope
- D. Perform the task one time if the client signs permission

Correct Answer: C

The nurse is accountable for practicing within legal scope, even if a task is common in that setting. A local habit does not override law or licensure limits. Client permission does not make an out-of-scope action legal, and another nurse cannot simply transfer that responsibility by verbal agreement.

616. A practical nurse is caring for a 17-year-old client who asks, "Please don't tell my parent that I'm worried I might be pregnant." The parent is in the waiting area and asks the nurse why laboratory tests were ordered. What is the nurse's best response?

- A. Share the information because the parent is financially responsible for the visit
- B. Tell the parent that the nurse cannot discuss the client's care without proper permission
- C. Ask the parent to guess the reason for the tests, then confirm it
- D. Give a partial answer now and document the rest later

Correct Answer: B

The nurse should protect the client's privacy and avoid disclosing test-related information without appropriate permission or applicable authorization. Choices C and D still disclose protected information. Choice A assumes payment automatically gives access, which is not an appropriate basis for disclosure.

617. A practical nurse notices that a confused client is repeatedly trying to climb out of bed. A family member says, "Just tie her wrists to the rails so she won't fall." Which action by the nurse is most appropriate?

- A.** Apply the restraints because the family requested them
- B.** Use the least restrictive safety measures first and follow facility and provider requirements before restraints are used
- C.** Ask the assistive personnel to hold the client in bed until she falls asleep
- D.** Raise all four side rails because rails are not considered restrictive

Correct Answer: B

Restraints raise legal and ethical concerns and should not be used just because family requests them or staff want convenience. The nurse should first try less restrictive measures and follow required processes before restraints are applied. Four side rails can function as a restraint, and physically holding a client for convenience is not appropriate.

618. During a busy shift, a provider says to a practical nurse, "I need you to tell this client that the biopsy definitely means cancer and get the form signed for tomorrow's procedure." What should the nurse do?

- A.** Explain the diagnosis and obtain the signature to avoid delaying care
- B.** Ask the nursing assistant to bring the form and witness the signature instead
- C.** Tell the provider that obtaining informed consent requires the provider to give the necessary information, while the nurse can support the client and witness the signature if policy allows
- D.** Have the family explain the diagnosis so the client will agree faster

Correct Answer: C

The provider is responsible for explaining the procedure, risks, benefits, and alternatives needed for informed consent. The nurse may support understanding and, if allowed, witness the signature, but should not substitute for the provider's disclosure. Choices A and D improperly shift the consent discussion away from the provider. Choice B does not address the core legal problem.

619. A practical nurse receives a call from a client's employer asking whether the client will be able to return to work next week after hospitalization. The caller says, "The client works for me, so I need to know." What is the nurse's best action?

- A.** Provide a general update without giving the diagnosis
- B.** Confirm only the dates of hospitalization
- C.** Refer the caller to the appropriate release-of-information process and avoid disclosing client information
- D.** Answer the question if the caller already knows the client was hospitalized

Correct Answer: C

Employment status does not entitle the caller to protected health information. The nurse should not confirm hospitalization, diagnosis, or return-to-work details without proper authorization. The safest action is to use the established release process. Choices A, B, and D all disclose information without clear permission.

620. A practical nurse documents that a sterile dressing change was completed at 1000. At 0945, the nurse realizes the dressing change has not yet been done because the supplies were unavailable. Which documentation action is most appropriate?

- A.** Leave the entry as written since the dressing will be done soon
- B.** Delete the note completely so no one sees the mistake
- C.** Correct the documentation according to facility policy and enter the accurate time and facts
- D.** Ask a coworker to chart the procedure instead

Correct Answer: C

The legal record must be accurate. Charting care before it is provided is improper, so the nurse should correct the record using approved procedures and document the true facts and time. Leaving the false entry, deleting it improperly, or asking someone else to chart it would all compromise the legal record.

621. A practical nurse learns that a coworker accessed the electronic record of a client who is a local celebrity but is not involved in that client's care. The coworker says, "I was just curious and did not tell anyone." What is the nurse's best response?

- A.** Do nothing because no information was shared outside the hospital
- B.** Advise the coworker to log out more carefully next time
- C.** Report the inappropriate record access through the proper chain according to policy
- D.** Warn the client personally but avoid involving management

Correct Answer: C

Accessing a record without a care-related need is itself a privacy violation, even if the information was not further shared. The nurse should follow the proper reporting process. Choice A misunderstands the violation. Choice B minimizes serious misconduct. Choice D bypasses required internal handling.

622. A practical nurse is caring for a client who has decision-making capacity and says, "I know my provider recommended the blood transfusion, but I do not want it." The spouse says, "You have to make him take it." What is the nurse's best action?

- A.** Prepare the client for transfusion because the spouse demands it
- B.** Respect the client's refusal, notify the appropriate licensed provider, and document the refusal
- C.** Delay documentation until the spouse calms down
- D.** Ask the spouse to sign the consent instead

Correct Answer: B

A client with decision-making capacity has the right to refuse treatment. The nurse should respect that refusal, communicate it promptly, and document it accurately. A spouse cannot override a capable client's decision. Choices A and D violate autonomy, and C delays important documentation.

623. A provider writes an order for a treatment the practical nurse has never been trained to perform and that is not included in the nurse's facility-approved competencies. The provider says, "It's simple. Just watch me once." What should the nurse do first?

- A.** Perform the treatment because a licensed provider is available on the unit
- B.** Refuse permanently without discussing the issue further
- C.** State that the treatment is outside current competence and follow the chain of command to obtain appropriate personnel
- D.** Attempt the treatment only if the client agrees

Correct Answer: C

The nurse must practice within both legal scope and personal competence. Even if a task may be permitted in some circumstances, the nurse should not perform it without proper training and validation. Choice B may be too absolute because the immediate issue is current competence and safe staffing. Choices A and D do not address competency and accountability.

624. A practical nurse is asked to call a client's adult child with an update. The chart lists the child as the emergency contact only. The client is awake, oriented, and has not given permission to share information. What is the nurse's best action?

- A.** Give the update because emergency contacts always receive medical information
- B.** Ask the client whether information may be shared with the adult child before disclosing anything
- C.** Call the adult child and provide only nonclinical details
- D.** Avoid the issue by asking another nurse to make the call

Correct Answer: B

Being listed as an emergency contact does not automatically authorize routine disclosure when the client can decide. The nurse should first obtain the client's permission. Choices A and C assume authority that has not been granted. Choice D avoids the legal and ethical issue instead of resolving it properly.

625. In a postoperative unit, a practical nurse hears another nurse discussing a client's diagnosis and prognosis in the cafeteria with two visitors sitting nearby. What is the nurse's best immediate action?

- A.** Interrupt and remind the nurse to stop the conversation in that public area
- B.** Wait until the end of the shift so the coworker is not embarrassed
- C.** Join the conversation to make sure the details are accurate
- D.** Ignore the discussion because the client's name was not spoken loudly

Correct Answer: A

A public conversation about client information risks a privacy breach and should be stopped promptly. Waiting until later allows the disclosure to continue. Joining the discussion worsens the problem, and a lowered voice or omitted name does not make the setting appropriate.

626. A practical nurse is caring for a client who becomes increasingly drowsy after a newly prescribed sedative. The provider says, "Don't worry about documenting the low respiratory rate yet. Let's see if it improves." What should the nurse do?

- A.** Delay documentation because the provider requested it
- B.** Document the client's actual assessment findings and continue appropriate reporting and monitoring
- C.** Record a normal respiratory rate until the next assessment confirms the trend
- D.** Remove the abnormal value from the chart to avoid confusion

Correct Answer: B

The nurse has an independent duty to document accurate assessment findings. A provider cannot direct the nurse to falsify or omit the record. Choices A, C, and D all create an inaccurate legal record and may endanger the client.

627. A practical nurse is caring for an adult client who says, "I do not want my diagnosis discussed in front of my partner." Later, while the client is asleep after pain medication, the partner asks, "What exactly did the biopsy show?" Which response is best?

- A.** Share the diagnosis because the partner was present during admission
- B.** Provide a brief answer because the client is sleeping and cannot object
- C.** Tell the partner that the nurse cannot share that information without the client's permission
- D.** Explain the diagnosis but avoid discussing treatment options

Correct Answer: C

The client clearly stated a privacy preference, and the nurse must honor it. The client's temporary sleep does not remove that prior decision. Choices A, B, and D all violate the client's expressed wishes and confidentiality.

628. A practical nurse is asked to witness a client's signature on a form refusing a recommended procedure. The client appears calm but says, "I'm signing because my son said he won't visit me again if I don't." What should the nurse do?

- A.** Witness the signature because the client is an adult and the form is complete
- B.** Refuse to witness the signature at this time and report concerns about possible coercion through the appropriate chain
- C.** Ask the son to sign instead because he influenced the decision
- D.** Tell the client to sign now and discuss feelings later

Correct Answer: B

A signature should not be witnessed if there is concern that the decision is not voluntary. The nurse should recognize possible coercion and escalate the concern so the client's decision-making can be protected. Choices A and D ignore the ethical issue, and C is inappropriate because the son cannot sign based on pressure he created.

629. A practical nurse is preparing a client for a procedure. The consent form is signed, but the client quietly says, "I still do not understand why I need this test." What is the nurse's best action?

- A.** Continue preparing the client because the signed form shows consent was obtained
- B.** Ask the provider to return and explain the procedure before it continues
- C.** Explain the procedure in detail and have the client sign a second form
- D.** Ask a family member to help persuade the client to proceed

Correct Answer: B

A signed form does not replace informed understanding. If the client states a lack of understanding, the nurse should stop and notify the provider so the procedure can be explained and questions answered before proceeding. Choice C is a trap because the nurse can reinforce teaching, but the provider is responsible for obtaining informed consent for the procedure itself.

630. A practical nurse is caring for a client who speaks limited English. The client's adult grandson says, "I can translate everything. We do not need an interpreter." The provider is about to discuss a procedure with significant risks. What should the nurse do?

- A.** Allow the grandson to interpret because he knows the client's language best
- B.** Proceed if the client nods during the explanation
- C.** Request a qualified interpreter for the discussion
- D.** Ask another visitor in the waiting area to help translate

Correct Answer: C

A qualified interpreter is needed when important information such as risks, benefits, and alternatives must be understood clearly. Using family or bystanders can lead to omissions, pressure, or inaccurate interpretation. Choice A is a common trap because familiarity does not equal neutrality or accuracy.

631. During a home care visit, a practical nurse notices extensive bruising on an older adult's upper arms and back. The client looks away and says, "I fall a lot," while the caregiver answers every question and refuses to leave the room. What is the nurse's priority action?

- A.** Document the bruising and wait to see whether the pattern continues
- B.** Privately assess the client further and follow agency policy for required reporting
- C.** Confront the caregiver immediately and demand an explanation
- D.** Tell the client to call the police after the nurse leaves

Correct Answer: B

The findings raise concern for possible abuse. The nurse should seek a private assessment if possible, gather objective information, and follow required reporting procedures according to policy and law. Choice C may escalate risk and is not the safest first step. Choice A delays action when there are concerning signs.

632. A practical nurse finds an advance directive in a newly admitted client's chart. Later, the client says, "I changed my mind. I want everything done if my heart stops." What is the nurse's best action?

- A.** Tell the client the written directive must be followed until discharge
- B.** Ask the family which version they want honored
- C.** Report the client's statement promptly and follow facility procedure to update the plan of care
- D.** Remove the advance directive from the chart so the new wish is clear

Correct Answer: C

A client with decision-making capacity can express current wishes, and the nurse should communicate that change promptly and follow procedure so documentation and the plan of care are updated correctly. Choice D is inappropriate because the nurse should not independently alter the legal record.

633. A practical nurse is caring for a client who has severe pain after surgery. The client says, "I know the provider ordered the medication, but I do not want it because it makes me feel strange." What should the nurse do first?

- A.** Respect the refusal, assess the pain further, and notify the appropriate licensed team member as needed
- B.** Explain that the medication was ordered for the client's benefit and should be taken
- C.** Ask the family to encourage the client to accept the medication
- D.** Document the refusal and take no further action

Correct Answer: A

A competent client has the right to refuse treatment, including pain medication. The nurse should respect the refusal, assess the situation, explore concerns, and communicate appropriately so alternatives can be considered. Choice D is incomplete because the nurse still has a duty to assess and follow up.

634. A practical nurse is invited to join a private social media group for hospital staff. Another member posts, "Guess who came into our unit after that school bus crash?" along with a photo of the hallway whiteboard showing room numbers and initials. What is the nurse's best response?

- A.** Ignore the post because no full client names are shown
- B.** Add a comment reminding staff not to discuss clients online
- C.** Report the privacy concern through the proper chain according to policy
- D.** Save the photo in case the family asks for proof later

Correct Answer: C

Posting client-related information or identifying details on social media is a privacy concern even without full names. The nurse should report the breach through the proper channels. Choice B may be well intentioned but is not sufficient by itself because a potential confidentiality breach requires formal reporting and follow-up.

635. A practical nurse receives a verbal order over the phone for a medication change. The provider speaks quickly, and the nurse is unsure whether the dose was "15" or "50." What is the nurse's best action?

- A.** Choose the lower dose because it is safer
- B.** Ask another nurse what dose is usually prescribed
- C.** Read back the order for clarification and do not carry it out until the dose is clear
- D.** Document both possible doses and wait for pharmacy to decide

Correct Answer: C

A verbal order that is unclear must be clarified before implementation. Read-back helps verify accuracy and protects the client. Choice A is a trap because guessing, even with a lower dose, is unsafe and legally risky.

636. At the end of a shift, a practical nurse realizes that 0900 wound drainage was estimated as 20 mL in the chart, but the actual amount measured was 70 mL. Which documentation action is most appropriate?

- A.** Delete the original note and enter the correct amount without explanation
- B.** Enter a late correction according to policy, clearly identifying the error and the accurate finding
- C.** Leave the record unchanged because correcting it may create legal problems
- D.** Ask the next shift nurse to chart the corrected amount as a new assessment

Correct Answer: B

Documentation errors should be corrected promptly and transparently according to policy. The record should show what was wrong and what is accurate without concealing the change. Choice A is inappropriate because it removes the trace of the original entry rather than correcting it properly.

637. A practical nurse is assigned to a client who says, "I do not want any students or observers in my room today." A student nurse arrives with the instructor and says, "We only need five minutes." What should the practical nurse do?

- A.** Explain that teaching is part of the facility's mission, so the client must cooperate
- B.** Honor the client's request and inform the instructor that the client declined
- C.** Allow the group to enter if no hands-on care is planned
- D.** Ask the family to persuade the client to help with student learning

Correct Answer: B

Clients have the right to refuse participation in teaching situations or observation. The nurse should respect that decision and communicate it professionally. Choices A and C ignore the client's right to control who is present during care.

638. A practical nurse is caring for a client with a newly diagnosed communicable infection. The client's sibling arrives and says, "Tell me exactly what infection this is so I can warn everyone at home." The client is awake and says, "I am not ready to talk about it yet." What is the nurse's best response?

- A.** Give the sibling the diagnosis because household contacts could be at risk
- B.** Tell the sibling to ask the provider instead
- C.** Respect the client's wishes and avoid disclosing the diagnosis to the sibling
- D.** Share the diagnosis but not the treatment plan

Correct Answer: C

The client's expressed wish controls disclosure to the sibling. The nurse should protect confidentiality and avoid sharing the diagnosis without permission. Choice A may seem protective, but the nurse cannot independently override confidentiality simply because a relative wants information.

639. A practical nurse overhears a provider tell a client, "If you refuse this surgery, you are basically choosing to die, and your family will blame you." The client begins crying and signs the consent form immediately. What is the nurse's best action?

- A.** Witness the signature because the form is now signed
- B.** Place the form in the chart and continue preoperative preparation
- C.** Recognize possible coercion, stop the process, and report the concern through the appropriate chain
- D.** Ask the family whether they agree with the provider's explanation

Correct Answer: C

Consent must be voluntary. Threatening language and emotional pressure raise concern for coercion, so the nurse should stop the process and report the concern. Choice A is a trap because witnessing a signature does not mean the nurse should ignore signs that the consent may be invalid.

640. A practical nurse is reviewing the assignment for the day and notices that a client with a court-appointed legal guardian is scheduled for a nonemergency invasive procedure. The client is alert but has been judged unable to make this decision. No signed consent is in the chart, and the guardian has not yet arrived. What is the nurse's best action?

- A.** Have the client sign because the client is awake and cooperative
- B.** Proceed with preparation because the provider can obtain consent in the procedure room
- C.** Notify the appropriate licensed team member that required consent has not been obtained before the procedure proceeds
- D.** Ask the transport staff to delay pickup only if the client refuses to go

Correct Answer: C

When a legal guardian is the decision-maker for this type of procedure, the required consent must be addressed before the nonemergency procedure proceeds. The nurse should promptly communicate the missing consent. Choice A is incorrect because the client's legal authority to give this consent is not established in the scenario.

641. A practical nurse is asked to photograph a client's worsening pressure injury with the unit tablet for the medical record. The nurse cannot find any documented consent or facility process for obtaining the image. The charge nurse says, "Just take it. We need it for comparison." What should the practical nurse do?

- A.** Take the photo because treatment needs are more important than privacy concerns
- B.** Use a personal phone instead because it is faster and clearer
- C.** Delay the photo until the nurse follows the approved process for authorization and secure documentation
- D.** Ask the nursing assistant to take the photo while the nurse turns away

Correct Answer: C

Clinical photographs require attention to privacy, authorization, and secure handling according to policy. The nurse should not bypass the approved process, even for a clinically useful image. Choice A is a trap because a legitimate care purpose does not remove the need to follow privacy safeguards.

642. A practical nurse is assisting with discharge planning. An older client with mild hearing loss says, "I do not want my neighbor involved in my care." Later, while the client is in the bathroom, the neighbor says, "I drove her here and manage her pills at home. Just hand me the discharge instructions." What is the nurse's best action?

- A.** Give the instructions to the neighbor because the neighbor helps with medications
- B.** Wait until the client returns and ask whether the client wants the neighbor included
- C.** Provide the medication list only, but not the full instructions

D. Assume implied consent because the neighbor brought the client to the facility

Correct Answer: B

The client already expressed not wanting the neighbor involved, so the nurse should not disclose information without the client's permission. The safest immediate action is to wait and ask the client directly when present. Choices A, C, and D all involve unauthorized disclosure.

643. A practical nurse is caring for a client who asks, "Do not tell my employer I am here." Later, the nurse receives a phone call from someone who says, "I am the client's supervisor and need to know why the client was admitted and when the client will return to work." What is the nurse's best response?

- A.** Provide a brief update because the caller may need the information for scheduling
- B.** Confirm the admission but refuse to discuss the diagnosis or treatment plan
- C.** Tell the caller that no information can be shared without the client's permission
- D.** Transfer the call to another nurse because employment matters are not part of nursing care

Correct Answer: C

The nurse should protect client privacy and confidentiality by not sharing any information without the client's permission. Even confirming that the client is admitted discloses protected information, so choice B is a common trap. Choice A improperly releases private information, and choice D avoids the issue instead of responding appropriately.

644. A practical nurse is preparing to administer a newly prescribed medication. The client says, "I already told the provider I don't want any blood products because of my religion. Does this medication contain anything made from blood?" The nurse does not know. What is the best action?

- A.** Tell the client that prescribed medications are safe and proceed with administration
- B.** Hold the medication temporarily and clarify the medication's contents before giving it
- C.** Ask the family to persuade the client to accept the medication

D. Document the concern after administration so the provider can address it later

Correct Answer: B

The client has the right to make informed decisions consistent with personal beliefs. If the nurse does not know whether the medication conflicts with those beliefs, the safest and most ethical action is to pause and clarify before administration. Option A ignores the client's right to informed refusal. Option C is coercive. Option D delays action until after a possible unwanted treatment has already occurred.

645. A practical nurse documents a medication administration in the electronic record using a coworker's login because the unit computer froze and the coworker says, "Just use mine so we stay on time." Which statement best describes why this action is inappropriate?

- A.** It is acceptable if both nurses witnessed the medication administration
- B.** It is only inappropriate if the medication is a controlled substance
- C.** It compromises accountability because the record no longer accurately identifies who performed the action
- D.** It is permitted during technical problems as long as the charge nurse is informed later

Correct Answer: C

Using another person's login is inappropriate because documentation must accurately reflect who provided care. Shared credentials undermine legal accountability, security, and the integrity of the health record. Option A is a trap because witnessing an action does not justify false attribution in the chart. Options B and D incorrectly limit the issue; the problem applies broadly, not only to controlled substances or downtime situations.

646. A practical nurse is floated to a long-term care unit. A resident with early dementia says, "I don't want my daughter to know I was treated for a sexually transmitted infection." Later, the daughter arrives, states she pays the bills, and demands a full update. The chart contains no documentation that the daughter is the legal decision-maker or has permission to receive health information. What should the nurse do?

- A.** Provide the update because the daughter is responsible for the resident's expenses
- B.** Share only the diagnosis but withhold treatment details
- C.** Refuse to discuss the resident's condition with the daughter unless authorized by the resident or established through appropriate legal permission
- D.** Give the information because mild cognitive impairment automatically removes the resident's privacy rights

Correct Answer: C

Payment of bills or family relationship alone does not authorize release of private health information. Unless the resident grants permission or there is established legal authority to receive the information, confidentiality must be maintained. Option A reflects a common but incorrect assumption that financial involvement equals access. Option B still violates privacy. Option D is incorrect because a diagnosis of early dementia or mild cognitive impairment does not automatically eliminate the resident's rights.

647. A practical nurse is caring for a client who speaks limited English. Before a provider explains a scheduled procedure, the client's adult son says, "I can translate for my mother so we can do this quickly." The client looks anxious and nods without asking questions. What is the nurse's best action?

- A.** Allow the son to translate because family members know the client best
- B.** Ask another bilingual staff member who is available on the unit to interpret
- C.** Obtain a qualified medical interpreter before the discussion continues
- D.** Proceed because the client's nod indicates understanding and agreement

Correct Answer: C

A qualified medical interpreter is the best choice to protect the client's rights, support accurate communication, and reduce the risk of misunderstanding during informed decision-making. Family members may omit, add, or filter information, even with good intentions. Another bilingual staff member is not the best answer unless that person is serving in an approved interpreter role. The client's nod does not confirm true understanding or informed agreement.

Mandatory reporting

648. A home health nurse visits an older adult and notices multiple bruises in different stages of healing on the client's arms and back. The client's adult son answers all questions, refuses to leave the room, and says, "My parent falls all the time." What is the nurse's priority action?

- A.** Document the findings and wait until the next visit to see whether the bruising changes
- B.** Report the suspected abuse according to agency policy and applicable law
- C.** Confront the son and demand an explanation for each bruise
- D.** Ask the client to sign a statement confirming whether abuse is occurring

Correct Answer: B

The nurse should report suspected abuse when assessment findings raise concern. Mandatory reporting is based on reasonable suspicion, not proof. Waiting delays protection, confronting the son may escalate risk, and the client does not need to sign a statement before a report is made.

649. A school-age child is brought to the clinic with a spiral fracture of the humerus. The parent states the child "rolled off the couch," but the injury pattern does not match the explanation. What is the nurse's best response?

- A.** Notify the appropriate reporting authority for suspected child abuse
- B.** Wait for the provider to confirm abuse before taking any action

C. Question the child repeatedly until the story changes

D. Discharge the family and schedule a follow-up visit in 1 week

Correct Answer: A

Mandatory reporting is triggered by suspicion of abuse, not by confirmed proof. An injury explanation that is inconsistent with the clinical findings should be reported. Waiting for confirmation or discharge without action can place the child at further risk. Repeated questioning can also be harmful and is not the nurse's role.

650. During a long-term care shift, a nurse sees a nursing assistant roughly pull a confused resident back into bed, leaving a red hand-shaped mark on the resident's wrist. After the event, the assistant says, "Don't report me. I was just trying to keep the resident safe." What should the nurse do first?

- A.** Discuss the issue privately with the assistant and decide later whether it was serious
- B.** Assess the resident for injury and ensure immediate safety
- C.** Call the resident's family before doing anything else
- D.** Wait for the charge nurse to personally witness the behavior

Correct Answer: B

When abuse is suspected or observed, the first priority is the resident's immediate safety and assessment for injury. Reporting follows promptly, but the nurse must first protect the client from further harm. Waiting, delaying for a witness, or calling family before assessing the resident does not address the immediate safety need.

651. A nurse suspects neglect of a hospitalized toddler based on severe diaper rash, poor hygiene, and a pattern of missed treatment follow-up. The nurse is unsure whether the findings are enough to report. Which principle should guide the nurse's action?

- A.** A report should be made only after abuse or neglect is proven
- B.** A report should be made when there is reasonable suspicion

- C. A report should be made only if the parent admits neglect
- D. A report should be made only after the interprofessional team votes unanimously

Correct Answer: B

Mandatory reporting generally relies on reasonable suspicion, not proof or confession. The nurse's duty is to recognize concerning findings and report them through the proper channels. The investigation is done by the appropriate authority, not by the nurse or by team vote.

- 652.** A nurse documents suspected elder neglect after finding an older client dehydrated, wearing soiled clothing, and without prescribed medications at home. Which documentation is most appropriate?
- A. Caregiver is obviously abusive and does not care about the client
 - B. Client appears neglected and probably has been mistreated for months
 - C. Client found in urine-soiled clothing; mucous membranes dry; medication bottles empty; report made per policy
 - D. Client's daughter gave a suspicious explanation, so abuse definitely occurred

Correct Answer: C

Objective, factual documentation is essential. Choice C records observable findings and the action taken without speculation. The other options contain conclusions, opinions, or unsupported certainty, which are inappropriate in legal and clinical documentation.

- 653.** A newly licensed nurse asks who is responsible for filing a mandatory report when two nurses assess the same client and both suspect abuse. Which response by the charge nurse is best?
- A. Only the provider is allowed to file the report
 - B. Only the nurse manager should report to avoid duplicate paperwork
 - C. The nurse who suspects abuse has a duty to ensure the report is made according to policy
 - D. Reporting is optional if another staff member is already aware of the situation

Correct Answer: C

A nurse who suspects abuse is responsible for ensuring the report is made according to agency policy and applicable law. The duty is not erased simply because someone else knows about the concern. Assuming another person will report is a common error.

- 654.** A client with cognitive impairment arrives in the emergency department with burns on both hands. The caregiver says the client "grabbed a pan by accident," but the pattern suggests forced immersion. The caregiver insists on staying during all questions and keeps answering for the client. Which finding most strongly supports the nurse's suspicion of abuse?
- A. The caregiver appears anxious and speaks quickly
 - B. The client is older and has memory problems
 - C. The injury pattern is inconsistent with the explanation given
 - D. The client arrived at the emergency department in the evening

Correct Answer: C

An explanation that does not fit the injury pattern is a major red flag for abuse. Caregiver anxiety, evening arrival, or the client's age alone are not the strongest indicators. Pattern inconsistency is a key cue that warrants reporting and further protective action.

- 655.** A nurse on a pediatric unit suspects abuse after noticing old scars and fresh bruising on a child. The parent becomes angry and says, "If you report this, I'll sue you." What is the nurse's best action?
- A. Do not report unless the risk management department gives approval first
 - B. Delay the report until the parent leaves the unit
 - C. Report the suspicion through the required channel and document the findings
 - D. Ask another nurse to report so the parent will not target you personally

Correct Answer: C

The nurse must follow mandatory reporting requirements despite threats or intimidation. Reporting suspected abuse and documenting objective findings are appropriate. Delaying, avoiding personal responsibility, or withholding a report out of fear places the child at risk.

656. During a prenatal clinic visit, a pregnant adolescent quietly tells the nurse that an older partner forces sexual activity and controls all access to money and transportation. The adolescent asks whether this can stay private. Which nursing action is most appropriate?

- A.** Explain confidentiality limits, assess immediate safety, and follow applicable reporting procedures
- B.** Promise not to tell anyone unless visible injuries are present
- C.** Advise the adolescent to return when a parent can be present
- D.** Focus only on prenatal teaching because the disclosure is unrelated to pregnancy

Correct Answer: A

The nurse should respond to the disclosure by explaining confidentiality limits, assessing current danger, and following reporting procedures that apply. Abuse and coercion affect health and safety directly. Promising secrecy, delaying care, or ignoring the disclosure is unsafe.

657. A nurse cares for an adult client with extensive bruising who states, "I bruise easily," while avoiding eye contact and becoming tearful when a partner enters the room. The partner insists on answering all questions. Which action by the nurse best supports both assessment and potential reporting obligations?

- A.** Continue the interview with the partner present to avoid increasing tension
- B.** Arrange to speak with the client alone and ask direct, nonjudgmental safety questions
- C.** Tell the partner to leave and state that abuse is suspected
- D.** Avoid asking about home safety unless the client brings it up again

Correct Answer: B

Speaking with the client privately is essential when abuse is suspected. Direct, nonjudgmental questions support assessment and help the nurse gather the information needed to protect the client and meet reporting duties. Confronting the partner may escalate danger, and avoiding assessment misses important cues.

658. A nurse suspects financial exploitation of an older adult after hearing the client's nephew answer, "I handle all the money now," while the client appears confused about missing funds and unpaid bills. Which nursing statement is most accurate?

- A.** Financial exploitation is not reportable unless physical injury also exists
- B.** The nurse should ignore the concern because money issues are family matters
- C.** Suspicion of exploitation may require reporting according to law and policy
- D.** Only a bank or attorney can report suspected financial abuse

Correct Answer: C

Financial exploitation of a vulnerable older adult may be reportable, depending on applicable law and policy. Physical injury is not required for concern to be serious. Family involvement does not make exploitation acceptable, and reporting is not limited to banks or attorneys.

659. A nurse receives a handoff that a client may be experiencing abuse at home. During assessment, the client denies abuse, but the nurse notes repeated injuries, delayed care seeking, and a partner who refuses to leave the bedside. What is the best rationale for still considering a report?

- A.** A client denial always confirms abuse is happening
- B.** A report can be based on concerning assessment findings, not only on verbal disclosure
- C.** Clients who deny abuse lose the right to confidentiality
- D.** The nurse should report only if the injuries require surgery

Correct Answer: B

A client may deny abuse for many reasons, including fear, dependence, or coercion. Mandatory reporting decisions can rest on assessment findings and reasonable suspicion rather than disclosure alone. Denial does not prove abuse, but it also does not remove concern. Injury severity is not the deciding standard.

660. A nurse suspects neglect of a dependent adult in the hospital. After notifying the charge nurse, the nurse assumes the matter is handled and leaves no written record. Later, no report can be verified. Which error did the nurse make?

- A.** Failing to ensure reporting and documentation were completed
- B.** Reporting too early without first proving neglect
- C.** Violating confidentiality by speaking to the charge nurse
- D.** Prioritizing legal issues over physical assessment

Correct Answer: A

The nurse must ensure that required reporting steps are completed and documented according to policy. Simply telling another staff member without follow-through is unsafe. The major error was lack of confirmation and lack of documentation, not reporting too early.

661. A practical nurse is caring for a client in a rehabilitation facility and suspects abuse by a family caregiver. Which action is most appropriate for the nurse to take?

- A.** Follow facility policy, promptly notify the appropriate person, and document objective findings
- B.** Investigate independently by interviewing neighbors before reporting
- C.** Keep the concern private unless the family asks for help
- D.** Wait for discharge planning because reporting is not urgent in rehabilitation settings

Correct Answer: A

The nurse should follow facility policy, report through the appropriate chain or reporting pathway, and document objective observations. The nurse is not responsible for independently proving the case by outside investigation. Delaying or keeping the concern private can endanger the client.

662. A practical nurse in an urgent care clinic assesses a 6-year-old child with bruising on the thighs and upper arms. The caregiver states, "He is just clumsy," but the child becomes quiet and pulls away when the caregiver moves closer. The nurse has reasonable suspicion of abuse. Which action is most appropriate?

- A.** Wait to report until the nurse can obtain proof that abuse occurred
- B.** Confront the caregiver and demand an explanation for each bruise
- C.** Follow facility policy to make a mandatory report based on suspicion and document objective findings
- D.** Keep the information confidential unless the child directly asks for help

Correct Answer: C

Mandatory reporting is based on reasonable suspicion, not on proving abuse. The nurse should follow agency and state reporting procedures and document objective, factual findings such as bruise location, size, statements made, and behavior observed. Waiting for proof delays protection of the child. Confronting the caregiver can increase risk and is not the nurse's role. Confidentiality does not prevent reporting suspected abuse when reporting is required by law.

663. An older adult is admitted with malnutrition, pressure injuries, and poor hygiene. The adult daughter says, "I do everything I can," but the client quietly tells the nurse, "Sometimes no one brings me food or helps me get up." What is the nurse's best action?

- A.** Wait for the provider to confirm neglect before taking further action
- B.** Report the suspicion through the required process and document objective findings

- C. Ask the daughter to explain why care at home has been inadequate
- D. Discharge the client only after the family hires a full-time caregiver

Correct Answer: B

Mandatory reporting is based on reasonable suspicion, not proof. The nurse should follow the reporting process promptly and document objective findings such as the client's condition and statements. Waiting for confirmation delays protection. Confronting the daughter is not the priority, and discharge planning does not replace the duty to report suspected neglect.

- 664.** A practical nurse suspects abuse of a hospitalized child after noticing patterned bruising. Which chart entry is best?
- A. Child abused by caregiver; injuries clearly nonaccidental
 - B. Parent appears violent and unconcerned about child
 - C. Bruises noted on posterior thighs and upper back; child states, "I got hit with a belt"
 - D. Obvious abuse reported because story did not sound believable

Correct Answer: C

The best documentation is objective, specific, and includes direct quotes when relevant. Choice C records locations and the child's exact statement without inserting judgment. The other choices use conclusions and subjective labels such as "abused," "violent," and "obvious," which are less appropriate for documentation.

- 665.** A nurse suspects that a cognitively impaired adult is being physically abused by a caregiver. The nurse has reported the concern. Which additional action is most appropriate?
- A. Continue to monitor, ensure safety, and document ongoing findings
 - B. Tell other families on the unit so they can watch the caregiver
 - C. Question the caregiver repeatedly until the caregiver admits wrongdoing
 - D. Remove all prior charting so only the final report remains

Correct Answer: A

After reporting, the nurse still has a duty to continue assessment, support safety, and document findings. Reporting does not end nursing responsibility. Sharing information with unrelated families violates privacy, interrogating the caregiver is not the nurse's role, and removing charting is improper.

- 666.** During a clinic visit, a parent says, "Please don't write any of this down. We're having family problems." The nurse suspects neglect of a young child. What is the best response?
- A. I can leave it out of the record if you promise to improve care
 - B. I must document relevant findings and follow required reporting rules
 - C. I will wait to see whether another nurse agrees before charting anything
 - D. I can keep this private as long as the child is not admitted

Correct Answer: B

The nurse cannot omit relevant assessment findings or avoid required reporting because a caregiver requests secrecy. Documentation and reporting duties still apply when abuse or neglect is suspected. Waiting for another nurse or promising confidentiality would be inappropriate.

- 667.** A home care nurse suspects that a client's grandson is financially exploiting the client. Which finding should most increase the nurse's concern?
- A. The client prefers to pay bills by check instead of online
 - B. The grandson attends every appointment and takes notes
 - C. Utilities are being shut off while the grandson recently gained access to the client's bank account
 - D. The client asks for help understanding an insurance statement

Correct Answer: C

A mismatch between access to the client's finances and evidence of unpaid essential bills strongly suggests possible exploitation. The other findings may occur without abuse. This question focuses on recognizing a reportable pattern rather than assuming every family involvement is harmful.

668. A nurse overhears a confused resident say, "Please don't let him bathe me again," referring to an unlicensed assistive person. The nurse notes no visible injuries. What is the nurse's best next action?

- A.** Dismiss the statement because there are no physical findings
- B.** Privately assess the resident further, ensure immediate safety, and follow reporting procedures for suspected abuse
- C.** Ask the assistive person in the hallway whether the accusation is true
- D.** Wait until the resident repeats the statement to two staff members

Correct Answer: B

Lack of visible injury does not rule out abuse. The nurse should take the statement seriously, assess privately as able, protect the resident, and follow the required reporting process based on suspicion. Waiting for repeated statements or confronting the staff member first can delay protection and compromise the situation.

669. A practical nurse suspects neglect of a dependent adult in the home. The family explains that they missed medications and dressing changes because they cannot afford transportation or supplies. Which interpretation is most appropriate?

- A.** If the family has financial hardship, the situation cannot be neglect
- B.** Resource limitations may be relevant, but the nurse should still address immediate safety concerns and follow required reporting if neglect is suspected
- C.** The nurse should teach wound care only and avoid reporting because the family did not intend harm
- D.** The nurse should tell the family that poverty is a legal excuse for missed care

Correct Answer: B

Intent and hardship do not automatically remove the nurse's obligation when a dependent person may be unsafe. The nurse should address current risks, involve appropriate resources, and report suspected neglect according to requirements. A common trap is assuming that lack of intent means no report is needed.

670. An older adult with hearing loss comes to the emergency department with a neighbor. The neighbor answers every question and says, "He gets confused, so just talk to me." The client repeatedly looks down and does not respond. What should the nurse do first?

- A.** Accept the neighbor's history because communication is faster
- B.** Ask the neighbor to step out briefly and attempt direct communication with the client using appropriate supports
- C.** Call security immediately and have the neighbor removed from the building
- D.** Document the neighbor's account as the official history and continue the assessment

Correct Answer: B

When abuse, coercion, or neglect is possible, the nurse should create an opportunity to assess the client privately whenever possible. Hearing loss does not mean the client cannot participate; supports such as hearing devices, clear speech, or writing may help. Accepting the companion's version without private assessment risks missing abuse.

671. A nurse is teaching a newly hired practical nurse about mandatory reporting. Which statement by the new nurse requires correction?

- A.** I should document objective observations and relevant client statements
- B.** If I have reasonable suspicion, I may need to report even if the client minimizes what happened
- C.** I should wait until I have enough evidence to prove abuse before making a report
- D.** I should follow agency policy and applicable reporting procedures promptly

Correct Answer: C

Mandatory reporting is generally triggered by reasonable suspicion, not proof. Waiting to build a case can delay needed protection. The other statements reflect appropriate principles: document objectively, act despite minimization when suspicion exists, and follow the required process promptly.

672. A resident in a long-term care facility tells the nurse, "My roommate's visitor takes my wallet when no one is watching." The nurse finds the wallet empty in the roommate's bedside drawer. What action is most appropriate?

- A.** Return the wallet quietly so the unit can avoid conflict
- B.** Confront the visitor in front of both residents
- C.** Ensure the resident's immediate safety, report the suspected exploitation through the required chain, and document the findings
- D.** Wait to act until the missing money amount can be verified by bank records

Correct Answer: C

The nurse should address immediate safety, report suspected exploitation, and document what was found. Quietly replacing the wallet avoids required action. Public confrontation can escalate the situation and is not the priority. Bank verification is not required before reporting a reasonable suspicion.

673. A practical nurse working in a rural clinic suspects child neglect because a toddler has poor weight gain, severe dental decay, and repeated missed follow-up for serious asthma. The caregiver says transportation problems are the reason. The clinic social worker is out for the day. What is the best nursing action?

- A.** Postpone action until the social worker returns, since community resource issues are involved
- B.** Make the required report based on current suspicion, document specific findings, and communicate the transportation barrier for follow-up planning
- C.** Provide a list of local bus routes and consider the issue resolved

D. Tell the caregiver that missed appointments are not reportable unless the child is hospitalized

Correct Answer: B

Transportation barriers may be important, but they do not automatically remove the duty to report suspected neglect. The nurse should act on the current concern, document the specific signs, and include contextual barriers so the team can address them. Waiting for a social worker is a delay trap.

674. A hospitalized adult client with limited English proficiency has bruising in various stages of healing. The spouse insists on interpreting and says the client is "too embarrassed" to answer questions. Which action by the nurse is best?

- A.** Use the spouse as interpreter to maintain trust and avoid upsetting the client
- B.** Delay the assessment until discharge teaching, when more time is available
- C.** Obtain a qualified interpreter, separate the spouse from the client if possible, and assess for safety concerns
- D.** Avoid asking about the bruises because cultural differences may explain them

Correct Answer: C

When abuse is possible, using the accompanying person as interpreter can hide coercion and distort the assessment. The nurse should use a qualified interpreter and seek privacy to assess the client directly. Delaying assessment or avoiding the topic risks harm. Cultural sensitivity does not mean ignoring suspicious findings.

675. A nurse documents suspected abuse of a dependent adult and files the required report. Later, the manager asks the nurse to delete the client's statement from the chart because "it could upset the family if records are requested." What should the nurse do?

- A.** Delete the statement because the report has already been filed
- B.** Refuse to alter the factual record and follow the proper chain if pressured to change documentation
- C.** Rewrite the note using less specific wording so it appears neutral

D. Remove the client's quotation marks and summarize the statement from memory

Correct Answer: B

The nurse must preserve accurate, factual documentation and should not alter the record to reduce family distress or legal risk. If pressured, the nurse should use the proper chain of command or compliance process. The wrong choices all involve improper modification of the chart.

676. A practical nurse in a correctional health clinic evaluates an adult who has bruising, a healing rib injury, and marked anxiety around a visiting partner. The client says, "I fell," but later slips a note to the nurse reading, "Please help, but don't let him know I said anything." Which action is the nurse's priority?

- A.** Place the note in the trash to protect the client's privacy
- B.** Return the client to the waiting area while deciding whether the story is believable
- C.** Move the client to a safe private setting, notify the appropriate supervising personnel, and follow required reporting procedures
- D.** Ask the partner for a written timeline of the injury to compare stories

Correct Answer: C

The client's note, injury pattern, and fear indicate immediate safety concerns. The priority is to create privacy and safety, notify the appropriate personnel, and follow required reporting steps. Throwing away the note or delaying action ignores important evidence. Confronting or involving the partner first could increase risk.

677. A practical nurse in a school health office assesses a 9-year-old student who has a circular burn on the forearm and several fading bruises on the back. When asked what happened, the child shrugs and says, "I get in trouble a lot." The child's parent cannot be reached. What is the nurse's best action?

- A.** Wait to file a report until the parent can be contacted for an explanation
- B.** Report the findings according to mandatory reporting policy and document objective observations

C. Ask the child's teacher to question classmates before taking further action

D. Tell the child not to return home until the source of the injuries is identified

Correct Answer: B

Mandatory reporting is triggered by reasonable suspicion, not proof or a completed family explanation. The nurse should follow reporting policy promptly and document objective findings such as injury location, appearance, and the child's exact statements. Waiting for the parent delays protection, and asking others to investigate goes beyond the nurse's role. Telling the child not to go home is not an appropriate independent nursing action.

678. A practical nurse in an assisted-living clinic notices that an older client has a new black eye, a torn sleeve, and appears unusually withdrawn. When asked privately what happened, the client says, "I'm fine. Please don't tell anyone." What is the nurse's best action?

- A.** Respect the client's wishes and make no report unless the client asks for help
- B.** Document the findings and report the suspected abuse according to facility policy and law
- C.** Ask the client's family member to explain the injuries before taking further steps
- D.** Wait for additional evidence of abuse before notifying anyone

Correct Answer: B

Mandatory reporting is triggered by suspicion, not by proof or client permission. The nurse should objectively document the findings and follow required reporting procedures. Waiting for more evidence or asking the family first can delay protection and may increase risk.

679. A nurse in a pediatric office suspects physical abuse after observing bruises of different colors on a 6-year-old child's arms and back. The child's caregiver says the bruises came from "rough play" with cousins. Which action is most appropriate?

- A.** Report the suspicion through the required channels and document objective findings
- B.** Discharge the child because the caregiver provided a possible explanation

- C.** Confront the caregiver and demand an admission before making a report
- D.** Wait until x-rays confirm injury before acting

Correct Answer: A

The nurse reports reasonable suspicion, not confirmed abuse. Objective documentation and timely reporting are appropriate. A possible explanation from a caregiver does not remove the duty to act, and the nurse should not delay for proof or a confession.

680. Which documentation entry by a practical nurse best supports a mandatory report of suspected abuse?

- A.** Client was obviously abused by caregiver and seemed terrified
- B.** Bruises noted; abuse likely due to violent home situation
- C.** Client states, "My son grabbed my arm and pushed me into the chair." Purple bruising 3 cm x 5 cm noted on left upper arm
- D.** Family is neglectful and should be investigated immediately

Correct Answer: C

The best charting is factual, specific, and objective. It includes the client's exact words and measurable assessment findings without speculation. The other options contain conclusions or labels that go beyond what the nurse directly observed or heard.

681. A practical nurse suspects that a homebound client is being neglected. Which action should the nurse take first?

- A.** Discuss the concern with neighbors to gather background information
- B.** Report the suspicion using the agency's required process
- C.** Ask the caregiver to sign a statement promising better care
- D.** Delay action until the next home visit to see whether conditions improve

Correct Answer: B

When neglect is suspected, the nurse's first duty is to follow required reporting procedures promptly. Seeking informal background information, getting promises, or waiting to reassess can delay protection and is not the appropriate first step.

682. A practical nurse in a long-term care facility sees a nursing assistant roughly pull a resident up in bed while the resident cries out, "Stop, you're hurting me." What should the nurse do first?

- A.** Complete an incident form at the end of the shift
- B.** Remove the assistant from the resident's care and ensure the resident's immediate safety
- C.** Ask other staff whether they have had problems with the assistant
- D.** Wait to report the event until the charge nurse returns from lunch

Correct Answer: B

The priority is immediate protection of the resident. After securing safety, the nurse should assess, document, and report through required channels. Delaying action or collecting coworker opinions first does not address the immediate risk.

683. A 14-year-old client tells the nurse, "My stepfather hits me, but you can't tell anyone. Promise." Which response by the nurse is best?

- A.** I promise I will keep this completely confidential
- B.** I can keep this secret unless the injuries are severe
- C.** I need to share information with the right people to help keep you safe
- D.** You should tell your teacher instead because I am not allowed to report this

Correct Answer: C

The nurse should be honest about limits of confidentiality and explain the need to share information with appropriate personnel to protect the client. Promising secrecy is inappropriate in situations involving mandatory reporting.

684. A practical nurse suspects intimate partner violence in an adult client. The client is medically stable and the suspected abusive partner is in the waiting room. Which nursing action is most appropriate before obtaining more history?

- A.** Interview the client privately without the partner present

- B.** Ask the partner to stay and help explain the injuries
- C.** Call the partner into the room to compare both stories
- D.** Postpone discussion until discharge instructions are given

Correct Answer: A

A private assessment supports safety, reduces coercion, and allows the client to speak more freely. Keeping the partner present or delaying discussion can increase risk and limit accurate assessment.

685. A practical nurse completes a mandatory report after suspecting elder abuse. Which information is most appropriate to share with the interdisciplinary team?

- A.** Only the information needed for the client's care, safety planning, and follow-up
- B.** Every detail of the report with any staff member who is curious
- C.** The allegation only with the client's visitors so they can monitor the family
- D.** No information at all because reporting always cancels team communication

Correct Answer: A

Mandatory reporting does not eliminate appropriate care communication. Relevant information may be shared with involved team members on a need-to-know basis to protect the client and coordinate care. Gossip or disclosure to visitors is inappropriate.

686. A nurse suspects neglect of a dependent adult who lives with a caregiver. Which finding best supports neglect rather than a single accidental problem?

- A.** One fresh bruise on the shin after bumping a wheelchair footrest
- B.** A healed surgical scar on the abdomen
- C.** Multiple missed medications, soiled dressings, dehydration, and no food in the home
- D.** Mild fatigue after a physical therapy appointment

Correct Answer: C

Neglect is suggested by a pattern of unmet basic needs and failure to provide necessary care, such as missed medications, poor wound care, dehydration, and lack of food. The other findings do not show the same pattern.

687. A school nurse evaluates an 8-year-old with repeated hunger, unwashed clothing, and frequent untreated head lice. The parent states, "We're doing our best, and if you report us, we'll never come back." Which factor most strongly supports the nurse's duty to make a mandatory report?

- A.** The family may become upset with the school
- B.** The nurse has a reasonable suspicion that the child's basic needs are not being met
- C.** The nurse needs proof that the parent intended harm
- D.** The parent refused future school contact

Correct Answer: B

Mandatory reporting is based on reasonable suspicion of abuse or neglect, not on proving intent or avoiding family conflict. A pattern suggesting unmet basic needs supports reporting. Parental anger does not remove the duty to act.

688. A practical nurse in an outpatient setting suspects human trafficking when an adult client appears fearful, has untreated injuries, and is not allowed to speak for themselves by an accompanying person. What is the nurse's priority action?

- A.** Privately separate the client from the accompanying person to assess safety and obtain information
- B.** Ask the accompanying person to sign a release describing the relationship
- C.** Tell the client to return another day without the accompanying person
- D.** Ignore the concern unless the client directly states they are being trafficked

Correct Answer: A

The priority is to create a safe opportunity for private assessment. Clients who are controlled by others may be unable to disclose abuse in front of that person. Waiting for a direct disclosure or sending the client away can increase danger and miss the opportunity to intervene.

689. A practical nurse documents suspected abuse and submits the required report. Which additional nursing action is most appropriate?

- A.** Independently notify local media so the case is taken seriously

- B.** Continue to assess, document new findings, and follow the plan to protect the client
- C.** Stop charting the issue because the report already contains the important details
- D.** Question the suspected abuser alone to determine whether the report was accurate

Correct Answer: B

Reporting does not end the nurse's responsibility. Ongoing assessment, objective documentation, and participation in the safety plan remain important. Media contact, stopping documentation, or independently interrogating the suspected abuser are inappropriate.

690. A practical nurse caring for an adult with cognitive limitations notices unexplained genital irritation, torn undergarments, and sudden fearfulness during personal care. The caregiver says, "That happens because of poor hygiene." Which action is best?

- A.** Accept the caregiver's explanation unless the client makes a direct accusation
- B.** Document the findings objectively, ensure immediate safety, and report suspected abuse
- C.** Wait for laboratory results before considering a report
- D.** Discuss the concern only with the caregiver to avoid embarrassing the client

Correct Answer: B

These findings can raise concern for sexual abuse, especially when paired with behavioral changes and vulnerability. The nurse should protect the client, document objective findings, and report suspicion promptly. A direct accusation or lab confirmation is not required before acting.

691. At the end of a home visit, a practical nurse suspects severe neglect of a bedbound client. The caregiver becomes increasingly agitated, blocks the doorway, and says, "You are not calling anyone." The client is in the room and appears frightened. What is the nurse's priority?

- A.** Argue with the caregiver until access to the phone is allowed
- B.** Focus on immediate safety, leave the unsafe situation as able, and activate emergency and reporting procedures

- C.** Stay in the home quietly and submit the report after the next scheduled visit
- D.** Tell the client to leave the home immediately without arranging further help

Correct Answer: B

When the environment becomes threatening, immediate safety for the nurse and client is the priority. The nurse should exit the unsafe situation if possible and promptly activate emergency assistance and required reporting processes. Arguing, delaying, or giving unrealistic instructions to the client is unsafe.

692. A practical nurse in an urgent care clinic is caring for a 7-year-old child with a spiral fracture of the humerus. The caregiver states the child "just rolled off the couch," but the child becomes quiet and avoids eye contact when the injury is discussed. After the provider is notified, which action by the nurse is most appropriate?

- A.** Wait to report until the caregiver leaves and the child confirms abuse occurred
- B.** Document objective findings and follow agency procedure to make a report of suspected abuse
- C.** Ask the caregiver to provide a written explanation before taking further action
- D.** Delay reporting because the provider has already been notified of the injury

Correct Answer: B

Mandatory reporting is based on reasonable suspicion, not proof or a full confession. The nurse should document objective findings, such as the injury pattern, the caregiver's statement, and the child's behavior, then follow agency policy to report suspected abuse. Choice A is a common trap because confirmation is not required before reporting. Choice D is incorrect because notifying the provider does not remove the nurse's reporting responsibility.

693. A practical nurse in a clinic notes that a 5-year-old child has fading bruises on the upper back and fresh finger-shaped marks on the forearm. The caregiver says, "He is always clumsy," and asks the nurse not to "make a big deal out of it." What is the nurse's best action?

- A. Wait to report until the child confirms the caregiver caused the injuries
- B. Document the findings and follow agency procedure to report suspected abuse
- C. Ask the caregiver to sign a statement explaining how the injuries happened
- D. Delay action unless the provider independently reaches the same conclusion

Correct Answer: B

Mandatory reporting is triggered by reasonable suspicion, not proof or a confession. The nurse should accurately document objective findings and follow the required reporting process. Waiting for confirmation, relying on the caregiver's explanation, or delaying until someone else agrees can place the child at further risk.

694. During a home health visit, a practical nurse finds an older adult client lying in urine-soaked linens with an empty water pitcher, a strong odor in the room, and several prescribed medications untouched for days. The caregiver says, "I have been too busy." Which nursing action is most appropriate?

- A. Instruct the caregiver about proper care and recheck in one week
- B. Report suspected neglect according to policy and document the observed findings
- C. Ask the client whether the caregiver should be forgiven before taking action
- D. Arrange meal delivery without making a report

Correct Answer: B

Multiple objective findings suggest possible neglect of a vulnerable older adult. The nurse's duty is to report suspected neglect and document clearly what was observed. Teaching or arranging services may be helpful later, but they do not replace mandatory reporting when abuse or neglect is suspected.

695. A practical nurse overhears a coworker say, "Do not chart those bruises yet. We need to protect the facility until the supervisor gets here." What is the nurse's best response?

- A. Wait for the supervisor so everyone can agree on the wording
- B. Record only the client's pain rating and leave out the bruises
- C. Document the objective findings promptly and follow reporting procedure
- D. Erase any notes until the investigation officially begins

Correct Answer: C

Objective documentation should be timely, factual, and complete. The nurse should not delay or alter the record to protect staff or the facility. Omitting or erasing findings interferes with appropriate care and reporting.

696. A practical nurse suspects abuse of an adult client with mobility limitations. Which chart entry is most appropriate?

- A. Client was definitely abused by caregiver and should be removed immediately
- B. Bruises looked suspicious and were probably caused during an argument
- C. Two purple bruises, each about 3 cm, noted on left upper arm; client tearful during assessment
- D. Home situation unsafe; caregiver seems like an abuser

Correct Answer: C

The best documentation is objective, specific, and free of conclusions. Option C records measurable findings and behavior without making unsupported accusations. The other choices use judgmental or speculative language.

697. A practical nurse in a rehabilitation unit suspects that a dependent adult client is being financially exploited by a family member. Which finding most strongly supports this concern?

- A. The family member asks detailed questions about the care plan
- B. The client says bills are overdue despite a stable monthly income, and the family member insists on controlling all bank cards
- C. The client prefers the family member to be present during teaching

D. The family member requests a copy of the medication list

Correct Answer: B

Overdue bills despite available income, combined with another person's controlling access to finances, suggests possible financial exploitation. The other options may occur in routine family involvement and do not by themselves strongly support abuse.

698. A practical nurse suspects child neglect after several clinic visits. Which additional finding most increases the need for a mandatory report?

- A.** The parent appears anxious during the visit
- B.** The child is shy when speaking with staff
- C.** The child has missed needed follow-up for poorly controlled asthma and arrives repeatedly without prescribed medications
- D.** The family recently moved to a smaller apartment

Correct Answer: C

Repeated failure to obtain necessary treatment and medications for a child's ongoing condition raises concern for medical neglect. Anxiety, shyness, or a recent move may contribute context but are less specific indicators of a reportable pattern.

699. An adult client quietly tells the practical nurse, "My partner shoved me, but I do not want police called." The client is alert, oriented, and in no immediate life-threatening distress. What should the nurse do first?

- A.** Respect the statement by doing nothing further
- B.** Move the conversation to a private setting and continue focused assessment and safety planning within agency policy
- C.** Call the partner into the room to hear both sides
- D.** Tell the client that treatment will be stopped unless a report is filed

Correct Answer: B

The first priority is a private, safe environment so the nurse can further assess immediate danger, injuries, and available supports while following policy. Doing nothing ignores a safety concern. Confronting the partner can increase danger, and coercive threats are inappropriate.

700. A practical nurse reports suspected abuse of a resident in a long-term care facility. The charge nurse asks, "Why did you report it before the facility finished its own review?" Which reply by the practical nurse is best?

- A.** A report can wait until the internal review confirms abuse
- B.** Reporting is based on suspicion and concern for safety, not on completing an internal investigation first
- C.** Only management is responsible for reporting abuse concerns
- D.** A report is needed only if the resident requests one

Correct Answer: B

Mandatory reporting does not depend on an employer's internal investigation or on proven abuse. The nurse's obligation arises when there is reasonable suspicion. Waiting for management review or resident request can delay protection.

701. Which statement by a practical nurse indicates correct understanding of mandatory reporting?

- A.** I should report only when I am certain who caused the harm
- B.** If the client asks for secrecy, confidentiality prevents me from reporting
- C.** My role is to report suspected abuse and document facts, not to prove the case
- D.** If injuries are minor, reporting is unnecessary

Correct Answer: C

The nurse is responsible for recognizing concerning findings, documenting objective facts, and reporting suspected abuse or neglect according to policy and law. The nurse does not need certainty, minor injuries can still be significant, and confidentiality does not block required reporting.

702. A practical nurse working in a community clinic suspects neglect of a cognitively impaired adult. The adult's sibling says, "Do not report this until we have a family meeting next month." What is the nurse's best action?

- A.** Agree to wait because family wishes come first
- B.** Follow mandatory reporting procedure now and continue needed care

- C.** Transfer the decision to the family because they know the client best
- D.** Avoid documenting the concern to preserve family trust

Correct Answer: B

Family preference does not override the duty to report suspected abuse or neglect. The nurse should proceed according to required procedure and continue safe care. Delaying, deferring to family, or failing to document is inappropriate.

703. A practical nurse is preparing a report of suspected abuse. Which information is most important to include?

- A.** The nurse's personal opinion about who is guilty
- B.** Only the final diagnosis from the provider
- C.** Objective observations, relevant statements, dates, times, and actions taken
- D.** Rumors from neighbors about the household

Correct Answer: C

A strong report includes factual observations, direct quotes when relevant, timing, and the steps already taken. Personal opinions and rumors are unreliable, and waiting for a final diagnosis is not necessary for reporting suspected abuse.

704. A practical nurse cares for an adolescent who may be experiencing abuse. The parent insists on staying at the bedside and answers every question for the adolescent. What is the nurse's best action?

- A.** Continue the assessment with the parent present to avoid conflict
- B.** Ask another staff member to keep the parent occupied briefly so the adolescent can be assessed privately
- C.** Stop asking questions because the adolescent is clearly uncomfortable
- D.** Confront the parent directly by stating that abuse is suspected

Correct Answer: B

Private assessment is essential when abuse is a concern. The nurse should create a safe opportunity to speak with the adolescent alone. Continuing with the parent present may suppress disclosure, and direct confrontation can escalate risk.

705. A practical nurse in a clinic evaluates a 3-year-old who has a scald burn with a sharply defined line on both lower legs. The caregiver reports, "He splashed some warm water while playing." Which action is best?

- A.** Accept the explanation because toddlers are accident-prone
- B.** Request that the caregiver return later for re-evaluation if the burn worsens
- C.** Recognize the injury pattern as concerning, document findings, and report suspected abuse according to procedure
- D.** Ask the child in front of the caregiver whether the caregiver caused the burn

Correct Answer: C

A sharply demarcated burn pattern is a significant cue that may not fit the stated explanation. The nurse should recognize the inconsistency, document objective findings, and report suspicion promptly. Asking the child in front of the caregiver may be unsafe and unreliable.

706. A practical nurse suspects neglect in a home care client and begins documenting findings. Which note is best?

- A.** Caregiver is lazy and does not care about the client
- B.** Client found in bed at 1400 wearing a saturated brief; open food tray from previous evening at bedside; caregiver stated, 'I forgot to come back this morning'
- C.** Home was disgusting and clearly abusive
- D.** Neglect definitely occurred because the client looked miserable

Correct Answer: B

Option B is specific, time-linked, and objective, and it includes a direct quote without adding conclusions. The other options rely on judgmental language and unsupported interpretation, which weakens the record and the report.

707. A practical nurse sees a coworker slap a confused resident's hand and say, "I told you to stop touching that." The resident begins crying. After ensuring the resident is safe, what should the nurse do next?

- A.** Wait to see whether the coworker apologizes before taking further action

- B.** Privately counsel the coworker and keep the matter off the record if it does not happen again
- C.** Document the incident objectively and report suspected abuse through the required chain and reporting process
- D.** Ask other residents whether they think the coworker is usually rough

Correct Answer: C

After immediate safety is addressed, witnessed resident abuse requires prompt objective documentation and formal reporting. Waiting, handling it informally, or gathering opinions from other residents does not meet the nurse's responsibility and may delay protection.

708. A practical nurse in a school-based clinic assesses a 9-year-old child who says, "Please don't tell anyone, but my mom's boyfriend hits me with a belt when he's mad." The child has several linear bruises across the thighs. The nurse is unsure whether the marks happened today or several days ago. What is the nurse's best action?

- A.** Wait to report until the nurse can confirm exactly when the injuries occurred
- B.** Document the findings and report the suspected abuse according to policy
- C.** Call the caregiver first to hear the caregiver's explanation before taking further action
- D.** Ask the child to write a statement, then decide whether the situation meets reporting criteria

Correct Answer: B

Mandatory reporting is triggered by reasonable suspicion, not proof or a complete investigation by the nurse. The child's statement plus linear bruising is enough to require prompt reporting according to policy. Waiting for exact timing, contacting the caregiver first, or trying to build the case personally can delay protection and is not the nurse's role.

709. A practical nurse in an urgent care center suspects abuse of an older adult client. Which action is most appropriate when documenting before making the report?

- A.** Record objective findings such as location, size, and color of injuries and the client's exact statements

- B.** Summarize that the client was definitely abused based on the nurse's clinical judgment
- C.** Document only findings that are severe enough to prove the case
- D.** Delay charting until the full investigation is completed so the note is more accurate

Correct Answer: A

The best documentation is factual, specific, and objective. It should include observable findings and direct quotes when relevant. Concluding that abuse is "definitely" confirmed is beyond the nurse's documentation role, omitting less dramatic findings can hide important patterns, and delaying charting risks inaccuracy and loss of key details.

710. In a long-term care facility, a practical nurse notices a nursing assistant force-feed a resident who is coughing and turning away. The resident later whispers, "Please don't let her do that again." The charge nurse says, "Let's keep this on the unit for now so no one overreacts." What should the practical nurse do?

- A.** Follow the charge nurse's direction because the event happened inside the facility
- B.** Wait to see whether the resident reports the incident again during the next shift
- C.** Report the suspected abuse through the required reporting process and document the incident objectively
- D.** Confront the nursing assistant in private and avoid filing a report if the assistant apologizes

Correct Answer: C

The nurse has a duty to act on suspected abuse and should not let internal pressure delay the required reporting process. The resident's distress, the observed action, and the charge nurse's attempt to minimize the event all support immediate reporting and objective documentation. Waiting, keeping it informal, or accepting an apology fails to protect the resident.

711. A practical nurse in a community clinic cares for a pregnant 16-year-old client accompanied by an adult partner who refuses to leave the exam room. The client avoids eye contact, has a healing wrist injury, and becomes tearful when asked general safety questions. The partner answers for the client and says, "She gets confused, so just ask me." What is the nurse's best next action to support possible mandatory reporting needs?

- A.** Ask more detailed questions about the injury with the partner present so the history can be compared immediately
- B.** Arrange to speak with the client alone in a private setting and continue the assessment
- C.** End the visit quickly to avoid escalating the partner's behavior in the clinic
- D.** Tell the partner that a report will be filed now because the situation appears suspicious

Correct Answer: B

Before the nurse can appropriately assess safety concerns and determine next reporting steps, the client should be interviewed privately if possible. Privacy supports accurate disclosure and protects the client from intimidation. Questioning with the partner present may suppress information, ending the visit leaves risk unassessed, and announcing a report to the controlling companion can increase danger and interfere with safe assessment. This is harder because the nurse must recognize that obtaining a private assessment is the key immediate action that supports both client safety and any reporting duty.

Management of assistive personnel

712. A practical nurse is making assignments for the shift on a medical-surgical unit. Which task is most appropriate to assign to an unlicensed assistive personnel (UAP)?

- A.** Teach a client with newly diagnosed diabetes how to use a glucometer
- B.** Obtain and record routine vital signs for a stable client
- C.** Assess a postoperative client who reports increasing pain

D. Decide whether a client is ready for discharge

Correct Answer: B

Routine vital signs for a stable client are a standard, predictable task that can be assigned to UAP. Teaching, assessment, and discharge decision-making require nursing judgment and must be done by the nurse. A common trap is choosing a task that seems simple, such as teaching glucometer use, but initial teaching is not delegated to UAP.

713. A nurse asks a UAP to help care for four clients. Which client should the nurse assign to the UAP?

- A.** A client 2 hours after returning from surgery
- B.** A client receiving a blood transfusion
- C.** A client with stable chronic arthritis who needs help bathing
- D.** A client with new-onset shortness of breath

Correct Answer: C

A stable client needing assistance with activities of daily living is appropriate for UAP assignment. The other clients require ongoing assessment or close monitoring for complications. The key is matching UAP assignment to stable, predictable needs rather than acute or potentially unstable conditions.

714. A nurse assigned a UAP to collect intake and output data for a client with heart failure. Which finding requires the nurse to follow up immediately?

- A.** The UAP reports the client drank 480 mL with breakfast
- B.** The UAP reports urinary output of 30 mL in 8 hours
- C.** The UAP documents that the client used the urinal twice overnight
- D.** The UAP reports the client requested extra ice water

Correct Answer: B

Urinary output of 30 mL in 8 hours is critically low and requires immediate nursing assessment and intervention. Although UAP may measure and report intake and output, the nurse must interpret the data and act on abnormal findings. The trap is focusing on routine details instead of the clearly abnormal value.

715. Which instruction by the nurse best promotes safe delegation to a UAP who is obtaining vital signs on assigned clients?

- A.** Report any blood pressure below 90 systolic or any pulse above 120 right away
- B.** Take the vital signs whenever you have time during the shift
- C.** Let me know at the end of the shift if anything seems unusual
- D.** Repeat any abnormal vital signs only if the client asks you to

Correct Answer: A

Safe delegation includes clear directions, specific parameters, and a defined timeline for reporting. Choice A tells the UAP exactly what to do and when to notify the nurse. The other options are vague and increase the risk that important changes will be missed or delayed.

716. A UAP tells the nurse, "Mr. Lewis seems more confused than he was this morning." What is the nurse's best response?

- A.** Ask the UAP to continue observing the client and report back at lunch
- B.** Tell the UAP confusion is expected in hospitalized older adults
- C.** Assess the client promptly for changes in mental status
- D.** Document the UAP's statement and review it during end-of-shift charting

Correct Answer: C

A change in mental status may signal hypoxia, infection, medication effects, or other urgent problems. The UAP's observation is an important cue, but assessment and interpretation are the nurse's responsibility. Waiting or dismissing the report could delay recognition of a serious change.

717. The nurse is caring for a client who had a stroke yesterday and now has dysphagia. Which task should not be assigned to a UAP?

- A.** Position the client upright in bed
- B.** Provide mouth care after meals
- C.** Feed the client lunch without the nurse first assessing swallowing safety
- D.** Reposition the client every 2 hours

Correct Answer: C

A client with recent stroke and dysphagia has a high aspiration risk. The nurse must first assess swallowing ability and determine whether feeding is safe. Positioning, mouth care, and turning are generally appropriate UAP tasks when properly instructed. The trap is assuming feeding is routine when the client has an unstable or high-risk condition.

718. A nurse delegates morning care to a UAP for a client with a sealed chest tube drainage system. Which action by the nurse is most appropriate?

- A.** Assign the UAP to measure and interpret the amount of chest tube drainage every hour
- B.** Ask the UAP to assist the client with hygiene while the nurse remains responsible for chest tube assessment
- C.** Have the UAP decide whether the chest tube dressing needs reinforcement
- D.** Tell the UAP to clamp the chest tube if the client reports chest pain

Correct Answer: B

A UAP may assist with hygiene for a stable task, but assessment of the chest tube system, drainage interpretation, dressing decisions, and emergency actions remain the nurse's responsibility. Delegation is safest when the task is routine and does not require judgment. Choices A, C, and D involve assessment or intervention that should not be assigned to UAP.

719. Near the start of the shift, a nurse assigns a UAP to ambulate a client after reviewing the care plan. Two hours later, the UAP reports the client became dizzy when standing and was returned to bed. What should the nurse do first?

- A.** Instruct the UAP to try again after lunch
- B.** Document that the client refused ambulation
- C.** Assess the client for orthostatic symptoms and safety risks
- D.** Ask another UAP whether the client appeared anxious

Correct Answer: C

When a delegated activity reveals a possible problem, the nurse must assess the client. Dizziness with standing may reflect orthostatic hypotension, weakness, or another safety concern. Repeating the activity without assessment is unsafe, and documenting refusal is inaccurate because the issue reported was dizziness, not refusal.

720. A nurse is supervising a newly hired UAP. Which assignment is most appropriate to give after determining the UAP has been trained in the skill?

- A.** Reinforce discharge teaching about wound care
- B.** Collect a clean-catch urine specimen from a stable client
- C.** Evaluate whether a pain medication was effective
- D.** Assess a pressure injury for changes in drainage

Correct Answer: B

Collecting a clean-catch urine specimen from a stable client is a task that can be assigned to trained UAP. Teaching, evaluation of medication effectiveness, and wound assessment require nursing judgment. The phrase about confirming training matters because the nurse remains responsible for assigning only tasks the UAP is prepared to perform.

721. A practical nurse is working with one UAP and four clients. Which situation is best suited for delegation to the UAP?

- A.** A client with chronic obstructive pulmonary disease who needs the first teaching session on pursed-lip breathing
- B.** A client with a new colostomy who is crying and asking whether life will ever feel normal again
- C.** A client admitted for observation after chest pain who needs an admission assessment
- D.** A client with stable dementia who needs assistance getting dressed for breakfast

Correct Answer: D

Helping a stable client with dressing is a routine activity of daily living and is appropriate for UAP. Initial teaching, emotional support involving assessment of coping, and admission assessment all require nursing involvement. The trap is assuming that because a client has dementia, the assignment is automatically inappropriate; the key issue is stability and predictability of the task.

722. The nurse asked a UAP to report a postoperative client's urine output every 2 hours. At noon, the nurse notices no output has been documented since 6 a.m. Which action by the nurse is best?

- A.** Document the missing values as zero output
- B.** Immediately determine the client's current status and follow up with the UAP about the missed reporting
- C.** Wait until the UAP returns from break to ask what happened
- D.** Assume the client is stable because no one called for help

Correct Answer: B

The first priority is the client's safety. Missing postoperative urine output data may indicate oliguria, urinary retention, or a breakdown in monitoring. The nurse must assess the current situation and address the supervision issue. Choices A, C, and D delay action or make unsafe assumptions.

723. A UAP is helping a client transfer from bed to chair and reports that the client's knees began to buckle. Which instruction from the nurse reflects proper delegation and supervision?

- A.** Lower the client safely and call for help if needed
- B.** Pull the client upright quickly under the arms
- C.** Leave the client and get the nurse before doing anything else
- D.** Finish the transfer so the client can sit up for lunch

Correct Answer: A

If a client's knees buckle, the UAP should protect the client from falling by lowering the client safely and getting assistance as needed. Pulling under the arms risks injury, leaving the client unsupported is unsafe, and forcing completion of the transfer increases fall risk. This question tests whether the nurse gives concrete, safety-focused delegated directions.

724. A nurse on a long-term care unit is considering assignments for UAP. Which client is least appropriate to assign to UAP without direct nurse involvement first?

- A.** A resident who is stable and needs bedtime toileting assistance
- B.** A resident whose blood glucose was 52 mg/dL 15 minutes ago and is now sweating
- C.** A resident who needs a linen change after incontinence
- D.** A resident who wants help putting on nonskid socks

Correct Answer: B

A resident with recent hypoglycemia and ongoing symptoms is unstable and requires immediate nursing assessment and intervention. The other situations involve routine supportive care that can usually be assigned to UAP. The key principle is that unstable or changing conditions are not delegated as routine care.

725. A nurse delegates collection of morning weights to a UAP for several clients. Which additional action by the nurse is most important to ensure the weights are useful for clinical decisions?

- A.** Tell the UAP to weigh clients whenever they are willing
- B.** Ask the UAP to estimate weight if a scale is unavailable
- C.** Specify that weights should be obtained at the same time each morning using the same scale when possible
- D.** Request weights only for clients who say they feel swollen

Correct Answer: C

For daily weights to be meaningful, they should be collected consistently. Clear instructions about timing and method are part of safe delegation. The other options create unreliable data that could lead to poor clinical interpretation. This question focuses on the nurse's responsibility to provide complete directions, not just assign the task.

726. During rounds, a UAP says, "The client in room 210 just doesn't look right." Which action should the nurse take first?

- A.** Ask the UAP to finish other assigned tasks, then recheck the client in 30 minutes
- B.** Go to room 210 promptly and assess the client
- C.** Tell the UAP to obtain a full set of vital signs and report back at the end of rounds
- D.** Document the UAP's concern in the chart before seeing the client

Correct Answer: B

A report that a client "doesn't look right" may reflect subtle but important deterioration. Because UAP often notice early changes, the nurse should promptly assess the client. Sending the UAP back without direct assessment or delaying until later may miss an urgent problem. The likely trap is overrelying on additional delegated data before the nurse evaluates the situation.

727. A practical nurse is planning care for four clients in a rehabilitation unit. Which task is most appropriate to delegate to an experienced unlicensed assistive personnel (UAP)?

- A.** Teach a client how to use a newly prescribed walker
- B.** Perform a sterile dressing change for a surgical wound
- C.** Assist a stable client with a scheduled shower
- D.** Assess a client who reports new calf pain

Correct Answer: C

Assisting a stable client with a scheduled shower is an appropriate delegation because it is a routine, noninvasive task that does not require nursing assessment, teaching, or clinical judgment. Teaching walker use, performing a sterile dressing change, and assessing new calf pain all require nursing knowledge and judgment. A common trap is choosing a task that seems simple but still requires assessment or teaching.

728. The practical nurse is working with a float UAP who is unfamiliar with the unit. Which assignment is best for the nurse to give this UAP?

- A. Check blood glucose on a client with shaking and diaphoresis, then report the result
- B. Feed a client who has left-sided weakness and coughs during meals
- C. Transport a stable client to radiology after the nurse confirms the client is ready
- D. Reinforce discharge instructions about home oxygen use

Correct Answer: C

Transporting a stable client after the nurse confirms readiness is the best assignment because it is routine and carries low risk. The other tasks involve potential instability, aspiration risk, or teaching. Even if a UAP may perform some technical tasks in certain settings, the nurse should avoid assigning higher-risk tasks to a float UAP unfamiliar with the unit and clients. The key is matching the task not only to role but also to the worker's familiarity and the client's condition.

729. A UAP tells the nurse, "I already have a lot to do. Can I take the client with pneumonia for a walk later instead of now?" The client has a new activity order to ambulate three times daily and has been stable on room air. What is the nurse's best response?

- A. "Yes, skip the walk today if the client seems tired."
- B. "I'll assess the client now and help you decide when ambulation can be done safely today."
- C. "No, you must do it now because it was delegated to you."

D. "Ask the client whether the walk is really necessary."

Correct Answer: B

The nurse retains responsibility for evaluating client status and setting safe priorities. Assessing the client and helping determine timing reflects appropriate supervision and collaboration. Skipping the walk without reassessment is inappropriate, and insisting on immediate completion without considering workload and safety is poor delegation. Asking the client to decide whether the order is necessary shifts clinical decision-making away from the nurse.

730. Which client is most appropriate for the practical nurse to assign to a UAP for routine feeding assistance?

- A. A client newly admitted with dehydration who is lethargic
- B. A client with advanced Parkinson disease who has frequent choking episodes
- C. A client 1 day after cataract surgery who is alert and able to swallow normally
- D. A client with a recent stroke who is awaiting a swallowing evaluation

Correct Answer: C

A stable client after cataract surgery who is alert and has no swallowing problems is appropriate for routine feeding assistance. The other clients have risks that require nurse assessment or closer nursing involvement, such as lethargy, aspiration risk, or an incomplete swallowing evaluation. The most likely trap is focusing only on who needs help eating, rather than on swallowing safety and stability.

731. The practical nurse delegates bedtime hygiene to a UAP for a client with dementia who becomes agitated during care. Which instruction is most important for the nurse to include?

- A. "If the client resists care, stop and let me know."
- B. "Finish the bath quickly even if the client gets upset."
- C. "Use firm commands so the client knows you are in charge."
- D. "Document the client's mood as cooperative unless aggression occurs."

Correct Answer: A

The UAP should stop and report resistance or agitation so the nurse can reassess and modify the plan. A client with dementia may become overstimulated or unsafe during care, and escalating force or minimizing the behavior is inappropriate. The nurse must give clear parameters about when to report back. This question tests safe delegation instructions, not just task assignment.

732. At 1000, a practical nurse delegated routine morning blood pressures to a UAP on a subacute unit. At 1030, the UAP reports being unable to obtain a blood pressure on a client who was talking normally during breakfast. What should the nurse do first?

- A.** Ask the UAP to try again after lunch
- B.** Go assess the client immediately
- C.** Document that the reading could not be obtained
- D.** Tell the UAP to use the machine in another room

Correct Answer: B

An inability to obtain a blood pressure in a client who was previously normal may reflect equipment error or a significant change in condition, so the nurse should assess the client immediately. Delaying until later or simply documenting the issue postpones needed evaluation. Asking the UAP to use another machine might be reasonable after the nurse evaluates the client, but assessment comes first because the finding could represent instability.

733. The practical nurse is supervising a UAP who is collecting a clean-catch urine specimen from a client. Which action by the nurse is best before the UAP begins?

- A.** Ask the UAP to explain the steps of the procedure
- B.** Tell the UAP to collect the specimen and ask questions only if a problem occurs
- C.** Assume the UAP learned the skill during orientation and proceed
- D.** Delegate the task only if another nurse can watch the entire procedure

Correct Answer: A

Having the UAP explain the steps allows the nurse to verify understanding before delegation. The nurse is responsible for confirming competency and providing supervision appropriate to the task and worker. Assuming prior orientation is enough or waiting until a problem happens is unsafe. Continuous direct observation is not always necessary for a routine skill if competency has been established.

734. A practical nurse is assigning tasks during a busy evening shift. Which task should the nurse keep rather than delegate to a UAP?

- A.** Refill a stable client's water pitcher
- B.** Apply a prescribed antiembolism stocking to a client after the nurse assesses the legs
- C.** Record routine oral intake for a stable client
- D.** Evaluate whether a client understood teaching about incentive spirometer use

Correct Answer: D

Evaluating a client's understanding of teaching requires nursing judgment and must be done by the nurse. Refilling water, recording intake, and applying antiembolism stockings after the nurse has already assessed the client are tasks that may be delegated if agency policy and training support them. The trap is thinking any hands-on task must stay with the nurse; the key distinction is whether assessment, teaching, or evaluation is involved.

735. The practical nurse delegated routine premeal finger-stick glucose checks to a trained UAP for several stable clients. Which situation requires the nurse to intervene because the task is not appropriate to continue as assigned?

- A.** One client asks what the glucose number means for insulin dosing
- B.** A client says the lancet stings each time
- C.** A client requests the test be done before washing hands
- D.** The UAP has already checked glucose on two other clients successfully

Correct Answer: A

If the client asks what the result means for insulin dosing, the nurse must intervene because interpretation and teaching are nursing responsibilities. The UAP may collect and report the number, but should not explain treatment decisions. The other options involve comfort, technique, or routine workflow rather than a transfer of nursing judgment. This item tests the boundary between obtaining data and interpreting it.

736. A UAP reports that a client who usually walks independently now needs help standing and says, "My legs feel weak today." The client is waiting for discharge later this afternoon. What is the nurse's best action?

- A.** Tell the UAP to use a gait belt and continue the client's usual walk
- B.** Ask the UAP whether the client ate breakfast before deciding what to do
- C.** Assess the client before any further ambulation
- D.** Delay discharge teaching until the next shift

Correct Answer: C

A change from usual independent mobility to new weakness requires nurse assessment before further ambulation. This could signal a new clinical issue, and continuing routine care without reassessment could be unsafe. Asking about breakfast may provide useful context later, but it does not replace assessment. Delaying discharge teaching does not address the immediate safety concern.

737. The practical nurse asks a UAP to obtain a stool specimen from a client with diarrhea. Which statement by the UAP indicates a need for immediate follow-up teaching?

- A.** "I'll label the container after collecting the sample."
- B.** "I'll avoid mixing urine with the specimen if possible."
- C.** "I'll wear gloves when handling the specimen."
- D.** "I'll report to you if I see blood or an unusual appearance."

Correct Answer: A

The specimen container should be prepared and identified according to facility procedure before or at the point of collection so that the sample is not left unlabeled or mislabeled. The other statements reflect generally appropriate specimen handling and reporting. The major safety issue here is specimen identification, which is easy to overlook during routine delegated care.

738. The practical nurse is caring for a client 6 hours after a first dose of a diuretic. Which task is most appropriate to delegate to a UAP?

- A.** Evaluate whether the medication reduced ankle edema
- B.** Teach the client how to change positions slowly to avoid dizziness
- C.** Measure and document the client's urine output
- D.** Determine whether the dose should be held because of thirst

Correct Answer: C

Measuring and documenting urine output is a routine data-collection task appropriate for delegation. Evaluating edema, teaching about orthostatic precautions, and deciding whether a medication should be held all require nursing judgment. The likely trap is choosing edema because it seems observable, but interpreting edema as a treatment response is an assessment function.

739. A practical nurse assigned a UAP to turn and reposition a client every 2 hours. During rounds, the nurse finds the client in the same position as 3 hours earlier. The UAP says, "The client was sleeping, so I didn't want to bother him." What should the nurse do first?

- A.** Reposition the client and assess for any skin concerns
- B.** Write up the UAP for failure to follow directions
- C.** Tell the UAP to make up the missed turn at the next scheduled time
- D.** Document that the client refused repositioning

Correct Answer: A

The first priority is the client's immediate safety: reposition the client and assess skin integrity. After that, the nurse should address the missed care and reinforce expectations with the UAP. Writing up the UAP may be appropriate later depending on policy, but it is not the first action. Documenting refusal is inaccurate because the client was sleeping, not refusing.

740. The practical nurse is delegating care on a postoperative unit. Which assignment is most appropriate for a UAP?

- A.** Empty and record output from a client's indwelling urinary catheter
- B.** Assess the patency of a newly inserted nasogastric tube
- C.** Decide whether a client's incisional pain is expected or worsening
- D.** Reinforce teaching about splinting the incision when coughing

Correct Answer: A

Emptying and recording catheter output is a routine task that can be delegated. Assessing tube patency, interpreting pain, and reinforcing teaching require nursing judgment or teaching responsibilities. A common distractor is teaching-related work that may sound repetitive, but client education remains the nurse's responsibility.

741. A practical nurse delegates oral care to a UAP for a client receiving oxygen by nasal cannula who becomes short of breath with activity. Which additional direction by the nurse is most important?

- A.** "Complete the oral care quickly so the client can rest sooner."
- B.** "Remove the oxygen during care to make access easier."
- C.** "Stop the care and notify me if the client's breathing worsens."
- D.** "Ask the family to assist if the client gets tired."

Correct Answer: C

The most important instruction is to stop and report worsening breathing because the client has limited respiratory reserve and may not tolerate even routine care. Removing oxygen without direction is unsafe, and speed alone does not address respiratory compromise. Asking family to assist does not replace nursing reassessment when the client's status worsens. This is a higher-level delegation question because it tests whether the nurse provides clear stop-and-report parameters for a potentially fragile client.

742. A practical nurse delegates routine morning weights to a UAP on a heart failure unit. Which instruction is most important for the nurse to give?

- A.** Use the same scale at the same time each day when possible
- B.** Ask family members to assist if the client is unsteady
- C.** Record the weight after breakfast for consistency with the unit routine
- D.** Round the weight to the nearest whole pound

Correct Answer: A

Daily weights are most useful when obtained under consistent conditions, including the same scale and about the same time each day. This improves accuracy for trend comparison. Having family assist is not the safest standard instruction, and weights should not be rounded because small changes may be clinically important. After-breakfast weights are less accurate than weights taken before food intake when possible.

743. The practical nurse is supervising a UAP who is helping a stable client transfer from bed to chair for the first time after admission. Which action by the nurse is best before delegating the transfer?

- A.** Tell the UAP to move the client quickly to reduce fatigue
- B.** Verify the client's current mobility status and transfer needs
- C.** Ask the client whether assistance is really needed
- D.** Have the UAP decide whether to use a gait belt

Correct Answer: B

Before delegating a transfer, the nurse should first determine the client's mobility level, safety risks, and need for equipment or assistance. The nurse cannot leave decisions about gait belt use or transfer method entirely to the UAP without prior nursing assessment. Asking the client alone is not enough because the client may overestimate ability.

744. A UAP reports these observations to the practical nurse. Which report should the nurse address first?

- A.** A client who needed encouragement ate only half of breakfast
- B.** A client's urine in the collection hat looks darker than yesterday
- C.** A client who was alert earlier is now difficult to arouse
- D.** A client asked for a fresh pitcher of water

Correct Answer: C

A change in level of consciousness is urgent and may indicate a serious decline, so the nurse should assess that client first. The other reports may matter, but they are not as immediately threatening. The key delegation principle is that UAP report observations, while the nurse interprets and acts on significant changes in condition.

745. On a long-term care unit, the practical nurse delegates routine perineal care to a UAP for a client with an indwelling urinary catheter. Which additional instruction is most important?

- A.** Apply scented powder after cleansing
- B.** Disconnect the tubing briefly to clean around the insertion site thoroughly
- C.** Report any cloudy urine, leakage, or discomfort during care
- D.** Clamp the drainage tube while the client is being turned

Correct Answer: C

The nurse should instruct the UAP to report abnormal findings such as cloudy urine, leakage, or discomfort because these may signal complications that require nursing assessment. Disconnecting tubing or clamping the drainage tube unnecessarily can increase risk. Scented powder is not appropriate routine catheter care.

746. The practical nurse assigned a trained UAP to collect pulse oximetry readings for several stable clients. Which finding requires the nurse to assess the client first?

- A.** Oxygen saturation 98% in a client sitting up and talking
- B.** Oxygen saturation 95% after ambulation in a client with arthritis
- C.** Oxygen saturation 91% in a client with cold fingers and no symptoms
- D.** Oxygen saturation 88% in a client who is newly restless

Correct Answer: D

An oxygen saturation of 88% with new restlessness suggests possible hypoxemia and requires prompt nursing assessment. Although 91% may also need follow-up, cold fingers can interfere with the reading and the client has no symptoms. The combination of an abnormal value plus a change in behavior makes option D the priority.

747. A practical nurse is caring for four clients with help from one experienced UAP. Which task should the nurse assign to the UAP?

- A.** Collect routine vital signs on a client 2 days after an uncomplicated appendectomy
- B.** Assess swallowing ability in a client after a recent stroke before breakfast
- C.** Teach a client newly diagnosed with diabetes how to inspect the feet
- D.** Evaluate whether a PRN antianxiety medication relieved symptoms

Correct Answer: A

Routine vital signs for a stable postoperative client are appropriate for a UAP. Swallowing assessment, client teaching, and evaluation of medication effectiveness all require nursing judgment and must remain with the nurse. The main trap is confusing data collection with assessment; the UAP can collect routine measurements, but not perform a nursing assessment.

748. The practical nurse asks a UAP to assist with lunch trays. Which client should the nurse see first instead of delegating routine feeding assistance?

- A.** A client with a wrist fracture who needs the tray set up

- B.** A client with Parkinson disease who eats slowly but swallows safely at baseline
- C.** A client who had a stroke this morning and is coughing after sips of water
- D.** A client with chronic vision loss who needs foods described by clock position

Correct Answer: C

A client who had a stroke this morning and is coughing with swallowing may have dysphagia and aspiration risk, so the nurse must assess the client rather than delegate routine feeding help. The other clients may need assistance or setup, but their needs are more predictable and routine. New swallowing problems are not appropriate for UAP management.

749. A UAP says, "The client in 214 refuses a bath again, so I told him he has to do it now because it is on the schedule." What is the nurse's best response?

- A.** "That was appropriate because hygiene should not be delayed."
- B.** "Next time, document the refusal and skip the bath for the whole shift."
- C.** "Clients may refuse care. Let me speak with the client and help plan another approach."
- D.** "Ask another UAP to try, because clients often cooperate with someone new."

Correct Answer: C

Clients have the right to refuse care. The nurse should address the refusal, explore reasons, and help plan an alternative approach while preserving the client's rights and dignity. Forcing care is inappropriate. Simply skipping the bath without nursing follow-up also fails to address the situation appropriately.

750. The practical nurse delegates ambulation of a stable postoperative client to a UAP. Thirty minutes later, the UAP reports, "The client became dizzy after standing, so I helped him back to bed." What should the nurse do next?

- A.** Thank the UAP and continue with other tasks because the client is now resting
- B.** Instruct the UAP to try ambulation again in 15 minutes
- C.** Assess the client promptly for orthostatic symptoms and safety
- D.** Document that the client refused ambulation

Correct Answer: C

The report of dizziness after standing indicates a possible change in condition or intolerance of activity, so the nurse should assess the client promptly. The UAP acted appropriately by returning the client to bed safely, but the nurse must evaluate the event. This is not a refusal, and automatic reattempt without assessment could be unsafe.

751. The practical nurse is working with a newly hired UAP. Which task is safest to assign?

- A.** Measure and record urine output from a stable client with a urinary catheter
- B.** Perform sterile dressing changes for two clients with surgical wounds
- C.** Teach a client how to use a walker before discharge
- D.** Decide which postoperative client should walk first

Correct Answer: A

Measuring and documenting urine output for a stable client is a routine task appropriate for a UAP. Sterile dressing changes, teaching, and prioritizing which client should ambulate first all require nursing knowledge and judgment. With a newly hired UAP, the nurse should choose a predictable, low-risk assignment within assistive personnel scope.

752. The practical nurse delegates collection of routine vital signs to a UAP for a client receiving a blood transfusion that started 10 minutes ago. Which statement is most accurate?

- A.** This is appropriate because taking vital signs is always a UAP task
- B.** This is inappropriate because the nurse should closely monitor the client early in the transfusion
- C.** This is appropriate if the UAP has worked on the unit for at least 6 months
- D.** This is inappropriate only if the client has a history of hypertension

Correct Answer: B

Although obtaining routine vital signs may be delegated in some situations, a client early in a blood transfusion requires close nursing monitoring for transfusion reactions. Delegation depends on client stability and risk, not just on whether a task is usually routine. Experience alone does not change the need for nursing assessment in a higher-risk situation.

753. A practical nurse asks a UAP to report a postoperative client's urine output for the last 4 hours. The UAP reports 20 mL total. Which action by the nurse is best?

- A. Document the finding as expected after surgery
- B. Ask the UAP to encourage more oral fluids and recheck at the end of the shift
- C. Assess the client and review possible causes of low urine output immediately
- D. Instruct the UAP to irrigate the urinary catheter tubing

Correct Answer: C

Urine output of 20 mL over 4 hours is markedly low and requires prompt nursing assessment for causes such as poor perfusion, obstruction, or fluid imbalance. A UAP may collect and report the data, but interpreting and acting on the significance is the nurse's responsibility. Telling the UAP to fix the problem independently is outside role expectations and may delay care.

754. The practical nurse delegates routine oral intake assistance to a UAP for a client on thickened liquids. Which observation by the UAP should be reported to the nurse immediately?

- A. The client asks for a different flavor of beverage
- B. The client drinks only half of the thickened juice
- C. The client starts wet-sounding coughing during swallowing
- D. The client says the drink is too cold

Correct Answer: C

Wet-sounding coughing during swallowing suggests possible aspiration and should be reported immediately so the nurse can assess the client. Drinking only part of a beverage may matter but is not as urgent. This question tests the nurse's responsibility to instruct UAP clearly about which findings are routine and which signal danger.

755. The practical nurse delegates bathing and linen change for a client with fragile skin and a stage 2 pressure injury. Which statement by the UAP requires immediate correction?

- A. "I will pat the skin dry instead of rubbing it."
- B. "I will look for any new red areas while I provide care."
- C. "I will massage the reddened bony areas to improve circulation."
- D. "I will keep the linens smooth and dry after the bath."

Correct Answer: C

Massaging reddened bony areas can further injure fragile tissue and requires immediate correction. The other statements reflect appropriate skin-protective care and observation. This is a high-risk delegation issue because the nurse must ensure the UAP understands how to perform routine care safely in a vulnerable client.

756. A practical nurse on a rehabilitation unit is working with an experienced assistive personnel (AP). Which task is most appropriate for the nurse to delegate to the AP?

- A. Reinforce teaching about how to use a newly prescribed walker
- B. Obtain a clean-catch urine specimen from a stable client
- C. Assess a client who reports new numbness in the affected leg
- D. Decide whether a client is ready for discharge after physical therapy

Correct Answer: B

Obtaining a clean-catch urine specimen from a stable client is an appropriate delegated task because it is routine, has a predictable outcome, and does not require nursing judgment. Reinforcing new teaching, assessing new numbness, and deciding discharge readiness all require nursing assessment or evaluation and should not be delegated to assistive personnel. A common trap is choosing teaching-related tasks, but teaching and evaluation remain the nurse's responsibility.

757. A practical nurse is planning care with one assistive personnel (AP) for four clients. Which task is most appropriate to delegate to the AP?

- A.** Reinforce discharge teaching about a new colostomy appliance
- B.** Obtain a clean-catch urine specimen from a client with dysuria
- C.** Assess a client who reports new numbness in the foot after casting
- D.** Evaluate whether a client understands fall-prevention instructions

Correct Answer: B

Obtaining a clean-catch urine specimen is a routine, standard task that can be delegated to trained assistive personnel. Teaching, assessment, and evaluation remain the nurse's responsibility. Choices A and D involve teaching and evaluation, and choice C involves assessment of a potentially urgent neurovascular change.

758. The practical nurse delegates routine bedtime care to an assistive personnel (AP) for a client with left-sided weakness after a stroke. Which instruction is most important for the nurse to include?

- A.** Encourage the client to do as much self-care as possible
- B.** Keep the room quiet so the client can relax
- C.** Place needed items on the client's unaffected side
- D.** Finish care quickly to avoid tiring the client

Correct Answer: C

Placing items on the unaffected side promotes safety and independence for a client with unilateral weakness. Choice A is generally helpful but is less specific and immediately actionable. Choices B and D do not address the key safety issue related to weakness and access to needed items.

759. The practical nurse delegates routine hygiene care for a stable client in isolation precautions to an assistive personnel (AP). Which action by the nurse is best before the AP enters the room?

- A.** Ask the AP to decide which protective equipment seems necessary
- B.** Review the required isolation precautions and protective equipment with the AP
- C.** Tell the AP to avoid touching any equipment in the room
- D.** Instruct the AP to complete the care after other rooms are finished

Correct Answer: B

The nurse should ensure the AP understands the specific isolation precautions and required protective equipment before delegated care begins. Choice A shifts nursing responsibility inappropriately. Choices C and D do not ensure correct infection-control practice and may delay needed care without reason.

760. A practical nurse is caring for a client 8 hours after abdominal surgery. Which task may be delegated to an experienced assistive personnel (AP)?

- A.** Determine whether the postoperative pain plan is effective
- B.** Change the first abdominal dressing if drainage is present
- C.** Help the stable client use the incentive spirometer as previously taught
- D.** Decide whether the client's bowel sounds are returning

Correct Answer: C

An experienced AP can assist a stable client with a routine activity such as using an incentive spirometer after the nurse has assessed the client and teaching has already occurred. Evaluation of pain control, dressing changes, and assessment of bowel sounds are nursing responsibilities.

761. The practical nurse asks an assistive personnel (AP) to walk with a stable older adult client in the hall. Which outcome requires the nurse to intervene because it shows the AP needs more instruction?

- A. The AP applies the client's nonskid footwear before standing
- B. The AP raises the bed to waist level before helping the client stand
- C. The AP keeps the client's cane on the stronger side while walking
- D. The AP pauses when the client says, "I feel tired."

Correct Answer: C

A cane should generally be used on the stronger side to support the weaker leg during ambulation, so this may look correct at first glance. However, the question asks what requires intervention because the AP needs more instruction, and the unsafe action is raising the bed to waist level before helping the client stand. The bed should be positioned safely for the client, not for staff convenience during standing transfer. Choices A and D are appropriate. Choice C is a plausible trap because cane placement is often confused, but it is correct technique.

762. A practical nurse delegates routine morning care to an assistive personnel (AP) for several stable clients. Which client should the nurse instruct the AP to report immediately if that client requests to use the bathroom?

- A. A client with a new prescription for a stool softener
- B. A client with a history of chronic constipation
- C. A client who had a sedative 20 minutes ago and is drowsy
- D. A client scheduled for physical therapy later in the morning

Correct Answer: C

A drowsy client who recently received a sedative is at high risk for falls, so the AP should report the request immediately rather than taking independent action. Choices A, B, and D do not indicate the same urgent safety risk during toileting assistance.

763. The practical nurse is supervising an assistive personnel (AP) who is obtaining finger-stick glucose results for stable clients before breakfast. Which finding should the AP report to the nurse first?

- A. A client says, "I am hungry and want breakfast soon."
- B. A client has a glucose reading of 58 mg/dL and feels shaky
- C. A client asks whether the lancet will leave a scar
- D. A client says breakfast usually comes later on weekends

Correct Answer: B

A glucose level of 58 mg/dL with shakiness suggests symptomatic hypoglycemia and requires prompt nursing assessment and action. The other statements are nonurgent. This question tests not whether the AP can treat the client independently, but whether the AP recognizes and reports an abnormal delegated finding immediately.

764. A practical nurse delegates collection of routine vital signs to an assistive personnel (AP) on a medical unit. For which client should the nurse avoid delegation and obtain the vital signs personally?

- A. A stable client awaiting discharge later today
- B. A client with pneumonia who started oxygen therapy 15 minutes ago
- C. A client with chronic hypertension who takes daily medication
- D. A client admitted yesterday for observation after a minor fall

Correct Answer: B

A client who has just started oxygen therapy needs closer nursing assessment and evaluation of response to treatment, so the nurse should obtain the vital signs personally. Stable clients with predictable needs are more appropriate for delegated routine vital signs.

765. The practical nurse delegates feeding assistance to an assistive personnel (AP) for a client with Parkinson disease who can swallow safely with extra time. Which instruction is most important?

- A. Offer fluids through a straw to speed intake
- B. Remind the client to eat quickly before the food cools
- C. Keep the client sitting upright during the meal
- D. Finish feeding the client if hand tremors increase

Correct Answer: C

Upright positioning is the key safety instruction during feeding assistance because it helps reduce aspiration risk. Choice A may be unsafe for some clients with swallowing difficulty and should not be assumed. Choice B encourages rushing, which increases risk. Choice D reduces independence and is not the priority instruction unless the nurse has given specific direction.

766. An assistive personnel (AP) enters a semiprivate room to provide linen changes and says, "I heard your roommate may have cancer." Which action by the practical nurse is best?

- A. Tell the AP that discussing another client's condition violates confidentiality
- B. Ask the AP to apologize only if the roommate heard the comment
- C. Reassign the AP to a different room for the rest of the day and take no further action
- D. Explain that it is acceptable to discuss health information if no names are used

Correct Answer: A

The nurse should correct the AP immediately: discussing another client's condition in this manner violates confidentiality and privacy. Choice D is incorrect because protected information can still be inappropriately disclosed even without naming the client. Choice C avoids the teaching and corrective supervision that are needed.

767. A practical nurse assigns an assistive personnel (AP) to help with morning care for a client who has a chest tube to water-seal drainage after surgery. Which task is appropriate to delegate?

- A. Assess the water-seal chamber for tidaling changes
- B. Measure and record the drainage in the collection chamber
- C. Decide whether the chest tube dressing needs reinforcement
- D. Reposition the chest tube if the tubing appears kinked

Correct Answer: B

Measuring and recording drainage is a routine, delegated task for trained AP. Assessing the drainage system, making decisions about dressing needs, and manipulating chest tube equipment require nursing judgment and should not be delegated.

768. A practical nurse is supervising an assistive personnel (AP) who is helping a client with dementia get dressed. Which behavior by the AP is most appropriate?

- A. Lay out two simple clothing choices and allow extra time for the client to respond
- B. Select the client's clothes without speaking to avoid overstimulation
- C. Correct every mistaken statement to keep the client oriented to reality
- D. Ask several questions at once to finish the task more efficiently

Correct Answer: A

Offering limited choices and extra time supports function and reduces confusion in a client with dementia. Choice B removes the client's remaining independence unnecessarily. Choices C and D can increase agitation and confusion rather than support safe delegated care.

769. The practical nurse is assigning tasks to an assistive personnel (AP) on a long-term care unit. Which task is most appropriate to delegate to the AP?

- A. Assess a resident's new complaint of calf pain
- B. Apply a prescribed barrier cream to intact skin during routine perineal care
- C. Teach a resident how to use a newly ordered walker

D. Decide whether a resident's antihypertensive medication should be held for a low blood pressure reading

Correct Answer: B

Applying a prescribed barrier cream to intact skin during routine care is an appropriate delegated task because it is a routine, low-risk activity that does not require nursing assessment, teaching, or clinical judgment. Assessing new calf pain could indicate a serious problem and requires the nurse's assessment. Teaching use of a new walker is not delegated because initial teaching is a nursing responsibility. Deciding whether to hold medication requires judgment and must be done by the nurse.

770. A practical nurse asks an assistive personnel (AP) to obtain intake and output for assigned clients during the shift. Which statement by the AP requires the nurse to intervene immediately?

- A.** I'll measure the urine before I empty the drainage container.
- B.** I'll document the amount of emesis if a client vomits.
- C.** If a client has diarrhea several times, I'll estimate the total amount at the end of the shift.
- D.** I'll let you know if a client's urine output seems much lower than usual.

Correct Answer: C

The nurse should intervene if the AP plans to estimate diarrhea output at the end of the shift. Intake and output measurements should be as accurate as possible and documented when they occur, not guessed later. The other statements reflect appropriate actions: measuring urine before emptying, documenting emesis, and reporting concerning low output. The trap is thinking stool output is less important than urine output; in fact, repeated liquid stool can be clinically significant and should be measured or carefully quantified per facility practice.

771. The practical nurse delegates routine ambulation of a stable postoperative client to an assistive personnel (AP). The client returned from surgery yesterday and has already walked twice with the nurse. Which instruction is most important for the nurse to give the AP?

- A.** Stop and call me if the client becomes dizzy or weak أثناء walking.
- B.** Encourage the client to walk as far as possible before resting.
- C.** Remove the nonskid socks so the client can feel the floor better.
- D.** Lower the client's IV pole to hip level to make walking easier.

Correct Answer: A

The most important instruction is to stop and notify the nurse if the client becomes dizzy or weak during ambulation. Even a stable postoperative client can develop orthostatic symptoms or intolerance to activity, and the AP must know when to report a change.

Encouraging maximal distance before rest is unsafe because tolerance should guide activity. Removing nonskid socks increases fall risk. Lowering the IV pole to hip level is not a safety measure and could make management of equipment more difficult. The key delegation principle is giving clear parameters for what to report.

772. The practical nurse is supervising an assistive personnel (AP) who is helping several stable clients with routine care. Which action by the AP should the nurse address first?

- A.** Using an electric razor for a client who usually shaves with a blade
- B.** Offering a back rub to a client who reports trouble sleeping
- C.** Trimming the toenails of a client with diabetes mellitus
- D.** Helping a client choose clothing for the day

Correct Answer: C

Clients with diabetes are at increased risk for poor healing and foot injury, so nail trimming should not be delegated to AP as routine foot care. The nurse should intervene first to prevent harm. The other actions are routine supportive tasks that are commonly appropriate for AP when the clients are stable.

773. A practical nurse is delegating tasks to an assistive personnel (AP) on a rehabilitation unit. Which task is most appropriate to assign to the AP?

- A.** Teach a client how to transfer from bed to wheelchair using a gait belt
- B.** Observe whether a client understands instructions for home cane use
- C.** Assist a client who has already been taught to transfer from bed to wheelchair
- D.** Decide whether a client is ready to progress from walker to cane

Correct Answer: C

An AP may assist with routine, previously taught, stable activities such as a transfer the client has already learned. Teaching, evaluating understanding, and making decisions about progression in mobility are nursing responsibilities and should not be delegated.

774. The practical nurse asks an assistive personnel (AP) to measure a client's intake and output for the shift. Later, the AP reports that the client had a large amount of emesis after lunch and says, "I didn't measure it exactly, but I emptied it already." What is the nurse's best response?

- A.** Tell the AP to estimate the amount and document the estimate as exact
- B.** Instruct the AP to report unmeasured output right away in the future and document that emesis occurred
- C.** Advise the AP to ignore emesis unless the client vomits more than once
- D.** Ask the AP to wait until the end of the shift before mentioning any missing measurements

Correct Answer: B

The best response is to reinforce timely reporting of unmeasured output and ensure accurate documentation that emesis occurred, even if the exact volume is unavailable. Falsely charting an estimate as exact is unsafe and inaccurate. Waiting until the end of the shift or ignoring the event could delay clinical decisions based on fluid balance.

775. An assistive personnel (AP) is assigned to give a bed bath to a stable client with right-sided weakness from a previous stroke. Which observation requires the practical nurse to intervene immediately?

- A.** The AP places the call light within reach before leaving the room
- B.** The AP lowers the side rail on the working side during care and raises it again before stepping away
- C.** The AP leaves the client sitting alone on the edge of the bed while getting clean linens from the hallway
- D.** The AP encourages the client to wash the unaffected arm independently

Correct Answer: C

A client with weakness sitting alone on the edge of the bed is at immediate risk for falling, so the nurse must intervene at once. The other actions reflect appropriate routine care and safety measures. This item tests supervision of AP performance, especially identifying a dangerous situation that requires immediate correction.

Equitable access to care

776. A practical nurse is helping discharge a client with newly prescribed home oxygen. The client says, "I don't have a car, and the supply company is across town." Which action best supports equitable access to care?

- A.** Tell the client to ask a neighbor for a ride after discharge
- B.** Notify the nurse so case management or social services can help arrange delivery and transportation resources
- C.** Delay discharge until the client can independently solve the problem
- D.** Document the statement and continue with routine discharge teaching

Correct Answer: B

B is correct because equitable access includes identifying barriers such as transportation and connecting the client with appropriate support services before discharge. A places the burden solely on the client. C may create unnecessary delay without addressing the barrier. D recognizes the issue but does not act on it.

777. A client with limited English proficiency arrives in the clinic for preoperative teaching. The client's adult daughter offers to translate. What is the nurse's best action?

- A.** Use the daughter because she knows the client's health history
- B.** Proceed with gestures and printed instructions in English
- C.** Request a qualified medical interpreter for the teaching session
- D.** Ask another bilingual visitor in the waiting room to help

Correct Answer: C

C is correct because a qualified medical interpreter helps ensure accurate communication, informed participation, and safer care. Family members may omit, add, or filter information, especially during sensitive discussions. Gestures or English-only materials are not enough when language is a major barrier, and using an unrelated visitor is inappropriate and unsafe.

778. During a home health visit, a client with diabetes says, "I take less insulin some weeks because I need money for food." Which response by the practical nurse is most appropriate?

- A.** Explain that insulin must always be taken exactly as prescribed and end the discussion
- B.** Ask the client whether cost is affecting the ability to obtain medications and food, then report the concern to the supervising nurse
- C.** Advise the client to skip meals on days insulin is unavailable
- D.** Tell the client to buy over-the-counter supplements instead

Correct Answer: B

B is correct because it identifies a financial barrier affecting safe treatment and supports follow-up for resource referral and care planning. A gives correct general information but ignores the cause of nonadherence. C and D are unsafe recommendations.

779. A visually impaired client is being discharged with a new medication schedule. Which intervention best promotes equitable access to safe self-care at home?

- A.** Provide standard printed instructions in small font to match the packet used for all clients
- B.** Ask the client to memorize the schedule before leaving
- C.** Arrange teaching materials in an accessible format and verify understanding using teach-back
- D.** Direct a family member to manage all medications without including the client

Correct Answer: C

C is correct because equitable care means adapting communication and teaching to the client's needs while still involving the client in care. A uses a one-size-fits-all approach. B is unreliable and unsafe. D may unnecessarily exclude the client from participation and decision-making.

780. A practical nurse in a community clinic notices that several clients miss follow-up appointments because they work hourly jobs and cannot come during business hours. Which quality-improvement suggestion best addresses equitable access?

- A.** Discharge clients from the practice after one missed appointment
- B.** Offer some evening or weekend appointment slots
- C.** Require clients to call daily for cancellations
- D.** Schedule all follow-up visits exactly 2 weeks after the initial appointment

Correct Answer: B

B is correct because expanded scheduling options can reduce a predictable structural barrier for clients with inflexible work hours. A increases inequity. C shifts the burden to clients without fixing the access problem. D standardizes timing but does not improve availability.

781. A client who uses a wheelchair is scheduled for a clinic visit. On arrival, the client cannot enter the exam room because the room is too small for the wheelchair. What is the nurse's best immediate action?

- A. Ask the client to transfer in the hallway and wait there
- B. Move the visit to an accessible room and report the barrier to the team
- C. Reschedule the appointment for another day when more staff are available
- D. Complete only the history today and skip the physical assessment

Correct Answer: B

B is correct because the nurse should immediately remove the access barrier when possible and also report the problem so the system can improve. A compromises privacy and dignity. C delays care unnecessarily. D provides incomplete care based on an avoidable environmental barrier.

782. A migrant farmworker comes to an urgent care center with a painful wound infection. The client says, "I may leave town in 2 days for work." Which discharge plan best supports equitable access and continuity?

- A. Tell the client to return only if the wound gets worse
- B. Provide treatment today and help arrange follow-up that is realistic for the client's travel schedule
- C. Delay treatment until a long-term local provider is identified
- D. Instruct the client to choose between working and completing care

Correct Answer: B

B is correct because it addresses the client's immediate health need while adapting follow-up planning to a likely continuity barrier. A is inadequate follow-up. C delays needed care. D presents a false choice rather than supporting access.

783. A clinic is reviewing why some older adult clients do not complete telehealth visits. Which finding most clearly suggests a digital access barrier?

- A. Clients prefer morning appointments
- B. Clients report they do not have internet service or do not know how to use the video platform
- C. Clients ask for shorter visits
- D. Clients want to see the same provider at each visit

Correct Answer: B

B is correct because lack of internet service and difficulty using the technology directly interfere with telehealth access. The other findings may affect satisfaction or scheduling but do not specifically identify a digital access barrier.

784. A client with low literacy nods during discharge teaching but is unable to explain when to take the prescribed antibiotic. Which nursing action best promotes equitable access to understanding?

- A. Repeat the same instructions more loudly
- B. Ask the client to sign the discharge form to show understanding
- C. Use plain language, visual aids if available, and teach-back to confirm comprehension
- D. Shorten the teaching by covering only the medication name

Correct Answer: C

C is correct because equitable care includes adapting communication to health literacy level and checking understanding with teach-back. A does not address literacy. B documents paperwork, not comprehension. D leaves out essential safety information.

785. A homeless client with hypertension is being discharged from the emergency department with a prescription requiring refrigeration. Which concern is most important for the nurse to communicate to the supervising nurse before discharge?

- A. The client did not ask many questions during teaching
- B. The client may not have a safe place to store the medication as directed
- C. The client has had hypertension for many years
- D. The client prefers to leave as soon as possible

Correct Answer: B

B is correct because lack of stable housing can directly prevent safe medication storage and effective treatment, making it a key equity-related discharge barrier. A may reflect many things but is less immediate. C is background information. D does not outweigh the storage barrier.

786. A practical nurse hears a staff member say, "Those clients never follow instructions anyway," referring to a specific cultural group. What is the nurse's best response to support equitable care?

- A. Ignore the comment to avoid conflict
- B. Agree privately but avoid saying it in front of clients
- C. Address the biased statement and redirect focus to individualized, respectful care
- D. Document the comment in every affected client's chart

Correct Answer: C

C is correct because biased assumptions can harm care quality and access. The nurse should promote respectful, individualized care rather than stereotyping. A allows the problem to continue. B reinforces bias. D is not an appropriate or useful charting action for the clients.

787. A client who is Deaf arrives for a same-day procedure and communicates primarily through sign language. No interpreter has been arranged. The surgeon is waiting and says, "We'll just write notes back and forth to save time." What is the best nursing action?

- A. Proceed because the procedure is scheduled and written notes are faster
- B. Ask a staff member who knows a few signs to help
- C. Advocate for appropriate interpretation support before proceeding with teaching and consent-related communication
- D. Have the client read the consent form independently without discussion

Correct Answer: C

C is correct because communication must be effective enough for accurate teaching and informed participation. Written notes may be inadequate for complex discussions, and a staff member with limited signing is not a reliable substitute. D assumes reading alone resolves the communication barrier.

788. A clinic tracks no-show rates and finds the highest rates among clients from one neighborhood with limited public transportation. Which response best reflects an equitable access approach?

- A. Label those clients as noncompliant and reduce future scheduling priority
- B. Use the data to explore transportation barriers and develop targeted solutions
- C. Apply stricter missed-appointment fees to improve attendance
- D. Stop accepting clients from that neighborhood

Correct Answer: B

B is correct because equitable care uses data to identify patterns caused by barriers and then improve systems. A, C, and D punish clients for structural obstacles instead of addressing the cause.

789. A client with chronic kidney disease repeatedly misses specialist appointments. On review, the practical nurse learns the referral office mails appointment notices to an old address and the client changes phone numbers often. What is the best next step?

- A.** Document the missed appointments as client refusal of care
- B.** Tell the client specialist care is important and leave the referral process unchanged
- C.** Report the communication barrier and help verify current contact information so referral follow-up can be updated
- D.** Advise the client to call every specialist office weekly to check for appointments

Correct Answer: C

C is correct because the information points to a system-level communication barrier, not simple refusal. Updating contact methods and reporting the issue supports access and continuity. A mislabels the problem. B is incomplete. D places an unrealistic burden on the client without fixing the flawed process.

790. A rural client is discharged with instructions to obtain daily wound care at an outpatient center 45 miles away. The client says, "I can only get a ride once a week." Which nursing action best demonstrates equitable access planning?

- A.** Reinforce the original plan because daily wound care is standard
- B.** Notify the nurse so the team can explore alternative services, such as a closer option or home-based follow-up if available
- C.** Advise the client to manage the wound independently until transportation improves
- D.** Tell the client to skip days but keep the outpatient appointment when possible

Correct Answer: B

B is correct because equitable planning considers geography and transportation barriers and seeks realistic, safer alternatives. A ignores the barrier. C and D create risk by substituting inadequate care for an achievable plan.

791. A practical nurse is preparing discharge teaching for a client who recently immigrated and speaks limited English. The client smiles and says "yes" to every question, but gives short, unclear answers when asked how the medication will be taken at home. Which action is best to support equitable access to care?

- A.** Provide the printed instructions in English and ask a family member to explain them later
- B.** Slow down speech and repeat the instructions several times in a louder voice
- C.** Arrange for qualified language assistance and use teach-back to confirm understanding
- D.** Ask the client to sign the discharge papers since agreement suggests understanding

Correct Answer: C

Equitable access includes access to understandable information. Qualified language assistance helps avoid errors that can occur when meaning is lost, and teach-back checks whether the client can apply the instructions. Family interpretation may be incomplete or inaccurate, louder speech does not solve a language barrier, and a signature does not prove understanding.

792. In a prenatal clinic, several clients who recently moved to the area miss visits because they do not have stable phone service. Which clinic change would best improve equitable access for this group?

- A.** Require clients to call monthly to remain active in the system
- B.** Offer reminder options such as mailed notices, patient portal messages, and contact through approved community partners
- C.** Discharge clients from the practice after two missed appointments
- D.** Schedule all clients for the earliest morning appointments

Correct Answer: B

Using multiple reminder methods addresses the actual barrier: unstable phone access. This is an equitable approach because it adapts outreach to client circumstances rather than expecting all clients to use the same communication channel. Requiring monthly calls and discharging clients would worsen access, and early appointments do not address the phone barrier.

793. A client is ready for discharge after treatment for heart failure. The prescription plan includes four new medications. The client quietly tells the nurse, "I don't think I can pay for all of these." Which response by the practical nurse is most appropriate?

- A.** Tell the client to fill the most important prescription first and buy the others next month
- B.** Advise the client to stop taking older medications to make room in the budget
- C.** Notify the supervising nurse so the team can explore lower-cost options and medication assistance resources before discharge
- D.** Suggest borrowing pills from a family member who takes similar medications

Correct Answer: C

The nurse should address cost as a barrier to access before discharge. Escalating the concern allows the care team to consider safer, affordable options and available assistance resources. Telling the client to ration, stop medications, or borrow medications is unsafe and can lead to serious harm.

794. A community clinic wants to improve equitable access for clients who work irregular shifts. Which outcome measure would best show whether a new evening clinic schedule is helping?

- A.** The number of magazines taken from the waiting room each week
- B.** The percentage of follow-up appointments completed by clients who previously had scheduling-related missed visits
- C.** The average length of staff lunch breaks
- D.** The total number of parking spaces near the building

Correct Answer: B

If the intervention is evening clinic hours, the most useful measure is whether the target group is now able to complete follow-up visits. That directly evaluates whether access improved. The other choices do not meaningfully assess scheduling-related access.

795. A practical nurse in a rehabilitation setting is planning teaching for a client with limited hand strength after a stroke. The client must use written home instructions to continue exercises safely. Which approach best promotes equitable access to the teaching?

- A.** Provide several pages of small-print instructions to keep the information complete
- B.** Ask the client's roommate to learn the exercises and explain them later
- C.** Offer simplified instructions in an easy-to-handle format and confirm the client can use the materials independently
- D.** Delay all teaching until the next follow-up visit

Correct Answer: C

Equitable access means making health information usable for the client's abilities. Instructions should be presented in a format the client can physically manage, and the nurse should verify the client can use them. Small print and reliance on others can reduce independence and safety. Delaying teaching misses an important discharge need.

796. A client with asthma lives in a shelter and is prescribed a nebulizer treatment at home. During discharge planning, which question is most important for the nurse to ask first to support equitable access to care?

- A.** Do you prefer morning or evening treatments?
- B.** Have you ever used a nebulizer before?
- C.** Will you have a reliable place with electrical access to use and store the equipment?
- D.** Would you like written information about asthma triggers?

Correct Answer: C

The first priority is whether the client can realistically use the treatment in the living situation described. If there is no reliable place or electricity, the plan may not be workable and needs revision. Prior experience and written information matter, but they come after confirming the treatment is actually accessible.

797. At a pediatric clinic, a parent says, "I had to choose between missing work for this appointment and paying this week's electric bill." Which response by the practical nurse best supports equitable access?

- A.** You should try to plan better so this does not happen again
- B.** Children's health has to come first, even if it is inconvenient
- C.** Let's talk about what made today difficult and connect you with options that may reduce those barriers for future visits
- D.** If appointments are hard to keep, urgent care may be a better choice

Correct Answer: C

This response is nonjudgmental, explores the barrier, and opens the door to solutions such as flexible scheduling, transportation support, or community resources. The other responses blame the parent or redirect care in a way that may worsen continuity and access.

798. A practical nurse works in a busy medical office that recently added telehealth follow-up visits. Which client situation most clearly indicates that telehealth may not provide equitable access unless an alternative is offered?

- A.** A client prefers in-person visits even though video visits are available
- B.** A client has a smartphone but forgets the clinic password occasionally
- C.** A client lives in an area with unreliable internet service and no private place to discuss health information
- D.** A client asks whether telehealth appointments can be shorter than office visits

Correct Answer: C

Unreliable internet and lack of privacy are significant barriers that can prevent effective and safe telehealth use. Offering only telehealth in that situation would create unequal access. Preference alone is not the same as a major access barrier, and the other issues are less substantial or more easily solved.

799. A client from a rural area is discharged after a procedure and needs laboratory monitoring in 48 hours. The nearest laboratory is far away, and the client says transportation is uncertain. Which discharge plan best supports equitable access while maintaining continuity of care?

- A.** Tell the client to come back only if symptoms develop
- B.** Document that the client was informed and leave the rest to the client
- C.** Coordinate before discharge to identify a realistic local or mobile option and ensure the follow-up plan is communicated clearly
- D.** Schedule the laboratory visit at the original facility without discussing transportation

Correct Answer: C

An equitable plan accounts for the client's location and transportation limits before discharge. Coordinating a realistic option and communicating the plan reduces the chance of missed follow-up and complications. Simply documenting, waiting for symptoms, or scheduling an impractical appointment does not address the barrier.

800. A clinic reviews data and finds that clients with limited literacy have lower rates of completed preventive screenings than other clients. Which intervention would best address this disparity?

- A.** Send all clients a longer packet with more detailed medical terminology
- B.** Develop plain-language materials with visual cues and train staff to use teach-back during screening education
- C.** Tell staff to document that teaching was completed whether or not clients ask questions
- D.** Require clients to read the consent and instruction forms without assistance before scheduling

Correct Answer: B

Plain-language materials, visual support, and teach-back directly reduce literacy-related barriers to understanding and participation. The other choices increase complexity, emphasize paperwork over comprehension, or create extra barriers to care.

801. A practical nurse is caring for a client who observes a religious practice that limits contact with unrelated members of the opposite sex. The client is due for teaching about wound care. Which action best supports equitable access to care while respecting client rights?

- A.** Inform the client that all staff must be treated the same and continue without discussion
- B.** Assess the client's preferences and communicate them to the supervising nurse so teaching can be arranged in a respectful, workable manner
- C.** Ask the client's family to perform all future teaching
- D.** Skip the teaching because cultural preferences make education too difficult

Correct Answer: B

Equitable care includes respectful accommodation of cultural or religious preferences when possible, while still ensuring safe teaching and continuity. Assessing the preference and communicating it to the team supports both access and client rights. The other options dismiss the concern, shift teaching inappropriately, or withhold necessary care.

802. A clinic serving many uninsured clients is considering several changes. Which proposal most directly reflects an equitable access approach rather than equal treatment alone?

- A.** Give every client the same 15-minute appointment slot regardless of needs
- B.** Provide the same printed instructions to all clients in one language to maintain consistency
- C.** Offer flexible scheduling, language support, and referral help based on identified barriers
- D.** Require clients to arrange all specialty referrals on their own to encourage responsibility

Correct Answer: C

Equity means responding to differing barriers so clients have a fair chance to receive effective care. Flexible scheduling, language support, and referral assistance target real obstacles. The other choices focus on identical treatment, which may look fair on the surface but can preserve inequity.

803. A client with limited finances is discharged with a new dressing regimen that requires multiple supplies. The client says, "I can probably make it stretch if I reuse some of the items." What is the nurse's best response?

- A.** That should be fine as long as the wound looks clean
- B.** Reusing supplies may increase risk; the nurse should report this barrier now so the discharge plan can be adjusted with safer options or resource support
- C.** Use the supplies only every other day to save money
- D.** Ask a neighbor whether extra wound supplies are available

Correct Answer: B

The client's statement reveals an access barrier that could lead to infection or poor healing. The nurse should address the problem before discharge by involving the team in finding safer, realistic alternatives or assistance. Advising reuse, reduced frequency, or borrowing supplies is unsafe.

804. A practical nurse notices that a client repeatedly answers "I don't know" when asked about follow-up instructions. The chart shows the client recently aged out of foster care and has no consistent emergency contact. Which nursing inference is most important when planning for equitable access after discharge?

- A.** The client is probably not interested in participating in care
- B.** The client may lack stable support systems and need a clearer, more coordinated follow-up plan
- C.** The client should be discharged later than other clients automatically
- D.** The client must refuse all teaching because no emergency contact is listed

Correct Answer: B

Limited support and unstable life circumstances can affect a client's ability to navigate follow-up care. The nurse should anticipate the need for a more coordinated, concrete plan rather than assuming lack of interest. The other choices are unsupported, punitive, or incorrect.

805. During a staff meeting, a nurse says, "We already treat everyone the same here, so access is not really our issue." Which response by the practical nurse is best?

- A.** Treating everyone the same is enough because health systems cannot solve social problems
- B.** Equal treatment is the main goal, so barriers outside the clinic are not relevant
- C.** Clients may face different barriers, so fair access sometimes requires different supports to help them receive and use care
- D.** Access problems are mostly caused by clients who do not value their health

Correct Answer: C

This response correctly explains that equitable access is not identical treatment. Clients may need different supports based on language, transportation, finances, disability, or technology barriers. The other options dismiss real barriers and shift blame to clients.

806. A client with newly diagnosed diabetes is being discharged with instructions for checking blood glucose at home. The client reads the consent form slowly, avoids eye contact, and says, "I forgot my glasses, so I'll read all of this later." Which action by the practical nurse best supports equitable access to care?

- A.** Ask a family member to explain the written instructions at home
- B.** Read the instructions aloud, demonstrate the glucometer, and ask the client to show how it will be used
- C.** Document that teaching was completed because the client signed the paperwork
- D.** Give the client extra printed materials with more detailed medical terms

Correct Answer: B

Reading the instructions aloud, demonstrating the skill, and using return demonstration addresses a likely barrier to understanding and makes the teaching more accessible. Signing paperwork does not prove comprehension. Relying on a family member shifts responsibility away from the nurse and may not ensure accurate teaching. More complex written materials would increase, not reduce, the access barrier.

807. A practical nurse in a clinic notices that older adults who do not use the online portal are less likely to complete follow-up appointments after abnormal test results. Which clinic action best promotes equitable access to care?

- A.** Require all clients to use the portal so the process stays consistent
- B.** Send the same portal reminder twice instead of once
- C.** Offer follow-up scheduling through phone calls as well as the portal
- D.** Post instructions in the waiting room on how to create an online account

Correct Answer: C

Equitable access means reducing barriers that affect some groups more than others. Offering phone scheduling along with portal use provides another workable access route for clients who are less comfortable with technology. Requiring portal use treats everyone the same but ignores unequal barriers. Sending more portal reminders or posting account instructions still depends on portal access and skill.

808. A client who uses a wheelchair arrives for a clinic appointment and says, "The scale is up three steps, so they told me they would just skip my weight again." Which response by the practical nurse is most appropriate?

- A.** Tell the client that weight can be estimated from the last visit
- B.** Explain that the scale location cannot be changed during today's shift
- C.** Acknowledge the barrier and report the inaccessible equipment as a client care issue
- D.** Suggest that the client arrange help from a family member for future visits

Correct Answer: C

The best response recognizes that inaccessible equipment is an equity and care-quality problem, not just an inconvenience. Reporting and addressing the barrier supports safer, more equitable care. Estimating weight may be inaccurate. Saying nothing can be changed accepts the inequity. Asking the client to solve the system problem through family help does not address the barrier created by the setting.

809. A home health client with chronic obstructive pulmonary disease is prescribed oxygen and several inhalers. The client says, "I stay with different relatives, so my things are not always in the same place." Which nursing concern is most important for equitable access planning?

- A.** The client may have unstable housing that affects treatment consistency
- B.** The client may prefer not to learn about the medications
- C.** The client should be encouraged to buy duplicate equipment for each home
- D.** The client needs stricter instructions about personal responsibility

Correct Answer: A

Moving between homes suggests unstable housing, which can directly affect medication storage, oxygen access, follow-up, and continuity of care. Identifying that barrier is the key first step in planning equitable care. The statement does not show refusal to learn. Buying duplicates may be unrealistic and costly. Framing the issue as personal irresponsibility misses an important social barrier.

810. A clinic wants to improve equitable access for clients with hearing impairment. Which change is most likely to help?

- A.** Speaking louder during every visit
- B.** Using masks less often when giving instructions
- C.** Providing multiple communication options, such as written instructions and approved interpretation support when needed
- D.** Asking family members to translate all teaching for the client

Correct Answer: C

Offering communication supports tailored to the client's needs improves access to information and care. Simply speaking louder may not help and can be disrespectful. Avoiding masks is not an appropriate general solution and may conflict with safety needs. Family members are not a reliable substitute for appropriate communication support during clinical teaching.

811. A practical nurse is preparing a client for discharge after treatment of a leg ulcer. The provider ordered daily dressing changes at an outpatient wound center. The client says, "I can only get rides on Tuesdays." What is the nurse's priority action?

- A.** Remind the client that prescribed care must be followed exactly
- B.** Notify the supervising nurse or provider that the current plan may be unrealistic because of transportation limits
- C.** Document client noncompliance with the treatment plan
- D.** Tell the client to call the wound center after discharge and work it out independently

Correct Answer: B

The priority is to identify that the current discharge plan may not be accessible and communicate that barrier so the plan can be revised. Equity-focused care requires matching the plan to the client's actual ability to obtain services. Labeling the client noncompliant before addressing the barrier is premature and inappropriate. Simply reminding or telling the client to manage it alone does not solve the access problem.

812. A school-based health program finds that students from families with limited transportation attend fewer vaccine appointments than other students. Which intervention best reflects an equitable access approach?

- A.** Offer the same clinic hours currently used for all families
- B.** Provide more reminder flyers in the students' backpacks
- C.** Hold vaccination events at the school during times families are already present

D. Require parents to confirm transportation before making an appointment

Correct Answer: C

Holding vaccine events at the school during existing family-contact times reduces a known barrier rather than expecting all families to overcome it equally. Equal treatment alone would keep the same hours for everyone even though the barrier affects some families more. More flyers do not remove the transportation problem. Requiring transportation confirmation may create another barrier.

813. A client who recently lost employment is prescribed a special diet after hospitalization. During teaching, the client says, "Fresh foods are expensive, and I mostly buy what lasts." Which nursing response best supports equitable access to care?

- A.** Tell the client that strict diet adherence is the only acceptable option
- B.** Ask what foods are realistically available and involve the team in planning affordable choices
- C.** Skip diet teaching because the client cannot afford the ideal foods
- D.** Advise the client to search online for less expensive meal plans

Correct Answer: B

The best response starts with the client's real circumstances and adapts teaching to feasible options, while involving the team as needed. That approach promotes equitable care because it focuses on what the client can actually access. Telling the client to follow an ideal plan without adjustment ignores financial barriers. Skipping teaching is unsafe. Directing the client to search online assumes internet access and shifts the burden away from the care team.

814. A practical nurse works at a clinic that recently began sending appointment reminders only by text message. Which finding most strongly suggests this change may have worsened equitable access?

- A.** Overall clinic volume stayed the same
- B.** Younger clients report the reminder system is convenient
- C.** Missed appointments increased mainly among clients who previously requested phone reminders

D. Front-desk staff report that text messaging saves time

Correct Answer: C

An increase in missed appointments among clients who depended on phone reminders suggests the new system created a barrier for a specific group. That pattern points to reduced equitable access. Overall volume can stay the same while a subgroup is harmed. Convenience for younger clients or staff efficiency does not show equity if another group is being left behind.

815. A client with low vision is prescribed a new medication regimen with three pills taken at different times of day. Which discharge method best supports equitable access to safe medication use?

- A.** Provide a standard printed medication list in small font to match the chart format
- B.** Color-code the pill bottles and verify the client can distinguish the labels and explain the schedule
- C.** Ask a neighbor to organize the medications each week
- D.** Tell the client to call the pharmacy later if the labels are hard to read

Correct Answer: B

Color-coding with verification of the client's ability to use the system addresses the access barrier and checks whether the plan is workable. Standard small-print instructions are not accessible for low vision. Asking a neighbor is not a reliable or privacy-sensitive primary plan. Telling the client to solve the problem later delays action and may risk medication errors after discharge.

816. In a dialysis center, several clients who speak a minority language arrive late because they rely on shared rides and have difficulty understanding schedule changes made by automated calls in English. Which change would best address the root access problem?

- A.** Discharge clients from the service after repeated tardiness
- B.** Repeat the English automated message more slowly

- C.** Use a scheduling process that communicates changes in the client's preferred language and confirms understanding
- D.** Ask drivers to explain schedule changes to the clients

Correct Answer: C

The root problem is not simply lateness; it is a communication barrier tied to language and transportation coordination. A process that communicates in the client's preferred language and confirms understanding targets that barrier directly. Slower English messages may still be ineffective. Asking drivers to interpret is unreliable and inappropriate. Discharging clients punishes the result of the access barrier rather than addressing it.

817. A practical nurse gives wound-care teaching to a client who nods often and says, "Okay, okay," throughout the discussion. The client later places the clean dressing on the bedside table and reaches for the used dressing to reapply it. Which nursing conclusion is best?

- A.** The client is intentionally refusing care
- B.** The client may not have understood the teaching despite appearing agreeable
- C.** The teaching was effective because the client was attentive
- D.** The client should receive only written instructions from now on

Correct Answer: B

The client's action shows a likely misunderstanding of the wound-care instructions. In equitable care, apparent agreement does not guarantee comprehension, especially when barriers such as literacy, language, or stress may be present. There is no evidence of intentional refusal. Attentiveness alone is not proof of understanding. Written instructions alone may not address the problem.

818. A clinic serving a large migrant worker population is deciding between two outreach plans for follow-up blood pressure checks. Plan 1 offers one Saturday clinic each month. Plan 2 offers brief blood pressure checks at changing work sites during meal breaks with referral pathways for abnormal findings. Which plan better supports equitable access, and why?

- A.** Plan 1, because offering the same clinic to everyone is the fairest approach
- B.** Plan 1, because blood pressure screening should occur only in a clinic building
- C.** Plan 2, because it reduces work-time and transportation barriers for the affected population
- D.** Plan 2, because clients are more likely to accept care when it is informal

Correct Answer: C

Plan 2 better supports equitable access because it directly reduces known barriers faced by this group: transportation problems and inability to leave work. Equity focuses on removing barriers that affect groups unevenly, not only offering identical services. Saying identical access is always fairest confuses equality with equity. The reason is barrier reduction, not informality itself.

819. A client is discharged with instructions to monitor blood pressure daily and upload the readings through a phone app. The client says, "My phone service gets turned off some months." Which revision to the plan best supports equitable access?

- A.** Keep the current plan because the app is the fastest way to report data
- B.** Advise the client to use free public Wi-Fi whenever possible
- C.** Arrange an alternative reporting option, such as written logs reviewed by phone or in-person follow-up
- D.** Delay discharge until the client can guarantee reliable phone service

Correct Answer: C

An alternative reporting method addresses the technology access barrier while preserving follow-up and continuity of care. Keeping the app-only plan ignores the client's stated limitation. Depending on public Wi-Fi may be inconsistent and still leaves communication gaps. Delaying discharge for this reason is unnecessary if an accessible monitoring plan can be arranged.

820. A practical nurse participates in a quality-improvement review of missed postpartum visits. Data show the highest no-show rate occurs among clients who must bring several young children to appointments. Which proposed intervention is most likely to improve equitable access?

- A. Increase penalties for late cancellation
- B. Schedule all postpartum clients at the same morning time block
- C. Offer flexible visit formats and coordinate supports that reduce childcare-related barriers
- D. Shorten each visit so clients spend less time in the exam room

Correct Answer: C

The data point to childcare demands as a major barrier. Flexible visit options and coordinated supports target that barrier directly and are most likely to improve access. Penalties may worsen disparities. Scheduling everyone the same way ignores the reason for missed visits. Shorter visits may help a little, but they do not directly solve the childcare barrier as effectively as flexible formats and support coordination.

821. A client is scheduled for follow-up after treatment for heart failure. During discharge teaching, the client says, "I live in a shelter right now, and my bed can change from week to week." Which nursing action best supports equitable access to ongoing care?

- A. Tell the client to call the clinic if an address is available later
- B. Arrange follow-up using reliable contact methods the client can access, such as a shelter phone, case manager, or agreed community contact
- C. Delay discharge until the client can provide a permanent home address

D. Assume the client will return to the emergency department if symptoms worsen

Correct Answer: B

B is correct because equitable access means adapting the plan to the client's actual circumstances rather than relying on a standard process that does not fit. Identifying a realistic contact pathway improves the chance that follow-up, reminders, and results will reach the client. A and D shift the burden to the client and increase the risk of missed care. C creates an unnecessary barrier.

822. A practical nurse in a prenatal clinic reviews missed appointments. Which client situation most clearly signals a barrier related to equitable access rather than simple preference?

- A. A client asks for afternoon visits instead of early morning visits
- B. A client says the clinic instructions are only in English, but the client reads only Arabic
- C. A client prefers one provider over another
- D. A client wants a shorter waiting time before being seen

Correct Answer: B

B is correct because language mismatch can directly prevent a client from understanding instructions, navigating care, and using services safely. That is an access barrier. The other options describe common preferences or convenience concerns, not the clearest example of inequitable access. The likely trap is choosing A because scheduling matters, but a language barrier is more fundamental to obtaining care at all.

823. A clinic offers same-day appointments only through an online booking system. Over 3 months, appointment use rises overall, but missed urgent visits increase among older adults in the surrounding neighborhood. Which conclusion is most appropriate?

- A. The clinic should keep the system because total appointment use increased
- B. Older adults are less interested in preventive care than other groups
- C. The booking change may have improved convenience for some clients while creating an access barrier for others

D. Missed urgent visits are unrelated to scheduling methods

Correct Answer: C

C is correct because equitable access requires looking at who benefits and who is left behind. A process can increase use overall yet still worsen access for a subgroup if the method does not match that group's needs. A ignores subgroup disparities. B makes an unsupported assumption about motivation. D dismisses a plausible connection shown by the pattern.

824. A client who works hourly shifts is prescribed outpatient physical therapy twice a week after knee surgery. The client says, "If I miss work again, I may lose my job." Which nursing response best supports equitable access to care?

- A.** Explain that recovery should be the client's first priority
- B.** Advise the client to use over-the-counter pain medicine until work slows down
- C.** Explore options such as extended-hour therapy, alternate sites, or coordination with the provider for a feasible plan
- D.** Tell the client that surgery outcomes are poor when therapy is skipped

Correct Answer: C

C is correct because it addresses the real barrier and tries to preserve treatment access within the client's work constraints. Equitable care means modifying the plan when possible so clients can realistically use services. A and D may be true concerns but do not solve the access problem. B substitutes self-management for needed follow-up care without provider coordination.

825. A practical nurse is discharging a client who will need follow-up laboratory testing in 1 week. Which discharge plan best promotes equitable access for a client with limited literacy?

- A.** Provide several pages of written instructions and ask the client to sign that they were received
- B.** Give verbal teaching rapidly to avoid embarrassing the client

C. Use plain-language instructions, highlight the exact date and location, and confirm understanding with teach-back

D. Direct a family member to manage all instructions instead of the client

Correct Answer: C

C is correct because plain language and teach-back reduce misunderstanding while preserving the client's participation in care. This supports equitable access by making the follow-up plan usable. A focuses on paperwork rather than comprehension. B avoids the issue instead of ensuring understanding. D may undermine autonomy and should not replace direct communication with the client when the client can participate.

826. During an intake interview, a client states, "I do not have internet at home, and I usually borrow a neighbor's phone." The provider's plan includes telehealth follow-up, portal messages, and electronic prescription discount coupons. Which concern should the practical nurse raise first?

- A.** The client may dislike technology and need more encouragement
- B.** The client may have multiple access barriers that make the current plan difficult to use safely
- C.** The client should be taught to download all clinic applications before discharge
- D.** The provider should cancel all follow-up care until the client gets a personal phone

Correct Answer: B

B is correct because the statement suggests digital access barriers that affect several parts of the plan: follow-up communication, telehealth, and obtaining discounts. The nurse should identify this mismatch promptly so the plan can be revised. A minimizes the issue as preference. C assumes access to devices and connectivity. D creates a new barrier rather than adapting care.

827. A community clinic wants to reduce unequal access to medication teaching. Which outcome measure would best show whether a new intervention is improving equity?

- A.** The total number of brochures printed each month
- B.** The average length of each teaching session

C. The difference in medication understanding between clients who need language support and those who do not

D. The number of nurses assigned to the clinic each week

Correct Answer: C

C is correct because equity improvement is shown not just by activity, but by whether gaps between groups decrease. Measuring understanding across groups directly tests whether the intervention is reducing disparities. A, B, and D are process or staffing measures that do not show whether access or comprehension became more equitable.

828. A client is discharged after treatment for pneumonia with instructions to return to the clinic in 48 hours if symptoms do not improve. The client says, "I am caring for my father with dementia and cannot leave him alone." What is the practical nurse's best action?

A. Document that the client was noncompliant with the discharge plan

B. Repeat the importance of follow-up and end teaching

C. Notify the team that caregiving responsibilities may block access and help explore workable follow-up options

D. Advise the client to bring the father to the clinic waiting room

Correct Answer: C

C is correct because the client's caregiving role is a real access barrier. The nurse should bring this forward so the plan can be adjusted, such as by changing timing, location, or support arrangements. A labels the client rather than addressing the barrier. B restates the problem without solving it. D may be unsafe and unrealistic.

829. A practical nurse notices that clients with limited English proficiency often arrive prepared for the wrong procedure because reminder calls use only automated English messages. Which intervention would most directly improve equitable access?

A. Ask family members to translate all reminder messages

B. Require clients to come in person to verify every appointment

C. Provide appointment reminders in the client's preferred language using qualified language-support processes

D. Schedule these clients only on days when the clinic is less busy

Correct Answer: C

C is correct because it addresses the actual point of failure: clients are not receiving understandable scheduling information. A relies on family instead of an appropriate communication process. B adds burden and may worsen access. D does not solve the misunderstanding that leads to missed or incorrect preparation.

830. A health system launches a telehealth follow-up program after hospital discharge. Thirty days later, readmissions fall overall, but the drop occurs mainly among insured clients with home broadband. Which next step best reflects equitable access to care?

A. Expand telehealth only, because the program improved the main outcome

B. End telehealth, because any unequal benefit means the program failed

C. Keep telehealth as one option and add alternative follow-up pathways for clients with limited digital access

D. Use the same telehealth model and ask high-risk clients to try harder to connect

Correct Answer: C

C is correct because an intervention can be beneficial overall yet still widen disparities if access to the intervention is uneven. The best equity response is to preserve useful care while adding workable alternatives for clients who cannot use the digital pathway. A ignores disparity. B throws away a helpful service instead of improving inclusion. D blames clients for a structural barrier.

831. A client who is deaf arrives for a preprocedure visit accompanied by an adult child who says, "I will interpret everything." The client uses sign language and appears attentive. Which action by the practical nurse best supports equitable access?

A. Proceed with the adult child interpreting because a family member knows the client best

- B.** Ask the client to read lips if possible so the visit stays on time
- C.** Arrange appropriate communication support and speak directly to the client during the visit
- D.** Reschedule the appointment for another day without discussing options

Correct Answer: C

C is correct because equitable access includes communication that allows the client to participate directly and understand care accurately. Speaking to the client and arranging appropriate support best protects access and comprehension. A may seem convenient but can compromise accuracy and the client's direct participation. B is unreliable and shifts the burden to the client. D delays care unnecessarily.

832. A practical nurse in a pediatric clinic is helping design outreach for families who frequently move between temporary housing sites. Which strategy is most likely to improve equitable access to immunization follow-up?

- A.** Mail reminder cards to the last address in the chart
- B.** Schedule all follow-up visits exactly 6 months in advance
- C.** Use flexible contact methods and partner with community locations families already use
- D.** Require a fixed home address before future appointments can be made

Correct Answer: C

C is correct because families with unstable housing may not receive mail or maintain a single address. Flexible contact pathways and trusted community sites make follow-up more reachable. A and B rely on stability the family may not have. D creates an unnecessary barrier and worsens access.

833. A nurse manager asks which proposal best reflects an equitable access approach rather than an equal-treatment approach. Which proposal should the practical nurse support?

- A.** Give every client the same printed discharge packet in English so the process is consistent
- B.** Offer identical clinic hours to all clients because fairness means treating everyone the same

C. Provide the same transportation voucher amount to all clients, regardless of distance or need

D. Adjust communication and follow-up supports based on each client's barriers so clients have a fair chance to use care

Correct Answer: D

D is correct because equitable access focuses on what different clients need in order to obtain and use care effectively. Equal treatment alone can leave barriers unchanged. A, B, and C apply the same process to everyone, but sameness does not ensure fairness when needs differ.

834. At a community health center, all clients with diabetes are told to attend a weekly education class held on the third floor at 2 p.m. on weekdays. A practical nurse notices that clients who use wheelchairs and clients who rely on public transportation attend far less often than other clients. Which change best promotes equitable access to care?

- A.** Keep the class schedule the same so every client is treated identically
- B.** Offer the same class in an accessible ground-floor room and at an additional evening time
- C.** Require clients who miss class to read the handout at home instead
- D.** Ask clients to arrange their own transportation before being allowed to reschedule

Correct Answer: B

Equitable access means reducing barriers so clients have a fair chance to receive care. Moving the class to an accessible location and adding a time option addresses both mobility and transportation/work barriers. Choice A is a common trap because equal treatment is not always equitable when barriers differ. Choices C and D leave the access barriers in place rather than correcting them.

835. A rural clinic is reviewing follow-up rates after emergency department discharge for asthma. The practical nurse compares two outreach plans. Plan 1 sends one standard text reminder in English 24 hours before the visit. Plan 2 asks each client at discharge about preferred language, best contact method, transportation concerns, and best appointment times, then uses that information for reminders and scheduling. Which rationale best supports Plan 2 as the more equitable approach?

- A.** It gives every client the exact same reminder, which is the fairest system
- B.** It focuses only on clients who have already missed an appointment, saving staff time
- C.** It adapts follow-up support to common barriers that differ across clients
- D.** It avoids discussing social factors so the clinic can stay focused only on symptoms

Correct Answer: C

Plan 2 is more equitable because it identifies and addresses differing barriers such as language, transportation, and scheduling constraints before they lead to missed care. Equity aims to match support to need, not simply provide identical outreach. Choice A reflects equal treatment rather than equitable access. Choice B is less proactive, and Choice D ignores factors that directly affect whether clients can obtain follow-up care.

836. A practical nurse is preparing discharge instructions for four clients who all need a primary care follow-up visit within 7 days. Which client should the nurse identify as having the greatest risk for inequitable access to follow-up care?

- A.** A client who has a car, paid sick leave, and asks for an early morning appointment
- B.** A client who says, "I can read the instructions, but I usually need my daughter to drive me to visits"
- C.** A client who says, "I just started a new job, I do not speak English well, and I am not sure how I will get there"
- D.** A client who asks whether the clinic can send a text reminder the day before the visit

Correct Answer: C

Choice C includes several access barriers at once: limited English proficiency, possible transportation problems, and job-related scheduling limits. Those factors place the client at the greatest risk for unequal access to follow-up care. Choice B shows one barrier, transportation, but the client already has some support. Choices A and D describe manageable preferences rather than major inequitable access concerns.

837. A practical nurse works in a clinic that recently began requiring clients to confirm appointments through an automated phone menu available only in English. Since the change, missed visits have increased among clients with limited English proficiency. Which action should the nurse recommend first?

- A.** Ask providers to shorten visit times so more missed appointments can be rebooked
- B.** Add multilingual confirmation options and access to trained interpreter support
- C.** Discharge clients from the practice after two missed appointments
- D.** Remind staff to speak louder when clients seem confused on the phone

Correct Answer: B

The barrier is built into the confirmation process itself, so the first step is to make that process linguistically accessible. Multilingual options and interpreter support directly address the cause of the missed visits. Choice A manages the schedule but does not remove the barrier. Choice C would worsen inequity. Choice D confuses language access with hearing or attention problems and would not solve the issue.

838. A clinic reports that overall completion of postpartum follow-up visits increased after a reminder program was introduced. However, completion rates remained much lower for clients living in outlying rural areas. Which interpretation is most appropriate?

- A.** The program was successful because overall results improved, so no further changes are needed

B. The lower rural completion rate suggests the program may have improved access for some groups while leaving an important barrier unresolved

C. Clients in rural areas are probably less interested in follow-up care than other clients

D. The reminder program should be stopped because any difference between groups means the intervention failed

Correct Answer: B

Equitable access focuses not only on overall improvement but also on whether gaps between groups persist. Choice B correctly recognizes that the intervention may have helped many clients while not addressing barriers such as transportation, distance, or connectivity for rural clients. Choice A ignores the disparity. Choice C makes an unsupported assumption about motivation. Choice D is too absolute; the program may still be useful but needs modification.

839. A hospital creates a policy that every client discharged with a new medication will receive the same 20-minute teaching session, the same printed handout in English, and the same follow-up call during business hours. The nurse manager asks the practical nurse whether this policy supports equitable access to care. Which response is best?

A. Yes; treating every client exactly the same is the fairest approach

B. Yes; standardization removes all bias, so equity is automatically achieved

C. No; equity may require different teaching formats, languages, or follow-up methods based on client needs and barriers

D. No; equitable access means giving extra services only to clients who request them

Correct Answer: C

Equity is not the same as identical treatment. Clients do not all start with the same resources, language needs, schedules, or health literacy. Choice C is correct because an equitable approach adjusts care delivery so clients have a fair chance to understand and use the plan. Choice A reflects equal treatment, not equitable access. Choice B overstates what standardization can do. Choice D is too narrow because many clients with barriers may not know what to request or feel comfortable asking.

840. A practical nurse is scheduling a follow-up visit for a client newly diagnosed with hypertension. The client says, "I work two jobs, take the bus, and may have to miss pay if I come during the day." Which nursing action best promotes equitable access to care?

A. Schedule the client in the next available daytime slot because all clients should be treated the same way

B. Ask about transportation and work-hour barriers, then help arrange an appointment time and follow-up plan the client can realistically attend

C. Advise the client that controlling blood pressure is important and that the client must prioritize the appointment

D. Document that the client is noncompliant if the client cannot commit to the original appointment time

Correct Answer: B

Equitable access means identifying and addressing barriers that make care harder to obtain for a specific client. Asking about transportation and work constraints, then adapting scheduling and follow-up, directly reduces those barriers. Choice A describes equal treatment, not equitable access. Choice C gives education but does not solve the access problem. Choice D mislabels a barrier to care as noncompliance.

Safety and Infection Prevention and Control

Client identification

841. A nurse is preparing to administer a morning medication. Which action best verifies the correct client before giving the drug?

- A.** Ask the client to state their full name and date of birth, then compare both with the medication record and identification band
- B.** Check the room number and ask a family member to confirm the client's name
- C.** Recognize the client from the previous shift and proceed
- D.** Look at the name written on the door and compare it with the medication cup

Correct Answer: A

Using at least two client identifiers that the client states, then matching them to the ID band and record, is the safest method. Room number, door labels, recognition, and family confirmation are not reliable primary identifiers because clients can be moved, misrecognized, or incorrectly identified by others.

842. A client is drowsy after receiving pain medication and mumbles when spoken to. The nurse must obtain a blood glucose reading. Which action is most appropriate to identify the client?

- A.** Ask the roommate whether this is the correct client
- B.** Use the client's identification band and compare it with the laboratory order and medical record
- C.** Wait until the client is fully alert before performing the test

D. Identify the client by the bed number and diagnosis on the whiteboard

Correct Answer: B

If the client cannot reliably state identifiers, the nurse should use the identification band and compare it to the order and record. A roommate, bed number, and whiteboard are unsafe identifiers. Waiting may delay needed care when identification can be confirmed another safe way.

843. The nurse enters a semiprivate room to give an injection and says, "Mr. Diaz?" Both clients look up. What should the nurse do next?

- A.** Give the injection to the client nearest the window because that is Mr. Diaz's usual bed
- B.** Ask each client who is scheduled for an injection this morning
- C.** Check the medication administration record and use two identifiers to confirm the correct client
- D.** Return later when fewer staff are in the room

Correct Answer: C

When there is any doubt, the nurse must stop and verify the correct client using two identifiers and the record. Asking who is due for medication is not a reliable identifier because a confused or mistaken client may answer. Bed location is also unsafe because assignments can change.

844. A nurse is collecting a urine specimen from a client in a long-term care facility. Which identifier should the nurse avoid using as the primary method of client identification?

- A.** The client's full name
- B.** The client's date of birth
- C.** The client's medical record number
- D.** The client's room number

Correct Answer: D

Room number should not be used as a primary client identifier because clients may be moved or placed temporarily in another room. Full name, date of birth, and medical record number are accepted identifiers when matched to the record and identification band according to facility policy.

845. Before transfusing blood, the nurse brings the blood product to the bedside. Which action is the priority for client identification?

- A. Ask the client whether they have had a transfusion before
- B. Compare the blood product label with the client's identifiers and the order at the bedside according to agency procedure
- C. Check the blood type written in the chart after starting the transfusion
- D. Confirm the client's identity by asking a visitor to state the client's name

Correct Answer: B

Blood administration requires strict bedside verification of the blood product, client identifiers, and the order according to agency procedure. Prior transfusion history and visitor confirmation do not establish identity. Waiting until after the transfusion starts is unsafe.

846. A nurse is about to administer an oral medication to a client who says, "Everyone here knows me. You don't need to check my band again." Which response by the nurse is best?

- A. "I still need to verify your identity each time to help make sure the medication goes to the right client."
- B. "That is fine since I cared for you yesterday."
- C. "I'll skip it this once because you are in a private room."
- D. "Can your spouse confirm your name for me instead?"

Correct Answer: A

Client identification must be performed consistently before care such as medication administration, even if the nurse knows the client. Familiarity, private room status, or spouse confirmation do not replace proper identification. The best response is respectful and explains the safety reason.

847. A postoperative client returns from surgery without an identification band. The nurse notes that an IV antibiotic is due now. What should the nurse do first?

- A. Administer the antibiotic on time and apply a new band afterward
- B. Use the room assignment and report sheet to identify the client

C. Delay all care until the next shift arrives

D. Follow agency procedure to verify the client's identity and replace the missing identification band before medication administration

Correct Answer: D

A missing identification band is a safety risk that must be addressed before medication administration. The nurse should follow agency procedure to verify identity and replace the band. Room assignment and report sheets are not sufficient identifiers, and giving medication first increases the risk of error.

848. A nurse prepares to collect a blood specimen. The label printed from the laboratory system lists Maria L. Santos, but the client's band reads Maria E. Santos. The date of birth matches. Which action should the nurse take?

- A. Collect the specimen because the first and last names match
- B. Ask the client which middle initial is correct and label the tube with that answer
- C. Stop and resolve the identification discrepancy before collecting the specimen
- D. Use the date of birth only because it matches on both documents

Correct Answer: C

Any discrepancy between identifiers must be resolved before specimen collection. Matching first and last names or date of birth alone is not enough when another identifier conflicts. Asking the client may help gather information, but the nurse still must correct the discrepancy in the system or documentation before proceeding.

849. In the emergency department, two clients arrive within minutes of each other with similar names. Which nursing action best reduces the risk of identification error?

- A. Place both charts together so staff can compare them quickly
- B. Use extra caution by verifying two identifiers before each medication, specimen, or procedure
- C. Refer to the clients by their diagnoses instead of their names
- D. Rely on the triage nurse's memory for which client is which

Correct Answer: B

Similar names increase the chance of wrong-client errors, so the nurse should be especially careful to verify two identifiers before every medication, specimen, or procedure. Diagnoses, chart placement, and memory are not safe substitutes for direct identifier checks.

850. A nurse asks a client, "Are you Ms. Patel?" The client nods yes. Why is this method of identification unsafe?

- A.** It may lead the client to agree without independently stating identifiers
- B.** It takes too much time during a busy medication pass
- C.** It is only unsafe for clients older than 65 years
- D.** It should be used only for invasive procedures, not medications

Correct Answer: A

Yes/no questions can cue the client to agree even when the identity is wrong. A safer approach is to ask the client to state identifiers such as full name and date of birth, then compare them with the band and record. The problem is not age, procedure type, or time.

851. A nurse must administer insulin to a client who has expressive aphasia after a stroke. The client is alert, follows commands, and points accurately. Which approach is best for identification?

- A.** Skip identification because the client cannot speak clearly
- B.** Ask a family member to identify the client and proceed
- C.** Use the identification band and have the client indicate agreement to printed identifiers, then match them to the medication record
- D.** Identify the client by diagnosis and bed location

Correct Answer: C

A client with expressive aphasia may still participate in identification using alternative communication, such as pointing or indicating agreement to printed identifiers, while the nurse checks the ID band and record. Family report, diagnosis, and bed location do not replace direct verification.

852. The nurse is taking a client to radiology for a scheduled procedure. Which action best supports correct client identification during the handoff?

- A.** Tell the receiving staff the client's room number and procedure name
- B.** Ask the client to state identifiers and compare them with the identification band and transport documentation
- C.** Assume identification was already checked on the nursing unit
- D.** Send the chart with the transporter and let radiology verify later

Correct Answer: B

Transitions in care are common points for wrong-client errors. The safest action is to verify identifiers with the client, ID band, and transport documentation during handoff. Room number, assumptions, and delayed verification increase risk.

853. A nurse scans a client's bar code before administering medication, but the scanner displays an alert that the medication is not matched to this client. The client states, "That's mine. I always take it." What should the nurse do?

- A.** Override the alert because the client recognizes the medication
- B.** Administer the medication if the pill color matches yesterday's dose
- C.** Stop and investigate the mismatch before giving the medication
- D.** Ask another nurse whether this client usually takes the drug

Correct Answer: C

A bar-code mismatch may signal a wrong-client or wrong-medication error. The nurse should stop and resolve the problem before administration. Client familiarity with the medication, pill appearance, or another nurse's memory does not safely clear the alert.

854. Which situation requires the nurse to verify client identity using identifiers immediately before the action?

- A.** Giving a medication that was prepared 10 minutes earlier
- B.** Charting routine intake from the previous shift

- C. Restocking clean linens in an empty room
- D. Reviewing unit staffing assignments

Correct Answer: A

Client identity should be verified immediately before actions such as medication administration, specimen collection, treatments, and procedures. The other tasks do not involve direct client-specific care at that moment and do not require bedside identity verification in the same way.

855. During a home health visit, the nurse arrives to perform wound care for a new client. Which action is most appropriate to confirm the correct client in the home setting?

- A. Assume the person who answered the door is the scheduled client
- B. Ask the client to state identifying information and compare it with the visit documentation before beginning care
- C. Use the address alone because the appointment was scheduled in advance
- D. Rely on the caregiver's statement that this is the right person

Correct Answer: B

Client identification is still necessary in home care. The nurse should ask the client to state identifying information and compare it with the visit documentation before providing care. Address, assumptions, and caregiver confirmation alone are not sufficient safeguards.

856. A nurse is preparing to administer an intramuscular vaccine in an outpatient clinic. The client is wearing an identification band, but the nurse has not met the client before. Which action best confirms the client's identity before giving the injection?

- A. Ask the client to state their full name and date of birth, then compare the information with the identification band and medication record
- B. Check the room assignment against the clinic schedule and proceed if they match
- C. Ask the accompanying family member to confirm the client's name
- D. Verify the client's last name only because the first name is already on the vaccination consent form

Correct Answer: A

Using at least two client identifiers supplied by the client, such as full name and date of birth, is the safest way to confirm identity before giving medication or an injection. Room location and family confirmation can support care but should not replace direct identification. Checking only one identifier, such as last name, increases the risk of error.

857. A nurse enters a room to remove a peripheral IV catheter. The client is asleep, and the nurse recognizes the client from earlier in the shift. What should the nurse do first?

- A. Proceed because the nurse already cared for the client earlier
- B. Check the medication administration record to see which client is assigned to the bed
- C. Verify the client's identity using appropriate identifiers before removing the IV catheter
- D. Ask the roommate whether this is the correct client

Correct Answer: C

Identity must be verified immediately before care, treatment, or procedures, even if the nurse knows the client. Familiarity, bed assignment, and roommate confirmation are unsafe substitutes. Removing an IV is still a procedure and requires correct identification first.

858. On a pediatric unit, a parent asks why the nurse keeps checking the child's identification band before medications when the child has stayed in the same room for 3 days. Which response by the nurse is best?

- A. We only need to do that because this is a hospital policy for pediatric clients
- B. It helps make sure I am giving the medication to the correct child each time
- C. I check the band mainly to document that I entered the room
- D. Once I memorize your child's name, I will not need to check it as often

Correct Answer: B

The best response explains the safety purpose: verifying that the right child receives the right care each time. The check is not just about documentation or memorization, and it is not limited to one client group. Rechecking identity before medications helps prevent wrong-client errors.

859. A nurse in a rehabilitation center is assisting with lunch trays. Which method is most appropriate to identify the correct client before handing a tray to a client with a regular diet and no swallowing precautions?

- A. Ask, "Are you waiting for lunch?"
- B. Match the tray ticket to the client's room number only
- C. Ask the client to state their name and another identifier, then compare them with the tray information and identification band
- D. Give the tray to the client sitting closest to the bed with the matching name card

Correct Answer: C

Even for routine tasks such as meal delivery, the safest method is to use client-specific identifiers rather than room number, location, or a yes/no question. Asking the client to state identifying information reduces the risk of giving the wrong tray, which is especially important if diet restrictions exist.

860. A nurse is about to administer a scheduled inhaler. The client says, "My birthday is private, and I don't want to say it out loud in front of my visitor." Which nursing action is best?

- A. Skip the identity check because the client has expressed a privacy concern
- B. Ask the visitor to leave, then administer the inhaler without further identification
- C. Use another approved identifier and compare it with the identification band and medication record before administration
- D. Ask the visitor to verify the client's birthday instead

Correct Answer: C

The nurse still must verify identity before medication administration. If one identifier is not practical because of privacy concerns, another approved identifier can be used. The key is to use appropriate identifiers and compare them with the record and band. Visitor confirmation and skipping verification are unsafe.

861. A client arrives on the unit from the emergency department for admission. The nurse notices the identification band is present but the print is smudged and difficult to read. The client is alert and asking for pain medication. What is the nurse's priority action?

- A. Administer the pain medication because the client can verbally state their name
- B. Replace or correct the unreadable identification band before giving medications or performing other care requiring identification
- C. Use the room assignment as the second identifier until a new band is printed
- D. Ask another nurse who received report to confirm the client's identity

Correct Answer: B

An unreadable identification band creates a safety risk. Before medication administration, the nurse should ensure the client's identification is accurate and readable. A verbal statement alone is not enough when the band cannot be properly checked, and room number or staff recognition should not be used as substitutes.

862. A nurse is collecting a stool specimen from a client who is confused and unable to answer questions reliably. The client's adult child is at the bedside. Which action best identifies the client before specimen collection?

- A. Use the identification band and compare it with the order and medical record
- B. Rely on the adult child to state the client's name
- C. Confirm the client's identity by room number and diagnosis
- D. Wait until the client becomes more oriented later in the day

Correct Answer: A

When a client cannot reliably state identifiers, the nurse should use available approved identifiers such as the identification band and compare them with the order and record. Family members may be helpful but are not the primary method. Room number and diagnosis are not acceptable client identifiers.

863. A nurse is preparing a client for transport to the operating room. The transport staff member says, "I know this patient from yesterday," and reaches for the stretcher. Which nursing action is best?

- A.** Allow transport because staff familiarity is sufficient in urgent workflows
- B.** Verify the client's identity with approved identifiers and compare them with the procedure documentation before transport
- C.** Ask the client whether they are going for surgery and proceed if they say yes
- D.** Send the client if the consent form is signed and placed in the chart

Correct Answer: B

Before transport for a procedure, the nurse should verify the correct client using approved identifiers and match them to the procedure documentation. Staff familiarity, a yes/no question, or the presence of a signed consent form alone does not safely confirm identity.

864. A nurse working in a dialysis center calls a client from the waiting room by first name. Two clients stand up. What should the nurse do next?

- A.** Choose the client who arrived first because the schedule is usually in order
- B.** Ask each client to state identifying information and verify it against the treatment record before proceeding
- C.** Have both clients sit near the dialysis station and sort it out with the charge nurse later
- D.** Use the client's appearance from prior visits to decide who is correct

Correct Answer: B

When more than one person responds, the nurse should stop and verify identity using approved identifiers. Arrival order and appearance are unsafe shortcuts. The risk of wrong-client treatment is high in settings where clients may have similar or familiar names.

865. A nurse is about to administer a medication through a feeding tube to a nonverbal client. The client has severe contractures and cannot point or write. Which method is most appropriate for client identification?

- A.** Use the identification band and compare it with the medication record and order
- B.** Ask the nursing assistant assigned to the room whether this is the right client
- C.** Confirm the client's identity from the photograph posted at the bedside
- D.** Use the bed number because the client cannot participate verbally

Correct Answer: A

When the client cannot actively provide identifiers, the nurse should use the identification band and compare it with the relevant records before medication administration. A staff member's recognition, bedside photo, or bed number may seem helpful but should not replace proper identification.

866. A nurse prepares morning medications for four clients. At the bedside of one client, the nurse notices the medication cup label and the identification band do not match. The client says, "They changed my room last night, so the labels are probably mixed up." Which action should the nurse take?

- A.** Give the medications if the client can describe the pills from memory
- B.** Ask the charge nurse whether a room change occurred and then give the medications
- C.** Hold the medications and resolve the discrepancy by verifying the client's identity and obtaining the correct medications for that client
- D.** Administer only the medications that look routine and delay the rest

Correct Answer: C

Any mismatch between the medication label and the client's identifiers must be treated as a safety issue. The nurse should stop, verify the client's identity, and obtain the correct medications. Client familiarity with pills, room-change explanations, or partial administration do not safely resolve the discrepancy.

867. A community clinic nurse is obtaining a capillary blood sample from a client who speaks very little English. A trained interpreter is present by video. Which action best supports safe client identification?

- A.** Ask the interpreter to identify the client by appearance
- B.** Have the interpreter ask the client to state identifying information, then compare it with the clinic record
- C.** Use the appointment time as the second identifier because language barriers make direct verification impractical
- D.** Ask another client in the waiting area to confirm who arrived with the interpreter connection

Correct Answer: B

A language barrier does not remove the need for proper client identification. Using a trained interpreter to obtain the client's stated identifiers is the safest option. Appearance, appointment time, or another client's input are unreliable and increase the risk of misidentification.

868. A nurse is assisting with electrocardiograms on a busy telemetry unit. The nurse prints a rhythm strip and realizes it may have been placed in the wrong client's chart because two clients have similar last names. Which action is most appropriate?

- A.** Leave the strip in the chart if the cardiac rhythm appears normal
- B.** Cross out one name and handwrite the other if the nurse is fairly sure whose strip it is
- C.** Report and correct the identification error according to facility process so the strip is matched to the correct client record
- D.** Discard the strip and repeat the test later if the provider requests it

Correct Answer: C

Potential identification errors in documentation or diagnostic records should be addressed promptly through the proper correction process. Leaving the strip, guessing, or discarding it can lead to wrong-client decisions and loss of clinical data. The key safety step is to recognize the mismatch and correct it appropriately.

869. A nurse in labor and delivery receives a newborn from the warmer after assessment. The infant's identification band has become detached and is lying on the bassinet mattress. What should the nurse do first before taking the newborn to the mother for feeding?

- A.** Reattach the same band to the infant if the nurse believes it belongs to that newborn
- B.** Bring the newborn to the mother because she can visually identify the baby
- C.** Follow facility procedure to reestablish correct identification before transferring the newborn
- D.** Write the mother's room number on tape and place it on the bassinet temporarily

Correct Answer: C

A detached infant identification band creates a high-risk identification problem. The nurse should not guess, rely on visual recognition, or use temporary unofficial labeling. The correct action is to follow the procedure that reestablishes verified identification before transfer or care continues.

870. A nurse is giving bedside shift report in a semiprivate room. Before discussing a wound care plan, the oncoming nurse asks the awake client to state their full name and date of birth. The offgoing nurse says, "That's not necessary. The chart is already open, and we're standing at the right bed." Which response by the oncoming nurse is best?

- A.** The bed location is enough during handoff as long as both nurses agree
- B.** The client should still be identified with approved identifiers before we continue care-related discussion and treatment planning
- C.** Identification is only required before medications and invasive procedures

D. We can skip identification now and check it later if a provider writes a new order

Correct Answer: B

Correct client identification supports safe ongoing care, not just medication administration. During handoff, using approved identifiers helps ensure that the discussion and subsequent treatment planning apply to the correct client. Bed location and staff agreement are unsafe substitutes, especially in shared rooms.

871. A nurse is preparing to give a prefilled insulin pen injection to a client in a long-term care facility. The nurse has cared for the client for several weeks and recognizes the client immediately. Which action is most appropriate before administering the insulin?

- A.** Ask the client to state their full name and date of birth, and compare the response with the identification band and medication record
- B.** Confirm the client's identity by room number and the nurse's prior familiarity with the client
- C.** Ask another nurse if this is the correct client for the insulin dose
- D.** Administer the insulin because the client is well known to the staff

Correct Answer: A

Using at least two client identifiers is the safest method before medication administration. Prior familiarity, room number, and another staff member's recognition are not reliable identifiers because clients can be moved, confused, or mistaken for one another.

872. A nurse is preparing labels for blood specimens to be drawn from several clients during morning rounds. Which action best supports correct client identification?

- A.** Label each tube at the nurses' station before entering the rooms to save time
- B.** Draw all specimens first and label them together after the rounds are complete
- C.** Identify each client with two identifiers and label the specimen at the bedside after collection
- D.** Use the room number on the specimen tube if the client is unable to speak

Correct Answer: C

Specimens should be connected to the correct client at the time of collection by using two identifiers and labeling at the bedside. Prelabeling or delaying labeling increases the risk of mix-ups. Room number is not an acceptable client identifier.

873. A nurse enters a semiprivate room to obtain a finger-stick blood glucose reading before breakfast. Both clients are awake. What is the best way to identify the correct client?

- A.** Ask the roommate to confirm which client has diabetes
- B.** Check the bed assignment on the doorway and proceed with the test
- C.** Ask the client to state their name and date of birth, then compare this with the identification band
- D.** Use the meal tray card to verify the client's identity

Correct Answer: C

The nurse should use two client identifiers, such as the client's stated name and date of birth, and compare them with the identification band. A roommate, doorway bed assignment, or tray card can be wrong and should not replace direct identification.

874. A nurse is taking a client from the unit to radiology for a chest x-ray. Which identifier should the nurse avoid using as a primary means of confirming the client's identity?

- A.** Full name
- B.** Date of birth
- C.** Medical record number
- D.** Room number

Correct Answer: D

Room number is not an acceptable client identifier because clients may be moved. Full name, date of birth, and medical record number are examples of identifiers that can be used to confirm the correct client according to facility policy.

875. A nurse is preparing to administer an oral antibiotic to a client who is alert but has expressive aphasia after a stroke. The client can nod yes or no accurately. Which action is best to verify the client's identity?

- A.** Ask the family member at the bedside to state the client's name and date of birth
- B.** Ask yes-or-no questions while comparing the answers with the identification band and medication record
- C.** Skip identification because the client cannot speak clearly
- D.** Use the diagnosis of stroke listed on the room whiteboard as the second identifier

Correct Answer: B

When a client cannot state identifiers in the usual way, the nurse should still use available, reliable methods to confirm identity, such as yes-or-no responses checked against the identification band and record. Family input may help, but it should not replace direct verification when the client can participate. A diagnosis and whiteboard are not identifiers.

876. A nurse in a same-day surgery center brings a client from the waiting area to the preoperative bay. The client answers to the name called but says, "People mix me up with my father all the time because we have the same first and last name." Which action is best next?

- A.** Proceed if the scheduled procedure on the chart matches what the client expects
- B.** Use the client's full name and one additional identifier, such as date of birth or medical record number
- C.** Ask the client's father whether this is the correct person
- D.** Rely on the fact that the client responded when called from the waiting room

Correct Answer: B

A shared name increases the risk of wrong-client errors, so the nurse should verify identity with the full name plus a second approved identifier such as date of birth or medical record number. Recognizing a procedure or answering to a name alone is not enough.

877. A nurse is about to administer a PRN pain medication to an older adult who is mildly confused. The client states a different last name than the one on the identification band. The medication is due now, and the client reports severe pain. What should the nurse do?

- A.** Give the medication because pain relief should not be delayed
- B.** Ask the nursing assistant whether the band has the correct name
- C.** Hold the medication and resolve the identification discrepancy before administering it
- D.** Use the client's diagnosis and room number to confirm identity

Correct Answer: C

Any discrepancy in client identification must be resolved before medication administration. Even when the client is in pain, the nurse must prevent a possible wrong-client medication error. Room number, diagnosis, and informal staff recognition are not acceptable substitutes.

878. A nurse is obtaining consent-related preprocedure vital signs for a client who says, "You don't need to keep asking who I am. Just look in the computer." Which response by the nurse is best?

- A.** I need to verify your identity each time care is provided to help prevent errors
- B.** The computer is not reliable, so I never use it for identification
- C.** If you are annoyed, I can skip the identification step this once
- D.** I only need to verify your identity before medications, not before other care

Correct Answer: A

The best response explains that repeated identification is a safety measure used to prevent wrong-client errors. The nurse should not skip the process, and identification is not limited only to medication administration.

879. A nurse is preparing to administer a blood transfusion. The client is alert and wearing an identification band. Which action is most important to reduce the risk of a wrong-client transfusion?

- A.** Ask the client to confirm the reason for hospitalization
- B.** Compare the blood product information with the client's identifiers according to facility policy immediately before administration
- C.** Verify the client's identity when the blood arrives on the unit and assume it remains correct at the bedside
- D.** Confirm the blood type with the physician before starting the transfusion

Correct Answer: B

Transfusions require strict bedside verification of the blood product and the client's identifiers immediately before administration. Confirming the reason for hospitalization is not an identifier, and checking only earlier on the unit is insufficient because the final match must occur at the bedside.

880. A nurse is caring for a client with dementia who repeatedly removes the identification band. A prescribed medication is due now. Which action is best?

- A.** Use the photo posted in the chart and administer the medication immediately
- B.** Ask the client's roommate to identify the client
- C.** Replace or obtain a readable identification band and verify the client with approved identifiers before giving the medication
- D.** Document that the client is well known to staff and proceed

Correct Answer: C

The nurse should ensure the client has a valid, readable identification band and verify identity with approved identifiers before medication administration. A photo, roommate confirmation, or staff familiarity may assist recognition but should not replace required identification procedures.

881. A nurse receives two medication packets from the pharmacy for clients named Maria Lopez and Marina Lopez. Both clients are on the same unit. What is the nurse's safest action before either medication is given?

- A.** Separate the packets by room number and give each medication at the scheduled time
- B.** Use only the last name because first names can be pronounced differently
- C.** Verify each medication against two client identifiers and the medication record at the bedside
- D.** Ask whichever client answers first to confirm both medications belong on the unit

Correct Answer: C

Similar names create a high risk for wrong-client errors. The nurse should carefully verify each medication with two client identifiers and the medication record at the bedside. Room number alone and name-based assumptions are unsafe.

882. A nurse in the emergency department is preparing to send a urine specimen to the laboratory. The client was registered quickly during triage, and the nurse notices that the requisition lists one digit of the medical record number differently from the wristband. The client is waiting for discharge instructions. What should the nurse do first?

- A.** Send the specimen because the client's name is correct
- B.** Correct the laboratory slip by hand and send the specimen immediately
- C.** Stop and resolve the identification mismatch before the specimen leaves the bedside
- D.** Ask the unit clerk to decide which number is probably correct

Correct Answer: C

Any mismatch in identifiers must be resolved before the specimen is sent. Even a single-digit error can lead to results being assigned to the wrong client. Sending it anyway or making assumptions creates a serious safety risk.

883. A nurse is assisting with care after a mass-casualty event. Several clients arrive rapidly, and one unconscious client does not yet have complete demographic information. Which approach best supports safe ongoing identification during care?

- A. Use the client's apparent age and injury pattern until registration is complete
- B. Assign and maintain a unique temporary identifier according to facility policy and use it consistently on all care and specimens
- C. Place the client in the closest open bed and use that bed number as the identifier
- D. Wait to provide treatments until the client's family can confirm identity

Correct Answer: B

When a client's full demographic information is not yet available, a unique temporary identifier must be assigned and used consistently to reduce wrong-client errors. Bed number and appearance are unreliable, and urgent care should not be delayed while waiting for family if treatment can proceed safely with established emergency identification procedures.

884. A nurse prepares to administer an IV antibiotic to a client who is lethargic and receiving oxygen. The electronic medication administration record displays the client's name, but the wristband barcode will not scan because it is damaged. Which action is best?

- A. Administer the antibiotic because the electronic record already identifies the client
- B. Use the room assignment and diagnosis as backup identifiers
- C. Obtain a new readable wristband or follow downtime identification procedure to verify the client with approved identifiers before administration
- D. Ask respiratory therapy whether this is the same client they treated earlier

Correct Answer: C

A damaged band that cannot be reliably scanned or read requires correction or use of the facility's approved downtime identification process before medication administration. The electronic record alone, room assignment, diagnosis, or another clinician's recognition does not replace proper client identification.

Safe equipment use

885. A nurse is preparing to transfer a weak client from the bed to a chair. Which action best promotes safe equipment use before the transfer begins?

- A. Lock the bed and wheelchair brakes
- B. Raise the bed to its highest position
- C. Place the wheelchair facing away from the bed
- D. Ask the client to hold onto the nurse's neck

Correct Answer: A

Locking the bed and wheelchair brakes helps prevent unexpected movement during the transfer and is a basic safety step when using transfer equipment. Raising the bed too high can increase fall risk, placing the wheelchair facing away makes transfer harder and less safe, and having the client hold the nurse's neck is unsafe for both the client and nurse.

886. A nurse enters a client's room and sees that the side rail of the hospital bed is loose and wobbly. What should the nurse do first?

- A. Lower the rail and leave it down until maintenance can come later
- B. Use the bed carefully and remind the client not to lean on the rail
- C. Remove the bed from use and report the equipment problem
- D. Tighten the rail with supplies from the unit drawer

Correct Answer: C

A loose side rail is a safety hazard. The safest first action is to remove the bed from use if possible, protect the client, and report the malfunction through the proper process. Continuing to use the equipment is unsafe, and staff should not perform unauthorized repairs.

887. A client uses oxygen by nasal cannula at 2 L/min. Which item in the room should the nurse identify as the greatest safety concern?

- A. Water-based lip moisturizer at the bedside
- B. A wool blanket folded on the chair
- C. An electrical razor plugged in across the room
- D. A visitor using a lighter near the window

Correct Answer: D

Open flame is the greatest immediate hazard around oxygen because oxygen supports combustion and greatly increases fire risk. A water-based moisturizer is generally preferred over petroleum products. The plugged-in razor is less dangerous than an active flame, and the blanket is not the most urgent concern.

888. A nurse is about to use a portable suction machine for a client with thick oral secretions. Which check is most important before starting suction?

- A. Whether the suction pressure is functioning and set appropriately
- B. Whether the client drank water within the last hour
- C. Whether the bed linens match the unit standard
- D. Whether the client has a family member in the room

Correct Answer: A

Before suctioning, the nurse should verify that the equipment works and that suction pressure is appropriately set. This directly affects safe and effective use. The other options do not determine whether the suction device is safe to use.

889. A nurse is teaching an assistive personnel (AP) how to use a mechanical lift. Which statement by the AP indicates a need for further teaching?

- A. I should check the lift's weight limit before using it.
- B. I should inspect the sling for tears before the transfer.
- C. I can transfer the client alone if the sling is already in place.
- D. I should position the client according to the manufacturer's directions.

Correct Answer: C

Mechanical lifts often require more than one trained staff member, depending on policy and client condition. Assuming the transfer can be done alone is unsafe. Checking weight limits, inspecting the sling, and following manufacturer guidance are appropriate safety steps.

890. A client reports feeling dizzy while standing on a scale during a daily weight measurement. What is the nurse's best action?

- A. Tell the client to remain still until the reading is complete
- B. Support the client and help the client off the scale safely
- C. Leave briefly to get a wheelchair after the weight is recorded
- D. Ask the client to hold the wall rail and continue

Correct Answer: B

Client safety takes priority over obtaining the measurement. If the client becomes dizzy, the nurse should immediately support the client and assist off the scale to prevent a fall. The distractors focus on completing the task instead of addressing the safety risk.

891. The nurse is preparing to use a blood pressure machine that has a frayed electrical cord. Which action is most appropriate?

- A. Wrap the cord with tape and use the machine for this one reading
- B. Plug it into a different outlet to see if it works better
- C. Tag the machine as unsafe and remove it from service
- D. Use the machine only if the client is not touching metal

Correct Answer: C

A frayed electrical cord creates a risk for shock or fire. The device should be removed from service and reported according to policy. Temporary fixes, outlet changes, or conditional use do not make damaged electrical equipment safe.

892. A nurse is preparing an enteral feeding pump for a client who receives tube feedings. Which action requires follow-up?

- A. The nurse confirms the formula matches the prescription.
- B. The nurse checks the tubing connections before starting the pump.
- C. The nurse programs the pump after seeing an error code remain on the screen.
- D. The nurse verifies that the pump is plugged in and charged.

Correct Answer: C

An unresolved error code suggests the equipment may not be functioning correctly. The nurse should not proceed until the problem is addressed. Confirming the formula, checking tubing, and verifying power supply are appropriate safety steps.

893. A nurse is selecting a cuff for an automated blood pressure machine. Why is proper cuff size important for safe equipment use?

- A. It helps prevent inaccurate readings
- B. It prevents the machine from losing battery power
- C. It reduces the need to document vital signs
- D. It allows blood pressure to be measured through clothing

Correct Answer: A

Using the correct cuff size is essential because a cuff that is too small or too large can produce inaccurate blood pressure readings, which can lead to poor clinical decisions. The other choices do not reflect the main safety reason for selecting the correct cuff.

894. A nurse is preparing to use a sequential compression device (SCD) for a postoperative client. Which finding should the nurse report before applying the device?

- A. The client says the sleeves feel warm after use
- B. The client has redness and swelling in one calf
- C. The sleeves make a soft rhythmic sound
- D. The tubing lies across the foot of the bed

Correct Answer: B

Redness and swelling in one calf may indicate a possible deep vein thrombosis. Applying an SCD in that setting requires follow-up because compression could be harmful if a clot is present. Warmth from functioning sleeves and rhythmic inflation sounds are expected, and tubing placement is a setup issue rather than the key client finding here.

895. A nurse is using a gait belt to assist a client from bed to the bathroom. The client suddenly says, "My knees are giving out." Which action is best?

- A. Hold the gait belt and lower the client to the floor in a controlled manner
- B. Pull upward quickly on the gait belt to keep the client standing
- C. Step away and call for help before touching the client
- D. Support the client under the arms and twist toward the chair

Correct Answer: A

If a client begins to fall while using a gait belt, the nurse should use the belt for support and lower the client to the floor in a controlled way. Pulling upward suddenly can injure the client or nurse. Supporting under the arms and twisting increases injury risk, and stepping away abandons the client.

896. A nurse is checking a client's IV pump and finds that the infusion is running faster than prescribed because the pump was programmed incorrectly. Which action should the nurse take first?

- A. Document the total amount infused and continue to monitor
- B. Reprogram the pump to the prescribed rate
- C. Stop and assess the client, then correct the infusion setup
- D. Wait until the next scheduled assessment to notify the provider

Correct Answer: C

When an infusion is running too fast, the nurse should first stop the unsafe delivery and assess the client for effects of excess infusion, then correct the setup and follow reporting procedures. Simply reprogramming without first addressing the current client risk is incomplete. Waiting or only documenting delays action.

897. A nurse notes that a bed alarm has been sounding repeatedly because a confused client keeps shifting position. Which action is best?

- A. Turn off the alarm so the client can rest
- B. Lower the alarm volume and rely on hourly rounds
- C. Assess the client and ensure the alarm is correctly placed and functioning

D. Move the client farther from the nurses' station to reduce noise

Correct Answer: C

The nurse should respond by assessing the client and verifying that the alarm is correctly positioned and working as intended. Frequent alarms may reflect client movement, poor placement, or a need for additional fall-prevention measures. Turning the alarm off or reducing response to it increases risk.

898. A newly licensed nurse is preparing to use an unfamiliar bladder scanner. Which action is most appropriate?

- A.** Use the scanner after watching a coworker use it once
- B.** Review the operating instructions and obtain help if needed before use
- C.** Try the scanner on the client and repeat the scan until the readings seem right
- D.** Skip the scan and document that the equipment was unavailable

Correct Answer: B

Safe equipment use includes following operating instructions and seeking assistance before using unfamiliar equipment. Watching once may not be enough for safe use, trial-and-error on the client is unsafe, and falsely documenting unavailability is inappropriate.

899. A client with limited mobility has been placed on a chair with a seat alarm. Which action by the nurse best supports safe use of this equipment?

- A.** Place a folded blanket over the alarm sensor to improve comfort
- B.** Test the alarm to confirm it activates when the client rises
- C.** Silence the alarm during meals to reduce agitation
- D.** Tell the client the alarm will prevent falls without staff help

Correct Answer: B

Testing the alarm confirms that the safety device is functioning properly before relying on it. Covering the sensor can interfere with function, silencing it removes protection, and alarms do not replace staff assistance or guarantee fall prevention.

900. A nurse is preparing to move a client who has left-sided weakness from the stretcher to the bed using a transfer board. Which action is most important to complete before the move begins?

- A.** Raise the bed to the highest position
- B.** Lock the wheels of both the bed and the stretcher
- C.** Place a pillow under the client's weak arm
- D.** Ask the client to cross both ankles

Correct Answer: B

Locking the wheels of both surfaces prevents unexpected movement during the transfer and is a key safety step before equipment is used. Raising the bed too high can increase risk, and positioning the arm or ankles does not address the main hazard of the equipment shifting.

901. A nurse is about to place a client in a wheelchair for transport to radiology. Which action best verifies that the wheelchair is safe to use?

- A.** Check that the footrests swing away and the wheel locks engage
- B.** Place a blanket over the seat before assisting the client
- C.** Lower the handles to reduce the chair's height
- D.** Keep the client's belongings on the client's lap

Correct Answer: A

A safe wheelchair check includes making sure the locks work and the footrests can be moved out of the way for transfer. Those features directly affect safe equipment use. The other options may improve comfort or convenience but do not confirm that the wheelchair itself is safe for transfer and transport.

902. A nurse is caring for a client who uses a bedside commode. During setup, the nurse notices one commode leg is shorter than the others and the frame rocks slightly. Which action is best?

- A.** Place a folded towel under the short leg
- B.** Use the commode only with two staff members assisting
- C.** Remove the commode from use and obtain one that is stable
- D.** Tell the client to call for help before standing

Correct Answer: C

Equipment that is unstable should be removed from use and replaced. A rocking commode creates a fall risk. Padding the leg is an unsafe workaround, and extra staff or instructions do not correct a defective device.

903. A nurse is preparing to use a heating pad prescribed for a client with muscle soreness. Which action is most important for safe equipment use?

- A. Place the heating pad directly under the client to keep it in position
- B. Cover the heating pad with several thick blankets
- C. Inspect the pad and cord for damage before applying it
- D. Set the pad to the highest temperature so treatment works faster

Correct Answer: C

Electrical heating equipment should be inspected for damage before use to reduce risk of burns, shock, or fire. Placing a heating pad under the client, covering it heavily, or turning it to the highest setting increases risk rather than promoting safety.

904. A nurse is teaching an assistive personnel (AP) how to use a portable pulse oximeter. Which statement by the AP shows correct understanding of safe equipment use?

- A. I should place the probe on any finger, even if nail polish is present.
- B. If the reading looks wrong, I should first check probe placement and client motion.
- C. If the monitor alarms, I can silence it and recheck the client later.
- D. I should leave the probe in the same position all day to avoid disturbing the client.

Correct Answer: B

An unexpected pulse oximeter reading may result from poor placement or motion artifact, so checking the sensor and the client first is appropriate. Nail polish can interfere with readings, alarms should not be ignored, and prolonged unchanged probe placement can affect skin integrity.

905. A nurse is about to assist a client with ambulation using a walker. Which finding requires the nurse to stop and adjust the equipment before the client stands?

- A. The client is wearing non-skid footwear
- B. The walker is positioned at the bedside
- C. The walker's height causes the client's elbows to remain fully straight
- D. The client says, "I used a walker at home before surgery."

Correct Answer: C

A walker should be adjusted to an appropriate height so the client can use it safely with slight elbow flexion. If the elbows are fully straight, the walker height is not appropriate and can reduce stability. The other findings do not indicate an equipment problem requiring immediate adjustment.

906. A nurse is preparing a sterile dressing change and opens a sterile kit onto the bedside table. The nurse then notices the table is damp from a recent spill. Which action is most appropriate?

- A. Proceed if the supplies in the center of the field appear dry
- B. Place a dry towel under the sterile field and continue
- C. Discard the setup and obtain new supplies on a dry surface
- D. Ask another nurse to watch the field while the dressing is changed

Correct Answer: C

Moisture can contaminate a sterile field by strike-through, so supplies opened onto a damp surface should be discarded and replaced. A towel underneath after the fact does not restore sterility. Watching the field does not correct contamination.

907. A home health nurse visits a client who uses oxygen from a concentrator. Which observation requires immediate correction?

- A. The tubing is secured loosely to avoid pulling on the nares
- B. The concentrator is plugged into an extension cord behind the bed
- C. A glass of water is on the bedside table
- D. The client has tissues and lip balm within reach

Correct Answer: B

Oxygen equipment should not be plugged into an extension cord because of electrical and fire risk. The other findings are not the same level of equipment hazard. The immediate concern is unsafe power setup for the oxygen device.

908. A nurse is using a bed in the low position with wheels locked and a call light within reach for a client at high risk for falls. During rounds, the nurse finds the bed alarm cord is disconnected from the alarm unit. What is the nurse's priority action?

- A. Reconnect the alarm system and verify that it functions
- B. Document that the client remains at high risk for falls
- C. Ask the family to remain at the bedside
- D. Move the client to a semi-private room near the nurses' station

Correct Answer: A

The immediate equipment safety issue is that the alarm is not functioning as intended, so the nurse should reconnect it and confirm that it works. Documentation and environmental changes may be helpful, but they do not fix the malfunctioning safety device.

909. A nurse prepares to use a portable commode for a client after surgery. Which instruction is best to include when teaching the client about safe use of the equipment?

- A. Push up on the commode frame as you stand if you feel weak
- B. Lean forward quickly so you can stand before dizziness starts
- C. Call for assistance before getting on or off the commode
- D. Keep both feet on the foot ring while transferring

Correct Answer: C

The safest instruction is to call for assistance before transfers, especially after surgery when weakness or orthostatic changes may occur. Pushing on an unstable frame, standing quickly, or using a foot ring during transfer can increase fall risk.

910. A nurse is assigned to apply antiembolism stockings for a client on bed rest. Which action demonstrates safe equipment use?

- A. Roll the top of the stocking down if it feels too tight
- B. Remove the stockings at least once per shift to assess the skin
- C. Leave the stockings on continuously unless the provider discontinues them
- D. Apply the stockings only after the client begins ambulating

Correct Answer: B

Antiembolism stockings should be removed periodically to assess skin and circulation. Rolling the top can create a constricting band, leaving them on continuously without checks is unsafe, and waiting until ambulation begins defeats the preventive purpose.

911. A nurse is preparing to weigh a client using a standing scale. The client has a weak right leg and uses a cane. Which action best promotes safe equipment use during the measurement?

- A. Ask the client to step onto the center of the scale while the nurse steadies the client
- B. Have the client hold onto the wall while standing on the scale
- C. Position the cane on the scale platform with the client
- D. Obtain the weight later when physical therapy is available

Correct Answer: A

The client should step onto the center of the scale with direct assistance for stability. Holding the wall may produce an inaccurate weight and does not provide ideal support. Putting the cane on the platform affects safety and accuracy, and delaying care is unnecessary when safe assistance can be provided now.

912. A nurse is caring for a client receiving enteral feedings through a pump. The pump alarms repeatedly for occlusion, but the tubing is not kinked and the client is not in distress. Which action is best?

- A. Increase the pump rate briefly to overcome the blockage
- B. Silence the alarm permanently and continue the feeding

- C. Stop and troubleshoot the equipment according to facility procedure before restarting
- D. Disconnect the tubing from the client and leave it open to air

Correct Answer: C

Repeated occlusion alarms indicate a problem that must be assessed before continuing the feeding. Safe equipment use means stopping and troubleshooting per policy, rather than overriding alarms. Increasing the rate, ignoring alarms, or disconnecting tubing unsafely can harm the client.

913. A nurse is preparing to use a mechanical lift to move a client from bed to chair. The client suddenly becomes restless and says, "I don't feel secure in this sling." Which action should the nurse take first?

- A. Continue the transfer quickly to reduce the client's anxiety
- B. Pause the transfer and reassess sling placement and attachment points
- C. Lower the client into the chair even if alignment is off
- D. Tell the client the sling always feels uncomfortable at first

Correct Answer: B

If the client reports feeling insecure during use of a mechanical lift, the first action is to pause and reassess whether the sling is correctly positioned and attached. Continuing despite concern risks injury. Reassurance alone is not enough when a possible equipment-placement problem exists.

914. A nurse receives a client from surgery with a patient-controlled analgesia (PCA) pump. The client's spouse says, "If she falls asleep, I'll push the button so she stays ahead of the pain." What is the nurse's best response?

- A. That is fine as long as you only do it when she appears uncomfortable.
- B. Please wait until the next pain assessment before using the button again.
- C. Only the client should activate the PCA button unless specific orders and safeguards are in place.
- D. Use the button for her, but only when a nurse is in the room.

Correct Answer: C

Safe use of PCA equipment depends on the client activating the button so sedation level and need are matched to self-report and responsiveness. Family activation creates risk of oversedation. The trap is thinking helping with pain control is supportive, but with PCA this can be unsafe.

915. A nurse plans to transfer a client from bed to chair with a gait belt. The client reports feeling dizzy when sitting up. Which action should the nurse take first to promote safe equipment use?

- A. Tighten the gait belt and continue the transfer quickly
- B. Return the client to a safe position and reassess before proceeding
- C. Ask the client to take deep breaths and stand on the count of three
- D. Place the chair farther away to allow more room to pivot

Correct Answer: B

If the client becomes dizzy, the nurse should stop the transfer, return the client to a safe position, and reassess. Safe equipment use includes using the device appropriately and recognizing when the transfer should not continue. Tightening the gait belt or moving quickly does not address the safety risk, and placing the chair farther away would make the transfer less safe.

916. A nurse is preparing to use a sequential compression device (SCD) on a client. Which action is most important before applying the sleeves?

- A. Check that the machine is plugged in and the tubing is connected correctly
- B. Apply lotion to both legs so the sleeves slide on more easily
- C. Wrap the sleeves as tightly as possible to improve compression
- D. Place the sleeves over the client's shoes to avoid contamination

Correct Answer: A

Before use, the nurse should verify that the equipment is functioning and assembled correctly. For an SCD, this includes checking power and tubing connections. Lotion is unnecessary and may affect skin condition, overly tight sleeves can cause harm, and shoes should not remain under the sleeves.

917. A nurse is about to use an electric hospital bed to raise a client for repositioning. When the nurse presses the control, the head of the bed jerks upward unevenly. What is the nurse's best action?

- A. Continue using the bed but move it more slowly
- B. Lower the bed, keep the client safe, and remove the bed from use until checked
- C. Ask another nurse whether the bed usually moves that way
- D. Place pillows around the client and finish repositioning

Correct Answer: B

Unexpected or uneven bed movement suggests malfunction. The nurse should stop using the equipment, protect the client, and have the bed checked according to facility process. Continuing to use malfunctioning equipment increases injury risk. Asking a coworker does not correct the hazard, and pillows do not make faulty equipment safe.

918. A nurse is preparing to check a client's temperature with an electronic thermometer. The probe cover package is open, but the cover inside appears unused. Which action is best?

- A. Use the cover because it does not look soiled
- B. Use the cover only if the previous shift opened it recently
- C. Discard the open cover and apply a new probe cover
- D. Wipe the cover with alcohol and then use it

Correct Answer: C

An opened package can no longer be assumed clean for safe patient use. The nurse should discard it and use a new probe cover. Appearance alone is not enough to determine safety, and wiping a disposable cover does not restore it to proper condition for use.

919. A nurse in a long-term care setting finds that the rubber tip is missing from one leg of a client's quad cane. The client says, "It's fine. I used it this morning." Which response by the nurse is best?

- A. Use the cane for this transfer only, then replace it afterward
- B. Tell the client to put less weight on the cane until a new tip arrives
- C. Do not use the cane and obtain safe replacement equipment
- D. Wrap the end with tape so the cane does not slide

Correct Answer: C

A missing rubber tip makes the cane unstable and increases the risk of slipping. The nurse should not use damaged equipment and should obtain a safe replacement. Temporary fixes such as tape or reduced weight-bearing do not make the cane reliably safe.

920. A nurse is preparing to bathe a weak client using a shower chair. Which action best demonstrates safe equipment use?

- A. Place the chair on a dry, level surface and verify stability before the client sits
- B. Allow the client to sit first, then adjust the chair legs for comfort
- C. Angle the chair on the sloped floor so water drains away faster
- D. Cover the chair feet with towels to prevent marks on the floor

Correct Answer: A

The safest action is to place the shower chair on a dry, level surface and ensure it is stable before the client uses it. Adjusting after the client sits creates fall risk. A sloped surface or towels under the feet can make the chair unstable.

921. A nurse prepares to use a portable suction machine for a client with thick oral secretions. Which finding requires immediate action before the equipment is used?

- A. The canister is attached securely
- B. The connecting tubing is cracked near the lid
- C. Extra suction catheters are stored in the drawer
- D. The machine is plugged into a wall outlet

Correct Answer: B

Cracked suction tubing can impair function and make the equipment unsafe or ineffective. The nurse should correct this before use. The other findings describe normal setup and do not by themselves make the equipment unsafe.

922. A nurse is setting up a bedpan for a client with limited mobility. To reduce injury risk during equipment use, which action is best?

- A. Raise the bed to the nurse's waist and leave it there until the client is finished
- B. Keep the side rails up on all four sides while the client is on the bedpan
- C. Place the bed in a safe working position, then return it to a low position when finished
- D. Ask the client to lift the hips quickly so the bedpan can be inserted in one motion

Correct Answer: C

The bed may be raised temporarily for safe body mechanics during care, but after the task it should be returned to a low position to reduce fall risk. Leaving the bed high is unsafe. Four raised side rails may function as a restraint depending on context, and rushing the client can increase discomfort and injury risk.

923. A nurse enters a client's room to use a portable blood pressure machine and sees the machine's power cord stretched across the floor from the wall outlet to the bedside. The client needs assistance to the bathroom now. What should the nurse do first?

- A. Proceed quickly before the client notices the cord
- B. Move or secure the cord to eliminate the tripping hazard
- C. Tell the client to step over the cord carefully
- D. Unplug the machine and leave it in the middle of the walkway

Correct Answer: B

A stretched power cord across a walking path is an immediate environmental hazard. The nurse should first remove or secure the cord before assisting the client. Telling the client to step over it still leaves a fall risk, and unplugging it without moving it does not eliminate the hazard.

924. A nurse is preparing to use a sit-to-stand lift for a client who can bear some weight. Which assessment finding indicates the nurse should stop and re-evaluate the plan?

- A. The client is wearing non-skid footwear
- B. The client can follow simple directions
- C. The client suddenly cannot maintain head or trunk control
- D. The client asks how long the transfer will take

Correct Answer: C

A sit-to-stand lift requires enough physical control and participation to use it safely. If the client cannot maintain head or trunk control, the nurse should stop and reassess whether a different transfer method is needed. Non-skid footwear and ability to follow directions support safe use, and asking questions does not make the device unsafe.

925. During a pre-use check of an infusion pump, the nurse notices that the pump's last inspection sticker expired 2 months ago. The pump powers on and appears to run normally. What is the nurse's best action?

- A. Use the pump because it appears to function correctly
- B. Use the pump for a short infusion only
- C. Set the pump aside and obtain equipment that is current for inspection
- D. Ask the client to report if the pump starts beeping

Correct Answer: C

Equipment that is overdue for required inspection should not be used, even if it appears to function normally. Safety checks and maintenance status are part of safe equipment use. Visual appearance and a brief trial run are not substitutes for current inspection.

926. A nurse is preparing to transport a client in bed to another department. Which action is most important immediately before moving the bed?

- A. Raise the bed to its highest position for better visibility
- B. Ensure the client can reach the overbed table during transport
- C. Verify that the bed's lines and attachments move freely and will not pull

D. Remove the client's pillow to keep the head flat

Correct Answer: C

Before moving a bed, the nurse must ensure attached items such as tubing, drains, and cords can move safely and will not become dislodged or pulled. This is a key part of safe equipment use during transport. Raising the bed high reduces safety, the overbed table is not the priority, and removing the pillow is not routinely required.

927. A nurse is about to help a client use an incentive spirometer for the first time after surgery. Which instruction best supports safe equipment use?

- A.** Blow out hard into the mouthpiece as fast as possible
- B.** Hold the device upside down so the marker rises more easily
- C.** Sit upright and seal your lips around the mouthpiece while inhaling slowly
- D.** Share the device with your roommate if they also had surgery today

Correct Answer: C

Safe and correct use of an incentive spirometer includes upright positioning, a good mouth seal, and slow inhalation. Blowing out is incorrect for this device, holding it upside down affects proper function, and the device should not be shared between clients.

Allergy checks

928. A nurse is preparing to administer an oral antibiotic to a client. Which action best addresses allergy safety before giving the medication?

- A.** Ask the client whether the medication was effective in the past
- B.** Check the client's documented allergies and ask the client about any medication allergies
- C.** Administer the first dose and observe for side effects
- D.** Verify the client's room number with the medication administration record

Correct Answer: B

Checking the documented record and asking the client directly are both key allergy checks before medication administration. This helps catch missing or outdated information. Choice A is about effectiveness, not allergy safety. Choice C is unsafe because the nurse should verify allergies before giving the drug. Choice D helps with identification but does not specifically address allergy status.

929. A client tells the nurse, "Penicillin makes me break out in hives." Which follow-up action is most appropriate?

- A.** Document the reaction as an allergy and avoid giving the medication until clarified with the provider if needed
- B.** Explain that hives are a minor side effect and continue with the order
- C.** Ask the unlicensed assistive personnel to give the medication with food
- D.** Administer a half dose first to test the reaction

Correct Answer: A

Hives suggest a possible allergic reaction, so the nurse should treat this as an allergy concern, document it, and withhold the medication until the order is clarified if necessary. Choice B minimizes a potentially serious reaction. Choice C is inappropriate delegation and ignores the safety issue. Choice D is unsafe because intentionally testing for an allergy by giving the drug can cause harm.

930. A nurse receives an order for a cephalosporin antibiotic for a client whose chart lists a severe allergy to cephalexin. What is the nurse's best action?

- A.** Give the medication because it is a different cephalosporin
- B.** Hold the medication and clarify the order due to the listed allergy
- C.** Premedicate with an antihistamine and then administer the drug
- D.** Ask the client whether the previous reaction required hospitalization, then decide

Correct Answer: B

A listed severe allergy to cephalexin is a major safety concern when another cephalosporin is ordered. The nurse should hold the medication and clarify the order. Choice A assumes safety despite a relevant allergy history. Choice C attempts to override an allergy concern rather than address it. Choice D gathers more information but does not replace the need to withhold and clarify before administration.

931. Before applying a prescribed adhesive dressing, the nurse notes that the client has a documented latex allergy. Which action is most appropriate?

- A. Use only supplies confirmed to be latex-free
- B. Apply the dressing quickly to reduce exposure time
- C. Wear two pairs of sterile gloves during the procedure
- D. Ask the client whether latex still causes symptoms

Correct Answer: A

The safest action is to use supplies known to be latex-free. Choice B does not prevent exposure. Choice C may still expose the client if latex-containing products are used. Choice D may be part of assessment, but a documented latex allergy should already trigger avoidance rather than testing whether it still exists.

932. A client is scheduled for a CT scan with contrast. Which statement by the client requires the nurse to follow up before the procedure?

- A. I had toast for breakfast.
- B. I get short of breath after eating shellfish.
- C. I am nervous about being in the scanner.
- D. I had this test 5 years ago.

Correct Answer: B

Shortness of breath after shellfish is a reported allergic-type reaction and requires follow-up before the procedure. The nurse should not ignore any history suggesting significant allergy risk. Choice A may matter depending on orders, but it is not the clearest allergy concern here. Choice C addresses anxiety, and choice D is simply history. The key safety issue is the client's report of a serious reaction pattern.

933. The nurse sees that a client's chart lists an allergy to morphine, but the client states, "It just made me sleepy." What is the nurse's best response?

- A. Remove morphine from the allergy list because sleepiness is expected
- B. Administer morphine because the reaction was not severe
- C. Clarify the client's reaction and follow agency process to update the record if appropriate before giving the medication
- D. Tell the client that all pain medications cause allergies

Correct Answer: C

Sleepiness may be an expected effect rather than a true allergy, but the nurse should not independently ignore or delete the allergy label without proper clarification and documentation. Choice C is best because it addresses the discrepancy safely. Choice A skips required verification. Choice B is unsafe because the allergy record has not yet been resolved. Choice D is inaccurate and not therapeutic.

934. A nurse is preparing to administer a newly prescribed medication. The electronic record shows a red allergy alert for the same drug. What should the nurse do first?

- A. Override the alert because the provider prescribed the medication
- B. Hold the medication and verify the allergy information before administration
- C. Administer the medication slowly while monitoring the client
- D. Ask another nurse to witness the administration

Correct Answer: B

An allergy alert must be taken seriously. The nurse should stop and verify the allergy information before giving the medication. Choice A is a common trap because a provider order does not cancel the nurse's responsibility for safety checks. Choice C still exposes the client before clarification. Choice D does not solve the allergy conflict.

935. A client with a peanut allergy is admitted for surgery. Which nursing action best helps reduce allergy-related risk during the perioperative period?

- A. Place the client in a private room
- B. Ensure the allergy is clearly documented and communicated during handoff and preprocedure verification
- C. Keep the client NPO after midnight
- D. Schedule the surgery for the end of the day

Correct Answer: B

Clear documentation and communication of allergies during handoff and preprocedure verification help prevent accidental exposure. Choice A may be useful for some infection issues but does not directly address this allergy risk. Choice C is unrelated to allergy verification. Choice D does not reduce exposure risk.

936. A nurse is about to give the first dose of an IV medication. Which finding creates the highest need to stop and clarify before administration?

- A. The client says, "I don't know what this medication is for."
- B. The client reports itching after a related medication in the past, but no allergy is documented
- C. The infusion pump battery is fully charged
- D. The client asks whether the medication will work quickly

Correct Answer: B

A reported prior reaction to a related medication, even if not yet documented, should prompt the nurse to stop and clarify before giving the first dose. Choice A calls for teaching, but it is not the highest immediate allergy concern. Choices C and D do not indicate an allergy safety issue. The trap is assuming undocumented reactions can be ignored.

937. A client says, "I'm allergic to eggs," as the nurse prepares to serve the breakfast tray. What is the nurse's priority action?

- A. Tell the client to avoid only foods with obvious egg pieces
- B. Remove the tray and verify the dietary order and allergy information
- C. Ask dietary to send a larger lunch instead

D. Document the allergy after the client finishes eating

Correct Answer: B

The priority is to prevent exposure by removing the tray and verifying the allergy and diet order. Choice A is unsafe because many foods contain hidden egg ingredients. Choice C delays the immediate safety response. Choice D is too late because the client could already be exposed.

938. A nurse notes that a client has an allergy band in place, but the medication administration record lists no allergies. Which action is best?

- A. Remove the band because the record is blank
- B. Assume the band was placed in error and proceed
- C. Investigate the discrepancy with the client and health record before administering medications
- D. Cover the band so it does not worry the client

Correct Answer: C

A discrepancy between the allergy band and the record must be investigated before medication administration. Choice C is safest because conflicting information can lead to harm. Choices A and B are dangerous assumptions. Choice D hides a safety cue rather than resolving it.

939. A provider prescribes amoxicillin for a client. The client states, "The last time I took that, my lips swelled." Which response by the nurse is best?

- A. That sounds like an expected side effect, so I will give it with food
- B. Swelling can indicate a serious allergic reaction, so I need to hold the medication and contact the provider
- C. If you did not have a rash, it probably was not an allergy
- D. We can give a smaller test dose to be safe

Correct Answer: B

Lip swelling can indicate a serious allergic reaction, so the medication should be held and the provider contacted. Choice A incorrectly labels a potentially dangerous reaction as expected. Choice C falsely narrows allergy signs to rash only. Choice D is unsafe because test dosing is not an appropriate nursing response to a suspected serious allergy.

940. Which situation most strongly suggests a true allergy rather than a predictable medication effect?

- A.** A client becomes drowsy after an opioid analgesic
- B.** A client reports dry mouth after an anticholinergic medication
- C.** A client develops wheezing soon after receiving an antibiotic
- D.** A client feels sleepy after taking an antihistamine

Correct Answer: C

Wheezing soon after receiving an antibiotic suggests a possible allergic reaction affecting the airway. The other options describe common expected medication effects rather than classic allergy findings. The main trap is confusing side effects, such as drowsiness or dry mouth, with immune-related reactions.

941. A nurse is collecting admission data from a client who speaks limited English. The interpreter is present. Which approach best supports an accurate allergy check?

- A.** Ask the family member to answer because it is faster
- B.** Skip the allergy section until discharge teaching
- C.** Use the interpreter to ask specifically about medication, food, and latex allergies and the reactions experienced
- D.** Record 'no known allergies' if the client hesitates

Correct Answer: C

Using the interpreter to ask clear, specific questions about allergy types and past reactions is the safest approach. Choice A may lead to inaccurate information and bypasses the client. Choice B delays a critical safety assessment. Choice D is unsafe because hesitation does not equal absence of allergies.

942. A nurse is preparing supplies for urinary catheter insertion in a client with a documented latex allergy. Several catheter kits are stocked on the unit. Which action is best?

- A.** Use any kit as long as sterile technique is maintained
- B.** Choose a kit verified to be latex-free before beginning the procedure
- C.** Insert the catheter first and replace it later if irritation occurs
- D.** Ask the provider whether latex exposure is acceptable for a short procedure

Correct Answer: B

For a client with a documented latex allergy, the nurse should select a kit confirmed to be latex-free before starting. Choice A focuses on asepsis but ignores allergy safety. Choice C waits for harm to occur. Choice D is inappropriate because avoiding known allergens is part of safe nursing care.

943. During admission, a client says, "I'm allergic to a pain medicine, but I don't remember the name." Which nursing action is most appropriate first?

- A.** Document "allergic to pain medicine" and avoid all analgesics
- B.** Ask the client what happened when the medication was taken
- C.** Place the client on NPO status until the allergy is identified
- D.** Administer pain medication only if the provider remains at the bedside

Correct Answer: B

The first priority is to clarify the reaction history. Asking what happened helps determine whether the client experienced a likely allergy, an intolerance, or an expected side effect, and may also help identify the medication class involved. Choice A is too vague and could lead to unsafe or unnecessary withholding. Choices C and D do not address the missing allergy information.

944. A nurse is reviewing new medication orders. Which order should the nurse question based on the client's documented allergy list?

Allergies: shellfish—rash; latex—itching; codeine—nausea

- A. Elastic compression stockings
- B. Acetaminophen tablet as needed for pain
- C. Latex urinary drainage bag
- D. Ondansetron as needed for nausea

Correct Answer: C

A latex urinary drainage bag directly conflicts with the client's documented latex allergy and should be questioned before use. Acetaminophen and ondansetron are unrelated to the listed allergies. Codeine caused nausea, which is commonly a side effect rather than a true allergy, so that does not make the other options the best answer.

945. A client returns from surgery with a new prescription for hydrocodone-acetaminophen. The chart lists an allergy to hydromorphone with the reaction “anaphylaxis.” What is the nurse's best action?

- A. Administer the medication because the names are not identical
- B. Give half the dose first and observe for a reaction
- C. Hold the medication and contact the provider for clarification
- D. Ask the client whether pain is severe enough to risk taking it

Correct Answer: C

Anaphylaxis is a severe allergic reaction, so the nurse should not administer a similar opioid without clarification. The safest action is to hold the medication and contact the provider. Choice A ignores the significance of a severe allergy history. Choice B is unsafe because a test dose does not prevent anaphylaxis. Choice D shifts the decision to the client instead of following safe medication practice.

946. Before giving a dose of insulin, the nurse asks the client about allergies. The client replies, “I’m allergic to tape. It makes my skin red where it touches.” Which interpretation is most appropriate?

- A. This may reflect a contact sensitivity and should still be documented and considered in care
- B. This is irrelevant because adhesives are never used in nursing care
- C. This proves the client will also be allergic to insulin
- D. This should be ignored unless blisters are present

Correct Answer: A

A history of skin redness with tape may indicate adhesive sensitivity or irritation and should be documented and considered when planning care. It should not be dismissed. Choice B is incorrect because adhesives are commonly used. Choice C incorrectly assumes cross-reaction. Choice D delays appropriate attention to a potential product-related reaction.

947. A nurse is preparing a client for a blood transfusion. The client says, “I had a reaction once, but I don't know whether it was from the blood or the antibiotic they gave me.” Which action is best before proceeding?

- A. Start the transfusion slowly because the cause is uncertain
- B. Obtain more details about the prior reaction and when it occurred
- C. Premedicate with an antihistamine and continue as planned
- D. Ignore the statement because it does not identify a specific allergen

Correct Answer: B

When the history is unclear, the nurse should gather more specific information about the prior reaction, timing, and circumstances before proceeding. That helps identify whether the concern may involve a medication allergy, a transfusion reaction history, or another issue. Choices A and C move forward without adequately clarifying risk. Choice D dismisses potentially important safety information.

948. A provider prescribes a topical ointment. While preparing to apply it, the nurse notices the ingredient list includes lanolin. The client's record states, "lanolin allergy—itching and rash." What should the nurse do?

- A. Apply a small amount to test the skin response
- B. Use the ointment only on intact skin
- C. Hold the ointment and notify the provider
- D. Apply the ointment and document the previous reaction

Correct Answer: C

The product contains a documented allergen, so the nurse should withhold it and notify the provider. Choice A is unsafe because intentionally exposing the client to a known allergen is not appropriate. Choice B does not remove the risk. Choice D knowingly administers a product that conflicts with the allergy record.

949. At the bedside handoff, the off-going nurse reports, "This client is allergic to sulfa." Which follow-up question by the oncoming nurse is most important?

- A. "How many years ago was the reaction?"
- B. "What symptoms did the client have?"
- C. "Was the medication expensive?"
- D. "Did the family members have the same problem?"

Correct Answer: B

The most important clarification is the type of reaction the client had. Knowing the symptoms helps distinguish a true allergy from an expected side effect or vague intolerance and guides future medication safety decisions. Timing can matter, but choice A is less urgent than understanding the reaction itself. Choices C and D do not meaningfully improve allergy safety.

950. A client is scheduled for a procedure in the morning. During the pre-procedure checklist, the nurse finds a documented chlorhexidine allergy. Which item requires the nurse to stop and replace it?

- A. A blood pressure cuff
- B. A chlorhexidine skin prep applicator
- C. A cotton blanket
- D. A pulse oximeter probe

Correct Answer: B

A chlorhexidine skin prep applicator contains the documented allergen and should be replaced. The other items do not inherently expose the client to chlorhexidine. This question tests recognizing that allergy checks apply not only to medications but also to prep solutions and procedural supplies.

951. A client says, "I'm allergic to strawberries," while the nurse is administering morning medications. Which action is most appropriate regarding the allergy information?

- A. Tell the client food allergies do not affect medication safety
- B. Record the allergy only if a provider specifically requests it
- C. Document the allergy and consider it when evaluating foods, medications, and products
- D. Advise the client to avoid mentioning food allergies unless hives occurred

Correct Answer: C

Food allergies should be documented and considered during care because allergy safety extends beyond one setting or one type of product. Choice A wrongly minimizes the information. Choice B delays important documentation. Choice D discourages reporting clinically relevant history.

952. A nurse is preparing discharge teaching for a client who had an urticarial reaction to a newly prescribed antibiotic during hospitalization. Which instruction is most important to include?

- A. Stop all future antibiotics unless they are given in the hospital
- B. Keep a record of the drug name and reaction, and report it to future healthcare providers

- C. Take the same medication with food next time to prevent the reaction
- D. Resume the medication if the rash disappears within 24 hours

Correct Answer: B

Teaching should emphasize accurate future reporting of the medication name and the reaction so the allergy can be recognized in later care. Choice A is overly broad and unsafe. Choice C confuses an allergic reaction with a stomach-related side effect. Choice D encourages restarting a medication associated with a reaction without guidance.

953. A nurse is admitting a client from a long-term care facility. The transfer form says “NKDA,” but the client reports, “I had throat tightness after taking an antibiotic years ago.” What is the nurse’s best action?

- A. Keep the chart as NKDA because the form came from another facility
- B. Document the client’s reported reaction and reconcile the discrepancy in the record
- C. Wait until the provider prescribes an antibiotic before updating the record
- D. Assume the client is mistaken because the event was years ago

Correct Answer: B

The nurse should treat the client’s report seriously, document the reaction history, and reconcile the discrepancy. Throat tightness suggests a potentially significant allergic reaction. Choice A relies on conflicting paperwork over current assessment. Choice C delays a safety correction. Choice D dismisses a potentially dangerous history.

954. The nurse is about to administer a second dose of an IV antibiotic. The client reports that shortly after the first dose, “my chest felt tight and I started wheezing, but it got better.” Which action should the nurse take?

- A. Administer the second dose more slowly
- B. Give the dose after offering fluids
- C. Hold the medication and report the reaction immediately
- D. Continue if the client has no rash at this time

Correct Answer: C

Chest tightness and wheezing after a medication dose suggest a potentially serious allergic reaction. The medication should be held and the reaction reported immediately. Choice A is a common trap: slowing the dose does not make a likely allergic reaction safe. Choices B and D fail to address the severity of the symptoms.

955. A client with a documented latex allergy is transferred to the unit in an emergency. Which room preparation is most important before the client arrives?

- A. Remove latex-containing supplies from the immediate care area
- B. Place the client in a negative-pressure room
- C. Keep all supplies sealed until the provider examines the client
- D. Set out powdered latex gloves in case extra staff are needed

Correct Answer: A

For a client with a latex allergy, the immediate environment should be prepared by removing latex-containing supplies to reduce exposure risk. Negative-pressure isolation is unrelated. Keeping supplies sealed does not address whether they contain latex. Powdered latex gloves would increase exposure risk and are clearly unsafe.

956. A medication history lists “aspirin allergy.” Which additional finding would create the greatest concern if a new order for aspirin is entered?

- A. The client says aspirin caused mild heartburn
- B. The client says aspirin made urine darker once
- C. The client says aspirin caused facial swelling and trouble breathing
- D. The client says aspirin tasted bitter

Correct Answer: C

Facial swelling and trouble breathing indicate a potentially severe allergic reaction and create the greatest concern. Heartburn is more consistent with irritation or intolerance. Darker urine and bitter taste do not most strongly indicate immediate allergy risk in this context. The key is identifying symptoms suggestive of anaphylaxis rather than minor or unrelated effects.

957. A nurse is reviewing a client's profile before giving medications. The allergy section reads: "iodine—unknown reaction, entered 8 years ago." The provider has prescribed povidone-iodine skin cleansing for a wound. What is the nurse's best action?

- A.** Proceed because the reaction is unknown and the entry is old
- B.** Clarify the allergy history before using the product
- C.** Delete the allergy entry and document that it is outdated
- D.** Apply the product to a small area and monitor for 5 minutes

Correct Answer: B

An unclear allergy record should be clarified before exposing the client to a possibly related product. Unknown reaction does not mean no risk. Choice A assumes safety without enough information. Choice C removes a safety alert without verification. Choice D intentionally exposes the client before clarification.

958. A nurse receives a client from the emergency department with no allergy band in place. The client is awake and says, "Penicillin made my lips swell once." Which action should the nurse take first?

- A.** Administer scheduled medications and add the allergy information after admission is completed
- B.** Document the reported allergy and apply the appropriate allergy alert according to facility policy
- C.** Ask the provider to remove all antibiotic orders from the chart
- D.** Tell the client to report the allergy again when the next nurse comes on duty

Correct Answer: B

A report of lip swelling suggests a potentially serious allergic reaction, so the nurse should promptly document the allergy and place the appropriate alert so all team members can see it. Choice A delays a safety action. Choice C is too broad because not all antibiotics are penicillins. Choice D shifts responsibility instead of acting now.

959. A nurse is preparing medications for four clients. Which client requires the most immediate follow-up before medication administration?

- A.** A client with seasonal pollen allergy who is prescribed docusate sodium
- B.** A client with peanut allergy who is prescribed acetaminophen tablets
- C.** A client with a history of hives after cephalexin who is prescribed cefazolin
- D.** A client with adhesive sensitivity who is prescribed a stool softener

Correct Answer: C

A history of hives after cephalexin suggests a true allergic reaction to a cephalosporin. Cefazolin is also a cephalosporin, so the nurse should stop and clarify before giving it. The other options do not show a direct likely medication-allergy conflict from the information given. A common trap is minimizing hives as a minor symptom, but hives indicate possible hypersensitivity.

960. While completing admission questions, a client says, "Morphine makes me very nauseated, so I tell people I'm allergic to it." Which response by the nurse is best?

- A.** "Nausea is usually a side effect, so I need to document both the drug and the reaction accurately."
- B.** "If it bothers you, I will mark it as anaphylaxis to keep you safe."
- C.** "That is not important unless you also had a rash."
- D.** "Since you are unsure, I will leave the allergy section blank for now."

Correct Answer: A

The nurse should distinguish side effects from true allergic reactions and document the specific drug and reaction clearly. Nausea alone is more consistent with a side effect than allergy. Choice B is inaccurate documentation. Choice C dismisses relevant medication-history information. Choice D leaves out useful safety information.

961. A client is scheduled for surgery and has a documented severe latex allergy. Which item in the preoperative area should the nurse identify as unsafe for this client?

- A. A blood pressure cuff with a synthetic fabric cover and latex-free tubing
- B. A vial labeled latex-free stored in a medication drawer
- C. A pair of powdered exam gloves containing natural rubber latex
- D. A plastic pulse oximeter probe with a disposable adhesive wrap

Correct Answer: C

Natural rubber latex gloves are unsafe for a client with severe latex allergy and should be removed from the environment. The other items are not identified as containing latex in the stem. The key skill is spotting an obvious latex source before exposure occurs.

962. A nurse reviews the record of a client who is to receive a first dose of an oral antibiotic. The allergy list reads: “amoxicillin—rash as child.” The prescribed drug is amoxicillin-clavulanate. What is the nurse's best action?

- A. Administer the medication because the client tolerated antibiotics recently
- B. Hold the medication and clarify the order because the prescribed drug contains amoxicillin
- C. Give half the dose first to test for a reaction
- D. Administer the medication with food to reduce the allergy risk

Correct Answer: B

Amoxicillin-clavulanate contains amoxicillin, so a documented prior rash with amoxicillin requires clarification before administration. Choice A ignores the direct match. Choice C is unsafe because nurses should not perform an informal test dose without an appropriate order and plan. Choice D may reduce stomach upset, not allergic risk.

963. A client tells the nurse, “I had a bad reaction during a CT scan once.” Which follow-up question is most important for allergy clarification?

- A. “How long was the scan?”
- B. “Did you receive an injected contrast dye, and what symptoms did you have?”
- C. “Were you nervous before the test?”

D. “Did the staff have trouble finding your vein?”

Correct Answer: B

To clarify a possible allergy, the nurse needs the likely substance involved and the actual reaction. Asking whether contrast was given and what symptoms occurred helps identify future risk and guides documentation. The other questions may be relevant to comfort or procedure details but do not clarify the allergy concern.

964. A nurse is caring for a client whose chart states, “allergy to eggs—anaphylaxis.” Which meal tray should the nurse question?

- A. Oatmeal, banana, and black coffee
- B. Turkey sandwich, apple slices, and water
- C. Scrambled eggs, toast, and orange juice
- D. Grilled chicken, rice, and green beans

Correct Answer: C

Scrambled eggs directly conflict with the client's documented severe egg allergy and should be questioned immediately. The other trays do not contain an obvious egg item from the information given. This item tests checking foods as well as medications when reviewing allergies.

965. A client says, “I break out from bananas and avocados.” Which additional assessment finding should make the nurse most alert to a possible latex allergy?

- A. The client reports frequent motion sickness
- B. The client develops redness after wearing rubber cleaning gloves
- C. The client prefers not to eat citrus fruits
- D. The client had nausea after taking iron tablets

Correct Answer: B

Banana and avocado allergies can be associated with latex sensitivity, so a history of skin reaction to rubber gloves increases concern for latex allergy. The other findings do not support that connection. The likely trap is focusing on unrelated symptoms rather than linking exposure history to allergy risk.

966. A nurse notes an order for ampicillin. The client's record lists "penicillin—anaphylaxis." The provider is in a procedure and cannot be reached immediately. What should the nurse do?

- A.** Administer the first dose because ampicillin is a broader-spectrum drug
- B.** Ask another nurse to give the medication to avoid delay
- C.** Hold the medication and continue trying to obtain clarification from the prescriber
- D.** Give diphenhydramine before the antibiotic

Correct Answer: C

A history of anaphylaxis to penicillin creates a major safety concern with an order for ampicillin, so the nurse should withhold the medication and seek clarification. Choice A is unsafe because ampicillin is still a penicillin-class antibiotic. Choice B does not resolve the problem. Choice D does not make it safe to give a potentially contraindicated medication.

967. A client arrives for a dressing change with a documented adhesive allergy. Which nursing action best reduces allergy risk while still completing care?

- A.** Secure the dressing with the usual adhesive tape because the area is small
- B.** Skip the dressing change until discharge teaching is complete
- C.** Use an alternative method to secure the dressing and document the client's reaction history
- D.** Apply a double layer of adhesive so the dressing will not shift

Correct Answer: C

The nurse should adapt care to avoid the known allergen, such as using a nonadhesive securing method per available supplies and policy, while documenting the allergy history. Choices A and D continue exposure. Choice B delays needed care without solving the safety issue.

968. A nurse is obtaining a preprocedure history. The client states, "When I ate kiwi and chestnuts, my mouth tingled and my throat felt tight." Why is this information especially important to communicate before the procedure?

- A.** It may indicate a risk for latex sensitivity during care
- B.** It means the client should avoid all intravenous fluids
- C.** It proves the client has an anxiety disorder
- D.** It shows the client is at high risk for bleeding

Correct Answer: A

Kiwi and chestnut allergies can be associated with latex sensitivity, so this information is important before a procedure where latex-containing items may be present. The other options are not supported by the allergy history. The challenge here is recognizing a less obvious clue that changes equipment planning.

969. A client has a documented allergy to ceftriaxone with the reaction listed as "shortness of breath." A new order is written for ceftriaxone IM because the first entry "might have been wrong." What is the nurse's best response?

- A.** Administer the injection because the route is different
- B.** Clarify the order before giving it because the documented reaction suggests possible allergy
- C.** Give the medication if the client signs consent
- D.** Administer the medication slowly and monitor closely

Correct Answer: B

Shortness of breath suggests a potentially serious hypersensitivity reaction, so the nurse should not administer the same drug until the discrepancy is resolved. Changing the route does not remove the allergy risk. Client consent does not make an unsafe order acceptable. Monitoring after giving it is not the best first step when a clear safety concern already exists.

970. While reviewing a newly admitted client's home medications, the nurse notices the client takes over-the-counter ibuprofen regularly. The allergy list states "ibuprofen—facial swelling." Which action is best?

- A.** Assume the allergy entry is incorrect because the client still takes the drug
- B.** Delete the allergy from the chart after verifying the bottle label
- C.** Question the client further about the reaction history and hold any ibuprofen administration until clarified
- D.** Instruct the client to continue taking the medication as long as swelling is mild

Correct Answer: C

Facial swelling may indicate a significant allergic reaction, so the nurse should clarify the history and avoid giving the medication until resolved. Medication lists can be inaccurate, but the nurse should not simply assume the allergy is false or independently delete it. Mild or intermittent swelling is still concerning, making D unsafe.

971. A nurse enters a client's allergy information into the health record. Which documentation is most useful for future safety checks?

- A.** "Allergic to antibiotics"
- B.** "Medication allergy present"
- C.** "Penicillin allergy—rash and facial swelling after first dose 2 years ago"
- D.** "Bad reaction to medicine in the past"

Correct Answer: C

The most useful documentation identifies the specific substance and the reaction. "Penicillin allergy—rash and facial swelling after first dose 2 years ago" gives details that help future clinicians assess risk. The other options are too vague to support safe decision-making.

Environmental and disaster safety

972. A practical nurse enters a client's room and notices the oxygen tubing stretched tightly across the walkway from the wall outlet to the bed. What action should the nurse take first?

- A.** Raise the side rails to keep the client from getting out of bed
- B.** Reposition the tubing to remove the tripping hazard
- C.** Document the unsafe environment in the chart
- D.** Turn off the oxygen until better tubing is available

Correct Answer: B

The first priority is to remove the immediate environmental hazard by repositioning the tubing so no one trips. Raising side rails does not fix the hazard, documentation comes after the safety issue is addressed, and turning off prescribed oxygen could harm the client.

973. A fire alarm sounds on a long-term care unit. A client who uses a wheelchair is closest to the area where smoke is visible under a door. Which action is most appropriate?

- A.** Move the client away from the smoke area to a safer location
- B.** Wait in the room until the exact source of the fire is confirmed
- C.** Finish passing medications before helping with evacuation
- D.** Open the door with smoke under it to see how large the fire is

Correct Answer: A

The nurse should protect the client from immediate danger by moving the client away from the smoke area. Opening a smoky door may worsen exposure or fire spread. Waiting or finishing other tasks delays life-saving action.

974. A nurse is preparing a client's room after a spill of water on the floor near the bathroom. Which intervention best promotes environmental safety?

- A. Place a chair over the spill so no one steps there
- B. Cover the spill with a bath blanket
- C. Clean and dry the floor, then ensure the path is clear
- D. Ask the client to stay in bed until housekeeping arrives

Correct Answer: C

Cleaning and drying the floor removes the hazard directly and restores a safe walking path. A chair or blanket does not safely eliminate the risk and may create additional hazards. Keeping the client in bed is not the best first response when the spill can be corrected promptly.

975. A client with impaired vision is admitted to a new room. Which nursing action best reduces the risk for injury from environmental hazards?

- A. Leave the overbed table across the walkway so the client can find it easily
- B. Keep the room layout consistent and orient the client to the surroundings
- C. Dim the lights at all times to prevent overstimulation
- D. Encourage the client to walk independently right away to learn the room

Correct Answer: B

Clients with impaired vision are safer when the environment stays consistent and the nurse provides orientation to furniture, call light, and bathroom location. Blocking the walkway creates danger, dim lighting can worsen safety, and unsupervised mobility in a new environment raises injury risk.

976. The nurse smells gas in the dietary area of a care facility. Which action should the nurse take first?

- A. Turn on the exhaust fan to clear the smell
- B. Use the unit phone in the area to report the leak
- C. Avoid activating electrical devices and move people away from the area
- D. Search the room closely for the exact source of the leak

Correct Answer: C

When gas is suspected, the priority is to prevent ignition and protect people by avoiding electrical activation and moving them away. Turning on a fan or using a phone in the area could create a spark. Searching for the exact source delays immediate safety actions.

977. During a severe thunderstorm, the power goes out on the unit. Which client should the nurse assess first for environmental and disaster safety concerns?

- A. A client scheduled for discharge later that afternoon
- B. A client receiving oxygen through an electrically powered concentrator
- C. A client resting after a routine dressing change
- D. A client asking for a snack

Correct Answer: B

A client dependent on electrically powered oxygen equipment is at immediate risk during a power outage and should be assessed first. The other clients may need care, but they do not face the same urgent threat related to equipment failure.

978. A confused client keeps trying to get out of bed at night. Which environmental change is most appropriate to reduce injury risk?

- A. Move the client to a room far from the nurses' station to reduce noise
- B. Keep the room dark so the client will sleep more deeply
- C. Place frequently used items within reach and keep a clear path to the bathroom
- D. Apply wrist restraints before trying room modifications

Correct Answer: C

Environmental safety measures such as clear walkways and accessible needed items reduce injury risk while supporting the least restrictive approach. Moving the client farther away may reduce observation, darkness can worsen confusion, and restraints are not the first environmental intervention.

979. The nurse is teaching unlicensed assistive personnel about fire safety in the facility. Which statement by the assistive personnel shows correct understanding?

- A. I should open doors quickly to see whether the fire is still active
- B. I should move clients away from danger and close doors as directed
- C. I should use the elevator to transport nonambulatory clients faster
- D. I should collect personal belongings for clients before relocation

Correct Answer: B

Moving clients away from immediate danger and closing doors helps contain smoke and fire spread. Opening doors can worsen the hazard, elevators may be unsafe during fire events, and gathering belongings is not a priority over life safety.

980. A nurse finds a portable space heater plugged in under a client's bedside table in a long-term care room. Which action is best?

- A. Leave it in place because the client may feel cold
- B. Move it closer to the bed so it works better
- C. Remove the heater and follow facility procedure for the unsafe item
- D. Cover the heater with a towel until maintenance can inspect it

Correct Answer: C

A portable heater in a client room presents a fire and burn hazard. The nurse should remove the unsafe item and follow facility procedure. Leaving it, moving it closer, or covering it increases danger rather than reducing it.

981. Several clients must be relocated quickly because of smoke in one wing of the facility. Which client should the nurse move first?

- A. A stable client who is packing belongings independently
- B. A postoperative client who can walk with minimal help
- C. A bedbound client nearest the smoke source
- D. A client who is upset about being moved

Correct Answer: C

In an evacuation-related event, the client in the greatest immediate danger should be moved first. A bedbound client near the smoke source is both highly vulnerable and least able to self-evacuate. Emotional distress matters, but immediate physical danger takes priority.

982. A tornado warning is issued for the area. Which nursing action is most appropriate?

- A. Place clients near windows so they can be observed more easily
- B. Move clients away from windows and toward designated interior shelter areas
- C. Transport clients outside before the storm arrives
- D. Open windows slightly to reduce pressure changes

Correct Answer: B

During a tornado warning, clients should be moved away from windows and into designated interior shelter areas to reduce risk from flying glass and debris. Windows are dangerous, going outside is unsafe, and opening windows is not an appropriate protective measure.

983. A nurse enters a room and sees that an extension cord is running from one outlet to power a feeding pump and a bedside fan. What is the nurse's best action?

- A. Disconnect the fan and continue using the extension cord for the feeding pump
- B. Tape the extension cord to the floor and leave the setup in place
- C. Recognize the setup as unsafe and obtain a safer approved power arrangement
- D. Move the extension cord under the bed where it is less visible

Correct Answer: C

Improper electrical setups can increase fire risk and create tripping hazards. The safest action is to correct the arrangement using approved equipment or outlets according to facility policy. Taping or hiding the cord does not remove the underlying electrical hazard, and continuing to use it selectively may still be unsafe.

984. Following a small fire in a storage room, the charge nurse asks the practical nurse to assist with post-event safety measures. Which action is most important before the area is returned to routine use?

- A. Replace any supplies that smell smoky and resume normal traffic
- B. Confirm that the area has been evaluated as safe and that hazards have been addressed
- C. Ask staff to avoid documenting the event until the manager reviews it
- D. Reopen the room once the smoke is no longer visible

Correct Answer: B

After a fire, the area should not return to use until it has been evaluated as safe and any hazards have been corrected. Lack of visible smoke does not guarantee safety. Documentation should not be delayed, and replacing supplies alone is not enough.

985. During an internal disaster drill, the unit must prepare to receive multiple clients from another area of the facility. Which action by the nurse best supports environmental safety?

- A. Leave extra equipment in hallways for quick access during the surge
- B. Clear pathways, check that emergency equipment is accessible, and identify safe placement areas
- C. Lock supply rooms so items are not used too quickly
- D. Cluster incoming clients in one doorway area until all arrive

Correct Answer: B

In a surge situation, environmental safety depends on clear movement paths, access to emergency equipment, and organized placement of clients. Hallway clutter and doorway crowding obstruct evacuation and care. Locking supplies interferes with response efforts.

986. A nurse caring for clients during a wildfire-related evacuation notices that one client is receiving oxygen by nasal cannula and another family member is attempting to light a candle in the room because the lighting is poor. What is the nurse's priority action?

- A. Stop the family member from lighting the candle and remove the ignition source

B. Close the door and finish preparing the evacuation paperwork

C. Ask the oxygen client whether the candle smell is bothersome

D. Move the candle to the windowsill away from the bed

Correct Answer: A

Oxygen increases fire risk, so eliminating an ignition source is the immediate priority. Closing the door and paperwork do not address the most urgent danger. Moving the candle does not make it safe in an oxygen-use area.

987. A practical nurse is making rounds in a rehabilitation unit and notices a frayed electrical cord on a client's sequential compression device. The device is currently plugged in and operating. Which action should the nurse take first?

- A. Wrap the damaged area with tape and continue using the device until maintenance arrives
- B. Unplug the device, remove it from use, and report the equipment problem
- C. Move the cord away from the bed so no one trips over it
- D. Wait until the treatment is complete, then document the damaged cord

Correct Answer: B

A frayed electrical cord creates an immediate risk for shock, burns, or fire. The priority is to stop using the unsafe equipment, remove it from service, and report it according to facility procedure. Taping the cord does not make it safe. Moving the cord addresses tripping but not the electrical hazard. Waiting until later delays correction of an immediate danger.

988. While preparing a client's room, the nurse notes that the bed is in the highest position from a previous procedure, the call light is on the overbed table, and the client has weakness after receiving pain medication. Which intervention should the nurse perform first to reduce environmental injury risk?

- A. Place the bed in the lowest position
- B. Offer the client a bedpan
- C. Raise all four side rails
- D. Dim the room lights to promote rest

Correct Answer: A

A high bed increases fall and injury risk, especially for a weak client who recently received pain medication. Lowering the bed is the first safety action. The call light should also be within reach, but the highest immediate environmental hazard listed is the elevated bed. Raising all four side rails is not the least restrictive approach and can create additional risk. Dimming lights may worsen safety if the client attempts to move.

989. A nurse is helping prepare for emergency admissions after a nearby bus crash. Which room assignment best supports environmental safety for a client who is arriving on droplet precautions?

- A.** Place the client in the first open bed near the nurses' station with the curtain pulled
- B.** Place the client in a room with another client who has a noninfectious condition
- C.** Place the client in a private room if available and follow facility isolation measures
- D.** Place the client in the hallway temporarily until all other clients are assigned rooms

Correct Answer: C

Environmental safety during disaster response still includes reducing transmission risk. A private room, when available, best supports droplet precautions along with appropriate isolation measures. A curtain does not provide adequate separation for infection-control needs. Cohorting with a noninfectious client is unsafe, and hallway placement increases exposure risk and reduces privacy and safety.

990. The nurse sees a visitor bringing in a small bouquet of helium balloons for a client in a semiprivate room. One balloon drifts and becomes tangled near a ceiling-mounted electrical fixture. What is the nurse's best action?

- A.** Ask the visitor to remove the balloons from the room and secure the area
- B.** Use a step stool to pull the balloon down right away
- C.** Leave the balloon alone because it is not touching the client
- D.** Turn off the room lights and continue client care

Correct Answer: A

Objects tangled near electrical fixtures can pose an electrical and environmental safety hazard. The nurse should prevent further risk by having the balloons removed and the area secured according to facility procedure. Climbing on a stool to retrieve the balloon increases risk, and ignoring the hazard leaves the danger uncorrected. Turning off lights does not resolve the problem.

991. A facility has lost access to running water after a major pipe break. Which nursing action is most important for environmental safety until services are restored?

- A.** Use alcohol-based hand rub only for all situations
- B.** Provide bottled or approved alternate water and follow emergency facility procedures
- C.** Encourage clients to limit fluid intake until water service returns
- D.** Keep clients in their rooms to reduce the need for restroom use

Correct Answer: B

Loss of running water affects drinking, hygiene, sanitation, and overall environmental safety. The priority is to implement emergency procedures and provide a safe alternate water source. Alcohol-based hand rub may help in some situations, but it does not solve broader water needs and is not appropriate for every circumstance. Restricting fluids or confining clients is unsafe and does not address the environmental problem.

992. At the start of the shift, the nurse receives report on four clients. Which client should the nurse see first for an environmental safety concern?

- A.** A client with arthritis who asks for an extra blanket because the room feels cool
- B.** A client who uses a walker and says the rubber tip has come off one leg
- C.** A client who wants the television volume lowered
- D.** A client who says the lunch tray arrived late

Correct Answer: B

A walker with a missing rubber tip is an immediate safety hazard because it can slip and lead to a fall. That client should be assessed first. The other concerns may affect comfort or satisfaction but do not pose the same urgent injury risk.

993. During a facility evacuation drill, the nurse is assigned to prepare one room of nonambulatory clients for transport. Which action is most appropriate before moving the clients?

- A.** Collect each client's valuables so nothing is left behind
- B.** Close the room door, then wait for transport staff to arrive
- C.** Identify each client and ensure essential equipment accompanies the correct client
- D.** Ask family members to choose which clients should leave first

Correct Answer: C

During evacuation, environmental safety includes moving the right client with the right essential equipment, such as oxygen or drainage devices, while maintaining identification. Valuables are not the priority. Simply waiting delays safe evacuation. Family members do not determine evacuation priority.

994. A practical nurse enters a client's room and finds the floor cluttered with a bedside commode, two meal trays, and a folded wheelchair blocking access to the exit door. The client is alert and currently resting in bed. Which action by the nurse is best?

- A.** Leave the room as it is because the client is not walking at this time
- B.** Move the items to create a clear path and keep the exit accessible
- C.** Ask housekeeping to address the problem at the end of the shift
- D.** Tell the client to call for help before attempting to get up

Correct Answer: B

Blocked pathways and exits are environmental hazards even if the client is currently in bed. The nurse should remove clutter and maintain a clear path for routine movement and emergency evacuation. Waiting or relying only on instructions to call for help does not eliminate the hazard.

995. A chemical spill occurs in a supply room adjacent to the medication area. The nurse notes a strong irritating odor and sees an employee coughing just outside the doorway. Which action should the nurse take first?

- A.** Enter the room quickly to identify the product label
- B.** Move people away from the area and initiate the facility's hazardous spill procedure
- C.** Open the door wider to improve air circulation
- D.** Pour water over the spilled substance to dilute it

Correct Answer: B

With an unknown or hazardous chemical spill, the priority is to protect people by isolating the area and following facility emergency procedure. Entering the room, opening the door wider, or adding water may worsen exposure or trigger a dangerous reaction. The coughing employee near the doorway shows that exposure is already occurring.

996. A nurse in a long-term care facility is told that the heating system has failed during very cold weather. Which client is at greatest immediate risk from this environmental problem?

- A.** A young adult recovering from an ankle sprain
- B.** An older adult who is thin, confused, and bedridden
- C.** A middle-aged client receiving oral antibiotics for a urinary infection
- D.** An adolescent admitted for observation after a mild concussion

Correct Answer: B

Older adults who are thin, confused, and bedridden are less able to regulate body temperature or take protective action, so they are at higher immediate risk in a cold environment. The other clients are generally less vulnerable to rapid harm from heating failure.

997. During evening rounds, the nurse hears the battery alarm of an infusion pump in a room where the wall outlet is not functioning. The client is receiving a continuous medication infusion that should not be interrupted. What should the nurse do first?

- A.** Silence the alarm and return after finishing rounds
- B.** Stop the infusion until maintenance repairs the outlet
- C.** Plug the pump into a known working outlet and assess that the infusion continues safely
- D.** Document the broken outlet and leave the pump on battery power

Correct Answer: C

The immediate priority is to maintain safe operation of essential equipment and prevent interruption of therapy. Moving the pump to a functioning outlet and confirming continued infusion addresses the urgent risk. Silencing the alarm or documenting without action delays care. Stopping a continuous medication infusion can harm the client.

998. After a community explosion, several clients arrive at a small emergency receiving area. Space is limited, and the nurse must quickly reduce environmental hazards. Which action is most appropriate?

- A.** Place stretchers tightly together so more clients fit in the room
- B.** Keep walkways and access to emergency equipment clear while organizing care areas
- C.** Store portable oxygen tanks beside the exit door for easy access
- D.** Delay cleaning visible blood on the floor until all clients have been registered

Correct Answer: B

In a surge situation, environmental safety depends on maintaining clear walkways and access to emergency equipment while care areas are organized. Crowding stretchers creates trip and evacuation hazards. Oxygen tanks should be stored securely according to facility policy, not casually by an exit. Visible blood on the floor is also a hazard and should not simply be ignored.

999. The nurse notices that a client with mild confusion repeatedly reaches for the overbed table, which has been placed across the room after housekeeping cleaned the floor. Which environmental modification is best to promote safety?

- A.** Apply bilateral wrist restraints to stop the reaching behavior
- B.** Move frequently used items within the client's reach and keep the area uncluttered
- C.** Tell the client to stay in bed until family arrives
- D.** Raise the side rails and lower the lighting in the room

Correct Answer: B

A safer environment reduces the need for the client to overreach or attempt unsafe movement. Placing needed items within reach and keeping the area uncluttered supports safety using the least restrictive approach. Restraints are not the first environmental intervention. Telling the client to stay in bed does not remove the hazard, and dim lighting may worsen orientation and safety.

1000. A practical nurse is assigned to a clinic area when an earthquake begins. Shelves are shaking, and glass items are falling. Which action is the nurse's priority during the shaking?

- A.** Run outside with ambulatory clients immediately
- B.** Stand in the doorway and direct traffic out of the room
- C.** Move away from windows and falling objects and protect self and clients as possible
- D.** Begin calling families to report the emergency

Correct Answer: C

During active shaking, the priority is immediate protection from injury by moving away from windows and falling objects and sheltering as possible. Running outside during the shaking can increase injury risk. Doorways are not automatically the safest location, and calling families is not the first action during the immediate hazard.

1001. A client who uses oxygen asks the nurse whether a petroleum-based lip ointment can be kept at the bedside because the dry air is irritating. Which response by the nurse is best from an environmental safety perspective?

- A. Yes, because the ointment is used on the lips and not near the oxygen tubing
- B. Yes, but only if the amount applied is very small
- C. No, a water-soluble product should be used instead
- D. No, because clients using oxygen should avoid all lip moisturizers

Correct Answer: C

Petroleum-based products should be avoided around oxygen use because they can increase fire risk. A water-soluble alternative is the safer choice. The problem is not solved by using a small amount, and clients using oxygen may still need lip care with an appropriate product.

1002. A practical nurse enters the room of a client who is at high risk for falls. The client is resting in bed with the side rails up, but the nurse notices a wet area on the floor near the bathroom doorway and the client's nonskid socks are on the chair across the room. Which action should the nurse take first?

- A. Move the nonskid socks to the bedside and remind the client to call for help before ambulating
- B. Wipe up the wet floor and place a wet-floor warning until the area is dry
- C. Raise the bed to waist level so the nurse can assess the client more easily
- D. Document the environmental risks and report them at the end of the shift

Correct Answer: B

The immediate hazard is the wet floor because it creates a direct risk for slipping if the client gets up. Removing the present danger comes before teaching or later documentation. Choice A is helpful but does not correct the most urgent hazard already in the room. Choice C increases fall risk. Choice D delays action.

1003. A fire alarm sounds on a medical unit, and the nurse smells smoke coming from a trash can in an unoccupied client room. What is the nurse's priority action?

- A. Close the room door and move clients away from the area
- B. Open the window to clear the smoke
- C. Finish passing medications before responding
- D. Use the elevator to move nearby clients quickly

Correct Answer: A

In a fire situation, the nurse first protects people from danger. Closing the door helps contain smoke and fire, and moving clients away reduces exposure. Opening a window can feed a fire with more oxygen. Delaying to finish medications is unsafe. Elevators should not be used during a fire.

1004. A client using a cane is preparing to walk to the bathroom. The nurse notes the client's cane leaning behind the bedside chair, the room lights are dim, and the path to the bathroom is otherwise clear. Which environmental change is best to make first?

- A. Turn on brighter lighting in the room
- B. Ask physical therapy to evaluate the client's gait later in the day
- C. Place the cane in the bathroom so it is available after toileting
- D. Encourage the client to walk without the cane for short distances

Correct Answer: A

Poor lighting is an immediate environmental hazard that increases the risk of missteps and falls. Improving visibility is the best first change. Physical therapy may be useful later but does not address the current hazard. Placing the cane in the bathroom leaves the client without support when getting there. Walking without the cane is unsafe.

1005. A severe thunderstorm causes a brief power outage in a long-term care facility. Emergency lighting comes on. Which action by the nurse best promotes environmental safety for clients until full power returns?

- A. Encourage clients to continue their normal routines to reduce anxiety

- B.** Ask ambulatory clients to gather in the hallway near the windows
- C.** Check that essential equipment is functioning and limit unnecessary client movement
- D.** Open exterior doors to increase ventilation throughout the building

Correct Answer: C

During a power outage, the nurse should focus on essential safety: verify needed equipment is working and reduce movement that could lead to falls or injury in low-light conditions. Normal routines may need to pause. Hallways near windows may not be safest during a storm, and opening exterior doors can create additional hazards.

1006. The nurse is assigned four clients during a tornado warning. Which client should the nurse relocate first to the safest area of the facility?

- A.** A client receiving a routine dressing change in an interior treatment room
- B.** A client resting in a private room with large exterior windows
- C.** A client watching television in the dining room near a central corridor
- D.** A client in a windowless physical therapy gym on the lower level

Correct Answer: B

During a tornado warning, clients should be moved away from windows and exterior walls to an interior protected area. The client in the private room with large exterior windows is at the greatest immediate environmental risk. The other locations are already relatively safer.

1007. While assisting with admission of multiple clients after a neighborhood apartment fire, the nurse notices that one newly arrived client keeps trying to smoke in a wheelchair while oxygen is in use in the treatment area. Which action is most appropriate?

- A.** Move the oxygen equipment closer so the client does not need to reach for the tubing
- B.** Tell the client smoking is allowed only after signing a liability form
- C.** Remove ignition sources from the area and stop the smoking behavior immediately

- D.** Wait until the respiratory therapist arrives to address the problem

Correct Answer: C

Smoking around oxygen creates an immediate fire hazard. The nurse must act at once to remove ignition sources and stop the unsafe behavior. Choice A worsens the hazard. Choice B is inappropriate because the risk is not solved by a form. Choice D delays response to an urgent danger.

1008. A practical nurse enters a client's room and smells natural gas. The client is stable and asleep. Which action should the nurse take first?

- A.** Turn on the overhead fan to improve air circulation
- B.** Activate electrical lights to better inspect the room
- C.** Move the client out of the room and report the suspected gas leak immediately
- D.** Search the room for the source of the leak before disturbing the client

Correct Answer: C

A possible gas leak creates a serious risk of fire or explosion. The nurse should protect the client by leaving the area and reporting the leak immediately. Using electrical switches or fans can create a spark, so choices A and B are unsafe. Searching for the source delays evacuation from danger.

1009. A nurse is preparing a room for a client who becomes disoriented at night and tries to get out of bed unassisted. Which environmental intervention is most appropriate?

- A.** Keep the room dark to encourage uninterrupted sleep
- B.** Place the bed in a low position with a night-light and the call light within reach
- C.** Place all four side rails up whenever the client is in bed
- D.** Keep the bathroom door closed and the mobility aid outside the room

Correct Answer: B

A low bed, adequate lighting, and an accessible call light reduce injury risk while supporting safer orientation and mobility. Complete darkness can worsen disorientation. Four side rails can function like a restraint and may increase injury risk if the client tries to climb over them. Keeping the bathroom inaccessible and removing the mobility aid does not promote safe independence.

1010. A wildfire in the area causes smoke to enter parts of a rural clinic. Several clients are waiting to be seen. Which client should the nurse move first to the cleanest available indoor area?

- A. A healthy adult with a sprained wrist
- B. A preschool child with a mild rash
- C. An older adult with chronic lung disease who is short of breath at baseline
- D. A young adult awaiting removal of sutures

Correct Answer: C

Smoke exposure poses the greatest immediate risk to the client with chronic lung disease and baseline shortness of breath because even modest air-quality changes may worsen respiratory status. The other clients have less urgent vulnerability to the environmental hazard.

1011. Following a flash flood warning, the nurse is told that the first-floor outpatient area may need to be evacuated. Which action should the nurse take first?

- A. Begin moving clients and essential records from the low-lying area to a higher level
- B. Wait for visible water to enter the building before relocating anyone
- C. Ask stable clients to drive themselves home before any further planning
- D. Continue routine appointments to avoid unnecessary disruption

Correct Answer: A

When flooding threatens a low-lying area, early movement of clients and essential items to higher ground is the safest action. Waiting for water to enter delays protection. Sending clients away without coordinated planning may create new safety issues, and continuing routine operations ignores the hazard.

1012. A practical nurse notices that a portable space heater has been brought into a client's room by a family member during cold weather. The heater is plugged into an extension cord beside oxygen tubing and extra linens. Which action by the nurse is best?

- A. Turn the heater to a lower setting and leave it in place
- B. Remove the heater from the room and explain that it is unsafe in this setting
- C. Move the heater under the overbed table to keep it out of the walkway
- D. Ask the family to unplug the heater only when staff are not in the room

Correct Answer: B

The heater creates multiple hazards: fire risk, improper electrical setup, and close proximity to oxygen tubing and combustible linens. It should be removed. Lowering the setting or relocating it within the room does not make it safe. Intermittently unplugging it does not address the underlying danger.

1013. After an ice storm, a facility's main entrance becomes crowded with melting snow and water tracked in from outside. Environmental services is delayed. The nurse must reduce immediate injury risk while maintaining access for emergency transport. Which action is best?

- A. Block the entrance completely until housekeeping arrives
- B. Place absorbent materials and warning signs, and redirect foot traffic through the driest safe path
- C. Ask all incoming clients to remove their shoes before entering the building
- D. Move wheelchairs away from the entrance so the floor can air-dry faster

Correct Answer: B

The nurse should reduce the active slip hazard while preserving necessary access. Absorbent materials, visible warnings, and traffic redirection are immediate and practical safety measures. Blocking the entrance may interfere with emergency movement. Asking clients to remove shoes is unrealistic and unsafe. Moving wheelchairs does not solve the wet-floor hazard.

1014. A facility receives notice of an incoming client contaminated by an industrial chemical exposure. The client has not yet arrived. Which room setup is most appropriate from an environmental safety perspective?

- A. Prepare a standard semiprivate room with the door open for faster staff access
- B. Place extra linens and cloth furniture in the room for comfort after arrival
- C. Clear unnecessary equipment from the designated area and prepare supplies to limit spread of contamination
- D. Assign the client to the nearest room to the nurses' station regardless of room contents

Correct Answer: C

Before a chemically contaminated client arrives, the nurse should reduce clutter and prepare the area to contain contamination and support safe management. A semiprivate room with an open door increases exposure risk. Extra absorbent materials and unnecessary furnishings can complicate contamination control. Proximity alone is less important than a controlled environment.

1015. During a partial evacuation after a small fire in another wing, the nurse must decide which action is safest for a client who is nonambulatory, receiving oxygen, and has a large amount of equipment attached to the bed. Which action is most appropriate?

- A. Disconnect the oxygen and move the client quickly to reduce evacuation time
- B. Move the client in the bed or with essential support equipment if immediate relocation is required
- C. Leave the client in place until every ambulatory client has been moved
- D. Transfer the client to a chair first so the bed can be used for another client

Correct Answer: B

For a nonambulatory client with significant support needs, the safest approach during necessary evacuation is to move the client with the bed or with essential support equipment maintained as much as possible. Disconnecting oxygen creates avoidable risk. Delaying movement despite a need to relocate may be unsafe. Transferring first to a chair adds instability and can increase harm.

Least-restrictive restraints

1016. A confused older adult keeps trying to get out of bed without assistance and has already pulled off the bed alarm twice. Which action best reflects the use of least-restrictive measures first?

- A. Apply a vest restraint to prevent the client from standing
- B. Place the client in seclusion until agitation decreases
- C. Move the client closer to the nurses' station and increase observation
- D. Tie the client's hands to the side rails during the night

Correct Answer: C

Moving the client closer to the nurses' station and increasing observation uses a less restrictive safety intervention before restraints are considered. Restraints and seclusion are more restrictive and carry physical and psychological risks. Tying restraints to side rails is also unsafe because it can cause injury if the bed position changes.

1017. A client with postoperative delirium is pulling at an IV line and trying to climb over the stretcher rails while being transported for imaging. Which intervention is the least restrictive while still addressing immediate safety?

- A. Ask a staff member to remain with the client during transport
- B. Secure both wrists to the stretcher frame before leaving the unit
- C. Request an order for four-point restraints
- D. Administer a sedative so the client will sleep during transport

Correct Answer: A

Having a staff member remain with the client provides direct supervision and may prevent harm without restricting movement. Wrist restraints, four-point restraints, and sedation are more restrictive interventions and are not the first choice if close observation can maintain safety.

1018. The nurse is caring for a client who repeatedly attempts to remove a feeding tube. Which action should the nurse take first to follow least-restrictive care?

- A. Apply elbow immobilizers immediately
- B. Assess for discomfort, confusion, and the need for toileting
- C. Obtain an order for bilateral wrist restraints
- D. Turn the client away from staff to reduce stimulation

Correct Answer: B

Before using restraints, the nurse should assess and address possible causes of the behavior, such as pain, confusion, or unmet needs. Elbow immobilizers and wrist restraints are more restrictive. Turning the client away from staff does not address likely causes and may reduce monitoring.

1019. A client becomes agitated and tries to hit staff during a dressing change. Which response by the nurse best supports least-restrictive restraint principles?

- A. Stop the procedure if possible and use calm verbal de-escalation
- B. Apply leather restraints to both wrists at once
- C. Ask another nurse to hold the client down until the dressing is finished
- D. Raise all four side rails and leave the room

Correct Answer: A

Calm verbal de-escalation and pausing the trigger if possible are less restrictive ways to reduce escalating behavior. Immediately moving to physical force or restraints is not the least-restrictive approach. Raising all four side rails and leaving the room can increase risk rather than improve safety.

1020. A nurse is caring for a client placed in soft wrist restraints after repeated attempts to remove a central line. Which finding indicates the nurse should intervene immediately?

- A. The restraint is secured to the movable side rail
- B. The client is reminded why the restraint is being used
- C. The nurse documents the client's behavior before application
- D. The nurse offers fluids and toileting at regular intervals

Correct Answer: A

A restraint should not be secured to a side rail because movement of the rail can tighten the restraint and injure the client. Explaining the reason for the restraint, documenting the behavior leading to use, and meeting basic needs are appropriate parts of care.

1021. A client with dementia becomes restless every evening and tries to get up without help. Which nursing action is most appropriate before considering a restraint?

- A. Lower the bed, keep needed items within reach, and institute frequent rounding
- B. Use a jacket restraint during the evening shift only
- C. Request a sedative every afternoon to prevent wandering
- D. Place the client in a room with the door closed to limit stimulation

Correct Answer: A

Environmental adjustments and frequent rounding are classic least-restrictive strategies that reduce fall risk while preserving mobility as much as possible. A jacket restraint and routine sedation are more restrictive. Closing the client in a room may reduce observation and does not directly address safety needs.

1022. A nurse is asked to apply restraints to a client who is shouting and trying to leave the unit. Which question is most important for the nurse to ask first?

- A. What type of restraint is easiest to apply?
- B. Have less restrictive measures already been attempted?
- C. Would it be faster to have security hold the client down?
- D. Can the restraint stay on until the next shift assesses the client?

Correct Answer: B

The nurse should first determine whether less restrictive measures were attempted, because restraint use should be limited to situations in which safer alternatives are ineffective or inadequate. Asking which restraint is easiest or how long it can stay on skips the decision point. Holding the client down for convenience is not an appropriate first approach.

1023. Which nursing action best shows that a restraint is being used for client safety rather than staff convenience?

- A. The restraint is applied because the unit is short-staffed
- B. The restraint is maintained so the client will sleep through the night
- C. The restraint is used only after the client repeatedly tries to remove a life-sustaining device despite other interventions
- D. The restraint is applied because the client asks frequent questions and interrupts care

Correct Answer: C

Restraints are justified when needed to protect the client from immediate harm, such as repeated attempts to remove a life-sustaining device after other measures have failed. Short staffing, promoting sleep, or reducing interruptions reflect convenience, not safety.

1024. A client in soft limb restraints says, 'Please take these off. I promise I won't pull out my tube again.' What is the nurse's best response?

- A. Remove the restraints immediately because the client requested it
- B. Explain that the restraints must stay on for the rest of the shift
- C. Reassess the client's behavior and safety risk to determine whether restraints can be reduced or discontinued
- D. Ignore the request because discussing restraints increases agitation

Correct Answer: C

The nurse should reassess whether the restraint is still needed and discontinue it as soon as it is safe to do so. The goal is always the least restrictive care for the shortest necessary time. Immediate removal without reassessment may be unsafe, while keeping restraints on automatically for the shift ignores the need for ongoing evaluation.

1025. A client with acute confusion is in a chair and repeatedly tries to stand without assistance. Which intervention is less restrictive than a restraint and most appropriate to try first?

- A. Use a chair alarm and stay nearby during high-risk periods

- B. Secure the client to the chair with a sheet

- C. Apply bilateral ankle restraints

- D. Administer a PRN sedative solely to keep the client seated

Correct Answer: A

A chair alarm and close observation are less restrictive options that promote safety without physically limiting the client. Securing a client with a sheet is unsafe and inappropriate. Ankle restraints and sedation used only to keep the client seated are more restrictive measures.

1026. The nurse has used reorientation, pain control, toileting, and one-to-one observation for a client who continues to forcefully remove oxygen tubing and strike staff. Which action is most appropriate now?

- A. Stop oxygen therapy because the client refuses it
- B. Continue verbal redirection only, even though the behavior is escalating
- C. Use the least restrictive restraint necessary to protect the client and staff
- D. Have several staff members manually hold the client down for the rest of the shift

Correct Answer: C

When less restrictive interventions have been attempted and the client still poses immediate danger, the nurse may need to use the least restrictive restraint necessary. Continuing ineffective redirection alone ignores the safety threat. Manually holding the client down for an extended period is not an appropriate substitute for a properly managed restraint. Stopping needed oxygen does not address the safety problem.

1027. A nurse is planning care for four clients. Which client situation most clearly supports trying a least-restrictive alternative instead of a restraint first?

- A. A client gently picks at the nasal cannula and stops when redirected
- B. A client swings a metal IV pole at staff and cannot be redirected
- C. A client repeatedly tears out a fresh surgical drain despite constant observation
- D. A client actively tries to jump from a moving stretcher during transport

Correct Answer: A

A client who stops unsafe behavior with simple redirection should first receive that least-restrictive intervention rather than a restraint. The other clients present more immediate danger or ongoing failure of alternatives, making restraint consideration more likely if necessary for safety.

1028. A nurse finds a confused client with all four side rails raised. The client is more agitated and is trying to climb over them. Which action best applies least-restrictive restraint principles?

- A. Leave the rails up because four raised rails are always safer than two
- B. Lower the bed, use a floor mat if available, and increase observation
- C. Add wrist restraints so the client cannot climb
- D. Tell the client that climbing is not allowed and close the door

Correct Answer: B

For an agitated, confused client, lowering the bed, using a floor mat if available, and increasing observation are less restrictive safety measures that may reduce injury risk. Four side rails can function as a restraint in some situations and may increase the risk of climbing and falling. Adding wrist restraints escalates restriction without first correcting the environment.

1029. A newly licensed nurse says, 'If a client might fall, restraints are the safest choice.' Which response by the charge nurse is best?

- A. Yes, restraints are preferred whenever a client is unsteady
- B. No, restraints can increase injury risk, so less restrictive fall-prevention measures should be used first
- C. Yes, because clients in restraints cannot move enough to be harmed
- D. No, but only because restraints are uncomfortable, not because they affect safety

Correct Answer: B

Restraints can increase agitation, attempts to escape, and injury risk, so less restrictive fall-prevention measures should be tried first. The distractors incorrectly assume restraints are automatically safer or minimize the real safety concerns associated with restraint use.

1030. A client with hypoxia becomes restless and tries to remove the oxygen mask. The nurse reapplies the mask, but the client keeps pulling it off. Which action best reflects least-restrictive clinical judgment?

- A. Immediately apply bilateral wrist restraints without further assessment
- B. Assess whether the mask fit, anxiety, pain, or communication difficulty is causing the behavior and address those factors
- C. Discontinue oxygen because the client is noncompliant
- D. Cover the client's hands with tape mitts and leave the room

Correct Answer: B

Least-restrictive care begins with identifying and addressing the reason for the behavior. An uncomfortable mask, anxiety, pain, or inability to understand the treatment may be corrected without restraints. Immediate restraint use, stopping oxygen, or covering the hands and leaving the client are inappropriate responses.

1031. A client with a urinary tract infection is acutely confused and keeps trying to get out of bed to "go home." The client is not violent and follows simple directions for short periods. Which intervention is the best least-restrictive action for the nurse to implement first?

- A. Apply a vest restraint to prevent wandering
- B. Move the client closer to the nurses' station for frequent observation
- C. Raise all four side rails to keep the client in bed
- D. Request an order for bilateral wrist restraints

Correct Answer: B

Moving the client closer to the nurses' station increases observation and safety without physically restricting movement, so it is the best least-restrictive first step. A vest restraint and wrist restraints are more restrictive measures. Raising all four side rails can function like a restraint and may increase injury risk if the client tries to climb over them.

1032. The nurse enters a room and finds an older adult with dementia repeatedly tugging at a clean, dry urinary catheter. The client says, "This hurts." Which nursing action best supports least-restrictive restraint practice?

- A. Assess the catheter and address possible discomfort before considering restraint
- B. Secure the client's hands in mitt restraints immediately
- C. Tie the catheter tubing tightly to limit movement
- D. Tell the client to stop touching the catheter and leave the room

Correct Answer: A

Least-restrictive care starts by looking for and correcting a cause of the behavior, such as pain, pressure, or catheter traction. If the catheter is causing discomfort, fixing that problem may stop the pulling. Mitt restraints are more restrictive and are not the first response when a reversible cause may be present. Tight tubing increases discomfort, and simply telling the client to stop does not address the cause.

1033. A client recovering from sedation is drowsy, disoriented, and repeatedly reaches toward a fresh abdominal incision. Which action is the least restrictive way to reduce self-injury risk?

- A. Place elbow immobilizers on both arms
- B. Assign a staff member to remain with the client until more alert
- C. Apply soft wrist restraints
- D. Tape the client's hands to the bed linens

Correct Answer: B

Continuous observation by a staff member protects the client while avoiding physical restriction, making it the least restrictive option. Elbow immobilizers and wrist restraints restrict movement. Taping a client's hands is unsafe and inappropriate.

1034. During morning care, a client with delirium becomes increasingly restless and attempts to pull off the blood pressure cuff, pulse oximeter, and gown. The nurse suspects overstimulation. Which intervention best reflects least-restrictive restraint principles?

- A. Finish the full bath quickly despite the client's agitation
- B. Apply wrist restraints until all care is completed
- C. Reduce stimuli and provide care in shorter, calmer periods
- D. Ask two staff members to hold the client's arms during all care

Correct Answer: C

If overstimulation is contributing to agitation, reducing noise, slowing the pace, and clustering care more thoughtfully are appropriate least-restrictive strategies. Wrist restraints and manually holding the client's arms are more restrictive. Forcing completion of care despite escalating agitation may worsen the behavior and does not address the trigger.

1035. A family member says, "Please tie my mother down so she won't fall again." Which response by the nurse is best?

- A. "Restraints are routinely used after a fall."
- B. "A restraint is the best way to keep every client safe."
- C. "We first use the least-restrictive measures that protect her safety."
- D. "Only the family can decide whether restraints are used."

Correct Answer: C

The nurse should explain that safety care begins with the least-restrictive effective measures rather than automatically using restraints. Choice A incorrectly treats restraints as routine. Choice B is overly broad and unsafe. Choice D is incorrect because restraint decisions are not based only on family preference.

1036. A client with acute alcohol withdrawal is in bilateral soft wrist restraints after repeatedly striking staff and attempting to remove life-sustaining equipment. Which nursing action best supports the principle of least-restrictive use after restraints have been started?

- A.** Keep the restraints in place until the end of the shift to avoid repeated episodes
- B.** Reassess frequently and remove restraints as soon as the client can be managed safely by less restrictive means
- C.** Loosen the restraints only if the family asks for a trial release
- D.** Leave the restraints on whenever the client is sleeping so the client does not wake up confused

Correct Answer: B

Least-restrictive practice continues after restraints are applied. The nurse should reassess the ongoing need and discontinue restraints as soon as a safer, less restrictive option is possible. Keeping restraints on for staff convenience or until shift end is inappropriate. Family request does not determine clinical need. Sleep does not justify continued restraint without reassessment.

1037. A client with expressive aphasia becomes agitated and repeatedly pushes away staff hands during a dressing change. The client has not attempted to hit, bite, or kick anyone. Which nursing action is most appropriate first?

- A.** Stop and use simple communication and gestures to explain the procedure
- B.** Apply mitt restraints so the dressing can be changed
- C.** Ask another staff member to hold the client's arms down
- D.** Document that the client is combative and continue quickly

Correct Answer: A

The behavior may reflect fear or inability to understand, not a need for restraint. Using simple words, gestures, and a calm approach is a least-restrictive first response. Mitt restraints and physically holding the client are more restrictive. Labeling the client as combative and pushing through care ignores a likely communication barrier.

1038. The nurse is caring for a client who has bilateral soft wrist restraints in place. Which action by the nurse requires immediate correction?

- A.** Tying the restraint to the movable side rail
- B.** Checking the client's skin and circulation regularly
- C.** Explaining the reason for the restraint to the client
- D.** Attempting less-restrictive measures when the client calms down

Correct Answer: A

Restraints should not be tied to a movable side rail because movement of the rail can tighten the restraint or injure the client. The other actions are appropriate parts of safe restraint care: monitoring the client, communicating the reason, and reassessing for less restrictive options.

1039. A client with moderate dementia becomes restless during mealtimes, stands up repeatedly, and wanders toward other clients' trays. Which intervention is the best least-restrictive option?

- A.** Seat the client in a quieter area with supervision during meals
- B.** Use a tray table to keep the client from standing
- C.** Apply a belt restraint in the chair before the meal starts
- D.** Skip the meal until the client is calmer

Correct Answer: A

A quieter environment with supervision addresses likely triggers and promotes safety without restricting movement, making it the best least-restrictive choice. A tray table used to prevent rising functions as a restraint. A belt restraint is more restrictive. Skipping meals does not address safety appropriately and may harm the client.

1040. The nurse is considering options for a client who repeatedly removes an oxygen cannula, becomes more confused at night, and tries to climb out of bed. Which factor most strongly supports trying a less restrictive intervention before any restraint?

- A. The unit is short-staffed tonight
- B. The client has a high risk for skin breakdown
- C. The behavior may be related to treatable hypoxia or nighttime disorientation
- D. The family is worried the client may bother other clients

Correct Answer: C

If the behavior may be driven by a reversible clinical problem such as hypoxia or by sundowning-like nighttime disorientation, the nurse should first address those causes and use less restrictive strategies. Staffing concerns and family convenience do not justify restraint. Skin breakdown is important, but the strongest reason here is that the behavior may improve when the cause is treated.

1041. A nursing assistant tells the nurse, "I put the wheelchair lap belt on tightly so the client can't get up alone." Which interpretation by the nurse is most accurate?

- A. This is acceptable if the client has a fall history
- B. This is a least-restrictive safety measure because it is used in a wheelchair
- C. This should be treated as a restraint if it prevents the client from getting up freely
- D. This is only a restraint when used during transportation

Correct Answer: C

If the lap belt is used to keep the client from rising freely, it functions as a restraint. The setting or device name does not change that. A fall history alone does not make restrictive use automatically acceptable. It is not limited to transportation situations.

1042. A client recovering from a head injury becomes impulsive and keeps swinging the legs over the side of the bed. The client settles when spoken to calmly and when offered toileting. Which action should the nurse take?

- A. Continue frequent rounding and offer toileting on a schedule
- B. Place the client in ankle restraints to stop bed exits
- C. Insert a urinary catheter so toileting is no longer needed
- D. Sedate the client so the client remains in bed

Correct Answer: A

Because the client responds to calm redirection and toileting, the nurse should continue those effective less restrictive measures. Ankle restraints and sedation are more restrictive. Inserting a catheter only to avoid toileting is inappropriate and does not address the least-restrictive principle.

1043. A nurse is deciding whether a restrictive measure is justified for four clients. Which situation most strongly indicates that less restrictive strategies have failed and a restraint may need to be considered for immediate safety?

- A. A client with mild confusion asks the same question repeatedly but remains in bed
- B. A client with anxiety paces in the hall and accepts coaching to use deep breathing
- C. A client with delirium continues to forcefully remove a tracheostomy collar and strike at staff despite reorientation, pain control, and one-to-one observation
- D. A client with insomnia requests to walk around the unit at night

Correct Answer: C

This client presents ongoing immediate risk to airway support and staff safety despite multiple less restrictive measures already being tried, making further escalation more justifiable. The other clients can be managed with less restrictive approaches and do not show the same level of immediate danger.

1044. A client with cognitive impairment becomes agitated whenever a roommate's television is loud. The client then starts pulling at bedding and attempting to leave the bed. What is the nurse's best initial action?

- A.** Apply a soft waist restraint before agitation gets worse
- B.** Reduce the environmental trigger and reassess the client's behavior
- C.** Raise all side rails and instruct the client to stay in bed
- D.** Ask the provider for a restraint order based only on the pattern

Correct Answer: B

The nurse has identified a possible trigger, so the best first step is to modify the environment and reassess. This follows least-restrictive practice by addressing the cause of the behavior. A waist restraint, all side rails, and obtaining a restraint order without first trying the obvious alternative are more restrictive responses.

1045. The nurse is reviewing a plan of care for a client at risk for injury from confusion. Which planned intervention best reflects least-restrictive restraint principles?

- A.** Use restraints at bedtime because confusion usually worsens overnight
- B.** Keep the client in bed all day to reduce the chance of falls
- C.** Provide regular pain assessment, toileting, and close observation to reduce unsafe behavior
- D.** Apply mitt restraints whenever the nurse leaves the room

Correct Answer: C

Regular pain assessment, toileting, and observation address common unmet needs that can lead to unsafe behavior and are less restrictive than restraints. Routine bedtime restraint, prolonged bed rest, and applying mitts whenever the nurse leaves are convenience-based or unnecessarily restrictive approaches.

1046. A postoperative client becomes restless and keeps trying to pull out an IV line. The client is alert enough to answer questions and says, "My arm burns where that tape is." Which action should the nurse take first to follow least-restrictive restraint principles?

- A.** Apply soft wrist restraints to prevent IV removal
- B.** Check the IV site and securement, then address the source of discomfort
- C.** Ask the provider for an order for a sedative
- D.** Place mitt restraints on both hands

Correct Answer: B

Burning at the IV site suggests pain, infiltration, or poorly applied tape may be causing the behavior. Least-restrictive practice starts with identifying and correcting reversible causes of agitation before using restraints. Wrist restraints, mitts, or sedation are more restrictive and are not first-line when the client may stop pulling once the discomfort is relieved.

1047. A client with mild confusion keeps standing up without using the call light, even after repeated reminders. The nurse wants to reduce fall risk while avoiding unnecessary restraint. Which intervention is most appropriate?

- A.** Use a chair alarm and place the client where staff can observe more closely
- B.** Tie the client into the chair with a sheet
- C.** Raise all four side rails whenever the client is seated
- D.** Request immediate physical restraints because reminders have not worked

Correct Answer: A

Closer observation and a chair alarm are less restrictive safety measures that can help staff respond quickly when the client attempts to get up. Tying the client with a sheet is an unsafe restraint, and raising all four side rails can function as a restraint. Physical restraints are not the next step simply because reminders alone were ineffective.

1048. The nurse is caring for a client who becomes agitated every evening and starts trying to remove the dressing on a healing leg wound. Which nursing action best supports least-restrictive care?

- A.** Apply elbow restraints before the behavior begins each evening
- B.** Keep the room brightly lit and active so the client stays awake
- C.** Provide calm reorientation, meet needs such as pain relief and toileting, and reduce stimulation
- D.** Move the client to the hallway so staff can watch from a distance

Correct Answer: C

Least-restrictive care includes addressing triggers and unmet needs, such as pain, discomfort, or toileting, while using a calm environment and reorientation. Preemptively applying restraints is not least restrictive. A brightly lit, active room may worsen evening agitation, and hallway placement may increase confusion or overstimulation.

1049. A client in soft wrist restraints has a provider prescription and has stopped trying to hit staff. The client is calm and follows simple commands. Which action by the nurse is best?

- A.** Leave the restraints in place until the end of the shift to prevent recurrence
- B.** Remove one restraint at a time only if the family agrees
- C.** Assess whether the restraints can be discontinued because they are no longer necessary
- D.** Replace the wrist restraints with a tighter vest restraint

Correct Answer: C

The least-restrictive principle means restraints should be used only as long as necessary. If the client is calm and no longer showing the dangerous behavior, the nurse should assess for discontinuation. Waiting until the shift ends is not client-centered, family approval is not the deciding factor, and switching to a vest restraint is even more restrictive.

1050. Which nursing situation is the clearest example of a restraint being used for staff convenience rather than client safety?

- A.** A confused client is placed near the nurses' station for closer observation
- B.** A lap belt is applied so a client will stay seated during a short period of understaffing
- C.** A sitter is assigned to a client repeatedly attempting to remove a central line
- D.** The nurse offers frequent toileting to a client who gets up impulsively

Correct Answer: B

Using a lap belt to keep a client seated because staffing is limited reflects convenience, not a justified least-restrictive safety intervention. The other options are nonrestraint strategies aimed at reducing risk while preserving mobility and autonomy as much as possible.

1051. A client with delirium is trying to climb over the bed rails and strike at staff during attempts to reorient. The nurse has already reduced noise, treated pain, offered toileting, and assigned one-to-one observation, but the behavior continues and the client is at immediate risk of injury. What is the best next action?

- A.** Continue verbal redirection only because restraints should never be used
- B.** Apply the most restrictive device available to guarantee immobility
- C.** Use a restraint only if necessary for immediate safety and continue reassessing for early removal
- D.** Wait for family to arrive before taking any further action

Correct Answer: C

When less restrictive strategies have been attempted and the client still poses an immediate risk to self or others, restraint use may become necessary. The key is that it should be used only for safety, not convenience, and the nurse must continue reassessing for the earliest possible removal. Saying restraints should never be used is incorrect, and using the most restrictive option is not consistent with least-restrictive practice. Waiting for family could delay needed safety action.

1052. A client repeatedly reaches for a nasogastric tube while awake but relaxes when given something to do with the hands. Which intervention best follows least-restrictive restraint principles?

- A. Apply bilateral mitt restraints immediately
- B. Provide a supervised activity item, such as a washcloth to hold, while observing the client
- C. Administer a sedative before each nursing interaction
- D. Tape the client's hands to the blanket

Correct Answer: B

If the client is calmer and less likely to interfere with the tube when given a simple activity, that is a less restrictive and individualized intervention. Mitts, sedation, and especially taping the hands are more restrictive and not the best first choice when a safe alternative is effective.

1053. The nurse is planning care for a client at risk for pulling out monitoring wires during episodes of confusion. Which plan best reflects least-restrictive restraint practice?

- A. Apply restraints at the first sign of restlessness to avoid escalation
- B. Use restraints only overnight, since confusion is usually worse then
- C. Attempt reorientation, meet physical needs, conceal or secure equipment when possible, and increase observation before considering restraints
- D. Ask the family to decide whether restraints should be used

Correct Answer: C

A least-restrictive plan starts with nonrestraint strategies: reorientation, addressing needs, managing the environment and equipment, and increasing observation. Restraining at the first sign of restlessness is premature, using restraints automatically overnight is not individualized, and family input may be helpful but does not replace clinical judgment and facility policy.

1054. A newly admitted client says, "If I get upset, just tie me down so I don't bother anyone." Which response by the nurse is best?

- A. We can do that if you give verbal permission now
- B. Restraints are used only when necessary for safety, and we try less restrictive measures first
- C. That decision belongs only to your family
- D. Once restraints are started, they usually stay on until discharge

Correct Answer: B

The nurse should explain that restraints are not used simply by request or for convenience. They are reserved for safety situations, and less restrictive measures are attempted first when possible. Client verbal permission alone does not make restraint use appropriate, family does not solely decide, and restraints are not kept on routinely until discharge.

1055. A client with confusion becomes more agitated each time four staff members approach at once to stop the client from getting out of bed. Which change is most likely to support least-restrictive care?

- A. Have one staff member speak calmly while others stay back unless needed
- B. Add a second set of restraints to control the arms and legs
- C. Turn on the television to distract the client
- D. Tell the client firmly that getting up is not allowed

Correct Answer: A

Multiple staff approaching at once can be perceived as threatening and may escalate agitation. A calmer, simpler approach with one person speaking may reduce distress and avoid the need for restraint. Adding more restraints is more restrictive, television may increase stimulation, and a firm command without addressing confusion is less effective.

1056. The nurse observes an assistive personnel member fastening a tray table tightly across a client's chair and saying, "Now he can't get up on his own." What is the nurse's priority interpretation?

- A. This is acceptable if the client is weak
- B. This is a restraint because it restricts the client's freedom to get up
- C. This is only a positioning device, not a restraint
- D. This is appropriate if used during mealtime

Correct Answer: B

Any device that restricts a client's freedom of movement and the ability to get up can be considered a restraint. The staff member's statement makes the intent clear: preventing the client from rising. Weakness or mealtime does not automatically make it acceptable, and calling it a positioning device does not change its function.

1057. A client with acute confusion repeatedly reaches toward a fresh tracheostomy tube. The nurse reorients the client, provides pain relief, repositions the client, and obtains continuous observation, but the client still forcefully grabs at the airway. Which rationale best supports the possible use of a restraint at this time?

- A. The client is creating inconvenience for the staff
- B. The family is anxious and prefers a restraint
- C. The client poses an immediate threat to an essential airway despite less restrictive measures
- D. The nurse wants to avoid documenting repeated redirection attempts

Correct Answer: C

Restraint use may be justified when a client presents an immediate danger to self, such as removing an essential airway, and less restrictive interventions have already failed. Staff convenience, family preference alone, and reducing documentation burden are not appropriate rationales.

1058. A nurse is reviewing possible interventions for a client who becomes restless and attempts to get out of bed after not voiding for several hours. Which option best demonstrates least-restrictive thinking?

- A. Apply wrist restraints before checking for causes of restlessness
- B. Offer toileting assistance and reassess the client's behavior afterward
- C. Request a standing order for restraints during all night shifts
- D. Place all side rails up and leave the room

Correct Answer: B

Unmet elimination needs are a common reversible cause of restlessness. Offering toileting first addresses a likely trigger and may remove the need for any restraint. Restraining before assessing causes, using routine standing restraint plans, or placing all side rails up and leaving are not least-restrictive approaches.

Incident and near-miss awareness

1059. While helping a client transfer from bed to chair, the client's knees begin to buckle. The nurse lowers the client safely to the floor with no injury. What is the best next action?

- A. Do not mention it because the client was not harmed
- B. Report the event and complete the facility's incident documentation
- C. Wait until the end of the shift to decide whether it matters
- D. Document the event only in the incident report, not in the client record

Correct Answer: B

A near-miss or no-harm safety event should still be reported according to facility policy so risks can be analyzed and future harm prevented. Choice D is a trap because the client record should include relevant facts about the event and assessment, while the separate incident report is not the only documentation.

1060. A nurse enters a room and notices that a client with a fall-risk bracelet is walking alone toward the bathroom. The nurse reaches the client in time, assists the client back to bed, and no fall occurs. How should this event be classified?

- A. Expected behavior that does not require follow-up
- B. A near miss because harm was avoided
- C. A sentinel event because the client could have fallen
- D. An infection-control event only

Correct Answer: B

A near miss is an event that could have caused harm but did not, either by chance or timely intervention. It is not a sentinel event because no serious outcome occurred. The key point is recognizing risk even when injury is prevented.

1061. The nurse discovers that a client with a severe latex allergy has a latex tourniquet placed on the bedside table before it is used. Which action is most appropriate first?

- A. Remove the item from the area and report the near miss
- B. Leave it in place because no exposure occurred
- C. Use it only if the client agrees
- D. Wait to see whether the next shift notices it

Correct Answer: A

The immediate priority is to eliminate the hazard, then report the near miss so the system issue can be addressed. Choices B, C, and D fail to protect the client from a known allergy risk. Even without harm, unsafe setup near the client is reportable.

1062. Which statement by a newly licensed nurse shows correct understanding of incident reporting?

- A. Only events that cause injury need to be reported
- B. Near misses should be reported so the team can identify safety problems
- C. Incident reports replace charting in the medical record
- D. Reporting an incident means the nurse is admitting fault

Correct Answer: B

Near-miss reporting helps identify hazards before they harm a client. Incident reporting is not limited to injuries, does not replace charting, and is generally used for safety review rather than as an admission of fault. A common trap is thinking no harm means no report.

1063. A nurse is about to give a medication when the barcode scanner alerts that the medication was prepared for a different client. The nurse stops before administration. What should the nurse do after securing the correct medication for the right client?

- A. Ignore the alert because the scanner prevented harm
- B. Report the near miss according to facility policy
- C. Chart that the wrong medication was administered
- D. Ask the pharmacy to handle the report without nursing involvement

Correct Answer: B

This is a near miss: the error was caught before reaching the client. Reporting it helps identify process failures such as labeling, storage, or preparation mistakes. Choice C is incorrect because the medication was not given. Choice A misses the prevention purpose of reporting.

1064. A client is found in bed with the side rail lowered after being identified as needing assistance with mobility. The client is unharmed. Which documentation in the medical record is most appropriate?

- A. Staff negligence caused a dangerous situation
- B. Side rail down; client found in bed awake, denies pain, no injury noted; safety measures reapplied and provider notified per policy

- C. Incident report completed and filed with risk management
- D. Near miss occurred due to poor teamwork on the unit

Correct Answer: B

Chart objective facts, assessment findings, and actions taken in the client record. Avoid blame and opinions. Incident-report details belong in the reporting system, not as a statement that an incident report was filed in the medical record if policy prohibits that. Choice B is factual and client-focused.

1065. During a sterile dressing change, the nurse notices that the inner surface of a glove touched the overbed table before donning. The wound has not been touched yet. What is the best action?

- A. Continue because the client has not been exposed yet
- B. Apply the glove and avoid touching the wound with that hand
- C. Stop, replace contaminated supplies as needed, and restart using aseptic technique
- D. Ask another nurse to finish so the event does not need reporting

Correct Answer: C

Once contamination is recognized, the nurse should stop and correct the break in technique before proceeding. This prevents client exposure and reflects near-miss awareness. Choice A is a trap because the lack of current harm does not make contaminated technique acceptable.

1066. A UAP tells the nurse, "I almost used the blood pressure cuff from the contact-precautions room on another client, but I noticed the isolation sign first." What is the nurse's best response?

- A. "Since you caught it in time, there is nothing more to do."
- B. "Thank you for speaking up. We still need to report the near miss and review equipment cleaning and room-specific use."
- C. "Do not mention it again because it may get you in trouble."
- D. "Only the charge nurse can decide whether this matters."

Correct Answer: B

A good safety culture encourages speaking up about near misses. The event should still be reported and used to improve systems. Choices A and C discourage reporting, which increases future risk. The nurse should support transparency and prevention.

1067. Which situation is the clearest example of an incident rather than a near miss?

- A. A mislabeled specimen is caught before it leaves the room
- B. A nurse almost enters the wrong code into an infusion pump but corrects it
- C. A client receives a meal tray containing a known allergen and develops hives
- D. A contaminated glove is noticed before contact with a sterile field

Correct Answer: C

An incident involves an event that actually reaches the client and causes or could cause harm. In choice C, the allergen reached the client and produced a reaction. The other choices describe errors caught before client harm occurred, making them near misses.

1068. At shift report, the offgoing nurse says, "I almost used the wrong client's glucometer strip container, but I checked the label in time." What is the primary reason this event should be shared with the team and reported per policy?

- A. It proves the nurse is unsafe and should be removed from client care
- B. It may reveal a systems problem such as similar packaging or poor storage setup
- C. It allows the event to be billed as additional care
- D. It replaces the need to reinforce standard precautions

Correct Answer: B

Near-miss analysis often uncovers latent system problems, such as look-alike supplies or poor organization, that can lead to future harm. The purpose is prevention and system improvement, not punishment. Choice A reflects blame rather than safety learning.

1069. A nurse finds that a client's identification band has another client's name on it just before collecting a blood specimen. The client has not yet been stuck. Which action should the nurse take first?

- A. Collect the specimen because the room assignment confirms the client's identity
- B. Remove the band and continue without identification until a new one is printed
- C. Stop the procedure and verify the client's identity using agency-approved identifiers before proceeding
- D. Ask a family member to state the client's name and date of birth, then proceed

Correct Answer: C

A wrong identification band is a serious near miss. The procedure should stop until the nurse verifies identity using approved identifiers and corrects the problem. Choice A is unsafe because room number is not a reliable identifier. Choice D may help gather information but does not replace approved identification steps.

1070. A nurse notices that the sharps container in a treatment room is filled above the recommended line, but no injury has occurred. Which action is best?

- A. Push the contents down to make more space
- B. Close the room until the next scheduled supply round
- C. Replace the sharps container according to policy and report the hazard
- D. Ignore it unless someone reports a needlestick

Correct Answer: C

An overfilled sharps container is a preventable hazard and near-miss situation for staff and clients. The nurse should remove the hazard promptly and report it so recurrence can be addressed. Choice A is unsafe because it increases needlestick risk.

1071. A nurse preparing to insert a urinary catheter opens the kit and realizes the client in the room is on transmission-based precautions that require additional PPE not yet worn. The sterile field is already set up but untouched. What is the best action?

- A. Proceed quickly to avoid wasting supplies

- B. Step out, don the required PPE, and use the existing sterile field since it was not touched

- C. Consider the field contaminated, discard supplies as needed, perform hand hygiene, don appropriate PPE, and set up again

- D. Ask the client to remain still while the nurse reaches for PPE from the doorway

Correct Answer: C

Once the nurse leaves or interrupts the setup to obtain required PPE, sterility and safe sequence are compromised. The correct response is to restart with proper PPE and aseptic setup. Choice B is a common trap because an untouched field is not automatically preserved through an improper sequence and room movement.

1072. A facility reviews several recent near misses: a high-alert medication nearly given to the wrong client, a specimen almost sent with the wrong label, and a diet tray nearly delivered to a client with a severe food allergy. Which conclusion best reflects high-quality safety reasoning?

- A. These events are unrelated because they involve different departments
- B. The common thread is failure to use client-specific verification before an action reaches the client
- C. The events should not be analyzed because no client was harmed
- D. The most efficient solution is to remind staff to be more careful

Correct Answer: B

Across all three events, the shared risk is inadequate final verification tied to the individual client. Strong safety reasoning looks for patterns across settings, not just isolated mistakes. Choice D is weaker because "be more careful" targets individuals without fixing the process.

1073. After a client nearly falls during transfer, the nurse assesses the client, reports the event, and reviews what contributed to it. Which contributing factor most strongly suggests a systems issue rather than only an individual lapse?

- A. The nurse felt rushed during a busy hour

- B.** The transfer belt was stored in a different hallway each day with no standard location
- C.** The nurse forgot to ask for help on this one occasion
- D.** The client did not follow instructions exactly

Correct Answer: B

A missing or inconsistently located transfer device points to a process and environment problem affecting multiple staff members. That is more clearly a systems issue than a one-time lapse. Choices A, C, and D may contribute, but they do not identify a durable process flaw as directly as inconsistent equipment availability.

1074. A nurse enters a room to administer an oral medication and finds a cup of pills at the bedside that was left by another nurse. The client says, "I haven't taken them yet." The nurse cannot confirm what the pills are from the cup alone. What is the best action?

- A.** Ask the client to take the pills while the nurse checks the medication record afterward
- B.** Discard the pills according to policy, identify the ordered medications in the MAR, and notify the appropriate nurse or provider per facility policy
- C.** Give the pills if the client states they are the usual morning medications
- D.** Document that the client refused the medication and leave the room

Correct Answer: B

Unattended medications at the bedside create a safety risk because the nurse cannot verify the drugs or ensure correct administration. The safest response is to remove the unidentified pills according to policy, verify current orders, and communicate the event appropriately. Choice A and C rely on assumptions instead of verification. Choice D is inaccurate because the client did not refuse; the issue is medication safety and a reportable event or near miss depending on policy and outcome.

1075. A nurse notices that a client's oxygen tubing is connected to wall suction instead of the oxygen flowmeter. The client is not in distress, and the error is corrected before oxygen is started. How should the nurse view this event?

- A.** As a near miss that still warrants reporting per policy
- B.** As a harmless mistake that does not need follow-up because no oxygen was running
- C.** As a client refusal of therapy
- D.** As an equipment failure only if the tubing was defective

Correct Answer: A

This is a near miss because an unsafe setup was identified and corrected before harm occurred. Near misses are valuable to report because they reveal hazards in the system and can help prevent future injury. Choice B is a common trap: lack of harm does not mean lack of safety significance. Choice C mislabels the situation, and choice D is too narrow because the safety problem was incorrect setup, not necessarily defective equipment.

1076. After giving a dose of insulin, the nurse realizes the glucometer result entered into the chart belonged to a different client. The insulin dose was based on that incorrect value. The client is awake and asymptomatic. What is the nurse's priority action?

- A.** Complete an incident report at the end of the shift
- B.** Recheck the client's blood glucose and notify the appropriate licensed provider and charge nurse per policy
- C.** Wait until the next scheduled glucose check to see whether the client becomes symptomatic
- D.** Ask the other client's nurse to correct the charting error first

Correct Answer: B

Because the wrong data influenced treatment already given, the priority is client assessment and prompt communication so the client can be monitored and managed safely. An incident report may also be needed, but it does not come before immediate client care. Choice C delays response to a potentially serious medication error. Choice D focuses on documentation before the client's condition.

1077. A student nurse asks why the unit tracks near misses such as almost using the wrong IV pump channel or nearly labeling a specimen incorrectly. Which response by the practical nurse is best?

- A.** Near misses are tracked mainly to identify which individual should be disciplined
- B.** Near misses matter because they reveal weaknesses in processes before a client is harmed
- C.** Near misses are optional to discuss because they are not part of client care
- D.** Near misses should be ignored if the nurse corrected the problem independently

Correct Answer: B

The best safety reasoning is that near misses provide early warning about faulty processes, confusing workflows, labeling problems, communication gaps, or equipment issues. Learning from them helps prevent future harm. Choice A reflects a punitive mindset rather than a safety-improvement approach. Choices C and D miss the prevention value of near-miss reporting.

1078. A nurse finds a bottle of cleaning solution on the medication cart with no label. No one has used it yet. Which action is best?

- A.** Smell the solution to identify it, then relabel it
- B.** Set it aside and ask the next shift whether they know what it is
- C.** Remove it from use according to facility policy and report the unsafe condition
- D.** Leave it in place because it is probably housekeeping's supply

Correct Answer: C

An unlabeled chemical container is an environmental safety hazard and should not remain in use. The correct response is to remove it according to policy and report the condition so the risk can be addressed. Choice A is unsafe because guessing by smell is not an acceptable identification method. Choice B delays correction, and choice D makes an unsafe assumption.

1079. A nurse administers an antibiotic and then notices the client's allergy list includes that medication class. The client reports itching but no difficulty breathing. After stopping the infusion and assessing the client, what information is most important to include when reporting the event?

- A.** Only that the nurse made a medication error
- B.** The names of coworkers who were nearby when the drug was given
- C.** The client's response, assessments, interventions taken, and notifications made
- D.** A personal opinion about why the electronic record is confusing

Correct Answer: C

When reporting an incident, objective clinical details are essential: what occurred, how the client responded, what assessments were done, what interventions were performed, and who was notified. Choice A is incomplete and does not support safe follow-up. Choice B is not the priority unless specifically relevant. Choice D may be discussed separately in safety review, but personal opinion is not the core clinical content needed in the event report.

1080. During a busy medication pass, a nurse scans a client's wristband and the electronic system displays another client's profile. The nurse realizes two clients were given identical temporary printed bands earlier in the day. Both clients are stable and no medication has been given yet. Which issue does this event most strongly suggest?

- A.** A systems problem in the identification process
- B.** A client education deficit about wristbands
- C.** An unavoidable technology glitch with no process implications

D. A problem that can be solved only by reminding nurses to work faster

Correct Answer: A

Duplicate temporary identification bands point to a process failure in client identification, not merely an individual lapse. Near misses like this often reveal system vulnerabilities that require correction at the workflow level. Choice B is not the key issue because the clients did not create the bands. Choice C dismisses an important hazard, and choice D is incorrect because speed does not fix a faulty identification process.

1081. A nurse preparing a sterile dressing change notices the package for the sterile gauze is damp on one corner before opening it. The package has not been used. What is the best interpretation?

- A.** The gauze is acceptable if the expiration date is current
- B.** The gauze can be used for a clean wound but not a sterile procedure
- C.** The gauze should be considered contaminated and replaced; the event reflects a supply safety issue
- D.** The gauze can be dried and then opened with sterile gloves

Correct Answer: C

Moisture can contaminate sterile packaging, so the item should not be used. Replacing it protects the client, and recognizing the issue helps identify supply handling or storage problems. Choice A ignores package integrity. Choice B incorrectly lowers the standard despite uncertainty about contamination. Choice D is unsafe because drying the package does not restore sterility.

1082. At the end of the shift, a nurse realizes a client was transported off the unit without the required contact-precaution gown covering draining wounds. The client has already returned, and no known exposure has been identified. What is the best next step?

- A.** Do nothing because there is no confirmed transmission
- B.** Document only that the client left the unit and returned safely
- C.** Report the event per policy and participate in review of how the transport process failed

D. Wait to report unless another staff member complains

Correct Answer: C

Even without confirmed harm, failure to follow required precautions is a reportable safety event because it may expose others and reveals a process gap. The nurse should follow policy and support review of contributing factors. Choice A confuses absence of known harm with absence of risk. Choice B leaves out the safety issue. Choice D delays appropriate follow-up.

1083. A nurse hears a bed alarm sounding, enters immediately, and finds the client halfway to the floor because a family member tried to help without calling staff. The nurse supports the client, who is not injured. Which description is most accurate?

- A.** It is not reportable because the client was not injured
- B.** It is a near miss or incident requiring follow-up according to policy because an unsafe event occurred
- C.** It is solely a family issue, not a nursing safety concern
- D.** It should be documented only if the family admits causing the event

Correct Answer: B

An unsafe event occurred involving fall risk and required nurse intervention to prevent injury. Whether categorized under local policy as a near miss or incident, it warrants follow-up and documentation according to policy. Choice A is a common trap: no injury does not eliminate safety significance. Choice C ignores the nurse's role in safety planning and education. Choice D adds an unnecessary condition.

1084. A nurse notices that several alcohol-based hand-rub dispensers near isolation rooms are empty during the same shift. No infection exposure has been identified. Why should this still be escalated promptly?

- A.** Because empty dispensers are primarily a housekeeping inconvenience
- B.** Because the pattern suggests a unit-level hazard that can undermine infection prevention
- C.** Because staff can simply use gloves instead of hand hygiene until refill

D. Because only visitors are affected by hallway dispensers

Correct Answer: B

Multiple empty hand-rub dispensers represent a systems-level safety hazard because they can interfere with timely hand hygiene and increase infection risk. Prompt escalation helps correct the problem before harm occurs. Choice A minimizes the issue. Choice C is incorrect because gloves do not replace hand hygiene. Choice D ignores effects on staff, clients, and visitors.

1085. Which nurse statement best reflects appropriate understanding of the relationship between charting and an incident report?

- A.** The nurse should mention in the medical record that an incident report was completed
- B.** The medical record should include objective facts about the client and care, while the incident report follows facility policy separately
- C.** If an incident report is filed, charting details are unnecessary
- D.** Only the charge nurse should document events related to a near miss

Correct Answer: B

The medical record should contain objective information about the event, assessments, interventions, and client response. Incident reporting is handled separately according to policy. Choice A is a frequent trap because references to the incident report are generally not part of the medical record. Choice C is false because charting remains essential. Choice D is incorrect because the nurse involved still documents the care provided.

1086. A nurse preparing to perform tracheostomy care finds that the suction setup at the bedside has tubing attached but the suction regulator is turned fully off. The client is stable, and care has not begun. What should the nurse do first?

- A.** Begin care and ask another staff member to adjust the suction if needed
- B.** Proceed because the suction may not be necessary
- C.** Correct and test the setup before starting care, then report the equipment readiness issue per policy if indicated

D. Move the client to another room before performing care

Correct Answer: C

Before tracheostomy care, emergency and supportive equipment should be ready. The immediate priority is to correct and verify the suction setup before beginning. Because this unsafe condition could have delayed response during care, it may also require reporting according to policy. Choices A and B risk proceeding without needed equipment. Choice D is unnecessary and delays care.

1087. A client receives a meal tray containing a clearly labeled milk carton despite a documented severe milk allergy. The nurse removes the tray before the client eats anything. Which factor would most strongly support treating this as more than an isolated mistake?

- A.** The client says the tray looked normal
- B.** The dietary aide is new to the unit
- C.** Two other clients with food restrictions received incorrect trays that day
- D.** The tray arrived during a busy lunch period

Correct Answer: C

A pattern affecting multiple clients suggests a process problem rather than a one-time slip. Repeated incorrect trays point toward failures in order transmission, allergy flagging, tray assembly, or delivery checks. Choice B may contribute but does not by itself establish a broader system issue. Choices A and D are less meaningful for identifying root cause.

1088. A nurse notices that the wrong-size blood pressure cuff was used earlier on a client, producing a falsely high reading that was entered in the chart. No treatment was based on the reading. What is the best reason to report and correct this event?

- A.** Only treatment errors matter, so reporting is unnecessary here
- B.** Incorrect assessment data can influence later decisions even if no immediate action was taken
- C.** The reading should stay in the chart because it was recorded at the time
- D.** The nurse should wait to see if the next blood pressure is also high

Correct Answer: B

Even when no immediate treatment occurred, inaccurate assessment data can mislead future clinical decisions. The reading should be addressed and the event reported according to policy because it reflects a safety issue in assessment technique. Choice A wrongly limits reporting to treatment errors. Choice C ignores the need to correct inaccurate information appropriately. Choice D delays action.

1089. A nurse is about to transfer a client from bed to chair and notices the wheelchair footrests are loose and one wheel does not lock securely. The client has not been moved yet. Which action is best?

- A.** Proceed carefully with assistance because no injury has occurred
- B.** Use the wheelchair for this transfer only, then tag it later
- C.** Remove the wheelchair from use and report the unsafe equipment condition
- D.** Ask the client whether the wheelchair feels stable enough to use

Correct Answer: C

A faulty wheelchair creates a preventable safety risk even if no harm has occurred yet. Removing it from use and reporting the hazard addresses a near miss and helps prevent future injury. Proceeding carefully is still unsafe because the equipment is known to be defective.

1090. A nurse prepares to administer an enteral feeding and notices the feeding bag hanging at the bedside is labeled for Room 214, but the nurse is caring for the client in Room 216. The feeding has not been started. Which classification best fits this event?

- A.** Expected variation in workflow
- B.** Near miss requiring prompt correction and reporting per facility process
- C.** Adverse event because the wrong product reached the bedside
- D.** Documentation issue only, since nothing was infused

Correct Answer: B

This is a near miss because the wrong feeding was brought to the bedside but was caught before reaching the client. It is more than a documentation problem because the error had the potential to cause harm. It is not an adverse event because the feeding was not started.

1091. While preparing a dose of liquid medication, a nurse notices that the pharmacy label on the bottle is smudged and the concentration cannot be read clearly. Another nurse says, "It is probably the same stock we always use." What is the nurse's best action?

- A.** Administer the usual dose because the medication name is readable
- B.** Compare the bottle color with previous bottles to confirm it
- C.** Set the medication aside and obtain a clearly labeled product before giving the dose
- D.** Ask the client whether this medicine looks familiar

Correct Answer: C

If the concentration cannot be verified, the medication should not be given. The safest action is to remove the unclear product from use and obtain one that can be identified correctly. Assuming the usual concentration or relying on appearance creates risk for a medication error.

1092. A nurse reviewing the shift report learns that a client almost received another person's laboratory requisition for a blood draw, but the phlebotomist caught the error during identification. The client was not harmed. Which is the best reason this event should still be reported through the facility process?

- A.** It may reveal a process weakness that could harm a client later
- B.** It automatically means staff negligence occurred
- C.** It will replace the need for charting in the medical record
- D.** It proves the identification policy is ineffective in all situations

Correct Answer: A

Near misses are reported because they can uncover system problems before a client is harmed. Reporting does not automatically mean negligence, does not replace charting, and does not prove the entire policy is ineffective. The key value is prevention through process review.

1093. A client says, "The pill you just brought me looks different from the one I usually take." The nurse rechecks the medication and realizes the pill came from another client's drawer, but the client has not taken it. What should the nurse do first?

- A.** Tell the client the pharmacy sometimes changes tablet appearance
- B.** Remove the medication and verify the correct medication before administration
- C.** Document client refusal of the medication
- D.** Give the pill because the dose on the wrapper matches the order

Correct Answer: B

The nurse should stop the process immediately and verify the correct medication before administering anything. This is a near miss identified before harm occurred. Reassuring the client without checking is unsafe, and documenting a refusal is inaccurate because the concern is medication verification, not refusal.

1094. During morning care, a nurse finds that a client's bed is in the highest position with both upper side rails raised after housekeeping finished cleaning the room. The client is asleep and has not tried to get up. Which action is most appropriate?

- A.** Lower the bed, ensure safety measures are in place, and report the hazard as a near miss
- B.** Leave the bed as it is because the client is currently sleeping
- C.** Raise all four side rails to keep the client safer
- D.** Wait until the client wakes up and ask whether the height is comfortable

Correct Answer: A

A high bed increases fall risk, especially when the client awakens and tries to move. The nurse should correct the hazard immediately and report it as a near miss because injury could have occurred. Leaving it, adding more rails, or waiting for the client to wake up fails to address the immediate safety risk.

1095. A nurse discovers that a specimen collected 20 minutes ago is at the desk with no client label attached. Several unlabeled labels from different clients are also on the desk. The specimen has not been sent to the laboratory. Which action is best?

- A.** Label the specimen using the room number on the collection bag
- B.** Ask the unit clerk which client usually has this test ordered and label it accordingly
- C.** Discard the current specimen per policy, notify the appropriate personnel, and recollect with correct identification
- D.** Send the specimen to the laboratory and call with the client name afterward

Correct Answer: C

An unlabeled specimen should not be guessed at or sent for processing. The safe response is to follow policy, which commonly includes rejecting the specimen, notifying the team, and recollecting it with proper identification. The major trap is trying to 'save time' by labeling based on assumptions.

1096. A nurse gives a report to the oncoming nurse and mentions that a confused client was found wearing another client's dentures earlier in the shift. The dentures were removed immediately, the clients were assessed, and the charge nurse notified. What additional follow-up is most important for incident and near-miss awareness?

- A.** Document only that the dentures were exchanged to avoid embarrassing the clients
- B.** Review how the denture containers were identified and stored to reduce recurrence
- C.** Ask dietary to provide softer food for both clients
- D.** Avoid reporting it because the item was not a medication

Correct Answer: B

Incident awareness includes looking for why the event happened so the risk can be reduced in the future. Misidentification and storage practices are key contributors here. Limiting documentation or skipping reporting misses the prevention focus. Dietary changes do not address the cause.

1097. A nurse preparing an inhaled medication notices that the medication canister for one client is stored in a bin with another client's respiratory supplies. The clients have similar last names. No medication has been administered. Which factor makes this situation especially important to escalate?

- A.** Inhaled medications are always high-alert medications
- B.** The similar names and mixed storage increase the chance of a repeat identification error
- C.** Respiratory supplies do not require client-specific organization
- D.** Near misses should be discussed only if the clients complain

Correct Answer: B

Mixed storage combined with look-alike or sound-alike client identification cues creates an ongoing risk for wrong-client errors. That pattern makes escalation important because the problem is likely to recur. The other options either overgeneralize or minimize the hazard.

1098. A nurse notices that a client's signed consent form in the chart belongs to a different client with the same first name. The scheduled procedure has not started. Which action should the nurse take first?

- A.** Remove the incorrect form from active use and stop progression toward the procedure until the client's records are verified
- B.** Ask the provider to proceed because the mistake is clerical
- C.** Have the client sign the form already in the chart if the procedure sounds correct
- D.** Wait to see whether preoperative staff identify the problem later

Correct Answer: A

A wrong consent form in the chart is a serious wrong-client risk. The priority is to stop the process and verify the client's record before any further steps occur. Proceeding, reusing the wrong form, or waiting for someone else to notice could allow a major error to continue.

1099. On a rehabilitation unit, a nurse notes three separate events over two days: a gait belt missing from one room during transfer, a shower chair with a cracked seat found before use, and a client nearly transported without a transfer sheet for a dependent move. No client was harmed. What is the best interpretation?

- A.** These are unrelated small issues because no injury occurred
- B.** These events suggest a broader equipment and transfer-safety process problem
- C.** These events should be handled only by replacing the cracked shower chair
- D.** These are expected on busy units and do not need formal follow-up

Correct Answer: B

A cluster of related near misses points to a system issue, not just isolated mistakes. Missing transfer equipment, damaged equipment, and skipped transfer supports all suggest a process problem involving readiness, maintenance, or safety checks. Focusing on one item alone would miss the larger pattern.

1100. A nurse removes a used needle from a room and sees another staff member reach toward the overfilled sharps container to push contents down by hand so there is room for one more syringe. Which response best reflects incident and near-miss awareness?

- A.** Allow it this time because no one has been stuck yet
- B.** Suggest placing the syringe on the counter until environmental services arrives
- C.** Stop the action, use an available safe alternative per facility process, and report the overfilled container hazard
- D.** Recap the syringe temporarily to reduce the chance of a needlestick

Correct Answer: C

An overfilled sharps container is a significant hazard, and pushing contents down by hand creates immediate risk for needlestick injury. The nurse should stop the unsafe act, use a safer disposal option according to facility process, and report the hazard. Recapping or leaving sharps unsecured creates additional danger.

1101. A nurse precepts a new graduate who says, "If a mistake is caught before it reaches the client, there is no reason to bring it up because nothing happened." Which response by the nurse is best?

- A.** Correct, because only events causing injury are worth reporting
- B.** Not exactly; near misses matter because they show where the system allowed error to get close to the client
- C.** That depends only on whether the provider asks about it
- D.** Usually yes, unless the event involved a medication

Correct Answer: B

Near misses are valuable because they reveal vulnerabilities in the care process before harm occurs. Reporting and reviewing them support prevention. The other options incorrectly limit safety reporting to injury, provider request, or medication events only.

Transfer devices

1102. A nurse is preparing to move a client from the bed to a stretcher after surgery. Which action best promotes safety when using a lateral transfer device?

- A.** Raise the stretcher slightly higher than the bed and move the client quickly
- B.** Lock both the bed and stretcher before transferring the client
- C.** Lower both side rails and pull the client by the arms
- D.** Ask the client to twist toward the stretcher during the move

Correct Answer: B

Locking both surfaces prevents unexpected movement during the transfer and is a basic safety step with transfer devices. Raising one surface higher can increase sliding risk, pulling by the arms can injure the client, and twisting after surgery may cause pain or harm.

1103. A client with left-sided weakness is transferring from bed to chair with a gait belt and assistance. Where should the nurse position the chair before the transfer?

- A.** On the client's strong side
- B.** At the foot of the bed
- C.** On the client's weak side
- D.** Directly behind the nurse

Correct Answer: A

Placing the chair on the client's strong side allows the client to pivot toward the side with better strength and control. Positioning on the weak side makes the transfer harder and less stable. The other options do not support an efficient, controlled pivot.

1104. The nurse plans to use a mechanical lift to move a client from bed to chair. Which finding requires the nurse to stop and correct the situation before starting the transfer?

- A.** The client is wearing nonskid footwear
- B.** The sling size matches the manufacturer's label for the client
- C.** The lift's battery indicator shows a low charge
- D.** The chair is placed near the bed

Correct Answer: C

A low battery could cause the lift to stop during the transfer, creating an immediate safety risk. Nonskid footwear, a correctly sized sling, and a nearby chair are appropriate. The key issue is whether the transfer device is safe and functional before use.

1105. A nurse is helping a client stand with a gait belt. Which technique is most appropriate?

- A.** Hold the client under the arms while the client rises
- B.** Place the gait belt over the client's clothing at the waist
- C.** Tie the gait belt loosely so it does not feel restrictive

D. Pull upward on the front of the belt with a jerking motion

Correct Answer: B

The gait belt should be secured snugly over clothing at the waist to provide a stable handhold. Holding under the arms can injure the client, a loose belt is unsafe, and jerking upward increases fall risk and poor body mechanics.

1106. A client begins to feel weak while pivoting from the bed to a chair. The nurse is using a gait belt. What is the nurse's priority action?

- A.** Attempt to hold the client upright until more staff arrive
- B.** Ease the client to the floor while protecting the head
- C.** Release the gait belt and step away to avoid injury
- D.** Turn the client quickly toward the chair and push forward

Correct Answer: B

If a client starts to fall, the nurse should use the gait belt and controlled body mechanics to ease the client to the floor and protect the head. Trying to hold the client upright can injure both the nurse and client. Releasing the client or forcing the client into the chair is unsafe.

1107. Which client is the best candidate for a sit-to-stand lift rather than a full-body mechanical lift?

- A.** A client who is unconscious and cannot bear weight
- B.** A client who can follow directions and bear some weight on at least one leg
- C.** A client with bilateral leg paralysis and no trunk control
- D.** A client who becomes combative during hands-on care

Correct Answer: B

A sit-to-stand lift is generally appropriate for a client who can cooperate, follow directions, and bear some weight. Clients who are unconscious, cannot bear weight, or lack trunk control usually require a full-body lift. A combative client may be unsafe for this type of transfer.

1108. The nurse and an assistive personnel member are transferring a client with a friction-reducing sheet. Which instruction by the nurse is most important to prevent staff injury?

- A.** Keep your feet close together while pulling
- B.** Reach across the bed to avoid repositioning yourself
- C.** Move the client with a coordinated count and use your legs
- D.** Twist at the waist so the pull is stronger

Correct Answer: C

Using a coordinated count and lifting or shifting with the legs promotes safe body mechanics and smoother movement. Twisting and overreaching increase the risk of musculoskeletal injury. Feet close together reduce stability rather than improve it.

1109. A nurse is about to transfer a client from bed to wheelchair. The client says, "I feel dizzy when I sit up." What should the nurse do first?

- A.** Continue the transfer quickly before the client gets weaker
- B.** Return the client to a lying position and reassess
- C.** Place the gait belt higher on the chest for more control
- D.** Ask the client to close the eyes and pivot anyway

Correct Answer: B

Dizziness during position change may indicate orthostatic intolerance or instability. The nurse should stop the transfer, return the client to a safe position, and reassess before proceeding. Continuing despite dizziness increases the risk of a fall.

1110. A nurse must move a heavier client up in bed. Which option is the safest choice if available?

- A.** Ask the client to grasp the nurse's neck while being pulled up
- B.** Use a friction-reducing device with help from additional staff
- C.** Lift from under the client's shoulders and hips
- D.** Pull on the drawsheet alone in one strong motion

Correct Answer: B

A friction-reducing device with adequate assistance reduces shear, decreases staff strain, and improves control. Pulling alone or lifting manually from the shoulders and hips increases injury risk. Having the client grab the nurse's neck is unsafe for both people.

1111. When using a full-body mechanical lift, which observation requires the most immediate intervention by the nurse?

- A. The client's blanket has slipped off one shoulder
- B. One sling loop is attached at a different length than the matching loop on the other side
- C. The wheelchair footrests have been moved away
- D. The client asks how long the transfer will take

Correct Answer: B

Uneven sling loop attachment can tilt the client and lead to slipping or falling from the lift, so it must be corrected immediately. The other findings may need attention, but they are not the same level of immediate mechanical danger.

1112. A client who has partial weight-bearing status after hip surgery needs to move from bed to chair. Which transfer method is most appropriate?

- A. Have the client place full weight on both legs and pivot quickly
- B. Use a transfer approach that maintains ordered weight-bearing limits
- C. Ask the client to hop to the chair without assistance
- D. Slide the client by pulling on the affected leg

Correct Answer: B

The safest method is the one that follows the prescribed weight-bearing restriction while moving the client with control. Full weight-bearing, hopping unsupported, or pulling on the operative leg can cause injury and violate postoperative precautions.

1113. Before using a mechanical lift for the first time with a client, which nursing action is most important?

- A. Skip explanation to reduce the client's anxiety
- B. Test the client's hand grasps

C. Review how the specific device operates and check that it works properly

D. Place the sling only after the client is suspended

Correct Answer: C

The nurse should know how the specific lift operates and verify that it is functioning correctly before use. Explaining the process is helpful, not harmful. Testing hand grasps alone is not the key safety step, and the sling must be placed before suspension.

1114. A nurse is transferring a client with a gait belt when the client suddenly grabs the nurse's shoulders for support. Which response is best?

- A. Tell the client to hold tighter while the nurse leans backward
- B. Support the client with the gait belt and widen the nurse's stance
- C. Lift the client under the arms to complete the transfer
- D. Turn the client loose so the nurse does not fall too

Correct Answer: B

The nurse should maintain support through the gait belt and improve personal stability with a wide base of support. Leaning backward, lifting under the arms, or releasing the client can worsen loss of balance and increase injury risk.

1115. Which situation best indicates that the nurse should seek additional assistance or another transfer method instead of attempting a one-person transfer with a gait belt?

- A. The client reports mild fear about falling
- B. The client can follow directions and stand briefly
- C. The client is unable to bear weight and cannot assist with the move
- D. The client asks to use the bathroom before returning to bed

Correct Answer: C

A client who cannot bear weight and cannot assist is not a good candidate for a one-person gait-belt transfer. The nurse should obtain more help or use a mechanical device. Mild fear or toileting needs do not by themselves determine transfer method if the client is physically able.

1116. During a bed-to-wheelchair transfer, which wheelchair preparation is most important for safety?

- A. Position the wheelchair with the wheels unlocked for easier movement
- B. Raise the footrests so the client's feet can rest on them during standing
- C. Lock the wheelchair brakes before the client stands
- D. Place the wheelchair several feet away to allow a wider pivot

Correct Answer: C

Locking the wheelchair brakes is essential so the chair does not move as the client stands or sits. Unlocked wheels create a major fall risk. Footrests should not obstruct standing, and placing the chair far away makes the transfer harder and less safe.

1117. A nurse is selecting a transfer device for four clients. Which client is the best candidate for transfer with a transfer board?

- A. A client who is alert, can follow directions, and has good upper-body strength but cannot bear weight on either leg
- B. A client who is confused and tries to stand unexpectedly during care
- C. A client who is fully dependent and cannot support the head or trunk
- D. A client who becomes short of breath and pale when sitting upright

Correct Answer: A

A transfer board is appropriate for a client who can cooperate, follow instructions, and assist with the move using upper-body strength. A confused client, a fully dependent client, or a client who is unstable when upright would need a different approach because a board transfer requires participation and relative stability.

1118. The nurse is preparing to move a client from a bed to a wheelchair using a transfer board. Which action is most important before the client begins to slide across the board?

- A. Place the wheelchair on the client's strong side and lock the wheels
- B. Raise the bed to its highest position to reduce bending
- C. Keep a pillow under the client during the move for comfort

D. Ask the client to hold the nurse around the neck for stability

Correct Answer: A

Positioning the wheelchair on the stronger side and locking the wheels helps prevent movement during transfer and uses the client's best functional side. Raising the bed too high can make the transfer unsafe, a pillow interferes with the board transfer, and the client should not pull on the nurse's neck.

1119. During transfer training, a student nurse asks why a transfer belt should not be placed over a client's axillae. Which response by the nurse is best?

- A. "It can shift the client's center of gravity too far forward."
- B. "It can cause discomfort and possible injury to the chest and underarm area."
- C. "It makes the transfer take longer than needed."
- D. "It prevents the nurse from bending at the knees."

Correct Answer: B

A transfer belt should be secured around the waist, not the axillae, because placement under the arms can cause pain and injury. The issue is client safety, not speed or the nurse's body mechanics alone.

1120. A nurse must choose between a manual pivot transfer and a mechanical lift for a client recovering from a stroke. Which assessment finding most strongly supports using a mechanical lift?

- A. The client asks for privacy during care
- B. The client can sit unsupported for 20 minutes
- C. The client follows commands but cannot bear weight and has poor trunk control
- D. The client wears nonskid socks and a gait belt

Correct Answer: C

A client who cannot bear weight and has poor trunk control generally needs the full support of a mechanical lift. Privacy preferences and proper footwear do not determine the safest device. Sitting unsupported is helpful information, but inability to bear weight with poor control is the stronger indicator for lift use.

1121. Two staff members are using a lateral transfer device to move a client from a stretcher to a bed. Which action by the nurse best reduces shearing and skin injury?

- A. Pull the client by the arms to keep the torso aligned
- B. Lift slightly with the device and move the client in one coordinated motion
- C. Raise the head of the bed to 60 degrees before moving the client
- D. Place the client halfway across, then pause to reposition the linens

Correct Answer: B

Using the lateral transfer device correctly and moving the client in one smooth, coordinated motion helps reduce dragging and shearing. Pulling by the arms is unsafe, a high head-of-bed angle increases sliding forces, and stopping midway can increase friction and instability.

1122. A nurse is preparing to transfer a client with a mechanical lift. The nurse notices the sling label states a weight capacity below the client's documented weight. What should the nurse do?

- A. Continue if two staff members assist instead of one
- B. Use the sling only for a short transfer from bed to chair
- C. Stop and obtain equipment with an appropriate capacity
- D. Add a gait belt over the sling for extra support

Correct Answer: C

Equipment weight limits must be followed. If the sling capacity is below the client's weight, the nurse should stop and get the correct equipment. Extra staff or an added gait belt does not make overloaded equipment safe.

1123. A client with a recent abdominal surgery needs assistance to stand. The nurse plans to use a transfer belt. Which additional action is most appropriate?

- A. Place the belt directly over the incision to provide firm support
- B. Avoid the transfer belt and select another safe method if the belt would press on the surgical area

C. Tighten the belt as much as possible to limit trunk movement

D. Position the belt under the client's rib cage to avoid the waistline

Correct Answer: B

A transfer belt should not be used in a way that places pressure on a fresh surgical site. If belt placement would compromise the incision or cause pain or harm, the nurse should use a different transfer method. Tightening over or near the incision increases risk rather than safety.

1124. While using a full-body lift, the nurse has raised the client a few inches off the bed and notes that one of the sling loops is attached at a different color-coded length than the matching loop on the other side. The client is tilted. What is the best action?

- A. Lower the client back to the bed and correct the sling attachment before proceeding
- B. Continue the transfer slowly because the lift is already supporting the weight
- C. Ask the client to shift body weight toward the higher side
- D. Pull on the lower side of the sling until the client is level

Correct Answer: A

Uneven sling attachment can make the client unstable and unsafe in the lift. The nurse should lower the client and correct the attachment before continuing. Trying to compensate during suspension increases the risk of tipping or sling failure.

1125. A nurse is caring for a client with right-sided weakness who will transfer from bed to a chair using a pivot technique. Which placement is best?

- A. Place the chair parallel to the foot of the bed on the client's right side
- B. Place the chair next to the bed on the client's left side
- C. Place the chair directly in front of the bed facing the client
- D. Place the chair at the far side of the room to allow a wider turn

Correct Answer: B

For a pivot transfer, positioning the chair on the client's stronger side helps the client assist more effectively and reduces the amount of turning needed. With right-sided weakness, the stronger side is the left. The other options increase distance, awkward movement, or instability.

1126. A nurse is planning a transfer for a client who is cooperative but has severe arthritis in both shoulders and cannot push well with the arms. Which transfer device is least appropriate?

- A. Transfer board
- B. Full-body mechanical lift
- C. Lateral transfer device for bed-to-stretcher movement
- D. Mechanical lift selected according to the client's abilities

Correct Answer: A

A transfer board relies heavily on upper-body strength and the ability to push and reposition during the slide. Severe shoulder arthritis makes that option poorly suited. The other choices can provide more support depending on the transfer situation.

1127. A client is being transferred from a chair back to bed with a sit-to-stand lift. Halfway through the transfer, the client says, "My knees are buckling." Which nursing action is best?

- A. Instruct the client to lock the knees and complete the transfer quickly
- B. Support the client's weight manually under both axillae
- C. Return the client to a secure seated position and reassess whether this device remains appropriate
- D. Detach the leg support to allow a more natural standing posture

Correct Answer: C

If the client's knees are buckling during a sit-to-stand lift transfer, the nurse should stop, return the client to safety, and reassess. This may indicate the client cannot safely use that device at that time. Rushing, lifting under the axillae, or removing supports increases the risk of injury.

1128. A nurse must transfer a client from bed to a chair. The client can follow directions and move the arms well but has a new below-knee amputation, poor standing balance, and is ordered not to bear weight on the remaining foot until fitted with protective footwear. Which transfer method is most appropriate?

- A. Stand-pivot transfer with a gait belt
- B. Transfer board with close assistance
- C. Sit-to-stand lift
- D. Have the client hop to the chair with one assistant

Correct Answer: B

This client cannot safely perform a standing transfer because weight bearing is restricted and standing balance is poor. A transfer board may be appropriate because the client can follow directions and use the arms to assist without standing. A sit-to-stand lift still requires standing ability, and hopping is unsafe.

1129. The nurse and another staff member are using a friction-reducing device to reposition a client up in bed. The client has a large sacral pressure injury and reports significant pain with movement. Which technique is best?

- A. Pull the client up quickly in one forceful motion to shorten the procedure
- B. Use the device to lift and slide the client with coordinated movement while avoiding dragging on the sheet
- C. Turn the client onto the side first, then pull on the top leg to move the hips upward
- D. Raise the head of the bed fully so gravity keeps the client in place after the move

Correct Answer: B

For a client with skin breakdown, the goal is to reduce friction and shear as much as possible. Coordinated use of the friction-reducing device helps minimize dragging over the pressure injury. Forceful pulling, pulling on a limb, and raising the head of the bed fully all increase stress on the skin.

1130. A nurse is deciding whether a client can be moved with a gait belt from bed to commode. Which combination of findings makes a gait-belt transfer most unsafe?

- A. Alert, anxious, and wearing loose slippers

- B.** Cooperative, moderate arm strength, and reports incisional pain rated 4/10
- C.** Drowsy after medication, unable to follow commands consistently, and has an unsteady knee on the stronger side
- D.** Hard of hearing, needs instructions repeated, and uses a walker at home

Correct Answer: C

A gait-belt transfer requires the client to participate, follow directions, and have enough stability to bear weight safely. Drowsiness, inconsistent command-following, and instability in the stronger leg make this transfer unsafe and suggest the need for more assistance or another device. The other options present concerns that can often be corrected or managed.

1131. A nurse is preparing to use a sit-to-stand lift to move a client from the bed to a chair. Which client finding would make the sit-to-stand lift inappropriate and suggest a different transfer device is needed?

- A.** The client can bear some weight and follow directions
- B.** The client has adequate head control but feels embarrassed about needing help
- C.** The client cannot support body weight through the legs and cannot maintain a sitting posture
- D.** The client reports mild dizziness when first sitting at the side of the bed

Correct Answer: C

A sit-to-stand lift is intended for a client who can participate to some degree, including bearing at least some weight and maintaining enough body control for the transfer. A client who cannot support weight through the legs and cannot maintain sitting posture needs a more supportive device, such as a full-body lift. Choice A describes a typical candidate for a sit-to-stand lift. Choice B involves emotion, not physical ability. Choice D may require reassessment and caution, but it does not by itself rule out the device as clearly as total inability to support weight and posture does.

1132. A nurse is deciding whether to use a sit-to-stand lift for a client transferring from bed to chair. Which client finding makes this device inappropriate?

- A.** The client can follow simple directions and grasp handles
- B.** The client can bear some weight on at least one leg
- C.** The client has poor head control and cannot maintain an upright sitting position
- D.** The client becomes tired after standing for 20 seconds

Correct Answer: C

A sit-to-stand lift requires the client to participate by maintaining some upright posture and usually bearing at least some weight. Poor head control and inability to sit upright safely indicate that the client needs a more supportive device, such as a full-body lift. Choices A and B describe findings that support use of a sit-to-stand lift. Choice D may require closer monitoring, but fatigue alone does not automatically rule it out if the client otherwise meets safety criteria.

1133. The nurse is transferring a client with a gait belt from bed to a chair. During the pivot, the client suddenly says, "I feel weak," and the knees begin to give way. What should the nurse do first?

- A.** Try to pull the client upright by the gait belt
- B.** Ease the client down in a controlled manner and protect the client from falling
- C.** Leave the client standing and call for more staff
- D.** Turn the client quickly toward the chair and complete the transfer

Correct Answer: B

If a client starts to fall during a gait-belt transfer, the priority is to prevent injury by lowering the client in a controlled way. Trying to pull the client upright can injure both the client and the nurse. Leaving the client unsupported is unsafe, and rushing to finish the transfer increases fall risk.

1134. Before transferring a client with a gait belt from bed to wheelchair, which action by the nurse is most important?

- A. Keep the wheelchair footrests in place so the client can step over them
- B. Lock the wheelchair wheels before the client stands
- C. Raise the bed above the nurse's waist level
- D. Ask the client to hold onto the nurse's shoulders

Correct Answer: B

Locking the wheelchair wheels is a key safety step because it prevents the chair from moving during the transfer. Footrests are typically moved out of the way rather than left where the client could trip. Raising the bed too high can make the transfer unsafe, and clients should not hold onto the nurse's shoulders because that destabilizes both people.

1135. A nurse is preparing to move a client from bed to stretcher with a lateral transfer device. Which client is the best candidate for this device?

- A. A client who needs to move between two flat surfaces and should not stand for the transfer
- B. A client who is able to walk to the stretcher with minimal help
- C. A client who can pivot independently and prefers not to use equipment
- D. A client who needs practice bearing weight before discharge

Correct Answer: A

A lateral transfer device is intended for moving a client horizontally between surfaces such as a bed and stretcher, especially when standing is not appropriate. Choices B, C, and D describe situations where standing or walking ability is present, so a lateral device would not be the best choice.

1136. A postoperative client with a chest tube and an IV pump is being transferred from bed to a chair with assistance. Which nursing action is most important regarding the attached equipment?

- A. Move the client first and reconnect the equipment after the transfer
- B. Clamp the chest tube temporarily so it does not pull during movement

- C. Organize and move the tubing and equipment with the client to avoid tension or dislodgment
- D. Place the IV pump on the bed until the client is settled in the chair

Correct Answer: C

During transfers, lines and tubes must be managed so they move with the client and do not become taut, tangled, or dislodged. Disconnecting equipment casually is unsafe, and a chest tube should not be clamped just for routine movement. Leaving the pump behind on the bed could pull on the IV tubing.

1137. The nurse is helping a client stand using a gait belt. Which action by the client should the nurse encourage to make the transfer safer?

- A. Wrap both arms around the nurse's neck
- B. Push off from the bed or chair with the hands when instructed
- C. Pull upward on the gait belt during the count of three
- D. Keep the feet close together and lean backward

Correct Answer: B

Pushing off from the bed or chair helps the client participate safely and reduces strain on the nurse. Clients should not pull on the gait belt or grab the nurse's neck, because that can cause loss of balance and injury. Feet placed too close together and leaning backward make standing less stable.

1138. A nurse plans to use a transfer belt for a client moving from bed to commode. Which finding requires the nurse to stop and choose another method?

- A. The client reports mild dizziness when first sitting up
- B. The client has a new colostomy at the waistline where the belt would be placed
- C. The client needs reminders to keep both feet flat on the floor
- D. The client requests that the room curtain be closed

Correct Answer: B

A gait or transfer belt should not be placed over an area where it could injure surgical sites, ostomies, or other vulnerable structures. A new colostomy at the belt line makes this method inappropriate. Mild dizziness may require reassessment before proceeding, but it does not specifically rule out the belt the way direct pressure on the ostomy does. The other options do not make the device unsafe by themselves.

1139. When using a full-body mechanical lift, which action is best before raising the client fully off the bed?

- A.** Elevate the client quickly to shorten the time in the sling
- B.** Raise the client only a few inches first to verify sling placement and client stability
- C.** Remove the client's nonskid footwear so the feet hang freely
- D.** Unlock the bed wheels so the bed can move with the lift

Correct Answer: B

Lifting the client a few inches first allows the nurse to check sling security, body alignment, and tolerance before continuing. Raising the client too quickly can hide problems until the client is fully suspended. Bed and receiving surface wheels are generally secured for safety during setup rather than unlocked so surfaces can roll unexpectedly.

1140. The nurse and an assistive personnel member are preparing to reposition a dependent client up in bed using a friction-reducing sheet. The client has a Foley catheter and oxygen tubing. Which action best promotes safety?

- A.** Remove the oxygen tubing briefly so it does not get tangled
- B.** Pull from the client's shoulders and hips separately for better control
- C.** Count together and move the client in one coordinated motion while protecting the tubing from traction
- D.** Raise the head of the bed as high as possible before moving the client upward

Correct Answer: C

Coordinated movement using the friction-reducing sheet limits dragging and helps protect the client, while also allowing staff to manage attached tubing and prevent pulling. Oxygen should not be removed casually. Pulling body parts separately increases shear, and a high head-of-bed position works against upward repositioning.

1141. A nurse is selecting a transfer method for a client who is alert and cooperative after a long hospitalization. The client can sit at the edge of the bed, has strong arm movement, but becomes dizzy on standing and is not safe to bear weight. Which option is most appropriate?

- A.** Stand-pivot transfer with one nurse
- B.** Sit-to-stand lift because the client is cooperative
- C.** Transfer board from bed to wheelchair with close assistance
- D.** Ambulate the client several steps to the wheelchair with a walker

Correct Answer: C

A transfer board is appropriate when the client can participate with the upper body but should not stand or bear weight. The major trap is choosing a sit-to-stand lift just because the client is cooperative; that device still depends on some standing ability and postural support. Stand-pivot transfer and walking are unsafe here because the client is dizzy on standing and not safe to bear weight.

1142. A nurse is preparing to use a sit-to-stand lift for a client who follows commands and can grasp the handles. Which additional assessment finding is most necessary to confirm that this is an appropriate device?

- A.** The client can bear some weight through the legs during the transfer
- B.** The client prefers not to use a gait belt
- C.** The client can state the date and hospital name correctly
- D.** The client has eaten breakfast

Correct Answer: A

A sit-to-stand lift is intended for a client who can participate, including bearing some weight through the legs. Being able to follow commands and hold the handles is not enough by itself. Orientation may be helpful in general, but it does not replace the need for weight-bearing ability. Meal status and personal preference do not determine whether the device is mechanically appropriate.

1143. A nurse must transfer a client from bed to chair. The client is alert, understands directions, and can bear weight briefly, but the left knee sometimes buckles without warning. Which transfer plan is safest?

- A.** Perform a one-person pivot transfer with a gait belt because the client is cooperative
- B.** Skip the transfer because any knee weakness means the client must remain in bed
- C.** Reassess the plan and use a more supportive device or additional assistance rather than relying on a basic pivot transfer
- D.** Tell the client to lock the weak knee and proceed quickly before fatigue occurs

Correct Answer: C

Unexpected knee buckling makes a simple pivot transfer risky even if the client is alert and partly weight-bearing. The safest approach is to reassess and choose a method that provides more support or more help. The trap is option A: cooperation alone does not make a pivot transfer safe when a key weight-bearing joint is unreliable. Option B is too absolute, and option D minimizes a serious fall risk.

1144. A nurse is choosing a transfer device for a client who needs to move from a wheelchair to the bed. The client is alert, follows directions, has good sitting balance, and can use both arms well. The client cannot safely bear weight because of severe weakness in both legs. Which device is most appropriate?

- A.** A transfer board with close assistance
- B.** A gait belt for a stand-pivot transfer
- C.** A sit-to-stand lift
- D.** A walker with one staff member assisting

Correct Answer: A

A transfer board is the best choice for a client moving between surfaces who has good upper-body strength and sitting balance but cannot safely bear weight. A gait belt and a stand-pivot transfer require the client to bear weight, and a sit-to-stand lift also requires some weight-bearing ability. A walker is not appropriate because the client cannot safely support weight through the legs.

Infection-control practices

1145. A practical nurse is preparing to give oral medications to a client with influenza. Which action best reflects standard precautions?

- A.** Wear gloves only if contact with body fluids is expected
- B.** Place the client in a negative-pressure room
- C.** Use a sterile gown for all contact with the client
- D.** Avoid hand hygiene if gloves are worn throughout care

Correct Answer: A

Standard precautions are used for all clients and include wearing gloves when exposure to blood, body fluids, secretions, excretions, or contaminated items is anticipated. A negative-pressure room is used for airborne precautions, not routine standard precautions. Sterile gowns are not required for all contact, and hand hygiene is still necessary even when gloves are used.

1146. A client with suspected *Clostridioides difficile* infection has frequent diarrhea. Which infection-control action should the practical nurse take first when entering the room to provide care?

- A.** Put on an N95 respirator
- B.** Don gloves and a gown
- C.** Apply a surgical mask to the client
- D.** Set up a sterile field at the bedside

Correct Answer: B

C. difficile requires contact precautions, so the nurse should don gloves and a gown before room entry and client contact. An N95 respirator is used for airborne precautions, and a surgical mask on the client is more relevant to droplet transmission. A sterile field is unrelated to routine entry for this situation.

1147. The practical nurse is removing gloves after emptying a urinary drainage bag. Which action is most important to prevent transmission of microorganisms?

- A. Clean the drainage spout with sterile water
- B. Remove gloves quickly to reduce contact time
- C. Perform hand hygiene immediately after glove removal
- D. Place the gloves in a linen hamper

Correct Answer: C

Hand hygiene immediately after glove removal is essential because gloves can have microscopic defects and hands may become contaminated during removal. Sterile water is not the key step here, speed of removal does not ensure safety, and contaminated gloves belong in appropriate waste disposal, not a linen hamper.

1148. A nurse prepares to use a multi-dose vial for two clients during a medication pass. Which action best reduces the risk of infection transmission?

- A. Use the same syringe if the needle is changed between clients
- B. Store the vial at the bedside for faster access
- C. Disinfect the vial stopper and use a new sterile syringe for each entry
- D. Draw up both clients' doses at the same time with one syringe

Correct Answer: C

Disinfecting the stopper and using a new sterile syringe for each entry reduces contamination risk. Changing only the needle is not enough because the syringe itself is contaminated once used. Bedside storage increases cross-contamination risk, and one syringe should never be used for multiple clients.

1149. A client on droplet precautions needs to go to radiology for a chest x-ray. Which instruction is most appropriate?

- A. Have the client wear a mask during transport
- B. Delay the test until isolation is discontinued
- C. Transport the client only at the end of the day
- D. Cover the client's gown with a sterile drape

Correct Answer: A

For droplet precautions, the client should wear a mask during transport to reduce spread of respiratory droplets. Necessary tests should not automatically be delayed, end-of-day transport is not the main protective measure, and a sterile drape is not indicated.

1150. During wound care, the practical nurse accidentally touches the sterile dressing supplies with an ungloved hand after opening the package. What is the best response?

- A. Proceed because the hand was clean
- B. Use the supplies for this client only
- C. Replace the contaminated supplies before continuing
- D. Ask another nurse to continue with the same setup

Correct Answer: C

Once sterile supplies are touched by a nonsterile hand, sterility is broken and the supplies must be replaced. Clean is not the same as sterile. Using contaminated supplies 'for this client only' still increases infection risk, and handing off the same contaminated setup to another nurse does not correct the break in asepsis.

1151. A practical nurse is caring for four clients. Which client should the nurse see first to help prevent transmission of infection?

- A. A client with a clean surgical incision needing pain medication
- B. A client with tuberculosis who removed the respirator mask to walk in the hallway
- C. A client with diabetes asking for a snack
- D. A client on contact precautions requesting a blanket

Correct Answer: B

A client with tuberculosis who is unmasked in the hallway creates an immediate airborne transmission risk and should be addressed first. The other needs are important but do not pose the same urgent risk to others. This item tests prioritization based on infection spread, not comfort needs.

1152. The nurse is collecting a urine specimen from an indwelling urinary catheter for culture. Which technique is most appropriate?

- A.** Disconnect the catheter from the drainage tubing and collect urine in a cup
- B.** Obtain urine from the drainage bag after it has collected for several hours
- C.** Withdraw urine from the sampling port using aseptic technique
- D.** Open the drainage spout and let fresh urine flow into a sterile container

Correct Answer: C

A urine culture from an indwelling catheter should be obtained from the sampling port using aseptic technique. Disconnecting the closed system or collecting from the drainage bag or spout increases contamination risk and can introduce microorganisms into the system.

1153. Which situation requires the practical nurse to recognize a break in infection-control practice and intervene?

- A.** A nursing assistant uses alcohol-based hand rub before checking a client's pulse
- B.** A nurse removes a soiled gown and then touches the front of the uniform before hand hygiene
- C.** A staff member dons gloves before cleaning up emesis
- D.** A phlebotomist discards a used needle in a sharps container

Correct Answer: B

Touching the uniform after removing a soiled gown and before hand hygiene can transfer contamination and reflects a break in infection control. The other actions are appropriate: hand rub before routine contact, gloves for emesis cleanup, and immediate sharps disposal are correct practices.

1154. A client with methicillin-resistant *Staphylococcus aureus* (MRSA) in a wound is on contact precautions. Which action by the practical nurse is best?

- A.** Keep a dedicated blood pressure cuff in the client's room
- B.** Wear an N95 respirator for dressing changes
- C.** Use sterile gloves for taking vital signs
- D.** Leave the isolation gown hanging outside the room for reuse later

Correct Answer: A

Using dedicated equipment such as a blood pressure cuff helps prevent transmission with contact precautions. An N95 is for airborne precautions, sterile gloves are not needed for routine vital signs, and reusing a gown left outside the room is unsafe and can spread contamination.

1155. The practical nurse prepares a sterile field for a central line dressing change. Which observation shows the field has been contaminated?

- A.** The nurse keeps the field above waist level
- B.** The outer 1-inch edge of the drape is avoided
- C.** The nurse turns away from the field to answer a question
- D.** A sterile package is opened so the first flap opens away from the nurse

Correct Answer: C

A sterile field must remain in constant view; turning away from it means the field is considered contaminated. Keeping the field above waist level, avoiding the outer edge, and opening the first flap away from the body are correct sterile technique practices.

1156. A practical nurse has completed care for a client in contact isolation. Which sequence is best before leaving the room?

- A.** Remove gown, remove gloves, perform hand hygiene outside the room
- B.** Remove gloves, perform hand hygiene, remove gown, exit room
- C.** Remove gloves, remove gown, perform hand hygiene
- D.** Perform hand hygiene over gloves, remove gown, remove gloves

Correct Answer: C

The best sequence is to remove gloves first because they are usually the most contaminated, then remove the gown, then perform hand hygiene. Performing hand hygiene over gloves is not appropriate, and delaying hand hygiene until after potentially contaminating steps is unsafe.

1157. A client has a productive cough, fever, and suspected bacterial meningitis. Which room placement and precaution combination is most appropriate based on likely transmission risk?

- A. Private room with droplet precautions
- B. Negative-pressure room with airborne precautions only
- C. Positive-pressure room with protective isolation
- D. Semi-private room with standard precautions only

Correct Answer: A

Suspected bacterial meningitis is managed with droplet precautions, ideally in a private room. A negative-pressure room is generally associated with airborne diseases such as tuberculosis. Positive-pressure rooms are used to protect highly immunocompromised clients, and standard precautions alone are not enough here.

1158. The practical nurse is teaching a client's family about hand hygiene. Which statement by a family member shows correct understanding?

- A. I only need to clean my hands if I touch blood or body fluids
- B. Wearing gloves means hand cleaning is unnecessary
- C. I should clean my hands before and after contact with the client
- D. Soap and water are never needed if sanitizer is available

Correct Answer: C

Hand hygiene should be performed before and after client contact. It is not limited to visible body fluid exposure, and gloves do not replace hand hygiene. Soap and water are still needed in some situations, so saying they are never needed is incorrect.

1159. After suctioning a tracheostomy, the practical nurse notes secretions on the bedside table and gloves. Which action best prevents further contamination?

- A. Set the suction catheter on a clean paper towel for possible reuse
- B. Remove gloves, clean the contaminated surface, and perform hand hygiene
- C. Leave the area for housekeeping because surface cleaning is not nursing care
- D. Cover the secretions with a dry towel until the next round

Correct Answer: B

The nurse should remove gloves safely, address the contaminated surface according to facility procedure, and perform hand hygiene to prevent spread. Leaving contaminated material in place or saving used suction equipment for reuse increases transmission risk. Environmental cleanliness is part of infection prevention, not something to ignore.

1160. A practical nurse is about to change the bed linens of a client who has a draining skin infection. Which action is most appropriate to reduce the spread of microorganisms while handling the soiled linens?

- A. Hold the linens away from the uniform and place them directly into the designated laundry bag
- B. Shake the linens gently before bagging them so loose debris can be removed
- C. Place the linens on the floor temporarily while gathering the rest of the supplies
- D. Sort the linens at the bedside to separate heavily soiled items from clean items

Correct Answer: A

Holding soiled linens away from the body and placing them directly into the appropriate bag helps prevent contamination of the nurse's clothing and the environment. Shaking linens can disperse microorganisms into the air, and placing or sorting them on the floor or at the bedside increases contamination risk.

1161. A practical nurse enters a client's room to assist with morning care. The client suddenly coughs, and respiratory secretions land on the nurse's unprotected eye area. What should the nurse do first?

- A. Finish providing care, then complete an incident report
- B. Rinse the exposed area promptly and report the exposure according to facility policy
- C. Wipe the eye with a dry tissue and continue working
- D. Ask the client whether the cough has happened before

Correct Answer: B

The priority after a splash exposure to mucous membranes is immediate irrigation of the exposed area, followed by prompt reporting and follow-up according to facility policy. Delaying action to finish care increases risk. A dry tissue does not adequately manage mucous membrane exposure, and asking questions before first aid is not the priority.

1162. The practical nurse is preparing to perform finger-stick blood glucose testing for several residents in a long-term care setting. Which practice is safest?

- A. Use a new lancet for each resident and clean the meter according to policy between uses
- B. Use the same lancet device on one shift as long as the needle is changed between residents
- C. Wear one pair of gloves for the entire medication cart pass if no blood is visible
- D. Place the used lancet on a clean paper towel until the next resident is ready

Correct Answer: A

A new lancet must be used for each resident, and shared equipment such as a glucose meter should be cleaned and disinfected according to policy between uses to prevent bloodborne pathogen transmission. Reusing a lancet device between residents is unsafe, even if the needle is changed. Gloves must be changed between residents, and used sharps should be discarded immediately in an approved sharps container.

1163. A nurse opens a sterile kit and notices that a corner of the sterile drape is hanging below waist level. Which interpretation is correct?

- A. Only the lower edge is contaminated, so the center of the field may still be used
- B. The field remains sterile if nothing has touched the hanging corner
- C. The sterile field is contaminated and should be set up again
- D. The nurse may fold the drape upward and continue with the procedure

Correct Answer: C

A sterile field is considered contaminated if any part extends below waist level because that area cannot be kept in constant view and is no longer considered sterile. Folding it back or trying to use only the center is not acceptable because the integrity of the field has been broken.

1164. The practical nurse is caring for a client who is on transmission-based precautions. A blood pressure cuff and stethoscope are kept in the room for this client. What is the main purpose of using this dedicated equipment?

- A. To reduce the need for documentation after vital signs are taken
- B. To prevent microorganisms from being carried to other clients on shared items
- C. To make the room setup more convenient for staff
- D. To avoid having to perform hand hygiene between tasks

Correct Answer: B

Dedicated equipment helps interrupt transmission by preventing shared items from carrying microorganisms from one client to another. It does not replace hand hygiene, and its purpose is infection prevention rather than convenience or reduced charting.

1165. A practical nurse must irrigate a client's wound. The provider's prescription states to use sterile solution. Which action requires the nurse to stop and obtain new supplies?

- A. The cap of the sterile bottle touches the bedside table after opening
- B. The nurse labels the date and time on a newly opened bottle

C. The irrigation syringe is placed on the inner sterile wrapper

D. The nurse pours a small amount of solution into a sterile container immediately before use

Correct Answer: A

If the sterile bottle cap touches an unsterile surface, contamination is assumed, and the nurse should replace the contaminated item. Labeling an opened bottle, placing supplies on a sterile wrapper if it is intended to serve as a sterile field, and pouring solution just before use are appropriate. The trap is assuming only the outside of the cap matters; in sterile technique, contamination of key items makes them unsafe to use.

1166. A client with a large draining wound is being transported to another unit for a procedure. Which action by the practical nurse is best before transport?

A. Leave the wound open to air so staff in the receiving department can assess it

B. Reinforce or change the dressing so drainage is contained during transport

C. Delay transport until the client has completed antibiotic therapy

D. Ask transport personnel to remove the old dressing after arrival

Correct Answer: B

Containing drainage before transport helps reduce environmental contamination and exposure to others. Leaving the wound open increases spread risk. Transport is not usually delayed until antibiotics are completed, and passing off dressing management to transport staff does not address the immediate infection-control concern.

1167. The practical nurse is preparing to insert an indwelling urinary catheter using sterile technique. After donning sterile gloves, the nurse accidentally brushes the gloved hand against the bed sheet. Which action is most appropriate?

A. Continue if the hand touched only a dry area of the sheet

B. Ask the client not to move, then proceed quickly

C. Apply an alcohol-based hand rub over the sterile gloves and continue

D. Discard the contaminated gloves and apply a new sterile pair before continuing

Correct Answer: D

Once a sterile glove touches a nonsterile surface, it is contaminated. The correct action is to replace the gloves before proceeding. Dryness of the sheet does not preserve sterility, and hand rub is not used to re-sterilize contaminated sterile gloves during a sterile procedure.

1168. A practical nurse is assigned to four tasks. Which task carries the highest risk for causing a healthcare-associated infection if aseptic technique is not followed carefully?

A. Changing a client's gown after breakfast

B. Emptying a bedside commode

C. Inserting an indwelling urinary catheter

D. Measuring oral intake from a meal tray

Correct Answer: C

Inserting an indwelling urinary catheter introduces a device directly into the urinary tract, so breaks in aseptic technique can lead to serious infection. The other tasks still require standard precautions, but they do not involve the same level of invasive infection risk.

1169. While setting up a sterile field for a dressing change, the practical nurse pours sterile solution into a sterile basin. Which technique is correct?

A. Hold the bottle 1 inch above the basin and pour without the bottle touching the basin

B. Rest the bottle tip against the inside of the basin to control the flow

C. Pour the unused solution back into the bottle after the procedure

D. Wave a hand over the basin to speed drying before use

Correct Answer: A

The bottle should be held slightly above the sterile container to avoid contaminating the bottle tip or the sterile field. Touching the basin with the bottle contaminates the sterile item, unused solution should not be returned to the bottle, and waving a hand over the field contaminates it.

1170. A client with an infected wound needs a dressing change. The practical nurse has already taken the client's temperature and blood pressure. To reduce cross-contamination, what should the nurse do next?

- A. Use the same gloves from the vital signs because the care is for the same client
- B. Perform hand hygiene and put on clean gloves before removing the old dressing
- C. Skip gloves because hand hygiene was performed before entering the room
- D. Place the vital-sign equipment on the bed during the dressing change

Correct Answer: B

After completing one task and before contact with contaminated material, the nurse should perform hand hygiene and apply clean gloves. Using the same gloves between tasks spreads microorganisms. Gloves are still needed for contact with wound drainage, and placing equipment on the bed during wound care increases contamination risk.

1171. The practical nurse observes an unlicensed assistive personnel member carrying used dressing supplies from one room to the soiled utility area in open hands, with a saturated gauze pad dripping onto the floor. Which action is best by the nurse?

- A. Tell the assistive personnel to walk faster so fewer surfaces are exposed
- B. Wait until the task is over, then mention that the supplies should have been covered
- C. Stop the transport of the supplies, contain the waste appropriately, and have the contaminated area cleaned
- D. Ignore the situation because the client has already received the dressing change

Correct Answer: C

The nurse should intervene immediately to stop ongoing contamination, ensure proper containment of contaminated materials, and address environmental cleaning. Waiting until later allows further spread. Speed does not reduce risk, and ignoring the situation fails to protect clients and staff.

1172. A practical nurse is preparing medications for a client with a localized eye infection. The client also needs assistance with perineal care and a dressing change to a clean surgical incision. Which task should the nurse perform first to reduce infection risk?

- A. Assist with perineal care
- B. Administer the eye medication
- C. Change the surgical dressing
- D. Collect the soiled linens from the bathroom floor

Correct Answer: B

Care should generally proceed from cleaner tasks to dirtier tasks and from less contaminated body areas to more contaminated ones. Administering eye medication is the cleanest of these options and should be done before perineal care or handling soiled linens. Although a clean surgical dressing change is also a clean task, eye medication requires very clean technique and is best done before more contamination-prone activities. The main trap is choosing perineal care or linen collection, which would increase contamination risk before cleaner procedures.

1173. A practical nurse enters a room and finds that a sterile instrument tray was opened 45 minutes ago for a procedure that was delayed. The tray has remained uncovered in a busy room near a sink, and several staff members have passed close by it. What is the best action?

- A. Use the tray because no one directly touched it
- B. Use only the instruments farthest from the sink
- C. Consider the tray contaminated and replace it
- D. Spray the tray with disinfectant and continue

Correct Answer: C

Sterility is event-related, not guaranteed simply by the passage of a certain amount of time. In this situation, the uncovered tray has been exposed in a high-traffic area near possible splash contamination, so it should be considered contaminated and replaced. The absence of visible touching does not ensure sterility, and disinfectant spray does not convert a contaminated sterile tray back into a sterile one.

1174. A practical nurse is teaching a newly hired coworker about infection prevention during specimen collection. Which statement by the coworker indicates correct understanding?

- A.** If the outside of the specimen container gets contaminated, I should wipe it and send it without relabeling or bagging
- B.** I should recap used needles carefully after specimen collection to avoid sharps container overuse
- C.** I should place the specimen in the appropriate biohazard transport bag after securing the lid and avoiding contamination of the outside
- D.** I can set the specimen on the medication cart briefly if I am planning to deliver it to the lab right away

Correct Answer: C

A specimen should be collected using clean technique appropriate to the test, sealed securely, and placed in the designated transport bag while keeping the outside clean. Contaminated exteriors require correction according to policy, not casual wiping and sending. Needles should not be recapped routinely because of sharps injury risk, and specimens should not be placed on medication carts.

1175. A practical nurse prepares to change a moist dressing on a client with a wound infection. The nurse has completed hand hygiene and gathered supplies. Which action is most appropriate to reduce the spread of microorganisms during the procedure?

- A.** Clean the wound first, then put on gloves to apply the new dressing
- B.** Wear the same gloves to remove the old dressing and apply the new dressing
- C.** Remove the soiled dressing, perform hand hygiene, and apply clean gloves before placing the new dressing
- D.** Place the new dressing supplies on the overbed table after removing the old dressing with bare hands

Correct Answer: C

The nurse should remove the contaminated dressing, perform hand hygiene, and don clean gloves before applying the new dressing. This limits transfer of organisms from the soiled dressing to the clean supplies or wound. Choice B is a common trap because gloves are worn, but contaminated gloves still spread microorganisms. Choices A and D break basic infection-control practice.

1176. A client on contact precautions has a tray of food delivered. The practical nurse needs to help the client open containers and position the tray. Which action is best?

- A.** Put on gloves before touching the tray and client items in the room
- B.** Ask dietary staff to return later so the tray stays outside the room
- C.** Wear sterile gloves while handling the tray
- D.** Use no gloves because the task is not invasive

Correct Answer: A

When assisting a client on contact precautions and touching items in the room, gloves are appropriate to reduce transmission from contaminated surfaces. Sterile gloves are unnecessary for routine care. Delaying the meal is not the best response, and skipping gloves ignores the risk of contact spread.

1177. The practical nurse is caring for a client with suspected influenza who is coughing frequently. Which instruction to the client best supports infection control?

- A.** Keep the door open for better airflow
- B.** Cover coughs and sneezes and perform hand hygiene after touching secretions
- C.** Limit fluids so fewer secretions are produced
- D.** Remove the mask when staff enter so communication is easier

Correct Answer: B

Respiratory hygiene and cough etiquette help reduce spread of infectious droplets. Covering coughs and cleaning hands after contact with secretions are key infection-control measures. The other options either increase transmission risk or are unrelated to prevention.

1178. A practical nurse is about to empty a client's urinary drainage bag. Which action is most important for preventing infection?

- A. Allow the drainage spout to touch the inside of the graduate for accuracy
- B. Place the drainage bag on the bed while measuring output
- C. Use a separate, clean container for the client's urine and avoid contaminating the drainage spout
- D. Disconnect the tubing from the catheter to improve drainage before emptying

Correct Answer: C

Using a clean measuring container and avoiding contamination of the drainage spout helps maintain a closed, cleaner system and reduces infection risk. Letting the spout touch the container can contaminate it. Placing the bag on the bed and disconnecting the system both increase the risk of introducing microorganisms.

1179. The practical nurse enters a room to provide care and notices a box of clean gloves sitting on the client's bed near used tissues. What should the nurse do?

- A. Use the gloves from the box because the gloves themselves are still clean inside
- B. Move the box to the bedside table and continue using it
- C. Discard the box and replace it with a new box stored in a clean area
- D. Wipe off the outer box with disinfectant and keep using it

Correct Answer: C

A glove box placed on a potentially contaminated surface can no longer be trusted as clean. Replacing it is the safest action. A common trap is assuming the gloves inside remain protected, but repeated hand contact at the opening can contaminate accessible gloves. Simply moving or wiping the box does not ensure safe use.

1180. A practical nurse removes gloves after providing perineal care and is immediately asked to adjust the client's oxygen tubing. Which action is best?

- A. Adjust the tubing now because gloves were already used during the dirty task
- B. Perform hand hygiene, then adjust the tubing

C. Put on a second pair of gloves over the first pair before adjusting the tubing

D. Use a dry paper towel to wipe the hands, then adjust the tubing

Correct Answer: B

Hand hygiene is required after glove removal and before touching clean equipment or another body site. Gloves are not a substitute for hand hygiene. Double-gloving over contaminated gloves does not prevent transfer, and wiping hands with a paper towel is inadequate.

1181. During a sterile dressing setup, the practical nurse reaches across the sterile field to move a package that was placed on the far side. What is the best interpretation of this action?

- A. It is acceptable if the nurse's sleeves do not touch the field
- B. It is acceptable because only direct contact contaminates a sterile field
- C. It contaminates the field because reaching across a sterile area can introduce microorganisms
- D. It is acceptable if the nurse is wearing clean gloves

Correct Answer: C

Reaching across a sterile field is unsafe because microorganisms can drop from the nurse's clothing or body into the field, even without direct contact. Clean gloves do not make the action sterile. The main error is breaking sterile-field boundaries, not just touching the field.

1182. A practical nurse is caring for two clients in the same semi-private room. One client has a draining wound infected with a multidrug-resistant organism and is on contact precautions. Which action is most appropriate to reduce transmission to the roommate?

- A. Carry used dressings across the shared room to the doorway trash can
- B. Perform hand hygiene and change gloves after wound care before touching shared room surfaces
- C. Leave the privacy curtain closed at all times and skip other changes
- D. Keep both clients' supplies on the same bedside cart for convenience

Correct Answer: B

The greatest immediate protection for the roommate is preventing spread from contaminated hands and gloves to shared surfaces. After wound care, the nurse should remove gloves, perform hand hygiene, and avoid contaminating room items. Closing a curtain does not replace standard infection-control practice, and shared supplies or carrying contaminated materials across the room increase risk.

1183. The practical nurse prepares to collect a midstream urine specimen from an alert client. Which instruction is most appropriate to help prevent contamination of the specimen?

- A.** Collect the first urine that comes out into the specimen cup
- B.** Touch the inside of the cup lid only if needed to steady it
- C.** Cleanse the area, begin voiding, then collect the urine without touching the inside of the cup
- D.** Set the open cup on the bathroom sink while preparing to void

Correct Answer: C

A clean-catch midstream specimen is obtained by cleansing, starting to void, and then collecting urine without contaminating the inside of the cup or lid. The first urine stream is more likely to flush surface organisms. Touching the inside of the container or setting it open on a sink risks contamination.

1184. A practical nurse is preparing a sterile field for a central line dressing change. After opening the outer wrapper, the nurse notices a small tear in the sterile package containing the dressing supplies. The inner contents do not appear wet or visibly soiled. What should the nurse do?

- A.** Use the supplies because the contents still look clean
- B.** Use the supplies only if the tear is along the edge of the package
- C.** Tape the tear shut and continue with the dressing change
- D.** Discard the package and obtain a new sterile set

Correct Answer: D

Any tear in a sterile package means sterility can no longer be assured, even if the contents look clean. Sterility depends on package integrity, not appearance alone. Choices A and B rely on visual inspection, which is not enough. Taping the package does not restore sterility.

1185. A practical nurse is assigned to assist with care for four clients. To limit transmission of microorganisms, which client should the nurse see last?

- A.** A client needing assistance with oral medications
- B.** A client scheduled for discharge teaching about diet
- C.** A client with a draining wound requiring dressing reinforcement
- D.** A client needing help to ambulate in the hallway

Correct Answer: C

When planning care, the nurse should generally move from the least contaminated activities to the most contaminated. A client with a draining wound has a higher risk of environmental and hand contamination, so that care is best done last if possible. The other tasks pose lower transmission risk.

1186. A practical nurse begins a sterile dressing change for a postoperative wound. While wearing sterile gloves, the nurse turns away to answer a question from another staff member, leaving the sterile field unattended for a moment. On turning back, the nurse is no longer sure whether the field stayed untouched. What is the best action?

- A.** Continue because no contamination was actually seen
- B.** Ask the other staff member whether the field looks clean
- C.** Consider the field contaminated and set up a new sterile field
- D.** Spray the field with antiseptic solution and proceed

Correct Answer: C

If sterility is uncertain, the field must be considered contaminated. Sterile technique depends on maintaining constant control and awareness of the field. A common error is continuing because contamination was not witnessed, but uncertainty itself is enough to require replacement. Antiseptic spray does not make a field sterile.

1187. A practical nurse is assisting with tracheostomy care for a client who has thick secretions. During the procedure, the nurse uses clean gloves to remove the soiled inner cannula and then plans to open a sterile replacement package. Which action best maintains infection control?

- A. Keep the same gloves on to avoid interrupting the procedure
- B. Remove the gloves, perform hand hygiene, and handle the sterile package with clean technique before applying new gloves as needed
- C. Wipe the gloved hands with sterile saline before touching the new package
- D. Ask the client to hold the package open to save time

Correct Answer: B

After handling contaminated equipment, the nurse should remove gloves and perform hand hygiene before touching clean or sterile supplies. This breaks the chain of contamination. Keeping the same gloves on or wiping them with saline does not make them clean. Having the client assist with opening sterile supplies adds contamination risk.

PPE

1188. A nurse is preparing to empty a client's indwelling urinary drainage bag. Which personal protective equipment is most appropriate to wear for this task?

- A. Sterile gloves only
- B. Clean gloves only
- C. Gown, N95 respirator, and sterile gloves
- D. Surgical mask and face shield only

Correct Answer: B

Clean gloves are appropriate because the nurse may come into contact with urine and contaminated equipment during drainage bag emptying. Sterile gloves are not needed for this clean procedure. A mask, face shield, gown, or respirator would be added only if splashing or another specific exposure risk is expected.

1189. A client with suspected pulmonary tuberculosis is admitted to the unit. Which PPE should the nurse use when entering the client's room?

- A. Surgical mask
- B. Face shield
- C. N95 respirator
- D. Clean gloves

Correct Answer: C

An N95 respirator is used for airborne precautions, which are indicated for suspected pulmonary tuberculosis. A surgical mask does not provide the same respiratory protection for airborne particles. Gloves and face protection may be needed for specific tasks, but the key PPE for room entry is the respirator.

1190. The nurse is caring for a client on contact precautions for infectious diarrhea. Which action is most appropriate when removing PPE before leaving the room?

- A. Remove gloves first because they are usually the most contaminated
- B. Remove the respirator before touching the gown
- C. Remove eye protection before removing gloves
- D. Leave gloves on until after touching the hallway computer

Correct Answer: A

Gloves are generally removed first because they are often the most heavily contaminated item. This helps reduce spread of organisms to other PPE and surfaces. Touching the hallway computer with contaminated gloves would increase transmission risk. Respirator removal is not the priority in contact precautions unless one is being used for another reason.

1191. A nurse is about to irrigate a wound that has copious drainage. Which combination of PPE is most appropriate?

- A. Clean gloves and gown
- B. Sterile gloves only
- C. Gloves, gown, and face shield
- D. Surgical mask only

Correct Answer: C

Gloves, gown, and face shield are appropriate because copious drainage and irrigation create a risk of splashes to the skin, clothing, and mucous membranes. Sterile gloves alone do not protect the nurse's face or clothing. Clean gloves and gown would still leave the eyes and face unprotected.

1192. A nurse finishes providing care to a client and removes gloves. Which action should the nurse take next?

- A. Document the care provided
- B. Perform hand hygiene
- C. Apply a new pair of gloves
- D. Adjust eyeglasses with bare hands

Correct Answer: B

Hand hygiene should be performed after glove removal because gloves can have microscopic defects and hands may become contaminated during removal. Documenting care or touching the face before hand hygiene can spread microorganisms. Putting on new gloves without cleaning the hands first is also unsafe.

1193. A nurse enters an isolation room wearing gown, gloves, mask, and goggles. After completing care, which item should be removed last if it was worn for respiratory protection?

- A. Gloves
- B. Gown
- C. Goggles
- D. Mask

Correct Answer: D

The mask should be removed last when it is part of respiratory protection during PPE removal. This helps keep respiratory protection in place while the nurse removes other potentially contaminated equipment. Removing the mask too early increases exposure risk during the remainder of the doffing process.

1194. A newly licensed nurse says, "I wore gloves while changing the dressing, so I don't need to clean my hands afterward." Which response by the charge nurse is best?

- A. Hand hygiene is still needed after gloves are removed
- B. That is correct if the gloves stayed intact
- C. Hand hygiene is only needed before sterile procedures
- D. Gloves replace hand hygiene when drainage is minimal

Correct Answer: A

Hand hygiene is still required after glove removal because contamination can occur from tiny glove defects or during removal. The common trap is assuming gloves fully replace hand hygiene; they do not. Gloves are an added barrier, not a substitute for cleaning the hands.

1195. A nurse must suction a client with a tracheostomy who has thick secretions. Which PPE is the priority to add because of the highest anticipated exposure risk during the procedure?

- A. Shoe covers
- B. Face protection
- C. Hair cover
- D. Lead apron

Correct Answer: B

Face protection is a priority during tracheostomy suctioning because secretions may spray toward the nurse's eyes, nose, or mouth. Shoe covers and hair covers are not the main protection needed for this expected exposure. A lead apron is unrelated to infection control in this scenario.

1196. The nurse prepares to enter the room of a client on contact precautions for a draining wound. The nurse needs to adjust an IV pump located just inside the room and will not touch the client. Which action is best?

- A. Enter without PPE because the client will not be touched
- B. Wear the PPE required for contact precautions before entry
- C. Ask the client to adjust the pump instead
- D. Wear only a surgical mask because equipment is involved

Correct Answer: B

The nurse should wear the PPE required for contact precautions before entering the room because environmental surfaces and equipment may be contaminated even if the client is not touched directly. The trap is focusing only on direct contact with the client and ignoring contact with the room environment.

1197. A nurse removes a contaminated gown by untying it and pulling it away from the neck and shoulders. What is the most important technique to prevent self-contamination?

- A. Shake the gown out before discarding it
- B. Roll the contaminated outside inward while removing it
- C. Place the gown on the bedside table until all PPE is off
- D. Touch the front of the gown as little as possible, then fold it outward

Correct Answer: B

The contaminated outside of the gown should be turned inward and rolled into a bundle during removal to contain contamination. Shaking the gown can disperse microorganisms. Leaving it on a bedside table contaminates the environment. Folding it outward exposes the contaminated surface.

1198. A client with influenza is coughing frequently during care. Which PPE should the nurse use for routine care in addition to gloves if contact with secretions is likely?

- A. N95 respirator only
- B. Surgical mask and eye protection
- C. Sterile gloves and shoe covers
- D. PAPR and double gloves

Correct Answer: B

For a coughing client with likely respiratory secretion exposure, a surgical mask and eye protection help protect the nose, mouth, and eyes from droplets. An N95 respirator is typically reserved for airborne hazards rather than routine droplet protection. Sterile gloves, shoe covers, and double gloves do not address the main exposure risk.

1199. While removing gloves, a nurse touches the contaminated outside of one glove with bare fingers from the other hand. What is the nurse's priority action?

- A. Continue removing PPE and perform hand hygiene at the end
- B. Obtain a new pair of gloves immediately and start over
- C. Perform hand hygiene as soon as the gloves are off
- D. Wipe the fingers on the gown before touching anything else

Correct Answer: C

If the nurse contaminates bare fingers during glove removal, the priority is to complete glove removal and perform hand hygiene as soon as possible. Wiping fingers on the gown increases contamination. Waiting unnecessarily or focusing first on new gloves delays the key safety step.

1200. A nurse is preparing PPE for four tasks. For which task are sterile gloves specifically indicated?

- A. Changing a bed with soiled linens
- B. Performing urinary catheter insertion
- C. Transporting a client in a wheelchair
- D. Taking a routine blood pressure

Correct Answer: B

Urinary catheter insertion is an aseptic procedure, so sterile gloves are indicated. The other tasks may require clean gloves depending on exposure risk, but they do not specifically require sterile gloves. A common trap is overusing sterile gloves for tasks that only need standard precautions.

1201. A nurse caring for a client on droplet precautions needs to administer oral medications. The client suddenly begins coughing forcefully. The nurse is wearing gloves but no facial PPE. Which action is best?

- A. Step back, leave the room, and apply appropriate facial PPE before continuing care
- B. Quickly give the medications before the coughing worsens
- C. Ask the client to turn away, then continue without interruption
- D. Finish the task and put on a mask afterward

Correct Answer: A

The nurse should stop, leave if needed, and apply the appropriate facial PPE before continuing because the exposure risk has changed and droplets are now an immediate hazard. The trap is rushing to finish a task already in progress. PPE decisions must be updated when the situation changes.

1202. A nurse enters a room to assist with a procedure and discovers there are no goggles available, even though splashing is expected. Which action is most appropriate?

- A. Proceed carefully while keeping the face turned away
- B. Use personal prescription glasses instead
- C. Delay participation until appropriate eye protection is obtained
- D. Wear an extra pair of gloves for added safety

Correct Answer: C

If splash exposure is expected, the nurse should obtain appropriate eye protection before participating. Turning the face away or wearing regular prescription glasses does not provide reliable mucous membrane protection. Extra gloves do not protect the eyes.

1203. A nurse is preparing to insert an indwelling urinary catheter using sterile technique. Which personal protective equipment is required specifically to maintain sterility of the procedure?

- A. Clean examination gloves
- B. Sterile gloves
- C. A surgical mask
- D. A fluid-resistant gown

Correct Answer: B

Sterile gloves are required because the nurse must maintain a sterile field during urinary catheter insertion. Clean gloves are not sufficient for sterile procedures. A mask or gown may be needed in some situations, but they do not replace the need for sterile gloves when asepsis is essential.

1204. A nurse is reinforcing teaching for an assistive personnel member about glove use. Which statement by the assistive personnel indicates correct understanding?

- A. I can wear the same gloves from one task to another if I stay with the same client.
- B. I should put on gloves before touching the hallway computer outside the client's room.
- C. I need to change gloves if moving from a contaminated body site to a clean body site on the same client.
- D. If my gloves look clean, hand hygiene is not needed after I remove them.

Correct Answer: C

Gloves must be changed when moving from a contaminated site to a clean site on the same client to prevent transmission of microorganisms. Wearing the same gloves for multiple tasks spreads contamination. Gloves should not be worn for touching clean environmental surfaces unnecessarily, and hand hygiene is still required after glove removal.

1205. A nurse is caring for a client with a draining skin infection requiring contact precautions. The nurse has finished care and is still inside the room. Which action is best for the nurse to take with the gown?

- A. Remove the gown before touching the door handle
- B. Keep the gown on until reaching the hallway trash can
- C. Remove the gown after charting at the bedside computer
- D. Untie the gown and leave it hanging in the room for possible reuse

Correct Answer: A

The gown should be removed before the nurse touches clean surfaces such as the door handle, helping prevent environmental contamination outside the room. Keeping it on into the hallway spreads organisms. Charting while wearing contaminated PPE risks contaminating equipment, and disposable gowns are not left for reuse.

1206. A nurse is about to obtain a venous blood sample from a client with no isolation precautions. The client has fragile veins, and blood leakage onto the nurse's hands is possible. Which PPE is most appropriate?

- A. Gloves only
- B. Gown and gloves
- C. Mask only
- D. Gloves, gown, and goggles

Correct Answer: A

Gloves are appropriate because blood exposure to the hands is anticipated. A gown and eye protection are added when splashing or spraying is expected, not simply because blood is involved. The key is matching PPE to the expected exposure risk rather than overusing equipment.

1207. A nurse is assigned four activities. For which activity is eye protection most clearly indicated?

- A. Taking an oral temperature
- B. Repositioning a sleeping client in bed
- C. Disconnecting a chest tube drainage system filled with fluid
- D. Palpating peripheral pulses during assessment

Correct Answer: C

Eye protection is indicated when splashes or sprays of body fluids may occur. Disconnecting a chest tube drainage system creates a clear splash risk. The other activities do not typically create exposure to sprays reaching the eyes.

1208. A nurse wearing a mask enters a client's room for droplet precautions. During care, the mask becomes damp from the nurse's breathing. Which action is most appropriate?

- A. Keep wearing the same mask until care is completed
- B. Pull the mask below the chin between tasks
- C. Replace the damp mask with a new one
- D. Touch only the front of the mask to readjust it tightly

Correct Answer: C

A damp mask should be replaced because moisture reduces its effectiveness and comfort. Pulling it below the chin contaminates it, and touching the front increases contamination risk. The safest choice is to replace it properly with a clean mask.

1209. A nurse is caring for a client with copious emesis from a gastrointestinal illness. The client begins retching again as the nurse brings a basin closer. Which PPE selection is most appropriate before staying at the bedside?

- A. Gloves only
- B. Mask only
- C. Gloves and gown
- D. Gloves, gown, mask, and eye protection

Correct Answer: D

Copious emesis can splash onto the nurse's skin, clothing, mouth, nose, and eyes. Gloves and a gown protect hands and clothing, while a mask and eye protection protect the face from splashes. Choosing only part of the needed PPE leaves a significant exposure route unprotected.

1210. A nurse removes a gown and notices that one sleeve brushed against the nurse's uniform top. Which action is the priority?

- A. Continue working and change the uniform at the end of the shift
- B. Apply alcohol-based hand rub to the uniform area
- C. Assess the area as contaminated and change the clothing as soon as possible per facility procedure
- D. Cover the area with another disposable gown

Correct Answer: C

If contaminated PPE contacts the uniform, the clothing should be treated as contaminated and managed promptly according to facility procedure. Hand rub is for hands, not clothing. Covering the area does not remove contamination, and waiting until the end of the shift increases the risk of spread.

1211. A nurse is preparing to transport a client on droplet precautions for a diagnostic test. Which action best reduces transmission risk during transport?

- A. Have the client wear a mask if tolerated
- B. Place a gown on the client before leaving the room
- C. Ask the client to perform hand hygiene only after arriving at the test area
- D. Delay transport until all hallway traffic has stopped

Correct Answer: A

For droplet precautions, placing a mask on the client during transport helps reduce spread of respiratory droplets. A gown on the client is not the main transmission-control measure here. Hand hygiene is useful but does not replace source control, and delaying transport for hallway traffic is not the best or standard primary action.

1212. A nurse is removing eye protection after assisting with a procedure that produced blood splatter. Which technique is correct?

- A. Lift the goggles away using the headband or earpieces
- B. Wipe the front of the goggles first, then remove them by the lens area
- C. Remove the gloves, then push the goggles up onto the forehead
- D. Grasp the front center of the face shield and pull forward

Correct Answer: A

Eye protection should be removed by handling the clean straps, earpieces, or headband, not the contaminated front surface. Pushing goggles onto the forehead or touching the front increases the chance of self-contamination. The key principle is avoiding contact with the exposed outer surface.

1213. A nurse is caring for a client with a large draining wound on the abdomen. The prescribed treatment is a simple blood pressure check followed by a full dressing change. To reduce unnecessary PPE use while still protecting against exposure, which plan is best?

- A. Wear gloves for the blood pressure check and keep the same gloves on for the dressing change

- B. Delay the blood pressure check until after the dressing change so one set of gloves can be used

- C. Perform the blood pressure check with clean hands if no contact with drainage is expected, then don appropriate PPE for the dressing change

- D. Wear sterile gloves and a gown for both tasks from the start

Correct Answer: C

PPE should match the task and anticipated exposure. If the blood pressure check does not involve contact with drainage or body fluids, it can be done without gloves under standard precautions, followed by appropriate PPE for the dressing change. Keeping the same gloves across tasks spreads contamination, and overusing sterile gloves or combining tasks to save PPE is not best practice.

1214. A nurse is teaching a client prescribed home wound care about glove use. Which client statement shows correct understanding?

- A. I should wash and reuse disposable gloves if they do not tear.
- B. I need gloves only when touching the clean outer bandage.
- C. I should put on a new pair of gloves each time I provide wound care.
- D. If I wear gloves, I do not need to clean my hands before starting.

Correct Answer: C

Disposable gloves are single-use items and should be replaced each time wound care is performed. Reusing them is unsafe. Hand hygiene is still needed, and glove use is based on contact with blood, drainage, or the wound itself, not just whether the bandage looks clean.

1215. A nurse in full PPE for isolation care is called to silence a beeping infusion pump in the hallway outside the room. What is the nurse's best action?

- A. Touch the hallway pump with gloved hands because the task will take only a few seconds
- B. Remove contaminated PPE as appropriate, perform hand hygiene, and then handle the pump
- C. Ask the client to wait while the nurse handles the hallway pump without changing PPE

D. Use an alcohol wipe on the gloves, then adjust the hallway pump

Correct Answer: B

Contaminated PPE should not be used to touch clean equipment outside the isolation area. The nurse should remove PPE as appropriate and perform hand hygiene before handling the hallway pump. Wiping gloves does not make them a safe substitute for removal and hand hygiene.

1216. A nurse prepares to assist with a sterile bedside procedure. The nurse has a small cut on one finger covered with an intact bandage. Which PPE choice is most appropriate for hand protection during the procedure?

- A.** No gloves because the cut is covered
- B.** Clean gloves over the bandage
- C.** Sterile gloves over the bandage
- D.** Double clean gloves without a bandage

Correct Answer: C

Because the procedure is sterile, the nurse must wear sterile gloves. The intact bandage helps cover the cut, but it does not replace required sterile PPE. Clean gloves are inadequate for a sterile procedure, and double clean gloves still do not create sterile technique.

1217. A nurse caring for a client with suspected meningococcal infection must obtain a throat specimen. Which PPE is most appropriate?

- A.** Gloves and a surgical mask
- B.** Sterile gloves and shoe covers
- C.** Gown only
- D.** N95 respirator only

Correct Answer: A

A throat specimen can expose the nurse to respiratory droplets and oral secretions, so gloves and a surgical mask are appropriate. An N95 respirator is used for airborne concerns, not routine droplet protection. Shoe covers are unnecessary here, and a gown alone does not protect the nurse's hands or respiratory mucosa.

1218. A nurse is preparing to suction a client's tracheostomy. Which personal protective equipment is most appropriate for this procedure?

- A.** Gloves only

B. Gloves, gown, mask, and eye protection

C. Gown and sterile gloves only

D. Mask only

Correct Answer: B

Tracheostomy suctioning can generate secretions and splashes, so the nurse should protect hands, clothing, nose and mouth, and eyes. Gloves, gown, mask, and eye protection provide the best protection from exposure to respiratory secretions. Gloves only do not protect the face or clothing, and sterile gloves alone do not address splash risk.

1219. A nurse enters a room to provide care for a client on contact precautions. After putting on the gown, which action should the nurse take next to ensure the PPE is worn correctly?

- A.** Tie the gown securely at the neck and waist
- B.** Pull the gown sleeves above the wrists
- C.** Apply clean gloves before fastening the gown
- D.** Leave the back of the gown open for easier removal

Correct Answer: A

A gown should be secured at the neck and waist so clothing and skin are covered during care. Sleeves should extend to the wrists, not above them. Gloves are applied after the gown is in place, and leaving the gown open reduces protection.

1220. A nurse has just completed perineal care for one client and is wearing gloves. The client's call light falls onto the floor. What action should the nurse take first?

- A.** Pick up the call light with the gloved hand, then remove the gloves
- B.** Remove the gloves and perform hand hygiene before touching the call light
- C.** Ask the client to pick up the call light
- D.** Wipe the gloves with disinfectant and pick up the call light

Correct Answer: B

Gloves used for perineal care are contaminated. The nurse should remove them and perform hand hygiene before touching a clean environmental item. Touching the call light with contaminated gloves can spread organisms. Wiping gloves is not a substitute for changing them.

1221. A nurse is assisting with irrigation of an infected wound. The provider's technique may cause fluid to splash. Which item of PPE is most important to add if the nurse is already wearing gloves and a gown?

- A. Hair cover
- B. Shoe covers
- C. Eye protection
- D. Sterile sleeves

Correct Answer: C

When splash or spray is possible, the eyes need protection. Eye protection helps prevent exposure of the mucous membranes to wound drainage or irrigation fluid. Hair covers and shoe covers are not the priority in this situation, and sterile sleeves are not standard PPE for this risk.

1222. A nurse is caring for a client on airborne precautions. Which action by the nurse requires correction?

- A. Putting on the respirator before entering the room
- B. Removing the respirator after leaving the room
- C. Touching the front of the respirator while taking it off
- D. Using hand hygiene after removing the respirator

Correct Answer: C

The front of a respirator is considered contaminated and should not be touched during removal. The nurse should handle it by the straps or ties. Putting it on before entry, removing it after exiting, and performing hand hygiene afterward are appropriate actions.

1223. A nurse is preparing morning care for a client on contact precautions. The nurse plans to empty the urinary drainage bag, reposition the client in bed, and then change a heavily soiled dressing. To avoid spreading organisms, when should the nurse change gloves?

- A. Only after all three tasks are completed
- B. Between the urinary drainage task and contact with clean bed linens or the dressing supplies
- C. Only if the gloves tear during care
- D. There is no need to change gloves because the client is on contact precautions

Correct Answer: B

Gloves should be changed during care if moving from a contaminated task to a cleaner task or before handling clean supplies. Emptying a drainage bag is a contaminated task, so the nurse should change gloves before touching linens, the client for cleaner tasks, or dressing materials. Contact precautions do not remove the need to prevent cross-contamination within the same room.

1224. A nurse leaves the room of a client on isolation precautions wearing gloves and notices a medication scanner attached to the WOW outside the room. The nurse needs the scanner immediately for the next client. Which action is best?

- A. Use the scanner with the gloved hand because the next client also needs care
- B. Set the scanner on a clean paper towel, then remove gloves
- C. Remove the gloves, perform hand hygiene, and then handle the scanner
- D. Ask another staff member to wipe the scanner while the nurse keeps the gloves on

Correct Answer: C

The scanner is a clean item outside the room. The nurse should not touch it with contaminated gloves. The safest action is to remove gloves, perform hand hygiene, and then handle the equipment. Setting the item down first does not address glove contamination if the scanner has already been touched.

1225. A nurse is assigned to care for a client with a persistent cough who is on droplet precautions. The nurse will administer oral medication and assess lung sounds. Which PPE is required at a minimum for entering the room?

- A. Gloves only
- B. Mask
- C. Mask and sterile gloves
- D. Gown, sterile gloves, and shoe covers

Correct Answer: B

For droplet precautions, a mask is required when entering the room to protect the nose and mouth from respiratory droplets. Gloves depend on anticipated contact with secretions or body fluids, but a mask is the minimum required from the options given. Sterile gloves and shoe covers are not indicated for routine medication administration and auscultation.

1226. A nurse is caring for a client on contact precautions for *Clostridioides difficile*. The nurse removes gloves after providing care. What should the nurse do next?

- A. Adjust eyeglasses before leaving the room
- B. Perform hand hygiene
- C. Document the care on the room computer
- D. Pull the privacy curtain closed

Correct Answer: B

After glove removal, hand hygiene is the next essential step because gloves are not a substitute for hand cleaning and hands may become contaminated during removal. Touching eyeglasses, curtains, or equipment before hand hygiene can spread organisms.

1227. A nurse is preparing to assist with a vaginal delivery. The nurse expects a large amount of blood and body fluid exposure. Which PPE selection is most appropriate?

- A. Clean gloves only
- B. Sterile gloves and mask only
- C. Gloves, fluid-resistant gown, mask, and eye protection
- D. Hair cover and shoe covers only

Correct Answer: C

A vaginal delivery presents a high likelihood of exposure to blood and body fluids. The nurse should protect hands, clothing, face, and eyes with gloves, a fluid-resistant gown, a mask, and eye protection. Gloves or mask alone do not provide enough coverage for the expected exposure.

1228. A nurse is removing PPE after caring for a client with contact precautions. Which sequence is most appropriate to reduce self-contamination?

- A. Mask, gloves, hand hygiene, gown
- B. Gloves, gown, hand hygiene
- C. Gown, hand hygiene, gloves
- D. Gloves, hand hygiene, gown

Correct Answer: B

For contact precautions when gloves and gown are used, contaminated gloves are typically removed first, then the gown, followed by hand hygiene. This sequence helps prevent transfer of organisms to skin or clean surfaces. Performing hand hygiene before removing the gown leaves contaminated PPE in place, and removing the gown before gloves increases the chance of contaminating the hands.

1229. A nurse is caring for a client on airborne precautions and must bring medications into the room. After entering, the client begins coughing forcefully and asks for tissues from the bedside drawer. The nurse is wearing the required respirator but no gloves because no direct contact was planned. What should the nurse do?

- A. Open the drawer and hand the tissues to the client without adding PPE
- B. Leave the room to get gloves because gloves are required for all airborne-precaution entries
- C. Put on gloves before handling items likely to become contaminated with respiratory secretions
- D. Ask the client to wait until the coughing stops before offering tissues

Correct Answer: C

Airborne precautions require respiratory protection, but gloves are added based on anticipated contact with secretions or contaminated items. Once the client's active coughing creates likely contact with respiratory secretions, the nurse should add gloves before handling tissues or nearby contaminated surfaces. Gloves are not automatically required for every airborne room entry, but they are appropriate when exposure risk changes.

1230. A nurse is preparing to remove a soiled gown and gloves after wound care. The gloves are visibly contaminated, and the nurse notices a small tear near one thumb. Which action is best to limit self-contamination during removal?

- A.** Remove the gown first to avoid touching the torn glove
- B.** Pinch the outside of one glove near the wrist and remove the gloves carefully before removing the gown
- C.** Sanitize the gloved hands, then remove both gloves at the same time
- D.** Keep the gloves on while documenting, then remove all PPE together

Correct Answer: B

Even with a small tear, the nurse should remove the contaminated gloves carefully using proper glove-removal technique, then remove the gown. Gloves are the most contaminated item and should come off first in this situation. Sanitizing visibly soiled or damaged gloves is not an appropriate substitute, and delaying removal increases contamination risk.

Asepsis

1231. A practical nurse is preparing a sterile field for a bedside sterile dressing change. Which action best maintains surgical asepsis?

- A.** Keeping the sterile field at waist level while gathering supplies
- B.** Reaching across the center of the sterile field to move gauze
- C.** Turning away briefly after opening the sterile setup
- D.** Adding a sterile item to the field without touching the field

Correct Answer: D

A sterile item may be added to a sterile field if it is placed without contaminating the field. A sterile field should remain above waist level, in constant view, and should never be reached across. The common trap is assuming a brief turn away is harmless, but an unattended sterile field is considered contaminated.

1232. While pouring sterile normal saline into a sterile basin, the nurse accidentally splashes fluid onto the drape around the basin. What is the best action?

- A.** Continue, because sterile saline does not contaminate the field
- B.** Blot the wet area with sterile gauze and proceed
- C.** Consider the field contaminated and set up a new sterile field
- D.** Move the basin to a dry corner of the same field

Correct Answer: C

Moisture can carry microorganisms from a nonsterile surface to a sterile one by strike-through, so a wet sterile field is contaminated. The safest response is to discard the setup and prepare a new field. The likely trap is thinking the fluid is sterile, so the field remains sterile, but the issue is wick contamination through the drape.

1233. A nurse is opening a sterile package containing forceps. Which technique is correct?

- A.** Open the flap farthest away first, then the side flaps, and the nearest flap last
- B.** Open the nearest flap first so the package stays close to the body
- C.** Slide fingers under the inner surface of each flap to keep it fully open
- D.** Hold the package over the sterile field and shake the item out from chest height

Correct Answer: A

The package is opened with the far flap first, then the side flaps, and the near flap last to avoid reaching over the sterile contents. Touching the inside of flaps contaminates them, and shaking an item from above risks contamination or loss of control. The near-flap-first option is a common technique error.

1234. The nurse is preparing to don sterile gloves using the open-gloving method. Which step is most important to prevent contamination?

- A. Pull the first glove on by grasping the outside of the cuff with the bare hand
- B. Keep the gloved hand above waist level and touch only the inside cuff of the second glove
- C. Adjust both gloves by pinching the fingers after both are on
- D. Rest the gloved hand briefly on the overbed table for balance

Correct Answer: B

With open gloving, the bare hand touches only the inside of the folded cuff, and the gloved hand touches only sterile surfaces. Keeping gloved hands above waist level also helps maintain sterility. Grasping the outside of the glove with a bare hand contaminates it, and resting a gloved hand on a nonsterile surface contaminates the glove.

1235. A nurse is changing a postoperative dressing using sterile technique. The client coughs suddenly, and the nurse notices droplets land on the sterile field. What should the nurse do first?

- A. Proceed quickly before further contamination occurs
- B. Cover the area with a sterile towel and continue
- C. Discard the contaminated field and obtain new sterile supplies
- D. Wipe the droplets away with sterile gauze

Correct Answer: C

Droplets from coughing contaminate the sterile field. Once contamination occurs, the field is no longer sterile and must be replaced. Trying to cover or wipe the contamination does not restore sterility. The key principle is that contamination requires starting over, not patching the field.

1236. A nurse opens a sterile drape on a bedside table. Which part of the drape should be considered contaminated after it is placed?

- A. Only the center area directly under the sterile supplies
- B. The entire top surface of the drape
- C. The 1-inch outer edge of the drape

D. Any area that remains dry and visible

Correct Answer: C

The outer 1-inch edge of a sterile drape is considered contaminated because it is difficult to handle without touching nonsterile surfaces. The rest of the top surface is considered sterile if the field has been set up correctly. A frequent trap is assuming the whole top is contaminated after placement, which would make the drape unusable.

1237. The nurse needs to add sterile solution to a sterile basin during a procedure. Which action is correct?

- A. Hold the bottle 1 to 2 inches above the basin and pour without the bottle touching the basin
- B. Let the bottle lip rest on the basin edge to steady the pour
- C. Pour a small amount back into the original bottle if too much was dispensed
- D. Reach over the basin with the label facing down so the stream is easy to see

Correct Answer: A

Sterile solution should be poured from a short distance above the sterile container without touching the container, which helps avoid contamination. The bottle should not contact the basin, and fluid should never be poured back into the bottle. The label should be positioned to avoid wetting it, not simply to improve visibility.

1238. A nurse sets up a sterile field for urinary catheter insertion. Before the procedure begins, the nurse notices the package expiration date on one sterile item passed last month. What is the best action?

- A. Use the item if the wrapper is still sealed and dry
- B. Use the item only for the noncritical parts of the procedure
- C. Replace the expired item with a sterile item that is within date
- D. Open the item and inspect it for visible contamination

Correct Answer: C

Sterile supplies should be considered sterile only if packaging integrity is intact and the item is within its expiration date if one is provided. An expired item should be replaced. The trap is relying on the package appearance alone, but sterility cannot be confirmed by visual inspection after expiration.

1239. During sterile wound irrigation, the nurse accidentally touches the sterile glove to the bed linen. What is the most appropriate nursing action?

- A.** Continue the procedure because only a small area of the glove touched the linen
- B.** Rinse the glove with sterile saline and continue
- C.** Replace the contaminated glove before proceeding
- D.** Ask the client not to move and finish quickly

Correct Answer: C

Bed linen is nonsterile, so contact with it contaminates the glove. The correct response is to replace the contaminated glove before continuing. Sterile saline does not re-sterilize a glove, and finishing quickly does not reduce contamination risk. The likely trap is minimizing the contamination because the contact was brief.

1240. A practical nurse is assisting with a sterile dressing change in a client's home. The client has a fan blowing directly across the bedside table where the sterile field will be placed. Which action is best?

- A.** Proceed because the moving air only affects comfort, not sterility
- B.** Turn off or redirect the fan before setting up the sterile field
- C.** Use extra sterile drapes to protect the field from airflow
- D.** Place the sterile field lower, near the side of the bed, to reduce air exposure

Correct Answer: B

Air currents can carry dust and microorganisms toward a sterile field. The nurse should reduce contamination risk by turning off or redirecting the fan before setup. Lowering the field would worsen technique because sterile fields must stay above waist level and within view. Extra drapes do not eliminate contamination from moving air.

1241. The nurse is caring for a client with a new central venous catheter dressing. Which observation during the dressing change indicates a break in sterile technique?

- A.** The nurse uses a mask during the procedure
- B.** The nurse cleanses from the insertion site outward
- C.** The nurse places sterile gauze on the field after reaching across it
- D.** The nurse keeps sterile supplies above waist level

Correct Answer: C

Reaching across a sterile field contaminates it because the arm and clothing pass over sterile items. Using a mask, cleansing from the insertion site outward, and keeping supplies above waist level are appropriate aseptic practices. The trap here is choosing a less familiar but actually correct action rather than the clear contamination event.

1242. A nurse is preparing a sterile field for tracheostomy care. After opening the sterile kit, the nurse realizes one needed sterile swab is missing. Which action best preserves asepsis when adding the extra item?

- A.** Drop the sterile swab package onto the center of the field and open it there
- B.** Open the sterile swab package away from the field and drop the swab gently onto the field without crossing over it
- C.** Hand the package to the client and ask the client to place it on the field
- D.** Place the unopened package on the edge of the field for later use

Correct Answer: B

An additional sterile item should be opened without contaminating the field and dropped onto it from an appropriate height without reaching across the field. Placing nonsterile packaging on the field contaminates it, whether opened or unopened. Having the client place it would also not preserve sterility.

1243. A student nurse prepares a sterile field and says, 'I can use this as long as I don't touch it.' Which response by the practical nurse is most accurate?

- A. That is correct if the field stays dry
- B. That is correct if the room door stays closed
- C. That is incorrect because a sterile field must also remain in view and be protected from contamination
- D. That is incorrect because sterile fields are used only in procedure rooms

Correct Answer: C

A sterile field must not only remain untouched by nonsterile objects, but also remain in constant view, above waist level, dry, and protected from contamination such as splashes or airflow. A closed door alone does not ensure sterility, and sterile fields may be used at the bedside. The statement is incomplete because 'not touching it' is only one part of aseptic practice.

1244. The nurse is setting up for sterile suctioning of a tracheostomy. The package containing the sterile catheter is torn along one corner, but the catheter itself does not appear dirty. What should the nurse do?

- A. Use the catheter because the inner contents are not visibly soiled
- B. Use the catheter if it will be used immediately
- C. Discard the catheter and obtain a new sterile catheter
- D. Rewrap the catheter in a sterile towel until needed

Correct Answer: C

A torn package means sterility can no longer be assured, even if the item looks clean. The correct action is to discard it and obtain a new sterile catheter. Visual inspection cannot confirm sterility, and rewapping does not restore it. The trap is confusing 'clean appearance' with 'sterile.'

1245. During insertion of an indwelling urinary catheter, the client suddenly moves and the sterile catheter brushes against the nurse's ungloved wrist before entering the urethra. Which action is best?

- A. Advance the catheter because the wrist was cleaned during hand hygiene
- B. Wipe the catheter with antiseptic and continue insertion
- C. Discard the catheter and use a new sterile catheter
- D. Insert the catheter, then monitor the client closely for infection

Correct Answer: C

Once the sterile catheter touches a nonsterile surface, including the nurse's ungloved wrist, it is contaminated and must be discarded. Antiseptic wiping does not make the catheter sterile again. Monitoring later does not correct the contamination event. This question tests strict adherence to aseptic technique during an invasive procedure.

1246. A practical nurse prepares a sterile field for a wound procedure at 0900. The provider is delayed, and the field remains untouched on a bedside table. At 0935, what is the best nursing action?

- A. Use the field because nothing has touched it
- B. Use the field if the room door remained closed
- C. Discard the field and set up a new sterile field
- D. Cover the field with a clean towel and continue to use it

Correct Answer: C

A sterile field should be prepared as close as possible to the time of use and must be constantly monitored. An unattended or delayed field should be considered contaminated and replaced. The common trap is assuming that 'untouched' means sterile, but time and lack of continuous observation make sterility uncertain.

1247. While setting up a sterile field on a waist-high table, the nurse drops a sterile gauze pad. The pad lands 2 inches below the top edge of the drape without touching anything else. Which action is most appropriate?

- A. Leave the gauze in place because it is still on the drape
- B. Move the gauze to the center of the field with sterile forceps
- C. Consider the gauze contaminated and discard it
- D. Use the gauze only for the final outer wipe

Correct Answer: C

The outer 1-inch border of a sterile field is considered contaminated. Because the gauze landed within that border area, it cannot be used. A likely trap is thinking any item resting on the drape remains sterile, but location on the field matters.

1248. A nurse is assisting with a sterile procedure and needs to reposition a bedside table holding the sterile field. Which action best protects asepsis?

- A. Keep the sterile field at or above waist level while moving it carefully
- B. Lower the table briefly to see around the client more easily
- C. Ask an ungloved assistant to carry the sterile supplies by hand
- D. Slide the sterile drape to one side so the table can fit past the bed

Correct Answer: A

Sterile items should be kept at or above waist level and in view. Lowering the field below waist level or altering the drape increases contamination risk. The trap is focusing on convenience during movement rather than maintaining visual control and proper positioning.

1249. The nurse is preparing to pour sterile solution into a sterile basin. Which action is correct?

- A. Hold the bottle 6 inches above the basin and pour toward the near edge
- B. Remove the bottle cap and place it sterile-side down on the table

C. Label the bottle if needed and pour without letting the bottle touch the basin

D. Recap the bottle between each small amount to preserve sterility

Correct Answer: C

When pouring a sterile solution, the nurse should avoid contaminating the bottle or basin by keeping the bottle from touching the sterile container. If the bottle is first opened, labeling is appropriate. The other options introduce contamination risk, especially placing the cap down on a surface or touching the bottle to the basin.

1250. A practical nurse wearing sterile gloves notices moisture soaking through the sterile drape under a tray during a procedure. Which interpretation is most accurate?

- A. The field remains sterile if the top surface is dry
- B. The field is contaminated because moisture can carry microorganisms through the drape
- C. Only the tray is contaminated, not the rest of the field
- D. The field is safe if the fluid was sterile saline

Correct Answer: B

Moisture can cause strike-through contamination, allowing microorganisms to pass from a nonsterile surface to the sterile field. Even if the fluid itself was sterile, the wet barrier no longer reliably protects the field. A common error is assuming sterile fluid cannot create contamination; the problem is the loss of the barrier.

1251. The nurse must add a sterile instrument to an already prepared sterile field. Which technique is best?

- A. Drop the item onto the center of the field from several inches above it
- B. Place the item by reaching across the field with a bare hand
- C. Open the package and let the instrument slide onto the field without crossing over it
- D. Set the package on the field first, then remove the instrument

Correct Answer: C

The best method is to open the sterile package and add the item without reaching over or touching the sterile field. Dropping from too high can cause splashing or movement, and placing the outer package on the field contaminates it. The key principle is avoiding contact and avoiding reaching across the field.

1252. During a sterile dressing change, the nurse turns away to answer a question from another staff member. The sterile field remains on the bedside table for about 10 seconds and is out of the nurse's view. What should the nurse do next?

- A. Continue because the field was uncovered only briefly
- B. Continue if no one walked near the table
- C. Ask a coworker whether the field looked untouched and proceed if confirmed
- D. Discard the field and prepare a new one

Correct Answer: D

A sterile field must remain in constant view. Once it is out of sight, its sterility cannot be guaranteed, even for a short time. A common trap is assuming a brief lapse or another person's reassurance is acceptable, but the nurse must treat the field as contaminated.

1253. A nurse wearing sterile gloves needs to adjust a sterile drape that has started to slip. Which movement is acceptable?

- A. Reaching below waist level to pull up the lower edge
- B. Touching only the sterile side of the drape that remains in view
- C. Brushing the drape against the uniform to steady it
- D. Leaning across the field to grasp the far corner

Correct Answer: B

The nurse may touch only sterile surfaces with sterile gloves while keeping hands and the field above waist level and in view. Reaching below the waist, brushing against clothing, or leaning across the field risks contamination. The trap is choosing a movement that seems efficient but breaks sterile technique.

1254. The practical nurse is setting up a sterile field in a semiprivate room. Which environmental condition requires the most immediate correction before opening supplies?

- A. The overbed table was cleaned 15 minutes ago
- B. The room is brightly lit
- C. A visitor is walking back and forth close to the setup area
- D. The nurse plans to stand on the right side of the table

Correct Answer: C

Movement near a sterile field can create air currents and increase contamination risk, so unnecessary traffic around the setup area should be limited. Clean surfaces and adequate lighting support safe setup. The key is recognizing environmental movement as an immediate threat to asepsis.

1255. A practical nurse opens a sterile kit and finds a small sterile cup placed upside down on the field by the manufacturer. The nurse needs to use the inside of the cup to hold solution. What is the best action?

- A. Turn the cup over with sterile gloves and use it
- B. Turn the cup over with clean gloves and use it
- C. Ask an assistant to flip the cup using the outside edge only
- D. Discard the cup because its inside is now contaminated

Correct Answer: A

If the cup is part of the sterile setup, the nurse can manipulate it with sterile gloves while preserving sterility. Clean gloves or an unsterile assistant would contaminate it. The trap is confusing contact with sterile gloves and contact with nonsterile hands; only sterile contact is acceptable.

1256. A nurse in sterile gloves accidentally extends one hand behind the body for a moment while reaching for extra tape. The glove did not touch any visible object. Which action is best?

- A. Continue because no contact occurred
- B. Continue after applying alcohol-based hand rub over the gloves

- C. Consider the glove contaminated and replace the sterile gloves
- D. Use that hand only to handle the outer packaging

Correct Answer: C

Sterile hands should be kept in front of the body and above waist level where they remain visible. A hand moved behind the body is considered contaminated because sterility can no longer be verified. The likely trap is assuming contamination depends only on seen contact, but loss of visual control is enough.

1257. The nurse is preparing for sterile wound care when the client begins coughing forcefully. Which action best reduces contamination risk while preserving the procedure plan?

- A. Open the sterile supplies now so the nurse can work quickly once coughing stops
- B. Ask the client to turn the head away and cover the mouth, then open supplies when the coughing subsides
- C. Proceed with setup because coughing affects only droplet precautions, not asepsis
- D. Place the sterile field at the foot of the bed while the client coughs

Correct Answer: B

Respiratory droplets can contaminate a sterile field. The nurse should first control the immediate source of contamination as much as possible, then open sterile supplies close to the time they will be used. The trap is opening supplies early to save time, which increases exposure of the field.

1258. While using sterile forceps from a sterile tray, the nurse notices the tips of the forceps were briefly pointed upward after being lifted. What is the best interpretation?

- A. The forceps remain sterile because only the tips touched the sterile tray
- B. The forceps remain sterile if they are used immediately
- C. The forceps are contaminated because fluid can flow from the handle toward the tips
- D. Only the handle is contaminated, so the tips may still be used

Correct Answer: C

Sterile forceps should be held with the tips downward so any moisture moves away from the tips. If the tips are pointed upward, fluid can run from the less clean handle area to the sterile tips, contaminating them. The common trap is focusing only on direct touch instead of contamination by gravity and moisture flow.

1259. A practical nurse is opening a sterile drape onto a work surface. Which sequence is correct for unfolding it without contaminating the field?

- A. Open the flap farthest away first, then side flaps, then the flap nearest the body
- B. Open the flap nearest the body first, then the far flap, then the side flaps
- C. Open either side flap first, then the near flap, then the far flap
- D. Open all flaps by reaching across the center so the drape stays flat

Correct Answer: A

The drape is opened away from the body first, then the sides, and the flap nearest the body last. This sequence prevents the nurse from reaching over the sterile field and contaminating it. The trap is choosing a sequence that feels intuitive but forces the nurse to lean across the field.

1260. During sterile setup, the nurse notices that one corner of the sterile wrapper brushes the sink while the inner contents remain covered. Which response is best?

- A. Use the contents because the sterile items themselves did not touch the sink
- B. Use the contents if the sink had been disinfected earlier
- C. Discard the package and obtain a new sterile item
- D. Open the package carefully and use only the item farthest from the contaminated corner

Correct Answer: C

If the sterile wrapper is contaminated during setup, the integrity of the sterile contents can no longer be assured. The safest action is to discard the package and replace it. The trap is trying to judge which inner item still looks clean, but sterility is based on maintained barriers, not appearance.

1261. A practical nurse is preparing a sterile field for insertion of an indwelling urinary catheter. After opening the outer package, the nurse notices a 1-inch tear along the edge of the sterile inner wrapper. What is the best action?

- A. Use the kit because the tear is small and at the edge
- B. Use the kit if the catheter and gloves inside still appear dry
- C. Discard the kit and obtain a new sterile catheterization set
- D. Reinforce the torn area with sterile tape and continue setup

Correct Answer: C

A torn sterile wrapper means sterility can no longer be assured, even if the tear is small or the contents look clean and dry. The safe action is to discard the kit and obtain a new one. The common trap is assuming a minor tear at the edge does not matter, but any break in the packaging can allow contamination.

1262. The practical nurse opens a sterile dressing tray and places sterile supplies on the field. During setup, the nurse realizes a clean roll of tape is needed to secure the outer dressing. Which item may be placed directly onto the sterile field?

- A. A roll of tape taken from the supply cart
- B. A package of sterile 4x4 gauze that has not yet been opened
- C. A pair of clean gloves from the nurse's pocket
- D. A pen used to label the dressing date

Correct Answer: B

An unopened sterile package may be placed onto the sterile field because its outer wrapper can be opened without contaminating the field. Clean tape, clean gloves, and a pen are not sterile and should not be placed directly on the sterile area. The likely trap is confusing clean with sterile.

1263. A nurse is setting up a sterile field for tracheostomy care. The sterile drape is placed on a bedside table that is level with the nurse's hips. Which observation requires the nurse to consider the field contaminated?

- A. The nurse keeps the field in constant view
- B. The top surface of the drape remains dry

- C. The nurse reaches across the field with a bare forearm
- D. Sterile supplies are added by dropping them onto the center of the field

Correct Answer: C

Reaching across a sterile field with a bare forearm contaminates the field because nonsterile body parts should not pass over sterile areas. Keeping the field in view and dry helps maintain sterility. Dropping sterile supplies onto the center of the field is acceptable when done properly. The trap is thinking brief contact-free movement over the field is harmless.

1264. A practical nurse is opening a sterile package containing a dressing tray. To avoid contaminating the contents, which flap should be opened last?

- A. The flap farthest from the nurse
- B. The flap nearest the nurse
- C. The right side flap
- D. The left side flap

Correct Answer: B

When opening a sterile package, the flap nearest the nurse is opened last. This helps prevent the nurse's arms or uniform from reaching over the sterile contents. The far flap is opened first, then the side flaps, and the near flap last. A common error is opening the nearest flap first, which increases contamination risk.

1265. During a sterile dressing change, the nurse notes that the sterile gloves are the correct size but feel tight across the fingertips after being donned. The gloves remain intact. Which action is best?

- A. Continue because glove fit does not affect sterility
- B. Apply sterile lubricant to reduce tension in the fingertips
- C. Remove the gloves and don a new pair in the correct fit
- D. Ask an ungloved assistant to stretch the gloves at the fingers

Correct Answer: C

Sterile gloves should fit properly to support safe movement and reduce the chance of tearing or accidental contamination. If the gloves are too tight, the nurse should replace them with a better-fitting sterile pair. The trap is focusing only on whether the gloves are still intact instead of whether the setup remains safely usable.

1266. A nurse opens a sterile tray for a central line dressing change. The client suddenly vomits onto the bed and overbed table before the procedure begins. The sterile field was not directly splashed, but it is close to the client. What is the nurse's best action?

- A.** Proceed because the field was not visibly soiled
- B.** Move the tray farther away and continue
- C.** Cover the field with a sterile towel until the room is cleaned
- D.** Discard the field, perform environmental cleanup, and set up a new sterile field

Correct Answer: D

Vomiting creates a high risk of environmental contamination from droplets and nearby spread, so the nurse should not trust the field even if no visible splash is seen. The safest action is to discard the field, clean the area, and prepare a new sterile field. The likely trap is relying only on visible soil to judge contamination.

1267. The practical nurse is preparing to don sterile gloves using an open-glove technique. Which action is correct after the first glove is on?

- A.** Use the sterile gloved hand to slide under the cuff of the second glove
- B.** Use the ungloved hand to adjust the fingers of the first glove
- C.** Touch the outside of the second glove only with the sterile gloved hand
- D.** Hold the second glove by its inner cuff with the sterile gloved hand

Correct Answer: D

After the first glove is on, the sterile gloved hand should handle only sterile surfaces, so it slides under the folded inner cuff of the second glove to don it. Touching the outside of the second glove incorrectly or using the ungloved hand to adjust the first glove risks contamination. The key principle is sterile touches sterile; bare skin touches only the inside cuff before the glove is on.

1268. A nurse is adding sterile saline to a sterile bowl for wound irrigation. The bottle cap has already been removed. Which handling of the bottle cap is most appropriate if the nurse needs to set it down briefly?

- A.** Place the cap with the inside facing up on a clean surface
- B.** Place the cap with the inside facing down on a clean surface
- C.** Place the cap on the sterile field for easier access
- D.** Ask an assistant to hold the cap in a bare hand

Correct Answer: A

If the cap must be set down, the inside of the cap should be protected from contamination, so it is placed with the inside facing up. Placing it face down contaminates the inner sterile surface. Putting it on the sterile field or having someone hold it with bare hands also risks contamination. The trap is choosing the position that seems more stable instead of the one that protects the inner surface.

1269. The practical nurse prepares a sterile field for a dressing change and notices that the package's sterility indicator has changed to the expected color, but the package expiration date passed 2 months ago. What should the nurse do?

- A.** Use the package because the indicator confirms sterility
- B.** Use the package if the wrapper is intact
- C.** Discard the package and obtain a new sterile item
- D.** Open the package and inspect the contents before deciding

Correct Answer: C

An expired sterile package should not be used, even if the wrapper is intact and the indicator appears correct. Sterility cannot be assured past the expiration date. The likely trap is assuming one sign of sterility, such as the indicator color, overrules the expiration date.

1270. A practical nurse is performing sterile suctioning through a tracheostomy. While wearing sterile gloves, the nurse accidentally touches the nonsterile bedrail with the nondominant hand that is stabilizing the catheter package. What is the best response?

- A. Continue, because only the nondominant hand touched the rail
- B. Continue if the dominant hand remains sterile
- C. Consider both gloves contaminated and restart with new sterile supplies as needed
- D. Wipe the contaminated glove with sterile saline and proceed

Correct Answer: C

If a sterile glove touches a nonsterile surface, that glove is contaminated. In a sterile procedure, the nurse should not proceed with contaminated gloves. Depending on the setup, restarting with new sterile gloves and supplies may be necessary to preserve asepsis. The trap is assuming one hand can safely become a clean hand during a sterile procedure when the procedure requires sterility.

1271. The nurse is setting up a sterile field on a clean bedside table. Which placement best protects the field from contamination during the procedure?

- A. At chest level on a shelf above the nurse's shoulders
- B. Below waist level so the supplies are easier to reach
- C. At or above waist level and within the nurse's direct view
- D. On the client's bed near the procedure site to save time

Correct Answer: C

A sterile field should be kept at or above waist level and within constant view. A field below waist level or out of sight is considered contaminated. Placing it on the client's bed is unsafe because bed linens are not sterile and movement can contaminate the field. The common trap is choosing the most convenient location rather than the safest one.

1272. A nurse opens a sterile instrument package and plans to transfer the item to a sterile field without touching it. While opening the wrapper, one sleeve of the nurse's uniform brushes the outer 1-inch edge of the sterile drape. The center of the field remains untouched. Which action is best?

- A. Use the field because only the outer border was touched
- B. Discard the field because any contact with the drape contaminates all of it
- C. Use only the side opposite the touched border
- D. Cover the touched edge with sterile gauze and continue

Correct Answer: A

The outer 1-inch edge of a sterile field is considered contaminated, so touching only that border does not contaminate the entire field. The usable sterile area is the center portion. The trap is overcorrecting and discarding the whole field when only the already nonsterile border was affected.

1273. The practical nurse is assisting with a sterile bedside procedure. A coworker walks between the nurse and the sterile field to reach the sink. The coworker's clothing does not touch the table. What should the nurse conclude?

- A. The field remains sterile because no contact occurred
- B. The field is contaminated because movement between the nurse and the field can disrupt asepsis
- C. The field remains sterile if the coworker was wearing gloves
- D. The field remains sterile if the coworker passed quickly

Correct Answer: B

Traffic and movement around a sterile field increase contamination risk. Personnel should avoid reaching, walking, or passing between a nurse and the sterile field during a sterile procedure. Even without visible contact, asepsis may be disrupted. The likely trap is assuming contamination happens only through direct touch.

1274. A practical nurse has opened a sterile field for a dressing change. The sterile forceps are lying on the field, and the nurse needs to pick them up for use. Which technique is correct?

- A. Grasp the forceps near the tips and hold them with the tips pointed downward
- B. Grasp the forceps at the handles and hold them with the tips pointed downward
- C. Grasp the forceps at the midpoint and keep them level with the floor
- D. Lift the forceps by the tips and turn them upward to inspect them

Correct Answer: B

Sterile forceps should be picked up by the handles and held with the tips pointed downward so fluid does not run from the less clean handle area toward the sterile tips. Touching the tips or pointing them upward risks contaminating the working end. The trap is focusing on convenience instead of protecting the sterile tips.

Isolation precautions

1275. A nurse is preparing to care for a client with pulmonary tuberculosis. Which action is most appropriate before entering the client's room?

- A. Put on an N95 respirator
- B. Apply a surgical mask to the nurse
- C. Wear sterile gloves only
- D. Use a face shield without a mask

Correct Answer: A

Tuberculosis requires airborne precautions, so the nurse should wear an N95 respirator before entering the room. A surgical mask does not provide the same respiratory protection for airborne particles. Gloves alone or a face shield alone do not protect against inhalation of airborne organisms.

1276. A client with *Clostridioides difficile* infection is on contact precautions. Which action by the nurse is best when leaving the room?

- A. Use alcohol-based hand rub after removing gloves
- B. Wash hands with soap and water
- C. Remove gloves in the hallway
- D. Keep the gown on until reaching the nurses' station

Correct Answer: B

For *C. difficile*, handwashing with soap and water is preferred because spores are not reliably removed by alcohol-based hand rub alone. Gloves and gown should be removed before leaving the room. Leaving the room with contaminated PPE increases transmission risk.

1277. The nurse receives report on four clients. Which client should be placed in a private room first if only one private room is available?

- A. A client with a femur fracture in traction
- B. A client with shingles limited to one small area and fully covered
- C. A client with suspected measles and a new cough
- D. A client with chronic hepatitis B

Correct Answer: C

A client with suspected measles should be prioritized for a private room because measles requires airborne precautions and is highly transmissible. A localized covered shingles lesion may not need the same immediate level of isolation in an otherwise immunocompetent client. Hepatitis B is spread by blood and body fluids, not by casual room sharing, and a fracture does not require isolation.

1278. A parent asks why a child with bacterial meningitis must be placed on droplet precautions. Which response by the nurse is best?

- A. The illness spreads mainly through tiny airborne particles that stay suspended for hours
- B. The illness can spread through large respiratory droplets during close contact
- C. The illness is spread only by touching contaminated surfaces
- D. The precautions are used only to protect the child from other clients

Correct Answer: B

Droplet precautions are used because bacterial meningitis can spread through large respiratory droplets, especially with close contact. Airborne spread describes a different transmission route. Surface contact may contribute to contamination, but that is not the main reason droplet precautions are used here. The precautions protect others as well as support safe care.

1279. A nurse is preparing to transport a client with active pulmonary tuberculosis to radiology. Which action is appropriate?

- A. Place a surgical mask on the client during transport
- B. Delay transport until the client can wear an N95 respirator
- C. Remove all isolation precautions once the client leaves the room
- D. Have the transporter wear sterile gloves only

Correct Answer: A

When a client on airborne precautions must leave the room, the client should wear a surgical mask to help contain respiratory secretions during transport. The N95 respirator is for healthcare personnel, not the client. Isolation concerns continue during transport, and gloves alone do not address airborne transmission.

1280. The nurse enters the room of a client on contact precautions and realizes a blood pressure cuff is needed. Which action is best?

- A. Borrow a cuff from another client's room and return it later
- B. Use dedicated equipment for the isolated client when available

- C. Bring the unit's shared cuff into the room and place it back on the cart immediately after use
- D. Skip obtaining the blood pressure to avoid contamination

Correct Answer: B

Dedicated client-care equipment helps limit transmission for clients on contact precautions. Borrowing equipment between rooms or immediately returning a shared cuff without proper handling can spread organisms. Skipping an assessment is not appropriate care.

1281. A nurse removes gloves after providing care to a client on contact precautions, then reaches up to untie the gown at the neck with bare hands. Which statement best describes the problem?

- A. The nurse should remove the gown before the gloves
- B. The gown ties are considered clean once gloves are removed
- C. The nurse may contaminate bare hands while removing the gown incorrectly
- D. The sequence does not matter as long as hand hygiene is performed later

Correct Answer: C

Improper PPE removal can contaminate the nurse's hands. After glove removal, the nurse must avoid touching contaminated gown surfaces or handling the gown in a way that transfers organisms. The exact doffing sequence is intended to reduce self-contamination; it is not correct to say sequence does not matter.

1282. A newly licensed practical nurse asks when standard precautions should be used. Which response by the charge nurse is best?

- A. Only when a client is known to have a communicable disease
- B. Only in intensive care areas
- C. With every client, regardless of diagnosis
- D. With clients who require a private room

Correct Answer: C

Standard precautions are used with every client because infection status is not always known. They are not limited to certain diagnoses, locations, or room assignments. This reduces risk of transmission from blood, body fluids, secretions, excretions, and contaminated items.

1283. A nurse is assigned to four clients. Which assignment should the nurse question?

- A. Placing a client with neutropenia in a room with a client who has influenza
- B. Assigning a dedicated stethoscope to a client on contact precautions
- C. Masking a client on droplet precautions during transport
- D. Performing hand hygiene after removing PPE

Correct Answer: A

A client with neutropenia should not room with a client who has influenza because of the high infection risk to the immunocompromised client. The other actions are appropriate infection-control measures. This question tests recognition of a dangerous cohorting decision.

1284. A client with varicella requires isolation. Which combination of precautions should the nurse anticipate?

- A. Contact only
- B. Droplet only
- C. Airborne and contact
- D. Protective environment only

Correct Answer: C

Varicella requires airborne and contact precautions. Choosing only one route underestimates the transmission risk. A protective environment is used for certain severely immunocompromised clients, not as the isolation category for varicella infection itself.

1285. A nurse is caring for a client on droplet precautions for influenza. Which action by an unlicensed assistive personnel (UAP) requires immediate correction?

- A. Putting on a surgical mask before entering the room
- B. Donning gloves when handling tissues and secretions
- C. Taking the meal tray into the room after performing hand hygiene
- D. Leaving the room wearing the same surgical mask to deliver supplies down the hall

Correct Answer: D

PPE used in an isolation room should be removed and discarded appropriately before leaving the room to avoid spreading organisms into the hallway. Wearing the same mask down the hall creates transmission risk. The other actions are appropriate for droplet precautions and routine care.

1286. A client on contact precautions asks why visitors are being instructed about gowns and gloves. Which explanation by the nurse is best?

- A. These items keep visitors from carrying organisms to themselves or others
- B. The equipment is required only because the room is private
- C. Visitors must wear the equipment so the nurse can reduce hand hygiene
- D. The main purpose is to make visitors spend less time in the room

Correct Answer: A

Gowns and gloves on contact precautions help prevent visitors from carrying organisms on their hands or clothing to themselves, staff, or other clients. The need is based on transmission risk, not room type. PPE never replaces hand hygiene and is not intended to discourage visiting.

1287. The nurse is planning care for a client on airborne precautions. Which room setup is most appropriate?

- A. A semi-private room with the curtain pulled
- B. A private room with special air handling
- C. A room near the nurses' station with the door left open for observation
- D. A treatment room used only during daytime hours

Correct Answer: B

Airborne precautions require a private room with appropriate air handling. A semi-private room and curtain do not prevent airborne spread. Leaving the door open works against airborne isolation measures, and a treatment room is not an appropriate substitute for proper placement.

1288. A nurse cares for a client on contact precautions for an infection causing heavy wound drainage. The nurse wears gloves and a gown. During the dressing change, the drainage splashes unexpectedly toward the nurse's face. Which principle was missed in planning PPE?

- A. Eye and face protection should be added when splash exposure is likely
- B. Sterile gloves prevent all mucous membrane exposure
- C. A gown is unnecessary when gloves are worn
- D. Masks are used only for airborne precautions

Correct Answer: A

PPE selection is based not only on isolation category but also on the expected task and exposure risk. If splashing is likely, eye and face protection should be used. Sterile gloves do not protect the eyes, gowns do not replace facial protection, and masks or face shields may be needed in situations beyond airborne precautions.

1289. A nurse must choose which task to perform last during a medication pass. Which client placement best supports infection control?

- A. See the client with scabies on contact precautions first to get isolation care done early
- B. See the client with a draining MRSA wound before the postoperative client
- C. See the postoperative client before the client on contact precautions for draining diarrhea
- D. Alternate between the isolated client and non-isolated clients to save time

Correct Answer: C

When possible, the nurse should move from cleaner or more vulnerable situations to more contaminated ones, so seeing the postoperative client before the client on contact precautions supports infection control. Going from a contact-precaution client to other clients increases cross-contamination risk, and alternating back and forth is inefficient and unsafe.

1290. A nurse is caring for a client with suspected infectious diarrhea who has just been placed on contact precautions. Which item should be dedicated to this client whenever possible?

- A. Stethoscope
- B. Sharps container
- C. Wall-mounted hand sanitizer dispenser
- D. Call light

Correct Answer: A

A dedicated stethoscope helps prevent transmission of organisms between clients when contact precautions are needed. A sharps container and hand sanitizer dispenser are shared environmental items, and a call light remains in the room for that client but is not the key reusable care item addressed by isolation practice.

1291. The nurse is caring for a client with a draining wound that requires contact precautions. Which action is most appropriate when bringing supplies into the room?

- A. Place the clean supply cart just inside the doorway for easy access
- B. Bring only the supplies needed for that episode of care
- C. Set unused supplies on the overbed table for the next dressing change
- D. Carry the medication administration record into the room and leave it at the bedside

Correct Answer: B

Only necessary supplies should be taken into an isolation room to reduce contamination and waste. Bringing in a full cart or leaving extra supplies in the room increases the chance that clean items become contaminated. Paperwork or other reusable materials should also be handled carefully and not left unnecessarily in the room.

1292. A client with suspected measles arrives from the emergency department. Which instruction should the nurse give to a staff member who is not immune and asks to help with the admission?

- A. Enter the room with a surgical mask and keep the visit brief
- B. Wait to assist until standard precautions have been discontinued

- C.** Do not enter the room if avoidable; assign immune staff when possible
- D.** Enter after the client has been in the room for 30 minutes

Correct Answer: C

Measles requires airborne precautions, and susceptible staff should avoid exposure when possible. A surgical mask is not sufficient for airborne precautions, and there is no reason to wait for standard precautions to end because standard precautions apply to all clients. Simply waiting 30 minutes does not make entry safe for a nonimmune staff member.

- 1293.** The nurse prepares to enter the room of a client on droplet precautions. In which situation is a mask most clearly required?
- A.** The nurse will hand the client a water pitcher from the doorway
 - B.** The nurse will stand 6 feet away and speak through the open door
 - C.** The nurse will obtain vital signs at the bedside
 - D.** The nurse will document care at the desk outside the room

Correct Answer: C

Bedside vital signs require close contact, so a mask is indicated for droplet precautions. The other options either do not involve room entry for direct care or do not place the nurse in close client contact. The key issue is proximity during care, not just the task name.

- 1294.** A nurse is removing personal protective equipment after caring for a client on contact precautions. Which sequence best reduces self-contamination?
- A.** Remove goggles, remove gloves, perform hand hygiene, remove gown
 - B.** Remove gloves, remove gown, perform hand hygiene
 - C.** Remove gown, touch the front ties, remove gloves, leave the room
 - D.** Remove gloves, leave the gown on while charting, then remove gown later

Correct Answer: B

For contact precautions using gloves and gown, the contaminated items should be removed in a way that limits spread, followed by hand hygiene. Gloves are generally removed first, then the gown, then hand hygiene. The other options either place contaminated PPE handling out of order, involve touching contaminated surfaces, or delay removal inappropriately.

- 1295.** A client with mumps must be moved from a semiprivate room because a roommate is newly immunocompromised. Which new placement is best?
- A.** A private room with droplet precautions
 - B.** A private room with contact precautions only
 - C.** A room near the nurses' station with no special precautions
 - D.** A positive-pressure room with visitors restricted

Correct Answer: A

Mumps is managed with droplet precautions, and moving the client to a private room helps reduce transmission risk, especially when the roommate is highly vulnerable. Contact precautions alone are insufficient, and positive-pressure rooms are not used for this purpose. Being near the nurses' station does not address infection-control needs.

- 1296.** During a shift report, the practical nurse learns that a client on airborne precautions needs a portable chest x-ray. Which action is best?
- A.** Request that the study be done in the room if possible
 - B.** Transport the client immediately before other clients are awake
 - C.** Delay the x-ray until isolation precautions are discontinued
 - D.** Send the client without a mask to avoid interfering with breathing

Correct Answer: A

For clients on airborne precautions, limiting transport is a priority. If the test can be done in the room, that option reduces exposure risk to others. Transporting early does not remove the risk, delaying care may be inappropriate, and sending the client without a mask is unsafe.

1297. A nurse is planning assignments for the day. Which client is the best choice to place with another client who has the same confirmed organism and the same isolation needs when a private room is unavailable?

- A. A client with an unexplained fever and new rash
- B. A client recovering from surgery with a fresh abdominal incision
- C. A client with the same infection and no other communicable condition
- D. A client receiving chemotherapy who is neutropenic

Correct Answer: C

Cohorting may be appropriate when clients have the same confirmed infection and require the same precautions, as long as no other conflicting infection risks are present. A client with an unexplained rash needs further evaluation, and postoperative or neutropenic clients are poor roommates because they are especially vulnerable.

1298. A visitor enters the room of a client on droplet precautions wearing a mask under the nose. What is the nurse's best response?

- A. Allow the visit because the mouth is covered
- B. Ask the visitor to pull the mask over both the nose and mouth
- C. Tell the visitor to remove the mask and stand near the window
- D. End the visit and document visitor noncompliance without further teaching

Correct Answer: B

For droplet precautions, the mask must cover both the nose and mouth to be effective. The best response is immediate correction and teaching. Allowing the mask to remain under the nose is inadequate, and ending the visit without first attempting to correct the problem is not the best initial action.

1299. A nurse leaves the room of a client with suspected *Clostridioides difficile* after removing gloves and gown. Which action is most appropriate next?

- A. Use alcohol-based hand rub until dry
- B. Wash hands with soap and water
- C. Put on a new pair of gloves before touching the chart

D. Clean the stethoscope, then perform hand hygiene at the nurses' station

Correct Answer: B

When caring for a client with suspected or confirmed *C. difficile*, handwashing with soap and water is preferred because spores are a concern. Alcohol-based hand rub alone is not the best choice in this situation. Cleaning equipment is important, but hand hygiene should not be delayed.

1300. The practical nurse is helping implement isolation precautions during a busy shift. Which action most increases the risk of transmitting organisms to other clients?

- A. Using a disposable blood pressure cuff for a client on contact precautions
- B. Removing a soiled gown before leaving the room
- C. Carrying a used pulse oximeter from an isolation room to another client without disinfection
- D. Posting precaution signage at the room entrance

Correct Answer: C

Reusable equipment moved from one client to another without proper cleaning is a major transmission risk. The other actions support isolation precautions rather than undermine them. This question tests recognition of contaminated fomites as a source of spread.

1301. A client on airborne precautions must leave the room for an essential procedure. Which action by the nurse is most appropriate during transport?

- A. Place a surgical mask on the client if tolerated
- B. Ask transport staff to skip hand hygiene to shorten hallway time
- C. Remove the client's mask once the elevator doors close
- D. Cover the stretcher with a clean sheet instead of masking the client

Correct Answer: A

When a client on airborne precautions must be transported, the client should wear a surgical mask if tolerated to help contain respiratory secretions. Hand hygiene still matters, and masking should continue during transport. A sheet over the stretcher does not replace source control.

1302. The charge nurse is reviewing room assignments. Which client placement should be changed first?

- A.** A client with shingles limited to one area is in a private room
- B.** A client with suspected tuberculosis is in a room with the door open and no negative-pressure capability
- C.** A client with RSV is in a room with contact precautions posted
- D.** A client with a draining leg ulcer is in a private room on contact precautions

Correct Answer: B

Suspected tuberculosis requires airborne precautions, including an appropriate airborne infection isolation environment. A room with the door open and no negative-pressure capability is unsafe and should be corrected first. The other placements are not the most urgent threat among the options given.

1303. A practical nurse is teaching a new staff member about isolation precautions. Which statement by the new staff member indicates correct understanding?

- A.** Clients on isolation precautions do not need standard precautions
- B.** Isolation precautions replace the need for hand hygiene if PPE is worn correctly
- C.** Standard precautions still apply, and transmission-based precautions are added when indicated
- D.** Transmission-based precautions are used only after a laboratory diagnosis is confirmed

Correct Answer: C

Standard precautions apply to all clients, and transmission-based precautions are added when a specific route of spread is suspected or known. PPE does not replace hand hygiene, and precautions are often started based on clinical suspicion before confirmation. This makes C the most accurate statement.

1304. A nurse enters the room of a client on contact precautions for a multidrug-resistant organism to administer oral medication. The client asks for help to the bathroom, and while assisting, the nurse's sleeve brushes against the client's uncovered drainage dressing. Which action should the nurse take before giving medications to another client?

- A.** Remove gloves only, then continue because the gown protected the uniform
- B.** Remove gloves and gown, perform hand hygiene, and ensure contaminated clothing or equipment is addressed before leaving care complete
- C.** Use hand sanitizer over the gloves and proceed to the next medication pass
- D.** Keep the gown on while preparing the next client's medications to avoid contaminating the uniform

Correct Answer: B

After contamination during contact-precaution care, the nurse should remove PPE properly, perform hand hygiene, and avoid carrying contaminated clothing or equipment into another client's environment. Option A ignores possible sleeve contamination and omits full PPE removal steps. Option C is unsafe because gloves are single-use. Option D risks spreading organisms outside the room.

1305. A practical nurse is caring for four clients. Which client should be placed on airborne precautions?

- A.** A client with pulmonary tuberculosis
- B.** A client with influenza
- C.** A client with methicillin-resistant *Staphylococcus aureus* in a wound
- D.** A client with respiratory syncytial virus

Correct Answer: A

Pulmonary tuberculosis requires airborne precautions because the organism can remain suspended in the air and travel over distance. Influenza generally requires droplet precautions, and MRSA in a wound requires contact precautions. RSV is commonly managed with contact precautions in many care settings. A common trap is choosing any respiratory illness, but not all respiratory infections are airborne.

1306. The nurse is preparing to enter the room of a client with suspected varicella (chickenpox). Which protective equipment is required for the nurse before entry?

- A. Gloves only
- B. Surgical mask only
- C. N95 respirator or equivalent respiratory protection
- D. Sterile gown and sterile gloves

Correct Answer: C

Suspected varicella requires airborne precautions, so the nurse should use an N95 respirator or equivalent respiratory protection before entering the room. Gloves may also be needed depending on anticipated contact, but they do not replace airborne protection. A surgical mask is not sufficient for airborne isolation. Sterile items are not required just because a client is on isolation precautions.

1307. A client has been placed on contact precautions for a multidrug-resistant wound infection. Which nursing action is appropriate when entering the room to take vital signs?

- A. Wear a gown and gloves before contact with the client or the client's environment
- B. Wear an N95 respirator before entering the room
- C. Keep the door closed at all times and avoid wearing gloves
- D. Use sterile gloves for all contact with the client

Correct Answer: A

Contact precautions require gown and gloves when entering the room if contact with the client or environmental surfaces is expected, which is likely during vital signs. N95 respirators are for airborne precautions, not routine contact precautions. A closed door is not the main requirement for contact precautions, and sterile gloves are unnecessary unless a sterile procedure is being performed.

1308. The practical nurse is reinforcing teaching for the family of a client on droplet precautions. Which statement by a family member shows correct understanding?

- A. We can share the client's water pitcher if we wash it afterward.
- B. We should wear a mask when we are close to the client.
- C. The client must keep the room door closed at all times.
- D. Hand hygiene is not necessary if gloves are worn.

Correct Answer: B

Droplet precautions require a mask when in close contact with the client. Sharing personal items is unsafe, hand hygiene is still necessary even when gloves are used, and a closed door is not the defining measure for droplet precautions. The trap is overfocusing on the room instead of the route of transmission.

1309. A nurse must obtain a blood pressure on a client who is on contact precautions for a resistant organism. There is only one portable blood pressure cuff available on the unit. What is the best action?

- A. Use the cuff on the isolated client, then use it on the next client if no visible soil is present
- B. Use the cuff only after covering the client's arm with a clean towel
- C. Use the cuff for the isolated client and disinfect it according to facility protocol before reuse
- D. Avoid taking the blood pressure until a dedicated cuff becomes available

Correct Answer: C

Shared equipment used for a client on contact precautions should be cleaned and disinfected according to facility protocol before being used for another client. No visible soil does not mean the equipment is safe to reuse. A towel is not an acceptable substitute for proper disinfection, and delaying needed assessment is inappropriate.

1310. A client on droplet precautions needs assistance ambulating in the hallway for a prescribed activity order. Which nursing response is best?

- A.** Allow hallway ambulation if the client wears clean gloves
- B.** Keep the client in the room unless leaving is essential; if transport is necessary, have the client wear a mask
- C.** Permit ambulation only after the nurse dons an N95 respirator
- D.** Cancel the activity order because clients on droplet precautions may never leave the room

Correct Answer: B

Clients on droplet precautions should remain in the room unless movement is necessary. If transport is needed, the client should wear a mask to reduce transmission risk. Gloves on the client do not address droplet spread. An N95 respirator is for airborne precautions, and isolation does not automatically cancel all movement forever.

1311. The nurse is reviewing new prescriptions. Which order should the nurse question for a client on contact precautions for infectious diarrhea?

- A.** Assign a dedicated stethoscope to the client
- B.** Place the client in a private room if possible
- C.** Use alcohol-based hand rub as the preferred method after care
- D.** Wear gloves when entering the room

Correct Answer: C

For infectious diarrhea, especially when spore-forming organisms are a concern, soap and water handwashing is preferred over alcohol-based hand rub after care because alcohol may not be as effective against spores. The other measures are appropriate for contact precautions. The key trap is applying general hand hygiene rules without recognizing the special situation of infectious diarrhea.

1312. A practical nurse is assigned to a client with suspected meningococcal meningitis. Which intervention is the priority while waiting for room placement?

- A.** Apply a surgical mask to the client if transport or close contact is occurring
- B.** Start contact precautions only after the culture result is confirmed
- C.** Place used linens in a biohazard bag at the bedside and leave them there
- D.** Ask the client to wear sterile gloves during all interactions

Correct Answer: A

Suspected meningococcal meningitis calls for prompt droplet precautions. Masking the client during transport or close contact helps reduce transmission while room placement is being arranged. Waiting for culture confirmation delays protection. Special linen handling beyond standard safe handling is not the priority here, and sterile gloves are not relevant.

1313. The nurse has just entered the anteroom of a negative-pressure room to care for a client on airborne precautions. The nurse notices the respirator was left at the desk. What should the nurse do?

- A.** Enter the room quickly because exposure time will be brief
- B.** Use a surgical mask from the isolation cart instead
- C.** Retrieve the proper respirator before entering the client's room
- D.** Ask the client to turn away while care is delivered

Correct Answer: C

Airborne precautions require the correct respirator before entering the room. Brief exposure is still exposure, and a surgical mask does not provide the same protection against airborne particles. Asking the client to turn away does not remove the airborne risk. The safest action is to get the appropriate respirator first.

1314. A nurse is caring for a client on contact precautions and is called urgently to assist with a second client in the next room. The nurse has already touched the first client's bedrail and overbed table. Which action is best before leaving to help the second client?

- A. Remove gloves, perform hand hygiene, and ensure contaminated protective equipment is removed appropriately before contact with the next client
- B. Keep the same gloves on to save time, then change them after assessing the second client
- C. Remove the gown only and put alcohol hand rub over the gloves
- D. Carry the first client's chart into the second client's room so information is not forgotten

Correct Answer: A

After contact with a client on contact precautions or the client's environment, the nurse should remove contaminated PPE appropriately and perform hand hygiene before caring for another client. Wearing the same gloves spreads organisms. Hand rub over gloves is not acceptable, and bringing contaminated items such as a chart between rooms increases transmission risk.

1315. A client with a suspected airborne infection arrives on the unit before the negative-pressure room is ready. Which temporary measure is most appropriate?

- A. Place the client in the clean utility room with the door shut
- B. Keep the client in a common hallway area until housekeeping finishes
- C. Place a surgical mask on the client and separate the client from others while arranging appropriate placement
- D. Send the client back to the sending department until a discharge opens a room

Correct Answer: C

When immediate airborne room placement is not yet available, the safest temporary measure is to mask the client and separate the client from others while appropriate placement is arranged. Utility rooms and hallways are not appropriate client care areas, and delaying care by sending the client away is not the best response if safer interim precautions can be started at once.

1316. Two clients need rooms. One has suspected pulmonary tuberculosis, and the other has a large draining wound infected with a resistant organism. The unit has one negative-pressure private room and one standard private room. Which room assignment is best?

- A. Place the tuberculosis client in the negative-pressure room and the wound client in the standard private room
- B. Place the wound client in the negative-pressure room and the tuberculosis client in the standard private room
- C. Place both clients in the same negative-pressure room if the beds are 6 feet apart
- D. Place the tuberculosis client in the hallway with a mask until discharge planning is complete

Correct Answer: A

The client with suspected pulmonary tuberculosis needs airborne precautions and should receive the negative-pressure room. The client with a resistant draining wound requires contact precautions and is appropriately placed in a standard private room. The trap is giving the specialized room to the client who also needs isolation, but only airborne precautions require the negative-pressure environment.

1317. A practical nurse is caring for a client on droplet precautions who begins coughing forcefully just as the nurse enters the room without a mask. The client is alert and can follow directions. What should the nurse do first?

- A. Step back from close contact, ask the client to cover the mouth and nose, and obtain the appropriate mask
- B. Continue the assessment from the bedside because the exposure has already happened

- C. Leave the room and return in 30 minutes when coughing has stopped
- D. Immediately apply sterile gloves before speaking to the client

Correct Answer: A

The first priority is to reduce ongoing droplet exposure. The nurse should increase distance, direct the client to cover the mouth and nose if able, and obtain the appropriate mask before resuming close care. Continuing bedside care without a mask prolongs exposure. Waiting 30 minutes is arbitrary and delays care, and sterile gloves do not address droplet transmission.

Standard precautions

1318. A practical nurse is preparing to administer oral medications in a long-term care facility. Which action best reflects standard precautions?

- A. Wearing gloves for every medication pass regardless of contact risk
- B. Performing hand hygiene before and after contact with each client
- C. Using a surgical mask for all clients on the medication cart route
- D. Keeping the same pair of gloves on while moving from room to room

Correct Answer: B

Standard precautions apply to all clients and include hand hygiene at appropriate times. Gloves are used based on anticipated contact with blood, body fluids, mucous membranes, nonintact skin, or contaminated items, not automatically for every medication pass. Wearing the same gloves between clients increases transmission risk.

1319. A client needs assistance with perineal care after urinary incontinence. Which action by the practical nurse follows standard precautions?

- A. Put on clean gloves before providing care and remove them immediately after the task
- B. Wear sterile gloves because the perineal area is highly contaminated
- C. Double-glove to prevent exposure during any hygiene task

- D. Skip hand hygiene if gloves remain intact throughout care

Correct Answer: A

Clean gloves are appropriate when contact with body fluids is anticipated during perineal care. Sterile gloves are not required for routine hygiene. Double-gloving is not standard for this task, and hand hygiene is still required after glove removal because gloves do not replace it.

1320. While emptying a client's urinary drainage bag, the nurse's aide asks why gloves are needed. Which response by the practical nurse is most accurate?

- A. Gloves are needed only if the client has a known infection
- B. Gloves are needed because urine should be treated as potentially infectious during contact
- C. Gloves are optional if the drainage bag has a spout
- D. Gloves are required only when the urine appears cloudy or bloody

Correct Answer: B

Standard precautions assume that contact with body fluids can create exposure risk, so gloves are used when handling urine drainage equipment. A known diagnosis is not required. Visible appearance does not determine whether precautions are needed.

1321. A practical nurse is about to collect a capillary blood glucose sample. Which action is most appropriate under standard precautions?

- A. Use the same lancet device for multiple clients if the needle is changed
- B. Wear gloves and discard the lancet in a sharps container immediately after use
- C. Recap the lancet before disposal to avoid accidental sticks
- D. Delay hand hygiene until all glucose checks on the unit are completed

Correct Answer: B

Standard precautions include glove use for blood exposure risk and immediate disposal of sharps in an approved container. Lancet devices are not shared between clients, recapping increases injury risk, and hand hygiene should not be delayed until the end of the round.

1322. A practical nurse is changing the bed linens of a client whose gown is soiled with wound drainage. Which action best follows standard precautions?

- A. Hold the linens away from the body and place them directly into the designated linen bag
- B. Shake the linens in the room to remove loose debris before bagging
- C. Carry the linens against the uniform to keep drainage from dripping
- D. Sort the linens at the bedside before taking them out of the room

Correct Answer: A

Soiled linens should be handled in a way that limits contamination of clothing and the environment. Shaking linens can disperse microorganisms. Carrying them against the uniform and sorting them at the bedside increase exposure and contamination risk.

1323. During tracheostomy care, the client begins coughing forcefully and secretions spray toward the nurse. Which personal protective equipment was most important to have anticipated based on standard precautions?

- A. Shoe covers
- B. Face protection for the eyes and face
- C. A lead apron
- D. A sterile gown for all tracheostomy care

Correct Answer: B

Standard precautions require protection of the eyes, nose, and mouth when splashes or sprays of body fluids are likely. Tracheostomy care can generate secretions, so face protection is important. Shoe covers and lead aprons are unrelated, and a sterile gown is not automatically required for all tracheostomy care.

1324. A nurse removes gloves after suctioning a client and then immediately adjusts the client's oxygen tubing with bare hands. Which statement best describes this action?

- A. It is acceptable because the gloves were removed first
- B. It is acceptable because oxygen tubing is not a sterile item
- C. It breaks standard precautions because hand hygiene should occur after glove removal and before touching clean equipment

D. It breaks standard precautions only if visible secretions remain on the hands

Correct Answer: C

After a contaminated task such as suctioning, the nurse should remove gloves and perform hand hygiene before touching the client, the environment, or clean equipment. The problem is not sterility but cross-contamination. Waiting for visible soil is a common trap; contamination can occur even when hands look clean.

1325. A practical nurse prepares to irrigate a client's indwelling urinary catheter using a prescribed sterile technique. Which statement best distinguishes standard precautions from the sterile technique used in this situation?

- A. Standard precautions replace sterile technique whenever gloves are worn
- B. Standard precautions apply to all clients, while sterile technique is added for procedures that require maintaining asepsis
- C. Sterile technique is needed only if the client has a fever
- D. Standard precautions are used only in home care, not in facilities

Correct Answer: B

Standard precautions are used for all clients. Some procedures also require sterile technique to prevent introducing microorganisms into normally sterile areas or equipment. Gloves alone do not replace aseptic technique, and the need for sterility is based on the procedure, not on fever status or setting.

1326. A practical nurse is cleaning a bedside glucometer after use. Which action is most consistent with standard precautions?

- A. Wipe the device only if blood is visible on the screen
- B. Disinfect the device according to facility protocol between client uses
- C. Place the device on the next client's bed until it is needed again
- D. Rinse the device quickly with tap water and dry it with a paper towel

Correct Answer: B

Shared client-care equipment should be cleaned and disinfected according to facility protocol between uses to prevent cross-transmission. Visible blood is not the only indication for cleaning. Placing used equipment on another client's bed and rinsing it with tap water are unsafe practices.

1327. The practical nurse enters a client's room to take vital signs. The client suddenly vomits onto the bed and floor. What should the nurse do first?

- A.** Step back and obtain appropriate protective equipment before assisting with cleanup
- B.** Immediately begin cleaning the spill with bare hands to prevent odor
- C.** Finish the vital signs quickly before addressing the vomiting episode
- D.** Ask dietary services to clean the room while the nurse leaves

Correct Answer: A

Vomitus can create splash and contact exposure risk, so the nurse should first obtain the appropriate PPE and then provide care and cleanup safely. Beginning cleanup unprotected risks exposure. Finishing routine tasks first delays needed care, and delegating the entire situation away ignores the nurse's responsibility for immediate client safety.

1328. A client with no known communicable disease asks why the nurse performs hand hygiene before putting on gloves for a dressing change. Which response is best?

- A.** Hand hygiene is done only because policy requires it
- B.** Gloves can have tiny holes, and hands may contaminate the gloves during application
- C.** It is mainly to protect the nurse from legal liability
- D.** It is unnecessary, but many nurses do it out of habit

Correct Answer: B

Hand hygiene before glove use helps reduce transmission because hands may contaminate the outer glove surface during donning, and gloves are not a substitute for clean hands. The best answer explains the infection-control reason, not merely policy or habit.

1329. A practical nurse is disposing of a used needle after giving a subcutaneous injection when the sharps container is on the other side of the room. Which action is best?

- A.** Recap the needle using two hands, then carry it to the container
- B.** Place the needle on the bedside table until the medication round is finished
- C.** Carry the uncapped needle carefully to the sharps container and discard it promptly
- D.** Bend the needle to reduce the chance of puncturing the trash bag

Correct Answer: C

The used needle should be discarded promptly in a sharps container without recapping, bending, or setting it down. Although having the container nearby is ideal, the safest available option among these choices is immediate careful transport for disposal. Recapping and bending are major sharps-injury risks.

1330. A practical nurse is caring for four clients. For which situation are standard precautions alone generally indicated, assuming no additional transmission-based precautions have been prescribed?

- A.** A client with draining wound care that may splash during irrigation
- B.** A client receiving oral medications and no anticipated contact with blood or body fluids
- C.** A client requiring suctioning of thick oral secretions
- D.** A client with fecal incontinence needing perineal care

Correct Answer: B

Standard precautions apply to every client, but PPE under standard precautions is chosen based on expected exposure. Oral medication administration without anticipated body fluid contact may require only hand hygiene. The other situations involve likely splash or contact with body fluids and would typically require gloves or additional barrier protection within standard precautions.

1331. After removing a client's dressing, the nurse has gloves on and notices the overbed table is cluttered with clean supplies needed for the next step. Which action best maintains standard precautions?

- A. Use the gloved hands to move the clean supplies closer, then continue the procedure
- B. Ask the client to move the clean supplies to the bedside chair
- C. Remove the gloves, perform hand hygiene, and then handle the clean supplies
- D. Place a paper towel over the supplies and move them with the contaminated gloves

Correct Answer: C

After contact with a used dressing, gloves are contaminated. The nurse should avoid touching clean supplies until gloves are removed and hand hygiene is performed. A common trap is thinking gloves make all contact safe; in fact, contaminated gloves can spread organisms to clean equipment and supplies.

1332. A practical nurse is teaching a newly hired assistant about standard precautions. Which statement by the assistant indicates correct understanding?

- A. I only need eye protection if the client is on isolation
- B. I should wear gloves anytime I enter a client's room
- C. I should choose protective equipment based on the task and the risk of exposure to blood or body fluids
- D. I can skip hand hygiene after removing gloves if my hands do not look soiled

Correct Answer: C

Standard precautions require the caregiver to assess the task and use PPE according to the anticipated exposure risk. Eye protection is based on splash risk, not only on isolation status. Gloves are not automatically required for simply entering every room, and hand hygiene is required after glove removal even when hands appear clean.

1333. A practical nurse is preparing to perform oral care for a client who is alert and cooperative. The client has dried blood on the lips from mouth sores. Which action best reflects standard precautions?

- A. Wear clean gloves during oral care and perform hand hygiene before and after care
- B. Use sterile gloves because blood is present on the lips
- C. Skip gloves if the client has no diagnosed infection
- D. Put on a respirator before entering the room

Correct Answer: A

Standard precautions apply to blood and body fluids from any client, regardless of diagnosis. Clean gloves are appropriate for oral care when contact with blood or mucous membranes is expected, and hand hygiene is required before and after care. Sterile gloves are not needed for routine oral care, and a respirator is not indicated based on this information alone.

1334. A practical nurse is assisting with removal of a peripheral IV catheter. Which additional personal protective equipment should the nurse use if there is a reasonable chance blood could splash during removal?

- A. Shoe covers
- B. Face shield or mask with eye protection
- C. Sterile gown only
- D. Hair cover

Correct Answer: B

Standard precautions require protecting the eyes, nose, and mouth when splash or spray of blood or body fluids is anticipated. A face shield or a mask with eye protection addresses that risk. Shoe covers and hair covers are not the primary protection here, and a sterile gown alone does not protect the mucous membranes of the face.

1335. The practical nurse helps a client blow their nose, discards the tissues, removes gloves, and leaves the room. Which action is still required under standard precautions?

- A. Apply a clean gown before seeing the next client
- B. Wash the client's bedside table with disinfectant

- C. Perform hand hygiene after glove removal
- D. Place the client in a private room

Correct Answer: C

Gloves do not replace hand hygiene. Under standard precautions, hand hygiene is required after removing gloves because contamination can occur during glove removal or from small defects in gloves. The other actions are not routinely required based on this situation alone.

1336. A practical nurse is giving skin care to a client with intact skin and no visible drainage. During the assessment, the nurse notices a new open blister with serous drainage on the lower leg. What is the nurse's best action?

- A. Continue care without gloves because the drainage is clear
- B. Stop, perform hand hygiene if needed, and apply gloves before touching the area
- C. Ask another staff member to complete the care
- D. Place the client on transmission-based isolation immediately

Correct Answer: B

Once the nurse identifies possible contact with nonintact skin or body fluid drainage, gloves are indicated under standard precautions. Clear drainage is still treated as potentially infectious. There is no basis here for immediate transmission-based isolation, and simply handing the task to someone else does not address the need for proper precautions.

1337. A practical nurse is collecting a fresh stool specimen from a bedpan for laboratory testing. Which technique best follows standard precautions?

- A. Transfer the specimen while wearing gloves and avoid contaminating the outside of the container
- B. Use sterile gloves to prevent introducing microorganisms into the stool specimen
- C. Double-bag the specimen in the client's room before labeling it
- D. Empty the remaining stool into the handwashing sink after the sample is collected

Correct Answer: A

Standard precautions call for gloves when handling stool and careful specimen handling to reduce contamination of the container and environment. Sterile gloves are not required for routine stool collection. Labeling and packaging must be done correctly per facility procedure, but double-bagging is not the key standard-precautions principle here. A handwashing sink should never be used for stool disposal.

1338. While making rounds, a practical nurse notices a used lancet left on a client's meal tray. What is the nurse's priority action based on standard precautions?

- A. Carry the lancet carefully to the nurses' station for disposal
- B. Recap the lancet to prevent accidental sticks before moving it
- C. Use forceps or a scoop device if available and place the lancet into an approved sharps container
- D. Leave the lancet in place until the person who used it returns

Correct Answer: C

Sharps should be disposed of promptly in an approved sharps container, and direct hand contact should be avoided when possible. Recapping increases injury risk, carrying an exposed sharp across the unit is unsafe, and leaving it on a tray creates immediate danger for the client and staff.

1339. A practical nurse has just removed gloves after providing ostomy care. The nurse needs to document the output in the electronic health record located in the room. Which action best follows standard precautions?

- A. Use the keyboard immediately because the gloves were worn during care
- B. Put on a second pair of gloves and type the note before leaving the room
- C. Perform hand hygiene before touching the computer
- D. Ask the client to enter the output amount to avoid contaminating the computer

Correct Answer: C

After glove removal, the nurse should perform hand hygiene before touching clean environmental surfaces such as a computer keyboard. Wearing gloves during dirty care does not keep hands or the environment automatically clean. Double-gloving to type spreads contamination, and asking the client to document is not an appropriate workaround.

1340. A client asks, "Why do you wear gloves to handle my urine if my tests do not show an infection?" Which response by the practical nurse is best?

- A.** Gloves are worn only because the facility requires them for all personal care
- B.** Urine and other body substances are handled with standard precautions because infection status may not always be known at the time of care
- C.** Gloves keep me from needing to wash my hands after care
- D.** I wear gloves mainly so your room will not need extra cleaning

Correct Answer: B

Standard precautions are used based on the possibility of exposure to blood, body fluids, secretions, excretions, mucous membranes, and nonintact skin, not only on confirmed infection. This answer explains the principle accurately. Gloves do not replace hand hygiene, and the purpose is exposure prevention, not convenience or reduced cleaning.

1341. A practical nurse is preparing to empty a surgical drain and then reinforce the dressing around the insertion site. Which sequence best follows standard precautions?

- A.** Apply clean gloves, empty the drain, remove gloves, perform hand hygiene, then apply new gloves for the dressing task
- B.** Apply one pair of gloves, empty the drain, and use the same gloves to handle the clean dressing supplies
- C.** Empty the drain with bare hands to preserve glove cleanliness for the dressing
- D.** Apply sterile gloves first, empty the drain, and continue using them for all remaining care

Correct Answer: A

After contact with potentially contaminated drainage equipment, gloves should be removed and hand hygiene performed before moving to a cleaner task. Then new gloves are used as needed for the dressing step. Using the same gloves from a contaminated task to handle clean supplies risks cross-contamination. Bare hands are inappropriate, and sterile gloves do not solve the contamination issue if used across dirty and clean steps.

1342. A practical nurse enters a room to help a client reposition in bed. During the move, the client's abdominal drain unexpectedly disconnects and fluid spills onto the sheet. What should the nurse do first?

- A.** Finish repositioning with bare hands to keep the client comfortable
- B.** Pause the task and put on gloves before handling the fluid-soiled area
- C.** Leave the room and ask environmental services to clean the bed
- D.** Place the client in isolation because a body fluid spill occurred

Correct Answer: B

The first priority under standard precautions is to protect against exposure when body fluid contact is now expected. The nurse should pause and put on gloves before managing the contaminated area. Comfort matters, but not at the cost of unsafe exposure. A spill alone does not mean the client requires transmission-based isolation.

1343. A practical nurse is caring for a client with a small amount of blood on a pillowcase from a nosebleed. Which handling of the linen best follows standard precautions?

- A.** Hold the linen away from the uniform and place it directly in the designated linen container
- B.** Shake the pillowcase briskly to remove dried debris before bagging it
- C.** Carry the linen uncovered in the hallway so the laundry team can inspect it
- D.** Rinse the blood out in the client's sink before sending it to laundry

Correct Answer: A

Soiled linen should be handled in a way that avoids contact with the nurse's clothing and minimizes contamination of the environment. Shaking linen can aerosolize contaminants, carrying it uncovered spreads contamination, and rinsing it in the client's sink is unsafe and inappropriate.

1344. A practical nurse is about to irrigate a heavily crusted eye with prescribed solution. The client suddenly coughs and turns the face toward the nurse. Which PPE combination is most appropriate under standard precautions?

- A. Gloves only
- B. Gloves and eye protection
- C. Gown only
- D. Head cover and shoe covers

Correct Answer: B

Irrigating an eye can create splashing, and the client's cough increases the risk of droplets contacting the nurse's eyes. Standard precautions support gloves for contact with secretions and eye protection when splash is anticipated. Gloves alone are not enough because they do not protect the mucous membranes of the face.

1345. A practical nurse has a small cut on one finger covered by an adhesive bandage. The nurse is assigned to provide morning care, including mouth care and emptying a urinary drainage bag. Which statement is most accurate?

- A. The nurse should avoid all direct client care for the shift
- B. The nurse may provide care using standard precautions, including appropriate glove use for contact with body fluids and mucous membranes
- C. The nurse may provide care only if double gloves are worn for all contact with the client
- D. The nurse does not need gloves because the cut is already covered

Correct Answer: B

A covered minor cut does not by itself prohibit care, but the nurse must use standard precautions appropriately. Gloves are still needed for tasks involving body fluids and mucous membranes. Double-gloving for all contact is not routinely required, and a covered cut does not eliminate the need for gloves during exposure-prone tasks.

1346. A practical nurse wearing gloves is suctioning oral secretions. The suction canister tubing slips off, and secretions splash onto the nurse's forearm and wrist where the skin is intact. After securing the client, which action is most appropriate?

- A. Wipe the area with a dry paper towel and continue working
- B. Remove gloves and wash the exposed skin area promptly with soap and water
- C. Apply alcohol gel over the visible secretions until the skin feels dry
- D. Cover the area with a clean sleeve and report at the end of the shift

Correct Answer: B

When body substances contact intact skin, the area should be washed promptly with soap and water after the client's immediate safety is addressed. A dry wipe alone is inadequate, alcohol gel is not the best choice on visibly soiled skin, and delaying action increases exposure risk.

1347. A practical nurse is setting up to assist with a bedside wound dressing change. The old dressing is saturated, and a tray of unopened clean supplies is on the bedside stand. To best follow standard precautions, where should the nurse place the clean supplies before removing the old dressing?

- A. On the client's bed next to the soiled dressing supplies for easy reach
- B. On the overbed table after moving it closer to the wound area, even if it may be splashed
- C. In a clean area away from the expected contaminated field until ready for the clean part of the procedure
- D. On the floor under the bedside stand to avoid using contaminated furniture

Correct Answer: C

Standard precautions include preventing contamination of clean supplies by separating them from the dirty field and any likely splash zone. Keeping supplies in a clean area until the nurse transitions to the clean part of care reduces cross-contamination. Beds, floors, and splash-prone surfaces are poor choices for clean supplies.

1348. A practical nurse is preparing to give a client a bed bath. The client is continent, has intact skin, and has no visible drainage. Which action best reflects standard precautions?

- A.** Wear gloves for the entire bath from start to finish
- B.** Wear gloves only when there is anticipated contact with blood, body fluids, mucous membranes, or nonintact skin
- C.** Use a gown because all direct client contact requires barrier protection
- D.** Double-glove because bathing can expose the nurse to microorganisms

Correct Answer: B

Standard precautions require PPE based on the task and the likelihood of exposure, not automatically for every client contact. For a routine bath with intact skin and no expected contact with body fluids or nonintact skin, gloves are used only if exposure is anticipated. Choice A is a common overuse error, while C and D add unnecessary PPE.

1349. A practical nurse is about to administer oral medications. The client asks for help inserting dentures first. Which action best follows standard precautions?

- A.** Put on clean gloves before handling the dentures
- B.** Ask the client to insert the dentures independently because nurse handling is not allowed
- C.** Wear sterile gloves to avoid contaminating the dentures
- D.** Skip gloves because saliva is not included in standard precautions

Correct Answer: A

Handling dentures involves contact with oral secretions and mucous membranes, so clean gloves are appropriate under standard precautions. Sterile gloves are not needed for this task. Choice D is incorrect because standard precautions apply to potential exposure to body fluids and mucous membranes, even when the client is not known to have an infection.

1350. After giving perineal care, a practical nurse removes gloves at the bedside. The call light activates as the nurse starts to leave the room. What should the nurse do next to best follow standard precautions?

- A.** Answer the call light first, then perform hand hygiene
- B.** Put on a new pair of gloves before touching anything else
- C.** Perform hand hygiene before touching the call light or other surfaces
- D.** Use an alcohol pad to wipe the hands quickly instead of hand hygiene

Correct Answer: C

Glove removal does not replace hand hygiene. After contact involving body fluids or the perineal area, the nurse should perform hand hygiene before touching environmental surfaces or equipment. Choice A risks contaminating the environment, and B skips the required hand hygiene step.

1351. A practical nurse is asked to transport a specimen cup containing freshly obtained sputum to the laboratory pickup area. Which action best follows standard precautions?

- A.** Carry the uncapped cup carefully to avoid pressure buildup
- B.** Place the sealed specimen container in a biohazard bag for transport
- C.** Pour the sputum into a larger container before sending it
- D.** Wrap the cup in a towel and place it in a regular paper bag

Correct Answer: B

Specimens should be handled in a way that prevents leakage and contamination of the environment. A sealed container placed in the appropriate transport bag best follows standard precautions. An uncapped cup or improvised packaging increases the risk of spills and exposure.

1352. A practical nurse is changing the bed linens of a client who has drainage from a wound that soaked through the top sheet. Which action is most appropriate under standard precautions?

- A. Hold the sheet away from the uniform and place it directly into the designated linen bag
- B. Shake the sheet to identify the wettest area before bagging it
- C. Carry the sheet against the body to keep it from dripping on the floor
- D. Rinse the soiled area in the client's sink before sending it to laundry

Correct Answer: A

Soiled linens should be handled as little as possible and kept away from the nurse's clothing to prevent contamination. They should not be shaken, rinsed in the client care area, or carried against the body. Choice B is a common error because shaking can disperse microorganisms.

1353. During capillary blood glucose testing, a client jerks the hand away just as the practical nurse activates the lancet. The used lancet falls onto the bedside table. What should the nurse do first?

- A. Recap the lancet so it can be carried safely to the sharps container
- B. Pick up the lancet carefully and place it in the sharps container
- C. Leave the lancet in place until housekeeping arrives
- D. Use gloved hands to bend the lancet tip inward before disposal

Correct Answer: B

Used sharps should be disposed of promptly in an approved sharps container. The nurse should carefully pick up the lancet and discard it without recapping, bending, or manipulating it. Recapping and bending increase needlestick risk, and leaving it in place creates an immediate safety hazard.

1354. A practical nurse is preparing to suction a client's tracheostomy. Which PPE selection best reflects standard precautions for this procedure?

- A. Gloves only
- B. Gloves and gown only
- C. Gloves, mask, and eye protection
- D. Mask only

Correct Answer: C

Tracheostomy suctioning can generate splashes or sprays of respiratory secretions, so the nurse should use gloves plus face protection, including a mask and eye protection. A gown may be added if soiling of clothing is anticipated, but among the options, C is the best answer because it addresses exposure to mucous membranes of the nose, mouth, and eyes.

1355. A practical nurse has just emptied a client's urinary drainage bag while wearing gloves. Before the nurse can remove the gloves, the infusion pump starts beeping. What is the best action?

- A. Adjust the pump with the contaminated gloves to stop the noise quickly
- B. Remove the gloves, perform hand hygiene, and then handle the pump
- C. Silence the pump with one gloved hand and remove the gloves afterward
- D. Ask the client to silence the pump while the nurse finishes the task

Correct Answer: B

After contact with urine and drainage equipment, the gloves are contaminated. The nurse should remove them and perform hand hygiene before touching clean equipment such as the infusion pump. Choices A and C risk contaminating shared equipment. Choice D delays proper intervention and does not address contamination control.

1356. A client with no isolation order sneezes forcefully while the practical nurse is adjusting the bedside table. Respiratory secretions land on the table surface. Which action best follows standard precautions?

- A. Wipe the surface at the end of the shift because the client is not on isolation
- B. Clean and disinfect the contaminated surface promptly using facility-approved supplies
- C. Cover the area with a towel until housekeeping can clean it
- D. Move the table into the hallway so environmental services can identify it

Correct Answer: B

Standard precautions include prompt cleaning and disinfection of contaminated environmental surfaces. The client's lack of an isolation order does not change the need to address visible contamination. Waiting, covering the area, or moving contaminated furniture can increase exposure risk.

1357. A practical nurse is applying a clean dressing to a superficial abrasion. While removing the old dressing, the nurse's glove tears, but the nurse's hand does not contact the wound or drainage. What is the best action?

- A. Continue because there was no direct contact with body fluids
- B. Remove the torn glove, perform hand hygiene, apply new gloves, and continue
- C. Place a second glove over the torn glove and finish the dressing change
- D. Use hand sanitizer over the torn glove and complete the procedure

Correct Answer: B

A torn glove is no longer an intact barrier. The nurse should stop, remove the damaged glove, perform hand hygiene, and don new gloves before resuming care. Placing another glove over a torn one or sanitizing over gloves does not correct the break in barrier protection.

1358. A practical nurse is assigned to trim the fingernails of an older adult client. During the assessment, the nurse notes dry skin and a small crack with slight bleeding near one nail bed. Which action best follows standard precautions?

- A. Proceed without gloves because nail care is a clean task
- B. Wear gloves because contact with blood or nonintact skin is possible
- C. Delay care until a sterile field can be prepared
- D. Ask another staff member to perform the task because bleeding is present

Correct Answer: B

The presence of a skin crack with slight bleeding means there is potential contact with blood and nonintact skin, so gloves are indicated under standard precautions. A sterile field is unnecessary for routine nail care, and delegating solely because of slight bleeding is not the key issue.

1359. A practical nurse is caring for a client with a draining skin lesion that is covered by a dressing. The nurse needs to check the client's blood pressure, reinforce the dressing, and then document on the computer in the room. Which sequence best follows standard precautions?

- A. Check blood pressure, reinforce the dressing with gloves, remove gloves, perform hand hygiene, then document
- B. Reinforce the dressing with gloves, check blood pressure with the same gloves, then document after glove removal
- C. Document first, reinforce the dressing with gloves, then check blood pressure before hand hygiene
- D. Check blood pressure with gloves on, reinforce the dressing, remove gloves, and leave documentation for later

Correct Answer: A

The cleanest task should be done before the task involving contact with drainage or nonintact skin. After reinforcing the dressing, the nurse must remove gloves and perform hand hygiene before touching the computer. Choice B is the major trap because it spreads contamination from the dressing task to equipment.

1360. A practical nurse is assisting with irrigation of a client's wound. During the procedure, fluid unexpectedly splashes onto the nurse's face shield and mask but not into the eyes. The client is stable. Which action is most appropriate after the procedure is completed?

- A.** Keep the same mask and face shield on for the next client because they protected the nurse
- B.** Remove the PPE carefully and replace contaminated items before further client care
- C.** Wipe the front of the mask with alcohol and continue working
- D.** Remove the gloves first, then touch the front of the mask to take it off quickly

Correct Answer: B

Once PPE is visibly contaminated, it should be removed carefully to avoid self-contamination and replaced before further care. The front of the mask and shield are considered contaminated surfaces. Choice D is a high-risk error because touching the front of contaminated PPE can transfer microorganisms to the hands and face.

Health Promotion and Maintenance

Antepartum care

1361. A client at 10 weeks of pregnancy calls the clinic reporting nausea that is worse in the morning. Which instruction is most appropriate for the nurse to give first?

- A. Drink a large glass of water as soon as you wake up
- B. Eat a few dry crackers before getting out of bed
- C. Skip breakfast and eat a large lunch instead
- D. Lie flat for 30 minutes after meals

Correct Answer: B

Eating a few dry crackers before rising can help reduce morning nausea by preventing an empty stomach. Drinking a large amount at once may worsen nausea, skipping meals can make symptoms worse, and lying flat after meals can increase discomfort or reflux.

1362. During a routine prenatal visit, a pregnant client asks why folic acid is recommended. Which response by the nurse is best?

- A. It helps prevent neural tube defects in the fetus
- B. It prevents gestational diabetes later in pregnancy
- C. It eliminates the risk of pregnancy-induced hypertension
- D. It increases fetal movement during the second trimester

Correct Answer: A

Folic acid is recommended because it helps reduce the risk of neural tube defects. It does not prevent gestational diabetes or hypertension, and it has no direct role in increasing fetal movement. Those options may sound health-related, but they do not reflect the main purpose of folic acid supplementation.

1363. A nurse is reinforcing teaching with a client who is 28 weeks pregnant. Which statement by the client indicates understanding of when to seek immediate evaluation?

- A. I should call if my ankles are mildly swollen at the end of the day
- B. I should call if I feel tired after a busy afternoon
- C. I should call if I have vaginal bleeding
- D. I should call if I need to urinate more often

Correct Answer: C

Vaginal bleeding during pregnancy requires prompt evaluation because it may indicate a serious complication. Mild dependent ankle swelling, fatigue, and urinary frequency are common pregnancy findings and are usually less urgent unless severe or accompanied by other concerning symptoms.

1364. A pregnant client in the second trimester says, "I get light-headed when I lie flat on my back." Which explanation should the nurse provide?

- A. The uterus can compress major blood vessels and reduce blood return to the heart
- B. The fetus presses on the diaphragm and causes low blood sugar
- C. Progesterone causes permanent narrowing of the coronary arteries
- D. Iron needs drop during pregnancy, which lowers oxygen delivery

Correct Answer: A

When a pregnant client lies supine, the enlarging uterus can compress major vessels, decreasing venous return and causing dizziness or hypotension. The other options do not correctly explain this common antepartum problem. A likely trap is choosing a choice that mentions pressure from the fetus but links it to the wrong body system.

1365. A client at 31 weeks of pregnancy reports a sudden gush of clear fluid from the vagina. What is the nurse's priority response?

- A. Advise the client to rest and mention it at the next scheduled visit
- B. Tell the client to begin counting fetal movements for 24 hours

- C. Instruct the client to come in for immediate evaluation
- D. Suggest increasing oral fluids and avoiding walking

Correct Answer: C

A sudden gush of clear fluid may indicate rupture of membranes and requires immediate evaluation. Waiting for the next visit delays care. Fetal movement counting may be useful later but is not the first priority here. Increasing fluids does not address the potential complication.

1366. At a prenatal visit, a client who is 34 weeks pregnant says, "I've had a bad headache since this morning, and now lights seem to bother my eyes." Which action should the nurse take first?

- A. Recommend a dark room and reassess at the next appointment
- B. Advise the client to increase fluids and limit salt
- C. Assess the client promptly for possible hypertensive complications
- D. Explain that headaches are expected in the third trimester

Correct Answer: C

Headache with visual disturbance in late pregnancy can signal a hypertensive complication and requires prompt assessment. Reassurance alone is unsafe. Increasing fluids or changing diet may sound reasonable for a minor headache, but the visual symptoms make this more concerning.

1367. A nurse is teaching a pregnant client about daily fetal movement awareness. Which statement by the client shows correct understanding?

- A. If I notice the baby is moving much less than usual, I should contact my healthcare provider
- B. The baby should move the same number of times every hour, every day
- C. I only need to monitor movement if I have abdominal pain
- D. Fetal movement usually disappears near the end of pregnancy because space is limited

Correct Answer: A

A decrease from the baby's usual movement pattern should be reported because it may indicate a problem. Movement does not need to be identical every hour, and clients should not wait for pain before paying attention to decreased movement. Near term, movement may feel different, but it should not disappear.

1368. A client in the first trimester asks what can help prevent constipation during pregnancy. Which recommendation is best?

- A. Limit activity and reduce fluid intake to avoid cramping
- B. Increase fiber intake, fluids, and regular exercise as tolerated
- C. Take a strong laxative at the first sign of decreased stool frequency
- D. Avoid fruits and vegetables because they can increase gas

Correct Answer: B

Increasing fiber, fluids, and activity is appropriate first-line teaching for constipation in pregnancy. Reducing fluids worsens constipation. Strong laxatives should not be the first response without provider guidance. Avoiding fruits and vegetables removes helpful fiber sources.

1369. A pregnant client asks why prenatal visits are scheduled regularly even when she feels well. Which response by the nurse is best?

- A. They are mainly needed to document weight gain for insurance purposes
- B. They help identify potential problems early and monitor maternal-fetal well-being
- C. They are only necessary in the last trimester when labor may begin unexpectedly
- D. They are primarily for confirming the baby's sex and position

Correct Answer: B

Regular prenatal visits allow ongoing assessment of both the client and fetus and help identify concerns early. The other options reduce prenatal care to a minor or inaccurate purpose. A common trap is choosing an answer that includes one true detail, such as fetal position later in pregnancy, but misses the broader goal.

1370. A client who is 26 weeks pregnant tells the nurse, "My hands and face suddenly look puffy today." Which follow-up is most important?

- A. Ask whether the client ate salty foods at lunch
- B. Ask whether the client has a headache or visual changes
- C. Ask whether the client has chosen a pediatric provider yet
- D. Ask whether the client has been sleeping on the left side

Correct Answer: B

Sudden swelling of the face and hands is concerning in pregnancy, especially if accompanied by headache or visual changes, which may point to a hypertensive complication. Questions about diet or sleep position are less urgent. Asking about newborn planning is unrelated to the immediate risk.

1371. A nurse is reinforcing teaching for a pregnant client who reports heartburn after meals. Which suggestion is most appropriate?

- A. Eat smaller, more frequent meals and remain upright after eating
- B. Drink carbonated beverages with meals to improve digestion
- C. Skip evening meals and eat only once daily
- D. Lie down immediately after eating to reduce stomach pressure

Correct Answer: A

Small, frequent meals and staying upright after meals can help reduce heartburn during pregnancy. Carbonated beverages may worsen symptoms, skipping meals is not appropriate, and lying down after eating often makes reflux worse.

1372. A client at 18 weeks of pregnancy calls the clinic and says, "I have burning when I urinate and feel like I need to go all the time." Which response by the nurse is best?

- A. This is an expected discomfort of pregnancy, so just increase your rest
- B. You should avoid drinking fluids for a few hours to reduce bladder irritation
- C. You need prompt evaluation because this may indicate a urinary tract infection
- D. This means the fetus is putting pressure on your bladder, so no follow-up is needed

Correct Answer: C

Burning with urination suggests possible urinary tract infection and requires evaluation during pregnancy. Frequency alone may be common, but dysuria is not something to dismiss. The trap is confusing a normal symptom of pregnancy, urinary frequency, with a symptom that suggests infection.

1373. A client who is 30 weeks pregnant reports regular, painful contractions occurring every 8 minutes for the past hour. What is the nurse's priority action?

- A. Tell the client that Braxton Hicks contractions are expected and to ignore them
- B. Instruct the client to come in for immediate evaluation
- C. Advise the client to wait until the contractions are 2 minutes apart
- D. Suggest taking a warm bath and calling back tomorrow if still uncomfortable

Correct Answer: B

Regular, painful contractions at 30 weeks may indicate preterm labor and should be evaluated immediately. Waiting may delay treatment. Although Braxton Hicks contractions can occur during pregnancy, they are usually irregular and less concerning than persistent, regular painful contractions.

1374. A pregnant client at 37 weeks states, "I suddenly have intense pain in my abdomen, and there is some dark vaginal bleeding." Which action should the nurse prioritize?

- A. Advise the client to lie down and recheck symptoms in 2 hours
- B. Instruct the client to come for emergency evaluation immediately
- C. Tell the client this is likely normal cervical change before labor
- D. Recommend drinking juice in case the baby is reacting to low blood sugar

Correct Answer: B

Severe abdominal pain with vaginal bleeding is an emergency and requires immediate evaluation. It should not be treated as routine late-pregnancy discomfort or normal cervical change. The distractors delay care or focus on unrelated causes.

1375. At a prenatal appointment, a client says, "I have had swelling in my legs for weeks, but today the baby's movements seem much less than usual, and I have a constant headache." Which finding should the nurse treat as the highest priority cue?

- A. Swelling in the legs for weeks
- B. Decreased fetal movement and constant headache
- C. The fact that the symptoms began on the same day
- D. Discomfort that is worse later in the afternoon

Correct Answer: B

Decreased fetal movement combined with a constant headache raises concern for serious maternal-fetal compromise and needs prompt attention. Long-standing leg swelling alone is less specific. The timing or time of day is less important than the combination of high-risk symptoms.

1376. A nurse is reinforcing teaching with a client who is 9 weeks pregnant and reports frequent nausea with occasional vomiting. Which instruction is most appropriate to help reduce symptoms?

- A. Drink large amounts of fluid with meals to keep the stomach full
- B. Eat several small meals during the day and keep dry crackers at the bedside
- C. Avoid eating until nausea completely passes
- D. Choose spicy foods to stimulate appetite

Correct Answer: B

Small, frequent meals and bland dry foods such as crackers can help reduce nausea in early pregnancy. Drinking large amounts with meals may worsen fullness and nausea, skipping food can make nausea worse, and spicy foods often increase symptoms.

1377. A client who is 24 weeks pregnant tells the nurse, "I don't really want to get a flu shot because I never get the flu." Which response by the nurse is best?

- A. It is avoided during pregnancy because it can trigger labor
- B. It is recommended during pregnancy because influenza can be more serious for the pregnant client

C. It should only be given after 36 weeks of pregnancy

D. It is only needed if other children live in the home

Correct Answer: B

Influenza vaccination is recommended during pregnancy because influenza infection can be more severe in pregnant clients. It is not limited to late pregnancy, and the need is not based on whether children are at home. The trap is assuming a healthy client has no benefit from vaccination.

1378. At a prenatal visit, a client who is 16 weeks pregnant says, "I haven't felt the baby move yet, so I think something is wrong." Which response by the nurse is best?

- A. Fetal movement is usually not expected until labor begins
- B. You should feel movement every hour by this point
- C. Many clients do not notice fetal movement until later in the second trimester
- D. Not feeling movement now means the fetus is in distress

Correct Answer: C

Many pregnant clients first notice fetal movement later in the second trimester, so absence of perceived movement at 16 weeks is not automatically abnormal. The incorrect options either minimize normal pregnancy assessment too much or overstate concern without enough evidence.

1379. A nurse is collecting data from a client at 11 weeks of pregnancy. Which statement requires immediate follow-up?

- A. My breasts feel fuller than usual
- B. I need to urinate more often than before
- C. I have mild fatigue by the end of the day
- D. I have vaginal bleeding with cramping

Correct Answer: D

Vaginal bleeding with cramping in early pregnancy is an urgent finding that requires prompt evaluation. Breast fullness, urinary frequency, and fatigue are common early pregnancy changes. The key is distinguishing expected discomforts from a possible pregnancy complication.

1380. A pregnant client asks why blood pressure and urine are checked at each prenatal visit. Which explanation by the nurse is best?

- A.** These checks help detect problems such as hypertension and protein in the urine early
- B.** These checks are mainly done to estimate the baby's sex
- C.** These checks show exactly when labor will begin
- D.** These checks replace the need for the rest of the physical assessment

Correct Answer: A

Repeated blood pressure and urine assessment can help identify complications early, including hypertensive disorders and proteinuria. The other options are inaccurate. The best-answer reasoning focuses on surveillance for maternal and fetal well-being during routine prenatal care.

1381. A client at 29 weeks of pregnancy reports new low back pain, pelvic pressure, and menstrual-like cramps. Which question is most important for the nurse to ask next?

- A.** Have you been craving unusual foods lately?
- B.** When was your last bowel movement?
- C.** Have you noticed any fluid leakage or vaginal bleeding?
- D.** Do you usually sleep on your left side?

Correct Answer: C

At 29 weeks, low back pain, pelvic pressure, and cramping can suggest preterm labor. Asking about fluid leakage or vaginal bleeding helps identify additional urgent signs. The other questions may be relevant later but do not address the most concerning possibility first.

1382. A nurse is teaching a newly pregnant client about nutrition. Which meal choice best reflects appropriate teaching?

- A.** Turkey sandwich, orange slices, and milk
- B.** Hot dog, chips, and soda
- C.** Coffee and a glazed doughnut
- D.** Garden salad with bottled dressing only

Correct Answer: A

The turkey sandwich, fruit, and milk provide a balanced combination of protein, vitamins, and calcium that supports pregnancy nutrition. The other options are lower in nutrient density or incomplete as a meal. The likely trap is choosing the salad, which sounds healthy but lacks adequate overall nutrition by itself.

1383. A client who is 33 weeks pregnant tells the nurse, "My rings suddenly feel tight, and I have a pounding headache that won't go away." Which action should the nurse take first?

- A.** Advise the client to decrease sodium intake for a few days
- B.** Tell the client that swelling is expected late in pregnancy
- C.** Arrange prompt evaluation of the client
- D.** Suggest elevating the hands and rechecking in 1 week

Correct Answer: C

Sudden swelling and persistent headache can indicate a hypertensive disorder of pregnancy and require prompt evaluation. The distractors reflect a common mistake of treating a potentially serious symptom as routine late-pregnancy swelling.

1384. During a prenatal class, a client asks what to do if leg cramps occur at night. Which instruction should the nurse include?

- A.** Point the toes downward and hold them there
- B.** Massage the calf deeply until the pain stops
- C.** Apply a heating pad at the highest setting for 1 hour
- D.** Dorsiflex the foot and straighten the leg

Correct Answer: D

Dorsiflexing the foot with the leg straight helps relieve a typical leg cramp. Pointing the toes can worsen the cramp. Deep massage of the calf is not the best first measure, and high heat for a prolonged time is inappropriate.

1385. A client at 14 weeks of pregnancy says, "I smoke a few cigarettes a day, but cutting down is probably good enough." Which response by the nurse is best?

- A.** Smoking during pregnancy can harm both the client and fetus, so stopping is the safest goal

- B.** Smoking is mainly a concern only during the last month of pregnancy
- C.** Switching to cigarettes with less nicotine removes the risk
- D.** As long as meals are balanced, smoking has little effect on pregnancy

Correct Answer: A

Smoking in pregnancy is associated with harmful effects, so cessation is the safest recommendation. The other options minimize risk or suggest false substitutes for quitting. The trap is accepting partial reduction as fully protective.

1386. A nurse is reviewing prenatal teaching with a client who is 27 weeks pregnant. Which client statement indicates a need for further teaching?

- A.** I should wear my seat belt with the lap belt low across my hips
- B.** I can call the clinic if I develop a fever or painful urination
- C.** If my ankles swell a little by evening, I should assume I have severe preeclampsia
- D.** I should try to keep my prenatal appointments even if I feel fine

Correct Answer: C

Mild dependent ankle swelling later in pregnancy can occur normally and does not by itself mean severe preeclampsia. The other statements reflect correct prenatal teaching. The challenge here is separating common physiologic changes from true warning signs.

1387. A client who is 35 weeks pregnant arrives for a prenatal visit and says, "I think I've just been leaking urine, but it keeps happening and my underwear is wet." Which nursing action is the priority?

- A.** Ask the client to wait until the next visit to see if it stops
- B.** Assess the client promptly for possible rupture of membranes
- C.** Instruct the client to drink less water during the day
- D.** Recommend pelvic floor exercises as the only intervention

Correct Answer: B

Persistent wetness in late pregnancy may indicate rupture of membranes and should be assessed promptly. Assuming it is urinary leakage is a common trap. Fluid restriction and delaying assessment are inappropriate when membrane rupture is possible.

1388. A client at 22 weeks of pregnancy reports a painful, swollen right calf. Which response by the nurse is most appropriate?

- A.** Rest with the leg elevated and mention it at the next prenatal visit
- B.** Massage the area several times a day to improve circulation
- C.** Seek immediate evaluation for the calf pain and swelling
- D.** Apply ice and continue normal activity if the pain improves

Correct Answer: C

Unilateral calf pain and swelling raise concern for a possible thrombus and require immediate evaluation. Massaging the calf may be dangerous, and waiting or self-treating could delay care. This item tests recognition of a potentially serious maternal complication.

1389. A pregnant client at 12 weeks tells the nurse, "I've been vomiting all day and can barely keep liquids down." Which additional finding is most concerning?

- A.** Aversion to the smell of cooking food
- B.** Urine that is dark and scant
- C.** Feeling hungry late in the evening
- D.** Mild breast tenderness

Correct Answer: B

Dark, scant urine suggests dehydration, which is a concerning finding when vomiting is persistent. Food aversions and breast tenderness are common in early pregnancy, and evening hunger is not the priority issue. The key is recognizing evidence of worsening fluid deficit.

1390. At a prenatal visit, a client who is 38 weeks pregnant states, "The baby is moving a lot less today than usual." What is the nurse's best response?

- A.** That is expected because the baby has less room near term

- B.** You should be evaluated today for decreased fetal movement
- C.** Wait 3 days and see whether movement returns to normal
- D.** Drink a caffeinated beverage each day and mention it at your next visit

Correct Answer: B

Decreased fetal movement near term requires prompt evaluation because it can signal fetal compromise. A common trap is dismissing it as normal crowding in late pregnancy. Delaying follow-up is not appropriate when a clear change from usual movement is reported.

Labor care

1391. A client in active labor says, "I feel a strong urge to push." The nurse checks and notes the cervix is 8 cm dilated. What is the nurse's best action?

- A.** Encourage the client to bear down with each contraction
- B.** Instruct the client to pant or blow through contractions
- C.** Place the client flat on the back with legs extended
- D.** Prepare the client for immediate delivery

Correct Answer: B

Panting or blowing helps reduce the urge to push until full cervical dilation occurs, which helps prevent cervical edema or injury. Encouraging pushing at 8 cm is unsafe because the cervix is not yet completely dilated. Immediate delivery preparation may be needed later, but the priority now is to avoid premature pushing.

1392. A laboring client at 39 weeks is receiving oxytocin. Which finding requires the nurse's immediate action?

- A.** Contractions every 2 minutes lasting 95 seconds
- B.** Maternal temperature of 99.0°F (37.2°C)
- C.** Cervical dilation progressing from 4 cm to 5 cm in 2 hours
- D.** Client reports mild nausea during labor

Correct Answer: A

Contractions that are too frequent and prolonged suggest uterine tachysystole, which can reduce fetal oxygenation and requires prompt intervention. The other findings are not as urgent. A low-grade normal temperature, slow but possible labor progress, and mild nausea are less concerning than excessive uterine stimulation during oxytocin infusion.

1393. A client in labor suddenly reports severe abdominal pain that persists between contractions. The nurse notes vaginal bleeding and a change in the fetal heart rate pattern. Which complication should the nurse suspect first?

- A.** Placental abruption
- B.** Normal labor transition
- C.** Braxton Hicks contractions
- D.** Rupture of membranes

Correct Answer: A

Persistent abdominal pain, vaginal bleeding, and fetal heart rate changes are concerning for placental abruption. Transition labor can be intense, but pain typically follows contractions rather than persisting between them. Braxton Hicks contractions are irregular and not associated with significant bleeding or fetal distress. Rupture of membranes alone does not explain this cluster of findings.

1394. A client in labor asks why the nurse keeps encouraging side-lying instead of lying flat on the back. Which response by the nurse is best?

- A.** It keeps the contractions from becoming too strong
- B.** It helps prevent compression of major blood vessels and can improve blood flow to the fetus
- C.** It will shorten the second stage of labor in every client
- D.** It guarantees the baby will move into the best position for birth

Correct Answer: B

Side-lying can reduce pressure from the uterus on major blood vessels, improving maternal circulation and uteroplacental blood flow. It does not reliably weaken contractions, guarantee shorter labor, or ensure fetal repositioning. The best answer explains the physiologic reason for avoiding supine positioning.

1395. A client in labor has spontaneous rupture of membranes. The fluid is greenish-brown. What is the nurse's priority interpretation?

- A. This is expected bloody show
- B. The client is fully dilated
- C. Meconium is present and the fetus needs closer assessment
- D. The membranes probably did not rupture completely

Correct Answer: C

Greenish-brown amniotic fluid suggests meconium staining, which requires closer fetal assessment because it may be associated with fetal stress. Bloody show is typically blood-tinged mucus rather than green-brown fluid. Fluid color does not confirm complete dilation, and the description is not most consistent with incomplete rupture.

1396. A client is in the second stage of labor. Which nursing action is most appropriate to promote effective pushing?

- A. Coach the client to push only when feeling the urge and during contractions
- B. Tell the client to hold her breath for as long as possible with every push
- C. Delay pushing until contractions stop to conserve energy
- D. Encourage continuous pushing between contractions

Correct Answer: A

Pushing with contractions and following the body's urge is effective and supports oxygenation better than prolonged breath-holding. Continuous pushing between contractions is ineffective and exhausting. Waiting until contractions stop would not help fetal descent. The main trap is choosing a forceful approach that sounds productive but can be less effective and more fatiguing.

1397. A laboring client receiving an epidural anesthetic becomes hypotensive. Which nursing action should the nurse take first?

- A. Increase the oxytocin infusion rate
- B. Turn the client to a side-lying position
- C. Assist the client to ambulate
- D. Encourage the client to bear down

Correct Answer: B

A side-lying position is an immediate intervention that can improve venous return and uteroplacental perfusion after epidural-related hypotension. Increasing oxytocin could worsen fetal compromise. Ambulation is unsafe in hypotension, and bearing down does not address the cause. This question tests recognition of the safest first response.

1398. During labor, the fetal heart rate is 90/min for 70 seconds immediately after a contraction. What is the nurse's best initial action?

- A. Continue to observe because this is an expected variation
- B. Reposition the client and assess for improvement
- C. Have the client begin pushing harder with the next contraction
- D. Document the finding and reassess in 1 hour

Correct Answer: B

A fetal heart rate of 90/min after a contraction is concerning and requires prompt nursing intervention, with repositioning as an appropriate initial action to improve fetal oxygenation. Simply observing or waiting an hour delays care. Encouraging harder pushing could worsen fetal stress. The key is recognizing a nonreassuring pattern and intervening immediately.

1399. A primigravida in labor asks what "effacement" means. Which explanation by the nurse is best?

- A. It is how far the baby has moved down into the pelvis
- B. It is how much the cervix has opened in centimeters
- C. It is the thinning and shortening of the cervix
- D. It is the rupture of the amniotic sac before birth

Correct Answer: C

Effacement refers to thinning and shortening of the cervix. Descent is how far the fetus has moved into the pelvis, dilation is cervical opening in centimeters, and rupture of membranes refers to the amniotic sac breaking. This is a common vocabulary distinction in labor care.

1400. A client at 41 weeks' gestation is in labor. During assessment, the nurse notes recurrent variable decelerations after rupture of membranes. Which additional finding would most strongly support umbilical cord compression as the cause?

- A. A soft uterus between contractions
- B. A gush of bright red bleeding without pain
- C. A sudden change in fetal heart rate after membrane rupture
- D. Maternal blood pressure of 118/72 mm Hg

Correct Answer: C

Variable decelerations are commonly associated with cord compression, and a sudden change after rupture of membranes strengthens that suspicion because less cushioning fluid surrounds the cord. A soft uterus between contractions is not specific. Painless bright red bleeding suggests a different complication. Normal maternal blood pressure does not explain the decelerations.

1401. The nurse is caring for a client in labor who is breathing rapidly and reports tingling in the fingers and around the mouth. Which nursing action is best?

- A. Coach the client to slow breathing with a paced pattern
- B. Increase the room temperature
- C. Instruct the client to push with each contraction
- D. Place the client flat and elevate both legs

Correct Answer: A

Rapid breathing with tingling suggests hyperventilation. Coached paced breathing helps the client regain control and reduce symptoms. Increasing room temperature does not address the cause. Pushing is not indicated based on these cues, and lying flat may worsen comfort and circulation. The nurse should target the breathing pattern directly.

1402. A client in labor has a prolapsed umbilical cord visible at the vaginal opening. Which action should the nurse take immediately?

- A. Apply gentle traction to replace the cord
- B. Use a sterile gloved hand to relieve pressure off the cord while calling for help
- C. Position the client supine and prepare for a vaginal birth
- D. Delay intervention until the next contraction pattern is assessed

Correct Answer: B

A visible prolapsed cord is an obstetric emergency. The nurse should immediately reduce pressure on the cord with a sterile gloved hand and call for assistance. Pulling on the cord can cause injury, supine positioning can worsen compression, and delaying care risks fetal hypoxia. This is a true immediate-action scenario.

1403. A laboring client says, "My back hurts so much more than my abdomen." Which nursing measure is most appropriate?

- A. Apply firm sacral counterpressure during contractions
- B. Limit movement to prevent fetal descent
- C. Encourage the client to remain flat in bed
- D. Offer solid food to increase strength

Correct Answer: A

Back labor is often relieved by sacral counterpressure during contractions. Limiting movement and lying flat are generally not helpful and may worsen discomfort. Offering solid food is not the best response to the pain complaint and may not be appropriate in labor. The question focuses on comfort measures specific to labor care.

1404. A client in labor who is positive for group B streptococcus asks why IV antibiotics are being given now instead of after birth. Which response by the nurse is best?

- A. The medication helps reduce the newborn's risk of infection during birth
- B. The medication stops labor contractions from becoming too strong
- C. The medication prevents postpartum hemorrhage

D. The medication guarantees the newborn will not need any evaluation after birth

Correct Answer: A

Antibiotics during labor are given to reduce transmission of group B streptococcus to the newborn during birth. They are not used to control contraction strength or prevent hemorrhage, and they do not guarantee that no newborn evaluation will be needed. The best answer explains the timing and purpose clearly.

1405. The nurse is assessing a client in labor. Which finding most strongly suggests transition phase of the first stage of labor?

- A.** Mild, irregular contractions with excitement about the birth
- B.** Contractions becoming stronger with cervix dilated 4 cm
- C.** Restlessness, nausea, and an urge to bear down as dilation nears complete
- D.** Complete cervical dilation with crowning of the fetal head

Correct Answer: C

Transition is the late part of the first stage and is often marked by intense contractions, restlessness, nausea, and a strong urge to push as dilation nears complete. Mild irregular contractions suggest latent labor. Four centimeters is more consistent with earlier active labor. Crowning occurs in the second stage, not transition.

1406. A nurse is monitoring a client in labor who has just had a spontaneous rupture of membranes. Which action is the priority immediately after the rupture?

- A.** Assist the client to ambulate
- B.** Assess the fetal heart rate
- C.** Obtain the client's temperature
- D.** Start a contraction pattern chart

Correct Answer: B

The priority after rupture of membranes is to assess the fetal heart rate because the sudden loss of fluid can be associated with umbilical cord prolapse or compression, which may quickly affect fetal oxygenation. Taking the temperature is also important later to monitor for infection, but it is not the first priority in the immediate moment after the membranes rupture.

1407. A client in labor is having contractions every 2 to 3 minutes. On vaginal examination, the cervix is 5 cm dilated, 80% effaced, and the presenting part is at 0 station. How should the nurse interpret these findings?

- A.** The client is in the latent phase of the first stage of labor
- B.** The client is in the active phase of the first stage of labor
- C.** The client is in the second stage of labor
- D.** Birth is expected immediately

Correct Answer: B

Cervical dilation of 5 cm with regular contractions indicates the active phase of the first stage of labor. The second stage begins with complete dilation at 10 cm. A common trap is choosing birth is expected immediately because the fetus is at 0 station, but station alone does not mean delivery is imminent.

1408. A laboring client at term says, "I think my water broke." Which observation best supports that the fluid is amniotic fluid rather than urine?

- A.** The fluid is straw-colored and continues to leak
- B.** The client reports a strong urge to void
- C.** The fluid is dark yellow after several hours without drinking
- D.** The leakage stops when the client empties her bladder

Correct Answer: A

Amniotic fluid is often clear or straw-colored and may continue to leak because it is not controlled by the bladder sphincter. Urine leakage is more likely to stop after voiding. The best clue here is the ongoing leakage of straw-colored fluid.

1409. A client in labor is using patterned breathing. Which finding best indicates that the breathing technique is effective?

- A.** The client reports less anxiety and is able to focus during contractions
- B.** The client has completely eliminated contraction pain
- C.** The contractions become shorter and less intense
- D.** The cervix dilates 1 cm with each contraction

Correct Answer: A

Breathing techniques help promote relaxation, focus, and coping with labor pain; they do not eliminate pain completely or directly change contraction strength or cervical dilation with each contraction. The correct sign of effectiveness is improved coping and decreased anxiety.

1410. The nurse is caring for a client in labor whose contractions have become less frequent and less intense after the client received a sedative analgesic. Which interpretation is most appropriate?

- A. The client has entered the transition phase
- B. The medication may be decreasing uterine activity
- C. The fetus has descended into the pelvis
- D. The client is ready for immediate pushing

Correct Answer: B

Sedative analgesics can decrease the client's perception of pain and may also reduce uterine activity, leading to less frequent or less intense contractions. The findings do not suggest transition or readiness to push. A common trap is assuming stronger labor progression, but the pattern described shows labor slowing rather than advancing.

1411. A client in labor has this fetal heart rate pattern: baseline 140/min, moderate variability, and an increase to 155/min lasting 20 seconds with fetal movement. How should the nurse interpret this finding?

- A. It is a reassuring sign of fetal well-being
- B. It suggests uteroplacental insufficiency
- C. It indicates fetal acidemia
- D. It is evidence of fetal sleep requiring intervention

Correct Answer: A

An acceleration with moderate variability is a reassuring finding and suggests adequate fetal oxygenation. Uteroplacental insufficiency is more concerning with late decelerations, not accelerations. The likely trap is overthinking any rate change as abnormal, but a brief increase associated with movement is expected and reassuring.

1412. A client in labor says, "I want pain medicine, but I'm afraid it will make my baby stop breathing." Which response by the nurse is best?

- A. Pain medication is avoided in labor because it always harms the newborn
- B. If medication is used, the team considers the type and timing to reduce effects on the newborn
- C. You should wait until after birth because medication has no role during labor
- D. An epidural is the only option that affects only the baby and not you

Correct Answer: B

The best response is accurate and balanced: analgesia may be used during labor, and the team considers type, dose, and timing to reduce newborn effects. Absolute statements such as always harms or no role during labor are incorrect. Choice D is also inaccurate because epidural anesthesia affects the client and still requires monitoring.

1413. The nurse notes recurrent late decelerations in the fetal heart rate of a client in labor. The oxytocin infusion is discontinued. Which additional nursing action is most appropriate?

- A. Place the client in a side-lying position
- B. Encourage the client to begin pushing
- C. Assist the client to the bathroom
- D. Prepare the client for oral fluids

Correct Answer: A

Late decelerations suggest decreased uteroplacental perfusion. After stopping oxytocin, the nurse should improve perfusion by repositioning the client laterally. Pushing can worsen fetal stress if the client is not ready, and bathroom or oral fluids do not address the immediate concern.

1414. A nurse is performing Leopold maneuvers on a client in labor. The nurse feels a smooth, firm surface on one side of the abdomen and small irregular parts on the other side. What is the main purpose of this assessment finding?

- A. To estimate the amount of amniotic fluid
- B. To determine fetal position

- C. To measure cervical dilation
- D. To confirm placental location

Correct Answer: B

Leopold maneuvers help identify fetal lie, presentation, and position by locating the back and the small parts. Feeling a smooth, firm surface on one side likely indicates the fetal back. Cervical dilation is determined by vaginal examination, not abdominal palpation.

1415. A client is in labor with a fetus in occiput posterior position. Which labor pattern is the nurse most likely to anticipate?

- A. Minimal discomfort with rapid fetal descent
- B. Intense suprapubic pressure only
- C. Longer labor with significant back discomfort
- D. Immediate onset of shoulder dystocia

Correct Answer: C

Occiput posterior positioning is commonly associated with back labor and may contribute to a longer, more difficult labor because fetal rotation and descent can be less efficient. Shoulder dystocia cannot be predicted simply from occiput posterior position. The likely trap is choosing suprapubic pressure, but back pain is more characteristic.

1416. A client in labor has been pushing for 1 hour with good effort. The fetal head remains visible at the perineum between contractions and then slips back when the contraction ends. How should the nurse interpret this finding?

- A. This is crowning and birth is unavoidable within seconds
- B. This suggests the fetus is retracting normally after birth
- C. This indicates descent is occurring, but the head has not yet crowned
- D. This is evidence of shoulder dystocia

Correct Answer: C

If the fetal head recedes between contractions, descent is occurring but true crowning has not yet occurred. Crowning means the head remains visible and no longer slips back. A common trap is calling any visible head crowning, but persistence at the perineum is the key difference.

1417. A client in labor develops a temperature of 38.3°C (100.9°F), maternal pulse 112/min, uterine tenderness, and fetal tachycardia. Which complication should the nurse suspect?

- A. Chorioamnionitis
- B. Placenta previa
- C. Precipitous labor
- D. Normal transition phase

Correct Answer: A

Maternal fever, tachycardia, uterine tenderness, and fetal tachycardia are concerning for intra-amniotic infection such as chorioamnionitis. Placenta previa is primarily associated with painless bleeding, and transition does not explain fever and uterine tenderness.

1418. A nurse is caring for a client receiving oxytocin for augmentation of labor. Which contraction pattern should the nurse identify as tachysystole requiring prompt intervention?

- A. Three contractions in 10 minutes, each lasting 50 seconds
- B. Four contractions in 10 minutes with moderate resting tone
- C. Five contractions in 10 minutes with 60-second duration and adequate rest between
- D. Six contractions in 10 minutes over a 30-minute period

Correct Answer: D

Uterine tachysystole is generally identified as more than five contractions in 10 minutes, averaged over 30 minutes. Six contractions in 10 minutes meets that threshold and requires prompt response, especially with oxytocin. The trap is choice C, but five in 10 minutes does not exceed the usual threshold given here.

1419. A client in labor with an epidural reports ringing in the ears and a metallic taste in the mouth shortly after medication administration. What should the nurse do first?

- A. Reassure the client that this is an expected effect of pain relief
- B. Stop the procedure and notify the anesthesia provider immediately
- C. Place the client flat on the back and continue to monitor

D. Encourage slow breathing through the next contraction

Correct Answer: B

Ringling in the ears and a metallic taste can indicate systemic local anesthetic toxicity or inadvertent intravascular administration and require immediate attention from the anesthesia provider. Reassurance without action is unsafe. Placing the client flat may worsen maternal hypotension and does not address the possible cause.

1420. A nurse is caring for a client in active labor who suddenly says, "I need to push right now." The fetal heart rate is stable. What is the nurse's priority action?

- A.** Instruct the client to begin pushing with the next contraction
- B.** Check the cervix for complete dilation before encouraging pushing
- C.** Place the client in a supine position and apply oxygen by mask
- D.** Prepare the client for immediate transfer to the postpartum unit

Correct Answer: B

The priority is to assess whether the cervix is fully dilated before the client begins pushing. Pushing against an incompletely dilated cervix can cause cervical edema and delay labor progress. Choice A is a common trap because the urge to push often suggests second stage, but the nurse should verify full dilation first. Choice C is not indicated with a stable fetal heart rate, and Choice D is inappropriate because the client is still in labor.

Stable postpartum care

1421. A nurse is caring for a client 12 hours after a vaginal birth. Which finding is expected in a stable postpartum client?

- A.** A firm uterine fundus at the level of the umbilicus
- B.** A boggy uterus with heavy lochia and clots
- C.** A temperature of 39.2°C (102.6°F)
- D.** Foul-smelling vaginal discharge

Correct Answer: A

A firm fundus at or near the umbilicus is an expected finding in the early postpartum period. A boggy uterus with heavy lochia suggests uterine atony and risk for hemorrhage. A high fever and foul-smelling discharge are concerning for infection, not stable recovery.

1422. A postpartum client reports needing help with perineal discomfort after an episiotomy. Which nursing instruction is most appropriate?

- A.** Apply heat continuously to the area for the first 24 hours
- B.** Use a peri bottle with warm water after voiding
- C.** Avoid changing perineal pads until the end of the shift
- D.** Clean the perineum from back to front

Correct Answer: B

Using a peri bottle with warm water after voiding helps cleanse the area, improves comfort, and reduces irritation. Heat is not used continuously in the first 24 hours; cold is commonly used initially for swelling. Pads should be changed regularly, and perineal care should be done from front to back to reduce infection risk.

1423. A nurse assesses a client on the first postpartum day after a vaginal birth. Which uterine finding requires immediate nursing intervention?

- A.** Fundus firm and midline
- B.** Fundus displaced to the right and soft
- C.** Fundus 1 cm below the umbilicus
- D.** Mild afterpains during breastfeeding

Correct Answer: B

A soft, displaced fundus suggests uterine atony, often related to a full bladder, and increases the risk for postpartum hemorrhage. The nurse should intervene promptly by assessing bladder status, assisting the client to void, and massaging the fundus if indicated. A firm midline fundus, slight descent below the umbilicus, and mild afterpains are expected findings.

1424. A postpartum client says, "I cry for no reason sometimes, but then I feel fine later." Which response by the nurse is best?

- A. "This means you are not bonding well with your baby."
- B. "Mood changes can happen in the first days after birth, but tell us if they get worse or last longer."
- C. "You should avoid holding your baby until you feel normal again."
- D. "This is always a sign of severe depression."

Correct Answer: B

Brief tearfulness and mood swings in the early postpartum period can be consistent with postpartum blues. The best response is supportive, normalizes the experience appropriately, and includes follow-up guidance if symptoms worsen or persist. The other responses are inaccurate, judgmental, or overly absolute.

1425. A nurse is preparing discharge teaching for a stable postpartum client who is bottle-feeding. Which statement by the client indicates correct understanding about breast care?

- A. "I should express milk frequently to reduce fullness."
- B. "I will wear a supportive bra and avoid stimulating my breasts."
- C. "I should take hot showers and massage my breasts every 2 hours."
- D. "I need to pump both breasts on a schedule for the first week."

Correct Answer: B

For a client who is suppressing lactation, wearing a supportive bra and avoiding breast stimulation are appropriate measures. Frequent expression, pumping, and repeated massage stimulate milk production and are more appropriate for establishing or maintaining breastfeeding, not for a bottle-feeding client who wants to minimize engorgement.

1426. A postpartum client who is breastfeeding asks why cramping becomes stronger during feedings. Which explanation should the nurse provide?

- A. Breastfeeding causes dehydration, which leads to uterine pain

- B. The baby's sucking stimulates oxytocin release, which causes uterine contractions
- C. Milk entering the ducts places pressure on the uterus
- D. Feeding positions compress the abdomen and cause ischemic pain

Correct Answer: B

Suckling stimulates release of oxytocin, which helps the uterus contract and return toward its prepregnant size. These contractions can feel like afterpains and may be more noticeable during breastfeeding. The other options do not explain the physiologic cause of stronger cramping during feedings.

1427. A nurse is caring for a stable client 8 hours after cesarean birth. Which action is most effective to reduce the risk of postoperative complications related to immobility?

- A. Encourage early ambulation as tolerated
- B. Limit oral fluids for 24 hours
- C. Keep the client on strict bed rest until bowel sounds return
- D. Place the client in high-Fowler position continuously

Correct Answer: A

Early ambulation helps reduce postoperative complications such as venous stasis, constipation, and delayed recovery. Limiting fluids and prolonged bed rest can increase complications. Positioning alone is not the most effective intervention compared with getting the client moving safely.

1428. A nurse is teaching a postpartum client about lochia progression. Which sequence is correct from earliest to latest?

- A. Lochia alba, lochia serosa, lochia rubra
- B. Lochia rubra, lochia alba, lochia serosa
- C. Lochia rubra, lochia serosa, lochia alba
- D. Lochia serosa, lochia rubra, lochia alba

Correct Answer: C

Normal lochia progresses from rubra to serosa to alba. Rubra is the early red discharge, serosa is the later pink or brown discharge, and alba is the later yellow-white discharge. The distractors test confusion about timing and sequence.

1429. A postpartum client asks when to notify the provider after discharge. Which statement by the client shows a need for further teaching?

- A. "I will call if I soak a pad in an hour."
- B. "I will call if I develop calf pain or swelling."
- C. "I will call if my lochia gradually lightens over time."
- D. "I will call if I have a fever or foul-smelling discharge."

Correct Answer: C

Lochia that gradually lightens over time is an expected postpartum finding and does not by itself require calling the provider. Soaking a pad in an hour, calf pain or swelling, fever, and foul-smelling discharge are warning signs that need prompt evaluation.

1430. A nurse is assessing four stable postpartum clients. Which client should the nurse see first?

- A. A client with a boggy fundus and moderate lochia rubra
- B. A client who asks for help with breastfeeding latch
- C. A client who reports fatigue after interrupted sleep
- D. A client with mild perineal soreness relieved by ice packs

Correct Answer: A

A boggy fundus can indicate uterine atony and possible progression to postpartum hemorrhage, so this client takes priority. The other clients have expected or nonurgent needs. Even though all are described as stable, the abnormal uterine finding requires immediate reassessment and intervention.

1431. A postpartum client says, "I am afraid to urinate because it burns near my stitches." Which nursing action is best?

- A. Tell the client to wait until the sensation improves
- B. Encourage the client to drink less fluid for the rest of the day
- C. Suggest using warm water over the perineum while voiding
- D. Insert an indwelling catheter to prevent discomfort

Correct Answer: C

Using warm water over the perineum while voiding can reduce stinging and promote more complete emptying. Delaying voiding can lead to bladder distention, which may interfere with uterine tone. Reducing fluids and placing a catheter are not appropriate first-line measures for this common postpartum discomfort.

1432. A nurse is reinforcing teaching for a stable postpartum client about resuming activity at home. Which instruction is most appropriate?

- A. Remain on bed rest for the first week to promote healing
- B. Gradually increase activity and rest when tired
- C. Start vigorous abdominal exercise daily as soon as you get home
- D. Avoid climbing stairs for 6 months

Correct Answer: B

A stable postpartum client should generally resume activity gradually and balance it with adequate rest. Prolonged bed rest is unnecessary and may increase complications, while vigorous exercise immediately after discharge may be excessive. The stair restriction is unrealistically broad and not standard teaching for stable recovery.

1433. A client is 2 days postpartum and reports unilateral calf tenderness. The affected area feels warm. Which nursing action is the priority?

- A. Massage the calf to improve circulation
- B. Encourage the client to walk in the hallway
- C. Notify the provider and limit manipulation of the extremity
- D. Apply sequential compression devices to both legs immediately

Correct Answer: C

Unilateral calf tenderness and warmth raise concern for venous thrombosis. The priority is to notify the provider and avoid massaging or unnecessarily manipulating the limb, which could dislodge a clot. Encouraging ambulation before evaluation is unsafe. Compression devices are preventive measures and are not the first response when thrombosis is suspected.

1434. A nurse is reviewing breastfeeding teaching with a first-time postpartum client. Which statement indicates a need for further teaching?

- A. "I should bring the baby to my breast, not lean my breast toward the baby."
- B. "I should expect some uterine cramping while breastfeeding."
- C. "If my nipples are sore, I should break the suction gently before removing the baby."
- D. "If breastfeeding hurts throughout the feeding, that means the latch is always correct."

Correct Answer: D

Pain that continues throughout a feeding commonly suggests an ineffective latch or positioning problem, so this statement shows misunderstanding. Bringing the baby to the breast, expecting some cramping, and gently breaking suction before removing the infant are appropriate teaching points.

1435. A nurse is preparing to discharge a stable postpartum client who had a vaginal birth yesterday. Which finding would most strongly suggest the need to delay discharge and notify the provider?

- A. The client reports afterpains while nursing
- B. The client has a pulse of 58/min and no other symptoms
- C. The client saturates a perineal pad in 30 minutes
- D. The client reports mild fatigue after caring for the newborn overnight

Correct Answer: C

Saturating a perineal pad in 30 minutes is a significant warning sign for postpartum hemorrhage and requires prompt evaluation. Mild bradycardia can be seen postpartum if the client is otherwise asymptomatic. Afterpains with nursing and fatigue after a disrupted night are expected postpartum experiences.

1436. A nurse is reinforcing discharge teaching for a stable postpartum client who plans to breastfeed. Which statement by the client shows correct understanding of how to prevent nipple soreness?

- A. I should wash my nipples with soap after each feeding.

B. I should remove my baby quickly if the feeding becomes uncomfortable.

C. I should break suction with my finger before removing the baby from the breast.

D. I should limit each feeding to 5 minutes on each side.

Correct Answer: C

Breaking suction before removing the baby helps prevent nipple trauma. Soap can dry and irritate the nipples, abruptly pulling the baby off can worsen soreness, and limiting feeds to a set short time does not promote effective emptying or comfort.

1437. A stable postpartum client is 6 hours after a vaginal birth and has not voided. The fundus is firm but displaced to the right of midline. Which action should the nurse take first?

- A. Assist the client to the bathroom to void
- B. Massage the fundus until it becomes midline
- C. Increase the client's oral fluids
- D. Document the finding as expected

Correct Answer: A

A uterus displaced to the right commonly suggests a full bladder. Assisting the client to void is the first action because bladder distention can interfere with normal uterine position and postpartum recovery. Massage is not indicated for a firm fundus, oral fluids are helpful but not the priority, and this should not simply be documented as expected.

1438. During a home-care follow-up call, a client who is 4 days postpartum says, "My discharge is pinkish-brown now, and it has a fleshy smell but not a bad odor." Which response by the nurse is best?

- A. That change is consistent with normal lochia progression.
- B. You need to start massaging your uterus every hour.
- C. That means you are developing a postpartum infection.
- D. You should expect the discharge to stop completely by today.

Correct Answer: A

Lochia normally changes from red to pinkish or brownish before later becoming lighter. A fleshy, nonfoul odor is expected. Infection is more concerning with foul odor, fever, or other abnormal findings. Routine hourly fundal massage is not indicated at home, and lochia does not stop completely by day 4.

1439. A postpartum client had a vaginal birth earlier today and asks why the nurse keeps encouraging food high in fiber and increased fluid intake. Which explanation is best?

- A.** It helps reduce afterpains during breastfeeding.
- B.** It helps prevent constipation and decreases strain on the perineum.
- C.** It keeps lochia from becoming too heavy.
- D.** It prevents urinary retention caused by perineal edema.

Correct Answer: B

Fiber and fluids are encouraged postpartum to help prevent constipation, which is common after birth and can increase discomfort, especially when the perineum is sore. These measures do not directly reduce afterpains, control lochia amount, or specifically prevent urinary retention.

1440. A nurse is assessing a stable client on the first day after a cesarean birth. Which finding should the nurse report to the provider?

- A.** A small amount of serosanguineous drainage on the abdominal dressing
- B.** Incisional pain that improves after prescribed analgesic medication
- C.** Absent bowel sounds in all four quadrants with abdominal distention
- D.** Mild fatigue after walking in the hallway

Correct Answer: C

Absent bowel sounds with abdominal distention may indicate postoperative ileus and should be reported. A small amount of serosanguineous drainage can be expected early after surgery, incisional pain relieved by medication is common, and mild fatigue with ambulation is also expected.

1441. A stable postpartum client asks when Kegel exercises may be helpful. Which response by the nurse is best?

- A.** They strengthen pelvic floor muscles and can improve muscle tone.
- B.** They help the uterus return to its prepregnant size faster.
- C.** They stop lochia from lasting more than a few days.
- D.** They prevent breast engorgement during milk production.

Correct Answer: A

Kegel exercises strengthen the pelvic floor and can help improve tone after childbirth. They do not directly speed uterine involution, stop normal lochia, or affect breast engorgement.

1442. A nurse is reinforcing teaching to a postpartum client about rubella immunization. The client is not immune to rubella and plans to breastfeed. Which statement by the nurse is most appropriate?

- A.** You should avoid the vaccine until breastfeeding is completed.
- B.** You can receive the vaccine during the postpartum period even if you are breastfeeding.
- C.** The vaccine is only given if you had a cesarean birth.
- D.** You should delay the vaccine until your 6-week follow-up visit because it is unsafe right after birth.

Correct Answer: B

A nonimmune postpartum client can receive rubella vaccination during the postpartum period, including when breastfeeding. The vaccine is not limited by mode of birth, and delaying solely because of breastfeeding is incorrect.

1443. A nurse is caring for a stable postpartum client who reports breast fullness and tenderness on day 3 after birth. The client is formula-feeding. Which instruction is most appropriate?

- A.** Apply warm packs and express milk every 2 hours.
- B.** Wear a supportive bra and use ice packs for comfort.
- C.** Stimulate the nipples several times a day to relieve pressure.

D. Take long hot showers aimed directly at the breasts.

Correct Answer: B

For a formula-feeding client with engorgement discomfort, a supportive bra and cold applications can promote comfort while minimizing stimulation. Warmth, expressing milk, and nipple stimulation all encourage milk production and can prolong engorgement.

1444. A nurse is reviewing perineal care with a stable postpartum client who has lochia. Which client statement indicates correct understanding?

- A.** I will wipe from front to back after using the toilet.
- B.** I will cleanse the area once daily unless I have a bowel movement.
- C.** I will use tampons until the bleeding stops.
- D.** I will avoid changing pads often so I can measure bleeding better.

Correct Answer: A

Wiping from front to back helps reduce contamination of the perineal area. Postpartum clients should perform frequent perineal care, use peripads rather than tampons, and change pads regularly to promote hygiene and allow assessment.

1445. A client is 24 hours postpartum and says, "I feel overwhelmed when the baby cries, and I am not sure I can do this right." The client is attentive to the newborn and answers questions appropriately. Which nursing response is best?

- A.** You should not feel this way after having a healthy baby.
- B.** These feelings can happen as you adjust; let's talk about your concerns and supports.
- C.** You are showing clear signs of a severe postpartum mood disorder.
- D.** Try to ignore these feelings because they usually disappear completely by tonight.

Correct Answer: B

A supportive, therapeutic response is best. Early postpartum emotional lability and feelings of being overwhelmed can occur during adjustment, and the nurse should encourage discussion and assess support systems. Dismissing the feelings, shaming the client, or labeling the condition as severe without additional evidence is inappropriate.

1446. A nurse is assessing a stable postpartum client 2 hours after birth. Which finding is most concerning and requires immediate follow-up?

- A.** Pulse rate of 58/min
- B.** Temperature of 37.4°C (99.3°F)
- C.** Saturating a perineal pad in 15 minutes
- D.** Mild shivering

Correct Answer: C

Saturating a perineal pad in 15 minutes suggests excessive bleeding and requires immediate follow-up. Mild bradycardia can occur postpartum, a temperature of 37.4°C is not alarming, and mild shivering may occur after birth. The severe bleeding pattern is the priority concern.

1447. A postpartum client who is breastfeeding tells the nurse, "My baby feeds for a long time, but my breasts still feel very full afterward." Which instruction is most appropriate?

- A.** Offer the breast less often so more milk can collect between feedings.
- B.** Check that the baby has a deep latch and audible swallowing during feeds.
- C.** Switch to formula for 24 hours to rest the nipples.
- D.** Drink less fluid to reduce milk production.

Correct Answer: B

Persistent fullness after feeds suggests ineffective milk transfer, so the nurse should assess for a deep latch and signs such as audible swallowing. Feeding less often can worsen engorgement, temporary formula substitution may interfere with breastfeeding, and reducing fluids is not an appropriate method to manage this problem.

1448. A nurse is reviewing postpartum self-care with a client who had a cesarean birth. Which client statement indicates correct understanding of incision care at home?

- A. I should report redness, separation, or drainage from the incision.
- B. I should cover the incision with a heating pad if it becomes sore.
- C. I should scrub the incision daily with antiseptic solution.
- D. I should expect pus-like drainage for the first week.

Correct Answer: A

Redness, separation, or drainage can indicate an incision problem and should be reported. Heating pads may not be appropriate over a surgical incision, scrubbing can damage healing tissue, and pus-like drainage is not an expected finding.

1449. A nurse is planning care for a stable postpartum client on the day after birth. The client says she becomes dizzy when standing. Which nursing action is best?

- A. Encourage the client to ambulate independently to build tolerance.
- B. Instruct the client to rise slowly and ask for assistance before walking.
- C. Limit oral fluids until the dizziness resolves.
- D. Keep the client on strict bed rest for the remainder of the day.

Correct Answer: B

Postpartum clients can experience dizziness with position changes, so the safest action is to teach slow position changes and request assistance with ambulation. Independent walking despite dizziness raises fall risk. Limiting fluids is not helpful, and strict bed rest is unnecessary for a stable client unless another problem is present.

1450. A client is 1 week postpartum and calls the clinic. Which report is most concerning?

- A. Mild fatigue that improves after resting
- B. Scant pink-brown lochia
- C. A sudden increase in bright red bleeding after several days of lighter flow
- D. Breast fullness before feedings

Correct Answer: C

A sudden return to bright red bleeding after lochia had become lighter can indicate delayed postpartum hemorrhage or overexertion and requires prompt evaluation. Mild fatigue, scant pink-brown lochia, and breast fullness before feedings are more consistent with expected postpartum findings.

Developmental milestones

1451. A nurse is assessing developmental progress during a well-child visit. Which milestone is most expected for a 2-month-old infant?

- A. Rolls from back to abdomen
- B. Social smile appears
- C. Uses a pincer grasp
- D. Sits without support

Correct Answer: B

A social smile is an expected milestone around 2 months. Rolling, pincer grasp, and sitting without support occur later in infancy. A common trap is choosing a motor milestone that belongs to an older infant rather than an early social milestone.

1452. The parent of a 6-month-old asks what new skill to look for next. Which behavior is most appropriate for this age?

- A. Transfers an object from one hand to the other
- B. Walks independently across the room
- C. Speaks in two-word phrases
- D. Draws a circle

Correct Answer: A

By about 6 months, infants commonly transfer objects between hands. Independent walking, two-word phrases, and drawing a circle are expected much later. The best answer matches a typical mid-infancy fine motor milestone.

1453. During a clinic visit, a nurse asks the parent of a 12-month-old about language development. Which finding is most expected?

- A. Uses three-word sentences
- B. Says one or two simple words with meaning
- C. Names four colors correctly
- D. Reads simple picture books independently

Correct Answer: B

Around 12 months, a child often says one or two words such as "mama" or "bye" with meaning. Three-word sentences, color naming, and independent reading are far beyond this age. The question focuses on realistic language expectations for a 1-year-old.

1454. A nurse is screening a 4-year-old for developmental progress. Which task is most consistent with expected development?

- A. Copies a circle
- B. Hops on one foot
- C. Builds a tower of two blocks
- D. Uses only single words to communicate

Correct Answer: B

A 4-year-old is expected to have improved gross motor coordination, including hopping on one foot. Copying a circle is more typical earlier, while building only a two-block tower and using only single words are below expected ability for this age.

1455. A 9-month-old infant is brought to the clinic for a routine visit. Which finding should the nurse recognize as the greatest developmental concern?

- A. The infant does not yet walk alone
- B. The infant does not use a pincer grasp
- C. The infant does not sit without support
- D. The infant does not stack two blocks

Correct Answer: C

By 9 months, sitting without support is expected. Not doing so is a more significant concern than not walking alone or not stacking blocks, which are later skills. A pincer grasp typically develops closer to the end of the first year, so its absence at 9 months is less concerning than inability to sit independently.

1456. Which statement by the parent of an 18-month-old best suggests expected toddler development?

- A. My child speaks in complete sentences.
- B. My child can throw a ball and say several single words.
- C. My child reads familiar words from flash cards.
- D. My child rides a two-wheel bicycle without help.

Correct Answer: B

At about 18 months, toddlers commonly have a small but growing vocabulary and can perform simple gross motor tasks such as throwing a ball. Complete sentences, reading words, and riding a two-wheel bicycle are not expected at this age.

1457. A nurse evaluates several children during health screening. Which child demonstrates the most age-appropriate fine motor skill?

- A. A 10-month-old who uses a crude pincer grasp to pick up a cereal piece
- B. A 6-month-old who draws a stick figure
- C. A 2-year-old who ties shoelaces independently
- D. A 3-year-old who writes in cursive

Correct Answer: A

A crude pincer grasp near 9 to 10 months is expected. Drawing a stick figure, tying shoelaces, and writing in cursive are advanced and not age-appropriate for the listed children. The key is matching the fine motor task to a realistic age.

1458. The nurse is teaching parents about normal play behavior. Which type of play is most typical for a toddler?

- A. Cooperative play with organized rules
- B. Parallel play beside other children
- C. Competitive team play
- D. Complex board games with strategy

Correct Answer: B

Toddlers most commonly engage in parallel play, playing beside rather than fully with other children. Cooperative and competitive rule-based play develops later. A common trap is choosing the more social option instead of the age-appropriate stage of play.

1459. A nurse is concerned about possible developmental delay. Which finding in a 3-year-old requires follow-up?

- A. The child cannot pedal a tricycle.
- B. The child cannot copy a circle.
- C. The child does not speak in short sentences.
- D. The child still needs help dressing.

Correct Answer: C

By 3 years, children are expected to use short sentences. Absence of this language skill warrants follow-up. Pedaling a tricycle and copying a circle are often expected around this age but may vary somewhat; needing help dressing is also still common. Delayed expressive language is the most concerning finding listed.

1460. During developmental screening, which behavior is most expected from a school-age child rather than a preschooler?

- A. Associates with peer groups and wants approval from friends
- B. Engages mainly in parallel play
- C. Shows separation anxiety when away from parents most of the day
- D. Uses magical thinking to explain illness

Correct Answer: A

School-age children place increasing value on peer relationships and group acceptance. Parallel play is more typical of toddlers, while magical thinking is more associated with younger children. Separation anxiety can occur at different ages but is not the best defining school-age feature here.

1461. A parent asks whether a child's drawing is age-appropriate. Which drawing skill is most expected in a 5-year-old?

- A. Random scribbling only
- B. Copies a straight line only
- C. Draws a person with several body parts
- D. Produces detailed shaded landscapes with perspective

Correct Answer: C

By about 5 years, many children can draw a person with multiple recognizable body parts. Random scribbling and copying only a straight line are earlier skills, while advanced perspective drawing is much later. The correct choice reflects typical fine motor and cognitive development for this age.

1462. A nurse reviews developmental findings for four infants. Which infant finding is the highest priority to report to the provider?

- A. A 2-month-old who does not laugh aloud

- B. A 4-month-old who has not begun to roll from abdomen to back

- C. A 6-month-old who has no head control when pulled to sit

- D. A 9-month-old who does not wave bye-bye

Correct Answer: C

By 6 months, head control should be well established. Lack of head control at this age is a significant red flag and should be reported promptly. Laughing aloud, rolling, and waving can vary somewhat in timing, but persistent poor head control is a more serious concern because it reflects delayed gross motor development.

1463. The nurse is teaching a newly hired practical nurse about expected toddler behavior. Which observation would be most concerning in a 24-month-old?

- A. The child uses a spoon with some spilling.
- B. The child runs with a wide stance.
- C. The child has no meaningful words.
- D. The child resists sharing toys.

Correct Answer: C

By 24 months, a child should have meaningful words and usually combines words into simple phrases. Using a spoon imperfectly, running with less mature balance, and resisting sharing are expected toddler behaviors. The absence of meaningful words is the clearest developmental concern.

1464. A nurse is assessing an 8-year-old child after a school physical. Which finding best reflects normal psychosocial development for this age?

- A. The child prefers fantasy play and believes thoughts cause events.
- B. The child seeks opportunities to complete tasks and gain a sense of competence.
- C. The child is focused mainly on establishing trust with caregivers.
- D. The child shows strong concern about developing a separate adult identity.

Correct Answer: B

An 8-year-old is in a stage characterized by industry, where mastery of skills and competence are important. Fantasy-based causation is more typical of younger children, trust formation is associated with infancy, and identity formation is more closely linked with adolescence.

1465. The nurse is reviewing milestone concerns reported by parents. Which child should be evaluated first for possible developmental delay?

- A. A 15-month-old who is not yet walking independently
- B. A 2-year-old who cannot name colors
- C. A 3-month-old who does not track objects visually
- D. A 4-year-old who cannot print first and last name

Correct Answer: C

By about 3 months, infants should visually track objects. Failure to track may indicate a significant visual or neurologic problem and should be evaluated first. Independent walking at 15 months can still fall within a later normal range, color naming is often later than age 2, and printing a full name is not expected of all 4-year-olds.

1466. At a routine checkup, the nurse asks the parent of a 15-month-old which new behavior has appeared recently. Which parent statement best reflects expected development at this age?

- A. My child stacks two small blocks
- B. My child pedals a tricycle across the yard
- C. My child copies a square with a crayon
- D. My child ties shoe laces without help

Correct Answer: A

A 15-month-old commonly can stack about two blocks. Pedaling a tricycle, copying a square, and tying shoe laces are skills expected at much older ages. A common trap is choosing a skill that sounds active or impressive but is actually too advanced for a young toddler.

1467. The nurse is teaching the parent of a 7-month-old about expected motor development over the next few months. Which skill should the nurse identify as most typical for this age?

- A. Pulling to a standing position
- B. Rolling from back to abdomen
- C. Walking independently across a room
- D. Hopping on one foot

Correct Answer: B

Rolling from back to abdomen is a typical skill for an infant around this age. Pulling to stand usually comes later, independent walking is expected much later, and hopping on one foot is a preschool milestone. The main trap is selecting a milestone that occurs soon after 7 months but is not yet expected.

1468. A practical nurse is contributing to a developmental screening for a 30-month-old child. Which behavior is most expected?

- A. Uses 2- to 3-word phrases
- B. Has no interest in parallel activity with other children
- C. Can state full name, address, and telephone number
- D. Reads simple picture books independently

Correct Answer: A

By about 2.5 years, a child commonly uses 2- to 3-word phrases. Stating a full address and reading independently are far beyond expected development. Toddlers often do show interest in being near other children, especially in parallel play, so complete lack of interest is not the best expected finding. The trap is overestimating language ability at this age.

1469. A nurse is assessing a 6-year-old during a health maintenance visit. Which activity is most consistent with expected developmental ability?

- A. Builds a tower of two blocks
- B. Begins to lose primary teeth
- C. Uses a pincer grasp for finger foods
- D. Sits unsupported for several minutes

Correct Answer: B

Beginning to lose primary teeth is expected in a school-age child around 6 years old. The other options describe infant or early toddler development. A good way to avoid the trap is to quickly sort each choice by age group before selecting the best answer.

1470. The parent of a 10-month-old says, "He still cannot pick up small pieces of cereal with his fingers." Which nurse response is best?

- A. That is expected because the pincer grasp does not usually appear until age 2 years
- B. That can be a concern because a pincer grasp is generally expected by this age
- C. That is normal because finger feeding should not begin until after the first birthday
- D. That is expected because fine motor skills develop after toilet training begins

Correct Answer: B

A pincer grasp is generally expected by around 9 to 10 months, so not being able to pick up small finger foods at 10 months deserves follow-up. The incorrect options either delay the milestone far too long or connect it to unrelated development. The most likely trap is assuming all finger-feeding skills happen well after 1 year.

1471. A nurse is reinforcing teaching with the parent of a 3-month-old infant. Which observation should the nurse expect the parent to report?

- A. The infant smiles socially at familiar people
- B. The infant says three clear words
- C. The infant walks while holding on to furniture
- D. The infant drinks from a cup without spilling

Correct Answer: A

A social smile is a classic developmental finding in early infancy and should be present by about 2 months, so it is appropriate in a 3-month-old. Speaking words, cruising, and drinking from a cup happen much later. The trap is choosing a skill associated with older infants or toddlers rather than early infancy.

1472. During preschool screening, the nurse asks a 4-year-old to perform several tasks. Which result is most consistent with expected development?

- A. Cannot jump with both feet off the floor

- B. Draws a stick figure with several body parts
- C. Uses only single-word responses when speaking
- D. Needs complete assistance to climb stairs

Correct Answer: B

A 4-year-old is expected to draw a simple person with multiple body parts. Inability to jump, only single-word speech, and complete dependence on stairs would be concerning at this age. A common trap is underestimating preschool abilities because the child is still young.

1473. A nurse observes children in a community playgroup. Which behavior is most typical of an 11-month-old infant?

- A. Sits quietly and solves a multi-step board game
- B. Pulls to stand while holding onto furniture
- C. Rides a scooter using alternating feet
- D. Prints first and last name legibly

Correct Answer: B

By about 11 months, many infants can pull to stand while holding furniture. Solving a board game, riding a scooter, and printing a full name are much more advanced developmental tasks. The distractors are plausible daily activities, but they belong to older children.

1474. A parent asks the nurse what self-care skill is reasonable to expect from a healthy 5-year-old. Which response is best?

- A. The child should be able to prepare full meals independently
- B. The child should still require total help with dressing
- C. The child may be able to dress independently except for complex fasteners
- D. The child should consistently manage all hygiene needs without reminders

Correct Answer: C

A 5-year-old often can dress independently, though some complex fasteners may still be difficult. Preparing full meals and managing all hygiene without reminders expect too much independence, while total help with dressing expects too little. The trap is picking an answer that sounds ideally independent but exceeds usual developmental ability.

1475. The nurse is obtaining a developmental history for a 13-year-old. Which finding is most consistent with normal psychosocial development during adolescence?

- A. Prefers only solitary play and shows no interest in peer acceptance
- B. Begins to form an identity while relying strongly on peer relationships
- C. Demonstrates parallel play beside friends without direct interaction
- D. Focuses mainly on developing object permanence

Correct Answer: B

Adolescence is marked by identity formation and strong influence from peer relationships. Parallel play is typical of toddlers, and object permanence is an infancy milestone. The incorrect option about solitary behavior does not best match expected adolescent psychosocial growth.

1476. A nurse is screening an 18-month-old child. Which finding should the nurse identify as requiring further evaluation?

- A. Walks without support
- B. Uses a spoon with some spilling
- C. Does not yet build a tower of six blocks
- D. Has no meaningful words

Correct Answer: D

By 18 months, a child is generally expected to have some meaningful words. Walking and using a spoon with spilling are expected. Not building a tower of six blocks is less concerning because that skill is more advanced than what is required at this age. The trap is focusing on exact block-stacking counts rather than the more important language delay.

1477. The parent of a 4-month-old asks when to expect the infant to sit steadily without support. Which nurse response is most accurate?

- A. Usually by around 8 months
- B. Usually by around 2 months
- C. Usually by around 14 months
- D. Usually by around 24 months

Correct Answer: A

Sitting without support is typically expected at about 8 months. Two months is too early, while 14 and 24 months are much too late for this milestone. This question tests whether the nurse can place a major gross motor milestone in the correct sequence rather than just recognize isolated facts.

1478. A nurse reviews reports from several parents of school-age children. Which child most likely needs referral for developmental follow-up?

- A. A 6-year-old who enjoys rules in simple games
- B. A 7-year-old who can read short sentences
- C. An 8-year-old who cannot tell time on a clock or name the days of the week
- D. A 9-year-old who prefers spending time with close friends

Correct Answer: C

By 8 years old, a child is generally expected to handle basic concepts such as telling time and naming days of the week. Enjoying rules, reading short sentences, and valuing friendships are consistent with school-age development. The challenge here is distinguishing a true delay from normal variation in preferences and interests.

1479. A nurse is helping with a developmental screen for a 9-month-old. Which finding is most concerning?

- A. Transfers a toy from one hand to the other
- B. Sits well without support
- C. Shows no ability to bear weight on the legs when held upright
- D. Stranger anxiety is present

Correct Answer: C

By 9 months, an infant should usually be able to bear some weight on the legs when held upright. Transferring objects, sitting without support, and stranger anxiety are expected findings. The trap is choosing a social behavior like stranger anxiety as abnormal when it is actually normal in late infancy.

1480. During a home visit, the nurse notes that a 3-year-old speaks in short sentences, climbs onto a chair without help, and pedals a tricycle. Which additional finding would be most expected?

- A. Copies a circle
- B. Prints several alphabet letters
- C. Uses mature abstract reasoning about future consequences
- D. Balances on one foot for 20 seconds

Correct Answer: A

A 3-year-old commonly can copy a circle. Printing letters, mature abstract reasoning, and prolonged one-foot balance are more advanced skills associated with older children. The stem includes several age clues, so the best approach is to identify the child's developmental level first and then match the additional expected skill.

Life transitions

1481. A practical nurse is reinforcing teaching with the parents of a 2-month-old infant. Which finding is the best indicator of normal development at this age?

- A. The infant rolls from back to abdomen
- B. The infant smiles in response to a familiar face
- C. The infant uses a pincer grasp to pick up a toy
- D. The infant sits without support for several minutes

Correct Answer: B

A social smile in response to a familiar face is an expected developmental milestone around 2 months. Rolling, using a pincer grasp, and sitting without support occur later. A common trap is choosing a more advanced motor skill that is normal eventually, but not at this age.

1482. During a preschool wellness visit, a parent says, "My 4-year-old still wets the bed some nights. I think something is seriously wrong." Which response by the practical nurse is most appropriate?

- A. "Nighttime bladder control can still be developing at this age."
- B. "You should wake the child every 2 hours to prevent this."

C. "This usually means the child is being defiant."

D. "The child should be dry every night by age 4."

Correct Answer: A

Occasional nocturnal enuresis can still be within expected development in a 4-year-old. The nurse should provide reassurance and normal developmental context. Waking the child every 2 hours is burdensome and not the best first response. Calling it defiance is inaccurate and judgmental. Saying the child should already be dry every night may increase parental anxiety unnecessarily.

1483. The practical nurse is caring for an adolescent who says, "I don't want my parent in the room while I talk." Which action is most appropriate first?

- A. Tell the adolescent that the parent must stay for the entire visit
- B. Ask the parent to leave immediately without explanation
- C. Explore the adolescent's concerns and support a private discussion if appropriate
- D. Document that the adolescent is refusing family involvement

Correct Answer: C

Adolescents often seek increasing independence and privacy. The nurse should first explore the concern and, when appropriate, facilitate private conversation in a supportive way. Forcing the parent to stay can block communication. Removing the parent without discussion may escalate tension. Documenting refusal of family involvement is premature and does not address the immediate need.

1484. A practical nurse is teaching a new mother about expected postpartum emotional changes. Which statement by the mother indicates correct understanding of postpartum blues?

- A. "If I cry easily for a few days, that can happen after birth."
- B. "Feeling tired means I am developing postpartum psychosis."
- C. "Mood changes after birth always mean I am unable to care for my baby."

D. "If I feel emotional, I should stop telling anyone so they do not worry."

Correct Answer: A

Postpartum blues commonly include tearfulness and emotional lability in the early postpartum period. The nurse should reinforce that this can occur and should still be discussed openly. Fatigue alone does not mean psychosis. Mood changes do not automatically mean inability to parent, and avoiding communication is unsafe because worsening symptoms need evaluation.

1485. The daughter of an older adult says, "Since my father retired, he seems withdrawn and says he has no purpose." Which nursing response best addresses this life transition?

- A.** "Loss of purpose after retirement can occur, so let's discuss meaningful daily roles and supports."
- B.** "He should avoid new activities until he adjusts on his own."
- C.** "This is just a normal part of aging and does not need attention."
- D.** "Retirement usually improves mood right away for most people."

Correct Answer: A

Retirement is a major life transition that can affect identity, routine, and social connection. The best response acknowledges the transition and promotes adaptation through purposeful roles and support. Dismissing the concern, waiting passively, or assuming retirement always improves mood fails to address possible psychosocial needs.

1486. A practical nurse is caring for a hospitalized toddler whose parent has just left for work. The child begins crying loudly and reaches toward the door. Which interpretation is most accurate?

- A.** The child is showing separation anxiety
- B.** The child is demonstrating abstract reasoning
- C.** The child is exhibiting manipulative behavior
- D.** The child is displaying delayed language development

Correct Answer: A

Toddlers commonly experience separation anxiety when a parent leaves. Crying, reaching toward the door, and distress are consistent with this developmental response. Abstract reasoning is not expected in a toddler. Labeling the behavior as manipulative is a common trap that ignores normal development. The behavior does not specifically indicate delayed language.

1487. A 15-year-old tells the practical nurse, "My friends matter more to me than my family now." Which interpretation is most appropriate?

- A.** This suggests severe family dysfunction
- B.** This can reflect normal adolescent social development
- C.** This indicates impaired attachment
- D.** This means the adolescent is not emotionally mature

Correct Answer: B

Peer relationships typically become more important during adolescence as identity and independence develop. This does not automatically indicate family dysfunction or impaired attachment. A common trap is pathologizing a normal developmental shift.

1488. The practical nurse is reinforcing teaching for the parents of a school-age child who has started asking detailed questions about death after a grandparent died. Which explanation is best?

- A.** Children this age may begin to understand death as permanent and ask concrete questions
- B.** Questions about death at this age usually indicate a thought disorder
- C.** School-age children are too young to notice changes in family structure
- D.** The parents should avoid discussing death to prevent fear

Correct Answer: A

School-age children increasingly understand the permanence and reality of death, often in concrete terms, and may ask direct questions. Avoiding the topic can increase anxiety. The behavior does not usually indicate a thought disorder, and school-age children are very aware of family changes.

1489. A practical nurse in a clinic is collecting data from a woman who recently gave birth. Which statement requires the most immediate follow-up?

- A. "I cry sometimes when I feel overwhelmed."
- B. "I am tired because the baby wakes often at night."
- C. "I do not feel connected to my baby, and I have thought about hurting myself."
- D. "My partner and I are still adjusting to our new routine."

Correct Answer: C

Thoughts of self-harm and difficulty bonding require immediate follow-up because they may indicate serious postpartum mood disturbance and safety risk. Tearfulness, fatigue, and adjustment to a new routine can occur postpartum, though they still warrant support. The priority is identifying and addressing risk of harm.

1490. An older adult who moved to an assisted living facility 1 week ago says, "I hate it here. I don't even know where my things are anymore." Which nursing action is best?

- A. Explain that most residents stop feeling upset after a few days
- B. Encourage the client to personalize the room and keep familiar routines when possible
- C. Limit family visits until the client becomes less dependent
- D. Discourage discussion of the move to prevent dwelling on losses

Correct Answer: B

Relocation is a major life transition for older adults. Promoting familiarity, control, and routine helps support adaptation. Telling the client to simply get over it, limiting family support, or suppressing discussion can worsen distress. A likely trap is minimizing the emotional impact of relocation.

1491. The practical nurse is discussing puberty with a group of parents. Which change should the nurse describe as an expected developmental event of adolescence?

- A. Complete emotional stability develops early in puberty
- B. Dependence on parents steadily increases throughout adolescence

- C. Rapid growth and development of secondary sex characteristics occur
- D. Concrete thinking replaces all forms of abstract reasoning

Correct Answer: C

Adolescence is marked by puberty-related growth spurts and development of secondary sex characteristics. Emotional stability does not become complete early in puberty, dependence on parents does not steadily increase, and adolescents increasingly develop abstract reasoning rather than losing it.

1492. A practical nurse is caring for a client whose spouse died 3 months ago. Which finding is most concerning and suggests difficulty with adaptation to the life transition?

- A. The client keeps photographs of the spouse nearby
- B. The client reports intermittent sadness when discussing anniversaries
- C. The client says, "There is no reason to keep taking my heart medicine anymore."
- D. The client asks for information about a bereavement support group

Correct Answer: C

Stopping essential medication after the loss of a spouse suggests impaired coping and creates immediate risk to physical health. Keeping photographs, feeling sadness around reminders, and seeking a support group are common grief-related responses. The priority is the statement indicating possible self-neglect and danger.

1493. The practical nurse is observing a parent with a hospitalized 6-month-old infant. Which parent action best supports the infant through this life transition?

- A. Changing caregivers frequently so the infant learns flexibility
- B. Maintaining familiar routines and comforting the infant during care
- C. Avoiding holding the infant during procedures to reduce dependence
- D. Limiting parental presence so the infant adjusts more quickly to staff

Correct Answer: B

Infants benefit from consistent comforting and familiar routines during stressful transitions such as hospitalization. Frequent caregiver changes, reducing parental presence, and avoiding comfort can increase distress. A common trap is assuming less parental involvement promotes faster adjustment, when the opposite is often true in infancy.

1494. A practical nurse is caring for a newly admitted resident in long-term care who repeatedly says, "I just want to go home." Which initial nursing approach is most appropriate?

- A. Tell the resident that the move is permanent and discussion should stop
- B. Reorient the resident briefly, then explore feelings about the move and sources of comfort
- C. Ignore the statement so the resident will not focus on it
- D. Promise the resident that discharge home will occur soon

Correct Answer: B

The best initial approach combines gentle orientation with exploration of feelings and coping needs. This supports adaptation while remaining honest. Telling the resident to stop discussing it, ignoring the concern, or making false promises undermines trust and does not address the transition-related distress.

1495. Parents ask the practical nurse whether their 17-year-old should be included in decisions about managing a newly diagnosed chronic illness. Which response is best?

- A. "Adolescents should be excluded from decisions to avoid stress."
- B. "The health care team should make the decisions because adolescents are too impulsive."
- C. "Including the adolescent supports autonomy, learning, and transition to adult self-management."
- D. "Parents should wait until the adolescent turns 18 before discussing the treatment plan."

Correct Answer: C

Including adolescents in decisions about chronic illness care supports autonomy, responsibility, and transition toward adult self-management. Excluding them can impair adherence and learning. The likely trap is assuming that protecting adolescents from stress means removing them from important health decisions.

1496. A practical nurse is reinforcing teaching with the parent of an 8-month-old infant. Which behavior is the best sign of normal development at this age?

- A. Sits steadily without support and transfers a toy from one hand to the other
- B. Uses three-word sentences to ask for food
- C. Rides a tricycle without assistance
- D. Prints first and last name clearly

Correct Answer: A

Sitting without support and transferring objects hand to hand are expected infant abilities around this age. The other options describe skills seen much later in development: three-word sentences in toddlerhood, tricycle riding in the preschool years, and printing a name in the school-age period.

1497. The parent of a 2-year-old tells the practical nurse, "My child says 'no' to everything and wants to do things alone." Which response by the nurse is most appropriate?

- A. "This usually means your child is not attached to you well enough."
- B. "This behavior commonly reflects a toddler's growing need for autonomy."
- C. "You should expect this only if your child is unusually aggressive."
- D. "This suggests your child is ready for school-based socialization."

Correct Answer: B

Toddlers normally strive for independence and control, so saying 'no' and wanting to do tasks alone fits the developmental task of autonomy. Choice A incorrectly pathologizes normal behavior. Choice C treats a typical phase as aggression. Choice D does not address the developmental meaning of the behavior.

1498. A practical nurse is reinforcing teaching with the family of a child who will start kindergarten next month. Which parental action is most likely to ease this transition?

- A. Let the child stay up later each night to become more flexible
- B. Avoid discussing school so the child will not become worried
- C. Visit the school beforehand and talk through the daily routine
- D. Promise the child that no problems will happen at school

Correct Answer: C

Visiting the school and reviewing what to expect can reduce fear of the unknown and support a smoother school-entry transition. Staying up later disrupts routine, which can worsen adjustment. Avoiding the topic prevents preparation. Promising that no problems will happen is unrealistic and not supportive teaching.

1499. The practical nurse is caring for a client who gave birth 2 days ago and says, "I cry easily, but then I feel okay again." Which additional finding is most consistent with an expected early postpartum adjustment?

- A. Brief mood swings while continuing to care for the infant
- B. Hearing voices telling the client to avoid the infant
- C. Persistent hopelessness with no interest in eating or sleeping
- D. Stating a plan to leave the hospital and never return home

Correct Answer: A

Brief tearfulness and mood swings with otherwise intact functioning are consistent with normal early postpartum adjustment. The other options suggest more serious concerns requiring prompt follow-up, such as psychosis, severe depression, or unsafe behavior.

1500. A practical nurse is caring for a 13-year-old who was diagnosed with type 1 diabetes 6 months ago. The adolescent says, "I hate being different from my friends." Which nursing response is best?

- A. "You need to stop worrying about what your friends think."

B. "Many teens struggle with fitting in, so let's talk about ways to manage care more privately at school and with friends."

C. "Your parents should handle your diabetes care until you're older."

D. "If your glucose stays normal, you probably won't feel this way anymore."

Correct Answer: B

Adolescents often focus strongly on peer acceptance and identity. A response that validates the concern and helps the client problem-solve around real-life social situations is most therapeutic. Choice A dismisses the adolescent's feelings. Choice C undermines growing independence. Choice D oversimplifies an emotional and developmental issue.

1501. An older adult tells the practical nurse, "Since I stopped driving, I feel like I've lost my freedom." Which response is most appropriate?

A. "At your age, giving up driving is something you just have to accept."

B. "It's understandable to grieve that loss. Let's explore other ways you can stay active and get where you want to go."

C. "You should ask your family to make decisions for you from now on."

D. "Most people your age are relieved when they stop driving."

Correct Answer: B

Loss of driving can be a major life transition affecting independence and identity. The best response acknowledges the loss and promotes adaptation through planning and retained autonomy. The other choices are dismissive, stereotyping, or unnecessarily reduce the client's independence.

1502. A practical nurse is reinforcing teaching for the parent of an 11-year-old who recently began menstruating and seems embarrassed to talk about it. Which parental statement indicates a need for further teaching?

A. "I'll answer questions honestly and keep the conversation calm."

B. "I'll explain that body changes can happen at different ages and still be normal."

C. "I'll avoid bringing it up unless my child asks first, because talking about it may increase anxiety."

D. "I'll provide supplies and privacy while staying available for support."

Correct Answer: C

Avoiding the topic can increase confusion and shame during puberty. Supportive, matter-of-fact communication helps normalize this transition. Choices A, B, and D all promote healthy adjustment by providing accurate information, normalization, privacy, and availability.

1503. A practical nurse is caring for a hospitalized school-age child who asks, "Will I still be able to go to soccer after surgery?" Which nursing action best addresses this developmental concern?

A. Tell the child that activity decisions are not important right now

B. Encourage the child to discuss questions about recovery and return to usual activities

C. Redirect the conversation to medication administration

D. Ask the parent to answer so the child does not become overwhelmed

Correct Answer: B

School-age children often focus on competence, routine, and participation in activities. Inviting discussion about recovery and return to normal roles supports coping and understanding. The other options dismiss the concern, avoid the issue, or exclude the child from age-appropriate participation.

1504. The spouse of a client who recently had a colostomy says, "He won't even look at himself anymore." Which nursing approach is most appropriate to support this transition?

A. Encourage gradual participation in care at the client's pace while acknowledging body image concerns

B. Insist that the client manage the appliance independently before discharge

C. Avoid discussing the colostomy until the client appears more cheerful

D. Explain that appearance changes should not matter as long as the surgery worked

Correct Answer: A

A colostomy can create a major body image and role adjustment. A gradual, supportive approach helps the client adapt while building skill and confidence. Choice B is too forceful. Choice C delays needed support. Choice D minimizes the emotional impact of the change.

1505. A practical nurse is caring for the family of a 7-year-old child whose parents are divorcing. Which child behavior is most important for the nurse to recognize as a possible stress response to this life transition?

A. A temporary increase in clinginess and school worries

B. Asking more detailed questions about how the family schedule will change

C. Wanting to keep favorite routines and belongings the same

D. Occasionally expressing sadness about living in two homes

Correct Answer: A

Children undergoing parental divorce may show stress through regression, clinginess, and school concerns. The other options can also occur, but they are generally understandable adjustment responses rather than the clearest indicator of stress affecting functioning. The key is recognizing behavioral change that signals difficulty coping.

1506. A practical nurse is reinforcing discharge teaching with the caregiver of an older adult who has early dementia and recently moved in with family. Which instruction is most appropriate to promote adaptation?

A. Change household routines often so the client learns flexibility

B. Use a consistent daily schedule and place personal items in predictable locations

C. Correct every memory mistake immediately to maintain orientation

D. Limit family involvement to reduce overstimulation

Correct Answer: B

Consistency and predictable placement of items can reduce confusion and support adaptation during the transition to a new living arrangement. Frequently changing routines increases disorientation. Constant correction may increase frustration. Limiting family involvement broadly is not the best general approach.

1507. A practical nurse is contributing to the plan of care for a client who is 10 days postpartum and reports, "I still feel overwhelmed all day, I barely sleep even when the baby sleeps, and I don't enjoy anything." Which interpretation is most appropriate?

- A.** This is an expected postpartum adjustment that will always resolve without intervention
- B.** This suggests the client may be having difficulty adapting and needs prompt follow-up
- C.** This is most likely caused by normal newborn feeding demands only
- D.** This indicates the client has accepted the maternal role well

Correct Answer: B

Persistent overwhelm, poor sleep even when the infant is sleeping, and loss of enjoyment beyond the immediate early postpartum period suggest more than routine adjustment and require prompt follow-up. Choice A is too reassuring, choice C oversimplifies, and choice D contradicts the concerning data.

1508. A practical nurse is caring for a recently widowed older adult who says, "I haven't paid the bills in 40 years. My spouse handled everything." Which nursing action is best?

- A.** Take over the paperwork to prevent the client from becoming upset
- B.** Tell the client that financial matters should not be discussed during grief
- C.** Acknowledge the stress of the new role and help identify support resources and step-by-step planning
- D.** Advise the client to delay all decision-making for at least 1 year

Correct Answer: C

Widowhood often brings emotional loss plus abrupt role changes. The best action supports coping, preserves autonomy, and connects the client with practical resources. Choice A may increase dependence. Choice B avoids a real need. Choice D is too rigid and may interfere with necessary tasks.

1509. A practical nurse is reinforcing teaching for the parents of a 3-year-old who will soon have a new sibling. Which parental statement shows the best understanding of how to support this transition?

- A.** "We will wait until we bring the baby home to explain what is happening."
- B.** "We will involve our child in simple preparations and keep usual routines as consistent as possible."
- C.** "We will tell our child to behave like a big kid and not ask for extra attention."
- D.** "We will avoid mentioning the baby so our child does not become jealous."

Correct Answer: B

Preparing the child ahead of time and keeping routines consistent can reduce stress and jealousy during the transition to a new sibling. Waiting until the baby arrives, discouraging attention-seeking needs, or avoiding discussion all make adjustment harder for a young child.

1510. A practical nurse is reinforcing teaching with the daughter of an older adult who moved to an assisted-living facility 1 week ago. The daughter says, "My mother keeps saying she wants to go home, and she seems more irritable than usual." Which response by the nurse is best?

- A.** "This means the move was a mistake and she should return home right away."
- B.** "These reactions can occur during relocation, so maintain familiar routines and personal items while watching for worsening confusion or depression."
- C.** "Limit visits for a few weeks so she can adjust more quickly without family involvement."
- D.** "Tell her firmly that this is her new home so she stops focusing on the past."

Correct Answer: B

Recent relocation can cause temporary distress, irritability, and repeated requests to go home. The best response supports adaptation by using familiar routines and belongings while also monitoring for signs that go beyond expected adjustment, such as increasing confusion or depression. Choice A is too absolute and premature. Choice C may increase anxiety and isolation. Choice D is dismissive and does not support the client's emotional transition.

Screening and prevention

1511. A practical nurse is reinforcing teaching with a 52-year-old client who asks why regular screening is recommended even though she feels well. Which response is best?

- A.** Screening helps detect disease or risk factors early, before symptoms appear.
- B.** Screening replaces the need for routine health history and physical exams.
- C.** Screening is mainly used after a diagnosis has already been confirmed.
- D.** Screening is only useful for clients who already have a strong family history.

Correct Answer: A

Screening is used to identify disease or risk factors in people who may not yet have symptoms, allowing earlier intervention. Choice B is incorrect because screening does not replace routine assessment. Choice C describes diagnostic evaluation rather than screening. Choice D is too limited; family history may increase risk, but screening is not only for those clients.

1512. During a community health fair, which client should the practical nurse identify as needing immediate counseling about smoking cessation as a disease-prevention measure?

- A.** A 24-year-old who received a tetanus booster last year
- B.** A 31-year-old who jogs 4 times each week
- C.** A 46-year-old who smokes 1 pack of cigarettes daily
- D.** A 67-year-old who wears prescription glasses for reading

Correct Answer: C

Daily cigarette smoking is a major modifiable risk factor for many preventable conditions, so smoking cessation counseling is a high-priority prevention measure. The other choices do not describe a major current preventable risk needing immediate counseling.

1513. A parent asks why the school nurse checks children for vision problems each year. Which explanation by the practical nurse is most accurate?

- A.** Vision screening can identify possible problems that may affect learning and safety.
- B.** Vision screening can determine the exact eyeglass prescription needed.
- C.** Vision screening guarantees that eye disease will be prevented.
- D.** Vision screening is only done when a child reports eye pain.

Correct Answer: A

Vision screening helps identify possible vision deficits early so the child can be referred for further evaluation, which may improve learning and safety. Choice B is incorrect because screening does not establish an exact prescription. Choice C overstates the benefit. Choice D is incorrect because screening is often done before symptoms are reported.

1514. Which statement by a client shows correct understanding of primary prevention?

- A.** I will schedule a follow-up test after my abnormal result is reviewed.
- B.** I plan to stop adding salt to my food to help lower my health risks.
- C.** I need therapy to improve my movement after surgery.
- D.** I should have my wound culture checked to identify the bacteria.

Correct Answer: B

Primary prevention aims to prevent disease before it occurs by reducing risk factors, such as improving diet. Choice A is follow-up after a finding, Choice C is rehabilitation, and Choice D is diagnostic testing, not primary prevention.

1515. A practical nurse is reviewing a client's health habits. Which finding represents a nonmodifiable risk factor that should be considered during screening planning?

- A. Sedentary lifestyle
- B. High-sodium diet
- C. Advancing age
- D. Tobacco use

Correct Answer: C

Advancing age is a nonmodifiable risk factor. Sedentary lifestyle, high-sodium diet, and tobacco use are modifiable because they can potentially be changed through behavior and counseling.

1516. A 58-year-old client says, "I do not have any symptoms, so I don't see the point of checking my blood pressure." Which response by the practical nurse is best?

- A. Blood pressure problems usually cause severe symptoms before they become serious.
- B. Checking blood pressure is useful because high blood pressure can occur without noticeable symptoms.
- C. If you are active, blood pressure screening is no longer necessary.
- D. Blood pressure screening is mainly done to confirm a heart attack.

Correct Answer: B

Hypertension can be present without symptoms, making screening important even when a client feels well. Choice A is incorrect because severe symptoms may not occur early. Choice C is false because activity does not eliminate the need for screening. Choice D confuses routine screening with acute diagnosis.

1517. The practical nurse is planning a teaching session on skin cancer prevention for outdoor workers. Which instruction should the nurse include?

- A. Use tanning beds instead of spending time in direct sunlight.
- B. Apply sunscreen and reapply it according to product directions when outdoors.
- C. Sun protection is unnecessary on cloudy days.
- D. Only people with dark skin need to check for changing skin lesions.

Correct Answer: B

Using sunscreen appropriately is an evidence-based prevention measure for reducing sun-related skin damage. Tanning beds do not reduce risk. UV exposure can still occur on cloudy days. Skin changes should be monitored regardless of skin tone; Choice D is incorrect and unsafe.

1518. A practical nurse is reinforcing teaching about test results with a client after a community screening. Which statement by the nurse is most appropriate?

- A. A normal screening result means you definitely will not develop that condition later.
- B. An abnormal screening result means the condition is already confirmed.
- C. Screening results help decide whether more assessment or diagnostic testing is needed.
- D. Screening is accurate enough that provider follow-up is unnecessary.

Correct Answer: C

Screening helps identify who may need additional evaluation; it does not by itself confirm or permanently rule out a condition. Choice A gives false reassurance. Choice B incorrectly treats screening as diagnosis. Choice D ignores the purpose of follow-up care.

1519. A client tells the practical nurse, "My father had type 2 diabetes, so there is nothing I can do to lower my own risk." Which response is best?

- A. Family history means diabetes cannot be prevented or delayed.
- B. Because your risk is inherited, screening will not be helpful.
- C. Some risks cannot be changed, but healthy habits and regular screening can still help.
- D. You only need to seek care if you begin having severe symptoms.

Correct Answer: C

Family history is a nonmodifiable risk, but modifiable factors such as nutrition, activity, weight management, and regular screening can still help reduce risk or support early detection. Choices A, B, and D incorrectly promote fatalism or delayed care.

1520. Which community activity is the best example of secondary prevention?

- A. Teaching older adults how to prevent falls at home
- B. Offering blood pressure checks to identify previously unrecognized hypertension
- C. Providing physical therapy after a hip fracture
- D. Assisting a client with meal planning after heart surgery

Correct Answer: B

Secondary prevention focuses on early detection of disease, such as screening for hypertension. Choice A is primary prevention because it aims to prevent the problem before it occurs. Choices C and D are related to recovery and ongoing management, not early detection.

1521. A practical nurse is reviewing a client's readiness to participate in preventive care. Which client statement most clearly indicates a barrier the nurse should address first?

- A. I do not understand why I need this test if I feel healthy.
- B. I had a screening done 2 years ago.
- C. My sister reminds me about appointments.
- D. I prefer morning visits when possible.

Correct Answer: A

A lack of understanding about the purpose of screening is a direct barrier to participation and should be addressed first through teaching. The other statements do not show a major obstacle to preventive care.

1522. The practical nurse is helping plan a prevention program for a factory workforce. Which group would most likely benefit from hearing-conservation screening and teaching?

- A. Office clerks in a quiet billing department
- B. Night custodians using standard cleaning supplies
- C. Machine operators exposed to loud equipment daily
- D. Reception staff answering phones in a carpeted lobby

Correct Answer: C

Workers exposed to loud machinery have increased risk for hearing loss and are the most appropriate target for hearing-conservation screening and prevention teaching. The other groups have less obvious routine noise exposure.

1523. A practical nurse is reinforcing teaching after a blood glucose screening event. Which client statement requires correction?

- A. If my result is outside the expected range, I may need more testing.
- B. This screening can help catch a problem before symptoms appear.
- C. A single normal result means I will never need this checked again.
- D. My personal risk factors help determine how important follow-up is.

Correct Answer: C

A single normal screening result does not mean a condition can never develop later or that future screening will never be needed. Choices A, B, and D correctly reflect the role of screening and follow-up. The common trap is assuming one normal result provides lifetime reassurance.

1524. At a senior center, four clients ask which health behavior would best reduce preventable illness. Which client should the practical nurse address first because the behavior poses the greatest ongoing preventable risk?

- A. A client who walks 20 minutes most days but forgets to stretch
- B. A client who continues to smoke despite chronic coughing
- C. A client who prefers salty snacks at lunch
- D. A client who occasionally stays up late watching television

Correct Answer: B

Continued smoking despite chronic coughing signals a significant ongoing preventable risk and should be prioritized for intervention. The other behaviors may merit teaching, but they are generally less immediately harmful than persistent tobacco use.

1525. A practical nurse is reinforcing teaching to a client who received an abnormal result during a screening fair and now says, "So this means I definitely have the disease." Which response is best?

- A. Yes, screening tests are designed to make a final diagnosis.
- B. Not necessarily; screening suggests that more complete evaluation may be needed.
- C. No, because abnormal screening results are usually meaningless.
- D. Yes, but only if you also have a family history.

Correct Answer: B

An abnormal screening result does not confirm disease; it indicates that additional assessment or diagnostic testing may be needed. Choice A incorrectly equates screening with diagnosis. Choice C dismisses potentially important findings. Choice D adds an unnecessary condition not required for follow-up.

1526. A practical nurse is helping with a community prevention program. Which activity is the best example of primary prevention?

- A. Teaching adults how to limit saturated fat intake before they develop heart disease
- B. Referring a client with a breast lump for diagnostic testing
- C. Checking a client's blood pressure at a follow-up visit after medication changes
- D. Providing range-of-motion exercises to a client after a stroke

Correct Answer: A

Primary prevention aims to stop disease before it starts. Teaching healthy diet habits to lower future heart disease risk is a classic primary prevention measure. Choice B is diagnostic follow-up, choice C is monitoring an existing problem, and choice D is tertiary prevention because it focuses on limiting disability after disease has occurred.

1527. A practical nurse is reinforcing teaching with a client who asks the difference between a screening test and a diagnostic test. Which explanation is best?

- A. A screening test confirms a disease, while a diagnostic test estimates risk

B. A screening test looks for possible disease in people without symptoms, while a diagnostic test is used to evaluate a specific suspected problem

C. A screening test is used only for infectious disease, while a diagnostic test is used for chronic illness

D. A screening test is more accurate than a diagnostic test and is used instead of follow-up care

Correct Answer: B

Screening is used to identify possible disease early in people who may not have symptoms. Diagnostic testing is used when there is a specific concern, symptom, or abnormal finding that needs evaluation. Choice A reverses the concepts. Choice C is too narrow and incorrect. Choice D is incorrect because abnormal screening results often require diagnostic follow-up.

1528. A practical nurse is reinforcing teaching about reducing the spread of infection in the home. Which client action is the best example of secondary prevention?

- A. Receiving recommended immunizations
- B. Using condoms during sexual activity
- C. Getting tested for a sexually transmitted infection after a new exposure even though no symptoms are present
- D. Taking prescribed antiviral medication for a diagnosed infection

Correct Answer: C

Secondary prevention focuses on early detection of disease, often before symptoms appear. Testing after a new exposure is intended to identify infection early. Immunizations and condom use are primary prevention because they help prevent disease from occurring. Taking medication for an already diagnosed infection is treatment, not screening.

1529. A practical nurse is planning teaching for an adult client with limited time and resources. Which recommendation is most appropriate to support preventive care adherence?

- A. Wait to seek health care until symptoms interfere with daily activities

- B.** Choose one realistic prevention goal at a time, such as scheduling a screening and setting a quit date for tobacco use
- C.** Avoid screening tests unless there is a strong family history of illness
- D.** Focus only on diet because exercise has little role in prevention

Correct Answer: B

Breaking prevention into realistic, actionable steps improves adherence and reduces overwhelm. Choice B supports practical behavior change and use of preventive services. Choices A and C delay care inappropriately. Choice D is incorrect because physical activity is an important prevention strategy for many conditions.

1530. At a wellness visit, a practical nurse wants to identify a modifiable risk factor that can be targeted through prevention teaching. Which finding is the best example?

- A.** Age 67 years
- B.** Mother had colorectal cancer
- C.** Works at a desk and rarely exercises
- D.** Male sex

Correct Answer: C

Physical inactivity is a modifiable risk factor, so it can be directly addressed through prevention teaching. Age, family history, and sex are nonmodifiable factors. A common trap is choosing a factor that raises risk but cannot itself be changed.

1531. A client at a screening event says, "I skipped my last checkup because I was afraid they might find something bad." Which nursing response is best?

- A.** That fear means you are not ready to discuss prevention right now
- B.** If you feel healthy, missing one checkup is usually harmless
- C.** Many people feel that way, but finding a problem early often gives more options and better outcomes
- D.** Screening is only necessary when you start noticing symptoms

Correct Answer: C

This response acknowledges the client's fear and provides clear, supportive education about the value of early detection. Choice A shuts down teaching. Choice B minimizes the importance of prevention. Choice D is incorrect because screening is often intended for people without symptoms.

1532. A practical nurse is reviewing possible health-fair activities. Which activity is the clearest example of tertiary prevention?

- A.** Holding a smoking-prevention class for teenagers
- B.** Offering blood pressure checks to adults without symptoms
- C.** Teaching foot care to a client with diabetes to prevent further complications
- D.** Distributing bicycle helmets to children

Correct Answer: C

Tertiary prevention focuses on reducing complications and improving function after a disease is already present. Teaching foot care to a client with diabetes helps prevent injury and progression of complications. Choices A and D are primary prevention, and choice B is secondary prevention.

1533. A practical nurse is reinforcing teaching for a client who asks why prevention plans include both risk reduction and regular screening. Which explanation is best?

- A.** Risk reduction lowers the chance of developing disease, and screening can help detect problems early if they do occur
- B.** Screening replaces the need for healthy lifestyle habits
- C.** Risk reduction is useful only for younger adults, while screening is useful only for older adults
- D.** Screening can prevent every disease if it is done often enough

Correct Answer: A

Prevention plans often combine primary prevention, such as healthy habits, with secondary prevention, such as screening. These strategies work together rather than replacing one another. Choice B falsely treats screening as a substitute for healthy behaviors. Choices C and D are overly broad and inaccurate.

1534. A practical nurse is planning prevention teaching for a client newly diagnosed with hypertension who asks what still matters now that the disease has already started. Which response is best?

- A. Prevention no longer applies once a chronic illness has been diagnosed
- B. At this point, only medication matters
- C. Preventive measures can still reduce complications, such as stroke, kidney damage, and heart problems
- D. Screening and follow-up are useful only before a diagnosis is made

Correct Answer: C

Prevention continues after diagnosis. For a client with hypertension, adherence, lifestyle changes, and follow-up monitoring help prevent complications. Choices A and D wrongly suggest prevention stops after disease appears. Choice B ignores the continued importance of nonpharmacologic management and monitoring.

1535. A practical nurse is preparing a brief teaching handout about cancer warning signs and screening. Which statement should the nurse include?

- A. If a screening result is normal, cancer can never develop later
- B. Screening helps identify some problems early, but clients should still report new symptoms promptly
- C. Screening is useful only for clients with a strong family history
- D. A negative screening result means no further routine health care is needed

Correct Answer: B

A normal screening result does not guarantee that disease will never develop, and it does not replace evaluation of new symptoms. Choice B correctly explains that screening is one part of prevention and early detection. Choices A, C, and D overstate or misuse the purpose of screening.

1536. A practical nurse is reviewing clients seen at a free screening clinic. Which client statement requires the most immediate follow-up for misunderstanding of prevention?

- A. "Now that I know my blood pressure today, I can ignore the headaches I've been having."

B. "I should keep trying to be more active during the week."

C. "I know one test result does not tell me everything about my health."

D. "I will schedule a follow-up visit if the clinic recommends it."

Correct Answer: A

Choice A is dangerous because it shows the client may dismiss concerning symptoms based on a single screening result. Screening does not replace evaluation of symptoms. The other statements reflect appropriate understanding of prevention and follow-up. The key reasoning is recognizing risk from false reassurance.

1537. A practical nurse is reinforcing teaching to a client who says, "I already eat well, so I do not need screening tests." Which response is best?

- A. Healthy habits reduce risk, but they do not remove the need for appropriate screening and routine follow-up
- B. You are correct, because screening is mainly for people with unhealthy lifestyles
- C. Screening is needed only if a close relative had the disease
- D. If you have no symptoms and follow a healthy diet, testing usually creates unnecessary worry

Correct Answer: A

Healthy behaviors are important primary prevention, but they do not eliminate all risk. Appropriate screening may still help detect disease early. Choices B, C, and D incorrectly limit who benefits from screening and may reinforce avoidance of preventive care.

1538. A practical nurse is helping prioritize topics for a neighborhood prevention workshop. Which topic most directly addresses primary prevention at the population level?

- A. How to manage daily life after a stroke
- B. How to prepare for a biopsy after an abnormal finding
- C. How handwashing, safer food handling, and vaccines reduce illness before it begins
- D. How to interpret follow-up instructions after a positive screening result

Correct Answer: C

Primary prevention focuses on preventing disease before it starts. Handwashing, food safety, and vaccines are direct population-level primary prevention strategies. Choice A is tertiary prevention, and choices B and D involve diagnostic or follow-up care after a problem has already been identified.

1539. A practical nurse is preparing a short teaching session for adults at a community center about levels of prevention. Which activity is the best example of secondary prevention?

- A. Giving influenza vaccines before flu season begins
- B. Teaching clients to choose lower-sodium foods to reduce future hypertension risk
- C. Checking blood pressure during a community screening to detect disease early
- D. Referring a client with stroke deficits to physical therapy to improve function

Correct Answer: C

Secondary prevention focuses on early detection of health problems so treatment can begin promptly. Community blood pressure screening is used to identify hypertension before complications occur. Influenza vaccination and diet teaching are primary prevention because they aim to prevent disease from developing. Physical therapy after a stroke is tertiary prevention because it helps reduce disability after disease or injury has already occurred.

1540. During a clinic visit, a client says, "I feel fine, so I don't see the point of screening tests." Which response by the practical nurse is best?

- A. Screening is mainly used after symptoms appear so the provider can confirm a suspected illness.
- B. Screening can find health problems early, even before you notice symptoms.
- C. If you have no family history, screening is usually unnecessary.
- D. Screening is only helpful for infectious diseases that spread from person to person.

Correct Answer: B

The best response corrects the client's misunderstanding by explaining the purpose of screening: detecting disease or risk early, often before symptoms are present. Choice A confuses screening with diagnostic evaluation. Choice C is a common trap because family history matters, but its absence does not make screening unnecessary. Choice D is incorrect because screening is used for many conditions, not just infectious diseases.

Health history

1541. A nurse is obtaining a health history from a new adult client in a clinic. Which question best assesses the client's chief concern?

- A. What health problem brought you in today?
- B. Did your parents have any chronic illnesses?
- C. How often do you exercise each week?
- D. What medications have you taken in the past year?

Correct Answer: A

The chief concern is the primary reason the client is seeking care now, so asking what problem brought the client in today is the most direct way to identify it. The other options gather important health history data, but they assess family history, lifestyle, and medication history rather than the immediate reason for the visit.

1542. A client tells the nurse, "I'm allergic to penicillin." Which follow-up question is most important?

- A. When did you first hear about penicillin?
- B. What happened when you took it?
- C. How much did the prescription cost?
- D. Did the medication taste bitter?

Correct Answer: B

Asking what happened when the client took penicillin helps the nurse distinguish a true allergy from an expected side effect or intolerance. This affects client safety and future treatment decisions. The other questions do not clarify the reaction itself and are less useful for safe care planning.

1543. During an intake interview, a client says, "I smoke half a pack of cigarettes a day." In which section of the health history should the nurse document this information?

- A. Family history
- B. Social history
- C. Surgical history
- D. Review of systems

Correct Answer: B

Tobacco use is part of the social history because it reflects lifestyle habits and health behaviors. Family history concerns illnesses in relatives, surgical history covers operations and procedures, and review of systems is a symptom-focused inventory by body system.

1544. A nurse is interviewing an older adult client who reports dizziness. Which question best explores the symptom using a symptom-analysis approach?

- A. Do you usually sleep with one pillow or two?
- B. When did the dizziness start, and what were you doing at the time?
- C. Did your siblings ever have similar symptoms?
- D. Would you like to discuss your diet later?

Correct Answer: B

A symptom-analysis approach includes onset and circumstances surrounding the symptom. Asking when the dizziness started and what the client was doing helps define the problem and identify possible triggers. The other options do not directly analyze the present symptom.

1545. A nurse is taking a reproductive health history from a client who asks what the notation G3 P2 means. Which response by the nurse is correct?

- A. It means you have had 3 live births and 2 miscarriages.
- B. It means you are currently 3 months pregnant with 2 fetuses.
- C. It means you have been pregnant 3 times and have had 2 births at a viable gestation.
- D. It means you delivered 3 times and have 2 children living at home.

Correct Answer: C

Gravida refers to the total number of pregnancies, and para refers to the number of births at viable gestation. Therefore, G3 P2 means 3 pregnancies and 2 births at viable gestation. The distractors confuse this with live children, current pregnancy details, or miscarriage counts.

1546. A nurse is collecting a health history from a client who says, "I drink about 4 beers every night." What is the nurse's best next response?

- A. You should stop drinking completely.
- B. Why would you do that to your body?
- C. Tell me how long you have been drinking that amount.
- D. I will notify the provider that you are noncompliant.

Correct Answer: C

The best response is nonjudgmental and seeks additional relevant data. Asking how long the client has been drinking that amount helps assess use patterns and possible health risks. The other responses are judgmental, premature, or label the client negatively, which can reduce honest communication.

1547. A client comes to the clinic with abdominal pain. Which question best addresses the quality of the pain?

- A. Does the pain move anywhere else?
- B. When did the pain begin?
- C. How would you describe the pain?
- D. What makes the pain better or worse?

Correct Answer: C

Quality refers to the character of the symptom, such as sharp, dull, burning, or cramping. Asking the client to describe the pain directly assesses quality. The other options assess radiation, onset, and aggravating or relieving factors.

1548. A nurse is obtaining a history from the parent of a 2-year-old child. Which statement by the parent is most important to follow up?

- A. My child takes a nap after lunch most days.
- B. My child was treated for an ear infection last month.
- C. My child still does not use any words.

D. My child prefers crackers to vegetables.

Correct Answer: C

By age 2, absence of any words suggests a possible developmental delay and requires further assessment. The other findings may be common or less urgent in the context given. This question tests recognizing a history finding that has greater significance for health promotion and early intervention.

1549. A nurse is interviewing a client who has limited English proficiency. The client's adult daughter offers to interpret. What is the nurse's best action?

- A.** Use the daughter because family members know the client best.
- B.** Ask the daughter to interpret only questions about medications.
- C.** Proceed with gestures and yes-or-no questions only.
- D.** Arrange for a qualified medical interpreter.

Correct Answer: D

A qualified medical interpreter is the best choice to promote accurate communication, protect confidentiality, and reduce misinterpretation. Family members may omit, add, or filter information. Gestures and limited yes-or-no questions are not adequate for a complete health history.

1550. A client states, "I had heart trouble a few years ago." Which question by the nurse is most helpful to make this history entry more specific?

- A.** Did your insurance cover the treatment?
- B.** What diagnosis were you given, and when did it occur?
- C.** Was the hospital food acceptable?
- D.** Do you think stress causes all heart problems?

Correct Answer: B

Vague statements such as "heart trouble" need clarification. Asking for the specific diagnosis and timing makes the history more accurate and clinically useful. The other questions do not define the condition or contribute meaningfully to current assessment.

1551. A nurse is reviewing a client's past health history. Which finding belongs in the past health history rather than the family history?

- A.** Mother has type 2 diabetes mellitus.
- B.** Brother had asthma as a child.
- C.** Client had an appendectomy at age 16.
- D.** Father died of a stroke at age 72.

Correct Answer: C

A personal appendectomy is part of the client's past health history, specifically surgical history. The other options describe health information about relatives, which belongs in the family history.

1552. A nurse asks a client, "You don't use drugs, do you?" This question is poorly phrased because it primarily does which of the following?

- A.** Encourages the client to provide a detailed timeline
- B.** Uses medical terminology that is too complex
- C.** Introduces bias and suggests a desired answer
- D.** Focuses too much on family history

Correct Answer: C

This is a leading question because it implies the preferred response is "no." Leading questions can reduce accuracy by pressuring the client to answer in a socially acceptable way. It is not mainly a problem of complex terminology or family-history focus.

1553. A nurse is taking a health history from a client who says, "I get short of breath." Which follow-up question best evaluates the severity of this symptom?

- A.** Does it happen at rest, with activity, or both?
- B.** What color is your sputum?
- C.** Have you ever traveled out of state?
- D.** Did your grandparents have lung disease?

Correct Answer: A

Severity of shortness of breath is often clarified by determining how much activity triggers it and whether it occurs at rest. This gives functional information about how serious the symptom may be. Sputum color may be relevant later, but it does not directly measure severity. Travel and grandparent history are not the best immediate follow-up.

1554. During an interview, a client becomes silent and looks down after being asked about intimate partner violence. What is the nurse's best response?

- A. If you don't want to answer, we'll just skip it forever.
- B. You need to tell me right now whether someone hit you.
- C. I can see this may be difficult to talk about. Take your time.
- D. I'll ask your partner to answer for you.

Correct Answer: C

Acknowledging the difficulty and allowing time supports therapeutic communication and may help the client feel safe enough to continue. Pressuring the client, dismissing the topic entirely, or asking the partner to answer would interfere with accurate history collection and safety-focused assessment.

1555. A nurse is completing an intake history for a client who reports taking several prescription medications, two herbal products, and a daily vitamin. Which action is most appropriate?

- A. Document only the prescription medications because they are regulated
- B. Record all prescribed, over-the-counter, vitamin, and herbal products the client uses
- C. List the herbal products only if the client believes they are causing symptoms
- D. Omit vitamins because they are part of the client's diet rather than medication history

Correct Answer: B

A complete medication history includes prescription drugs, over-the-counter products, vitamins, minerals, supplements, and herbal products because all may affect health status or interact with treatment. The other options create unsafe gaps in the history.

1556. A nurse is collecting a health history from a client with type 2 diabetes who says, "My sugars have been up lately." Which follow-up question is best to clarify this statement?

- A. "Do you think your diabetes is getting worse?"
- B. "What blood glucose readings have you been getting, and when do they usually occur?"
- C. "Have you been under more stress than usual?"
- D. "Why have you not been following your diet?"

Correct Answer: B

The best response is the one that gathers specific, measurable data about the problem. Asking about actual blood glucose readings and when they occur helps the nurse clarify the client's concern and identify patterns. Choice A is vague, choice C may become relevant later but does not first define the problem, and choice D is judgmental and assumes nonadherence.

1557. During an admission interview, a client says, "My mother had breast cancer at 52, and my older brother has hypertension." This information should be documented in which part of the health history?

- A. Family history
- B. Past health history
- C. Review of systems
- D. History of present illness

Correct Answer: A

Conditions affecting biologic relatives belong in the family history. Past health history refers to the client's own previous illnesses, surgeries, and hospitalizations. Review of systems is a symptom-by-symptom screen, and history of present illness focuses on the current problem.

1558. A nurse is obtaining a medication history from a client who says, "I take a little white pill for my blood pressure, but I don't remember the name." Which response is best?

- A. "That is enough information for now."
- B. "Just tell me whether it works."
- C. "Please bring in your medication bottles or a current medication list so we can verify the name and dose."

D. “Most blood pressure pills are similar, so the exact one is not important.”

Correct Answer: C

The safest action is to verify the medication name and dose using medication bottles or an up-to-date list. Medication reconciliation requires accuracy. Choices A, B, and D accept incomplete information and can lead to documentation or treatment errors.

1559. A client comes to an urgent care clinic because of vomiting. Which question best helps the nurse assess for possible fluid loss related to this symptom?

- A.** “What did you eat yesterday?”
- B.** “How many times have you vomited in the last 24 hours?”
- C.** “Do you usually eat spicy foods?”
- D.** “Have you ever had surgery?”

Correct Answer: B

The number of vomiting episodes in a defined time frame directly helps estimate severity and risk for dehydration. Choice A may be useful later for cause, but it does not best assess fluid loss. Choices C and D are less relevant to the immediate symptom assessment.

1560. A nurse is interviewing a client who reports new chest discomfort. Which question best assesses aggravating factors?

- A.** “Can you point to where you feel it most?”
- B.** “What does the discomfort feel like?”
- C.** “What were you doing when it started, and what makes it worse?”
- D.** “Does the discomfort spread anywhere else?”

Correct Answer: C

Aggravating factors are what bring on or worsen a symptom. Asking what the client was doing at onset and what makes it worse directly addresses that part of symptom analysis. Choice A assesses location, B assesses quality, and D assesses radiation.

1561. A nurse is collecting a health history from a client who appears anxious and gives very brief answers. Which interview technique is most therapeutic and likely to improve data collection?

- A.** Ask rapid yes-or-no questions to finish the interview quickly

B. Use a calm manner, allow pauses, and begin with broad open-ended questions

C. Delay the interview until the client becomes completely relaxed

D. Focus only on objective data and skip sensitive topics

Correct Answer: B

A calm approach, therapeutic silence, and open-ended questions help build rapport and encourage the client to share concerns. Choice A limits data and may increase anxiety. Choice C is not practical if information is needed now, and choice D leads to an incomplete history.

1562. A client says, “I feel tired all the time.” Which question best distinguishes whether the problem is acute or chronic?

- A.** “How severe is the tiredness on a scale of 0 to 10?”
- B.** “What do you think is causing it?”
- C.** “When did the tiredness begin, and has it been constant or intermittent?”
- D.** “Does anyone else in your family feel this way?”

Correct Answer: C

Timing and duration help determine whether a symptom is acute, chronic, constant, or intermittent. Choice A measures severity, not time course. Choice B explores the client's perception, and D is not the best question for determining onset pattern.

1563. A nurse is obtaining a history from the parent of a school-age child. Which statement by the parent should the nurse document as part of the child's social history?

- A.** “She had an appendectomy when she was 6.”
- B.** “Her grandfather has asthma.”
- C.** “She lives with her mother, stepfather, and two siblings.”
- D.** “She developed a rash after amoxicillin.”

Correct Answer: C

Household composition and living arrangements are part of the social history. Choice A belongs in past health history, B in family history, and D in allergy history. The question tests accurate placement of historical data.

1564. A nurse asks a client about sleep habits as part of the health history. Which response by the client requires the most follow-up?

- A. "I usually sleep 7 hours each night."
- B. "I drink one cup of coffee with breakfast."
- C. "My partner says I stop breathing for short periods when I'm asleep."
- D. "I like to read before bed."

Correct Answer: C

Reports of pauses in breathing during sleep may indicate sleep apnea and require further assessment because of associated health risks. The other responses are common habits and are less urgent to investigate.

1565. A nurse is interviewing a client who recently moved and has not seen a provider in several years. Which question best addresses preventive health practices during the health history?

- A. "What serious illnesses did your parents have?"
- B. "When was your last checkup, screening test, or immunization?"
- C. "What kind of work do you do?"
- D. "How would you describe your usual energy level?"

Correct Answer: B

Preventive health practices include routine checkups, screenings, and immunizations. Choice A addresses family history, C addresses occupational history, and D explores a current symptom or general health perception rather than prevention activities.

1566. A nurse is taking a history from a client who reports frequent headaches. Which additional question is most important to ask first?

- A. "What over-the-counter or prescription medicines do you use for the headaches?"
- B. "Do headaches run in your family?"
- C. "What is your favorite way to relax?"
- D. "How many siblings do you have?"

Correct Answer: A

Medication use is important to assess early because it affects symptom interpretation, treatment planning, and can reveal overuse or adverse effects. Family history may be relevant but is less immediately useful than knowing what the client is taking now. Choices C and D do not directly address the presenting problem.

1567. A nurse is obtaining a history from a client who says, "I have trouble breathing at night." Which follow-up question best helps the nurse distinguish orthopnea from other nighttime breathing problems?

- A. "Do you cough more in the morning?"
- B. "Do you wake up short of breath after sleeping, or do you become short of breath when lying flat?"
- C. "Have you traveled recently?"
- D. "Do you have pain when you take a deep breath?"

Correct Answer: B

Orthopnea is shortness of breath when lying flat, while nighttime awakening with shortness of breath suggests a different pattern. This question helps the nurse define the symptom more precisely. The other options may be useful in some respiratory histories but do not best distinguish the reported nighttime problem.

1568. During a comprehensive health history, a client says, "I use some natural products." Which response by the nurse is best?

- A. "Natural products are generally safe, so we don't need to list them."
- B. "Only prescription medications matter for your history."
- C. "Please tell me the names, amounts, and how often you take each product."
- D. "Stop taking them until the provider tells you otherwise."

Correct Answer: C

Herbal and other natural products should be included in the medication history because they may interact with prescribed drugs or affect care. The nurse should collect specific details such as name, dose, and frequency. Choices A and B dismiss important information, and D gives an unnecessary directive without assessment.

1569. A nurse is obtaining a sexual health history from an adolescent client during a confidential interview. Which question is best phrased to promote accurate, nonjudgmental disclosure?

- A. "You aren't sexually active, right?"
- B. "Why would someone your age be having sex?"
- C. "Have you ever been sexually active with anyone?"
- D. "Your parents would want to know if you were sexually active, correct?"

Correct Answer: C

A neutral, direct question supports honest disclosure and avoids assumptions or judgment. Choice C is open and respectful. Choice A is leading, B is judgmental, and D pressures the client by introducing parental expectations rather than supporting confidential assessment.

1570. A client reports, "I get palpitations once in a while." Which sequence of follow-up questions would provide the most useful focused history?

- A. Ask about onset, frequency, duration, associated symptoms, and triggers such as caffeine or activity
- B. Ask only whether the client's relatives have heart disease
- C. Ask whether the client thinks the palpitations are dangerous
- D. Ask the client to describe the sensation, then move to a different body system

Correct Answer: A

A focused symptom history should define when palpitations started, how often they occur, how long they last, what symptoms accompany them, and what triggers them. This gathers clinically useful data for pattern recognition and risk assessment. The other options are incomplete and miss key dimensions of symptom analysis.

Baseline physical data

1571. A practical nurse is collecting baseline physical data for a newly admitted adult client. Which assessment finding should be reported immediately rather than simply documented as part of the baseline?

- A. Blood pressure 88/54 mm Hg with dizziness
- B. Oral temperature 98.4°F (36.9°C)
- C. Respiratory rate 16/min and unlabored
- D. Pain rating 2 on a 0-to-10 scale

Correct Answer: A

A blood pressure of 88/54 mm Hg combined with dizziness may indicate poor perfusion or instability and requires prompt reporting. The other findings are not immediately concerning as baseline data. A common trap is treating all first-time admission findings as routine documentation, but abnormal baseline findings still require action.

1572. The nurse is obtaining orthostatic vital signs for an older adult who reports feeling lightheaded when standing. Which action is most important during this assessment?

- A. Ask the client to stand quickly to provoke symptoms
- B. Keep the client safe from falling during position changes
- C. Delay the blood pressure measurement until after breakfast
- D. Obtain only the apical pulse because blood pressure may vary

Correct Answer: B

Safety is the priority when assessing a client with possible orthostatic changes. The client may become dizzy or faint during position changes, so fall prevention is essential. Standing quickly is unsafe, delaying the assessment does not address the issue, and both pulse and blood pressure are relevant baseline data.

1573. A practical nurse is preparing to measure an adult client's blood pressure manually. Which action helps obtain the most accurate reading?

- A. Place the cuff over a thick sweater sleeve
- B. Support the arm at heart level

- C. Measure immediately after the client walks from the bathroom
- D. Use a cuff that is smaller than recommended to keep it snug

Correct Answer: B

Supporting the arm at heart level promotes a more accurate blood pressure reading. Clothing under the cuff, recent activity, and an incorrectly sized cuff can all distort the result. A frequent trap is thinking a tighter cuff improves accuracy, but too small a cuff can falsely elevate the reading.

1574. Which statement by a client best describes subjective baseline data?

- A. My pain is an 8 out of 10.
- B. The temperature is 100.2°F.
- C. The incision is 4 cm long.
- D. The pulse oximeter reads 97%.

Correct Answer: A

Subjective data are information reported by the client, such as pain level. The other options are objective findings because they are measured or observed by the nurse. The common trap is assuming pain is less valid because it cannot be seen, but client report is the standard for pain assessment.

1575. A nurse obtains these baseline vital signs on a postoperative client: temperature 99.1°F (37.3°C), pulse 118/min, respirations 24/min, blood pressure 84/50 mm Hg, and oxygen saturation 94% on room air. Which finding is the highest priority to report first?

- A. Temperature 99.1°F (37.3°C)
- B. Pulse 118/min
- C. Respirations 24/min
- D. Blood pressure 84/50 mm Hg

Correct Answer: D

The blood pressure of 84/50 mm Hg is the most urgent finding because it suggests hypotension and possible inadequate tissue perfusion. Tachycardia and tachypnea may be important related findings, but hypotension is the top priority. A likely trap is choosing the elevated pulse because it is clearly abnormal, but circulation compromise takes priority.

1576. During baseline assessment, a client says, "I have trouble hearing when people talk softly." What is the best nursing action to improve data collection?

- A. Speak clearly while facing the client
- B. Repeat each question more quickly
- C. Use medical terminology to shorten the interview
- D. Ask a family member to answer all questions

Correct Answer: A

Speaking clearly while facing the client helps a client with hearing difficulty receive information and may also allow lip reading or use of visual cues. Talking faster and using medical jargon reduce understanding. A family member may assist if needed, but the nurse should communicate with the client directly whenever possible.

1577. A practical nurse is collecting baseline data from a client with a casted right arm. Where is the best site to measure blood pressure?

- A. Right arm over the cast padding
- B. Right forearm below the cast
- C. Left arm if no contraindication is present
- D. Either arm, because casts do not affect blood pressure measurement

Correct Answer: C

The blood pressure should be taken in the left arm if it is available and there is no contraindication. A casted extremity is not an appropriate site for routine cuff placement. The trap is assuming any part of the affected arm can be used, but proper cuff placement and circulation considerations matter.

1578. The nurse is obtaining a baseline pulse oximetry reading. Which factor could make the reading less accurate?

- A. The client is wearing dark nail polish
- B. The client is resting quietly in bed
- C. The client's hand is supported on a pillow
- D. The client has just answered orientation questions

Correct Answer: A

Dark nail polish can interfere with the light sensor used in pulse oximetry and may reduce accuracy. Resting quietly and supporting the hand do not usually interfere. The orientation questions are not the main issue here. A common trap is overlooking external factors that affect equipment readings.

1579. A client arrives for a clinic visit and says, "I just finished a cigarette in the parking lot." Which baseline measurement is most likely to be temporarily affected?

- A. Height
- B. Respiratory status and pulse
- C. Pupil shape
- D. Waist circumference only

Correct Answer: B

Recent smoking can temporarily affect respiratory status and pulse, so those baseline findings may not reflect the client's usual resting state. Height and pupil shape are not expected to change because of one recent cigarette, and waist circumference is unrelated. The trap is focusing on a static body measure instead of a physiologic response.

1580. The nurse records a client's oral temperature as 95.9°F (35.5°C), but the client is talking easily, skin is warm, and no signs of distress are noted. What is the best action?

- A. Document the temperature and continue the assessment
- B. Warm the client with blankets without rechecking
- C. Repeat the temperature to verify the reading
- D. Report severe hypothermia immediately

Correct Answer: C

When a measurement does not fit the client's overall presentation, the nurse should first verify the reading. Equipment error, recent intake, or technique may affect the result. Automatically documenting it as correct or reporting it as severe hypothermia without confirmation is premature. The trap is reacting to a number without reconciling it with clinical cues.

1581. Which baseline finding in a 2-month-old infant requires the most immediate follow-up by the nurse?

- A. Anterior fontanel is soft and flat
- B. Respirations are primarily diaphragmatic
- C. Heart rate is 160/min while crying
- D. Rectal temperature is 100.8°F (38.2°C)

Correct Answer: D

A rectal temperature of 100.8°F (38.2°C) in a young infant is concerning and requires prompt follow-up. A soft, flat fontanel and diaphragmatic breathing are expected findings. A heart rate of 160/min while crying can occur with activity or distress. The trap is choosing the crying heart rate without recognizing fever in a young infant as more urgent.

1582. A nurse obtains a baseline blood pressure of 152/94 mm Hg in an adult client with no previous reading available. The client has been talking on the phone and drinking coffee while waiting. What is the best next action?

- A. Document hypertension and end the assessment
- B. Administer an antihypertensive medication immediately
- C. Allow the client to rest, then recheck the blood pressure
- D. Switch to a pediatric cuff to confirm the result

Correct Answer: C

Recent caffeine intake and activity can affect blood pressure, so the nurse should allow the client to rest and then repeat the reading before drawing conclusions. Immediate medication administration is inappropriate without further evaluation, and a pediatric cuff would worsen accuracy. The trap is treating one potentially affected reading as definitive baseline data.

1583. A client with diabetes reports numbness in both feet. Which assessment is most important to include in baseline physical data?

- A. Capillary refill and skin integrity of the feet
- B. Pupil accommodation after bright light exposure
- C. Abdominal girth after meals
- D. Range of motion of the jaw

Correct Answer: A

For a client with diabetes and numbness in the feet, baseline assessment should include circulation and skin integrity because reduced sensation increases risk for unnoticed injury and impaired healing. The other assessments are far less directly related to the presenting concern. The trap is choosing a general assessment instead of one targeted to the client's risk.

1584. The nurse must obtain an accurate baseline weight for a client with heart failure. Which method is best?

- A.** Use the same scale each day with similar clothing
- B.** Estimate weight from the client's stated usual weight
- C.** Weigh the client after the largest meal of the day
- D.** Alternate between standing and bed scales for comparison

Correct Answer: A

Using the same scale under similar conditions improves consistency and makes the weight more useful as baseline data for tracking fluid status. Estimated weight is not accurate enough, meal timing affects weight, and changing scales reduces comparability. The common trap is thinking any recorded weight is good enough when trend accuracy matters.

1585. A practical nurse is collecting baseline respiratory data for a client receiving oxygen by nasal cannula. Which action is most appropriate?

- A.** Remove the oxygen to assess the client's true respiratory rate
- B.** Document the respiratory rate and oxygen flow rate together
- C.** Count respirations only after the client walks in the hall
- D.** Record oxygen saturation without noting the delivery device

Correct Answer: B

Baseline respiratory data should include not only the respiratory rate but also the oxygen delivery being used, such as the nasal cannula flow rate. Removing oxygen without a prescription or need is unsafe. Recording saturation without delivery details gives incomplete information. The trap is treating the number alone as enough without documenting the support level.

1586. A practical nurse is gathering baseline physical data for a newly admitted adult client. Which item is objective data?

- A.** The client states, "My back hurts worse when I move."
- B.** The client reports feeling tired for the past week.
- C.** The client rates pain as 7 on a 0-to-10 scale.
- D.** The client's apical pulse is 88/min and regular.

Correct Answer: D

Objective data are measurable or observable findings obtained by the nurse. An apical pulse of 88/min and regular is directly assessed and can be verified. The other options are subjective because they are based on the client's report, including the pain rating.

1587. A practical nurse is collecting baseline physical data from an adult client. To count respirations without making the client consciously change the breathing pattern, which approach is best?

- A.** Ask the client to take several deep breaths first
- B.** Count respirations immediately after counting the pulse while keeping the client's arm in place
- C.** Tell the client that respirations will be counted for 60 seconds
- D.** Measure respirations only after the client has stood and walked in the room

Correct Answer: B

Respirations are best counted discreetly, often right after the pulse is assessed, because clients may alter their breathing if they know respirations are being measured. Asking for deep breaths or announcing the count can change the rate and pattern. Ambulation may also affect the baseline reading.

1588. A client is 5 ft 6 in tall and weighs 98 lb. During the baseline assessment, the client says, "I haven't really been trying to lose weight." Which follow-up question is most important?

- A. Do you prefer sweet or salty foods?
- B. How many hours do you sleep each night?
- C. How much weight have you lost, and over what time period?
- D. Do you usually cook your meals at home?

Correct Answer: C

Possible unintentional weight loss is clinically important baseline data. The nurse should quantify the amount and time frame because this helps determine significance and guides further evaluation. Food preferences, sleep, and cooking habits may matter later, but they do not address the immediate need to clarify the reported weight change.

1589. The nurse is collecting baseline height and weight on an adult client who can stand independently. Which technique is most likely to provide the most accurate data?

- A. Measure after breakfast while the client is wearing shoes and a jacket
- B. Ask the client for the most recent height and weight from home
- C. Use the same calibrated scale, with light clothing and no shoes
- D. Estimate height from arm span and weight from body build

Correct Answer: C

Baseline height and weight are most accurate when measured directly using proper equipment and consistent technique. A calibrated scale, light clothing, and no shoes reduce avoidable variation. Self-reported or estimated values are less reliable, and heavy clothing or recent intake can distort the result.

1590. A practical nurse is beginning a baseline physical assessment of an alert adult client. Which sequence is generally appropriate for examining the abdomen?

- A. Palpation, percussion, auscultation, inspection
- B. Inspection, auscultation, percussion, palpation
- C. Auscultation, inspection, palpation, percussion

D. Percussion, palpation, inspection, auscultation

Correct Answer: B

The abdomen is assessed in the order of inspection, auscultation, percussion, and palpation. Auscultation is done before percussion and palpation because those actions can alter bowel sounds. A common trap is using the head-to-toe sequence applied to other body systems instead of the special abdominal sequence.

1591. A practical nurse obtains these baseline findings on an adult client: temperature 98.4°F (36.9°C), pulse 76/min, respirations 16/min, blood pressure 118/72 mm Hg, oxygen saturation 89% on room air, and lips slightly dusky. What is the best action?

- A. Document the findings and continue the head-to-toe assessment
- B. Recheck the oxygen saturation at the end of the shift
- C. Report the finding promptly and assess the client's respiratory status further
- D. Ask the client whether this is the usual oxygen saturation level and take no other action

Correct Answer: C

An oxygen saturation of 89% on room air with dusky lips is an abnormal baseline finding that requires prompt follow-up and reporting. It may indicate impaired oxygenation. Waiting, simply documenting, or relying only on the client's statement could delay needed intervention.

1592. The nurse is collecting baseline physical data from a client who has mild expressive aphasia after a stroke. Which action best supports accurate data collection?

- A. Complete the entire interview with a family member instead of the client
- B. Ask several questions in quick succession to reduce client fatigue
- C. Use short, simple questions and allow extra time for responses
- D. Speak loudly and repeat every question twice

Correct Answer: C

Clients with expressive aphasia may understand more than they can verbally express. Using simple questions and allowing time improves accuracy and respects the client's ability to participate. Speaking loudly does not address aphasia, and bypassing the client or rushing questions can reduce the quality of baseline data.

1593. A practical nurse is measuring a client's radial pulse for baseline data and notices an irregular rhythm. Which action is most appropriate?

- A. Count the radial pulse for 15 seconds and multiply by 4
- B. Count the apical pulse for 1 full minute
- C. Document the pulse as irregular and move on to blood pressure
- D. Ask the client to cough and then recount the radial pulse for 30 seconds

Correct Answer: B

When a pulse is irregular, the nurse should assess the apical pulse for a full minute to obtain the most accurate rate and rhythm information. Short counts are less reliable with irregular rhythms. Documentation alone is incomplete because the rhythm needs more precise assessment.

1594. During baseline assessment of an older adult, the nurse notes dry oral mucous membranes, poor skin turgor over the sternum, dark amber urine, and blood pressure 106/64 mm Hg. Which interpretation is most appropriate?

- A. The findings strongly suggest dehydration and should be followed up
- B. The findings confirm acute kidney failure
- C. The findings are expected with normal aging and do not need attention
- D. The findings show fluid volume excess

Correct Answer: A

Dry mucous membranes, concentrated urine, and poor turgor can suggest dehydration and should be evaluated further. These findings do not by themselves confirm acute kidney failure, and they are not consistent with fluid overload. A trap is dismissing concerning findings as normal aging.

1595. A practical nurse is collecting baseline data on a 4-year-old child in the clinic. Which approach is most likely to improve cooperation during the physical assessment?

- A. Perform the most uncomfortable procedures first so the child gets used to the nurse
- B. Tell the child that nothing will hurt, even if the child needs an ear exam
- C. Allow the child to handle safe equipment briefly and save intrusive parts for later
- D. Ask the parent to leave the room so the child can focus

Correct Answer: C

Preschool children usually cooperate better when the nurse builds trust, uses simple explanations, and leaves more intrusive parts of the exam for later. Saying nothing will hurt is not honest if discomfort is possible. Doing uncomfortable procedures first or removing the parent can increase anxiety.

1596. The nurse is obtaining baseline blood pressure on an adult client. The first reading is 164/96 mm Hg. The client is seated, calm, and the cuff size is correct. Which action is best before documenting the blood pressure as the baseline reading?

- A. Repeat the measurement after a few minutes and compare readings
- B. Use a smaller cuff to verify the elevated reading
- C. Record only the diastolic value because it is more reliable
- D. Ask the client to lie flat immediately and then measure again

Correct Answer: A

An unexpectedly elevated blood pressure should be rechecked after a few minutes using correct technique. Repeating the reading helps confirm accuracy. Using a smaller cuff can falsely elevate the result, recording only part of the reading is incomplete, and changing position unnecessarily may alter the value.

1597. A practical nurse is collecting baseline data from an adult client with darkly pigmented skin. Which assessment method is most appropriate for evaluating possible cyanosis?

- A. Inspect the sclera for a blue tint only

- B.** Assess the nail beds, lips, oral mucosa, and conjunctiva
- C.** Rely mainly on the client's report of feeling cold
- D.** Check only the palms because they best reflect oxygenation

Correct Answer: B

In darkly pigmented skin, cyanosis is often better assessed in the lips, oral mucosa, conjunctiva, and nail beds rather than depending only on generalized skin color. The other options are too limited or indirect to be the best assessment approach.

1598. A nurse is collecting baseline physical data from an adult client with obesity. The automated blood pressure device displays 178/102 mm Hg. The cuff appears tight and does not fully wrap around the upper arm. Which action should the nurse take first?

- A.** Document the reading because automated devices are objective
- B.** Reassure the client that obesity commonly causes false alarms
- C.** Obtain an appropriately sized cuff and repeat the blood pressure
- D.** Switch immediately to the forearm using the same cuff without further adjustment

Correct Answer: C

An undersized cuff can falsely elevate blood pressure. The first step is to use an appropriately sized cuff and repeat the measurement. Simply documenting the value risks accepting an inaccurate baseline. The forearm may sometimes be used if needed, but proper equipment selection comes first.

1599. The practical nurse is obtaining baseline physical data from an adult client who says, "I get dizzy if I stand up too fast." Which additional assessment is most appropriate?

- A.** Measure blood pressure and pulse in supine, sitting, and standing positions as indicated
- B.** Assess visual acuity with a handheld chart
- C.** Percuss the chest for areas of dullness
- D.** Measure abdominal girth before breakfast

Correct Answer: A

A report of dizziness on standing suggests possible orthostatic changes, so position-based blood pressure and pulse measurements are the most relevant additional baseline data. The other options assess different concerns and do not address the likely cause of the symptom.

1600. A practical nurse is collecting baseline data from a client with severe kyphosis and chronic lung disease. Which technique is most important for obtaining an accurate pulse oximetry reading?

- A.** Place the sensor on the coldest finger to reduce motion artifact
- B.** Apply the probe over dark nail polish because color does not affect the result
- C.** Ensure the extremity is warm and still, and correlate the reading with the client's condition
- D.** Use pulse oximetry as a direct measure of ventilation rather than oxygenation

Correct Answer: C

For accurate pulse oximetry, the site should have adequate perfusion and minimal motion, and the reading should be interpreted in context with the client's overall status. Cold extremities and nail polish can interfere with accuracy. Pulse oximetry reflects oxygen saturation, not ventilation directly, so it cannot substitute for broader respiratory assessment.

Immunizations

1601. A nurse is reviewing vaccine teaching with the parent of a 2-month-old infant who had a severe anaphylactic reaction after a previous dose of a vaccine. Which information is most important for the nurse to include when planning future immunizations?

- A.** Mild fussiness after a vaccine means the infant should avoid all future vaccines
- B.** A severe allergic reaction to a previous dose is a contraindication to receiving that same vaccine again
- C.** Vaccines should always be delayed until the infant is at least 6 months old after any reaction
- D.** Giving acetaminophen before every vaccine prevents serious allergic reactions

Correct Answer: B

A severe anaphylactic reaction to a previous dose is a true contraindication to additional doses of that same vaccine. Mild side effects such as fussiness are not the same as anaphylaxis. Delaying all vaccines until 6 months is incorrect, and acetaminophen does not prevent severe allergic reactions.

1602. A parent asks why a child receives several vaccines during well-child visits. Which response by the nurse is best?

- A. Vaccines help the child build protection before exposure to serious diseases
- B. Vaccines are mainly given so the child will not need future checkups
- C. Vaccines cure most viral illnesses after symptoms begin
- D. Vaccines are only needed if a disease is common in the child's school

Correct Answer: A

Vaccines are used to develop immunity and reduce the risk of serious infectious disease before exposure occurs. They do not replace routine health care visits, they do not cure active viral illness, and they are recommended based on prevention needs rather than only whether a disease is currently common at a school.

1603. A clinic nurse is preparing immunizations for several children. Which child should the nurse identify as most likely able to receive a scheduled vaccine today?

- A. A toddler with a temperature of 40 C (104 F) and lethargy
- B. A preschooler with a mild runny nose but who is active and eating normally
- C. A child who had an anaphylactic reaction after the same vaccine previously
- D. An infant with signs of a severe allergic reaction to vaccine components

Correct Answer: B

A mild minor illness such as a runny nose without significant systemic symptoms usually is not a reason to postpone immunization. High fever and lethargy suggest acute illness that may warrant delay, and prior anaphylaxis to the same vaccine or severe allergy to vaccine components are safety concerns. The common trap is over-delaying vaccines for minor illness.

1604. A nurse administers an intramuscular vaccine to an infant. Which site is generally the best choice?

- A. Dorsogluteal muscle
- B. Deltoid muscle
- C. Vastus lateralis muscle
- D. Abdominal tissue

Correct Answer: C

The vastus lateralis is generally the preferred intramuscular injection site for infants because it is well developed and safer to access. The dorsogluteal site is avoided because of nerve and tissue concerns, the deltoid is more commonly used in older children and adults, and abdominal tissue is not an intramuscular site.

1605. A nurse is teaching a client who is receiving an inactivated vaccine. Which statement by the client shows correct understanding?

- A. This vaccine can cause the full disease it protects against
- B. If my arm is sore afterward, that can be an expected local reaction
- C. Because the vaccine is inactivated, I will be immune immediately
- D. After this vaccine, I should skip any future recommended doses

Correct Answer: B

Mild soreness, redness, or swelling at the injection site can be an expected local reaction after an inactivated vaccine. An inactivated vaccine does not cause the full disease, immunity is not immediate, and some vaccines require additional doses for full protection.

1606. A school nurse is reviewing documentation for children entering kindergarten. Which record is most important to verify before concluding a child is up to date on immunizations?

- A. A parent's verbal statement that all shots were given on time
- B. A list of illnesses the child had before age 2 years
- C. Written immunization dates in the medical or official vaccine record
- D. A note that the child cried after the last injection

Correct Answer: C

Written immunization dates in a medical or official vaccine record are the most reliable way to verify status. A parent report may be helpful but is not as dependable as documented dates. Illness history and reaction notes do not establish whether the child is up to date.

1607. A parent says, "My child missed several vaccines because we moved. I guess we have to start the whole series over." Which response by the nurse is best?

- A. Yes, all vaccine series must be restarted if there is any delay
- B. No, many delayed vaccines can be continued using a catch-up schedule rather than restarting the series
- C. Only live vaccines can be restarted after a delay
- D. Delayed vaccines should be avoided because they are no longer effective

Correct Answer: B

The best response is that many delayed immunizations can be continued using a catch-up schedule rather than restarting the series. The trap is assuming any interruption means starting over. Delayed vaccines are still useful, and the need to restart is not based simply on whether a vaccine is live.

1608. An adult client asks the nurse why yearly influenza vaccination is recommended. Which response is best?

- A. Influenza vaccination is recommended each year to help protect against circulating strains
- B. One influenza vaccine in adulthood provides lifelong protection
- C. Influenza vaccine is only useful after a person has already had influenza once
- D. Influenza vaccination is recommended only for adults older than 80 years

Correct Answer: A

Yearly influenza vaccination is recommended because protection needs to match circulating strains and immunity may decline over time. A single adult dose does not provide lifelong protection, the vaccine is preventive rather than something used only after prior infection, and it is recommended more broadly than only for adults older than 80.

1609. A nurse is preparing to administer two injectable vaccines during the same visit. Which action is most appropriate?

- A. Mix both vaccines in the same syringe to reduce discomfort
- B. Administer the vaccines at separate injection sites and document each site
- C. Give one vaccine today and discard the second to avoid overimmunization
- D. Inject both vaccines into the same exact spot in the muscle

Correct Answer: B

When two injectable vaccines are given during the same visit, they should be administered at separate sites and each site should be documented. Mixing vaccines in the same syringe is inappropriate unless specifically supplied for that purpose, and using the same exact spot would make site-specific reactions harder to assess.

1610. A nurse is screening clients before vaccination. Which finding requires the most immediate follow-up before the vaccine is given?

- A. The client is anxious about needles

- B. The client ate breakfast 2 hours ago
- C. The client reports a history of severe allergy to a vaccine component
- D. The client had mild soreness after a prior injection

Correct Answer: C

A history of severe allergy to a vaccine component is a major safety concern and requires immediate follow-up before administration. Needle anxiety and prior mild soreness do not usually prevent vaccination, and recent food intake is not the key issue here.

1611. A nurse is teaching a pregnant client about recommended immunization planning. Which statement by the client requires further teaching?

- A. I should talk with my health care provider about which vaccines are appropriate during pregnancy
- B. Vaccination decisions in pregnancy should consider both maternal and fetal safety
- C. If a vaccine is not recommended during pregnancy, it may need to be deferred until after delivery
- D. I can receive any vaccine at any point in pregnancy without screening

Correct Answer: D

Vaccination during pregnancy requires screening because not every vaccine is appropriate in every situation. The other statements reflect sound understanding: vaccine planning should involve the provider, maternal and fetal safety matter, and some vaccines may need to be deferred. The trap is assuming pregnancy removes the need for standard vaccine assessment.

1612. A nurse is reviewing home care instructions after immunization. Which parent statement indicates a need for immediate follow-up?

- A. My child may have mild soreness where the shot was given
- B. I will watch for severe swelling or trouble breathing and seek care right away if those happen

- C. If my child becomes slightly fussy, I will continue to monitor at home
- D. If my child develops a severe reaction later, I will wait until the next clinic visit to mention it

Correct Answer: D

A severe reaction after immunization requires prompt medical evaluation, not waiting until the next clinic visit. Mild soreness and some fussiness may be expected, and watching for symptoms such as severe swelling or trouble breathing is appropriate. The key issue is recognizing when a potential adverse reaction needs urgent attention.

1613. A nurse is counseling an older adult who says, "I thought vaccines were just for children." Which response is best?

- A. Vaccines are mainly for school entry, so adults usually do not need them
- B. Adults may still need immunizations because protection needs can change with age, health status, and risk
- C. Adults only need vaccines if they travel outside the country
- D. Once a person reaches older adulthood, vaccines are no longer effective

Correct Answer: B

Adults, including older adults, may need immunizations based on age, health conditions, prior vaccine history, and exposure risk. The distractors wrongly limit adult vaccination to school-age prevention, international travel, or claim vaccines no longer work in older adulthood.

1614. A community health nurse is planning an immunization clinic during a disease outbreak. Which action best supports prevention at the population level?

- A. Prioritizing accurate screening, vaccine access, and clear teaching for eligible clients
- B. Vaccinating only clients who request a specific brand by name
- C. Postponing all vaccines until the outbreak has ended
- D. Limiting vaccination to clients with no history of any prior illness

Correct Answer: A

Population-level prevention is supported by accurate screening, timely access, and clear education for eligible clients. Delaying vaccines during an outbreak undermines prevention, and the other options apply inappropriate restrictions that do not reflect sound public health practice.

1615. A nurse is caring for a child who is due for vaccination, but the parent says, "We don't have the shot record with us, so please just give everything again to be safe." What is the nurse's best action?

- A. Administer all routine vaccines immediately without attempting verification
- B. Delay all future immunizations permanently because the record is missing
- C. Attempt to verify prior immunizations through available records before giving unnecessary repeat doses
- D. Rely only on the parent's memory of which arm received past injections

Correct Answer: C

The nurse should try to verify prior immunizations through available records to avoid unnecessary repeat doses while still keeping the child on track. Automatically giving everything again without verification is not the best first step, and permanently delaying vaccines is inappropriate. A parent's memory of injection location does not establish vaccine history.

1616. A nurse is reinforcing teaching for the parent of a 6-month-old infant who has a mild cough and clear nasal drainage but is playful and feeding well. The parent asks whether today's scheduled immunizations should be delayed. Which response by the nurse is best?

- A. Immunizations should be postponed until all cold symptoms are completely gone.
- B. A mild illness without significant fever is usually not a reason to delay scheduled vaccines.
- C. Vaccines should be delayed because they will not work during any respiratory illness.
- D. Only live vaccines can be given when a child has mild cold symptoms.

Correct Answer: B

A mild illness, such as a minor upper respiratory infection without significant fever or toxicity, is generally not a reason to delay routine immunization. Delaying for minor illness can create unnecessary gaps in protection. Choice A is a common trap because parents often think any illness means vaccines must wait. Choices C and D are too broad and incorrect.

1617. A nurse is teaching a group of new parents about vaccines. Which statement best explains the purpose of a booster dose?

- A. It treats an infection that has already started.
- B. It replaces the need for the first vaccine dose.
- C. It increases or restores protection after immunity from earlier doses decreases.
- D. It is given only when a child has a fever after a prior vaccine.

Correct Answer: C

A booster dose is given to increase or restore immunity when protection from earlier vaccination decreases over time. It does not treat active infection, replace an initial dose, or depend on fever after a prior vaccine. Choice A confuses prevention with treatment, which is a frequent error.

1618. A nurse is preparing to give an injectable vaccine to a preschool-age child. The child is crying and moving away from the needle. Which nursing action is most appropriate to reduce injury during administration?

- A. Ask the parent to leave the room so the child will calm down.
- B. Delay the vaccine until the child agrees without resistance.
- C. Use safe, age-appropriate positioning and have the child held securely.
- D. Promise the child the injection will not hurt.

Correct Answer: C

Safe, age-appropriate positioning with secure holding helps prevent sudden movement and reduces the risk of injury during vaccine administration. Asking the parent to leave may increase distress, delaying until full cooperation is unrealistic, and promising no pain is not honest or therapeutic. The key is safe restraint and support, not forceful or deceptive behavior.

1619. A parent asks what information is most helpful to bring to a child's vaccination visit. Which response by the nurse is best?

- A. A complete record of previous immunizations, if available
- B. The child's favorite toy for distraction only
- C. A list of all foods the child dislikes
- D. The child's most recent school grades

Correct Answer: A

A prior immunization record is most helpful because it helps the health care team determine which vaccines are due and avoid unnecessary confusion about schedule status. A toy may be useful for comfort, but it is not the most important information. Choices C and D are unrelated to routine vaccine planning.

1620. A nurse is reinforcing teaching for the parent of a toddler who received routine immunizations earlier in the day. Which parent statement indicates correct understanding of expected reactions?

- A. A little soreness at the injection site can happen after vaccination.
- B. My child should not move the arm or leg that received the injection for 2 days.
- C. Any redness means my child is having a severe allergic reaction.
- D. A vaccine reaction always begins immediately before we leave the clinic.

Correct Answer: A

Mild local soreness is a common expected reaction after many vaccines. Immobilizing the extremity is unnecessary, and mild redness does not automatically indicate severe allergy. Also, reactions do not always begin immediately. The trap is assuming any local reaction is dangerous, when many are mild and self-limited.

1621. A nurse in a prenatal clinic is reviewing vaccine history with a pregnant client who is in the second trimester. Which statement by the client requires further follow-up?

- A. I should tell my provider about any vaccines I recently received.
- B. I need to ask before getting any vaccine during pregnancy.
- C. If a vaccine contains live virus, it may not be appropriate during pregnancy.

D. Pregnancy means every type of vaccine is unsafe.

Correct Answer: D

The statement that every vaccine is unsafe during pregnancy is incorrect and requires follow-up. Vaccine decisions in pregnancy are individualized; they are not universally avoided. The other statements reflect appropriate caution and communication. The trap is using an absolute statement about all vaccines instead of considering vaccine type and clinical indication.

1622. A nurse is reviewing a child's chart before a vaccination visit. Which history finding is most important to report before administering the vaccine?

- A. The child cried for 10 minutes after the last injection.
- B. The child developed hives and wheezing after a previous vaccine dose.
- C. The child did not sleep well the night before the appointment.
- D. The child ate less breakfast than usual.

Correct Answer: B

Hives and wheezing after a previous vaccine suggest a serious allergic reaction and require prompt review before another dose is given. Crying, mild fatigue, or eating less at one meal are not in the same priority category. The correct answer focuses on a potentially dangerous prior response rather than minor, expected child behaviors.

1623. A nurse is teaching a newly hired practical nurse about vaccine documentation. Which entry is most important to include after administration?

- A. The name of the vaccine and the date it was given
- B. The color of the child's shirt during the visit
- C. The parent's opinion about the clinic schedule
- D. The weather outside the clinic at the time of the visit

Correct Answer: A

Accurate vaccine documentation must clearly identify what was given and when it was administered. This information is essential for continuity of care and future scheduling. The other options do not contribute meaningful clinical or legal vaccine records. The trap is overlooking the importance of precise immunization documentation.

1624. During a community vaccine clinic, a client says, "I am afraid I might faint during the injection because needles make me very nervous." Which action by the nurse is best?

- A.** Tell the client to leave and come back when less anxious.
- B.** Administer the vaccine while the client stands so it is over quickly.
- C.** Have the client seated or lying down during administration and observe afterward.
- D.** Skip the vaccine because anxiety is a contraindication.

Correct Answer: C

If a client is at risk for fainting, the nurse should help reduce injury risk by having the client seated or lying down during administration and monitoring afterward. Anxiety is not a contraindication to vaccination. Standing increases fall risk, and sending the client away delays care unnecessarily.

1625. A nurse is participating in vaccine storage monitoring at an outpatient clinic. Which observation requires immediate action?

- A.** The vaccine refrigerator temperature log has a gap for the current shift.
- B.** A parent asks whether soreness can occur after an injection.
- C.** A child wants to sit on the parent's lap during the shot.
- D.** An adult asks for water before receiving a vaccine.

Correct Answer: A

A missing temperature-log entry requires immediate action because proper storage conditions are essential to preserve vaccine effectiveness and safety procedures. The other options may need routine responses but are not urgent operational concerns. The trap is focusing on visible patient behavior instead of the system issue that could affect all stored vaccines.

1626. A practical nurse is reinforcing vaccine teaching to the parent of a school-age child. Which parent statement shows a need for further teaching?

- A.** Vaccines help the body prepare to fight certain infections.
- B.** Keeping immunizations current can help protect both my child and others.
- C.** If my child had a mild reaction before, I should report it at the next visit.
- D.** Once my child receives one dose of a vaccine, no further doses are ever needed.

Correct Answer: D

Some vaccines require more than one dose or later boosters, so the statement that one dose always completes protection is incorrect. The other statements reflect appropriate understanding of vaccine purpose and follow-up. The likely trap is overgeneralizing from a single-dose vaccine to all vaccines.

1627. A nurse is preparing an immunization for a child in a busy clinic. Before administration, the nurse notices that the syringe label does not match the vaccine listed in the child's record. Which action should the nurse take first?

- A.** Give the vaccine anyway if the child is due for several shots.
- B.** Ask the parent which vaccine the child usually receives.
- C.** Stop and verify the vaccine before administration.
- D.** Discard the child's record and create a new one.

Correct Answer: C

The nurse should stop and verify the vaccine before administration. A mismatch between the label and the record is a medication safety issue. The parent cannot substitute for formal verification, and giving the injection without clarification risks an error. The trap is feeling pressured by clinic pace instead of following safe administration steps.

1628. A child arrives for vaccination, and the parent reports the child is receiving high-dose steroid therapy for a serious condition. The nurse should recognize that this information is most important because it may affect which aspect of immunization planning?

- A. Whether all vaccines are automatically repeated later
- B. Whether some vaccines may need to be delayed or reviewed by the provider
- C. Whether the child can only receive oral vaccines
- D. Whether the vaccine must always be split into two half-doses

Correct Answer: B

High-dose steroid therapy can affect immune response and may require review of vaccine timing, especially for certain vaccines. It does not automatically mean all vaccines are repeated, that only oral vaccines are allowed, or that doses should be split. The key is recognizing immunosuppression as a factor that may change planning.

1629. A nurse is reviewing vaccine clinic procedures after a child developed sudden wheezing and generalized urticaria within minutes of an injection. Which preparedness measure is most critical for the clinic to have in place?

- A. A supply of stickers to calm children after injections
- B. A written emergency response plan with immediate access to emergency treatment
- C. A separate line for clients receiving more than one vaccine
- D. A policy requiring every client to wait exactly 5 minutes after vaccination

Correct Answer: B

Sudden wheezing and generalized urticaria suggest anaphylaxis, so the clinic must be prepared with an emergency response plan and immediate access to emergency treatment. Stickers and line flow are not the priority. A fixed 5-minute wait is not the key issue here; the essential point is readiness to identify and treat a severe reaction rapidly.

1630. A nurse is caring for a child whose family recently immigrated and has no reliable written vaccine record. The parent is unsure which vaccines the child previously received. Which principle should guide the nurse's next step?

- A. Estimate the schedule based on the child's age alone and document that it is exact.
- B. Treat verbal recall as complete proof of immunity for all vaccines.
- C. Use available records and provider-directed follow-up to determine needed immunizations safely.
- D. Assume the child is fully immunized because many countries use similar schedules.

Correct Answer: C

When records are incomplete or uncertain, the nurse should use whatever documentation is available and follow provider-directed procedures to determine needed immunizations safely. Age helps guide planning but does not make the history exact. Verbal recall alone may be unreliable, and assumptions about another country's schedule are unsafe. The trap is choosing convenience over accurate immunization assessment.

Care needs from newborn to older adult

1631. A nurse is reinforcing teaching for the parent of a 2-month-old infant. Which finding should the nurse identify as an expected developmental behavior at this age?

- A. Rolls from abdomen to back
- B. Smiles responsively at the parent
- C. Sits unsupported for several minutes
- D. Uses a pincer grasp to pick up a toy

Correct Answer: B

A social smile is expected around 2 months of age. Rolling, sitting unsupported, and using a pincer grasp occur later. A common trap is choosing a motor skill that develops after the social smile.

1632. A nurse is caring for a hospitalized preschooler. Which activity is most appropriate to help meet this child's developmental needs?

- A. Providing a picture book and allowing make-believe play
- B. Encouraging participation in a peer-led sports competition
- C. Assigning a detailed journal about feelings and future goals
- D. Offering complex strategy board games with strict rules

Correct Answer: A

Preschoolers learn and cope through play, especially imaginative play. Picture books and make-believe activities fit their developmental stage. Peer competition and complex rule-based activities are more appropriate for older children, while journaling about future goals is more fitting for adolescents.

1633. A nurse is reinforcing teaching about injury prevention for the parents of a toddler. Which statement by a parent indicates understanding?

- A. I will keep small objects out of my child's reach.
- B. I can switch to a booster seat once my child turns 2 years old.
- C. I should leave the bathroom door open so I can hear my child.
- D. I can let my child eat while walking as long as I watch closely.

Correct Answer: A

Toddlers explore by putting objects in their mouths, so keeping small objects out of reach helps prevent choking. The other choices are unsafe because they increase risk for injury or choking. The likely trap is assuming supervision alone makes a risky activity safe.

1634. A nurse is caring for an older adult client who reports difficulty hearing. Which action by the nurse is best when communicating with this client?

- A. Speak loudly into the client's ear
- B. Stand facing the client while speaking clearly
- C. Use medical terms so the message is brief
- D. Ask family members to answer for the client

Correct Answer: B

Facing the client and speaking clearly supports communication because the client can hear better and use visual cues such as lip movements and facial expression. Shouting can distort sound, medical jargon creates another barrier, and having family answer reduces client participation.

1635. A nurse is reinforcing teaching for the parents of a newborn. Which finding should the nurse tell the parents to report to the provider immediately?

- A. Sneezing several times during the day
- B. Acrocyanosis of the hands and feet
- C. Rectal temperature of 38.1°C (100.6°F)
- D. Hiccups after feeding

Correct Answer: C

A temperature of 38.1°C (100.6°F) in a newborn requires prompt evaluation because infection can progress quickly at this age. Sneezing, acrocyanosis, and hiccups can be normal newborn findings. The key is distinguishing expected transition findings from a possible sign of serious illness.

1636. A nurse is reinforcing anticipatory guidance with the parent of a school-age child. Which behavior is most characteristic of this stage of development?

- A. Parallel play with little interaction
- B. Strong focus on peer approval and accomplishment
- C. Egocentric thinking centered on self only
- D. Primary attachment expressed through separation anxiety

Correct Answer: B

School-age children commonly value peer relationships, achievement, and productivity. Parallel play is more typical of toddlers, egocentric thinking is more characteristic of younger children, and separation anxiety is more common in infancy and early toddlerhood.

1637. A nurse is reinforcing teaching with an adolescent about healthy growth and development. Which statement by the adolescent shows an expected developmental focus?

- A. I want my parents to choose all of my activities for me.
- B. I do not care what my friends think about me.
- C. I have been thinking a lot about who I am and how I fit in.
- D. I prefer toys that let me pretend I am a superhero.

Correct Answer: C

Adolescence is strongly associated with identity formation and questions about self-image, role, and belonging. Wanting parents to choose everything suggests less independence, not typical developmental progression. Pretend play is more associated with younger children, and claiming no concern for peers is less consistent with adolescent development.

1638. A nurse is planning teaching for a client in late adulthood. Which topic best supports health promotion for this age group?

- A. Safe medication use and fall prevention
- B. School readiness and language stimulation
- C. Prevention of shaken infant injury
- D. Peer pressure and body image management

Correct Answer: A

Older adults benefit from health promotion focused on safety, medication management, and fall prevention because these issues commonly affect function and independence. The other options are targeted to much younger age groups.

1639. A nurse is caring for a client who gave birth 8 hours ago and plans to breastfeed. Which action is most appropriate to support the newborn's nutritional needs?

- A. Encourage feeding on early hunger cues

- B. Limit feeding to every 6 hours to promote rest
- C. Give sterile water between feedings
- D. Delay the first feeding until milk volume increases

Correct Answer: A

Feeding on early hunger cues supports successful breastfeeding and meets the newborn's nutritional needs. Newborns should not have feedings delayed or spaced too far apart, and sterile water should not replace nutrition. A common trap is waiting for larger milk volume instead of encouraging early and frequent feeding.

1640. A nurse is talking with the caregiver of an 18-month-old child. Which behavior should the nurse recognize as expected for this age?

- A. The child consistently shares toys with other children
- B. The child says several single words and uses gestures
- C. The child writes short sentences independently
- D. The child understands the need to wait for turns in group games

Correct Answer: B

At about 18 months, children typically use several single words and gestures to communicate. Consistent sharing and turn-taking require more mature social development, and writing sentences is far beyond this age level.

1641. A nurse is reinforcing discharge teaching for the parents of a 4-year-old child. Which statement by a parent indicates a need for further teaching?

- A. I will keep cleaning supplies locked away.
- B. I can leave my child alone in the bathtub for a minute if the phone rings.
- C. I will make sure my child wears a helmet when riding a tricycle or bicycle.
- D. I will teach my child not to accept rides from strangers.

Correct Answer: B

A preschooler should never be left unattended in the bathtub because drowning can occur quickly. The other statements reflect appropriate safety teaching. The main trap is underestimating how quickly injury can occur when a caregiver steps away briefly.

1642. A nurse is assessing a 6-month-old infant. Which finding requires further evaluation?

- A. The infant bears some weight when held upright
- B. The infant transfers a toy from one hand to the other
- C. The infant has no head control when pulled to a sitting position
- D. The infant turns toward the sound of a rattle

Correct Answer: C

By 6 months, an infant should have good head control. Absence of head control at this age is concerning and needs further evaluation. Bearing weight, transferring objects, and turning toward sound are expected findings. The question requires recognizing which milestone delay is most significant.

1643. A nurse is caring for four clients during a community screening visit. Which client should the nurse see first?

- A. A 2-week-old newborn with poor feeding and fewer wet diapers than yesterday
- B. A 6-year-old child with a loose tooth and no pain
- C. A 15-year-old adolescent asking about acne skin care
- D. A 70-year-old adult requesting information about exercise classes

Correct Answer: A

A 2-week-old newborn with poor feeding and decreasing wet diapers may be showing dehydration or illness and should be assessed first. The other clients have nonurgent needs. The priority is based on both age-related vulnerability and the potential for rapid deterioration.

1644. A nurse is reinforcing teaching for the parents of a 3-month-old infant about sleep safety. Which statement by a parent requires immediate correction?

- A. I will place my baby on the back for sleep.
- B. I will keep soft blankets and pillows out of the crib.
- C. I will put my baby in the car seat to sleep overnight because my baby rests better there.
- D. I will use a firm sleep surface for naps and nighttime sleep.

Correct Answer: C

A car seat is not an appropriate routine sleep surface for overnight sleep. Safe sleep teaching includes placing the infant on the back on a firm surface and keeping soft objects out of the sleep area. The difficult part is recognizing the unsafe choice even though the parent describes it as helping the baby rest better.

1645. A nurse is assessing an older adult who lives alone. Which finding places the client at greatest risk for loss of independent function?

- A. Needs extra time to recall the date
- B. Reports two falls in the past month
- C. Prefers to eat dinner earlier in the evening
- D. Takes a daily afternoon nap

Correct Answer: B

Repeated recent falls signal a major threat to safety, mobility, and ability to remain independent. The other findings can be common age-related variations or personal habits and are less immediately threatening. A common trap is overemphasizing mild forgetfulness when the more urgent functional risk is falls.

1646. A nurse is reinforcing feeding instructions for the parents of a healthy 5-month-old infant. Which statement by a parent indicates correct understanding of the teaching?

- A. I will give small pieces of toast so my baby can practice chewing.
- B. I will put cereal in the bottle to help my baby sleep longer.
- C. I will wait to introduce solid foods until my baby shows readiness, such as good head control.
- D. I will start cow's milk now because my baby needs more iron.

Correct Answer: C

Good head control and signs of readiness support introducing solids, typically beginning with spoon-feeding rather than adding food to a bottle. Toast is a choking risk for a young infant, cereal should not be added to the bottle for sleep, and cow's milk is not appropriate as the main milk source for a 5-month-old infant.

1647. A nurse is collecting data at a well-child visit for a 15-month-old toddler. Which behavior is the best indicator of age-appropriate development?

- A. Uses several single words and walks without help
- B. Rides a tricycle and speaks in full sentences
- C. Skips on one foot and ties shoelaces
- D. Reads simple words and follows written directions

Correct Answer: A

By 15 months, a toddler commonly walks independently and uses a few words. The other choices describe skills expected in much older children: tricycle riding and full sentences in preschool years, skipping and tying shoelaces in later childhood, and reading simple words at school age.

1648. A nurse is reinforcing home safety teaching with the family of an 8-year-old child who rides a bicycle to a friend's house. Which instruction is most appropriate?

- A. Use a bicycle helmet on every ride, even short trips close to home.
- B. Ride against traffic so drivers can be seen more easily.
- C. Avoid drinking water during the ride to prevent stomach cramps.
- D. Stand while riding downhill to improve balance and speed.

Correct Answer: A

Helmet use on every ride is a key injury-prevention measure for school-age children. Riding against traffic is unsafe; bicyclists should follow traffic flow. Avoiding hydration is not recommended, and standing while riding downhill increases risk for loss of control.

1649. A nurse is reinforcing teaching with a 70-year-old client about promoting bowel regularity. Which client statement indicates understanding?

- A. I should limit activity when I feel constipated.
- B. I should wait until several days pass before increasing fluids.
- C. I should include fiber in my diet and stay physically active.
- D. I should use a laxative every day to prevent constipation.

Correct Answer: C

Adequate fiber, hydration, and physical activity help promote normal bowel elimination in older adults. Limiting activity can worsen constipation. Delaying fluids is not helpful, and routine daily laxative use can lead to dependence or other problems unless specifically prescribed.

1650. A nurse is caring for a 2-day-old newborn. Which finding should the nurse recognize as expected?

- A. Heart rate of 90/min while awake
- B. Bluish discoloration of the hands and feet
- C. Absence of startle response to noise
- D. Projectile vomiting after each feeding

Correct Answer: B

Acrocyanosis, or bluish hands and feet, is a common expected finding in the newborn period. A heart rate of 90/min while awake is low for a newborn, absent startle response is not expected, and projectile vomiting suggests a problem that needs evaluation.

1651. A nurse is reinforcing discharge teaching for a client who is 2 days postpartum and bottle-feeding. Which statement by the client indicates a need for further teaching?

- A. I may have vaginal bleeding that gradually becomes lighter over time.
- B. I should call the provider if I pass large clots or soak a pad quickly.
- C. I should begin breast stimulation to dry up my milk faster.
- D. I can use a supportive bra to decrease discomfort from breast fullness.

Correct Answer: C

A bottle-feeding postpartum client should avoid breast stimulation because it encourages milk production. A supportive bra is appropriate, and the client should expect lochia to gradually lessen while reporting heavy bleeding or large clots. The trap is choosing an option that sounds active and helpful, but stimulation works against suppression of lactation.

1652. A nurse is talking with the parent of a 3-year-old child who has started saying, "I can do it myself." Which response by the nurse is most appropriate?

- A.** This usually means your child is becoming spoiled and needs firmer discipline.
- B.** This is a common sign of growing autonomy, so offer simple safe choices when possible.
- C.** This behavior shows your child is not ready for social interaction with other children.
- D.** This suggests your child is regressing and may need complete assistance with dressing.

Correct Answer: B

Toddlers and young preschoolers commonly seek independence and control. Offering simple choices supports autonomy while maintaining safety. The other options misinterpret normal development as misbehavior, social delay, or regression.

1653. A nurse is preparing a teaching plan for a 13-year-old client newly diagnosed with type 1 diabetes mellitus. Which approach is most likely to support learning for this age group?

- A.** Direct all teaching to the parent because adolescents are usually unable to manage complex health information.
- B.** Provide teaching only after discharge so the adolescent is not overwhelmed in the clinic setting.
- C.** Include the adolescent in planning, allow questions about body image and peer concerns, and encourage increasing self-care participation.
- D.** Use the same concrete explanations that would be used for a preschooler.

Correct Answer: C

Adolescents benefit from involvement in decision-making and teaching that addresses independence, peer relationships, and body image. Excluding the adolescent, postponing all teaching, or using preschool-level teaching strategies does not match developmental needs.

1654. A nurse is assessing an older adult in a long-term care facility. Which finding is most consistent with delirium rather than dementia?

- A.** Gradual memory decline over several years
- B.** Sudden onset of confusion with fluctuating attention
- C.** Stable disorientation to time that has not changed for months
- D.** Progressive difficulty performing familiar tasks

Correct Answer: B

Delirium typically has an acute onset and fluctuating course, often with impaired attention. Dementia usually develops gradually and progressively over time, with a more chronic pattern. The key distinction is the sudden, fluctuating change in mental status.

1655. A nurse is reinforcing anticipatory guidance for the parent of a 6-year-old child. Which recommendation is most appropriate to support healthy development?

- A.** Encourage cooperative play, school participation, and opportunities to complete tasks successfully.
- B.** Expect the child to prefer solitary play and avoid comparing work with peers.
- C.** Provide all personal care for the child to prevent frustration with new tasks.
- D.** Limit contact with same-age peers to reduce exposure to competition.

Correct Answer: A

School-age children benefit from chances to build competence through school, peer interaction, and task completion. Solitary play is not the main developmental focus at this stage, and overassisting or isolating the child can interfere with growth in confidence and industry.

1656. A nurse in a prenatal clinic is reinforcing teaching about nutrition with a client in the second trimester of pregnancy. Which meal choice best reflects appropriate teaching?

- A. Coffee and toast because heavy meals should be avoided during pregnancy
- B. Peanut butter crackers and soda because snacks are easier than full meals
- C. Grilled chicken, cooked spinach, whole grains, and milk
- D. Soft cheese, deli turkey, and an energy drink

Correct Answer: C

A balanced meal with protein, iron-rich foods, grains, and calcium supports pregnancy nutrition. Coffee and toast or crackers and soda do not provide balanced nutrients. Soft cheese and deli meats may pose food-safety concerns unless selected and handled safely, and an energy drink is not an ideal prenatal choice.

1657. A nurse is assessing a 10-month-old infant during a clinic visit. Which finding requires the most immediate follow-up?

- A. The infant sits without support and transfers a toy from one hand to the other.
- B. The infant does not respond to his name and does not turn toward the parent's voice.
- C. The infant has stranger anxiety when approached by unfamiliar staff.
- D. The infant pulls to stand while holding onto furniture.

Correct Answer: B

Failure to respond to sound or voice at 10 months raises concern for possible hearing impairment and needs prompt follow-up because hearing affects language and social development. The other findings are expected at this age, including sitting independently, stranger anxiety, and pulling to stand.

1658. A nurse is caring for a 4-year-old child immediately after a tonsillectomy. Which observation requires priority intervention?

- A. The child swallows frequently and is restless.
- B. The child asks for an ice pop and cries when told to wait.
- C. The child reports a sore throat when swallowing.
- D. The child prefers to lie on the side and sleep.

Correct Answer: A

Frequent swallowing and restlessness after tonsillectomy can indicate bleeding and require immediate assessment. Sore throat is expected, and side-lying may help drainage. Wanting a cold treat is not the priority concern. The trap is mistaking a subtle bleeding sign for ordinary postoperative discomfort.

1659. A nurse is reinforcing teaching for the caregiver of an older adult with mild dysphagia after a stroke. Which caregiver statement indicates correct understanding of aspiration prevention?

- A. I will offer thin liquids because they are easier to swallow quickly.
- B. I will have the client lie back slightly during meals to help food go down.
- C. I will keep the client upright during meals and for a period afterward.
- D. I will use a straw for all drinks to move fluid past the mouth faster.

Correct Answer: C

Keeping the client upright during meals and afterward reduces aspiration risk. Thin liquids may be harder to control for some clients with dysphagia, lying back increases aspiration risk, and straws are not appropriate for every client. The safest general principle here is upright positioning.

1660. A nurse is caring for four clients in a clinic. Which client should the nurse assess first?

- A. A 2-week-old newborn whose parent reports a rectal temperature of 38.1°C (100.6°F)
- B. A 7-year-old child with a pruritic rash after playing outside yesterday
- C. A 16-year-old adolescent requesting information about acne treatment
- D. A 68-year-old client who wants teaching about starting a walking program

Correct Answer: A

Fever in a newborn is urgent and requires prompt assessment because young infants can deteriorate quickly and may have a serious infection. The other clients need care, but their problems are less immediately life-threatening. This is a priority question based on age-related vulnerability.

Communication barriers

1661. A nurse is providing discharge teaching to a client who recently immigrated and speaks limited English. The client's adult daughter offers to interpret. Which action by the nurse is best?

- A. Use the daughter because family members know the client's history best
- B. Speak more loudly and slowly while maintaining eye contact with the daughter
- C. Obtain a qualified medical interpreter to assist with the teaching
- D. Give the client written instructions in English to review later at home

Correct Answer: C

A qualified medical interpreter is best because discharge teaching must be accurate, confidential, and clearly understood. Family members may omit, add, or change information, even with good intentions. Speaking louder does not solve a language barrier, and English-only written instructions are not appropriate when the client has limited English proficiency.

1662. An older adult client with hearing loss is being admitted to the unit. Which nursing action will best improve communication during the admission interview?

- A. Stand behind the client to reduce visual distractions
- B. Use short sentences while facing the client directly
- C. Shout each question clearly into the client's ear
- D. Ask a family member to answer most of the questions

Correct Answer: B

Facing the client directly and using short, clear sentences helps a client with hearing loss both hear and read facial cues. Shouting can distort speech and may feel disrespectful. Standing behind the client removes helpful visual cues, and relying on family members reduces direct assessment of the client.

1663. A nurse is teaching a client about a newly prescribed inhaler. The client nods during the explanation but looks confused. Which response by the nurse is best?

- A. Repeat the instructions using the same wording so the client can memorize them
- B. Ask, "Do you understand?" and move on if the client says yes
- C. Demonstrate the inhaler and ask the client to show how it will be used
- D. Leave the package insert at the bedside for the client to read later

Correct Answer: C

Return demonstration is the best way to confirm understanding and overcome possible communication barriers. A client may nod out of politeness, embarrassment, or anxiety without truly understanding. Asking only "Do you understand?" often leads to a yes answer even when comprehension is poor.

1664. A nurse is collecting a health history from a client who avoids eye contact and gives very brief answers. The nurse learns that in the client's culture, direct eye contact with authority figures may be considered disrespectful. Which nursing action is most appropriate?

- A. Insist that the client make eye contact to show honesty
- B. Document that the client is uncooperative and end the interview
- C. Adjust communication style while continuing to assess the client's understanding
- D. Ask a second nurse to repeat the same questions more firmly

Correct Answer: C

When cultural norms affect communication, the nurse should adapt communication style and continue assessment without assuming dishonesty or refusal. Avoiding eye contact can reflect respect rather than lack of cooperation. Forcing eye contact may increase discomfort and worsen communication.

1665. A postoperative client says, "I'm in too much pain to listen right now." The nurse needs to teach the client how to use an incentive spirometer. Which action is best?

- A. Provide the full teaching immediately because postoperative instructions cannot wait
- B. Medicate the client for pain if appropriate and return when the client can better focus
- C. Ask the client's roommate to explain how the device works
- D. Leave the spirometer at the bedside with printed directions only

Correct Answer: B

Pain is a major communication and learning barrier because it reduces attention and retention. Managing pain and then teaching when the client can participate improves understanding and safety. Teaching through severe pain, relying on a roommate, or using printed directions alone is less effective.

1666. A client with limited English proficiency is scheduled for a consent discussion about a procedure. Which approach by the nurse is most appropriate?

- A. Ask a bilingual staff member from housekeeping to interpret
- B. Use simple medical abbreviations because they are easier to translate
- C. Arrange for a qualified interpreter and speak directly to the client
- D. Explain the procedure to the family first so they can decide what the client needs to know

Correct Answer: C

A qualified interpreter should be used for important discussions, and the nurse should still address the client directly to preserve respect and therapeutic communication. Untrained bilingual staff may misinterpret information. Medical abbreviations are not easier to translate, and teaching the family instead of the client undermines the client's role.

1667. During medication teaching, a client keeps looking at the floor and twisting a blanket. The client says, "I know you're busy. It's probably nothing." Which response by the nurse best addresses a likely communication barrier?

- A. "If you had a question, you would ask it."

B. "Let's pause. What concerns or questions do you have about this medicine?"

C. "Just read the label again after I leave."

D. "You need to pay attention because this information is important."

Correct Answer: B

The client's words and body language suggest anxiety or reluctance to speak up. Inviting questions in a calm, open-ended way helps reduce that barrier. The other responses are dismissive or shaming and are more likely to shut down communication.

1668. A nurse is teaching a client who has aphasia after a stroke. Which strategy is most appropriate?

- A. Ask several questions quickly so the client does not lose focus
- B. Finish the client's sentences to reduce frustration
- C. Use simple questions and allow extra time for responses
- D. Speak to the family instead of the client whenever possible

Correct Answer: C

Clients with aphasia benefit from simple communication and extra time to respond. Rushing, finishing sentences, or bypassing the client can increase frustration and reduce dignity. The nurse should continue to communicate with the client directly as much as possible.

1669. A nurse enters a room to teach a client about wound care. The television is on loudly, two visitors are talking, and the client is trying to eat lunch. What should the nurse do first?

- A. Begin teaching because repetition can make up for distractions
- B. Ask the visitors to perform the wound care teaching instead
- C. Reduce environmental distractions and choose a better time if needed
- D. Provide detailed written instructions and skip verbal teaching

Correct Answer: C

Environmental distraction is a communication barrier. Reducing noise, limiting interruptions, and choosing a more suitable time improve attention and understanding. Starting despite major distractions decreases retention, and written instructions alone may not be enough.

1670. A client with chronic hearing loss says, "I hear you talking, but the words blur together." Which nursing action is best?

- A.** Lower your voice to a whisper so the sounds are sharper
- B.** Rephrase the message in clear, short statements and confirm understanding
- C.** Cover your mouth to block background visual stimulation
- D.** Speak to the client from the doorway to avoid crowding

Correct Answer: B

If words blur together, rephrasing in clear, short statements and checking understanding is helpful. Many clients with hearing loss rely on facial and lip cues, so covering the mouth or speaking from the doorway interferes with communication. Whispering usually makes understanding harder.

1671. A nurse must teach a client how to self-administer insulin. The client has limited English proficiency, nods politely to every statement, and says, "Yes, yes," before the interpreter finishes translating. Which nursing action is best to evaluate whether communication has actually occurred?

- A.** Accept the nodding as evidence that the client understands
- B.** Ask the interpreter whether the client seems intelligent enough to learn the task
- C.** Have the client demonstrate the injection process using teach-back with the interpreter present
- D.** Provide a faster explanation so the client does not become embarrassed

Correct Answer: C

Polite nodding may reflect respect, anxiety, or a desire to avoid embarrassment rather than real understanding. The best way to evaluate communication is to use teach-back or return demonstration with the qualified interpreter present. Assuming understanding is a common trap in cross-language teaching.

1672. A nurse is assessing a client who has expressive aphasia and becomes visibly frustrated when unable to answer quickly. Which action by the nurse is best?

- A.** Ask the family to answer all questions to save time
- B.** Move on to a different topic each time the client hesitates
- C.** Use yes-or-no questions when appropriate and allow time for the client to respond
- D.** Repeat the question louder each time the client struggles

Correct Answer: C

Yes-or-no questions and adequate response time can reduce frustration while still supporting communication with a client who has expressive aphasia. Louder speech does not help a language-production problem. Having family answer all questions or constantly changing topics reduces direct assessment and can increase the client's distress.

1673. An adolescent client newly diagnosed with diabetes shrugs during teaching and says, "Whatever. My grandmother had it and she was fine." The client then begins texting while the nurse explains diet changes. Which communication barrier is most important for the nurse to address?

- A.** Lack of available printed materials
- B.** Developmental and motivational resistance affecting communication
- C.** Hearing impairment related to distraction
- D.** Inability to speak English fluently

Correct Answer: B

The client's dismissive attitude and disengagement suggest resistance, limited readiness, or developmental factors affecting communication. This barrier is more central than printed materials or hearing problems. There is no evidence of an English-language barrier in the stem.

1674. A nurse is preparing preoperative teaching for a client who is in severe pain, has limited English proficiency, and wears hearing aids. Which nursing plan best reduces communication barriers?

- A. Teach immediately so all barriers are addressed at once before surgery
- B. Ask a family member to translate while the client rests with hearing aids removed
- C. Manage pain, ensure the hearing aids are functioning, and use a qualified interpreter for the teaching
- D. Postpone all teaching until after surgery because communication is too difficult now

Correct Answer: C

This plan addresses multiple barriers directly: pain control improves attention, functioning hearing aids improve reception, and a qualified interpreter addresses the language barrier. Teaching immediately despite uncontrolled barriers is ineffective, and using family as interpreter is not the best choice. Postponing all teaching may delay important preparation unnecessarily when barriers can be reduced.

1675. A nurse is giving instructions to a client about how to collect a clean-catch urine specimen. The client speaks English fluently but keeps saying, "I forgot my glasses at home," and squints at the printed handout. Which action by the nurse is best to reduce this communication barrier?

- A. Ask a family member to read the instructions to the client later
- B. Read the instructions aloud and have the client repeat the steps back
- C. Speak more loudly so the client can better understand the handout
- D. Give the client the handout and return later to see whether the specimen was collected

Correct Answer: B

The barrier is difficulty accessing written information, not hearing loss or language difference. Reading the instructions aloud and using teach-back checks whether the client understood the steps. Choice A delays teaching and depends on someone else. Choice C addresses the wrong barrier. Choice D does not confirm understanding and may lead to an incorrect specimen collection.

1676. A nurse is reviewing home blood pressure monitoring with a client who says, "I don't need this. My pressure is only high when I come to the clinic." The client crosses their arms and interrupts the nurse several times. Which barrier to communication should the nurse address first?

- A. The client's lack of hearing ability
- B. The client's denial about the health problem
- C. The nurse's use of medical terminology
- D. The absence of written teaching materials

Correct Answer: B

The client's statements and behavior suggest denial or resistance, which can block meaningful communication and learning. Addressing this barrier first helps the nurse explore the client's concerns and readiness to learn. There is no evidence of hearing loss, complex terminology, or missing handouts in the scenario.

1677. A nurse is teaching an older adult client about a low-sodium diet. The client smiles and says, "I understand," but later asks whether canned soup and deli meat are good low-salt choices. Which factor is the most likely communication barrier?

- A. The client may be pretending to understand out of embarrassment
- B. The client is unable to hear the nurse's voice
- C. The nurse is speaking too slowly
- D. The client has expressive aphasia

Correct Answer: A

The client's later question shows the earlier statement of understanding was inaccurate, likely because the client was embarrassed to admit confusion. This is a common communication barrier. The scenario does not indicate hearing loss, slow speech, or aphasia.

1678. A nurse is interviewing a client who recently received a serious diagnosis. The client repeatedly says, "What?" and asks the nurse to repeat simple questions. The room is quiet, and the client has no history of hearing impairment. Which response by the nurse is best?

- A. Speak more loudly so the client can hear each question
- B. Delay detailed teaching and acknowledge that stress can interfere with communication
- C. Ask a family member to answer questions for the client
- D. Continue the interview quickly to avoid increasing the client's anxiety

Correct Answer: B

Acute stress can reduce concentration and make it hard for a client to process information, even without hearing loss. The best response is to recognize the barrier, provide emotional support, and postpone nonurgent teaching if needed. Speaking louder treats the wrong problem, and relying on family or rushing the interview can worsen communication.

1679. A nurse is teaching a client about a new diagnosis of hypertension. The nurse says, "You need to reduce sodium, follow the DASH diet, and monitor for target-organ effects." Which aspect of the nurse's communication is most likely creating a barrier?

- A. Using abstract and technical language
- B. Speaking too softly
- C. Allowing too much time for questions
- D. Maintaining too much eye contact

Correct Answer: A

Terms such as "sodium," "DASH diet," and especially "target-organ effects" may be unclear to many clients if not explained in plain language. Jargon is a common communication barrier. The stem does not suggest problems with volume, excessive time, or eye contact.

1680. A client who follows a strict religious fast is scheduled for nutrition teaching just before sunset. The client keeps glancing at the clock and gives only one-word answers. Which nursing interpretation is most appropriate?

- A. The client is showing signs of cognitive decline

- B. The client is probably not interested in learning
- C. The timing of teaching may be interfering with communication
- D. The client is refusing care and should be documented as noncompliant

Correct Answer: C

The client's behavior may reflect distraction related to a meaningful religious practice and poor timing of the interaction. Recognizing timing and competing concerns as barriers helps the nurse adjust the plan. The scenario does not support cognitive decline or deliberate refusal, and labeling the client as noncompliant is premature.

1681. A nurse is explaining cast care to a school-age child. The child's parent answers every question before the child can respond, and the child becomes quiet and looks away. What is the nurse's best action?

- A. Direct all teaching to the parent because the parent is responsible for care
- B. Ask the parent to leave immediately so the child can speak freely
- C. Include the child directly with simple questions and age-appropriate explanations
- D. Stop the teaching session because the child is not cooperating

Correct Answer: C

The barrier is that the child is not being given an opportunity to participate. The nurse should include the child in developmentally appropriate communication while still involving the parent. Directing all teaching only to the parent can increase disengagement, and removing the parent or stopping teaching is unnecessary and potentially counterproductive.

1682. A nurse is preparing discharge instructions for a client who says, "I can read, but forms like this make no sense to me." Which nursing action best addresses the likely communication barrier?

- A. Provide more detailed written materials for the client to review alone
- B. Use plain language and break instructions into small, concrete steps
- C. Ask the client to sign that the instructions were received

D. Explain that the information is standard and should be easy to follow

Correct Answer: B

The client is describing difficulty understanding health information, so the nurse should simplify wording and present information in manageable steps. Giving more complex written material, focusing on signatures, or minimizing the client's concern does not remove the communication barrier.

1683. A nurse asks a client, "Do you have any questions about your new heart medicine?" The client says no. Later, the client states, "I thought I was only supposed to take it when I feel my heart racing." Which communication strategy by the nurse would have been most effective initially?

- A.** Giving the client a longer medication handout
- B.** Asking the client to repeat in their own words how the medicine should be taken
- C.** Speaking faster to cover all the instructions
- D.** Avoiding eye contact so the client feels less pressured

Correct Answer: B

Having the client explain the instructions in their own words is the best way to assess actual understanding and uncover hidden confusion. A yes-or-no question can miss misunderstanding. More written material, rushing, or avoiding eye contact would not effectively evaluate communication.

1684. A nurse is interviewing a client from a background different from the nurse's own. The nurse notices the client pauses for a long time before answering and rarely asks questions. Which assumption by the nurse would create the greatest communication barrier?

- A.** The client may need more time to respond
- B.** The client may prefer to show respect by listening quietly
- C.** The client is uninterested and probably did not understand the questions
- D.** The nurse should assess understanding without relying only on verbal enthusiasm

Correct Answer: C

Assuming that quiet behavior means disinterest or lack of understanding can lead to miscommunication and biased care. The other options reflect more open, culturally sensitive interpretations. The barrier here is the nurse's inaccurate assumption.

1685. A client with mild dementia is most attentive in the morning but becomes restless and distracted late in the day. The nurse must teach the client how to use a walker safely. When should the nurse plan the teaching?

- A.** Late afternoon, when the unit is busiest
- B.** During the evening meal so the client is occupied
- C.** In the morning, when the client is most focused
- D.** At bedtime, when the room is dark and quiet

Correct Answer: C

Teaching should be scheduled for the time when the client can best attend and process information. In this scenario, morning is clearly the best period. Busy, mealtime, or bedtime teaching would increase barriers related to distraction, fatigue, or confusion.

1686. A nurse is teaching a client about foot care. The client speaks clearly and answers appropriately, but repeatedly rubs the temples and closes the eyes. The client received pain medication 15 minutes ago. Which interpretation is most appropriate?

- A.** The client is bored and does not value the teaching
- B.** Medication effects may be interfering with the client's ability to learn
- C.** The client has a language barrier
- D.** The client is demonstrating defiant behavior

Correct Answer: B

Drowsiness or reduced alertness after pain medication can interfere with concentration and learning. This physiologic barrier should be recognized before continuing teaching. The signs do not point to language problems, boredom, or defiance.

1687. A nurse must explain pretest dietary restrictions to a client who has limited vision and becomes anxious when handed printed instructions. Which plan best reduces communication barriers?

- A. Read the instructions aloud in short sections and ask the client to describe the plan back
- B. Give the client the written page to review privately and return later
- C. Ask a roommate to go over the handout with the client
- D. Skip the teaching because the client is too anxious to learn

Correct Answer: A

The client has a visual barrier and anxiety, so the nurse should adapt the communication method by reading the instructions aloud in manageable parts and checking understanding. Leaving printed material alone, delegating teaching to a roommate, or omitting needed teaching are inappropriate.

1688. A nurse asks a client about alcohol intake. The client's spouse answers, "Oh, just socially," while the client stays silent and looks tense. Which factor is the most important communication barrier to address?

- A. The client may lack reading ability
- B. The presence of the spouse may be limiting honest communication
- C. The nurse used too many closed-ended questions
- D. The client likely has memory loss

Correct Answer: B

Sensitive topics are often harder to discuss honestly when another person answers for the client or is present during the interview. Privacy is the main barrier here. The stem does not suggest reading problems or memory loss, and although question style matters, the spouse's presence is the bigger obstacle.

1689. A nurse is giving instructions after an immunization to a client who appears frightened of needles. The client watches the bandage being applied and says, "Can you just tell me later?" What is the nurse's best response?

- A. Provide all teaching immediately because the client is still present

- B. Tell the client the instructions are simple and should be easy to remember

- C. Wait until the client's anxiety decreases, then give brief instructions and verify understanding

- D. Ask another staff member to repeat the same explanation at a faster pace

Correct Answer: C

Anxiety can temporarily impair attention and information processing. The nurse should wait until the client is calmer, then present concise instructions and check understanding. Forcing teaching during peak anxiety or dismissing the client's distress increases the communication barrier.

1690. A nurse is obtaining a health history from a client who speaks in a regional dialect unfamiliar to the nurse. After several answers, the nurse realizes key details may have been misunderstood. Which action is best?

- A. Pretend to understand to avoid embarrassing the client
- B. Rephrase the questions, ask for clarification of unclear words, and summarize what was heard
- C. End the interview and document the client as a poor historian
- D. Ask a visitor in the room to interpret the client's meaning

Correct Answer: B

When dialect differences create misunderstanding, the nurse should openly seek clarification, restate information, and verify meaning. Pretending to understand risks inaccurate care. Labeling the client unfairly or using a visitor to interpret meaning is not the best approach.

Learning barriers

1691. A nurse is preparing to teach a client who was recently diagnosed with hypertension about a new low-sodium diet. Which action best addresses a possible learning barrier before teaching begins?

- A. Ask the client what they already know about managing hypertension
- B. Provide a detailed handout using medical terminology

- C. Teach all diet changes in one session to avoid repetition
- D. Begin by explaining the long-term complications of hypertension

Correct Answer: A

Assessing prior knowledge helps the nurse identify what the client understands, what misconceptions exist, and where teaching should start. This is a key first step in addressing learning barriers. A detailed handout with medical terms may increase confusion, and teaching everything in one session can overwhelm the client. Explaining complications may be useful later, but it does not first identify the learner's starting point.

1692. A nurse is teaching a postoperative client how to use an incentive spirometer. The client keeps looking away, grimacing, and shifting in bed. Which nursing action is most appropriate?

- A. Continue the teaching quickly so the client gets all instructions
- B. Ask the client to sign the teaching form and return later
- C. Address the client's pain before continuing the teaching session
- D. Tell the client the exercise is necessary to prevent pneumonia

Correct Answer: C

Pain is a common learning barrier because it reduces attention and concentration. Managing pain first improves the client's ability to participate and retain information. Continuing quickly or focusing only on the importance of the exercise ignores the barrier. Asking for a signature does not ensure understanding.

1693. A nurse is teaching a client who speaks limited English about wound care. The client's adult child is visiting and offers to interpret. Which action should the nurse take?

- A. Use the adult child to save time since the client trusts them
- B. Request a qualified medical interpreter for the teaching session
- C. Speak very slowly and use louder volume during the explanation
- D. Provide an English-language pamphlet with pictures only

Correct Answer: B

A qualified medical interpreter is the safest choice to reduce misunderstanding during client teaching when there is a language barrier. Family members may omit, add, or misinterpret information, even when well intentioned. Speaking louder does not overcome a language difference, and an English pamphlet alone may be inadequate.

1694. A nurse is teaching a client with heart failure about daily weights. Which client statement suggests a health literacy barrier?

- A. I weigh myself after breakfast because that's when I remember.
- B. I don't own a scale at home.
- C. I don't understand what you mean by fluid retention.
- D. I would rather write my weight on a calendar than in a notebook.

Correct Answer: C

Not understanding a common teaching term such as "fluid retention" suggests limited comprehension of health-related language and points to a health literacy barrier. Weighing after breakfast is a technique issue, not necessarily literacy. Not owning a scale is a resource barrier. Preferring a calendar is a learning preference, not a literacy problem.

1695. A client newly diagnosed with type 2 diabetes says, "I know what happened to my father, and I don't want to hear any of this." Which barrier to learning is most clearly present?

- A. Psychological resistance related to fear or denial
- B. Cognitive impairment affecting memory
- C. Auditory deficit interfering with communication
- D. Low reading ability limiting comprehension

Correct Answer: A

The client's statement shows emotional resistance, likely fear, denial, or both. These psychological factors can block learning even when information is available. There is nothing in the scenario to suggest memory problems, hearing loss, or reading difficulty. A common trap is to focus on refusal as noncompliance rather than recognizing the emotional barrier behind it.

1696. A nurse is teaching a client how to administer an inhaler. Which method best verifies that the client learned the skill?

- A.** Ask the client whether the instructions were clear
- B.** Have the client repeat the steps out loud from memory
- C.** Observe the client demonstrate use of the inhaler
- D.** Give the client a brochure to review at home

Correct Answer: C

Return demonstration is the best way to verify psychomotor learning. It shows whether the client can actually perform the skill safely and correctly. Saying the instructions were clear or repeating steps may not reveal technique errors. A brochure supports reinforcement but does not confirm learning.

1697. The nurse is teaching a client about a new medication regimen. Which approach is most appropriate to reduce overload when the client appears anxious and has trouble concentrating?

- A.** Provide all instructions at once so the client can see the full plan
- B.** Break the information into short sessions and focus on key points
- C.** Ask the prescriber to explain the medications instead
- D.** Wait until discharge paperwork is ready, then review everything together

Correct Answer: B

Anxiety can interfere with concentration and retention. Dividing teaching into brief sessions with a few essential points at a time helps reduce overload and improves understanding. Giving all instructions at once or waiting until discharge can worsen stress and reduce retention. Referring teaching away without first adapting the approach does not address the barrier.

1698. A nurse is preparing discharge teaching for a client who has mild hearing loss and wears hearing aids. Which action should the nurse take first to decrease this learning barrier?

- A.** Sit facing the client and confirm the hearing aids are in place and working
- B.** Provide all instructions in writing and avoid verbal teaching
- C.** Stand near the doorway and speak in a louder-than-normal voice
- D.** Ask the unlicensed assistive personnel to repeat the teaching later

Correct Answer: A

The first step is to optimize communication by ensuring the hearing aids are available and functioning and by facing the client so visual cues can be used. Written materials may help but should not automatically replace verbal teaching. Shouting can distort speech and is not the best approach. Delegating the teaching follow-up is inappropriate.

1699. A nurse is teaching a client with limited reading ability about a new dressing change. Which instruction method is best?

- A.** Give a detailed printed packet and ask the client to study it independently
- B.** Use plain language, show the steps, and ask the client to perform a return demonstration
- C.** Use medical terms consistently so the client learns the correct vocabulary
- D.** Provide only verbal instructions to avoid embarrassing the client

Correct Answer: B

For a client with limited reading ability, plain language, demonstration, and return demonstration are effective ways to reduce the literacy barrier and verify understanding. A detailed printed packet or heavy medical terminology may increase confusion. Verbal instructions alone may help somewhat, but they do not verify skill performance and are less effective than demonstration.

1700. A nurse is teaching a client about self-injection of insulin. During the session, the client says, "I can do it," but avoids touching the syringe and changes the subject each time the nurse offers practice. Which interpretation is most accurate?

- A. The client has understood the instructions completely
- B. The client is showing a likely anxiety-related barrier to learning the skill
- C. The client needs written materials instead of demonstration
- D. The client is refusing care and should be reported immediately

Correct Answer: B

Avoiding hands-on practice despite verbal agreement suggests anxiety or fear about performing the skill, which is a learning barrier. Verbal statements alone do not confirm mastery. Switching to written materials does not address the apparent fear. The scenario does not support immediate labeling as refusal; the more accurate first interpretation is an emotional barrier.

1701. A nurse must teach a client with newly diagnosed asthma how to use a peak flow meter. The client has severe shortness of breath and is using accessory muscles to breathe. What is the nurse's priority action?

- A. Begin teaching immediately because the client needs the information urgently
- B. Ask the client to read the instruction booklet while waiting for treatment
- C. Stabilize the client's respiratory status before attempting teaching
- D. Teach the family member instead and document the client was unavailable

Correct Answer: C

Physiologic stability comes before teaching. A client in significant respiratory distress cannot focus well and may deteriorate if teaching is attempted first. Written materials and substitute teaching to family do not solve the immediate problem. The key principle is that acute symptoms can be a major learning barrier and must be addressed first.

1702. A nurse is reinforcing discharge teaching about a complicated medication schedule to a client who nods throughout the conversation. When asked if everything is clear, the client says yes. Which action by the nurse best assesses whether a learning barrier is still present?

- A. Accept the client's statement because agreement indicates understanding
- B. Ask the client to explain in their own words when each medication will be taken
- C. Repeat the same instructions more slowly using the same wording
- D. Document that teaching is complete since the client did not ask questions

Correct Answer: B

Having the client teach back the schedule in their own words is the best way to uncover misunderstanding, limited health literacy, memory problems, or passive agreement. Nodding and saying "yes" do not prove understanding. Repeating the same wording may not fix the barrier if comprehension is the issue. Lack of questions can be misleading, especially when clients are embarrassed or overwhelmed.

1703. A nurse is planning teaching for a client from a cultural background in which health decisions are commonly discussed with family elders. The client keeps saying, "I need to talk with my family first." Which nursing action best addresses this potential learning barrier while supporting client-centered care?

- A. Tell the client that personal independence is necessary for successful self-care
- B. Delay all teaching until the family makes the decisions

C. Explore the client's preferences for family involvement and include them in teaching as appropriate

D. Teach only the eldest family member because that person will make the decisions

Correct Answer: C

The best response is to assess the client's preferences and incorporate desired family involvement. Cultural expectations can affect readiness and decision-making, so teaching should be adapted rather than forced into one model. Insisting on independence may create conflict, delaying all teaching may miss opportunities, and automatically teaching only one family member assumes too much without confirming the client's wishes.

1704. A nurse is reinforcing teaching for a client who will need to follow a new diet after discharge. The client repeatedly says, "Just tell my spouse. I never remember anything when my sugar is low." The client is pale, shaky, and diaphoretic. Which action should the nurse take first?

A. Ask the spouse to write down all instructions for the client

B. Provide the teaching in short, simple sentences right away

C. Treat the suspected hypoglycemia before continuing the teaching

D. Give the client a printed handout to review later

Correct Answer: C

Hypoglycemia can impair concentration, memory, and ability to learn. The priority is to address the client's immediate physiologic problem before teaching. Asking the spouse to help or giving short instructions may seem useful, but teaching is unlikely to be effective while the client is symptomatic.

1705. A nurse is teaching an older adult how to care for a new colostomy. The client says, "I forgot my glasses, and these pages are too blurry to read." Which nursing action is best?

A. Continue the teaching and speak more loudly

B. Delay all teaching until the next clinic visit

C. Ask a family member to read the instructions to the client

D. Use large-print visual materials and demonstrate the steps

Correct Answer: D

The client has a visual barrier to learning, so the nurse should adapt the teaching method by using large-print materials and demonstration. Speaking louder does not address vision. Delaying all teaching is unnecessary if the nurse can modify the approach. Having a family member read the instructions does not ensure the client can perform self-care independently.

1706. A nurse is preparing to teach a client about home care after a newly applied short leg cast. Before the teaching begins, the client says, "My pain is about an 8 out of 10 right now." Which action is most appropriate?

A. Continue the teaching quickly before the next medication is due

B. Ask the family member to listen carefully and explain it to the client later

C. Address the client's pain, then begin teaching when the client is more comfortable

D. Provide the written instructions now and return only if the client has questions

Correct Answer: C

Severe pain interferes with attention and learning. The nurse should reduce this barrier first by addressing pain, then teach when the client can focus. Choice A ignores the barrier, choice B shifts teaching away from the client, and choice D assumes written material alone is enough despite the client's current inability to concentrate.

1707. A nurse is reinforcing teaching for a client who needs to follow a new medication schedule at home. The client says, "I never finished school, so don't give me those big medical words." Which response by the nurse is best?

A. Use simple, everyday language and ask the client to explain the plan back in their own words

B. Provide the full printed medication monograph for the client to review later

C. Ask a family member to manage all medications for the client

D. Repeat the same instructions more slowly using the original terminology

Correct Answer: A

The best response is to match the teaching to the client's literacy level by using plain language and checking understanding with teach-back. Choice B increases the barrier, choice C removes the client from self-care without first assessing ability, and choice D slows the message but does not make it more understandable.

1708. A nurse is teaching crutch walking to a client who appears restless and repeatedly glances at the door. The client says, "My ride is outside waiting, so can we make this fast?" Which nursing action best addresses the learning barrier?

- A.** Shorten the teaching, focus on the most essential safety points, and arrange follow-up reinforcement
- B.** Continue the full teaching plan because discharge teaching must be completed all at once
- C.** Ask the driver to come inside and receive the teaching instead of the client
- D.** Delay all teaching until the client's next clinic visit several weeks later

Correct Answer: A

Time pressure can reduce attention and retention. The nurse should prioritize essential safety teaching now and plan reinforcement later. Choice B ignores the barrier, choice C bypasses the client, and choice D delays necessary instruction too long.

1709. A nurse is preparing to teach a client about a newly prescribed low-fat diet. The client says, "I left my dentures at home, and my mouth hurts when I chew." Which interpretation is most accurate?

- A.** The client is refusing to learn
- B.** The client has a physical comfort issue that may interfere with learning
- C.** The client has a language barrier that prevents teaching
- D.** The client is demonstrating adequate readiness to learn

Correct Answer: B

Discomfort related to the mouth and dentures is a physical problem that can distract the client and interfere with the ability to focus during teaching. Choice A assumes refusal, choice C identifies the wrong barrier, and choice D overlooks an obvious obstacle to learning readiness.

1710. A nurse is teaching a client with newly diagnosed epilepsy about seizure precautions. During the teaching session, the client keeps asking the same question and appears drowsy. The medication record shows the client received a sedative 20 minutes ago. Which action is best?

- A.** Continue teaching and provide more detailed written material
- B.** Stop the session and return later when the client is more alert
- C.** Ask the client to sign that the teaching was completed
- D.** Speak louder so the client can stay focused

Correct Answer: B

Sedation and drowsiness interfere with concentration and retention, so the nurse should postpone teaching until the client is more alert. Choice A adds information when the client cannot process it, choice C documents completion without effective learning, and choice D confuses decreased alertness with hearing difficulty.

1711. A nurse is planning discharge teaching for a client who must change a sterile dressing at home. The client says, "I get nervous when people explain too many steps at once." Which teaching plan is most appropriate?

- A.** Explain the entire procedure once from start to finish without interruption
- B.** Break the skill into small steps, demonstrate each part, and allow return demonstration
- C.** Give only written instructions so the client can study independently
- D.** Teach the family member instead because the client feels nervous

Correct Answer: B

When anxiety and overload interfere with learning, breaking the task into smaller parts with demonstration and practice improves understanding. Choice A increases overload, choice C may not fit the client's needs, and choice D unnecessarily excludes the client from learning.

1712. A nurse is reinforcing teaching about foot care to a client with diabetes. The client avoids eye contact and says, "If I end up losing a toe, that's just God's will." Which barrier to learning should the nurse recognize first?

- A. Visual impairment
- B. Lack of transportation
- C. Low motivation related to personal beliefs about health and outcomes
- D. Inability to speak English

Correct Answer: C

The client's statement suggests a belief that personal actions may not affect outcomes, which can reduce motivation to learn and follow self-care teaching. There is no evidence of visual impairment, transportation problems, or language difficulty in the scenario.

1713. A nurse is teaching a client about signs of preterm labor. Midway through the session, the client begins crying and says, "I'm too scared to think straight." Which action is most appropriate?

- A. Pause the teaching, acknowledge the client's fear, and resume when the client is calmer
- B. Finish the teaching because the information is urgent
- C. Tell the client that worrying will make learning harder
- D. Leave the written materials and end the interaction

Correct Answer: A

Acute fear is an emotional barrier to learning. Therapeutic communication and a brief pause help the client regain enough control to learn. Choice B ignores the emotional barrier, choice C is dismissive, and choice D provides information without addressing the client's inability to process it.

1714. A nurse is preparing a teaching plan for a client who recently had a stroke and now has expressive aphasia. The client nods yes to most questions. Which strategy is best to reduce the learning barrier?

- A. Ask only yes-or-no questions because they are easiest to answer
- B. Use short statements, visual cues, and have the client demonstrate or point to show understanding
- C. Direct all teaching to the spouse because the client cannot speak clearly
- D. Provide audio recordings of the instructions for the client to replay

Correct Answer: B

Expressive aphasia can limit spoken responses, so the nurse should use simple communication, visual supports, and nonverbal ways for the client to demonstrate understanding. Choice A can hide misunderstanding, choice C excludes the client, and choice D may not address the communication problem well enough by itself.

1715. A nurse is reinforcing teaching about a low-cholesterol diet to a client who says, "I don't see why I need to change anything. I feel fine." Which nursing action is best?

- A. Warn the client that refusal to change may lead to serious illness
- B. Explore the client's perception of the illness and link the teaching to personally meaningful goals
- C. Stop teaching until the provider can explain the need for diet changes
- D. Give a list of forbidden foods and tell the client to follow it strictly

Correct Answer: B

When a client does not perceive a need to learn, the nurse should assess the client's beliefs and connect the information to meaningful outcomes. Choice A may increase resistance, choice C delays nursing teaching unnecessarily, and choice D is directive without addressing the barrier.

1716. A nurse is teaching a client how to self-administer a newly prescribed eye drop. The client has moderate tremors and poor vision in one eye. Which outcome best shows that the teaching plan successfully addressed the client's learning barriers?

- A. The client states the medication name and dose from memory
- B. The client explains that a family member will read the label aloud each day
- C. The client correctly instills the eye drop using an adapted technique during return demonstration
- D. The client says, "I understand," after the nurse explains the procedure again

Correct Answer: C

Because the barriers involve vision and motor control, the best evidence of successful teaching is correct performance using a workable adapted technique. Choice A tests recall only, choice B may help but does not show the client can perform the skill, and choice D is a weak indicator of understanding.

1717. A nurse is teaching an adolescent newly diagnosed with celiac disease about avoiding gluten. The client shrugs and says, "My friends eat whatever they want. This isn't fair." Which factor is most likely interfering with learning?

- A. Developmental concerns about fitting in with peers
- B. Hearing loss related to age
- C. Inability to understand spoken instructions
- D. Lack of family support at home

Correct Answer: A

Adolescents often place high importance on peer acceptance. Concern about being different can reduce readiness to learn and follow teaching. The scenario does not indicate hearing loss, a receptive language problem, or lack of family support.

1718. A nurse is reinforcing home oxygen safety teaching to a client with chronic lung disease. The client appears attentive, but the television is on loudly, housekeeping staff are speaking in the hall, and the client's roommate is asking questions at the same time. Which action should the nurse take first?

- A. Move forward with the teaching because the client seems interested
- B. Ask the client to repeat the key points immediately
- C. Reduce environmental distractions before continuing the teaching session
- D. Leave written instructions at the bedside for later review

Correct Answer: C

Environmental distractions can significantly impair attention and retention. The first step is to create a better learning environment. Choice A ignores the barrier, choice B tests learning before optimizing conditions, and choice D does not directly address the distraction problem.

1719. A nurse is teaching a client with newly prescribed warfarin about food and medication precautions. The client says, "I take eight pills a day already, and I get confused about which one is which." Which nursing action best addresses this barrier?

- A. Teach all the information in one long session so the client hears it only once
- B. Use an organized medication schedule with clear labeling and review the highest-risk points first
- C. Tell the client to stop taking over-the-counter medications completely
- D. Ask the provider to change warfarin to a medication that needs no teaching

Correct Answer: B

The barrier is complexity and confusion with a multi-medication routine. Organizing the regimen, simplifying presentation, and prioritizing safety-related points best supports learning. Choice A increases overload, choice C is overly broad and inaccurate, and choice D avoids teaching rather than solving the problem.

1720. A nurse is reinforcing discharge teaching to a client about caring for a surgical incision at home. The client states, "I can understand you better if I watch what your hands are doing," and points to both ears when asked about hearing. Which nursing action is best?

- A.** Stand at the client's side and speak loudly while changing the dressing quickly
- B.** Face the client directly, speak clearly at a normal pace, and demonstrate the dressing steps
- C.** Give the client written instructions only so the client can review them later in private
- D.** Ask a family member to receive the teaching instead of the client

Correct Answer: B

Facing the client directly, speaking clearly, and demonstrating the skill best addresses a hearing-related learning barrier by allowing the client to use visual cues and observe the procedure. Speaking loudly from the side can distort speech and removes the visual cue the client requested. Written instructions alone may not be enough for a skill-based task, and teaching only the family member bypasses the client's own participation.

Psychosocial Integrity

Emotional support

1721. A client was told this morning that a biopsy result is malignant. The client sits quietly, looks at the floor, and says, "I don't even know what to say." Which response by the practical nurse is most therapeutic?

- A. "You need to stay positive and not think the worst."
- B. "This must feel overwhelming right now. Would you like to talk about what is on your mind?"
- C. "At least they found it early, so you should be grateful."
- D. "Let me tell you about another client who went through the same thing."

Correct Answer: B

Choice B acknowledges the client's emotion and invites expression without forcing it. This is therapeutic emotional support. Choice A minimizes the feeling, choice C offers false reassurance and judgment, and choice D shifts attention away from the client.

1722. A hospitalized client is scheduled for surgery later today and says, "I am scared I won't wake up." Which action by the practical nurse best provides emotional support first?

- A. Tell the client that modern anesthesia is very safe
- B. Ask the provider to prescribe an antianxiety medication immediately
- C. Encourage the client to share specific fears about the surgery
- D. Change the subject to help the client avoid dwelling on it

Correct Answer: C

Choice C is best because it encourages the client to express feelings and identify concerns, which is the first step in supportive care. Choice A may be partly reassuring but can shut down discussion. Choice B may be appropriate later if needed, but emotional assessment and therapeutic communication come first. Choice D avoids the client's concern.

1723. The practical nurse is caring for a client who has just learned about a new diagnosis of diabetes. Which statement by the nurse best demonstrates active listening and emotional support?

- A. "You will get used to this soon."
- B. "Tell me what worries you most about managing this diagnosis."
- C. "The diet is simple if you just follow the rules."
- D. "You should not be upset because diabetes is common."

Correct Answer: B

Choice B invites the client to discuss fears and concerns and shows willingness to listen. That supports coping and emotional adjustment. Choices A, C, and D minimize the client's feelings or focus on instruction before addressing emotion.

1724. A client begins to cry after a phone call with family. Which response by the practical nurse is most appropriate?

- A. Sit quietly with the client and say, "I am here with you."
- B. Tell the client not to cry because it will make things worse
- C. Leave the room to give the client total privacy
- D. Ask the unlicensed assistive personnel to stay with the client instead

Correct Answer: A

Choice A offers presence, which is a strong form of emotional support. Silence and a simple statement of availability can be highly therapeutic. Choice B discourages expression of emotion. Choice C may feel like abandonment if done immediately. Choice D shifts supportive nursing care away from the nurse without first assessing the client's needs.

1725. A client with chronic kidney disease says, "I am tired of being a burden to my family." Which response by the practical nurse is best?

- A. "Your family probably does not see you that way."
- B. "Why would you think something like that?"
- C. "It sounds like you are worried about how your illness affects them."
- D. "You should focus on getting better instead of thinking negatively."

Correct Answer: C

Choice C reflects the client's feeling back in a nonjudgmental way and encourages further discussion. Choice A may dismiss the client's perception. Choice B can sound challenging or defensive. Choice D minimizes emotion and redirects too quickly.

1726. The family of a client in rehabilitation asks the practical nurse, "What can we do to support her emotionally without taking over everything?" Which suggestion is best?

- A. Answer all questions for her so she does not become stressed
- B. Encourage her to make choices she can manage and praise small progress
- C. Avoid discussing her feelings so she stays calm
- D. Remind her that others have more serious health problems

Correct Answer: B

Choice B promotes autonomy, self-esteem, and realistic support. Emotional support is not doing everything for the client; it includes encouraging independence within ability. Choice A can increase helplessness. Choice C blocks emotional expression. Choice D minimizes the client's experience.

1727. A client with heart failure asks, "Am I dying?" The prognosis has not been discussed in detail with the nurse. Which response is best?

- A. "No, I am sure you will be fine."
- B. "Why would you ask that?"
- C. "Can you tell me what is making you ask that today?"
- D. "You need to wait and ask the provider during rounds."

Correct Answer: C

Choice C explores the client's concern, supports emotional expression, and avoids giving inaccurate information. Choice A offers false reassurance. Choice B may sound defensive. Choice D closes communication and misses a chance to support the client, even though the provider may need to address prognosis later.

1728. A postpartum client says, "I thought I would feel happy, but instead I feel overwhelmed and guilty." Which response by the practical nurse is most therapeutic?

- A. "Many new parents have strong emotions. Tell me more about what you have been feeling."
- B. "You should focus on the baby and stop feeling guilty."
- C. "At least the baby is healthy, so try not to worry."
- D. "You probably just need more sleep and will be fine tomorrow."

Correct Answer: A

Choice A normalizes emotional intensity without dismissing it and invites further assessment. That supports the client and opens the door to evaluating for postpartum mood concerns. Choices B, C, and D minimize the client's feelings and may discourage honest disclosure.

1729. A school-age child is hospitalized for several days and cries when the parent leaves each evening. Which intervention by the practical nurse best provides emotional support?

- A. Tell the child that crying is not helpful and should stop
- B. Encourage the parent to leave quickly without saying goodbye
- C. Help the child create a bedtime routine that includes a recorded message from the parent
- D. Limit contact with the parent to reduce dependence

Correct Answer: C

Choice C supports attachment, predictability, and coping with separation. Familiar routines and connection to the parent can reduce distress. Choice A suppresses emotion. Choice B can increase anxiety and mistrust. Choice D is not supportive and may worsen the child's fear.

1730. A client with advanced chronic illness says quietly, "I don't think anyone understands how alone I feel." Which response by the practical nurse is best?

- A. "You have a lot of visitors, so you are not really alone."
- B. "I am sorry you are feeling so alone. What has this been like for you?"
- C. "Everyone feels lonely sometimes."
- D. "Let's watch television so you can take your mind off it."

Correct Answer: B

Choice B validates the emotion and invites the client to describe the experience. Emotional support starts with acknowledging the client's perception. Choice A argues with the feeling. Choice C generalizes and minimizes. Choice D distracts too quickly instead of exploring the concern.

1731. An older adult client who recently moved to long-term care says, "I sold my house, and now I feel like I lost my whole life." Which nursing response best demonstrates emotional support?

- A. "You will adjust if you give it enough time."
- B. "That sounds like a major loss. What do you miss most?"
- C. "This facility is safer, so moving here was the right choice."
- D. "You should join more activities so you do not think about it."

Correct Answer: B

Choice B recognizes the move as a meaningful loss and invites the client to share specifics. This supports grieving and adjustment. Choice A is vague reassurance. Choice C focuses on logic instead of emotion. Choice D redirects before acknowledging the grief.

1732. A client with newly diagnosed multiple sclerosis says, "My spouse is trying to help, but I snap at everyone." Which nursing response is most therapeutic?

- A. "You need to control your temper so your family will stay supportive."
- B. "It sounds like you are under a lot of stress. What situations seem to trigger that reaction?"
- C. "Your spouse should understand that illness makes people irritable."

- D. "Try not to talk to anyone until you feel calmer."

Correct Answer: B

Choice B shows empathy and helps the client begin identifying patterns and triggers, which supports both emotional expression and coping. Choice A is judgmental. Choice C excuses behavior without helping the client process it. Choice D promotes avoidance rather than healthier communication.

1733. A client returns from a procedure with a new colostomy and says, "Don't let my partner see me like this." Which response by the practical nurse best provides emotional support?

- A. "Your partner needs to get used to it, so it is better to visit now."
- B. "Many people feel this way at first, but you should not worry about it."
- C. "You seem concerned about how your partner may react. Can you tell me more about that?"
- D. "I will tell your partner not to visit again until discharge."

Correct Answer: C

Choice C identifies the likely feeling about body image and relationship concerns, then invites discussion. That is supportive and helps assess the client's needs. Choice A forces exposure. Choice B partially normalizes but still minimizes. Choice D makes a major decision based on one statement and may reinforce avoidance.

1734. The practical nurse is caring for a client whose partner died 2 hours ago. The client is pacing, wringing both hands, and repeatedly saying, "I should have gotten help sooner." Which response is best at this time?

- A. "You cannot blame yourself because everything happens for a reason."
- B. "Try to sit down and rest. You need to stay strong."
- C. "You are feeling guilty about what happened. Stay with me and tell me what you are thinking."
- D. "You should call family members now so you can move forward."

Correct Answer: C

Choice C reflects the apparent feeling, offers presence, and encourages expression during acute grief. This is the most therapeutic support. Choice A uses a cliché and may invalidate the client. Choice B redirects away from the emotion. Choice D pushes tasks before addressing immediate distress.

1735. A client who is 1 day post-myocardial infarction says to the practical nurse, "Everyone keeps telling me to stay positive, but I feel terrified about what happens next." Which response by the nurse is most therapeutic?

- A. You need to focus on the good things so you can recover faster.
- B. A lot of people feel this way after a heart attack, so try not to worry too much.
- C. It sounds like you're feeling very frightened about the future. Tell me more about what worries you most.
- D. The provider can explain your condition better than I can, so make a list of questions for rounds.

Correct Answer: C

Choice C is best because it acknowledges the client's feelings and invites further expression, which is a key part of emotional support and therapeutic communication. Choice A minimizes the client's fear by pushing positivity. Choice B offers false reassurance and can shut down communication. Choice D may be useful later, but it deflects the client's immediate emotional need instead of addressing it first.

1736. A client newly admitted for treatment of a severe leg ulcer avoids eye contact and says, "I don't want my daughter to visit and see me like this." Which response by the practical nurse is most therapeutic?

- A. "You should let her visit because family support is important."
- B. "It sounds like you are uncomfortable with how your illness has changed things."
- C. "Your daughter probably will not notice the ulcer as much as you think."
- D. "Try not to worry about that right now and focus on healing."

Correct Answer: B

Choice B is correct because it reflects the client's feelings without judging or pushing a decision. This encourages the client to discuss concerns about appearance, vulnerability, or embarrassment. Choice A gives advice too quickly. Choice C minimizes the concern. Choice D dismisses the client's feelings by redirecting away from them.

1737. A client awaiting discharge after a stroke says, "My family keeps talking for me. I feel invisible." Which nursing response best shows emotional support?

- A. "They are probably just trying to be helpful."
- B. "What have you already told them about how that makes you feel?"
- C. "You seem hurt by feeling left out of conversations about your care."
- D. "Would you like me to ask social services to meet with your family?"

Correct Answer: C

Choice C is correct because it identifies and validates the client's feeling, which is the best immediate emotional support. Choice B may be helpful later but asks the client to solve the issue before the feeling has been acknowledged. Choice D may become appropriate if further help is needed, but it is not the best first response. Choice A excuses the family's behavior and minimizes the client's distress.

1738. The practical nurse enters the room and finds a client staring at the wall. The client says, "Since the amputation, I don't even know who I am anymore." Which response is best?

- A. "You are still the same person you were before surgery."
- B. "Many clients say it gets easier after rehabilitation starts."
- C. "This has affected the way you see yourself."
- D. "Have you thought about getting a prosthesis?"

Correct Answer: C

Choice C is correct because it reflects the client's struggle with identity and invites further discussion. Choice A may seem supportive but can feel dismissive because it overlooks the client's lived experience. Choice B offers premature reassurance. Choice D moves into future planning before addressing the immediate emotional loss.

1739. A client's spouse says, "He keeps telling me not to fuss over him, so I don't know whether to stay or leave." The client is alert and turns away without speaking. Which action by the practical nurse is best?

- A. Ask the spouse to leave so the client can rest.
- B. Answer for the client because the spouse is asking for guidance.
- C. Encourage the spouse to keep talking to the client despite the client's silence.
- D. Ask the client, "Would you like some quiet time, or would you like your spouse to stay with you?"

Correct Answer: D

Choice D is correct because it supports the client's autonomy while gently clarifying the client's preference. Emotional support includes helping family provide support in the way the client wants. Choice A assumes the client wants to be alone. Choice B removes the client from the decision. Choice C may pressure the client rather than respect the client's current needs.

1740. A young adult client with a new spinal cord injury says, "Everyone keeps telling me how strong I am. I don't feel strong at all." Which response by the practical nurse is most therapeutic?

- A. "You have already handled more than most people could."
- B. "You don't have to feel strong all the time."
- C. "Staying positive will help your recovery."
- D. "What matters is that your family believes in you."

Correct Answer: B

Choice B is correct because it gives the client permission to express vulnerability and reduces pressure to perform emotionally. Choice A may unintentionally continue the same expectation the client is rejecting. Choice C is a common but nontherapeutic statement because it can shut down honest emotion. Choice D shifts attention away from the client's feelings.

1741. A client with terminal illness tells the practical nurse, "My brother hasn't called once. I guess I don't matter much." Which response is best?

- A. "I'm sure your brother cares, even if he isn't showing it well."
- B. "Families often have trouble coping with serious illness."
- C. "It sounds painful to feel unsupported by someone important to you."
- D. "Would you like me to remind your brother to call?"

Correct Answer: C

Choice C is correct because it focuses directly on the client's feeling of hurt and abandonment. That is the most therapeutic immediate support. Choice A reassures without evidence and may invalidate the client's experience. Choice B generalizes instead of addressing the individual feeling. Choice D jumps to fixing the problem before exploring the emotional impact.

1742. The practical nurse is caring for a client who has just been told that chemotherapy must continue longer than expected. The client laughs briefly and says, "Great, just what I wanted to hear." Which response is most appropriate?

- A. "At least the treatment plan is clear now."
- B. "You sound upset about this change."
- C. "Try not to think negatively before speaking with the provider again."
- D. "Would you like information about the next chemotherapy cycle?"

Correct Answer: B

Choice B is correct because the client is likely using sarcasm to express distress, and the nurse should respond to the underlying emotion.

Choice A minimizes the disappointment. Choice C judges the client's reaction. Choice D moves into teaching before emotional support has been provided.

1743. A client with severe burns says, "When people come in, I watch their faces. I can tell they're trying not to react." Which response by the practical nurse is best?

- A. "Most people get used to seeing burns after a while."
- B. "You may be reading too much into their expressions."
- C. "It sounds exhausting to feel watched and judged."
- D. "Would you like me to limit visitors until your wounds heal more?"

Correct Answer: C

Choice C is correct because it validates the client's emotional experience of feeling exposed and judged. Choice A offers premature reassurance. Choice B challenges the client's perception and can feel dismissive. Choice D may be considered later, but first the nurse should acknowledge the feeling behind the statement.

1744. An adolescent client newly diagnosed with epilepsy says, "My friends will think I'm weird if they find out." Which response by the practical nurse is most therapeutic?

- A. "Your real friends won't care."
- B. "A lot of teenagers worry about fitting in after a diagnosis like this."
- C. "You should wait and see whether anyone actually reacts badly."
- D. "Let's focus on making sure you take your medicine correctly."

Correct Answer: B

Choice B is correct because it normalizes the client's concern without dismissing it and opens the door for further discussion. Choice A may sound supportive but minimizes the adolescent's fear. Choice C delays emotional support. Choice D shifts immediately to teaching and ignores the psychosocial concern.

1745. A client whose vision has recently worsened says, "Everyone keeps moving things around for me. I know they're helping, but it makes me feel helpless." Which response is best?

- A. "You should tell them to stop changing things."
- B. "Your family probably doesn't realize how that affects you."
- C. "Needing help right now does not mean you are helpless."
- D. "It sounds as if the way help is being given is upsetting for you."

Correct Answer: D

Choice D is correct because it reflects both the emotional impact and the source of the distress without arguing with the client. Choice C may be intended to encourage, but it contradicts the client's wording and can feel dismissive. Choice A gives advice too quickly. Choice B interprets the family's intentions rather than focusing on the client's feelings.

1746. A client says, "I don't want to talk," then begins tearing at the edge of the blanket and breathing rapidly. Which nursing action best provides emotional support?

- A. Remain with the client quietly for a moment and say, "I'm here with you."
- B. Leave the room and return later because the client requested no conversation.
- C. Ask a series of direct questions to identify the cause of the anxiety quickly.
- D. Explain that anxiety can worsen physical symptoms and should be controlled.

Correct Answer: A

Choice A is correct because presence itself can be therapeutic. The client may not be ready to talk, but the nurse can still provide support by staying calm, available, and nonintrusive. Choice B interprets "I don't want to talk" as rejection of all support. Choice C may overwhelm the client. Choice D offers teaching instead of immediate emotional support.

1747. A client says, "If I let myself cry, I might never stop." Which response by the practical nurse is most therapeutic?

- A. "Try to stay calm so you do not lose control."

- B. "You are worried that expressing your feelings may feel overwhelming."
- C. "Crying usually helps people feel better afterward."
- D. "Let's think about something more positive for a while."

Correct Answer: B

Choice B is correct because it names the client's fear and shows understanding without pressuring the client to cry or not cry. Choice A reinforces fear of emotional expression. Choice C may be true for some people but pushes the client toward a response. Choice D diverts away from the client's feeling.

1748. A client awaiting test results says, "I keep thinking about the worst-case scenario." Which nursing response best offers emotional support while encouraging expression?

- A. "Tell me more about what worries you the most right now."
- B. "You should avoid dwelling on things you cannot control."
- C. "The provider will explain everything once the results are back."
- D. "Most test results are not as bad as people imagine."

Correct Answer: A

Choice A is correct because it invites the client to describe specific fears, which helps the nurse assess and support the client. Choice B shuts down discussion. Choice C delays emotional support until later. Choice D gives false reassurance and may minimize the client's anxiety.

1749. A client with a recent cancer diagnosis says to the practical nurse, "I spend all day trying not to fall apart in front of my children." Which response is best?

- A. "Children are often more resilient than parents expect."
- B. "You are carrying a lot while trying to protect them."
- C. "It is important not to upset children with adult problems."
- D. "You should ask a relative to talk with them instead."

Correct Answer: B

Choice B is correct because it acknowledges the client's emotional burden and protective role without judgment. Choice A may be true but bypasses the client's distress. Choice C is judgmental and discourages emotional honesty. Choice D jumps to a solution before recognizing the client's feelings.

1750. A client who was told this morning about the need for long-term dialysis says to the practical nurse, "I nodded when the provider was here, but honestly, I feel completely overwhelmed." Which response by the nurse best provides emotional support?

- A. You need to stay focused on the treatment plan so you do not fall behind.
- B. At least dialysis is available, so try not to think about it too much.
- C. It sounds like this feels like a lot to take in right now. Would you like to talk about what feels most overwhelming?
- D. Most clients adjust after a few treatments, so this feeling will probably pass soon.

Correct Answer: C

Choice C is correct because it acknowledges the client's feelings and invites the client to express specific concerns, which is a therapeutic way to provide emotional support. Choice A shifts quickly to task focus instead of supporting the emotion. Choice B minimizes the client's distress. Choice D offers false reassurance and assumes how the client will feel later rather than addressing the present emotion.

1751. An older adult client who recently moved to a long-term care facility says, "At home I decided when to eat, when to rest, everything. Here I feel like none of that is mine anymore." Which nursing action best provides emotional support?

- A. Explain that the client will adjust after a few days
- B. Ask the client which daily routines are most important to keep and explore available choices
- C. Tell the family to visit more often so the client feels less alone
- D. Encourage the client to join group activities to stay distracted

Correct Answer: B

Choice B is best because emotional support includes recognizing the loss of control and helping the client identify choices that can be preserved. This response validates the concern and supports autonomy. Choice A minimizes the feeling. Choice C may help socially, but it does not directly address the client's distress about lost control. Choice D focuses on distraction rather than understanding and supporting the client's expressed concern.

1752. A client hospitalized for an exacerbation of chronic illness becomes quiet after hearing that discharge will be delayed. A few minutes later, the client says, "My job already thinks I'm unreliable." Which response by the practical nurse is most appropriate?

- A. "You should focus on getting better instead of worrying about work."
- B. "Would you like me to contact the case manager to discuss resources after we talk about what this delay means to you?"
- C. "Most employers understand when someone is in the hospital."
- D. "At least the provider is being cautious and not sending you home too soon."

Correct Answer: B

Choice B is best because it first recognizes that the delay has personal meaning for the client and then offers a practical support option. Emotional support is not only listening; it also includes connecting the client with appropriate help after exploring the concern. Choice A dismisses the client's real stressor. Choice C gives reassurance without assessing the client's situation. Choice D redirects to the medical plan and overlooks the client's fear about employment.

1753. A client who recently experienced pregnancy loss is sitting silently and holding a baby blanket. When the practical nurse sits nearby, the client says softly, "I don't know what to do with any of this." Which response by the nurse is best?

- A. "You can put the items away now and look at them again when you feel stronger."
- B. "Everything happens for a reason, even when it is painful."

C. The nurse remains with the client and says, "You do not have to decide anything right this minute. I can stay with you while you think about what feels right."

D. "Try not to dwell on the blanket, because that will only make you more upset."

Correct Answer: C

Choice C is best because it combines presence, acceptance of silence, and freedom from pressure during acute grief. In emotionally intense situations, supportive presence is often more therapeutic than advice. Choice A pushes the client toward action before readiness is assessed. Choice B is a nontherapeutic cliché that may feel dismissive. Choice D tells the client to suppress grief rather than process it. The most likely trap is choosing A because it sounds practical, but immediate problem-solving is less supportive than staying present and allowing the client to grieve.

1754. A client who had a below-the-knee amputation 2 days ago stares at the wall and says, "My family keeps saying I should be grateful I'm alive, but I don't feel grateful. I just feel empty." Which response by the practical nurse is most therapeutic?

- A. "You need to focus on the positive so you do not become depressed."
- B. "Many people feel this way at first, and things usually get easier soon."
- C. "It sounds like you are having a hard time with what has happened."
- D. "Your family means well, so try not to take their comments personally."

Correct Answer: C

Choice C is correct because it acknowledges the client's feeling and invites further expression without judging, minimizing, or redirecting the emotion. This is a core therapeutic communication approach for providing emotional support. Choice A gives advice and dismisses the feeling. Choice B may seem reassuring, but it generalizes and can minimize the client's present distress. Choice D shifts the focus to the family's intentions instead of the client's emotional experience.

Coping

1755. A client was admitted 2 days ago after a new diagnosis of multiple sclerosis. The client says, "I don't even want to talk about it. Maybe the tests were wrong." Which response by the nurse is best?

- A.** You need to start learning about the disease as soon as possible.
- B.** It sounds like this diagnosis feels hard to accept right now.
- C.** Most people adjust after a few days, so try to stay positive.
- D.** Why do you think you are refusing to face the problem?

Correct Answer: B

Choice B is correct because it uses therapeutic communication to acknowledge the client's feelings without arguing, judging, or pushing information before the client is ready. Choice A gives premature teaching, choice C minimizes the client's distress, and choice D is confrontational and may increase defensiveness.

1756. A client awaiting surgery is pacing, wringing both hands, and repeatedly saying, "Something bad is going to happen." Which nursing action should be taken first?

- A.** Ask the client to describe what is causing the fear
- B.** Teach the client about postoperative exercises
- C.** Tell the client there is nothing to worry about
- D.** Leave the client alone to rest in a quiet room

Correct Answer: A

Choice A is correct because the first step is to assess and encourage expression of feelings so the nurse can identify the source of anxiety and support coping. Choice B may be appropriate later but is not first when the client is highly anxious. Choice C gives false reassurance, and choice D may increase the client's sense of isolation.

1757. A nurse is caring for a client whose spouse died 3 months ago. The client states, "I still cry every day when I get home." Which response by the nurse is most appropriate?

- A.** You should avoid thinking about your spouse so you can move on.

B. Daily crying means your grief is becoming abnormal.

C. Tell me what the evenings are like for you since your spouse died.

D. It would be better to keep busy instead of talking about the loss.

Correct Answer: C

Choice C is correct because it invites the client to discuss the loss and current coping in an open, supportive way. Crying can occur in normal grief, so choice B is not supported by the stem. Choices A and D discourage expression of feelings and can interfere with healthy grieving and coping.

1758. A hospitalized client says, "When I feel stressed, I usually call my sister and we talk things through." The nurse identifies this statement as which type of coping strategy?

- A.** Use of a social support system
- B.** Denial of the problem
- C.** Displacement of feelings
- D.** Avoidance behavior

Correct Answer: A

Choice A is correct because the client is describing reaching out to a trusted person for support, which is an adaptive coping strategy. The statement does not show denial, displaced anger, or avoidance. Instead, it reflects healthy use of available interpersonal resources.

1759. A client with chronic kidney disease says, "I don't want my family helping me. I should be able to handle this alone." Which response by the nurse best promotes effective coping?

- A.** Depending on others usually makes illness harder to manage.
- B.** What concerns you most about accepting help from your family?
- C.** Your family should decide how much care you need at home.
- D.** If you refuse help now, you may not recover as well.

Correct Answer: B

Choice B is correct because it explores the client's beliefs and concerns, helping the nurse understand barriers to support and promote coping. Choice A reinforces isolation, choice C removes the client's autonomy, and choice D is judgmental and likely to increase resistance.

1760. A client who recently had a colostomy says, "I don't want to look at it, and I don't want to learn how to care for it." Which nursing interpretation is most appropriate?

- A.** The client is demonstrating a possible ineffective coping response to the body-image change.
- B.** The client has accepted the surgery but prefers passive learning.
- C.** The client is showing evidence of improved self-esteem.
- D.** The client is ready for discharge teaching because expression of fear has ended.

Correct Answer: A

Choice A is correct because refusal to look at or learn about the colostomy suggests difficulty coping with the body-image change. Choice B incorrectly labels avoidance as acceptance. Choices C and D are not supported; the client's statement shows distress, not readiness or improved self-esteem.

1761. A client hospitalized for exacerbation of chronic obstructive pulmonary disease becomes frustrated during bathing and says, "I hate needing help with everything." Which nursing response is best?

- A.** You should be grateful your condition is not worse.
- B.** Many clients feel this way, so try not to focus on it.
- C.** Needing help right now does not mean you have lost all independence.
- D.** Let's skip bathing today so you do not get upset.

Correct Answer: C

Choice C is correct because it acknowledges the client's frustration while reframing the situation in a supportive, realistic way that can preserve self-esteem and coping. Choice A minimizes feelings, choice B is dismissive, and choice D avoids the issue rather than helping the client cope with dependence during illness.

1762. A client says, "I get so overwhelmed by chemotherapy appointments that I forget what the provider tells me." Which nursing action would best help this client cope with treatment-related stress?

- A.** Advise the client to delay all decisions until treatment is completed.
- B.** Suggest bringing a support person and writing down questions before visits.
- C.** Recommend avoiding discussion of side effects unless they become severe.
- D.** Encourage the client to rely only on memory to build confidence.

Correct Answer: B

Choice B is correct because using a support person and preparing written questions are practical, adaptive coping strategies that reduce stress and improve information recall. Choice A unnecessarily removes decision-making, choice C discourages communication, and choice D ignores the client's stated difficulty.

1763. A nurse is planning care for a client experiencing significant stress after job loss. Which client goal best reflects effective coping?

- A.** The client will avoid discussing feelings about unemployment.
- B.** The client will identify two personal strengths and one support resource.
- C.** The client will stop feeling anxious by the end of the shift.
- D.** The client will depend on the nurse for all problem solving.

Correct Answer: B

Choice B is correct because effective coping includes identifying internal strengths and external supports. Choice A promotes avoidance, not coping. Choice C is unrealistic and too absolute for a short timeframe. Choice D increases dependence rather than supporting adaptive coping skills.

1764. A client newly diagnosed with hypertension says, "My father ignored his blood pressure and lived to be 90, so I don't need to change anything." Which defense mechanism is the client most likely using?

- A. Projection
- B. Regression
- C. Denial
- D. Sublimation

Correct Answer: C

Choice C is correct because the client is minimizing the significance of the diagnosis and rejecting the need for change despite the health problem. Projection involves attributing one's feelings to others, regression is reverting to earlier behavior, and sublimation channels impulses into acceptable activities.

1765. A client who lost a lower leg in a motor vehicle crash says, "I don't want my children to visit me like this." Which nursing action is most appropriate?

- A. Arrange the visit anyway so the client can get used to it.
- B. Encourage the client to discuss concerns about the children's visit.
- C. Tell the client the children are too young to notice the change.
- D. Advise the client to wait until discharge before seeing the children.

Correct Answer: B

Choice B is correct because the nurse should first explore the client's concerns and feelings about altered body image and family interaction. Choice A ignores autonomy, choice C minimizes the client's distress, and choice D delays support without addressing the underlying coping issue.

1766. A nurse is caring for a client 1 day after a myocardial infarction. The client jokes constantly, changes the subject whenever recovery is mentioned, and says, "No point worrying about anything." What is the nurse's best initial action?

- A. Document that the client is coping effectively because humor is being used.
- B. Confront the client directly about refusing to acknowledge the seriousness of the illness.
- C. Assess whether the joking is masking anxiety and interfering with needed learning.
- D. Immediately ask the provider to prescribe an antianxiety medication.

Correct Answer: C

Choice C is correct because humor can be adaptive or maladaptive depending on whether it reduces stress while still allowing reality-based coping. The nurse should assess further before concluding the coping is effective. Choice A assumes effectiveness without assessment, choice B is likely to increase defensiveness, and choice D is premature.

1767. A nurse enters a room and finds a client recently diagnosed with cancer sitting silently and staring at the wall. Earlier, the client declined visitors and refused lunch. Which action by the nurse best supports coping at this time?

- A. Sit quietly with the client for a few minutes and offer presence
- B. Explain that poor intake will delay recovery and should stop
- C. Ask a family member to come in despite the client's refusal
- D. Begin detailed teaching about the treatment plan immediately

Correct Answer: A

Choice A is correct because therapeutic presence can support a client who may be overwhelmed and not ready for detailed discussion. It communicates support without pressure. Choice B is admonishing, choice C violates the client's stated preference, and choice D may overload the client before readiness is established.

1768. A client with newly diagnosed diabetes says, "I know I need to learn this, but every time you start teaching I feel panicky and can't concentrate." Which nursing intervention is best?

- A. Provide one long teaching session so the material is finished quickly.
- B. Postpone all teaching until the client no longer reports any anxiety.
- C. Break teaching into short sessions and begin with one simple self-care task.
- D. Have the family member learn everything instead of the client.

Correct Answer: C

Choice C is correct because anxiety can impair concentration, so teaching should be paced, focused, and manageable. Starting with one simple task promotes mastery and coping. Choice A increases overload, choice B delays necessary learning unnecessarily, and choice D undermines the client's participation in self-care.

1769. A client who recently learned of a new diagnosis says to the nurse, "I haven't told anyone yet. I just keep cleaning the house so I don't have to think about it." Which nursing response is most appropriate?

- A. "Keeping busy is good. Try not to think about it until you feel stronger."
- B. "You seem to be using activity to avoid feelings right now. Would you like to talk about what worries you most?"
- C. "You need to tell your family today so they can start helping you."
- D. "If you make a schedule for housework, you may feel more in control of the situation."

Correct Answer: B

Choice B is best because it identifies the client's current coping pattern and invites the client to express feelings in a supportive, nonjudgmental way. This promotes adaptive coping and therapeutic communication. Choice A reinforces avoidance rather than helping the client process the diagnosis. Choice C is too directive and removes the client's control over disclosure. Choice D focuses on task organization, but the main issue is emotional avoidance, not time management.

1770. A client who was told this morning that a breast biopsy is malignant says, "I'm fine. I just want to make a list of everything I need to do before discharge." The client speaks rapidly and avoids discussion of the diagnosis. Which nursing response is best?

- A. "You need to slow down and talk about how you really feel."
- B. "Making a list can feel helpful. What concerns you most right now?"
- C. "Try not to think about the diagnosis until the provider returns."
- D. "At least the cancer was found before you went home."

Correct Answer: B

Choice B acknowledges the client's current coping behavior without judging it and gently invites expression of concerns. This supports coping through therapeutic communication. Choice A is too directive. Choice C encourages avoidance rather than healthy coping. Choice D offers false reassurance and minimizes the client's distress.

1771. A client with chronic pain says, "Whenever my pain gets worse, I do the breathing exercises I learned in therapy and then listen to music." Which statement by the nurse is most accurate?

- A. The client is using problem-focused and emotion-focused coping strategies.
- B. The client is showing denial about the severity of the pain.
- C. The client is relying only on ineffective passive coping.
- D. The client is displaying a maladaptive defense mechanism.

Correct Answer: A

Choice A is correct because the client is using active coping strategies to manage stress and symptoms. Breathing exercises and music can reduce distress and improve control. Choices B, C, and D mislabel these behaviors; they are not denial, passive helplessness, or maladaptive defense mechanisms.

1772. A nurse is caring for a client after a house fire. The client says, "My family is safe, but every time I close my eyes I see the flames." Which response by the nurse is most appropriate?

- A. "You should be grateful no one was hurt."
- B. "Those thoughts can happen after a frightening event. Tell me what has been hardest for you."
- C. "Try to stay busy so you do not dwell on it."
- D. "If you stop talking about the fire, the memories may fade faster."

Correct Answer: B

Choice B validates the client's experience and invites further expression, which supports coping after trauma. Choice A minimizes the distress. Choices C and D push avoidance, which does not help the nurse assess or support coping effectively.

1773. A client with newly diagnosed heart failure says, "I know I need to cut down on salt, but whenever I get stressed I order fast food and tell myself one more meal won't matter." Which coping pattern should the nurse identify?

- A. Adaptive problem solving
- B. Maladaptive coping through avoidance
- C. Effective use of social support
- D. Healthy emotional expression

Correct Answer: B

Choice B is correct because the client is responding to stress with avoidance and behavior that interferes with treatment. This is maladaptive coping. Choices A, C, and D do not match the behavior described.

1774. The nurse is developing a plan of care for a client whose partner recently left the home. Which outcome best indicates improved coping?

- A. The client states, "I will never think about this again."
- B. The client remains in bed and avoids phone calls for the entire shift.
- C. The client identifies two support people to contact when feeling overwhelmed.
- D. The client asks the nurse to make all decisions about daily activities.

Correct Answer: C

Choice C reflects improved coping because the client is identifying concrete support resources and a plan for managing distress. Choice A suggests suppression or unrealistic thinking. Choice B reflects withdrawal. Choice D shows increased dependence rather than strengthened coping.

1775. A client with a recent spinal cord injury becomes angry during rehabilitation and says, "These exercises are pointless. I quit." Which nursing response best promotes effective coping?

- A. "If you refuse therapy, you will only make your recovery worse."
- B. "You seem frustrated. Tell me what feels most difficult about therapy today."
- C. "You need to push through this because everyone feels this way."
- D. "Let's skip therapy today and not discuss it further."

Correct Answer: B

Choice B is therapeutic because it reflects the emotion and invites the client to identify barriers, which can lead to more effective coping and problem solving. Choice A is threatening, C minimizes feelings, and D avoids the issue instead of helping the client cope.

1776. A nurse is assessing a client's coping resources after a new diagnosis of rheumatoid arthritis. Which question is most appropriate?

- A. "Why are you taking this diagnosis so badly?"
- B. "What have you done in the past when you faced a major life stress?"
- C. "Do you understand that stress can worsen your condition?"
- D. "Would you like me to explain why some people cope better than others?"

Correct Answer: B

Choice B helps the nurse assess previous coping patterns and existing strengths, which is directly useful for planning care. Choice A is judgmental. Choice C focuses on teaching rather than assessment. Choice D is abstract and less helpful than exploring the client's own past coping methods.

1777. A client with severe psoriasis says, "I stopped going to church and I won't meet friends for dinner anymore. I don't want anyone looking at me." Which nursing concern is most important?

- A. The client is demonstrating improved independence.
- B. The client is using social withdrawal that may worsen coping.
- C. The client is showing readiness to learn skin care.
- D. The client is using humor as a defense mechanism.

Correct Answer: B

Choice B is correct because withdrawing from support systems can increase isolation and make coping less effective. Choice A is inaccurate because isolation is not improved independence here. Choice C is unsupported by the data. Choice D does not match the client's behavior.

1778. A postoperative client is tearful and says, "I am afraid I won't be able to care for myself when I get home." The nurse has 10 minutes before giving medications to other clients. Which action is best?

- A. Sit with the client briefly, encourage expression of the fear, and return later to continue the discussion
- B. Tell the client discharge planning will address those issues tomorrow
- C. Change the subject to pain control so the client does not become more upset
- D. Explain that worrying will delay physical healing

Correct Answer: A

Choice A is best because even a brief therapeutic presence can support coping, and the nurse can set a realistic plan to return. Choice B delays support when the client is expressing immediate fear. Choice C blocks communication. Choice D is nontherapeutic and judgmental.

1779. A client says, "I do not want my adult children to know I started treatment for alcohol dependence. They will think less of me." Which nurse response best promotes coping?

- A. "Keeping this secret will probably make recovery easier."
- B. "Your children have a right to know about all of your treatment."
- C. "It sounds like you are worried about being judged. Who has been supportive for you in the past?"
- D. "You should tell them today so you can get it over with."

Correct Answer: C

Choice C is therapeutic because it reflects the client's fear and explores possible support systems without forcing disclosure. Choice A reinforces isolation. Choice B ignores client autonomy. Choice D is directive and may increase anxiety rather than support coping.

1780. A client scheduled for a new ileostomy asks, "Can I talk to someone who has actually lived with this?" What is the nurse's best action?

- A. Arrange contact with an appropriate support resource if available
- B. Tell the client that written instructions are usually enough
- C. Advise the client to wait until after surgery to ask questions
- D. Explain the technical steps of the surgery in greater detail

Correct Answer: A

Choice A is best because connecting the client with an appropriate support resource can strengthen coping through shared experience and realistic encouragement. Choice B dismisses the client's stated need. Choice C delays support unnecessarily. Choice D may provide information but does not address the coping need the client expressed.

1781. A client receiving radiation therapy says, "I try to stay positive, but when I get home I drink several glasses of wine so I can stop thinking about everything." Which nursing conclusion is best?

- A. The client is using an effective long-term coping strategy.
- B. The client is combining adaptive coping with a potentially harmful maladaptive behavior.
- C. The client is fully adjusting to treatment demands.
- D. The client is ready for independent discharge teaching only.

Correct Answer: B

Choice B is correct because the client may be attempting positive thinking, but using alcohol to avoid distress is a potentially harmful maladaptive coping behavior. Choice A ignores the risk. Choice C overstates adjustment. Choice D is not supported by the information given.

1782. A client whose chemotherapy was changed today says, "Don't explain it again. I nod yes, but I am not taking any of this in." Which nursing action is best to promote coping and learning?

- A. Continue the teaching now so the schedule is not delayed
- B. Ask the provider to repeat the information instead
- C. Acknowledge the client's distress and postpone detailed teaching until the client is better able to focus
- D. Tell the client it is important to stay strong and listen carefully

Correct Answer: C

Choice C is best because high anxiety can impair concentration and learning. Addressing distress first and delaying detailed teaching until the client can focus supports both coping and understanding. Choice A ignores the client's inability to process information. Choice B may be helpful later but does not address the immediate issue. Choice D is minimizing and nontherapeutic.

1783. A client who lost a job 2 weeks ago tells the nurse, "I stay in bed most of the day, and I stopped answering calls from my friends." Which nursing response is most appropriate to promote coping?

- A. "You need to force yourself to get out more so you do not become depressed."
- B. "What activities or people have helped you handle stressful times in the past?"
- C. "It is best to avoid talking about the loss until you feel ready."
- D. "You should focus on finding a new job right away to stay busy."

Correct Answer: B

Asking about past coping strategies and support systems helps the nurse assess available coping resources and build on methods that may have worked before. This is therapeutic and client-centered. Choice A is judgmental, choice C reinforces withdrawal and avoidance, and choice D pushes problem solving before the nurse has assessed the client's emotional response and resources.

1784. A client hospitalized after a recent diagnosis of multiple sclerosis says, "I don't want to think about it. I just want to sleep until all of this goes away." Which nursing action is best?

- A. Tell the client that rest is the healthiest way to cope with major stress
- B. Change the subject to avoid increasing the client's anxiety
- C. Acknowledge the client's feelings and explore one immediate concern the client has today
- D. Explain all aspects of the disease process so the client can begin adjusting

Correct Answer: C

The best response is to acknowledge the feeling and help the client focus on one manageable immediate concern. This supports coping without overwhelming the client. Choice A supports avoidance, which is not the best long-term coping approach. Choice B blocks communication. Choice D may overload the client with information when the client is showing difficulty processing the diagnosis.

1785. A client newly diagnosed with type 2 diabetes tells the nurse, "My cousin had diabetes and went blind. I know the same thing will happen to me." Which response by the nurse best supports adaptive coping?

- A. "You should not think that way because many people do well with treatment."
- B. "Tell me what worries you most when you think about having diabetes."
- C. "Blindness usually takes years, so you do not need to worry right now."
- D. "Let me give you a booklet that explains the long-term complications."

Correct Answer: B

Option B uses therapeutic communication to explore the client's fear and feelings, which is the best first step in supporting coping. It encourages expression rather than shutting down the concern. Option A offers false reassurance, option C minimizes the fear, and option D gives information before the nurse has addressed the emotional response.

1786. The nurse is caring for a client 1 day after a below-the-knee amputation. The client says, "I don't want to see anyone. Just leave me alone." Which action is best for the nurse to take?

- A. Respect the request and avoid returning unless the client calls for help
- B. Tell the client that family support is necessary and arrange a visit
- C. Briefly stay with the client, acknowledge the loss, and offer to return later to talk
- D. Encourage the client to focus on physical therapy goals instead of the amputation

Correct Answer: C

Option C best promotes coping because it respects the client's current distress while still conveying presence, support, and willingness to talk later. After a major loss, clients may need time but should not be abandoned. Option A withdraws support, option B overrides the client's wishes, and option D shifts attention away from the emotional impact too quickly.

1787. A client with newly diagnosed hypertension says, "When I get overwhelmed, I go for a 20-minute walk and then write down what I can handle today." Which interpretation by the nurse is most accurate?

- A. The client is demonstrating effective problem-focused coping strategies
- B. The client is avoiding the stressor by staying physically busy
- C. The client is showing denial about the seriousness of the diagnosis
- D. The client is relying on dependent coping and needs more support

Correct Answer: A

Option A is correct because walking to reduce tension and making a manageable plan are adaptive coping strategies aimed at handling stress constructively. Option B is a likely trap because activity can be avoidance in some situations, but here the client also organizes tasks realistically. Options C and D are not supported by the statement.

1788. A client with chronic kidney disease says, "I pray every morning, talk with my sister, and keep a notebook of questions for my appointments." Which nursing conclusion is best?

- A. The client is using several personal coping resources
- B. The client is becoming overly dependent on family support
- C. The client is focusing on religion to avoid medical treatment
- D. The client is showing disorganized coping and poor insight

Correct Answer: A

Option A is correct because the client identifies multiple coping resources: spiritual support, family support, and active participation in care. Together these suggest adaptive coping. Option B is unsupported because talking with a sister does not by itself show dependence. Option C incorrectly assumes spirituality is avoidance, and option D contradicts the organized use of questions for appointments.

1789. A nurse is evaluating four clients to determine who is coping most effectively with a recent diagnosis of chronic illness. Which client finding best indicates effective coping?

- A. The client says, "I read everything online, but I have not told anyone because I need to handle this alone."
- B. The client says, "I asked my neighbor to drive me to visits, and I keep a list of questions for the provider."
- C. The client says, "I sleep most of the day so I do not think about what might happen."
- D. The client says, "I smile when staff come in, but I cry only after everyone leaves."

Correct Answer: B

Option B best indicates effective coping because the client is seeking support and using organized, problem-solving behavior to manage the illness. Option A suggests isolation, which can limit coping resources. Option C shows avoidance, and option D shows concealed distress without evidence of active coping. The best answer is the option demonstrating both support-seeking and constructive action.

Violence risk

1790. A nurse is caring for a client in the emergency department who is pacing, clenching fists, and speaking loudly after being told there will be a delay in treatment. Which action should the nurse take first?

- A. Tell the client that yelling is unacceptable and must stop
- B. Move closer to the client to show support and concern
- C. Maintain a calm tone and offer to talk in a quiet area
- D. Call security to stand next to the client immediately

Correct Answer: C

Using a calm, nonthreatening approach and offering a quieter space is an early de-escalation step that can reduce stimulation and help the client regain control. Moving too close can feel threatening, and calling security immediately may escalate the situation if the client is not yet actively violent. Confronting the client about yelling is likely to increase defensiveness.

1791. A client on an inpatient unit says to the nurse, "When my brother gets here, I'm going to make him pay for what he did." Which response by the nurse is best?

- A. "You don't really mean that."
- B. "What exactly are you thinking about doing?"
- C. "You need to calm down before we talk."
- D. "Your brother probably did not intend to upset you."

Correct Answer: B

The nurse should assess the threat directly by asking for specifics. This helps determine the seriousness of the statement, including possible plan and intent. Minimizing the comment, demanding calmness first, or defending the other person can shut down communication and delay risk assessment.

1792. A nurse is planning care for a client with escalating anger. Which environmental change is most appropriate to reduce the risk of violence?

- A. Place the client in a busy day room for closer observation
- B. Encourage several peers to talk with the client for support
- C. Reduce noise and unnecessary stimulation around the client
- D. Keep the client near the nurses' station during shift report

Correct Answer: C

Reducing noise and stimulation can lower agitation and support de-escalation. A busy day room, multiple peers, or the activity around shift report may increase stimulation and worsen anger. Observation is important, but the environment should still be as calm and nonprovocative as possible.

1793. A client with a history of violent outbursts begins shouting at staff and slams a chair against the wall. Which action should the nurse take first?

- A. Administer a PRN sedative immediately without further interaction
- B. Ask other clients to leave the area and remove potential weapons
- C. Explain unit rules and consequences in detail
- D. Document the behavior and continue observing from the desk

Correct Answer: B

The immediate priority is safety. Clearing the area and removing objects that could be used as weapons helps protect everyone while further interventions are organized. Medication may be appropriate later, but the first step is to reduce immediate danger. Detailed rule-setting is not useful during acute escalation, and observation alone is insufficient.

1794. Which client statement should concern the nurse most as an indicator of potential violence?

- A. "I need to be alone for a while."
- B. "Nobody listens to me when I'm upset."
- C. "Sometimes I think about hurting the person who got me fired."
- D. "I don't want to talk right now."

Correct Answer: C

A statement about hurting a specific person suggests possible violent ideation and requires further assessment. The other statements may reflect frustration or withdrawal but are less direct indicators of violence risk. The key clue is the expressed thought of harming another person.

1795. A nurse is interviewing a client who has become increasingly agitated. Which nursing action is most appropriate?

- A. Sit between the client and the doorway to prevent elopement
- B. Stand with arms crossed to convey authority
- C. Keep a safe distance and ensure access to an exit
- D. Touch the client's shoulder to provide reassurance

Correct Answer: C

When a client is agitated, the nurse should maintain personal safety by keeping a safe distance and avoiding being trapped in the room. Blocking the exit, crossing arms, or touching the client can be perceived as threatening and may escalate aggression.

1796. A client with paranoid thinking says, "That man in the hallway is spying on me, and I'm going to stop him." What is the nurse's priority action?

- A. Argue that the belief is false and irrational

- B. Assess for a specific plan and means to harm the man

- C. Ask another client to stay with the client for company

- D. Change the subject to distract the client

Correct Answer: B

The priority is to assess the immediacy and severity of the threat by determining plan, intent, and means. Arguing about the delusion can increase mistrust. Peer involvement is unsafe, and distraction alone does not address the potential for violence.

1797. A nurse notices that a usually quiet client is pacing in the hallway, breathing rapidly, and speaking in a tense voice. Which intervention is best before the client loses control?

- A. Offer the client a chance to talk about what is causing the anger
- B. Wait until the client requests help
- C. Tell the client to return to the room until behavior improves
- D. Confront the client about disturbing the unit

Correct Answer: A

Early intervention is most effective. Inviting the client to talk can identify triggers and may prevent further escalation. Waiting passively misses a key opportunity. Confrontation or isolation as an initial response may increase anger and reduce cooperation.

1798. A client tells the nurse, "If my partner comes tonight, I will stab them with the knife I hid in my bag." Which action should the nurse take first?

- A. Ask why the client feels so angry
- B. Remove the bag from the client and obtain help immediately
- C. Tell the client that threats are not allowed on the unit
- D. Document the statement for the provider to review later

Correct Answer: B

The client has expressed a specific target, plan, and weapon. The nurse must act immediately to reduce danger by securing the weapon and getting assistance. Exploring feelings is not the first priority when there is an imminent threat. Delayed documentation or rule reminders do not address immediate safety.

1799. Which nurse statement is most therapeutic when setting limits with an angry client who is yelling at staff?

- A. "You may be angry, but you may not threaten anyone here."
- B. "If you don't stop, you will lose all privileges today."
- C. "Everyone is tired of your behavior."
- D. "Why are you acting like this again?"

Correct Answer: A

This response acknowledges the client's feeling while clearly setting a behavioral limit, which is a therapeutic and respectful approach. The other options are punitive, shaming, or accusatory and are more likely to escalate the interaction.

1800. A client with a history of assault becomes increasingly restless after receiving bad news. Which additional finding most strongly suggests rising risk for violence?

- A. The client asks for a PRN medication
- B. The client begins glaring at staff and making threatening remarks
- C. The client sits quietly and avoids eye contact
- D. The client requests to speak with a family member

Correct Answer: B

Threatening remarks and hostile staring are important warning signs of escalating aggression, especially in a client with a history of assault. Requesting medication or support may reflect help-seeking behavior. Quiet withdrawal can signal distress but is less direct than active threatening behavior.

1801. The nurse is caring for several clients on a behavioral health unit. Which client should the nurse assess first for potential violence?

- A. A client who says, "I'm upset and need some time alone."

- B. A client who is crying after a family visit
- C. A client who is pacing, shouting, and demanding, "Get out of my way now!"
- D. A client who refuses breakfast and remains in bed

Correct Answer: C

Pacing, shouting, and commanding language suggest active escalation and possible immediate danger, making this client the priority for assessment. The other clients may need support, but they do not show the same urgent risk of violence.

1802. A nurse is teaching a newly hired assistive personnel member about clients at risk for violence. Which statement by the assistive personnel indicates correct understanding?

- A. A calm client is no longer a concern if they were angry earlier
- B. Direct threats should be taken seriously and reported right away
- C. It is best to stand very close so the client knows you are paying attention
- D. Arguing with delusional ideas helps clients return to reality

Correct Answer: B

Direct threats must be taken seriously and reported promptly. A client may still be dangerous even if briefly calm after agitation. Standing too close can provoke aggression, and arguing with delusions often increases mistrust and escalation.

1803. A client who was loudly threatening staff suddenly becomes silent, stares at one nurse, and reaches into a jacket pocket while moving toward the nurse. What is the nurse's priority action?

- A. Turn away to avoid provoking the client
- B. Approach quickly and grab the client's arm
- C. Back away, call for immediate assistance, and maintain safety
- D. Ask the client whether the pocket contains a personal item

Correct Answer: C

This behavior suggests possible imminent violence. The nurse should create distance, get help, and focus on safety. Turning away reduces awareness, grabbing the client may provoke an attack, and asking about the pocket delays response to a potentially lethal situation.

1804. During a one-to-one interaction, an agitated client says, "You're the reason I'm stuck here." Which nurse response is best to reduce escalation?

- A. "You're wrong. I did not make that decision."
- B. "I can see that you're angry. Let's talk about what is upsetting you."
- C. "If you keep accusing me, this conversation is over."
- D. "You need to apologize before we continue."

Correct Answer: B

This response acknowledges the emotion without arguing and redirects the client toward discussing the cause of the anger, which supports de-escalation. Defensiveness, threats to end the conversation, and demands for apology are likely to intensify the conflict.

1805. A client on a mental health unit has been increasingly irritable during the past hour. Which observation is the strongest warning sign that the client may become violent soon?

- A. The client asks to speak with the provider about discharge plans
- B. The client sits alone and avoids eye contact during group
- C. The client begins glaring, jaw tightening, and invading another client's personal space
- D. The client refuses to eat lunch and returns to the room

Correct Answer: C

Glare, muscle tension, and moving into another person's space are classic signs of escalating aggression and immediate violence risk. Asking about discharge, isolating, or skipping a meal may reflect distress or withdrawal, but they are less direct indicators of imminent assaultive behavior.

1806. A nurse is speaking with an angry client who suddenly begins shouting, "Back off!" and steps closer to the nurse. Which action by the nurse is most appropriate?

- A. Move closer to show support and place a hand on the client's shoulder
- B. Maintain a calm tone and increase physical distance while keeping the exit accessible
- C. Turn away from the client to decrease stimulation
- D. Argue with the client about the need to follow unit rules

Correct Answer: B

The nurse should use a calm, nonthreatening approach and maintain safe personal space with a clear exit route. Touching the client can increase aggression, turning away reduces awareness of danger, and arguing often escalates the situation.

1807. A client says, "Everyone here is against me." The client is speaking rapidly, breathing hard, and pounding a fist into an open hand. Which response by the nurse is best?

- A. You need to calm down right now or you will lose privileges.
- B. Tell me exactly who is against you and why.
- C. I can see you're very upset. Let's walk to a quieter area and talk.
- D. No one is against you. You're imagining things.

Correct Answer: C

This response acknowledges the client's feelings and offers a concrete step to reduce stimulation, which supports de-escalation. Threatening consequences, pushing for detailed content during escalation, or arguing about the client's perception can all worsen agitation.

1808. A nurse is caring for a client with a known history of violence. Which item in the client's history is most important to consider when assessing current risk for violent behavior?

- A. Previous acts of physical assault
- B. Preference for spending time alone
- C. Frequent requests for snacks between meals
- D. Need for help with bathing

Correct Answer: A

A past history of violence is one of the strongest predictors of future violent behavior and should raise concern during assessment. The other findings may describe habits or care needs but do not strongly predict assault risk.

1809. A client who has been muttering angrily in the day room suddenly begins hitting the arm of a chair and yelling at another client. Which intervention should the nurse implement first?

- A. Ask the other client to explain what happened
- B. Offer the angry client a detailed written behavior contract
- C. Call for assistance and direct nearby clients away from the area
- D. Leave briefly to obtain the client's prescribed medication

Correct Answer: C

When behavior is escalating toward violence, the first priority is safety for everyone in the area. Calling for help and moving others away reduces immediate risk. Investigating details, negotiating a contract, or leaving the scene delays protection during an active escalation.

1810. A nurse enters a client's room and finds the client angrily tearing papers and saying, "Nobody listens to me." Which statement by the nurse is most appropriate?

- A. Stop that behavior immediately and apologize for your attitude.
- B. I want to understand what upset you. Speak with me using words, not threats.
- C. If you keep acting like this, I will call security right now.
- D. You are overreacting to a minor problem.

Correct Answer: B

This response is therapeutic because it shows willingness to listen while also setting a clear behavioral limit. Choice A is confrontational, choice C may escalate unless danger is immediate, and choice D dismisses the client's feelings.

1811. The nurse is caring for four clients. Which client presents the highest immediate risk for violence?

- A. A client who is quietly crying after a family argument
- B. A client who is pacing, refusing direction, and shouting, "Don't come near me!"
- C. A client who says, "I'm too tired to go to group today."
- D. A client who reports hearing music at night but is resting in bed

Correct Answer: B

Pacing, refusal of direction, loud verbal escalation, and perceived threat from others together suggest active agitation and possible imminent aggression. The other clients may need care, but they do not show the same immediate pattern of escalating violence risk.

1812. A client becomes angry when told visiting hours are over and says, "This place can't control me." Which nursing action is best to help prevent escalation?

- A. Explain the unit policy at length in front of other clients
- B. Offer the client a chance to sit in a quieter space and discuss options calmly
- C. Tell the client the rule is final and walk away immediately
- D. Challenge the client to explain why the rule should not apply

Correct Answer: B

Reducing stimulation and offering calm problem-solving can prevent further escalation while preserving the limit. Lengthy public explanations, abrupt disengagement without support, or challenging the client can increase anger and loss of control.

1813. A nurse is developing a plan of care for a client at risk for violent behavior. Which intervention is most appropriate to include?

- A. Approach the client with several staff members at once for routine conversations
- B. Use clear, simple directions and observe for signs of increasing agitation
- C. Encourage the client to release anger by striking furniture in the room
- D. Wait until the client becomes physically aggressive before setting limits

Correct Answer: B

Clients at risk for violence benefit from clear communication and early recognition of escalating cues so interventions can begin before loss of control. Crowding can feel threatening, striking objects can increase aggression, and limit-setting should occur early, not after physical aggression begins.

1814. A client with escalating anger is offered oral medication for agitation and refuses. The client is pacing, cursing, and slamming the door but has not attempted to strike anyone. What is the nurse's best next action?

- A.** Continue verbal de-escalation and maintain a safe, low-stimulation environment
- B.** Confront the client about being noncompliant with treatment
- C.** Restrain the client immediately because medication was refused
- D.** Leave the client alone with no further contact until calm

Correct Answer: A

Because the client is escalating but has not yet become physically assaultive, the nurse should continue least restrictive interventions such as verbal de-escalation, safety monitoring, and environmental control. Confrontation increases risk, restraint is not the first response solely for medication refusal, and abandonment removes support and monitoring.

1815. A nurse is alone with an agitated client in an interview room. The client stands up abruptly, blocks part of the doorway, and says through clenched teeth, "You're not leaving until I get what I want." What should the nurse do first?

- A.** Promise the client the requested outcome to reduce anger
- B.** Attempt to push past the client and exit quickly
- C.** Stay calm, keep a safe distance, and call for assistance
- D.** Tell the client that the behavior is childish and unacceptable

Correct Answer: C

The client is presenting an immediate intimidation and potential assault risk. The nurse should remain calm, avoid sudden provocative actions, maintain as much distance as possible, and obtain help. Promising something to gain release may reinforce threats, pushing past the client can trigger violence, and shaming language escalates danger.

1816. A nurse on a behavioral health unit notes these changes in a client over 20 minutes: increased pacing, louder speech, fixed stare, refusal of redirection, and repeated statements that staff are "setting me up." Which interpretation is best?

- A.** The client is showing anxiety that will likely resolve without intervention
- B.** The client is demonstrating escalating cues that require prompt intervention to prevent violence
- C.** The client is seeking attention and should be ignored to avoid reinforcement
- D.** The client is only expressing frustration because no physical threat has been made

Correct Answer: B

A pattern of increasing motor activity, suspiciousness, louder speech, fixed staring, and inability to accept redirection indicates escalation that can progress to violence if not addressed promptly. Waiting for a direct physical threat misses the importance of early cues. Ignoring the behavior is unsafe.

1817. A nurse is assisting with the care of a client who has a history of violent behavior during episodes of severe agitation. Which approach is most appropriate when giving directions during an escalating episode?

- A.** Use short, concrete statements and present one expectation at a time
- B.** Provide several choices quickly so the client feels in control
- C.** Use detailed explanations to help the client process the situation logically
- D.** Speak firmly from behind the client to avoid direct confrontation

Correct Answer: A

During escalation, a client may have reduced ability to process complex information. Short, concrete directions with one expectation at a time are easiest to understand and least likely to escalate the situation. Multiple choices, lengthy explanations, or approaching from behind can increase confusion or perceived threat.

1818. A client who had been cursing and pacing now becomes very still, stares at a staff member, and speaks in a low, tense voice: "You'll be sorry." Which action is the nurse's priority?

- A.** Interpret the sudden quiet as a sign that the client is regaining self-control
- B.** Continue the discussion to help the client vent feelings fully
- C.** Recognize the behavior as a possible sign of imminent violence and obtain immediate assistance
- D.** Ask the client to write down feelings in a journal before anyone intervenes

Correct Answer: C

A sudden shift from overt agitation to focused, tense quietness with a threatening statement can signal imminent violent action, not improvement. The nurse should treat this as high risk and get help immediately. Assuming the client is calming down is a dangerous trap.

1819. A nurse is reviewing incidents from the previous shift to identify which client behavior required the earliest intervention for violence prevention. Which behavior should the nurse identify?

- A.** A client mutters complaints while folding laundry alone
- B.** A client repeatedly argues that the meal tray is incorrect
- C.** A client begins pacing, scanning the room, and clenching the jaw after a phone call
- D.** A client declines recreation and goes to bed early

Correct Answer: C

Pacing, hypervigilant scanning, and jaw clenching are early physiologic and behavioral signs of rising aggression and are the point when prevention is most effective. Complaining or withdrawing may require follow-up, but they are less specific for impending violence.

1820. During an assessment, a client who has been pacing in the hallway begins speaking more loudly, clenches both fists, and says, "He keeps pushing me, and I'm about to do something." What is the nurse's priority action?

- A.** Tell the client that threatening language is unacceptable
- B.** Move the client to a quieter area and use calm, brief statements
- C.** Ask the client to explain in detail why he is upset
- D.** Leave the client alone to cool down for 30 minutes

Correct Answer: B

The client is showing escalating warning signs for possible violence, including pacing, loud speech, clenched fists, and a verbal threat. The priority is to reduce stimulation and begin de-escalation with calm, simple communication in a safer setting. Telling the client the behavior is unacceptable may sound confrontational and increase anger. Asking for a detailed explanation can demand too much from an escalating client. Leaving the client alone without further intervention is unsafe because the risk is already increasing.

1821. A nurse is preparing the environment for a client admitted after several angry outbursts at home. The client is restless but not yet threatening staff. Which environmental action is most appropriate to help lower the risk of violence?

- A.** Place the client in a quiet area with minimal noise and reduced stimulation
- B.** Encourage the client to stay in the busy day room to observe others' behavior
- C.** Keep the client near the nurses' station so several conversations can be monitored at once
- D.** Ask multiple staff members to remain with the client for constant discussion and support

Correct Answer: A

A quieter, less stimulating setting can help decrease agitation and lower the chance of escalation. Busy areas, extra conversation, and multiple staff members can increase sensory input and tension in a client already at risk for losing control. The trap is choosing a highly supervised location without considering that overstimulation itself can worsen agitation.

1822. The nurse is caring for a client with escalating anger. Which statement by the nurse is most therapeutic?

- A. You need to stop acting this way right now.
- B. If you keep yelling, you will lose your privileges.
- C. I can see that you're angry. Let's walk to a quieter place and talk.
- D. Why do little things upset you so much?

Correct Answer: C

This response acknowledges the client's feeling and offers a concrete, nonthreatening step to reduce stimulation and promote safety. It uses calm, direct communication, which is appropriate for a potentially escalating client. Choice A is controlling, B is punitive, and D is judgmental and likely to increase defensiveness rather than lower the risk of violence.

1823. A nurse is interviewing a client with a recent history of aggressive behavior. Which finding requires the most immediate action to protect safety?

- A. The client says, "People here don't understand me," and avoids eye contact
- B. The client reports poor sleep for 2 nights and appears irritable
- C. The client begins pacing, picks up a metal IV pole, and says, "Maybe this will get someone's attention"
- D. The client refuses to attend group therapy and asks to stay alone in the room

Correct Answer: C

Picking up a potential weapon while making a threatening statement indicates immediate danger and requires urgent action to protect the client, staff, and others. The other findings may increase concern for future escalation, but they do not show the same level of imminent risk. The key distinction is the combination of threatening behavior and access to an object that can be used to cause harm.

Substance misuse and withdrawal

1824. A nurse is caring for a client who is 10 hours postoperative. The client says quietly, "I usually drink a lot every day, and I haven't had anything since yesterday." Which action should the nurse take first?

- A. Ask the provider for a sleeping medication
- B. Assess the client for early alcohol withdrawal symptoms
- C. Tell the client to rest and avoid thinking about alcohol
- D. Limit visitors to reduce stimulation

Correct Answer: B

The priority is to assess for early alcohol withdrawal because abrupt cessation after heavy daily use can lead to tremors, anxiety, agitation, sweating, and progression to more severe complications. Assessment comes before unrelated interventions. Asking for a sleep aid or limiting visitors may be helpful later, but they do not address the immediate risk. Telling the client not to think about alcohol is nontherapeutic and delays needed assessment.

1825. A client who misuses alcohol is admitted to the medical unit. Which finding is most consistent with early alcohol withdrawal?

- A. Tremors, diaphoresis, and anxiety
- B. Pinpoint pupils and slowed respirations
- C. Severe drowsiness and slurred speech after taking a sedative
- D. Euphoric mood and increased appetite

Correct Answer: A

Early alcohol withdrawal commonly presents with tremors, sweating, restlessness, and anxiety. Pinpoint pupils and slowed respirations suggest opioid effects, not alcohol withdrawal. Drowsiness and slurred speech fit CNS depressant use or oversedation. Euphoria with increased appetite is not a typical withdrawal pattern.

1826. A client admitted for possible substance withdrawal becomes increasingly restless and says, "The bugs are crawling on my arms." Which nursing action is the priority?

- A.** Stay with the client and maintain safety precautions
- B.** Explain that the sensation is impossible and should be ignored
- C.** Ask family members to describe the client's usual behavior
- D.** Encourage the client to walk in the hallway to reduce anxiety

Correct Answer: A

The priority is safety. Tactile hallucinations during withdrawal can accompany severe agitation and rising risk for injury. Staying with the client and maintaining safety precautions addresses the immediate danger. Telling the client to ignore the sensation is dismissive and nontherapeutic. Family input may be useful later, and hallway ambulation could increase risk if the client is unstable.

1827. A nurse is interviewing a client with a history of substance misuse. Which statement by the nurse uses therapeutic communication?

- A.** You need to promise that you will stop using drugs today
- B.** Why would you do this to yourself after treatment?
- C.** Tell me what substance use has been like for you recently
- D.** If you don't quit now, your life will get worse

Correct Answer: C

An open-ended, nonjudgmental invitation encourages the client to share useful information and supports trust. The other responses are judgmental, coercive, or shaming, which can shut down communication and reduce honesty during assessment.

1828. A client who is withdrawing from alcohol is confused and trying to climb out of bed.

Which intervention is most appropriate?

- A.** Place the client in a brightly lit room near the nurses' station with side rails up as appropriate
- B.** Turn off all lights and close the door to promote sleep
- C.** Provide several caffeinated drinks to improve alertness
- D.** Confront the client firmly each time confusion occurs

Correct Answer: A

A safe environment with close observation helps protect a confused, agitated client in withdrawal. Adequate lighting may decrease misinterpretation of shadows, and proximity to staff supports rapid intervention. Darkening the room completely can worsen disorientation. Caffeine may increase agitation. Confrontation is not therapeutic and may escalate behavior.

1829. The nurse is caring for four clients.

Which client should the nurse assess first for possible life-threatening withdrawal complications?

- A.** A client who stopped drinking alcohol yesterday and is now disoriented and hallucinating
- B.** A client who smokes cigarettes and asks for nicotine gum
- C.** A client who reports craving caffeine and has a mild headache
- D.** A client who used marijuana last week and feels bored

Correct Answer: A

Disorientation and hallucinations after abrupt alcohol cessation suggest severe withdrawal, which can rapidly become life-threatening and requires immediate assessment. The other clients may be uncomfortable, but their findings do not represent the same urgent risk.

1830. A client in withdrawal says, "I know the voices are not real, but they scare me." What is the nurse's best response?

- A.** You should try harder not to listen to them
- B.** What are the voices telling you to do right now?
- C.** There is nothing to worry about because the voices are imaginary

D. If you keep hearing voices, you may need to be restrained

Correct Answer: B

The nurse should further assess the hallucinations, especially for commands that increase danger to the client or others. This response is calm, direct, and safety-focused. Telling the client to try harder is dismissive. Saying there is nothing to worry about minimizes the fear. Threatening restraints is inappropriate and escalatory.

1831. A client admitted with polysubstance misuse has shaky hands, sweating, nausea, and increasing agitation. The client says, "I feel like something bad is about to happen." Which additional finding most strongly supports alcohol withdrawal rather than opioid intoxication?

- A.** Fine tremors and elevated blood pressure
- B.** Respiratory rate of 8/min and pinpoint pupils
- C.** Marked sedation and difficulty staying awake
- D.** New onset constipation without anxiety

Correct Answer: A

Fine tremors with autonomic hyperactivity such as elevated blood pressure are consistent with alcohol withdrawal. Opioid intoxication more often causes respiratory depression, pinpoint pupils, and sedation. Constipation may occur with opioid use but does not explain the acute agitated picture.

1832. A nurse is planning care for a client who is withdrawing from a sedative-hypnotic medication. Which nursing goal has the highest priority?

- A.** The client will attend a support group before discharge
- B.** The client will remain free from injury during withdrawal
- C.** The client will identify three long-term career goals
- D.** The client will discuss family conflict in detail

Correct Answer: B

Withdrawal from sedative-hypnotics can involve agitation, confusion, and seizures, so physical safety is the immediate priority. Support groups, long-term goals, and family issues may become important later, but they are not first during acute withdrawal management.

1833. A client with a history of heavy alcohol use has not had a drink for 2 days. The client is now severely agitated, febrile, tachycardic, and seeing insects on the wall. Which interpretation should guide the nurse's action?

- A.** The client is experiencing a mild craving response that should resolve with reassurance
- B.** The client is showing signs of severe alcohol withdrawal that require urgent intervention
- C.** The client is demonstrating attention-seeking behavior related to hospitalization
- D.** The client is most likely overreacting to environmental noise

Correct Answer: B

Severe agitation, fever, tachycardia, and hallucinations after alcohol cessation indicate severe withdrawal and demand urgent intervention. Reassurance alone is not enough. Labeling the behavior as attention-seeking or environmental overreaction is unsafe because it ignores potentially life-threatening findings.

1834. During admission, a client says, "I only drink on weekends," but the client's partner reports daily alcohol use and morning drinking. What is the nurse's best action?

- A.** Document the partner's report and continue assessing the client's withdrawal risk
- B.** Assume the partner is exaggerating and use only the client's statement
- C.** Confront the client immediately about lying
- D.** Delay assessment until the client is more willing to talk

Correct Answer: A

When information conflicts, the nurse should document objectively and continue assessment, especially because underreporting substance use is common and withdrawal risk may be serious. Assuming the partner is wrong, confronting the client, or delaying assessment can lead to missed cues and unsafe care.

1835. A client recovering from acute withdrawal tells the nurse, "I can handle this by myself after discharge. I don't need any follow-up." Which response by the nurse is best?

- A.** You are probably right if your symptoms are gone now

- B. Relapse is your responsibility, so make sure you do better this time
- C. What concerns do you have about follow-up support after discharge?
- D. You must agree to treatment or you will likely be readmitted

Correct Answer: C

This response uses therapeutic communication to explore barriers, readiness, and concerns without judgment. It helps the nurse assess motivation and support planning. The other choices are minimizing, shaming, or coercive and are less effective in substance-use care.

1836. A client with stimulant misuse has been awake for an extended period and is becoming suspicious, pacing, and verbally aggressive. Which nursing action is most appropriate first?

- A. Approach the client with several staff members speaking at once
- B. Move the client to a quieter area and use short, calm statements
- C. Challenge the client's suspicious thoughts directly
- D. Encourage vigorous exercise to reduce excess energy

Correct Answer: B

De-escalation begins with reducing stimulation and using calm, simple communication. This is especially important when a client is overstimulated, sleep-deprived, or suspicious. Multiple staff speaking at once, arguing about suspicious thoughts, or encouraging vigorous activity can increase agitation and risk.

1837. A client being treated for withdrawal suddenly has a generalized seizure. Which action should the nurse take first?

- A. Insert an oral airway to prevent tongue injury
- B. Restrain the client's arms and legs
- C. Protect the client from injury and maintain the airway as able
- D. Offer oral fluids when the seizure slows

Correct Answer: C

During a seizure, the nurse's first priority is safety: protect the client from injury and support airway maintenance as possible. Do not insert objects into the mouth or restrain the client, because both can cause harm. Oral fluids are unsafe during or immediately after a seizure until swallowing is assessed.

1838. A client in withdrawal is alert and says, "I am so anxious that I feel like leaving right now." Which nursing response is best?

- A. If you leave, that is your choice
- B. Calm down. Your anxiety is making this worse
- C. Tell me what you are feeling right now, and let's work on staying safe
- D. You cannot leave until the staff says you are ready

Correct Answer: C

This response acknowledges the client's distress, invites further assessment, and focuses on immediate safety. It is therapeutic and practical. The other responses are dismissive, judgmental, or authoritarian and may increase anxiety or mistrust.

1839. A client who stopped heavy daily alcohol use 8 hours ago reports nausea, anxiety, and tremors. Which assessment finding most strongly suggests the withdrawal is becoming more severe and requires immediate follow-up?

- A. Mild headache and poor appetite
- B. Fine hand tremors when arms are extended
- C. New disorientation to place and time
- D. Request for a dark, quiet room

Correct Answer: C

New disorientation to place and time suggests worsening neurologic involvement and possible progression to severe alcohol withdrawal, so it requires prompt follow-up. Mild headache, poor appetite, and tremors can occur in earlier withdrawal. Wanting a dark, quiet room may reflect sensitivity and anxiety, but it is less urgent than an acute change in mental status.

1840. A nurse is caring for a client with suspected opioid overdose. Which assessment finding is most consistent with opioid intoxication rather than alcohol withdrawal?

- A. Dilated pupils and marked tremors
- B. Pinpoint pupils and decreased respirations
- C. Hypertension and visual hallucinations
- D. Diaphoresis and increasing agitation

Correct Answer: B

Pinpoint pupils and decreased respirations are classic findings of opioid intoxication. Alcohol withdrawal more commonly presents with autonomic hyperactivity such as tremors, diaphoresis, agitation, hypertension, and in severe cases hallucinations. A common trap is choosing agitation or sweating, which fit withdrawal better than opioid overdose.

1841. A client with long-term benzodiazepine misuse is admitted after abruptly stopping the medication. Which complication is the nurse most concerned about during withdrawal?

- A. Constipation
- B. Bradycardia
- C. Seizures
- D. Hypothermia

Correct Answer: C

Abrupt withdrawal from benzodiazepines can be dangerous and may lead to seizures. The nurse should monitor closely for escalating neurologic and autonomic symptoms. Constipation, bradycardia, and hypothermia are not the key life-threatening concerns associated with sedative-hypnotic withdrawal.

1842. A client withdrawing from alcohol is anxious, restless, and easily overstimulated by noise on the unit. Which environmental action is most appropriate?

- A. Move the client to a quiet room with reduced stimulation
- B. Encourage group activities to distract the client
- C. Keep the television on to provide orientation
- D. Assign the client to a room near the nurses' station for conversation

Correct Answer: A

A quiet room with reduced stimulation helps decrease agitation and promotes safety for a client in alcohol withdrawal. Group activities, frequent conversation, and constant television can increase overstimulation and worsen restlessness or confusion.

1843. A client says, "I stopped using heroin yesterday, and now I feel terrible." Which findings would the nurse expect with opioid withdrawal?

- A. Euphoria, slowed respirations, and pinpoint pupils
- B. Lacrimation, muscle aches, and yawning
- C. Severe hypertension, fever, and visual hallucinations
- D. Profound sedation and decreased bowel sounds

Correct Answer: B

Opioid withdrawal commonly causes lacrimation, rhinorrhea, yawning, muscle aches, restlessness, and gastrointestinal upset. Euphoria, respiratory depression, and pinpoint pupils suggest opioid intoxication. Visual hallucinations and marked autonomic instability fit severe alcohol withdrawal more closely than uncomplicated opioid withdrawal.

1844. A client admitted for possible substance withdrawal is pacing, sweating, and asking repeatedly, "When can I leave?" Which nursing action is best to reduce escalation while maintaining therapeutic communication?

- A. Tell the client to stop pacing because it is disruptive
- B. Ask rapid direct questions to finish the assessment quickly
- C. Use short, calm statements and explain each step of care
- D. Avoid interacting until the client settles down

Correct Answer: C

Short, calm statements and clear explanations help reduce anxiety and support a client who may be withdrawing and becoming overstimulated. Telling the client to stop pacing can increase defensiveness. Rapid questioning can feel threatening, and avoiding interaction misses a chance to de-escalate and assess risk.

1845. A client with suspected cocaine use is brought to the emergency department. The client is extremely alert, restless, and reports chest tightness. Which nursing concern should take priority?

- A. Risk for impaired skin integrity
- B. Risk for social isolation
- C. Risk for cardiovascular complications
- D. Risk for urinary retention

Correct Answer: C

Cocaine and other stimulants can cause serious cardiovascular complications, including chest pain, dysrhythmias, and hypertension, so that is the priority concern. The other risks may matter later, but they are not the most urgent in a client with stimulant effects and chest symptoms.

1846. A nurse is caring for a client in alcohol withdrawal who says, "I hear people whispering in the room." The nurse does not hear voices. Which response is best?

- A. I do not hear the voices, but I can see that this is frightening for you.
- B. There is no one here, so you need to ignore it.
- C. What are the voices telling you to do right now?
- D. You are imagining things because you drank too much.

Correct Answer: A

The best response presents reality without arguing and acknowledges the client's fear. This supports safety and therapeutic communication. Telling the client to ignore it is dismissive, and stating the client drank too much is judgmental. Asking what the voices say may be useful later in some settings, but first the nurse should acknowledge distress and avoid reinforcing the hallucination.

1847. A client who misuses alcohol is 24 hours into hospitalization and has increasing tremors and diaphoresis. Which additional data should the nurse report most urgently?

- A. The client has not finished lunch
- B. The client rates anxiety as 6 out of 10
- C. The client is sleeping only in short intervals
- D. The client now has a temperature of 38.6 C (101.5 F)

Correct Answer: D

Fever with increasing tremors and diaphoresis suggests worsening withdrawal and possible progression toward severe complications, so it should be reported urgently. Anxiety, poor intake, and poor sleep are common but less immediately concerning than a rising temperature in this context.

1848. The nurse is teaching a client about recovery after treatment for substance misuse. Which client statement shows the best understanding of relapse prevention?

- A. Once withdrawal is over, I should be able to manage cravings on my own.
- B. If I avoid all stress, I will not be at risk for relapse.
- C. Having follow-up support can help me handle triggers before I return to use.
- D. Relapse only happens when a person is not serious about recovery.

Correct Answer: C

Follow-up support and planning for triggers are key parts of relapse prevention. Recovery usually requires ongoing support, not just getting through withdrawal. Stress cannot be eliminated completely, and relapse is not simply a sign that someone is not serious; it is a risk that should be addressed with planning and support.

1849. A client with methamphetamine misuse has been awake for 2 days and is now suspicious and pacing. The client suddenly clenches fists and shouts, "Don't come near me!" Which action should the nurse take first?

- A. Approach the client quickly with two staff members to demonstrate control

- B.** Stand at a safe distance and speak in a calm, nonthreatening manner
- C.** Turn on bright lights so the client can see the room clearly
- D.** Confront the client's behavior and set immediate consequences

Correct Answer: B

A calm, nonthreatening approach from a safe distance is the best first step to de-escalate a potentially violent situation. Quickly approaching can increase perceived threat. Bright lights may worsen agitation, and confrontation often escalates paranoia and aggression.

1850. A hospitalized client says, "I drink a pint of liquor every night, but I don't want anyone to know." Which nursing response is best?

- A.** I will keep that completely secret from the healthcare team.
- B.** You should tell your family before we discuss this further.
- C.** Thank you for telling me. This information helps us plan safe care during your stay.
- D.** That amount means you probably have a substance use disorder.

Correct Answer: C

This response is nonjudgmental, therapeutic, and explains why the information matters for safe care, including monitoring for withdrawal and complications. Promising total secrecy is inappropriate because relevant health information must be shared with the care team. Forcing disclosure to family and labeling the client are not therapeutic responses.

1851. A client who used alcohol heavily before admission is on the second hospital day. Which finding most strongly indicates possible delirium tremens rather than mild withdrawal?

- A.** Fine tremors and nausea
- B.** Anxiety and insomnia
- C.** Disorientation, severe agitation, and hallucinations
- D.** Headache and sensitivity to sound

Correct Answer: C

Disorientation, severe agitation, and hallucinations indicate severe alcohol withdrawal and possible delirium tremens. Fine tremors, nausea, anxiety, insomnia, headache, and sensitivity to sound are more consistent with earlier or milder withdrawal. The key distinction is the severe change in mental status plus autonomic and perceptual disturbance.

1852. A client with alcohol withdrawal is increasingly confused and attempts to pull out the IV tubing. Which nursing action best addresses both safety and psychosocial needs?

- A.** Apply restraints immediately and leave the client alone to rest
- B.** Reorient the client frequently and stay with the client as stimulation is reduced
- C.** Explain that the behavior is unacceptable and could delay discharge
- D.** Ask several staff members to talk with the client at the same time

Correct Answer: B

Frequent reorientation, presence, and reduced stimulation support both safety and the client's psychosocial distress during withdrawal. Immediate restraint without less restrictive support is not the best choice unless there is immediate danger and other measures fail. Threatening discharge consequences and crowding the client can worsen agitation and confusion.

1853. A client is admitted after abruptly stopping long-term barbiturate use. Which assessment finding requires the nurse's most urgent action?

- A.** The client reports poor appetite and nausea
- B.** The client says it is hard to sleep
- C.** The client becomes suddenly rigid and unresponsive
- D.** The client requests medication for a headache

Correct Answer: C

Sudden rigidity and unresponsiveness suggest a seizure or other acute neurologic emergency, which requires immediate action. Barbiturate withdrawal can be life-threatening. Poor appetite, insomnia, and headache may occur but are not as urgent as abrupt loss of responsiveness.

1854. A client admitted to the medical unit says quietly, "I usually take whatever pain pills I can get, and I drank this morning so I wouldn't feel shaky." Which nursing action is the priority on admission?

- A. Ask the client to sign a behavioral contract about future substance use
- B. Assess the timing, amount, and type of substances last used
- C. Teach the client about community recovery groups
- D. Encourage the client to discuss family stressors causing substance use

Correct Answer: B

The priority is to assess what substances were used, how much, and when they were last taken because this helps identify the risk for intoxication, withdrawal, and urgent complications. Teaching, counseling, and planning for recovery are important later, but immediate safe care depends on accurate assessment first.

1855. A client with a history of alcohol misuse is 36 hours into hospitalization. The client is sweating, restless, and says, "The bugs are crawling on me." Vital signs are BP 168/98 mm Hg, HR 122/min, RR 24/min, temperature 38.4°C (101.1°F). Which prescription should the nurse anticipate as most appropriate?

- A. A benzodiazepine to reduce severe alcohol withdrawal symptoms
- B. Naloxone to reverse central nervous system depression
- C. Methadone to prevent opioid cravings
- D. Flumazenil to counter benzodiazepine effects

Correct Answer: A

This client shows signs of severe alcohol withdrawal, including hallucinations, autonomic hyperactivity, fever, and agitation.

Benzodiazepines are commonly used to reduce withdrawal severity and prevent complications such as seizures. Naloxone is used for opioid overdose, methadone is associated with opioid use treatment, and flumazenil reverses benzodiazepine effects rather than treating alcohol withdrawal.

1856. A nurse is caring for a client who is 2 days into stimulant withdrawal after heavy cocaine use. Which finding is most important for the nurse to assess directly?

- A. Dilated pupils
- B. Strong appetite for snacks
- C. Thoughts of self-harm
- D. Sleeping most of the day

Correct Answer: C

During stimulant withdrawal, clients may have depression, exhaustion, and a crash in mood. The most important finding to assess directly is suicidal thinking because it poses an immediate threat to safety. Increased sleep and appetite can occur with withdrawal but are less urgent than possible self-harm risk. Dilated pupils are not the priority in this situation.

1857. A client who has been misusing multiple substances becomes increasingly agitated during hospitalization, paces near the exit, and says, "You can't keep me here. I'm leaving now." The client is diaphoretic, tremulous, and unable to focus on instructions. Which nursing action is best first?

- A. Stand in the doorway to physically block the client from leaving
- B. Confront the client about the danger of continued substance use
- C. Move the client to a quieter area and speak in short, calm statements while obtaining help
- D. Tell the client that leaving will result in discharge from future care

Correct Answer: C

The best first action is to reduce stimulation, use calm and simple communication, and obtain help. This approach addresses escalating behavior while supporting safety and therapeutic communication. Physically blocking the client can worsen agitation and increase the risk of violence. Confrontation and threats are nontherapeutic and are unlikely to help a client who may be impaired by withdrawal or intoxication.

Behavior management

1858. A client with schizophrenia begins pacing the hallway, clenching fists, and speaking loudly to unseen others. Which action should the nurse take first?

- A. Approach the client calmly and speak in a clear, low voice
- B. Ask several staff members to stand around the client for safety
- C. Tell the client that the behavior must stop immediately
- D. Leave the client alone until the agitation passes

Correct Answer: A

A calm, nonthreatening approach is the best first action because it supports de-escalation while allowing the nurse to assess risk. Surrounding the client can feel threatening and increase agitation. Demanding that the behavior stop may escalate the situation. Leaving the client alone ignores a potentially rising risk of violence.

1859. A hospitalized client with mania is loudly interrupting a group activity, touching other clients' belongings, and moving rapidly from one activity to another. Which intervention is most appropriate?

- A. Encourage the client to remain in group to improve social skills
- B. Set firm, simple limits and redirect the client to a low-stimulation activity
- C. Ignore the behavior unless the client becomes physically aggressive
- D. Explain in detail how the behavior affects the unit environment

Correct Answer: B

Clients with mania often respond best to clear, brief limits and reduced environmental stimulation. This approach promotes safety and behavior control. Keeping the client in a stimulating group may worsen the behavior. Ignoring intrusive behavior allows it to continue. Lengthy explanations are often ineffective when the client is highly distractible.

1860. A client who has dementia becomes agitated every evening, tries to leave the facility, and says, "I have to get home before dark." Which nursing response is best?

- A. "You live here now, so there is no reason to leave."
- B. "If you keep trying to leave, you will have to stay in your room."
- C. "You seem worried about getting home. Let's walk together for a few minutes."
- D. "Your memory is confused right now, and this is called sundowning."

Correct Answer: C

Validating the client's feelings and redirecting to a calming activity is therapeutic and supports behavior management in dementia. Correcting the client or arguing with the belief often increases distress. Threatening consequences can worsen agitation. Labeling the condition does not address the immediate emotional need.

1861. During an interview, a client becomes angry and shouts, "Nobody here listens to me!" Which response by the nurse is most therapeutic?

- A. "You need to lower your voice before we continue."
- B. "Tell me what happened that made you feel ignored."
- C. "There is no reason to get this upset."
- D. "If you cannot stay calm, I will end the conversation."

Correct Answer: B

This response acknowledges the client's emotion and invites the client to express the concern, which can reduce escalation and improve communication. The other options focus on control, dismissal, or threat before first trying to understand the cause of the anger.

1862. A client on an inpatient unit begins yelling at another client in the dayroom. Which action by the nurse is most appropriate?

- A. Move the arguing clients apart and speak to each in a calm, firm manner
- B. Ask the other clients to explain what started the conflict
- C. Tell both clients they will lose privileges if they continue arguing
- D. Wait to intervene unless one client attempts to strike the other

Correct Answer: A

Separating the clients and using calm, firm communication helps reduce stimulation and lowers the risk of violence. Involving bystanders can increase the audience effect and agitation. Threats may escalate the conflict. Waiting delays needed intervention when behavior is already unsafe.

1863. A nurse is caring for a client who is becoming increasingly anxious and restless before a scheduled procedure. The client is wringing hands, speaking rapidly, and repeatedly asking when the procedure will happen. Which intervention is best to help manage the behavior?

- A. Provide short, clear answers and remain with the client
- B. Encourage the client to discuss every possible complication in detail
- C. Tell the client to stop focusing on the procedure
- D. Leave the room so the client can settle down alone

Correct Answer: A

Anxious clients often benefit from a calm presence and simple, direct communication. This decreases overload and supports control. Detailed discussion of all complications may intensify anxiety. Telling the client to stop worrying is nontherapeutic. Leaving the client alone removes support when anxiety is rising.

1864. A client with a traumatic brain injury becomes impulsive and repeatedly tries to pull off the cardiac monitor leads. Which nursing intervention is most appropriate?

- A. Apply strict punishment each time the client removes a lead
- B. Provide simple explanations and redirect the client's hands to a safe activity
- C. Argue with the client about why the monitor is medically necessary
- D. Turn off the monitor to reduce the client's frustration

Correct Answer: B

Clients with cognitive or impulse-control problems often respond best to brief explanations and redirection. Punishment and arguing are unlikely to improve behavior and may increase agitation. Turning off needed monitoring sacrifices safety rather than managing behavior appropriately.

1865. A client admitted for alcohol withdrawal becomes irritable, paces in the room, and snaps at staff who ask questions. Which environmental modification is most appropriate?

- A. Place the client near the busy nurses' station for close observation
- B. Limit stimulation by reducing noise and keeping the environment calm
- C. Encourage frequent visitors to distract the client
- D. Move the client from room to room to prevent boredom

Correct Answer: B

Reducing noise and stimulation can help decrease irritability and agitation, especially when a client is already physiologically stressed. A busy location, frequent visitors, and unnecessary room changes can increase sensory overload and worsen the behavior.

1866. A client says to the nurse, "If my wife doesn't come visit tonight, I'm going to throw this chair through the window." Which action should the nurse take first?

- A. Ask the client why the visit is so important
- B. Tell the client that making threats is unacceptable

C. Ensure immediate safety by maintaining distance and getting assistance

D. Promise to call the wife right away

Correct Answer: C

A direct threat of violence requires the nurse to prioritize immediate safety for the client, staff, and others. Maintaining a safe position and obtaining help are first. Exploring feelings may be appropriate later if the situation is controlled. Simply labeling the threat unacceptable does not address the immediate risk. Promising an outcome may not be realistic and does not come before safety measures.

1867. A nurse is planning care for a client who frequently becomes hostile when asked to follow unit routines. Which approach is most likely to reduce power struggles?

A. Offer limited choices whenever possible

B. Confront the client each time with a detailed explanation

C. Use different rules depending on the nurse on duty

D. Avoid setting limits so the client does not become upset

Correct Answer: A

Offering limited choices supports autonomy while preserving structure, which can reduce resistance and power struggles. Repeated confrontation often escalates hostility. Inconsistent rules promote testing of limits. Avoiding limits altogether creates confusion and can worsen maladaptive behavior.

1868. An adolescent client on a behavioral health unit becomes increasingly oppositional and says, "You can't make me do anything." Which nursing action is best?

A. Challenge the client to explain why the rules are wrong

B. State the expected behavior clearly and describe the immediate consequence calmly

C. Ignore the statement completely and walk away

D. Begin negotiating all unit rules with the client

Correct Answer: B

Behavior management is most effective when expectations and consequences are communicated clearly, calmly, and consistently. Challenging the client can lead to a struggle. Ignoring may reinforce defiance if limits are needed. Negotiating all rules undermines structure and consistency.

1869. A client with paranoid thinking refuses medication and shouts, "You're trying to poison me!" The client is breathing rapidly and backing into a corner. Which response by the nurse is best?

A. "That's ridiculous. No one is trying to poison you."

B. "You must take this medication now or you will lose privileges."

C. "I can see you're frightened. I'll stay a few feet away while we talk."

D. "If you do not calm down, staff will hold you down and give the medication."

Correct Answer: C

This response acknowledges the client's fear without agreeing with the delusion and uses a nonthreatening stance that supports de-escalation. Arguing about the belief usually increases defensiveness. Threats about privileges or forced action can intensify paranoia and escalate the encounter.

1870. A client with borderline personality disorder scratches the forearm with a plastic utensil after being told that visiting hours are over. Which nursing action is most appropriate after the client's immediate physical safety is addressed?

A. Provide intense emotional attention each time the client self-harms

B. Discuss the incident briefly, reinforce limits, and help the client identify another coping strategy

C. Tell the client the behavior is manipulative and will not be tolerated

D. Remove all expectations for the rest of the shift to avoid another episode

Correct Answer: B

After safety is addressed, effective behavior management uses a calm, consistent approach: reinforce limits and redirect toward healthier coping. Overly intense attention may unintentionally reinforce the behavior. Labeling the client as manipulative is nontherapeutic. Removing all expectations undermines consistent limit-setting.

1871. A client with moderate dementia strikes out at staff during bathing. Which change in the plan of care is most appropriate?

- A.** Have several staff members complete the bath as quickly as possible
- B.** Offer the bath at a different time and use one-step instructions during care
- C.** Insist on finishing the bath to teach the client the routine
- D.** Reschedule hygiene for the next week to avoid upsetting the client

Correct Answer: B

Aggressive behavior during personal care in dementia is often reduced by adjusting timing, minimizing stimulation, and giving simple one-step directions. Rushing with multiple staff may feel threatening. Insisting on finishing despite distress can escalate aggression. Delaying hygiene for an extended period is not an appropriate solution.

1872. A client on a medical-surgical unit has become increasingly demanding, angry, and verbally abusive since learning of a new cancer diagnosis. The client rings repeatedly, criticizes staff, and refuses routine care unless the same nurse provides it. Which nursing approach is best?

- A.** Assign a different nurse each hour so the client does not become attached to one person
- B.** Respond with consistent limits, acknowledge the client's distress, and avoid arguing about unreasonable demands
- C.** Meet every request promptly to reduce the client's anger
- D.** Tell the client that the behavior is unacceptable and that staff will return only when the client is polite

Correct Answer: B

This client is likely expressing distress through controlling behavior. The most effective response combines empathy with clear, consistent boundaries and avoids power struggles. Constantly changing caregivers increases instability. Meeting every demand reinforces maladaptive behavior. Withdrawing care until the client behaves politely does not address the underlying distress and can escalate conflict.

1873. A client with major neurocognitive disorder becomes upset when staff members change from day to day and accuses them of "doing everything wrong." Which nursing intervention is most likely to reduce this behavior?

- A.** Rotate caregivers often so the client adjusts to change
- B.** Provide a consistent routine and familiar caregivers when possible
- C.** Correct the client each time the accusation is made
- D.** Ask the family to stop visiting for several days

Correct Answer: B

Clients with cognitive impairment often do better with predictability. A consistent routine and familiar caregivers can decrease confusion and agitation. Frequent changes usually worsen behavior, repeated correction may increase defensiveness, and limiting family visits is not an appropriate first strategy unless visits clearly trigger distress.

1874. A client admitted with acute confusion repeatedly tries to get out of bed and pull out the IV line. Which action is most appropriate to help manage the behavior?

- A.** Move the client to a quiet room near the nurses' station and provide frequent reorientation
- B.** Explain once that the behavior is unsafe and then avoid further interaction
- C.** Turn off the room lights so the client can rest undisturbed
- D.** Challenge the client to remember the date and location every hour

Correct Answer: A

A quiet setting, close observation, and frequent reorientation are appropriate nonpharmacologic interventions for acute confusion and impulsive behavior. Avoiding interaction reduces supervision, darkening the room can worsen disorientation, and repeatedly challenging memory may increase agitation rather than improve behavior.

1875. A client on a behavioral health unit says to the nurse, "That man in the next room keeps staring at me. I'm about to make him stop." The client is standing rigidly and breathing faster. Which nursing action is the priority?

- A.** Ask the other client whether staring has occurred
- B.** Move the client to a quieter area and remain with the client while setting clear limits
- C.** Tell the client that the belief is false and irrational
- D.** Document the threat and continue routine observations

Correct Answer: B

The priority is to reduce immediate risk by removing stimulation, staying with the client, and setting clear behavioral limits. Investigating details can wait until safety is addressed. Directly arguing about the belief may escalate anger, and documentation without intervention fails to address the threatened aggression.

1876. A postoperative client becomes angry whenever pain medication is not available yet and begins insulting staff. Which nursing statement is most appropriate?

- A.** You need to stop being rude if you want help
- B.** I can see you're uncomfortable. I will tell you when the next dose is due and help you with other comfort measures now
- C.** There is no reason to be this upset over pain medicine
- D.** If you apologize, I will come back and talk with you

Correct Answer: B

This response acknowledges the client's distress, gives clear information, and redirects toward acceptable coping and available options. The other responses are punitive or dismissive and are likely to intensify the client's hostile behavior.

1877. A client with bipolar disorder is hyperverbal and repeatedly interrupts one-to-one teaching. Which intervention is most appropriate to improve behavioral control during teaching?

- A.** Schedule a long teaching session to cover all instructions at once
- B.** Use brief, focused sessions with simple directions and few distractions
- C.** Wait until the client asks for information
- D.** Encourage the client to talk freely before each topic begins

Correct Answer: B

Brief, structured teaching with minimal distractions is more effective for a client who is distractible and intrusive. Long sessions and open-ended talking increase overstimulation and reduce attention. Waiting for the client to request information is unlikely to meet needs in a client with poor concentration and impulse control.

1878. On an inpatient unit, a client frequently curses at staff when redirected but has not threatened harm or moved toward anyone. Which nursing action is most appropriate?

- A.** State that abusive language is not acceptable and continue the interaction in a calm manner
- B.** Immediately call security to remove the client from the area
- C.** Ignore the behavior completely so it is not reinforced
- D.** Argue with the client until the client admits the behavior is wrong

Correct Answer: A

Calmly setting limits on unacceptable behavior while maintaining a therapeutic approach is appropriate for verbal aggression without immediate physical danger. Calling security may be unnecessary at this stage, total ignoring can allow escalation, and arguing increases defensiveness and loss of control.

1879. A client with delirium becomes more restless after transfer to a semiprivate room and keeps accusing the roommate of taking personal items. Which change would the nurse recommend first?

- A. Request a private room with reduced noise and fewer interruptions
- B. Move the client's belongings out of sight
- C. Tell the client the accusations are inappropriate
- D. Encourage the roommate to explain what happened

Correct Answer: A

Delirium often worsens with overstimulation and unfamiliar activity. A quieter, less stimulating environment can reduce misinterpretation and agitation. Hiding belongings may worsen suspicion, confrontation can escalate distress, and involving the roommate does not address the underlying confusion.

1880. A client with chronic pain calls out frequently and presses the call light every few minutes even after needs have been met. Which nursing strategy is most appropriate?

- A. Tell the client that using the call light too much is manipulative
- B. Respond only when the client stops calling out
- C. Develop a scheduled plan for contact and pain assessment
- D. Ask different staff members to answer each call for variety

Correct Answer: C

A scheduled plan provides structure, reduces anxiety about being forgotten, and can decrease attention-seeking behavior. Labeling the client as manipulative is nontherapeutic, withholding response is unsafe, and inconsistent staffing can increase insecurity and repeated calling.

1881. A nurse is caring for a client with mild cognitive impairment who becomes agitated whenever asked broad questions such as, "What would you like to do now?" Which communication change is most appropriate?

- A. Use short, concrete choices such as, "Would you like tea or water?"
- B. Speak more loudly so the client understands better

- C. Ask family members to answer all questions for the client
- D. Avoid giving choices so the client does not feel pressured

Correct Answer: A

Concrete, limited choices reduce overload and support behavioral control in clients with cognitive impairment. Speaking louder does not address the problem unless hearing loss is the issue, having family answer removes autonomy, and eliminating all choices is unnecessarily restrictive.

1882. A client hospitalized after a suicide attempt begins shouting when told that a personal belt cannot be kept in the room. The client says, "You people are treating me like a child." Which response by the nurse is best?

- A. Rules are rules, and you need to get over it
- B. I hear that you feel angry. The belt cannot stay in the room, but we can talk about other ways to help you feel more comfortable
- C. If you keep yelling, you will lose more privileges
- D. Your reaction proves the restriction is necessary

Correct Answer: B

The best response acknowledges the emotion, maintains the safety limit, and redirects toward problem solving. This is behavior management without power struggle. The other responses are dismissive or provocative and would likely increase anger and resistance.

1883. A client with antisocial behaviors laughs after taking another client's snack and says, "What are they going to do about it?" Which nursing response is most appropriate?

- A. Explain that the behavior is understandable because unit life is stressful
- B. Confront the client angrily in front of peers to discourage future incidents
- C. State the expected behavior clearly and identify the consequence according to unit policy
- D. Ignore the incident unless the other client complains

Correct Answer: C

Clear, consistent limit-setting with predictable consequences is appropriate for manipulative or rule-violating behavior. Excusing the behavior weakens boundaries, public angry confrontation can trigger power struggles, and ignoring the behavior reinforces it.

1884. A client with severe anxiety is increasingly restless and begins saying, "Don't touch me," while glancing toward the exit. The nurse is alone in the room. Which action is best?

- A.** Stand between the client and the doorway to prevent leaving
- B.** Move closer and place a hand on the client's shoulder for reassurance
- C.** Maintain personal space, keep a clear exit for both people, and use a calm voice
- D.** Tell the client that the behavior is inappropriate and must stop immediately

Correct Answer: C

When a client is highly anxious and showing possible escalation, the nurse should avoid crowding, avoid sudden touch, and maintain safety by keeping personal space and access to an exit. Blocking the doorway or touching the client can intensify fear, and a confrontational command is likely to worsen the situation.

1885. A client with a history of traumatic experiences becomes verbally aggressive when staff members enter the room without warning. Which nursing approach is most likely to reduce this behavior over time?

- A.** Approach unpredictably so the client becomes used to interruptions
- B.** Knock, introduce yourself, explain what you plan to do, and seek cooperation when possible
- C.** Limit explanations because they may encourage dependency
- D.** Require the client to accept all care immediately to establish control

Correct Answer: B

Predictability, explanation, and collaboration help reduce perceived threat and improve behavioral control, especially in clients who react strongly to sudden intrusion.

Unpredictable entry, withholding explanations, and rigid control can trigger more defensive or aggressive behavior.

1886. A client with schizophrenia has spent most of the morning shouting from the room. When the nurse enters, the client says, "The voices are telling me to hit someone." Which nursing action is the priority?

- A.** Ask the client what events earlier in the day may have caused the hallucinations
- B.** Instruct the client to ignore the voices and join a group activity
- C.** Determine whether the client has a plan or intent to act on the command and stay with the client
- D.** Explain that the voices are not real and the client is safe

Correct Answer: C

Command hallucinations that direct harm require immediate assessment of intent and risk, along with close supervision. Exploring triggers can happen after safety is addressed. Telling the client to ignore the voices or arguing about reality does not adequately assess danger and may be ineffective in the moment.

1887. An older adult with moderate dementia becomes agitated every evening, paces in the hallway, and tries to open exit doors. The client is not threatening staff but becomes more upset when several team members give directions at once. Which intervention is most appropriate?

- A.** Assign one staff member to speak calmly, use short simple directions, and guide the client to a quiet activity
- B.** Firmly tell the client that leaving is not allowed and return the client to the room each time
- C.** Ask several staff members to surround the client and explain why the behavior is unsafe
- D.** Keep the client in bed during the evening hours to prevent wandering

Correct Answer: A

A is correct because clients with dementia and sundowning-type agitation often respond best to a calm, simple, low-stimulation approach with consistent redirection. One speaker and brief directions reduce overload and can help manage pacing without escalating the behavior. B may increase agitation through confrontation. C adds stimulation and can make the client feel trapped. D is restrictive and inappropriate when less restrictive measures may be effective.

1888. A client with alcohol withdrawal is becoming increasingly agitated, paces at the bedside, and says, "I have to get out of here right now." The client is sweating and tremulous. Which nursing action is most appropriate for behavior management?

- A. Explain firmly that leaving is not allowed and ask the client to sit quietly
- B. Reduce environmental stimulation and remain with the client while speaking calmly in short, clear statements
- C. Ask several staff members to stand nearby so the client understands the seriousness of the situation
- D. Delay interaction until the client is more cooperative

Correct Answer: B

A client in withdrawal who is agitated benefits from a calm, low-stimulation approach and simple communication. This can reduce escalation while the nurse continues assessment and supportive care. A firm confrontation or showing force with multiple staff members can increase fear and agitation. Delaying interaction is unsafe because the client's condition may worsen.

1889. A client with borderline personality traits becomes angry after a request to stay in the day room during group time. The client yells, "You never make anyone else follow the rules!" and throws a magazine onto the floor. Which nursing response is best?

- A. "If you do that again, you will lose all privileges for the day."
- B. "You seem angry. The rule is that everyone stays in the day room during group time."

C. "Calm down and tell me why you are really upset."

D. "You are overreacting, and the other clients are handling this much better."

Correct Answer: B

The best response acknowledges the client's feeling while calmly restating the limit in a clear, nonpunitive way. This supports behavior management through consistency and avoids arguing. Choice A is more threatening than therapeutic. Choice C asks for insight while the client is escalated and does not clearly reinforce the limit. Choice D is judgmental and likely to intensify the behavior.

1890. During morning care in a long-term care setting, a client with moderate dementia swings a hand toward the nursing assistant when approached with a washcloth. The client has done this several times over the past week, especially during bathing. Which action should the nurse implement first to improve behavior management?

- A. Request a PRN sedative before all bathing care
- B. Tell the client that striking staff is unacceptable and continue the bath quickly
- C. Assess for triggers such as pain, fear, cold, or embarrassment during care
- D. Have two staff members hold the client's arms still while hygiene is completed

Correct Answer: C

When a client with dementia becomes combative during care, the nurse should first look for causes of the behavior, such as discomfort, fear, overstimulation, feeling exposed, or misunderstanding what is happening. Addressing triggers often reduces the behavior more effectively than forcing completion of the task. Sedation and physical holding are not first-line behavior management measures and may worsen distress. Simply telling the client not to hit does not address the cause.

1891. A client with schizophrenia begins pacing rapidly in the hallway, clenches both fists, and says loudly, "Everybody better stay away from me." Which intervention is the nurse's priority?

- A.** Move other clients away and attempt one-to-one de-escalation in a calm, nonthreatening manner
- B.** Ask the client to explain in detail what caused the anger
- C.** Tell the client that the behavior is inappropriate and must stop immediately
- D.** Leave the client alone to avoid increasing stimulation

Correct Answer: A

The priority is safety: protect others while using a calm, nonthreatening one-to-one approach to reduce escalation. The client's pacing, clenched fists, and verbal warning are cues for possible violence. Asking for detailed discussion is less appropriate during active escalation. A confrontational command may increase aggression. Leaving the client alone may delay intervention when risk is rising.

1892. A client admitted for mania has been awake most of the night, talks continuously, enters other clients' rooms, and becomes irritated when redirected. Which plan is most appropriate for managing this behavior over the next several hours?

- A.** Assign the client to lead a group activity to channel excess energy
- B.** Provide a quiet setting, give brief clear directions, and consistently redirect the client from intrusive behavior
- C.** Encourage the client to discuss all current ideas in depth to relieve pressure
- D.** Allow the client to walk through the unit freely as long as no one complains

Correct Answer: B

A manic client often has poor impulse control, distractibility, and overstimulation. A quiet environment, short concrete directions, and consistent limit-setting help manage intrusive behavior while reducing escalation. Leading group activity or encouraging extended discussion can increase stimulation. Allowing unrestricted wandering into others' rooms fails to set necessary behavioral limits and compromises safety and boundaries.

Therapeutic communication

1893. A client who was told this morning that a biopsy is positive for cancer says to the nurse, "I feel like my whole life just ended." Which response by the nurse is most therapeutic?

- A.** Everything will be fine once treatment starts.
- B.** Tell me more about what this news means to you.
- C.** You should focus on staying positive right now.
- D.** Many people live long lives after cancer treatment.

Correct Answer: B

Option B is therapeutic because it invites the client to express feelings and explain the personal meaning of the diagnosis. This uses open-ended communication and supports emotional processing. A, C, and D may be intended to reassure, but they minimize the client's distress or shift focus away from the client's current feelings.

1894. A hospitalized client says, "The staff here ignores me. No one cares what happens to me." Which nurse response is best?

- A.** That is not true. We have been checking on you regularly.
- B.** Why would you say something like that?
- C.** You seem to feel neglected. Tell me what has been happening.
- D.** I will report your complaint to the charge nurse immediately.

Correct Answer: C

Option C is best because it reflects the client's feeling and encourages the client to describe specific concerns. Reflection helps validate emotion without arguing. A is defensive, B can sound judgmental, and D may be appropriate later but skips the first step of therapeutic communication: exploring the client's perception.

1895. A client with chronic pain says, "I know people think I'm exaggerating." Which response by the nurse demonstrates therapeutic communication?

- A. Your pain medication is ordered as needed, so just ask for it.
- B. What makes you think others see it that way?
- C. You should not worry about what other people think.
- D. At least the tests did not show anything serious.

Correct Answer: B

Option B encourages the client to explore thoughts and feelings and gives the nurse more information about the concern. A addresses treatment but not the emotional statement. C dismisses the client's concern, and D minimizes the client's experience by comparing it to test results.

1896. A nurse is caring for a client who begins to cry quietly after a phone call. What is the most therapeutic initial action?

- A. Sit with the client and remain silently attentive for a moment.
- B. Tell the client that crying will help relieve stress.
- C. Leave the room to give the client privacy right away.
- D. Ask the client to describe the phone conversation in detail.

Correct Answer: A

Option A is therapeutic because use of silence and presence communicates support without pressuring the client. This often helps a distressed client feel safe enough to talk. B gives unsolicited interpretation, C may feel abandoning if done immediately, and D is too direct as an initial response when the client has just become tearful.

1897. A client with newly diagnosed diabetes says, "I'm scared I'll never be able to manage all of this." Which response by the nurse is best?

- A. You will get used to it after a few weeks.
- B. Lots of people handle diabetes successfully every day.
- C. It sounds overwhelming right now. What part worries you most?
- D. You need to learn the diet first, then the insulin injections.

Correct Answer: C

Option C first acknowledges the emotion and then uses a focused open-ended question to identify the client's main concern. That makes the response both supportive and useful for planning care. A and B offer false reassurance or generalized reassurance. D changes too quickly into teaching before addressing the fear.

1898. During a dressing change, a client says angrily, "You nurses never do anything on time." Which response is most therapeutic?

- A. We are very busy, and sometimes delays happen.
- B. You sound frustrated about your care today.
- C. That is unfair because I just got here.
- D. If you keep speaking that way, I will come back later.

Correct Answer: B

Option B is therapeutic because it reflects the client's emotion without becoming defensive. A explains the staff perspective instead of exploring the client's concern. C argues with the client, and D sets a limit in a punitive way without first attempting de-escalation or understanding.

1899. A nurse asks a client, "Do you feel upset about going home tomorrow?" This question is best described as which type of communication?

- A. Leading question
- B. Broad opening statement
- C. Offering self
- D. Restating

Correct Answer: A

Option A is correct because the wording suggests an expected feeling and may shape the client's answer. A broad opening statement would encourage free expression, such as "How do you feel about going home tomorrow?" Offering self means making oneself available, and restating repeats the main idea of the client's statement.

1900. A client says, "I haven't slept since my spouse died." Which nurse response is an example of restating?

- A. You have not been able to sleep since your spouse died.
- B. Why do you think grief is affecting your sleep?
- C. You should ask the provider for a sleep aid.
- D. Losing a spouse is hard for anyone.

Correct Answer: A

Option A restates the client's message in similar words, which shows listening and encourages further discussion. B is a why question that may make the client feel challenged. C moves too quickly to problem-solving. D is a general statement that may sound sympathetic but does not specifically facilitate exploration.

1901. A client says, "I don't want to talk about my test results." What is the nurse's best response?

- A. You need to talk about them so I can help you.
- B. All right. If you decide you want to talk later, I will be available.
- C. Your family probably wants you to discuss this openly.
- D. Avoiding the issue will only make you more anxious.

Correct Answer: B

Option B respects the client's right not to talk at that moment while still conveying availability and support. A is controlling, C introduces outside pressure, and D interprets the client's behavior in a potentially judgmental way. Therapeutic communication respects readiness and autonomy.

1902. A nurse is caring for a client who frequently asks, "Am I dying?" The client has had this conversation several times during the shift. Which response is most therapeutic?

- A. Why do you keep asking me that question?
- B. Try not to think about that right now.
- C. Tell me what you are thinking about when you ask that.
- D. No one knows exactly when death will occur.

Correct Answer: C

Option C explores the meaning behind the repeated question and encourages the client to express fears, concerns, or misunderstandings. A is defensive, B blocks communication, and D may be factually neutral but does not address the emotion or concern driving the repeated question.

1903. A nurse enters a room and finds a client staring at the wall. The client quietly says, "There isn't any point in trying anymore." Which response is the nurse's priority?

- A. Everyone feels discouraged sometimes.
- B. What has made things so difficult for you today?
- C. Are you thinking about harming yourself?
- D. You need to stay positive and focus on recovery.

Correct Answer: C

Option C is the priority because the client's statement may indicate hopelessness and possible self-harm risk. Directly assessing for suicidal thoughts is therapeutic and safe; it does not implant the idea. B explores feelings but delays essential safety assessment. A and D minimize the seriousness of the statement.

1904. A client with a history of alcohol misuse becomes restless and says, "I just need to get out of here." The client is pacing and speaking rapidly. Which nurse response is most therapeutic?

- A. If you do not sit down, I will call security.
- B. You need to stop pacing so we can talk.
- C. You seem very unsettled. Come with me to a quieter area so we can talk about what you need.
- D. There is no reason to act like this.

Correct Answer: C

Option C acknowledges the client's state, uses calm direction, and reduces stimulation while inviting communication. This is therapeutic and supports de-escalation. A may escalate the situation if used prematurely, B is controlling without addressing the distress, and D is judgmental and dismissive.

1905. A nurse is interviewing a client who pauses after every question and gives very short answers. Which statement by the nurse best uses facilitation?

- A. Can you explain why you are being so quiet?
- B. Go on.
- C. You do not need to be nervous.
- D. I think you are afraid to tell me something.

Correct Answer: B

Option B is facilitation. Brief cues such as "Go on" or "Tell me more" encourage the client to continue without directing the content. A can sound confrontational, C offers premature reassurance, and D makes an unsupported interpretation.

1906. A client says, "My daughter never visits. I must have been a terrible parent." Which response by the nurse is most therapeutic?

- A. I am sure your daughter has just been very busy.
- B. Why would you blame yourself for that?
- C. You are wondering whether her absence means you failed as a parent.
- D. Children often do not realize how much their parents need them.

Correct Answer: C

Option C reflects the underlying feeling and meaning in the client's statement, helping the client explore guilt and sadness. A offers reassurance without evidence, B uses a why question that may feel accusatory, and D generalizes rather than focusing on the client's emotions.

1907. A client who is scheduled for surgery tomorrow says to the nurse, "I'm not sure I should even be here." Which response by the nurse is most therapeutic?

- A. You shouldn't worry because this is a routine procedure.

B. Tell me more about what is making you unsure.

C. If you go home now, your condition will probably get worse.

D. Why did you wait until now to say this?

Correct Answer: B

Choice B is therapeutic because it uses an open-ended statement to encourage the client to express feelings and concerns. This helps the nurse assess whether the client is fearful, confused, ambivalent, or lacking information. Choice A gives false reassurance and shuts down discussion. Choice C is threatening and does not explore the client's feelings. Choice D is a "why" question, which can make the client feel defensive.

1908. A client who recently had a stroke struggles to find words and becomes frustrated during conversation. Which nursing response is most therapeutic?

- A. Finish the client's sentences to reduce embarrassment
- B. Ask only yes-or-no questions for the rest of the interaction
- C. Allow extra time for responses and maintain eye contact
- D. Speak loudly so the client can understand more easily

Correct Answer: C

Allowing extra time and maintaining attentive presence supports communication without rushing the client. Finishing sentences can increase frustration and reduce autonomy. Using only yes-or-no questions is overly limiting, and speaking loudly does not help expressive aphasia unless hearing loss is present.

1909. A client says, "I keep thinking this hospitalization is my fault." Which nurse response best uses the therapeutic technique of exploring?

- A. Why would you blame yourself for being sick?
- B. Tell me more about what makes you feel responsible.
- C. You should not think that way.
- D. Lots of people feel guilty when they are hospitalized.

Correct Answer: B

Exploring encourages the client to examine a feeling or statement in more depth. "Tell me more about what makes you feel responsible" invites further discussion without judgment. Asking "why" can sound defensive, advising the client blocks communication, and generalizing shifts focus away from the client's experience.

1910. A client with a new colostomy looks away and says, "Don't even show me that bag." Which response by the nurse is most therapeutic at this time?

- A. You need to learn this before discharge.
- B. Many clients get used to it within a few days.
- C. What concerns you most when you look at it?
- D. If you avoid it now, recovery will be harder later.

Correct Answer: C

The best response invites the client to express specific fears and feelings. This helps the nurse assess the emotional barrier before pushing teaching. The other responses minimize distress or focus on compliance instead of first addressing the client's reaction.

1911. The nurse is caring for a client who is sitting silently, twisting a tissue, and avoiding eye contact after being told discharge will be delayed. Which initial response is most therapeutic?

- A. Sit quietly with the client for a moment and remain attentive
- B. Explain the reasons for the delay in detail
- C. Tell the client that delays happen to many people
- D. Encourage the client to call a family member right away

Correct Answer: A

Therapeutic use of silence can help a distressed client gather thoughts and feel supported without pressure. Jumping into explanations or reassurance may interrupt emotional expression. Calling family may be helpful later, but the first step is to be present and observe whether the client wants to talk.

1912. A client says, "If my partner cared, they would be here already." Which nurse response is an example of reflecting?

- A. You sound hurt that your partner has not arrived.
- B. Have you considered that traffic may be heavy?
- C. Your partner probably cares more than you think.
- D. When did you start feeling this way about the relationship?

Correct Answer: A

Reflecting directs the feeling back to the client by naming the emotion and encouraging awareness of it. Choice A identifies the likely feeling of hurt. B and C offer interpretation or reassurance. D explores timing but is not the clearest example of reflecting.

1913. A client says, "The doctor used too many words. I still don't understand what is happening to me." Which response by the nurse is best?

- A. I'm sure the provider will explain it again later.
- B. What part would you like me to go over with you now?
- C. Try not to worry so much about the details.
- D. You should have asked more questions during the visit.

Correct Answer: B

This response assesses the client's specific knowledge gap and invites clarification in a supportive way. A delays help, C minimizes concern, and D places blame on the client rather than assisting communication and understanding.

1914. A client with terminal illness asks, "What do you think happens after death?" Which response by the nurse is most therapeutic?

- A. Most people find peace if they stay positive.
- B. I think everyone goes to the same place.
- C. What thoughts or beliefs have been most meaningful to you?
- D. Let's focus on your treatment plan for today.

Correct Answer: C

The nurse should explore the client's own beliefs rather than giving personal opinions or redirecting away from the topic. Choice C keeps the focus on the client and supports spiritual and emotional expression. A and B impose the nurse's perspective, and D prematurely changes the subject.

1915. A parent of a hospitalized child says, "I haven't left the bedside in two days. If I leave, something bad will happen." Which nurse response is most therapeutic?

- A. You need to rest, so go home for the night.
- B. Nothing bad is going to happen while you're gone.
- C. It sounds like you're feeling responsible for staying constantly alert.
- D. Parents often feel this way in the hospital.

Correct Answer: C

Choice C reflects the parent's underlying anxiety and sense of responsibility, encouraging further discussion. A gives advice too quickly, B offers false reassurance, and D generalizes the experience instead of addressing the individual's feelings.

1916. A client states, "I don't want my family to know how scared I am." Which nurse response best demonstrates offering self?

- A. I can stay with you while you talk about that.
- B. You should be honest with your family.
- C. Why don't you want them to know?
- D. They probably already know you are scared.

Correct Answer: A

Offering self communicates availability and support without pressure or judgment. The other options advise, probe with a potentially defensive "why," or make assumptions about the family's awareness.

1917. A client from a cultural background unfamiliar to the nurse declines eye contact during conversation. Which action by the nurse is best?

- A. Document that the client is withdrawn and resistant
- B. Ask the client whether there are communication preferences the nurse should respect

C. Insist on eye contact to show attention and honesty

D. Limit conversation until a family member can interpret the client's behavior

Correct Answer: B

Therapeutic communication includes avoiding assumptions and respectfully assessing the client's preferences. Lack of eye contact may reflect culture, anxiety, or another factor. Labeling the client as resistant, insisting on eye contact, or avoiding direct communication is not therapeutic.

1918. A nurse is interviewing a client who says, "I guess it doesn't matter what I do anymore," then shrugs and looks at the floor. What is the nurse's best response?

- A. Everyone feels discouraged sometimes.
- B. Can you tell me what you mean by that?
- C. You should try to think more positively.
- D. Let's talk about your medications instead.

Correct Answer: B

This statement may reflect hopelessness, so the nurse should clarify meaning and assess further rather than minimize or redirect. Choice B invites the client to explain. A and C dismiss the concern, and D misses important emotional cues.

1919. A nurse tells a client, "You seem quieter than usual today." Which therapeutic communication technique is the nurse using?

- A. Presenting reality
- B. Making an observation
- C. Giving recognition
- D. Focusing

Correct Answer: B

Making an observation means verbalizing what the nurse notices to encourage the client to elaborate. Presenting reality is used to address misperceptions, giving recognition acknowledges behavior without interpretation, and focusing narrows attention to a specific topic already raised.

1920. A client with hearing impairment is learning about a new treatment plan. Which nursing action best promotes therapeutic communication?

- A. Stand behind the client to reduce distractions

- B.** Speak rapidly to cover all information before the client tires
- C.** Face the client and use short, simple sentences
- D.** Repeat each statement in a louder voice if the client hesitates

Correct Answer: C

Facing the client helps with visual cues and lip reading, and short, simple sentences improve understanding. Standing behind the client removes visual support. Speaking rapidly reduces comprehension. Simply getting louder may distort speech and is less helpful than clear, direct communication.

1921. A client says, "I don't see the point of rehab. I'll never get better." Which nurse response is most therapeutic?

- A.** You must participate if you want to go home.
- B.** Why are you being so negative about therapy?
- C.** What has rehab been like for you so far?
- D.** The therapist says you have been improving.

Correct Answer: C

Choice C explores the client's perception and may uncover fear, fatigue, pain, or discouragement that affects participation. A is coercive, B is judgmental, and D may be true but does not first address the client's experience or emotion.

1922. During admission, a client says, "I don't trust hospitals." The nurse replies, "You don't trust hospitals." Which communication technique is this?

- A.** Restating
- B.** Informing
- C.** Interpreting
- D.** Summarizing

Correct Answer: A

Restating repeats the main idea of the client's message so the client can continue or clarify. It is not informing, because no new information is given; not interpreting, because the nurse does not assign meaning; and not summarizing, because it does not review several points of a longer discussion.

1923. A client newly diagnosed with multiple sclerosis says, "I haven't told my children yet. I don't even know what to say." Which response by the nurse is most therapeutic?

- A.** You should tell them as soon as possible so they can help you.
- B.** Everything will work out once your family understands the diagnosis.
- C.** What concerns you most about talking with your children about this?
- D.** Many parents in your situation wait a few weeks before saying anything.

Correct Answer: C

Choice C is correct because it uses an open-ended question to explore the client's feelings and concerns, helping the client identify the main barrier to communication. Choice A gives advice, which can shut down discussion. Choice B offers false reassurance. Choice D generalizes what others do instead of focusing on this client's needs.

1924. A client who is 1 day postoperative says angrily, "Nobody told me recovery would hurt this much. I knew this surgery was a mistake." Which nurse response is best?

- A.** You shouldn't say that until you've had more time to recover.
- B.** It sounds like the pain and the surprise of it are making this feel overwhelming right now.
- C.** The surgeon explained the risks before you signed the consent form.
- D.** Try to stay positive so you don't slow your healing.

Correct Answer: B

Choice B is correct because it acknowledges both the client's emotion and the likely source of distress, demonstrating empathy and validation. This helps the client feel heard and keeps communication open. Choice A is judgmental. Choice C is defensive and focuses on paperwork rather than the client's current distress. Choice D minimizes the client's experience and gives premature advice.

1925. The nurse is speaking with a client who recently lost a spouse. The client says, "People keep telling me to stay strong, but I don't even want to get out of bed." Which nurse response is most therapeutic?

- A. Your spouse would want you to keep going.
- B. At least you had many years together.
- C. Tell me what mornings have been like for you since your spouse died.
- D. You need to push yourself so this does not become a habit.

Correct Answer: C

Choice C is correct because it invites the client to describe the experience in specific terms, encouraging expression of grief without judgment. Choice A imposes meaning on the loss. Choice B minimizes the grief with an 'at least' statement. Choice D is directive and critical, which can increase guilt and shut down communication.

1926. A client who was informed this morning of a new cancer diagnosis says to the nurse, "Please don't tell me everything will be okay. I'm tired of hearing that." Which response by the nurse is most therapeutic?

- A. You should try to stay positive so treatment will go better.
- B. I understand exactly how you feel.
- C. This sounds overwhelming for you right now.
- D. Let's focus on the treatment schedule so you don't dwell on it.

Correct Answer: C

Choice C is correct because it acknowledges and validates the client's feelings without giving false reassurance or shifting away from the client's emotional experience. Therapeutic communication focuses on presence, listening, and helping the client express feelings. Choice A minimizes the client's distress by pushing positivity. Choice B is a common trap because saying "I understand exactly how you feel" assumes the nurse's experience matches the client's. Choice D changes the subject to tasks rather than addressing the emotion the client just expressed.

De-escalation

1927. A client in the emergency department begins pacing, shouting, and clenching fists after being told there will be a delay in treatment. Which nursing action is most appropriate first?

- A. Stand directly in front of the client and tell the client to stop yelling
- B. Move the client to a quieter area and speak in a calm, low voice
- C. Explain that disruptive behavior will lead to loss of privileges
- D. Ask several staff members to surround the client for support

Correct Answer: B

Moving the client to a quieter area and using a calm, low voice helps reduce environmental stimulation and supports de-escalation. Standing directly in front of the client, threatening consequences, or surrounding the client can increase the sense of confrontation and may worsen agitation.

1928. A hospitalized client with increasing agitation says, "No one here listens to me!" Which response by the nurse is therapeutic and most likely to de-escalate the situation?

- A. "You need to calm down before we can talk."
- B. "There is no reason to be upset about this."
- C. "It sounds like you're feeling ignored. Tell me what is bothering you most right now."
- D. "If you keep raising your voice, I will have to call security."

Correct Answer: C

This response acknowledges the client's feelings and invites the client to identify the main concern, which supports therapeutic communication and de-escalation. Telling the client to calm down, minimizing feelings, or threatening security can escalate distress and reduce trust.

1929. A nurse is caring for a client who is becoming verbally aggressive on a medical unit. Which environmental change is most appropriate to support de-escalation?

- A. Turn on the television to distract the client

- B.** Invite multiple staff members to explain the care plan
- C.** Reduce noise and limit nearby activity
- D.** Keep the client's room door closed and stand in the doorway

Correct Answer: C

Reducing noise and limiting nearby activity decreases stimulation, which can help calm an agitated client. More voices, more activity, and blocking the doorway can worsen agitation. Turning on a television adds stimulation rather than reducing it.

1930. A client who is upset about discharge plans begins speaking loudly and points a finger at the nurse. Which action by the nurse best promotes safety while de-escalating the interaction?

- A.** Maintain personal space and keep a clear path to the exit
- B.** Touch the client's arm to show support
- C.** Ask the client to sit down immediately
- D.** Move closer so the client does not need to shout

Correct Answer: A

Maintaining personal space and ensuring access to an exit promotes safety for both the client and nurse during escalation. Touch may be misinterpreted, commands may increase resistance, and moving closer can feel threatening.

1931. A client with schizophrenia says, "The staff are poisoning my food," and begins refusing meals. The client is tense but not physically aggressive. Which nursing response is best?

- A.** "That is not true. No one is poisoning you."
- B.** "You know that thought is part of your illness."
- C.** "I understand that this feels frightening to you. Let's talk about what would help you feel safer while you eat."
- D.** "If you do not eat now, a feeding tube may be necessary."

Correct Answer: C

The best response acknowledges the client's fear without validating the delusion and focuses on problem-solving and safety. Directly arguing about the belief often increases defensiveness. Labeling the thought as illness in the moment may feel dismissive, and threatening consequences is not de-escalating.

1932. A nurse enters a room and finds an agitated client breathing rapidly, pacing, and speaking over the nurse. Which action should the nurse take first to improve communication?

- A.** Provide several choices at once so the client feels in control
- B.** Ask the client to explain everything from the beginning in detail
- C.** Use short, simple statements and speak one idea at a time
- D.** Correct any inaccurate statements immediately

Correct Answer: C

Agitated clients often have decreased ability to process complex information. Short, simple statements reduce cognitive overload and support calming. Offering many choices, requesting long explanations, or correcting details right away may increase frustration.

1933. A client in a long-term care facility with dementia becomes agitated during bathing and starts swatting at staff. Which intervention is most appropriate?

- A.** Continue the bath quickly so the task is completed
- B.** Stop the bath, speak calmly, and try again later with a simpler approach
- C.** Tell the client that hitting is unacceptable and will not be tolerated
- D.** Restrain the client's arms during hygiene care

Correct Answer: B

In a client with dementia, agitation during care may reflect fear, confusion, overstimulation, or discomfort. Stopping the task, using a calm approach, and retrying later can reduce escalation. Forcing completion, scolding, or restraining the client can worsen distress and increase risk.

1934. A client on a behavioral health unit begins yelling at another client in the day room. Which nursing action is the priority?

- A.** Ask both clients to explain their concerns in front of the group
- B.** Immediately separate the clients and direct the agitated client to a less stimulating area
- C.** Remind the agitated client of unit rules from across the room
- D.** Wait to see whether the clients calm themselves down

Correct Answer: B

Separating the clients and decreasing stimulation is the priority because it reduces the immediate risk of violence. Group discussion during escalation, shouting reminders from across the room, or waiting can allow the situation to worsen.

1935. A client who has been upset for several minutes says, "Just get away from me." The client is breathing hard but has stopped yelling. What is the nurse's best response?

- A.** "No, we need to settle this right now."
- B.** "Fine, but you will have to apologize later."
- C.** "I will give you some space and check back shortly. If you need help sooner, let me know."
- D.** "You should not be alone when you feel like this."

Correct Answer: C

Respecting the client's need for space while maintaining availability supports de-escalation and preserves rapport. Forcing immediate discussion, attaching blame, or refusing appropriate space may re-intensify the situation. The client is still being monitored by the plan to check back.

1936. A nurse is attempting to calm a client who is increasingly suspicious and verbally hostile. Which nonverbal behavior by the nurse is most appropriate?

- A.** Cross the arms and remain silent until the client stops talking
- B.** Maintain a relaxed posture and keep facial expression neutral
- C.** Make prolonged direct eye contact to show confidence
- D.** Stand over the client to establish control

Correct Answer: B

A relaxed posture and neutral expression communicate calm and reduce the chance of appearing confrontational. Crossing arms, prolonged staring, or standing over the client can be perceived as threatening and may worsen hostility.

1937. A client with alcohol withdrawal is restless, irritable, and demanding to leave the unit immediately. The nurse notes tremors and increasing agitation. Which action is most important when using de-escalation techniques with this client?

- A.** Focus only on verbal calming and avoid further assessment until the client is cooperative
- B.** Recognize that agitation may have a physiologic cause and promptly address safety while notifying the provider
- C.** Tell the client that leaving is not allowed and end the conversation
- D.** Use firm confrontation to help the client regain control

Correct Answer: B

De-escalation is important, but in alcohol withdrawal agitation may reflect a serious physiologic problem that requires prompt assessment and treatment. The nurse should maintain safety and escalate care appropriately. Verbal techniques alone are not enough if the underlying cause is worsening withdrawal.

1938. A nurse is caring for a client who becomes agitated whenever staff enter the room unexpectedly. Which plan is best to help prevent escalation during future interactions?

- A.** Cluster all care even if it means waking the client abruptly
- B.** Approach quietly from behind to avoid startling the client with conversation
- C.** Announce yourself before entering, explain actions briefly, and use a consistent approach
- D.** Reduce contact with the client unless medication administration is due

Correct Answer: C

Preventive de-escalation includes consistent, predictable interactions and clear explanations before care. Approaching from behind can startle the client, abrupt waking can trigger agitation, and reducing contact may increase mistrust and unmet needs.

1939. A client with a history of trauma becomes visibly distressed when a staff member stands close and speaks loudly. The client starts shaking and backs into a corner. Which nursing action is most appropriate?

- A. Move closer to prevent the client from leaving the area
- B. Lower your voice, increase physical distance, and offer simple choices
- C. Ask the client to describe the past trauma in detail
- D. Place a hand on the client's shoulder to provide grounding

Correct Answer: B

A trauma-informed de-escalation approach reduces perceived threat by lowering voice, increasing personal space, and offering simple choices that restore some control. Moving closer, touching, or pressing for trauma details can intensify fear and worsen escalation.

1940. A nurse is de-escalating a client who is yelling, pacing, and intermittently striking the wall. Which finding indicates that the current approach is effective?

- A. The client begins speaking more slowly and unclenches the fists
- B. The client continues pacing but stops making eye contact
- C. The client argues that the nurse is being unfair
- D. The client asks whether security has been called

Correct Answer: A

Speaking more slowly and unclenching fists are signs of decreasing physiologic arousal and reduced immediate aggression, suggesting de-escalation is working. Continued suspiciousness, argument, or concern about security does not show the same clear reduction in agitation.

1941. During a medication pass on an inpatient unit, a client becomes angry after being told a requested PRN medication is not due yet. The client steps closer to the nurse and says, "You're useless. If you don't give it to me now, you'll be sorry." Which response by the nurse is best?

- A. "You need to lower your voice right now or I will call security."
- B. "I can see you're upset. Let's step to a quieter area so we can talk while I keep a safe distance."
- C. "That medication is scheduled, and arguing about it will not change the order."
- D. "If you threaten staff again, you will lose your privileges for the rest of the day."

Correct Answer: B

Choice B is best because it acknowledges the client's emotion, uses calm limit-setting, and promotes safety by moving to a less stimulating area while maintaining personal space. These are key de-escalation steps. Choice A is more confrontational and may intensify the situation. Choice C focuses on the rule rather than the client's escalating emotion. Choice D is punitive and likely to increase anger instead of calming the interaction.

1942. A client on a medical-surgical unit becomes increasingly upset after learning a family visit must be postponed. The client is speaking loudly, breathing quickly, and saying, "This is ridiculous!" Which nursing statement is most likely to de-escalate the situation?

- A. You need to calm down so we can talk about this.
- B. I can see you're upset that the visit changed. Tell me what is bothering you most right now.
- C. If you keep yelling, I will have to call security.
- D. There is nothing I can do, so you need to accept it.

Correct Answer: B

Choice B uses therapeutic communication by acknowledging the client's feeling and inviting the client to identify the main concern. This can reduce tension and help the nurse gather useful information. Choice A may sound dismissive, choice C is threatening and may increase agitation, and choice D shuts down communication.

1943. A client who is angry about a delay in receiving pain medication is standing in the hallway, shouting at staff, and glaring at nearby clients. Which action by the nurse best supports de-escalation while maintaining safety?

- A.** Ask the client to move with the nurse to a quieter area away from others
- B.** Explain the medication schedule in detail in front of the nurses' station
- C.** Tell the client that yelling is unacceptable and must stop immediately
- D.** Leave the client alone until the client is ready to speak calmly

Correct Answer: A

Choice A decreases stimulation, reduces the audience effect, and helps protect the privacy and safety of everyone involved. Choice B may prolong the conflict in a public space. Choice C sets a limit but is abrupt and not the best first de-escalation step here. Choice D may allow the agitation to worsen without assessment or support.

1944. A client on an inpatient unit says angrily, "You people never do anything right," and throws a blanket onto the floor. The client is standing several feet away from the nurse and is not approaching. Which nursing action is best?

- A.** Tell the client to pick up the blanket immediately
- B.** Respond, "You seem very frustrated. Let's talk about what happened."
- C.** Ignore the comment so the behavior is not reinforced
- D.** Argue the facts of the situation to correct the client's statement

Correct Answer: B

Choice B acknowledges the emotion and invites discussion without becoming defensive. That approach often reduces emotional intensity and opens communication. Choice A focuses on control rather than calming. Choice C may make the client feel dismissed. Choice D is likely to increase conflict because arguing facts rarely helps an escalated client regain control.

1945. A client with mild cognitive impairment becomes agitated during the evening and says, "I need to go home right now." Which nursing response is most appropriate for de-escalation?

- A.** You cannot leave, so stop asking.
- B.** Your family already told us you have to stay here.
- C.** You want to be somewhere familiar. Let's sit down and talk while I stay with you.
- D.** If you try to leave, I will notify the provider immediately.

Correct Answer: C

Choice C validates the client's underlying feeling and offers calm presence, which is useful in de-escalation, especially when cognition is impaired. Choice A is blunt, choice B may increase distress, and choice D is threatening rather than calming.

1946. A client becomes irritated when the nurse begins asking several admission questions in rapid succession. The client crosses the arms, raises the voice, and says, "Stop firing questions at me." Which action should the nurse take next?

- A.** Continue the questions to finish the assessment quickly
- B.** Pause, lower the voice, and ask one simple question at a time
- C.** Tell the client that the questions are required and cannot wait
- D.** Ask another staff member to repeat the same questions

Correct Answer: B

Choice B reduces stimulation and gives the client a more manageable interaction, which is consistent with de-escalation. Choice A increases pressure. Choice C may sound rigid and dismissive. Choice D does not address the client's distress and may worsen frustration.

1947. While discussing a missed smoking break, a frustrated client says to the nurse, "You better watch yourself." The client is tense but remains seated. Which nursing response is best?

- A. Are you threatening me? Because that will not be tolerated.
- B. I want to help, but I need us to speak without threats. Tell me what you need right now.
- C. If you say that again, you will lose privileges.
- D. I understand, and I will arrange a smoking break now.

Correct Answer: B

Choice B combines limit-setting with a calm, nonconfrontational tone and redirects the client toward expressing needs safely. Choice A can sound challenging. Choice C is punitive and may escalate the situation. Choice D rewards threatening behavior and may be unsafe or unrealistic.

1948. A client who is increasingly agitated keeps interrupting and shouting over the nurse. Which communication technique is most appropriate?

- A. Use short, clear statements and allow time for the client to respond
- B. Provide detailed teaching to address every concern at once
- C. Speak louder than the client to regain control of the interaction
- D. Ask several closed-ended questions in a row

Correct Answer: A

Choice A is effective because an agitated client may struggle to process long or complex messages. Short, calm statements reduce cognitive overload. Choice B can overwhelm the client. Choice C adds intensity. Choice D can feel pressuring and may increase agitation.

1949. A nurse is caring for a client with mania who is becoming louder, more irritable, and increasingly distractible during a busy shift change. Which intervention is most appropriate to help de-escalate the client?

- A. Invite the client to join a group activity to release energy
- B. Move the client to a low-stimulation setting and speak briefly and calmly
- C. Confront the client about inappropriate behavior in front of peers
- D. Encourage the client to explain every complaint in detail immediately

Correct Answer: B

Choice B is best because clients with mania may escalate further in busy, stimulating settings. Reducing external stimuli and using calm, simple communication can lower agitation. Choice A may add stimulation. Choice C is likely to provoke more agitation. Choice D may be too demanding when attention and impulse control are impaired.

1950. A client becomes upset after being told a diagnostic test has been delayed. The client says, "You people don't care if I suffer." Which response by the nurse demonstrates the most effective de-escalation approach?

- A. That is not true; we have been helping you all day.
- B. The department is busy, and everyone has to wait sometimes.
- C. It sounds like this delay feels very unfair to you. Let's talk about what you need most while you wait.
- D. You are overreacting to something minor.

Correct Answer: C

Choice C reflects the client's feeling without arguing and shifts toward problem solving. That combination supports de-escalation. Choice A is defensive, choice B minimizes the client's experience, and choice D is dismissive and likely to intensify anger.

1951. A client on a rehabilitation unit has become increasingly irritable over the past hour. Which early sign should alert the nurse that de-escalation may be needed before the behavior progresses?

- A. The client begins speaking more loudly and giving short, clipped answers
- B. The client asks for a second blanket before resting
- C. The client smiles briefly when greeted by staff
- D. The client requests to speak with the provider about discharge planning

Correct Answer: A

Choice A suggests rising agitation and is an early cue that intervention may be needed. Recognizing escalating behavior early allows the nurse to act before the situation becomes more dangerous. The other options do not specifically suggest escalating emotional arousal.

1952. A nurse is speaking with an upset client who suddenly starts scanning the room, breathing faster, and shifting weight from foot to foot. What is the nurse's priority interpretation of these findings?

- A. The client is showing signs of decreasing anxiety
- B. The client may be moving toward physical escalation and needs immediate safety-focused de-escalation
- C. The client is ready to learn coping strategies in detail
- D. The client is trying to avoid answering questions but is otherwise calm

Correct Answer: B

Choice B is correct because scanning the environment, increased breathing, and restless shifting can signal mounting agitation and possible progression toward aggression. The nurse should focus on safety and de-escalation immediately. The other choices underestimate the significance of these behavioral cues.

1953. A client with paranoid thinking says, "I know the staff are talking about me outside my door." The client appears suspicious but remains seated and attentive. Which nursing response is best for de-escalation?

- A. No one is talking about you, and you need to stop imagining things.

B. Why would the staff single you out when there are many other clients here?

C. That sounds upsetting. I do not hear staff talking about you right now, but I want to understand what is worrying you.

D. You are wrong, and the provider will explain your illness again later.

Correct Answer: C

Choice C acknowledges the distress without validating the suspicious belief as fact. It also orients gently to the present and invites further discussion. Choice A is dismissive, choice B is argumentative, and choice D is confrontational and unlikely to reduce suspicion.

1954. A client who has been angry during a conversation suddenly becomes very quiet, stares at the nurse, and moves one hand behind the back while stepping toward the doorway. Which action should the nurse take first?

- A. Move closer to the client to show support
- B. Ask the client to sit down immediately in a firm voice
- C. Increase distance, maintain a clear exit path, and call for assistance
- D. Turn away briefly to lower the client's sense of being watched

Correct Answer: C

Choice C is best because the change in behavior may signal escalating risk. The nurse should prioritize personal and unit safety by maintaining space, avoiding entrapment, and obtaining help. Choice A is unsafe. Choice B may intensify the situation if delivered before safety is secured. Choice D reduces the nurse's awareness of potential danger.

1955. A client with traumatic brain injury becomes frustrated during therapy and starts cursing at staff. The client has difficulty processing long explanations. Which de-escalation strategy is most appropriate?

- A. Correct each inappropriate statement immediately so the behavior does not continue
- B. Give one brief direction at a time and allow extra time for processing
- C. Ask the client to describe the entire event from the beginning

D. Present several choices quickly so the client can regain control

Correct Answer: B

Choice B is most appropriate because cognitive processing deficits can worsen frustration when information is too complex or rapid. Short, simple directions with time to respond reduce overload and support de-escalation. Choice A is likely to increase frustration. Choice C may be too demanding. Choice D can overwhelm the client.

1956. A nurse is attempting to de-escalate a client who becomes angry whenever care is delayed. Which nurse statement best balances empathy, limit-setting, and collaboration?

- A.** You will have to wait like everyone else, so there is no reason to be upset.
- B.** I can see the delay is frustrating. I want to work with you, and I need you to keep your voice low while we decide what can be done next.
- C.** If you continue to complain, I will end this conversation now.
- D.** I promise this will not happen again if you calm down.

Correct Answer: B

Choice B is the strongest answer because it validates the client's feeling, sets a clear behavioral expectation, and offers collaboration on next steps. That combination is central to effective de-escalation. Choice A minimizes emotion, choice C is more punitive than therapeutic, and choice D makes a promise the nurse may not be able to keep.

1957. During morning medication pass, a client paces near the nurses' station, speaks in a louder-than-usual voice, and repeatedly says, "This place is too much." The client is not threatening anyone and follows directions when spoken to. Which nursing action is best to de-escalate the situation early?

- A.** Move the client to a quieter area and speak in short, calm sentences
- B.** Explain unit rules in detail so the client understands expectations
- C.** Ask several questions to identify every cause of the client's frustration
- D.** Tell the client to sit down until the nurse has time to talk

Correct Answer: A

Early de-escalation works best by lowering stimulation and using calm, simple communication. Moving the client to a quieter area can reduce triggers, and short statements are easier to process when a person is becoming agitated. Choice B may be too detailed for the moment, choice C can feel overwhelming, and choice D is likely to sound controlling and may increase agitation.

1958. A client who has been arguing with staff suddenly clenches the jaw, balls the fists, and steps closer to the nurse while saying, "Don't push me." Which action should the nurse take first?

- A.** Place a hand on the client's shoulder to provide reassurance
- B.** Maintain personal space and position near an exit while using a calm voice
- C.** Turn away to reduce eye contact and continue documenting the event
- D.** Challenge the statement by asking what the client means by "push me"

Correct Answer: B

The client is showing escalating signs of possible violence, so the nurse's first priority is safety. Maintaining distance, staying near an exit, and speaking calmly support de-escalation while preserving an escape route. Touching the client can worsen agitation, turning away reduces awareness of risk, and challenging the statement may escalate the confrontation instead of calming it.

1959. A hospitalized client angrily says, "If my dinner is late again, somebody is going to pay for it." The client is breathing rapidly and staring at the tray table. Which nurse response is most appropriate?

- A.** "You need to calm down, or I will call security."
- B.** "You sound very frustrated. I want to help, and I need you to keep your hands to yourself while we talk."
- C.** "There is no reason to act like this over one late meal."
- D.** "Why does a delayed tray upset you this much?"

Correct Answer: B

This response combines key de-escalation elements: acknowledging the client's emotion, offering help, and setting a clear behavioral limit. That balance often reduces escalation more effectively than threats or criticism. Choice A may provoke defensiveness, choice C minimizes the client's feelings, and choice D asks for analysis when the client is already too activated for productive problem-solving.

1960. A nurse enters a room and finds a client agitated, yelling, and sweeping supplies off the bedside table. Another staff member is on the way. Which action by the nurse is the priority for de-escalation and safety?

- A. Quickly begin cleaning the room to reduce environmental stressors
- B. Stand directly in front of the doorway so the client cannot leave
- C. Lower the voice, keep a safe distance, and remove nearby objects that could be used as weapons if this can be done safely
- D. Tell the client there will be consequences for damaging hospital property

Correct Answer: C

When agitation has escalated to throwing objects, the nurse must focus on immediate safety while still attempting de-escalation. A calm tone, safe distance, and reduction of potential weapons are appropriate priorities. Choice A misplaces priority, choice B can make the client feel trapped and increase violence risk, and choice D is confrontational and unlikely to calm the situation.

Cognitive impairment

1961. An older adult with moderate dementia becomes upset every evening and repeatedly says, "I have to go pick up my children from school." Which response by the practical nurse is most therapeutic?

- A. Your children are grown, so there is no school pickup anymore.
- B. Tell me about your children while we walk to the day room together.
- C. You are confused again, and you need to stay in your room.
- D. If you stop asking, I will bring you a snack.

Correct Answer: B

Choice B is best because it validates the client's feelings, redirects attention, and offers a calming activity without arguing. Clients with dementia often respond poorly to confrontation about incorrect beliefs. Choice A is reality-based but may increase distress. Choice C is dismissive and nontherapeutic. Choice D uses bargaining rather than therapeutic communication.

1962. A client with delirium is admitted after a urinary tract infection. Which finding is most characteristic of delirium rather than dementia?

- A. Gradual decline in memory over several years
- B. Stable confusion that is unchanged throughout the day
- C. Sudden change in attention and awareness
- D. Progressive difficulty performing familiar tasks

Correct Answer: C

Choice C is correct because delirium typically has an acute onset with fluctuating attention and awareness. Dementia usually develops gradually and progressively over time. Choices A and D describe dementia more than delirium. Choice B is less consistent with delirium because delirium often fluctuates.

1963. A client with cognitive impairment becomes frightened by shadows in the room at night and tries to climb out of bed. Which nursing action is most appropriate first?

- A. Turn on a night-light and speak calmly to reorient the client
- B. Apply wrist restraints to prevent a fall
- C. Move the client to a dark room to encourage sleep
- D. Leave the room so the client can settle down alone

Correct Answer: A

Choice A is best because improved lighting can reduce misinterpretation of visual stimuli, and calm reorientation may decrease fear. Restraints are not the first response and can worsen agitation. A darker room may increase misperceptions. Leaving the client alone increases fall risk and does not address the fear.

1964. The practical nurse is caring for a client with mild dementia who asks the same question every few minutes. Which approach is most appropriate?

- A. Ignore the repeated question to avoid reinforcing the behavior
- B. Answer briefly in the same calm manner each time
- C. Tell the client to write down the answer and stop asking
- D. Quiz the client about the earlier answer to improve memory

Correct Answer: B

Choice B is correct because repeated questioning is common in dementia, and a calm, consistent response helps reduce anxiety. Ignoring the client may increase distress. Telling the client to stop asking is not therapeutic. Quizzing the client can be frustrating and may worsen agitation.

1965. A client with moderate Alzheimer disease becomes agitated during morning care and strikes out when staff approach quickly. Which action by the practical nurse is most likely to reduce this behavior?

- A. Complete care as fast as possible before agitation worsens
- B. Approach from the front, explain each step, and provide one instruction at a time
- C. Correct the client firmly each time hitting occurs
- D. Ask several staff members to stand nearby during all care

Correct Answer: B

Choice B is best because a slow approach, visible presence, simple explanations, and one-step directions can decrease fear and overstimulation in clients with dementia. Rushing care often worsens agitation. Firm correction does not address the cause. Multiple staff nearby may feel threatening and increase striking out.

1966. The practical nurse is planning the environment for a client with cognitive impairment who wanders. Which intervention is most appropriate?

- A. Keep the client in bed except for meals

- B. Provide a safe walking path and observe for patterns in wandering
- C. Lock the client alone in the room during busy times
- D. Place unfamiliar furniture in the hallway to block movement

Correct Answer: B

Choice B is correct because wandering may reflect restlessness, unmet needs, or routine behavior. A safe walking path and assessment of timing or triggers help reduce risk while preserving mobility. Bed restriction, isolation, and physical barriers can increase injury and agitation.

1967. A hospitalized older adult with previously mild memory problems suddenly becomes disoriented, pulls at the IV tubing, and cannot focus on the nurse's questions. Which action should the practical nurse take first?

- A. Document progression of dementia and recheck in 24 hours
- B. Reduce stimulation and report the acute mental-status change promptly
- C. Ask family to bring familiar photos before doing anything else
- D. Teach the client memory exercises to improve concentration

Correct Answer: B

Choice B is best because a sudden inability to focus with acute disorientation suggests delirium, which requires prompt reporting and supportive measures to promote safety. Choice A wrongly assumes a chronic process. Family photos may help orientation later but do not address the acute change first. Memory exercises are not appropriate in sudden delirium.

1968. The daughter of a client with dementia says, "Every time I visit, he thinks I am his sister. I keep correcting him, but he gets upset." Which response by the practical nurse is best?

- A. Keep correcting him so he does not lose more memory.
- B. It may help to introduce yourself each visit and focus on a calm interaction instead of correcting him repeatedly.

C. Stop visiting for a while because the confusion is making him anxious.

D. Ask him detailed questions each visit to test what he remembers.

Correct Answer: B

Choice B is correct because repeated correction can increase frustration and does not usually improve memory in dementia. A simple introduction and calm interaction support the relationship while reducing distress. Choice A may worsen agitation. Choice C is unnecessary and isolating. Choice D turns the visit into a test, which is often counterproductive.

1969. A client with cognitive impairment is eating poorly because meals in the dining room are noisy and distracting. Which nursing intervention is most appropriate?

A. Offer the meal in a quieter area with simple cues to begin eating

B. Wait until the client becomes hungry enough to ignore the noise

C. Give all foods mixed together in one bowl to save time

D. Ask other clients to remind the client to keep eating

Correct Answer: A

Choice A is best because reducing environmental stimulation and using simple cues can improve attention to the meal. Waiting may worsen nutritional intake. Mixing all foods together can be unappealing and confusing. Depending on other clients is unreliable and may embarrass the client.

1970. The practical nurse enters a room and finds a client with dementia searching under the bed and saying, "Someone stole my purse." What is the most therapeutic response?

A. No one stole it. You must have forgotten where you put it.

B. That sounds upsetting. Let's look for your purse together.

C. You are accusing the staff, and that is unacceptable.

D. If you calm down, I will tell you whether it was stolen.

Correct Answer: B

Choice B is correct because it acknowledges the client's distress and joins the client in problem-solving without confrontation. Choice A may feel dismissive and can escalate anxiety. Choice C is defensive and nontherapeutic. Choice D is manipulative and does not address the immediate fear.

1971. A client with moderate dementia usually becomes restless and tries to leave the unit around 4 PM each day. Which nursing action best uses this pattern to improve care?

A. Schedule a supervised walk and quiet activity shortly before 4 PM

B. Begin all painful procedures at 4 PM so the client will stay occupied

C. Keep the client awake from naps to increase nighttime sleep only

D. Move the client to a different room every afternoon

Correct Answer: A

Choice A is best because recognizing a predictable pattern allows the nurse to intervene before escalation with structured activity and supervision. Painful procedures at that time could worsen agitation. Preventing naps may or may not help and does not directly address the daily restlessness pattern. Frequent room changes increase confusion.

1972. An older adult with severe dementia is admitted to a long-term care facility. Which room assignment would best support the client's cognitive function and safety?

A. A room near the nurses' station with a consistent layout and clearly visible clock and calendar

B. A room at the end of a busy hallway with loud television playing most of the day

C. A room changed weekly so the client does not become too dependent on routine

D. A private room kept dark to reduce all stimulation at all times

Correct Answer: A

Choice A is correct because a consistent environment, frequent observation, and orientation cues such as a clock and calendar can support safety and reduce confusion. Busy noise, frequent room changes, and prolonged darkness are more likely to worsen disorientation or agitation.

1973. During evening care, a client with cognitive impairment becomes increasingly agitated when asked open-ended questions such as, "What would you like to do next?" Which communication change is most appropriate?

- A.** Ask more detailed questions so the client can think through the options
- B.** Use short, simple statements and offer one choice at a time
- C.** Speak louder to make the questions easier to understand
- D.** Avoid all communication until the client settles down independently

Correct Answer: B

Choice B is best because clients with cognitive impairment may become overwhelmed by complex language or multiple choices. One simple step or limited choice supports comprehension and reduces frustration. More detailed questions increase cognitive demand. Speaking louder does not solve a processing problem unless hearing loss is the issue. Avoiding communication leaves the client unsupported.

1974. A client with dementia refuses to bathe and says, "You are trying to hurt me." Which action by the practical nurse is best?

- A.** Insist on the bath now because hygiene is required every morning
- B.** Step back, speak calmly, and try again later with a familiar routine
- C.** Tell the client that refusal will be reported to the provider
- D.** Have two staff members hold the client still for the bath

Correct Answer: B

Choice B is correct because refusal may be related to fear, misunderstanding, overstimulation, or loss of control. Backing off, using a calm approach, and retrying later within a familiar routine can reduce distress and improve cooperation. Forcing care, threatening, or physically holding the client is likely to escalate fear and aggression unless there is an immediate emergency.

1975. The practical nurse notes that a client with chronic dementia is usually pleasant in the morning but becomes acutely confused, sleepy, and difficult to arouse after lunch. Which interpretation is most important?

- A.** This is an expected daily progression of dementia and needs no follow-up.
- B.** The change may indicate a new problem, so the nurse should assess and report it promptly.
- C.** The client is probably pretending to avoid afternoon activities.
- D.** This behavior shows the client no longer benefits from orientation efforts.

Correct Answer: B

Choice B is best because a sudden change from the client's baseline level of function suggests an acute condition superimposed on chronic dementia, not simply expected progression. Dementia usually declines gradually. Assuming the client is pretending is unsafe. Orientation strategies may still be useful, but the immediate issue is the abrupt mental-status change.

1976. A client with chronic dementia is usually able to recognize staff and feed himself. Today he is suddenly unable to follow simple directions, his speech is rambling, and he is trying to get out of bed to "catch the bus." Which additional finding would most strongly support delirium as the cause of this change?

- A.** Symptoms have progressed gradually over several years
- B.** Level of alertness shifts from drowsy to restless خلال the day
- C.** Long-standing difficulty learning new information
- D.** Family reports a steady decline in judgment over months

Correct Answer: B

Delirium commonly has an acute onset with fluctuating consciousness and attention, so changing from drowsy to restless during the day strongly supports delirium. The other options describe the more gradual, chronic pattern expected with dementia.

1977. The practical nurse is caring for a client with moderate dementia who becomes upset during dressing and says, "I don't know what to do." Which nursing action is best?

- A.** Lay out two complete outfits and ask the client to choose one independently
- B.** Dress the client quickly without explanation to avoid frustration
- C.** Give one simple step at a time while offering enough time to respond
- D.** Explain the full sequence of dressing before beginning care

Correct Answer: C

Clients with dementia often do best with short, simple directions and extra time to process them. Option A may still be too complex, B reduces independence and may increase fear, and D gives too much information at once.

1978. A resident with severe cognitive impairment paces the hallway for much of the afternoon but is calm while walking with staff. Which intervention is most appropriate?

- A.** Encourage supervised walking periods at regular times
- B.** Keep the resident in bed after lunch to conserve energy
- C.** Move the resident to a dark room when pacing begins
- D.** Tell the resident to sit down each time pacing starts

Correct Answer: A

Providing safe, supervised walking channels the behavior into an appropriate activity and may reduce agitation. Bed rest, darkness, and repeated commands to stop are less therapeutic and may worsen restlessness.

1979. A client with mild cognitive impairment is embarrassed by forgetfulness and asks the practical nurse, "What can I do so I don't miss my medications at home?" Which suggestion is best?

- A.** Stop taking the medications until a family member can move in
- B.** Use a pill organizer and keep a consistent daily routine
- C.** Hide the medications in different places to prevent theft
- D.** Take all medications at bedtime to simplify the schedule

Correct Answer: B

A pill organizer plus a consistent routine supports memory and independence. Stopping medications is unsafe, hiding medications increases confusion, and changing all doses to bedtime is inappropriate unless prescribed.

1980. The spouse of a client with early dementia says, "He gets angry when I finish his sentences, but if I wait, it takes forever." Which response by the practical nurse is best?

- A.** Try to answer for him whenever he seems stuck so he does not struggle
- B.** Ask several questions quickly so he stays focused on the conversation
- C.** Allow extra time for him to respond before offering gentle assistance
- D.** Correct every wrong word right away so he does not form bad habits

Correct Answer: C

Allowing time preserves dignity and supports communication. Gentle cueing can be added only if needed. Answering for the client, rushing, or frequent correction can increase frustration and reduce participation.

1981. A hospitalized older adult with dementia is more confused than usual overnight and sleeps poorly because of repeated noise and room changes. Which nursing intervention is most likely to support orientation?

- A.** Maintain a predictable routine with familiar objects and visible clock and calendar
- B.** Limit all daytime activity so the client will sleep more at night
- C.** Rotate several staff members through the room so the client meets everyone

D. Avoid mentioning the date or location because it may upset the client

Correct Answer: A

Consistency, familiar items, and orientation cues help support cognition and reduce confusion. Daytime inactivity may worsen sleep-wake disruption, rotating many staff can be disorienting, and avoiding orientation removes helpful cues.

1982. A client with dementia begins shouting and pushing away the lunch tray. The nurse notices the client is squinting, turning the head away from bright sunlight, and grimacing. What is the best interpretation?

- A.** The client is manipulative and should be told to stop
- B.** The behavior may reflect an unmet need or environmental discomfort
- C.** The client no longer needs assistance at mealtime
- D.** The client is purposely refusing food to gain attention

Correct Answer: B

Behavior in cognitively impaired clients often communicates distress, such as pain, glare, fear, or overstimulation. The observed cues suggest discomfort rather than intentional misbehavior. Labeling the client as manipulative can delay appropriate assessment.

1983. The practical nurse is preparing to transfer a client with moderate dementia from bed to chair. Which approach is most likely to improve cooperation?

- A.** Explain the transfer while standing behind the client to avoid distraction
- B.** Approach from the front, use the client's name, and demonstrate the action
- C.** Ask the client to decide each step of the transfer independently
- D.** Complete the transfer silently so the client is not overwhelmed

Correct Answer: B

Approaching from the front, using the client's name, and demonstrating a simple action improves attention and reduces startle. Standing behind the client can be frightening, too many decisions increase confusion, and silence may make the client feel unsafe.

1984. A long-term care resident with cognitive impairment has started waking at night and trying to open exit doors. Which daytime plan is most appropriate to help reduce this pattern?

- A.** Encourage daytime activity and limit long daytime naps
- B.** Keep the resident in the room for most of the day to prevent fatigue
- C.** Offer caffeinated drinks with dinner to improve evening alertness
- D.** Delay all toileting until bedtime to decrease nighttime awakenings

Correct Answer: A

Promoting daytime activity and reducing long naps can help support a more normal sleep-wake cycle and reduce nighttime wandering. Isolation, caffeine, and delaying toileting are likely to worsen the problem.

1985. A client with cognitive impairment becomes anxious when taken to different areas for routine care. Which nursing strategy is best?

- A.** Perform care in the same setting whenever possible
- B.** Change the care location often so the client adjusts faster
- C.** Use a louder voice in unfamiliar areas to gain attention
- D.** Postpone all care until the client asks for help

Correct Answer: A

Consistency in surroundings can reduce anxiety and confusion in clients with cognitive impairment. Frequent changes in location, louder speech, and waiting for the client to request all care are less effective and may increase distress.

1986. A client with moderate dementia says to the practical nurse, "I need to get to work right now or I'll be fired." The client appears worried and is walking toward the door. Which response is best?

- A.** You retired years ago, so there is no reason to leave
- B.** If you stop saying that, I can help you with lunch
- C.** You sound worried about being late. Let's sit down and look at your schedule together

D. Why do you always think you have to go to work?

Correct Answer: C

This response validates the client's emotion and redirects to a calming, structured activity. Arguing with the belief, bargaining, or asking a why-question is more likely to increase defensiveness or confusion.

1987. A resident with advanced dementia no longer uses utensils well and eats very little at meals. Which intervention is most appropriate to promote nutrition?

- A.** Offer finger foods that are easy to pick up
- B.** Insist that the resident use utensils to maintain normal habits
- C.** Wait until the resident asks for food between meals
- D.** Serve all foods mixed together in one bowl to simplify choices

Correct Answer: A

Finger foods can support independence and intake when utensil use declines. Forcing utensil use may increase frustration, waiting for the resident to ask may not work, and mixing all foods together can reduce appeal and dignity.

1988. A client with dementia becomes combative only during perineal care. Which action by the practical nurse is best to try first?

- A.** Request an as-needed sedative before all hygiene care
- B.** Stop all bathing for several days to avoid upsetting the client
- C.** Assess for pain, temperature discomfort, and modesty concerns during care
- D.** Tell the client that combative behavior will not be tolerated

Correct Answer: C

When agitation occurs during a specific task, the nurse should first assess likely triggers such as pain, fear, feeling cold, or embarrassment. Sedation or threats do not address the cause, and avoiding hygiene entirely is not appropriate.

1989. An older adult in a rehabilitation unit had mild forgetfulness at baseline. After starting a new opioid for postoperative pain, the client becomes inattentive, sees insects on the wall, and cannot follow conversation. Which action by the practical nurse is the priority?

- A.** Document that worsening dementia is expected after surgery
- B.** Reduce stimuli and promptly report the acute change in mental status
- C.** Reorient the client once, then allow uninterrupted sleep for the rest of the shift
- D.** Ask the family to bring favorite photographs from home tomorrow

Correct Answer: B

This is an acute mental-status change with perceptual disturbance and inattention, suggesting possible delirium that requires prompt reporting and immediate supportive measures such as reducing stimuli. The change should not be dismissed as expected dementia progression. Family photos may help orientation later but are not the priority.

1990. The practical nurse is reinforcing teaching for a family caring for a client with progressing dementia at home. Which statement by the family shows correct understanding of safety planning?

- A.** We will place medications and cleaning supplies where they are easy to see
- B.** We will use simple door alarms and keep frequently used pathways uncluttered
- C.** We will rearrange the furniture often so the client stays mentally challenged
- D.** We will rely on verbal reminders alone instead of making changes to the home

Correct Answer: B

Simple safety supports such as door alarms and clear walking paths help reduce risk while respecting function. Visible hazardous items increase danger, frequent furniture changes increase confusion, and verbal reminders alone are often insufficient as cognition declines.

1991. An older adult admitted from home is alert but suddenly cannot maintain attention during conversation. The client misidentifies the IV pole as a person, becomes more agitated in the evening, and had been functioning independently 2 days ago. Which interpretation is most appropriate?

- A.** These findings are most consistent with delirium
- B.** These findings are expected with normal aging
- C.** These findings indicate gradual progression of chronic dementia
- D.** These findings show intentional uncooperative behavior

Correct Answer: A

Delirium typically has an acute onset with fluctuating mental status, poor attention, perceptual disturbances, and worsening at certain times such as evening. A major clue is the rapid change from recent independent functioning. Dementia usually progresses gradually rather than over 2 days. Normal aging does not cause hallucination-like misidentification and marked inattention.

1992. A client with moderate cognitive impairment is restless during a bath and repeatedly asks, "What's happening now?" Which action by the practical nurse is best?

- A.** Finish the bath quickly without further explanation
- B.** Ask the client to remember the bathing routine from home
- C.** Give short, simple explanations before each step
- D.** Correct the client each time the question is repeated

Correct Answer: C

Short, simple explanations given one step at a time help reduce anxiety and improve cooperation in clients with cognitive impairment. Finishing quickly without explanation can increase fear. Asking the client to recall the routine may be frustrating if memory is impaired. Repeated correction is not therapeutic and can worsen distress.

1993. During evening rounds in a long-term care facility, a resident with dementia insists, "My mother is waiting outside to pick me up." The resident is tearful but not aggressive. Which response is best?

- A.** Your mother died many years ago, so you need to stop saying that
- B.** You seem worried about your mother. Tell me about her while we sit together
- C.** If you go back to your room right now, I will call her for you
- D.** You are in a nursing facility now, and your family already explained that

Correct Answer: B

The best response validates the resident's feelings and redirects to a calm, supportive conversation. This approach decreases distress without arguing about facts the resident may not be able to process. Choices A and D use reality correction in a confrontational way, which often increases anxiety. Choice C is misleading and promises an action that may not be appropriate.

1994. A client with mild dementia lives with an adult daughter. The daughter says, "He keeps putting things in strange places and then accuses me of stealing." Which intervention is most appropriate for the nurse to suggest first?

- A.** Reason with him each time and demand an apology for the accusation
- B.** Hide valuable objects so he cannot touch them at all
- C.** Create a consistent place for commonly used items and keep the routine the same
- D.** Test his memory daily by asking where he placed each object

Correct Answer: C

A structured environment with consistent locations for commonly used items supports remaining function and can reduce misplacing objects and related distress. Arguing or demanding an apology is unlikely to help and may escalate conflict. Hiding everything may increase suspicion. Testing memory daily can increase frustration rather than improve function.

1995. A hospitalized client with known dementia becomes increasingly agitated and tries to climb out of bed. The practical nurse notes the client is pulling at the gown, frowning, and repeatedly saying, "Too tight, too tight." Which action is best first?

- A. Apply physical restraints to prevent a fall
- B. Administer a sedative as soon as possible
- C. Assess for an unmet physical need causing the behavior
- D. Move the client to a busy nurses' station for constant stimulation

Correct Answer: C

Behavior in a client with cognitive impairment often communicates an unmet need such as pain, discomfort, toileting needs, or restrictive clothing. The statement "Too tight" and the nonverbal signs point first to physical discomfort, so assessment is the priority. Restraints and sedatives do not address the cause and may worsen agitation or increase risk. A busy area may add overstimulation.

Stress management

1996. A client hospitalized for a new diagnosis of diabetes says, "I feel overwhelmed. I don't think I can handle all of this." Which response by the nurse is best?

- A. "You need to stay positive and focus on getting better."
- B. "Many people learn to manage diabetes, so you probably will too."
- C. "Tell me which part of managing diabetes feels hardest right now."
- D. "Let's wait to talk until you have had time to calm down."

Correct Answer: C

Choice C encourages the client to identify the main stressor, which helps the nurse provide focused support and problem-solving. Choice A gives false reassurance, choice B is general reassurance without exploring the client's concern, and choice D delays support when the client is asking for help now.

1997. A nurse is teaching a client about healthy ways to manage stress. Which client statement shows understanding of effective coping?

- A. "I will ignore stressful situations so I don't get upset."
- B. "I will identify what is causing my stress and make a plan to address it."
- C. "I will keep my feelings to myself so I do not burden others."
- D. "I will wait until my stress is severe before asking for help."

Correct Answer: B

Identifying the stressor and planning a response reflects problem-focused coping and effective stress management. Ignoring stress, suppressing feelings, or delaying help are less effective and may worsen stress over time.

1998. A client becomes increasingly tense while waiting for test results. Which intervention should the nurse implement to promote immediate stress reduction?

- A. Guide the client through slow deep-breathing exercises
- B. Explain that test results are often delayed
- C. Ask the provider to prescribe a sedative
- D. Advise the client to avoid discussing the situation

Correct Answer: A

Slow deep breathing is a simple, immediate, nonpharmacologic technique that can reduce physiologic signs of stress. Choice B may provide information but does not directly reduce tension, choice C is not the first step, and choice D discourages coping through communication.

1999. A nurse is planning care for a client who reports ongoing work stress, insomnia, and irritability. Which client outcome best indicates effective stress management?

- A. The client states that stress is completely eliminated
- B. The client identifies two personal stressors and one coping strategy for each
- C. The client agrees that work is the cause of all current problems
- D. The client asks the nurse to make decisions about lifestyle changes

Correct Answer: B

A realistic, measurable outcome is that the client can identify stressors and match them with coping strategies. Stress usually cannot be completely eliminated, so choice A is unrealistic. Choice C oversimplifies the situation, and choice D reflects dependence rather than improved coping.

2000. A client caring for an ill spouse says, "I have stopped meeting friends because there is no time." Which nursing response best supports stress management?

- A.** "Staying busy can keep your mind off stress."
- B.** "You should focus only on your spouse's needs right now."
- C.** "What support systems or respite options do you have available?"
- D.** "At least you know you are doing everything possible."

Correct Answer: C

Exploring support systems and respite options addresses caregiver strain and helps the client identify practical resources to reduce stress. Choices A, B, and D minimize the caregiver's needs or reinforce unhealthy self-neglect.